The Complete NCMHCE Study Guide

The Next-Move Method: Clinical Reasoning, Diagnostic Reference & Practice for the Case-Simulation Exam

Second Edition

GA Integrated Therapeutic Perspectives LLC

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For every licensed counselor candidate who has stared at a table of diagnoses and wondered: “But how do I actually use this on the exam?”

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Preface

If you are holding this book, you have already done the hard part. You sat through graduate seminars that ran long past the end of a full workday. You wrote case conceptualizations at midnight, drove across town for practicum and internship hours, and sat with clients whose pain you carried home with you. You finished the coursework, logged the supervised hours, and kept going on the nights when stopping would have been easier. None of that was small, and none of it was guaranteed. All of it was you choosing, again and again, to become the counselor your future clients need. That road was not easy — and it was never meant to be. Becoming a clinician asks something real of a person: patience, humility, and the willingness to keep showing up for other people's hardest moments. You have already proven you can do that. The credential ahead of you is not the beginning of your competence; it is the formal recognition of competence you have been building for years. So let us be clear about what the NCMHCE actually is. It is not a verdict on whether you belong in this profession — you answered that question long ago, with your training and your hours. It is one last structured hurdle: a test of clinical reasoning under exam conditions. It rewards a specific way of thinking — gathering information before acting, prioritizing safety, and matching each intervention to the client's stage — and that way of thinking can be learned, practiced, and mastered. That is exactly what this book is built to do. Everything ahead is designed to meet you where you are and carry you the rest of the way: the reasoning behind each diagnosis, the traps the exam sets and why they work, and worked cases that show how a strong clinician moves through a scenario. And when you are ready to pressure-test that reasoning under realistic conditions, PassReady Prep™ (www.passreadyprep.com) puts

everything you learn here into full-length case simulations built for the exam's current format — the natural next step after the pages that follow. Read at your own pace. Trust your preparation. And when the doubt shows up — because it will — remember how far you have already come to arrive here.

You have carried yourself through the hardest years of this journey. This is the final stretch. Let's finish it together.

— KekeTheLPC

How to Use This Book

This guide was built on a single observation: most NCMHCE prep resources teach you what to know but not how to think. They give you tables. The exam gives you a human being in crisis — and four options that all look plausible. The chapters ahead do something different.

The Next-Move Method. The NCMHCE doesn't test only what you know. You also have to know the next move. Every item, whatever it looks like, asks the same question: given this client, this moment, and this much information, what is the counselor's next move? This book trains that decision as a repeatable routine — read the last line first, scan for safety, ask what changed, pick the matching reasoning tool, eliminate the options that are too early, too late, out of scope, or absolute, then choose. The Priority Ladder settles what comes first; the rest of the Clinical Reasoning Toolkit settles how to think about it. Run the routine on every practice question until it is automatic, and the exam's four plausible options stop looking equal.

Every diagnosis family gets:

- A narrative overview that explains why the exam tests this family and what the examiners are actually looking for
- Diagnosis reference tables (retained as quick-reference tools)
- "The clinical picture" — prose describing how these disorders actually present inside a case simulation
- Differential discussion — not just "rule out X," but why X loses and how to detect the deciding feature
- Assessment tools in context — when you reach for each instrument and what the score tells you
- "What the exam rewards / What it punishes" — scored priorities and the traps, each with a reason

- Worked mini-cases — a vignette, the reasoning chain, and why each distractor loses
- Practice questions with expanded rationales

Four units cover the work the 2027 exam added: Counselor Self-Awareness and Professional Development (in Chapter 1), Continuity of Care (end of Chapter 6), Practice Management and Compliance (in Chapter 25), and Chapter 28, Indirect Client Care. Their headings carry NBCC's 2027 task codes, such as 3B.1 or 5.4, so you can match each section to the work task it teaches (National Board for Certified Counselors [NBCC], 2026).

How to Think Through Each Question

No single framework answers every NCMHCE question. The next section, **The Clinical Reasoning Toolkit**, shows how to match each question type to the right reasoning tool. It covers the Priority Ladder, case conceptualization, risk formulation, crisis intervention, developmental fit, functional analysis, the window of tolerance, cultural frameworks, boundary checks, and test-taking strategy for the multi-section case format. Read it before Part One, and come back to it whenever a practice question trips you up. The **Reasoning Skills Workbook**, just before the practice exams, has eleven drills that train each tool on its own, plus a ten-week study plan and a log for tracking your practice exam results.

How This Book Maps to the NCMHCE

The NCMHCE evaluates clinical reasoning through case simulation, not isolated recall. For exams given on or after July 1, 2027, NBCC's examination specifications organize the exam around 101 work tasks grouped into six domains, and every exam form follows the same domain weights (NBCC, 2026). The table below shows where each domain is taught and where it is practiced in this book.

> **Testing before July 1, 2027?** You will take the current NCMHCE: 11 cases built on NBCC's 2023 content outline, with 100 scored items. The clinical content in this book applies to both versions. The format and domain map in this edition are the 2027 ones. Check nbcc.org to confirm which version you will take.

NBCC Domain (2027)	Exam Share*	Taught In	Practiced In
1 Professional Development & Counselor Self-Awareness	15%	Ch 1 The Helping Relationship, including Core Counseling Attributes and Counselor Self-Awareness and Professional Development; Appendix A Multicultural Counseling Competencies; Appendix B Research Methods and Evidence-Based Practice	Countertransference, consultation, supervision, competence, and boundary questions in all three practice exams

NBCC Domain (2027)	Exam Share*	Taught In	Practiced In
2 Intake & Assessment	18%	Ch 5 Intake, Assessment & the Mental Status Exam; the diagnostic chapters (Ch 8–24); Ch 19 Crisis, Risk & Safety; Assessment Tools Compendium	The intake section of all 30 practice cases; consent, risk, screening, and diagnosis questions
3 Treatment Planning & Continuity of Care	15%	Ch 6 Treatment Planning, including Continuity of Care; the first-line treatment sections of Ch 8–24 and Ch 26; Ch 7 Worked Case	Goal-setting, sequencing, level-of-care, referral, relapse-prevention, discharge, and termination questions
4 Provision of Counseling Interventions	20%	Ch 2 Theories & Orientations, Ch 3 Skills & Techniques, Ch 4 Group Work, Ch 19 Crisis, Risk & Safety, Ch 26 Couples & Family, Ch 7 Worked Case	In-session response and intervention questions in all three practice exams and the supplemental bank
5 Indirect Client Care	12%	Ch 28 Indirect Client Care, including the psychopharmacology reference	Documentation, care-coordination, medication-effect, referral-list, and interpreter questions
6 Legal & Ethical Compliance	20%	Ch 25 Ethical and Legal Decision Points, including Practice Management and Compliance; Ch 19 Crisis, Risk & Safety; Appendix C Administrative and Professional Practice Cases	Ethics, confidentiality, telemental health, records, billing, and court questions

*Weights are the share of items in each domain, from NBCC's NCMHCE examination specifications effective July 1, 2027 (NBCC, 2026). The six domains total 100%.

Section 7 content guidance. The 2027 specifications no longer have an Areas of Clinical Focus domain. Instead, Section 7 lists the presenting concerns, theoretical frameworks, and tools and assessments counselors reported seeing or using most, and says candidates "may benefit from reviewing" them (NBCC, 2026). Most of the presenting concerns are covered in Chapter 27 and the diagnostic chapters (Ch 8–24), most of the frameworks in Chapters 2, 4, and 26, and the assessment instruments in Chapter 5 and the Assessment Tools Compendium.

How the Book Is Organized

Part One — Foundations, Theories & Skills (Ch 1–7). Builds the clinical reasoning infrastructure the exam scores: the helping relationship, core counseling attributes, and counselor self-awareness; counseling theories; intervention skills; group work; intake and assessment (including the mental status exam); treatment planning and continuity of care; and a fully worked case walkthrough that models the Priority Ladder decision sequence from intake through disposition.

Part Two — Diagnostic Reference (Ch 8–28). 103 diagnoses across 17 diagnostic families, followed by ethics and practice management, couples and family counseling, the nondiagnostic areas of clinical focus (Chapter 27), and indirect client care (Chapter 28). Each diagnostic chapter presents the clinical picture, key differentials, discriminating features, first-line treatments, and counselor scope — exactly the information the exam tests through case presentation.

Part Three — Clinical Quick Reference. Three rapid-review tools: the Defense Mechanisms Quick Reference, the Cognitive Distortions Quick Reference, and the Assessment Tools Compendium.

Back Matter and Appendices. The Alphabetical Diagnosis Index and references, followed by supplemental practice questions with an answer key, a guide to the

2027 NCMHCE format, Appendices A–D (multicultural counseling competencies, research methods and evidence-based practice, administrative and professional practice case simulations, and test-day logistics), and the Reasoning Skills Workbook.

Practice Exams. Three full-length NCMHCE-format exams: 30 cases and 390 questions across the three exams (10 cases and 130 questions per exam, mirroring the 2027 NCMHCE structure), with complete answer keys, rationales, and identified clinical errors. Each exam is built to the six 2027 domain weights, with every domain within about three percentage points of its target, and together the three exams draw cases from across the diagnostic chapters. **Each case opens with the client's working diagnosis.** The 2027 specifications do not say whether exam cases open with a diagnosis. Treat the one given here as a working diagnosis to confirm or revise, not a settled fact: the 2027 specifications include selecting the diagnosis from the most recent DSM or ICD (task 2B.12), so expect items that ask you to confirm, revise, add, or select a diagnosis, along with the assessment sequence, prioritized interventions, and evidence-based treatment decisions that follow (NBCC, 2026). Work each exam under timed conditions before consulting the key.

The Clinical Reasoning Toolkit

The NCMHCE does not reward memorized facts in isolation. It rewards applying the right way of thinking to the question in front of you. This section collects every reasoning tool the book uses into one place. Learn them here, then watch for them in every chapter, worked case, and practice exam.

The toolkit has four parts:

1. **Match the question to the tool**: a quick way to decide what kind of question you are facing
2. **Thinking through the case**: tools for understanding the client
3. **Staying in bounds**: checks that keep the counselor's actions ethical and within scope
4. **Taking the test**: tools for reading answer choices and the multi-section case format

A one-page routine at the end puts them together. That routine is the Next-Move Method, and every tool in this section feeds it.

Part 1 — Match the Question to the Tool

No single framework answers every NCMHCE question. Start by reading the stem's final line and deciding what kind of question it is. The keyword usually tells you:

If the stem asks...	Use this tool	Taught in
What to do **FIRST**, **NEXT**, or **most immediately**	The Priority Ladder	This section; Chapters 5–7, 19

If the stem asks…	Use this tool	Taught in
The **most likely** diagnosis, or what to **rule out**	Differential diagnosis: medical and substance causes first, then duration and timing, then the discriminating feature	Chapters 8–24
How to **understand** why the problem developed and persists	Case conceptualization (the 4 Ps)	This section; Chapter 6
How serious the **risk** is and what response it calls for	Risk formulation	This section; Chapter 19
How to respond to an acute **crisis**	The seven-stage crisis intervention model	This section; Chapter 19
The **most appropriate intervention** for this client now	Stage of change: match the intervention to the client's readiness	Chapters 6 and 11
Whether an intervention fits a **child, adolescent, or older adult**	Developmental fit	This section; Chapters 14 and 27
What **triggers** or **maintains** a behavior	Functional analysis (antecedent, behavior, consequence)	This section
Which **treatment** or approach is indicated	Evidence-based fit for the diagnosis, the client's preferences and culture, and the counselor's scope	Chapters 2 and 8–24
When to **process** trauma	Phase-oriented care and the window of tolerance	This section; Chapters 10 and 22
The appropriate **level of care**	The least restrictive level that is safe; the American Society of Addiction Medicine (ASAM) dimensions for substance use	Chapter 5

If the stem asks...	Use this tool	Taught in
What the counselor should do about an **ethical or legal** conflict	The ethical decision-making model, plus the scope and consent checks	Chapter 25; this section
The **best response** or most **empathic** reply	Core counseling attributes: the answer that is accurate, genuine, warm, and nonjudgmental	Chapters 1 and 3
How to handle a presentation that may be **culturally shaped**	Cultural formulation and the ADDRESSING framework	Chapter 5; this section; Appendix A
How to work with a **couple or family**	Systems thinking: the pattern is the client; screen privately for partner violence before joint sessions	Chapter 26

Two rules connect the tools:

- **Safety overrides every tool.** If the stem contains a safety signal, address it first, whatever the question type.
- **The ladder settles ties.** When two answers are both defensible under another tool, pick the one that comes first on the Priority Ladder.

The Priority Ladder

The Priority Ladder answers sequencing questions: what comes first when several needs compete.

The frame around every rung: informed consent and the limits of confidentiality come before any clinical work begins. Ethics is not a final step; it shapes every rung. Mandated reporting sits inside rung 1, and scope of competence sits inside rung 5.

Rung	Priority	Clinical Translation
1	Imminent safety	Suicidality, homicidality, abuse or neglect (including mandated reports), danger from intoxication or withdrawal, medical emergency — nothing else happens first
2	Medical / substance rule-outs	Could this be organic? Could substances explain it? Rule them out before diagnosing
3	Stabilization	Acute symptom relief, grounding, crisis reduction, basic needs
4	Engagement and assessment	Build the alliance *while* clarifying the picture — the client feels heard as history, mental status, measures, and collateral information (with consent) are gathered
5	Evidence-based treatment	The right approach for the diagnosis, matched to stage of change and culture and within the counselor's competence
6	Maintenance and relapse prevention	Skills consolidation, early warning signs, supports, planned termination

When two answers both seem correct on a sequencing question, ask which rung is lower-numbered. The lower rung wins.

Part 2 — Thinking Through the Case

Case Conceptualization: The 4 Ps

A diagnosis names the problem. A case conceptualization explains it — why this client, why now, and why it has not resolved — and that explanation points to treatment. A widely used structure organizes the presenting problem around four kinds of factors (Macneil et al., 2012):

Factor	Question it answers	What it points to in treatment
Predisposing	What made this client vulnerable? (genetics, temperament, early experiences, trauma)	Long-term themes; psychoeducation about vulnerability
Precipitating	What triggered the problem now? (loss, transition, stressor)	Focus of early sessions; what changed
Perpetuating	What keeps it going? (avoidance, substance use, negative thinking, reinforcement, family patterns)	**The main target of intervention**
Protective	What strengths and supports can be used? (relationships, skills, faith, motivation)	Resources the plan builds on

How the exam uses it: When a stem asks what the counselor should target or address, the best answer usually aims at a **perpetuating** factor. The past cannot be changed, and the trigger is often over. What maintains the problem today is where treatment has leverage.

Example: A client developed panic after a car accident (precipitating). She has an anxious temperament (predisposing). She now avoids driving and carries a water bottle and phone “in case” (perpetuating: avoidance and safety behaviors). She has a supportive partner (protective).

Treatment targets the avoidance and safety behaviors through exposure.

Risk Formulation

A risk assessment gathers facts. A risk formulation turns them into a judgment about level of risk and the response it calls for. SAMHSA's Suicide Assessment Five-Step Evaluation and Triage (SAFE-T) lays out the sequence (Substance Abuse and Mental Health Services Administration [SAMHSA], 2009):

1. **Identify risk factors**: prior attempts, psychiatric diagnoses, substance use, hopelessness, recent losses, access to lethal means
2. **Identify protective factors**: reasons for living, social support, responsibility to children, engagement in care
3. **Conduct a suicide inquiry**: ideation, plan, behaviors (including preparation and rehearsal), and intent
4. **Determine risk level and intervention**: match low, moderate, or high risk to the response
5. **Document**: the assessment, the risk level, the rationale, and the plan

Two distinctions sharpen the judgment:

- **Static vs. dynamic factors.** Static factors, such as a prior attempt or a family history of suicide, raise baseline risk but cannot change. Dynamic factors, such as current hopelessness, intoxication, agitation, or access to means, change over time and are what intervention can address.
- **Risk level drives the response.**

Risk level	Typical picture	Typical response
Low	Thoughts of death without plan, intent, or behavior; strong protective factors	Outpatient care, safety planning, follow-up

Risk level	Typical picture	Typical response
Moderate	Ideation with a plan but no intent or behavior; mixed risk and protective factors	Safety plan with means restriction, more frequent contact, consider a higher level of care, involve supports with consent
High	Plan with intent, preparatory behavior, or a recent attempt; few protective factors	Do not leave the client alone; arrange emergency evaluation or hospitalization

How the exam uses it: Answers that jump straight to hospitalization for passive ideation are too restrictive. Answers that respond to a plan with intent using a "no-harm contract" are unsafe. The correct response matches the level of risk. Chapter 19 covers the full risk and safety content.

The Seven-Stage Crisis Intervention Model

A crisis is a state of acute disequilibrium in which a person's usual coping has been overwhelmed. It is not limited to suicide risk: assault, disaster, sudden loss, a frightening diagnosis, or a family emergency can all precipitate one. Roberts's seven-stage model gives a clear sequence (Roberts, 2005):

1. **Assess lethality and safety**: psychosocial and imminent-danger assessment
2. **Make psychological contact** and rapidly establish rapport
3. **Identify the major problems**, including what precipitated the crisis
4. **Deal with feelings and emotions**: active listening and validation
5. **Generate and explore alternatives**: draw on the client's past coping and resources
6. **Implement an action plan**: concrete, short-term steps

7. **Follow up**: check on the plan, and schedule booster contact

How the exam uses it: Crisis stems test order. Safety comes before rapport-building exploration. Feelings are acknowledged before problem-solving. Long-term insight work does not belong in the acute phase. An answer that offers insight-oriented therapy or a detailed history in the middle of a crisis is premature.

Developmental Fit

NBCC's 2027 specifications list skill in aligning interventions with client need and identity, and in choosing instruments that fit the client's developmental considerations (NBCC, 2026). Before choosing an answer, ask: *Does this intervention fit this client's age and developmental stage?*

Client	What fits	What does not
Young child	Play-based and expressive approaches, caregiver involvement, concrete language	Abstract talk therapy, cognitive restructuring that relies on adult reasoning
Adolescent	Respect for growing autonomy, peer and identity themes, the minor's assent alongside parental consent (American Counseling Association [ACA], 2014, Standard A. 2.d)	Treating the adolescent as a small adult or excluding them from decisions
Adult with intellectual disability	Simplified language, visual supports, involvement of supports with consent	Assuming incapacity or bypassing the client
Older adult	Attention to sensory and cognitive changes, life review, medical coordination	Assuming depression or memory loss is "normal aging"

Developmental fit also covers informed consent: consent must be communicated in a developmentally appropriate way (ACA, 2014, Standard A.2.c).

Functional Analysis: Antecedent, Behavior, Consequence

Functional analysis asks what a behavior *does* for the person. Behavior is shaped by what comes before it (the **antecedent** or trigger) and by what follows it (the **consequence**, which reinforces it). Once the function is clear, the intervention replaces the behavior with one that serves the same need more safely. Chain analysis in dialectical behavior therapy (DBT) applies the same logic to self-harm and other target behaviors (Linehan, 2015).

Example: A teenager self-harms after arguments with her mother (antecedent). The cutting brings rapid relief from overwhelming emotion (consequence: negative reinforcement). The function is emotion regulation. Treatment builds distress tolerance and emotion regulation skills that serve the same function, and it addresses the conflict that triggers it.

How the exam uses it: For behaviors like self-harm, substance use, avoidance, school refusal, or tantrums, the rewarded answer identifies the function before trying to stop the behavior. Punishing or simply prohibiting a behavior without understanding its function is a common distractor.

The Window of Tolerance

The window of tolerance is the zone of arousal in which a person can think, feel, and function (Siegel, 1999). Outside it, a person is either **hyperaroused** (panic, agitation, rage, flashbacks) or **hypoaroused** (numbness, shutdown, dissociation). Learning and trauma processing happen only inside the window.

How the exam uses it: If a client in session becomes flooded or dissociates, the rewarded answer is to pause the content and use grounding or regulation to bring

them back into the window — not to push ahead with processing. The window of tolerance explains *why* phase-oriented trauma treatment puts stabilization before processing (see Chapters 10 and 22).

The ADDRESSING Framework

Hays's ADDRESSING framework is a checklist of cultural influences to consider with every client, so that the counselor does not reduce a client to one visible identity (Hays, 2022):

Letter	Cultural influence
A	Age and generational influences
D	Developmental disabilities
D	Disabilities acquired later in life
R	Religion and spiritual orientation
E	Ethnic and racial identity
S	Socioeconomic status
S	Sexual orientation
I	Indigenous heritage
N	National origin
G	Gender

How the exam uses it: The counselor asks what these identities mean to the client rather than assuming. The framework also prompts self-reflection: the counselor considers their own identities and where they hold privilege relative to the client. Pair it with the Cultural Formulation Interview (Chapter 5) and the multicultural competencies (Appendix A).

Part 3 — Staying in Bounds

The Scope Check: Is This the Counselor's Decision?

Before choosing an answer, ask whether the action belongs to the counselor's role. Counselors practice only

within the boundaries of their competence (ACA, 2014, Standard C.2.a).

Outside the counselor's scope	What the counselor does instead
Prescribing, adjusting, or advising on medication	Refers for a medication evaluation and coordinates with the prescriber
Diagnosing or ruling out medical conditions	Refers for a medical evaluation and coordinates care
Giving legal advice	Refers to legal resources while supporting the client emotionally
Treating a specialty problem without training	Consults, gets supervised training, or refers

How the exam uses it: An answer can describe good care that is simply not the counselor's to give, such as recommending a specific medication or dose. Eliminate it, and choose the answer that refers and coordinates.

The Release Check: Is There Consent to Share?

Before contacting anyone about a client — a family member, a physician, a school, a previous counselor — ask:

1. **Is there a signed release** authorizing this specific communication?
2. **Is the client a minor or an adult without capacity?** If so, what consent or assent is required, and who holds the legal right (ACA, 2014, Standard B.5)?
3. **Does an exception apply?** Imminent danger, mandated reporting, or a court order can permit or require disclosure without consent (ACA, 2014, Standard B.2.a).

How the exam uses it: "Call the client's physician to coordinate care" is correct only once the client has authorized it, or when an exception applies. The rewarded answer often *first obtains the release*. Under HIPAA, releasing protected health information generally requires written authorization (U.S. Department of Health and Human Services [HHS], 2025).

Least Intrusive Effective Action

Among the answers that would actually work, choose the one that preserves the most client autonomy and privacy. The principle applies well beyond level of care:

- **Confidentiality**: disclose only the minimum necessary, and only to the people who need it
- **Level of care**: the least restrictive setting that is safe (Chapter 5)
- **Confrontation**: explore before challenging, and challenge before directing
- **Family involvement**: invite the client to decide who is involved, unless safety requires otherwise
- **Referral**: coordinate care rather than transferring the client when the counselor can remain competently involved

The limit: least intrusive never means unsafe. When safety requires a more intrusive action, such as breaking confidentiality to protect life, safety wins (Priority Ladder, rung 1).

Part 4 — Taking the Test

Why Wrong Answers Are Wrong

Almost every wrong answer on the NCMHCE is wrong in one of a few recognizable ways. Name the flaw, and you can eliminate the option with confidence.

Distractor type	What it looks like	Example
Right action, wrong time	A good intervention offered before safety, assessment, or engagement	Starting exposure therapy before assessing a new suicide statement
Outside scope	Good care that is not the counselor's role	Recommending that the client increase their antidepressant dose

Distractor type	What it looks like	Example
Too passive	Ignores a risk or clinical signal	"Continue to monitor" after a client discloses a plan with intent
Too aggressive	An overreaction to the level of risk or need	Hospitalizing a client with passive ideation and strong protective factors
Counselor-centered	Serves the counselor's comfort, values, or curiosity	Referring out because of a values conflict; disclosing personal reactions
Mismatched to the client	Ignores stage of change, development, or culture	An action plan for a precontemplative client; a cognitive technique with a young child
Absolute language	*always, never, only, must, guarantee, immediately terminate*	"Assure the client that everything shared will always remain confidential"

Absolute wording is rarely right because clinical and ethical standards almost always have exceptions. The main exceptions are genuine bright lines, such as the prohibition on sexual relationships with clients.

What Changed Since the Last Section?

Each NCMHCE case is presented in three sections: an initial intake summary and two later counseling sessions (NBCC, 2026). Each new session usually adds information that matters. The 2027 specifications describe review by exam half: you can review questions before you submit each half, and once a half is submitted you cannot return to it. They do not say whether earlier sections of a case lock as you move forward, so confirm the navigation rules in the current candidate handbook, and practice as if you cannot go back.

At the start of every section, ask **what is new**:

- **New safety information?** A new statement about suicide, violence, abuse, or substance use moves the answer back to rung 1.
- **New diagnostic information?** A history of hypomania, a medical finding, a substance pattern, or a symptom duration can revise the working diagnosis.
- **A change in progress?** Improvement suggests continuing or stepping down care. No improvement calls for reviewing the plan (Chapter 6).
- **A change in the relationship?** A rupture, a missed session, or a new request calls for attention to the alliance.
- **A new person or system?** A family member, a school, a court, or another provider raises consent, confidentiality, and coordination questions.

How the exam uses it: The best answer in Session Two often responds to the new detail, not to the original intake. Candidates who keep answering from the intake picture miss the shift the case is testing.

The Next-Move Method: A Routine for Every Question

1. **Read the last line of the stem first.** Identify the question type (Part 1).
2. **Scan for safety.** If there is a safety signal, rung 1 decides the answer.
3. **Ask what changed** since the last section.
4. **Apply the matching tool**: the ladder, differential diagnosis, the 4 Ps, risk formulation, the crisis model, stage of change, developmental fit, functional analysis, the window of tolerance, culture, or core attributes.
5. **Eliminate by flaw**: too early, out of scope, too passive, too aggressive, counselor-centered, mismatched, or absolute.

6. **Run the boundary checks**: scope, release, and least intrusive effective action.
7. **Choose**, and move on. Answer each section as if you cannot return to it; the current candidate handbook gives the exact review rules.

Worked Example — Using Several Tools Together

Intake: Lily, 16, is referred by her school counselor for "anger problems." Her grades have dropped since her parents separated four months ago. She lives with her mother.

Session Two: Lily says she has been "scratching my arms to calm down" after fights with her mother, and she mentions that "sometimes it feels like it'd be easier to not be here."

The question: What should the counselor do FIRST?

- **Question type:** "FIRST" means the Priority Ladder.
- **What changed:** Session Two adds self-harm and a statement that may be passive suicidal ideation. That is new safety information, so rung 1 applies.
- **Risk formulation:** Conduct a suicide inquiry (ideation, plan, behavior, intent), and weigh risk factors (self-harm, family disruption) against protective factors.
- **Developmental fit and release check:** As a minor, Lily's safety plan involves her parent or guardian as the law and the level of risk require. The counselor explains the limits of confidentiality in a developmentally appropriate way.
- **Functional analysis (later, once she is safe):** The scratching follows conflict and brings relief, so its function is emotion regulation. Treatment will build regulation skills and address the family conflict.
- **4 Ps (for the plan):** Predisposing — to be assessed. Precipitating — the separation. Perpetuating — conflict with her mother and self-harm as a coping strategy.

Protective — the school counselor's involvement and her engagement.

- **Distractors to eliminate:** starting anger-management skills (right action, wrong time); telling her the scratching will stay confidential (absolute and unsafe); hospitalizing her immediately before assessing risk (too aggressive).

Answer: Assess suicide risk directly and thoroughly now, then develop a safety plan that involves her parent as appropriate.

Check Your Understanding — Reasoning Tools

Toolkit — perpetuating factor. A client with social anxiety was bullied in middle school, lost his job last month, and now avoids all phone calls and job interviews. Counseling should MOST directly target:

A. The middle school bullying that first shaped his fear

B. The job loss that triggered the current episode

C. The avoidance of calls and interviews

D. His temperament, including lifelong shyness

Answer: C. Avoidance is the perpetuating factor that maintains the anxiety today, and it is where treatment has leverage (Macneil et al., 2012). A and D are predisposing factors; B is the precipitating event.

Toolkit — what changed. At intake, a client was diagnosed with major depressive disorder and started cognitive behavioral therapy (CBT). In Session Two, she reports that last spring she went four days without sleep, felt "on top of the world," and spent $6,000. The counselor's BEST next step is:

A. Continue CBT for depression as planned, keeping the focus on her current depressive symptoms.

B. Praise her for sharing a positive memory, reinforcing her openness about good times in her life.

C. Revise the working diagnosis to consider bipolar disorder and coordinate a psychiatric evaluation.

D. Increase the session frequency to twice weekly so the depressive symptoms receive closer attention.

Answer: C. New information about a likely past hypomanic episode changes the diagnosis and the treatment. Antidepressant treatment alone can carry risk in bipolar disorder. A answers from the intake picture. B misses the clinical significance. D does not address the new diagnostic question.

Toolkit — distractor type. A client with moderate risk (a suicide plan but no intent, with strong protective factors) asks whether the counselor will "call the police." Which answer choice is the TOO AGGRESSIVE distractor?

A. Collaboratively develop a safety plan with means restriction and schedule a follow-up within days.

B. Explain the limits of confidentiality honestly and answer the client's question directly.

C. Arrange immediate involuntary hospitalization.

D. Involve a trusted support person with the client's consent to strengthen the safety plan.

Answer: C. Involuntary hospitalization overreacts to moderate risk with strong protective factors. A, B, and D are proportionate, collaborative responses consistent with a risk formulation (SAMHSA, 2009).

PART ONE: FOUNDATIONS, THEORIES & SKILLS

These chapters turn the reference tables into teachable, testable knowledge.

Chapter 1 — The Helping Relationship

Overview

Why Alliance Outpredicts Technique

Here is something the research has been saying for decades that exam writers clearly believe: the therapeutic alliance is the single strongest predictor of outcome, accounting for more variance in client improvement than the specific technique used. This is not a soft finding. Meta-analyses consistently show that alliance explains 5–7 times more outcome variance than treatment modality. The exam knows this. You will see questions that tempt you to pick an intervention — confront the distortion, assign the homework, interpret the dream — when the clinically correct answer is first to repair the relationship. Understanding why the alliance works also helps you spot alliance items in disguise. The exam rarely says "the alliance is rupturing." Instead, it shows a client who "seems disengaged," "pushes back on the homework," "misses appointments," or "says the counseling isn't helping." Your job is to recognize these as alliance signals and respond accordingly — before you pivot to technique.

The mechanism of rupture and repair. Ruptures happen in two forms. A withdrawal rupture looks like silence, vagueness, sudden compliance, or emotional flatness — the client has pulled back without saying so. A confrontation rupture looks like direct disagreement, irritation, or challenging the counselor's approach. Both forms carry the same message: "Something in our relationship isn't working." The research-supported repair sequence is: (1) notice the shift, (2) name it without blame ("I want to check in — it feels like something shifted for you just now"), (3) explore the client's experience, (4) acknow-

ledge your contribution if relevant, and (5) re-contract on the working arrangement. The exam rewards noticing and naming over plowing ahead.

How the exam disguises alliance items. Alliance questions frequently appear as "what do you do next" items in which the client has just expressed doubt, frustration, or disconnection. The trap is to pick the answer that moves the work forward — psychoeducation, a new technique, a referral. The correct answer almost always validates the client's experience first. "Validate before you challenge" is the cardinal rule (see Chapter 3). It applies to alliance ruptures just as strongly as to skill use.

Rogers's Core Conditions

Carl Rogers proposed six conditions that are necessary and sufficient for therapeutic change, three of them provided by the counselor (Rogers, 1957). The NCMHCE tests these by showing you a counselor response and asking you to identify the condition it demonstrates — or by asking which condition is missing in a scenario where the client isn't improving. Unconditional Positive Regard (UPR) is not approval. It is the commitment to value the client's personhood regardless of behavior, choices, or disclosures. A client who describes something the counselor finds morally troubling still receives warmth and nonjudgment. Exam tip: UPR is violated when a counselor's response implies the client should feel differently, or when the counselor's tone signals disapproval. Dialogue example:

> Client: "I've been drinking every night. I know it's bad, but honestly? It's the only time I feel like myself."
> Counselor (UPR): "That's worth understanding — what does feeling like yourself mean to you in those moments?"
> (Not: "Have you considered what alcohol is doing to your body?")

Empathy (accurate empathic understanding) requires the counselor to track not just content but underlying emo-

tion — and to communicate that tracking back to the client in a way the client recognizes. It is not sympathy ("I feel sorry for you") and not agreement ("You're right, your boss is terrible"). It is: "I am with you in your experience." Dialogue example:

> Client: "I've tried to talk to my husband about this a hundred times. He just doesn't get it."
> Counselor (empathy): "There's something exhausting about explaining yourself over and over to someone you love and still feeling unseen."

Congruence (Genuineness) means the counselor's internal experience, outward expression, and words align. The counselor does not perform warmth while feeling bored. Congruence does not mean self-disclosure of every feeling — it means authenticity, which sometimes includes saying "I'm noticing something in our conversation I want to name."

Bordin's Working Alliance: Bond, Goals, Tasks

Bordin's model breaks the alliance into three components that must all be intact for treatment to proceed (Bordin, 1979). Knowing which component is strained tells you what kind of repair to attempt.

Component	What It Means	When It's Strained
Bond	Emotional connection, trust, felt sense of being valued	Client feels judged, misunderstood, or unsafe
Goals	Mutual agreement on what treatment is for	Client doesn't believe the stated goals are their goals
Tasks	Agreement that the methods used will reach the goals	Client questions why they are doing an exercise or technique

Rupture-repair vignette: A client diagnosed with depression has been doing behavioral activation for three weeks (Martell et al., 2022; Safran & Muran, 2000). She arrives and says, "I don't see why I keep filling out these activity

logs. My problem isn't that I don't do things — it's that nothing feels worth doing." This is a task rupture. The repair: explore her understanding of the technique, validate that the tasks can feel meaningless when motivation is low, and collaboratively renegotiate whether behavioral activation is the right fit or whether the approach needs adjustment. Do not defend the technique. Do not skip straight to reassigning the homework.

Stages of Counseling

The exam tests whether you know the clinical priorities at each stage — not just the names.

Exploration / Initial Stage: Safety assessment comes first. Alliance-building is the primary task. Do not move to goal-setting before the client feels heard and the presenting problem is adequately explored. Counselors who jump to treatment planning in the first session lose points.

Working / Middle Stage: The treatment plan is active. Resistance appears here (see Chapter 3). Ruptures are most likely here because the work gets harder. The counselor moves between support and challenge.

Termination Stage: The exam tests two things: (1) how far in advance to raise termination (early enough for processing, not so early it disrupts the work), and (2) how to respond when clients regress near termination. Regression near termination is normal and expected — it is not a sign of failure and does not warrant extending treatment indefinitely. The correct response is to name the pattern, normalize it, and continue the planned termination.

Cultural Responsiveness

Cultural humility vs. cultural competence (Norcross & Wampold, 2018). Cultural competence implies mastery — that you can learn enough about a group to work with any of its members. Cultural humility recognizes that the client is the expert on their own cultural experience. The exam favors humility: the counselor asks, learns, and remains curious rather than applying assumptions. Saying

"I know that in many Latino families, mental health treatment carries stigma" is competence language. Saying "I'd like to understand how your family thinks about getting help — can you tell me about that?" is humility language. Humility wins on the exam. Managing countertransference. Countertransference is the counselor's emotional reaction to the client — rooted in the counselor's own history, biases, or unresolved material. It is not inherently harmful; unexamined countertransference is. The exam expects you to: recognize it (name the feeling), contain it (don't act on it impulsively), use it (mine it for clinical information about the client's impact on others), and consult or seek supervision when it feels unmanageable. Never act out countertransference by changing the treatment contract, providing dual-relationship services, or disclosing at length about your own experience.

Core Counseling Attributes: Hidden in Every Case

NBCC's 2027 specifications list core counselor attributes — empathy, unconditional positive regard, authenticity, genuineness, and congruence — as knowledge and skills within Provision of Counseling Interventions, the 20% domain, and give counselor self-awareness its own subdomain (1B) in Domain 1 (NBCC, 2026). You will never see a question labeled "attributes." Instead, they show up inside ordinary case questions: the answer choices differ in *how* the counselor responds, and only one response sounds like a counselor who is self-aware, genuine, warm, and culturally humble. When two options are both clinically defensible, the attribute is usually the tiebreaker.

The research behind these attributes is strong. Meta-analyses of the relationship conditions Rogers (1957) described find that empathy, positive regard, and congruence each predict client outcome independent of the treatment model used (Elliott et al., 2018; Farber et al., 2018; Kolden et al., 2018). The exam treats these as clinical skills, not personality traits — they can be done well

or badly, and the difference is observable in a single response.

Self-Awareness and Countertransference Management

Self-awareness means knowing your own values, reactions, cultural lens, and emotional triggers well enough that they inform your work instead of steering it. Countertransference is the counselor's emotional reaction to the client, shaped by the counselor's own history. It is not a failure — it is information. The failure is acting it out: rescuing, withdrawing, arguing, over-disclosing, or quietly steering a client toward the choice the counselor would make.

Research on countertransference management points to the same protective factors the exam rewards: self-insight, self-integration (a stable sense of one's own identity), anxiety management, empathy, and the ability to conceptualize what is happening (J. A. Hayes et al., 2018).

How this shows up on the exam: A stem describes the counselor noticing irritation, protectiveness, boredom, or a strong wish to "fix" the client. The best answer almost always involves recognizing the reaction and taking it to supervision or consultation — not disclosing it to the client, not ignoring it, and not transferring the client at the first sign of discomfort. Referral is appropriate only when the counselor genuinely lacks competence, never solely because of a values conflict (ACA, 2014, Standard A.11.b).

Congruence and Genuineness

Congruence means the counselor's outward behavior matches their inner experience. A congruent counselor does not perform calm while feeling alarmed, and does not pretend to agree. Congruence is not total transparency. It means the counselor is not faking, and when something about the relationship needs to be named, the counselor can name it honestly and with care.

Congruent	Incongruent
"I notice I'm feeling a little lost in what we're working on today — can we slow down?"	Nodding along while privately confused
"I'm concerned about what you just told me, and I want to understand it better."	"That's fine" when it clearly isn't
Owning a mistake: "I think I moved too quickly last week."	Defending the plan when the client signals a rupture

Empathic Attunement

Empathy is the accurate understanding of the client's inner world *and* the communication of that understanding back to the client (Elliott et al., 2018). Both halves are required. Accurate but uncommunicated empathy does nothing for the client; communicated but inaccurate "empathy" feels like being misread.

Empathic responses on the exam fall on a rough ladder:

Level	Example (client: "My sister got the promotion and everyone's celebrating her.")
Subtractive — misses or minimizes	"Well, your turn will come."
Interchangeable — reflects the surface	"Your sister was promoted and your family is celebrating."
Additive — reflects the feeling and its meaning	"It sounds like it hurts to watch everyone celebrate her while you're still waiting for your own moment."

The exam rewards additive empathy that stays close to what the client actually said. An "insightful" interpretation that jumps far beyond the client's words is not additive — it is premature.

Positive Regard and the Nonjudgmental Stance

Unconditional positive regard is warmth and respect for the client as a person, regardless of their behavior (Farber et al., 2018). It is not approval of everything the client does. A counselor can hold positive regard for a cli-

ent who has relapsed, lied, or harmed someone while still being clear about safety, legal duties, and consequences.

The nonjudgmental stance shows up in word choice. Avoid questions that start with "why" when they imply blame ("Why did you do that?"). Avoid evaluative adjectives ("That was a bad decision"). Prefer curious, open questions ("What was happening for you right before you decided to drink?").

Multicultural Sensitivity and Cultural Humility

Cultural humility is an interpersonal stance that is other-oriented rather than self-focused: the counselor is open to the client's cultural identity, aware of their own limitations, and willing to learn from the client (Hook et al., 2013). Clients who perceive their counselor as culturally humble report stronger working alliances. The exam contrasts humility with two common errors:

- **Assuming**: applying group-level knowledge to an individual ("Latino families always…").
- **Avoiding**: never raising culture, race, religion, or identity at all, even when it is clearly relevant.

The rewarded move is to *ask*: invite the client to describe what their identity, family, faith, or community means for the problem at hand. Appendix A covers the multicultural and social justice competencies in detail.

Foundational Listening Skills as Attributes

Attending, open questions, paraphrasing, reflecting feeling, and summarizing are covered as techniques in Chapter 3. Here they are evaluated as *dispositions*: does the counselor stay with the client's agenda, tolerate silence, and resist the urge to advise? A counselor who reflects before recommending, and summarizes before shifting topics, is demonstrating the attribute the exam rewards.

What the Exam Rewards (and Punishes) — Core Attributes

Rewards: Recognizing and managing countertransference through supervision or consultation. Additive empathic reflections that stay close to the client's words. Naming a rupture honestly and nondefensively. Asking about the client's cultural identity rather than assuming. Warmth toward the person while holding firm on safety and legal duties.

Punishes: Disclosing the counselor's own reactions to meet the counselor's needs. Referring a client out because of the counselor's personal values. Subtractive responses ("At least…", "Your turn will come"). Advice-giving before the client feels understood. "Why" questions that imply blame. Avoiding cultural topics the client has raised.

Worked Mini-Case — Core Attributes

Vignette: Jordan, a 29-year-old client, has been in counseling for six sessions for anxiety. Today he mentions, almost in passing, that he left his church after coming out as gay and that his parents no longer speak to him. The counselor, who is active in a faith community, notices a tightening in her chest and an urge to change the subject back to his panic symptoms.

Reasoning chain:

Step 1 — Notice the reaction: The counselor's discomfort is countertransference. It is information, not a directive.

Step 2 — Stay with the client: The rewarded in-session response is an empathic reflection of the loss Jordan just disclosed — "It sounds like coming out cost you your church and, for now, your relationship with your parents." Changing the subject would communicate that the topic is unwelcome.

Step 3 — Manage the reaction outside the session: The counselor seeks supervision or consultation to examine her reaction so it does not shape the work.

Step 4 — Do not refer on values grounds: Transferring Jordan because the topic touches the counselor's faith would violate ACA Standard A.11.b (ACA, 2014). Referral would be appropriate only if she lacked the competence to treat his anxiety, and she does not.

Check Your Understanding — Core Attributes

Attributes — best response. A client says, "Everyone keeps telling me I should be over my divorce by now." The response that BEST demonstrates empathic attunement is:

A. "Divorce recovery usually takes about a year, so honestly, you're right on schedule with healing."

B. "It sounds like on top of the grief itself, you're feeling judged for how long it's taking."

C. "Who is telling you that — your family, your friends, or people at work?"

D. "Let's focus on what you can control this week, like your sleep and daily routine."

Answer: B. B is an additive reflection: it captures both the feeling (judged) and its meaning (grief has its own timeline). A offers reassurance and a norm the client did not ask for. C shifts to fact-gathering. D redirects before the client feels understood.

Attributes — countertransference. A counselor notices strong irritation every time a particular client arrives late. The MOST appropriate first step is:

A. Tell the client how the lateness makes the counselor feel, so the client understands its impact.

B. Refer the client to a colleague to protect the therapeutic relationship before the irritation begins to affect the work.

C. Explore the reaction in supervision and consider what it may reveal about the client's relational patterns.

D. Ignore the reaction, because the counselor's feelings are not clinically relevant to treatment.

Answer: C. Countertransference is managed through supervision and used as clinical information (J. A. Hayes et al., 2018). A discloses to meet the counselor's needs. B abandons the client over a manageable reaction. D ignores useful data and risks acting out.

Counselor Self-Awareness and Professional Development

2027 task map: 1A.1 Seek consultation from a colleague for assistance with a specific skill or modality · 1A.2 Seek supervision from a counseling supervisor · 1A.3 Complete continuing education · 1A.4 Develop knowledge of empirically supported counseling interventions or approaches · 1A.5 Engage in advocacy for the client and the counseling profession · 1A.6 Engage in training related to counseling ethics · 1A.7 Engage in training related to ethical and legal use of artificial intelligence (AI) · 1A.8 Engage in training related to working with different cultures or populations · 1B.1 Address your compassion fatigue (e.g., burnout) potential · 1B.2 Assess your exposure to vicarious trauma · 1B.3 Assess your need for specialized training and/or certification · 1B.4 Assess your strengths and limitations as a counselor · 1B.5 Assess your suitability to work with a specific client based on their needs and sociocultural considerations · 1B.6 Utilize self-regulation techniques · 1B.7 Determine the client-counselor suitability (e.g., counselor has the skills and experience needed, availability, no conflict of interest) · 1B.8 Determine your competency for administering assessments · 1B.9 Maintain appropriate boundaries · 1B.10 Practice self-care · 1B.11 Practice self-reflection

The Counselor Is Part of the Treatment

Every other task assumes a counselor who is competent, regulated, honest about limits, and current. This unit keeps that counselor in working order. **Self-awareness** (1B) looks inward: Am I right for this client? Am I worn down? Is this reaction about the client or about me? **Professional development** (1A) looks outward: Whom do I consult? What training do I need? What does the research support?

Items here share one shape. A counselor hits a limit, a reaction, or a gap. The wrong options **ignore** the problem, **overcorrect** it (usually with an abrupt referral), or **push it onto the client** (through disclosure or by making the client the teacher). The keyed answer **names the problem and takes a proportionate step to fix it** while care continues.

Part 1 — Who Owns It

Owner	What it owns
ACA Code of Ethics (2014), Section C	**C.2.a** boundaries of competence · **C.2.b** new specialty areas · **C.2.d** monitor effectiveness · **C.2.e** consultations on ethical obligations · **C.2.f** continuing education · **C.2.g** impairment · **C.3.a** accurate advertising · **C.4.a** accurate representation · **C.4.b** credentials · **C.7.a–c** scientific basis, innovation, harmful practices · Section C introduction: **self-care** · **C.1** knowledge of and compliance with standards

Owner	What it owns
ACA Code (2014), Sections A and B	**A.1.a** primary responsibility · **A.1.b** records · **A.2.c** developmental and cultural sensitivity · **A.4.b** personal values · **A.5.d** friends or family members · **A.6, A.6.a, A.6.e** boundaries, previous relationships, nonprofessional relationships · **A.7.a–b** advocacy and consent · **A.10.e** bartering · **A.10.f** receiving gifts · **A.11.a, A.11.b, A.11.d** competence- and values-based referral, transfer · **A.12** abandonment · **B.1.c** confidentiality · **B.7.a–b** case consultation
ACA Code (2014), Sections D, E, F, H, I	**D.1.c** interdisciplinary teamwork · **D.2.a** consultant competency · **E.2.a** limits of assessment competence · **E.2.b** appropriate use · **E.7** conditions of administration · **F.1.a** supervision and client welfare · **F.2.a** supervisor preparation · **F.4.a** informed consent for supervision · **H.1.a** technology knowledge and competency · **H.2.a** technology informed consent · **I.1.b** ethical decision making · **I.1.c** conflicts between ethics and laws
Christina Maslach & Susan Jackson (1981)	**Burnout** and the **Maslach Burnout Inventory (MBI)**: emotional exhaustion, depersonalization (cynicism), reduced personal accomplishment
Charles Figley (1995)	**Compassion fatigue** (secondary traumatic stress): the “cost of caring”
Lisa McCann & Laurie Anne Pearlman (1990)	**Vicarious traumatization**: cumulative change in the counselor’s beliefs about safety, trust, esteem, intimacy, and control

Owner	What it owns
Beth Hudnall Stamm (2010)	**Compassion satisfaction**; the **Professional Quality of Life Scale (ProQOL)**: compassion satisfaction, burnout, secondary traumatic stress
Thomas Skovholt (2001)	**The resilient practitioner**; the **cycle of caring**; balancing other-care with self-care
John Norcross & Gary VandenBos (2018)	Evidence-based **self-care strategies** for therapists; personal therapy as one strategy
Sigmund Freud (1910); **Hayes, Gelso, & Hummel** (2011)	**Countertransference** (Freud) and its **management** (Hayes et al.)
Donald Schön (1983)	**Reflective practice**: reflection-in-action and reflection-on-action
Walfish et al. (2012)	**Self-assessment bias**: clinicians overrate their own effectiveness
James Gross (1998)	**Emotion regulation**: cognitive reappraisal vs. expressive suppression
Gerald Caplan (1970)	**Mental health consultation**: a coordinate, nonhierarchical relationship
Bernard & Goodyear (2019)	Definition of **clinical supervision**: evaluative, hierarchical, extends over time
Janine Bernard (1979)	**Discrimination model** of supervision
Stoltenberg & Delworth (1987)	**Integrated Developmental Model (IDM)** of supervision
Elizabeth Holloway (1995)	**Systems approach** to supervision
Ladany et al. (1996)	**Supervisee nondisclosure**
Sackett et al. (1996); **APA Presidential Task Force on Evidence-Based Practice** (2006)	**Evidence-based practice**: best available research + clinical expertise + client characteristics, culture, and preferences

Owner	What it owns
Dianne Chambless & Steven Hollon (1998)	**Empirically supported therapy criteria**: efficacious and specific, efficacious, possibly efficacious
Jacobson & Truax (1991)	**Clinical significance**; the Reliable Change Index
Paul Meehl (1954)	**Clinical vs. statistical (actuarial) prediction**
Jeffrey Mitchell (1983)	**Critical incident stress debriefing (CISD)**
Rose, Bisson, Churchill, & Wessely (Cochrane review, 2002)	Evidence that **single-session debriefing** does not prevent PTSD and may harm; **WHO** (2013) guidance against routine debriefing
Lewis, Arnold, House, & Toporek (2002); **Toporek & Daniels** (2018)	**ACA Advocacy Competencies**: two dimensions, six domains
Karen Kitchener (1984)	**Moral principles** (autonomy, nonmaleficence, beneficence, justice, fidelity); intuitive vs. critical-evaluative levels
ACA Code Preamble	Six principles, adding **veracity**
Forester-Miller & Davis (1996)	**Practitioner's seven-step** decision-making model
Vilia Tarvydas (1998)	**Integrative decision-making model**: four themes, four stages
Meara, Schmidt, & Day (1996)	**Principle ethics vs. virtue ethics**
Mandatory vs. aspirational ethics	Minimum compliance vs. the highest standard
Automation bias; algorithm aversion (Dietvorst et al., 2015)	Two opposite errors with AI and decision aids
HIPAA	**Business associate agreement** (45 CFR 164.504(e)); **Safe Harbor** de-identification
ADA Title III	**Effective communication** and auxiliary aids, including interpreters (28 CFR 36.303)
Hook et al. (2013)	**Cultural humility**

Owner	What it owns
Ratts et al. (2016)	**Multicultural and Social Justice Counseling Competencies (MSJCC)**

Part 2 — Strengths, Limitations, and Specialized Training (1B.3, 1B.4)

Boundaries of Competence

ACA C.2.a limits practice to the counselor's education, training, supervised experience, credentials, and appropriate professional experience. Self-assessment of strengths and limitations is how you apply C.2.a to a real client.

Error	What it looks like	Why it fails
Overestimating competence	"My general training and a good alliance cover everything."	Ignores needs outside the counselor's scope, such as medication management or acute crisis care
Underestimating competence	"This diagnosis needs a specialist. I'll decline."	Counselors treat many serious conditions within a collaborative plan; refusal is not required and can delay care

The keyed answer **names what the counselor offers and where others must fill in**: for a serious mood disorder, the counselor may own relapse-prevention planning and mood monitoring while psychiatric and crisis services handle medication and acute episodes. A clear role plus clear referrals.

When the gap is too large to close with consultation or training in time, **ACA A.11.a** directs referral. When the only "gap" is a values conflict, referral is not allowed (Part 3).

Checking Self-Assessment Against Data

Walfish et al. (2012) found that clinicians, on average, rate their own effectiveness well above their peers', and almost none rate themselves below average. **Felt confidence and years in practice are poor gauges of skill** with a specific disorder. **ACA C.2.d** (monitor effectiveness) directs counselors to keep checking their efficacy, including through peer supervision. So **compare self-ratings with outside data**: outcome measures and supervisor or peer feedback. A client's or relative's opinion cannot carry that weight.

Reflective Practice and the Near Miss

Donald Schön (1983) described the **reflective practitioner**. **Reflection-in-action** happens during the session ("I'm pushing and she's pulling back"). **Reflection-on-action** happens afterward ("Why did I miss that?").

When a counselor finds a near miss, such as almost skipping a screen for past hypomania in a client presenting with depression, reflection-on-action gives the keyed answer: **name the gap and change the routine** (screen at every intake). Wrong answers **overcorrect** (transfer the client; one fixable gap is not incompetence), **externalize** (blame the client's self-report; asking is the counselor's job), or **lean on a strength** (rapport cannot replace a systematic screen).

Specialized Training and Certification

ACA C.2.b (new specialty areas): counselors practice in new areas only after appropriate education, training, and supervised experience, and protect clients while skills develop. Assessing your need for specialized training starts from the **caseload**: what do these clients need, what training do I have, and what credential rules apply where I practice?

Idea	Rule
License ≠ competence in every specialty	A license sets a general floor. It does not confer competence with every population, problem, or instrument
Training is the basis for practice; a credential is a claim	Many modality certifications, such as EMDR certification, are **voluntary** advanced credentials. A counselor with completed training and its required consultation may use the method but may not **claim** the certification. **ACA C.4.a** (accurate representation) and **C. 4.b** (claim only credentials that are current and in good standing) govern the claim; **C. 3.a** (accurate advertising) governs what goes on a website
A certificate is only as good as its content	Collecting any available certificate does not build competence unless its training matches what clients need

Some roles do require specific preparation. **ACA F.2.a** (supervisor preparation): counselors are trained in supervision methods before offering supervision, and many state boards add their own supervisor requirements. Years of practice do not substitute. Verify your board's requirements first; the supervision agreement (**F.4.a**) comes after.

Part 3 — Suitability for a Specific Client (1B.5, 1B.7)

The Three Suitability Tests

The 2027 outline names them directly. A counselor is suitable for a client when the counselor has:

1. **The skills and experience the client needs** (or a realistic path to them through consultation and supervision)

2. **Availability** at the level the client needs (session frequency, crisis access, scheduling)
3. **No conflict of interest** (no competing role or tie)

Not suitability tests: shared background or interests (rapport, not competence), the fee the insurer pays, or whether the client already has insight (limited insight is often a target of treatment).

Values Conflicts: Serve, Don't Refer

- **ACA A.4.b (personal values):** counselors are aware of and avoid imposing their own values, attitudes, beliefs, and behaviors. They **seek training** in areas where they risk imposing values, especially when their values conflict with the client's goals.
- **ACA A.11.b (values within termination and referral):** counselors **do not refer** prospective or current clients **based solely on the counselor's personally held values**.

When a counselor's personal or religious beliefs conflict with a client's identity, relationship, or goals, the keyed answer is to **serve the client and seek training, consultation, or supervision as needed.** Sharing the beliefs so the client can "decide" is value imposition by self-disclosure. Referral for **lack of competence** (A.11.a) is permitted; referral for values is not.

Conflicts of Interest and Prior Contact

ACA A.6 (managing and maintaining boundaries) and **A.6.a** (previous relationships) direct counselors to weigh the risks and benefits of accepting a client with whom they have had another relationship. **A.5.d** prohibits counseling friends or family members when the counselor cannot remain objective. **A.6.e** asks counselors to avoid nonprofessional relationships with former clients and their families when the interaction is potentially harmful.

Prior or current tie	Typical judgment
The counselor holds **ongoing evaluative power** over the client (instructor, employer, grader)	A real conflict of interest: discuss it and arrange a referral. Having someone else grade leaves the power relationship in place; asking the client to drop the class shifts the problem onto the client
A **single brief transaction** years ago with no contact since	Rarely a conflict. Acknowledge it, weigh the risk, document the decision. Automatic referral overreacts; silence risks trust if the client recognizes the counselor later
A close friend or relative	Not suitable when objectivity is compromised (A.5.d)

Sociocultural and Language Fit

ACA A.2.c (developmental and cultural sensitivity) directs counselors to communicate in ways clients understand and, when needed, to arrange **a qualified interpreter or translator**. For a Deaf client who uses American Sign Language, offer a qualified interpreter or a signing counselor, with the client's input. **ADA Title III** requires **effective communication** through auxiliary aids, including qualified interpreters (**28 CFR 36.303**). Written notes rarely meet that bar for counseling; a family member as interpreter compromises confidentiality, accuracy, and candor.

Cultural difference alone is not a reason to refer. The counselor builds the needed competence (Part 14).

Part 4 — Competence to Administer Assessments (1B.8)

ACA E.2.a (limits of competence): counselors use only the testing and assessment services for which they have been trained and are competent. **ACA E.2.b** (appropriate use): counselors are responsible for the appropriate application, scoring, interpretation, and use of instruments,

whether they score them themselves or use technology or other services. **ACA E.7** asks that instruments be administered under the conditions established in their standardization.

Four traps recur:

Trap	Why it fails
"I read the manual."	Reading instructions is not training. Structured rating scales and risk instruments need instrument-specific training to give reliable scores
"The publisher let me buy it."	Publisher **qualification levels** set a minimum to purchase. They do not show skill with a given instrument
"Front-desk staff can give it."	The counselor stays responsible for who administers the instrument (E.2.b). Untrained administration threatens validity and privacy
"I was trained years ago."	Skill decays without use. Before relying on new scores, refresh the manual and scoring rules and **co-rate with a trained colleague** to confirm **interrater reliability**

Also wrong: interpreting by impression instead of norms, changing the administration format (parents completing a clinician-rated scale), skipping assessment, or leaving scores out of the record (**A.1.b**, records). If structured assessment is needed before training is complete, **consult or refer to a qualified evaluator** in the meantime.

Part 5 — Maintaining Appropriate Boundaries (1B.9)

The ethics chapter covers boundary crossings and violations in full. Two patterns carry most items here.

Gifts (ACA A.10.f). The Code does not ban gifts. Counselors weigh **the therapeutic relationship, the monet-**

ary value of the gift, the client's motivation, and the counselor's motivation. A young child's drawing usually fits: accept it warmly and explore what it means. Refusing can feel like rejection to a child. Matching it with a gift of equal value turns the relationship into a social exchange. Displaying it with the child's name reveals that the child is a client.

Services and second roles. Free labor for the counselor's business creates an obligation and a second relationship; paying the client makes the client a vendor. Decline warmly and explore what the offer means. When parents ask the counselor to take a second paid role, such as tutoring their child, decline the role (**A.6**), explain why, and **refer** to someone who can meet the need. Changing the location or billing leaves the dual role intact; the clinical goal stays on the counseling agenda.

Keep **bartering** (**A.10.e**) separate: it is an exchange of goods or services for counseling that the **client requests**, that is an accepted practice among professionals in the community, that is not exploitative, and that is documented in a written contract. A thank-you offer of free labor is not bartering.

Part 6 — The Costs of Caring: Burnout, Compassion Fatigue, and Vicarious Trauma (1B.1, 1B.2)

The exam asks you to tell three conditions apart, match each to its owner, and choose the response that fits its cause.

Burnout (Maslach & Jackson)

Christina Maslach defined **burnout** as a prolonged response to **chronic interpersonal stressors on the job**. With **Susan Jackson** she built the **Maslach Burnout Inventory (MBI; Maslach & Jackson, 1981)**, which measures three dimensions:

Dimension	What it looks like in a counselor
Emotional exhaustion	Drained, depleted, nothing left to give at the end of the day
Depersonalization (later called **cynicism**)	Detached, callous, or irritable toward clients; seeing them as cases or labels
Reduced personal accomplishment (later called **inefficacy**)	Feeling ineffective; doubting the work makes a difference

Two exam points: **cynicism toward clients is a burnout dimension, not healthy professional distance.** Because burnout grows from **work conditions**, the fix targets the **workload** as well as the counselor. A week off eases exhaustion briefly, but burnout returns if the caseload is unchanged. The keyed answer adjusts the caseload with the supervisor **and** builds in recovery.

Compassion Fatigue (Figley)

Charles Figley (1995) described **compassion fatigue** as the "cost of caring" for people who are suffering. He used it interchangeably with **secondary traumatic stress (STS)**: stress reactions that come from **helping or wanting to help a traumatized or suffering person**. STS can resemble PTSD: intrusive thoughts, avoidance, and arousal, picked up from clients' accounts rather than from direct exposure. Compassion fatigue can come on **quickly**, after a run of intense cases, where burnout builds slowly.

Early signs include **numbness, going through the motions**, dreading certain sessions, and reduced empathy. The exam rewards acting **at the early sign**: self-care, consultation or supervision, and caseload limits **now**, not "once the semester ends" or "once clients complain."

Vicarious Traumatization (McCann & Pearlman)

Lisa McCann and Laurie Anne Pearlman (1990) described **vicarious traumatization**: a **cumulative transformation** of the counselor's inner experience that comes from **empathic engagement with clients' trauma** over time. It is grounded in their **constructivist self-development theory**, and its signature is **disrupted cognitive schemas**, the counselor's core beliefs about **safety, trust, esteem, intimacy, and control**. Intrusive imagery from clients' stories can also occur.

Signs include intrusive images, dread before sessions, hypervigilance **outside work**, loss of trust, and reading ordinary client events as hidden danger. A counselor whose sense of danger **outruns the client's data** should **check the reaction against what the client reports and assess the counselor's own trauma exposure**. Treating it as "clinical intuition" and adding sessions over-treats the client.

Compassion Satisfaction and the ProQOL (Stamm)

Beth Hudnall Stamm added the positive side. **Compassion satisfaction** is the pleasure and meaning a helper draws from doing the work well. In her model, **compassion fatigue** has two parts, **burnout** and **secondary traumatic stress**, and compassion satisfaction can offset them.

Her **Professional Quality of Life Scale (ProQOL)** measures all three: **compassion satisfaction, burnout, and secondary traumatic stress**. It is a standard tool for assessing the effects of trauma exposure systematically, and its results belong in supervision. Compassion satisfaction comes from **meaningful work, not more work**; adding clients to rebuild purpose deepens fatigue.

Keep Them Apart

	Burnout	Compassion fatigue / STS	Vicarious traumatization
Owner	Maslach (& Jackson, MBI)	Figley	McCann & Pearlman
Source	Chronic work demands and conditions	Caring for suffering or traumatized clients	Cumulative empathic exposure to client trauma
Onset	Gradual	Can be sudden	Cumulative
Signature	Exhaustion, cynicism, inefficacy	PTSD-like intrusion, avoidance, arousal; numbness	Changed beliefs about safety, trust, esteem, intimacy, control
Trauma-specific?	No	Mostly (trauma or suffering)	Yes

Distractors: **countertransference** (Part 7) comes from the counselor's own history, not trauma exposure. The **Beck Depression Inventory-II (BDI-II; Beck et al., 1996)** measures depression severity. The **MBI** measures burnout only.

When Fatigue Touches Clinical Judgment

ACA C.2.g (impairment): counselors **monitor themselves for signs of impairment** and refrain from offering services **when impaired**, seeking help and limiting or suspending work if needed. The duty is to act **early**, not wait for impairment to show.

The most tested scenario: a drained counselor feels a pull to accept a client's "I'd never do it" without further questions. Keyed sequence: **name the fatigue, finish the safety work now, then seek consultation and support.** Accepting the denial lets exhaustion lower the bar; handing the client off mid-session without assessing risk is neglect (**A.12**).

General order: notice and name the sign → finish urgent client work → assess (ProQOL) and take it to supervision → change the conditions (caseload, recovery, personal counseling) → C.2.g steps if impairment is present or likely. Wrong answers dismiss the sign as "part of the job," tell the client, ask the client to share less trauma detail, or transfer the client at once.

Part 7 — Self-Care, Self-Regulation, and Self-Reflection (1B.6, 1B.10, 1B.11)

Self-Care Is Ongoing, Preventive, and Ethical

The **introduction to Section C** of the ACA Code says counselors engage in **self-care activities** to maintain their emotional, physical, mental, and spiritual well-being so they can meet their professional responsibilities. Self-care is part of ethical practice, not a crisis response.

Thomas Skovholt (*The Resilient Practitioner*) frames counseling as **one-way caring**: the counselor gives care without expecting it back, so resilience requires **balancing other-care with self-care**. His **cycle of caring** describes each helping relationship in four phases, repeated across a career:

1. **Empathic attachment** (forming the bond)
2. **Active involvement** (doing the work)
3. **Felt separation** (ending well and letting go)
4. **Re-creation** (restoring energy before the next attachment)

Skipping felt separation and re-creation carries each client into the next. The resilient practitioner builds renewal into ordinary weeks.

John Norcross (with VandenBos, *Leaving It at the Office*) lists research-informed self-care strategies, such as minding the body, nurturing relationships, setting boundaries, and **personal therapy**, which is **one** strategy, not the whole of self-care.

Three wrong views: self-care only when stress peaks (it is **ongoing and preventive**), only after impairment (self-care **prevents** it), or only personal therapy.

Self-Regulation in Session

Alarm, a rising pulse, irritation, the urge to escape: **self-regulation** keeps these from driving the response. The rule: **regulate first, then respond.** Techniques include **paced breathing** with a slow exhale, slowing speech, noticing body cues, **grounding** for a few minutes between sessions, and **cognitive reappraisal** ("This anger is fear, not about me").

James Gross separates **cognitive reappraisal**, which changes the emotion, from **expressive suppression**, which hides the expression but leaves physiological arousal high and impairs memory. "Push it down and start the next session" is suppression, not regulation.

Situation	Keyed response	Common wrong answers
Client speeds up and grows irritable; counselor's pulse rises	Slow own breathing, keep a calm, measured tone	Match the client's volume (escalates); tell the client to stop (reactive enactment); end the session abruptly
Client says quietly that her family would be better off without her; counselor feels alarm	Slow breathing and speech, then **ask directly about suicide**	Reassure and move on; change the topic until the counselor calms; call emergency services because the counselor feels alarmed (risk comes from assessment, not the counselor's emotion)

Situation	Keyed response	Common wrong answers
Counselor is still keyed up after a hostile session; next client is waiting	Brief grounding pause, then begin	Vent to front-desk staff or in a hallway (breaches confidentiality, ACA Section B, **B.1.c**); suppress and start at once; cancel the hostile client's next appointments

Asking directly about suicide does not increase risk; regulation makes the direct question possible.

Self-Reflection and Countertransference

Freud introduced **countertransference** as the analyst's unconscious reactions to the patient, an obstacle to overcome. Contemporary views treat it as reactions shaped by the counselor's own history and needs, useful once examined. **Hayes, Gelso, and colleagues** name the qualities that **manage** it: **self-insight, self-integration, anxiety management, empathy, and conceptualizing skills**.

Self-reflection keeps countertransference from steering care: What am I feeling? Where does it come from? How is it shaping what I do?

- **A belief leaks into a reaction.** The counselor feels impatient and thinks the client "isn't trying." Reflect on the belief and **bracket** it. Telling the client to try harder voices the bias as feedback.
- **Personal history drives a reaction.** A counselor who struggled in school feels fiercely protective of a child client and angry at her teacher. Reflect on the link; anger is not evidence about the school, and long self-disclosure centers the counselor's story.
- **A session went sideways.** The counselor kept pushing suggestions while the client tried to describe a worry. Reflect on what drove the push (a need to show

progress) and plan to listen more. Labeling the client "resistant" externalizes; a long apology is self-focus.

Across all three, **reflection comes before any referral**, and supervision is the place to bring what reflection turns up.

Part 8 — Consultation and Supervision (1A.1, 1A.2)

Two Different Relationships

	Consultation	Clinical supervision
Owner	**Caplan**: a **coordinate, nonhierarchical** relationship; the consultee is **free to accept or reject** the advice	**Bernard & Goodyear**: an intervention by a more senior member of the profession that is **evaluative, hierarchical, and extends over time**
Who is responsible for the client	The treating counselor keeps **primary responsibility** (A.1.a)	Responsibility is **shared**; the supervisor monitors client welfare (**F.1.a**) and can be held vicariously liable
Evaluation	None	Yes, including gatekeeping
Typical use	A licensed counselor needs help with a specific skill or modality	A counselor working toward licensure, or anyone in a supervised role

Remember Caplan's core point: **no authority, free to accept or reject**. Referral differs again: it **moves the client's care** to someone else (**A.11.d**, appropriate transfer of services).

Consultation Done Right

When. A counselor with some training but little experience in a modality (behavioral activation, imaginal expos-

ure, sleep restriction) consults an experienced colleague **while building skill** (**ACA C.2.b**). Online summaries mean learning the method on the client; referral is premature with training underway.

Whom. Match the consultant to the **specific skill gap**. Seniority or friendship is not expertise. A physician coordinates medical care (**D.1.c**, interdisciplinary teamwork) but is not the source for a counseling technique. **ACA D.2.a** (consultant competency) is the bar the consultant should meet.

What to share. ACA B.7.b: no identifying information without consent unless unavoidable, and **only what the consultation needs**. **B.7.a** limits consultation discussion to professional purposes. A full chart is too much; a public forum can identify a client even without a name; dinner conversation is not consultation. A release is **not** needed for a de-identified consultation.

What to do with the advice. Weigh it against your own assessment, keep responsibility (A.1.a), and document it. Advice does not transfer responsibility.

When the plan stalls. One consultation is not standing approval. If outcome scores stay flat, **return to the consultant with the data** (C.2.d).

Supervision Done Right

A supervisee unsure about a risk judgment, or shaken by a client's content, brings **both the clinical question and the counselor's own reactions** to supervision, so the supervisor can monitor client welfare (F.1.a). A supervisee facing an unfamiliar diagnosis reviews the plan with the **assigned supervisor before using it**. A peer's approval cannot replace the supervisor, and reporting after the fact removes the oversight.

Ladany et al. (1996) found that most supervisees withhold something, such as mistakes or negative reactions, often to manage impressions. Hiding doubts defeats su-

pervision; referring a treatable client out to escape discomfort shifts the problem onto the client.

Model	Owner	Signature
Discrimination model	**Bernard**	Three supervisor **roles** (teacher, counselor, consultant) crossed with three **foci** (intervention, conceptualization, personalization)
Integrated Developmental Model (IDM)	**Stoltenberg & Delworth**	Supervisees move through **levels**, tracked by **self/other awareness, motivation, and autonomy**
Systems approach to supervision (SAS)	**Holloway**	The **supervisory relationship** at the center, surrounded by contextual factors

Part 9 — Continuing Education (1A.3)

ACA C.2.f (continuing education): counselors pursue continuing education to stay current with scientific and professional information, maintain competence in the skills they use, and stay informed about best practices with diverse populations. It owns the **duty to stay current**; do not confuse it with **C.2.a** (practice within competence) or **C.2.g** (impairment). Four rules:

1. **Plan from needs.** Choose CE from a review of the **current caseload and practice gaps**. Comfort-zone topics, free courses, or the most hours in one sitting satisfy a number, not the purpose.
2. **Check the provider first.** Credentialing bodies accept hours only from **approved or qualifying providers**. Price says nothing about whether hours count.
3. **Check each credential separately.** Each board and certifying body sets its **own** hours, required topics (often ethics), and accepted providers. The issuing

jurisdiction's rules apply, not the state where you trained. Check your state board and certifying body for current requirements.

4. **Keep records; report honestly.** Retain **certificates of completion** (provider, date, hours) for **audits**; recall or a catalog listing proves nothing. If short at renewal, **contact the board about its options before attesting**. A false attestation is misrepresentation (**C.4.a**); uncredited reading does not count.

Part 10 — Empirically Supported Interventions (1A.4)

Evidence-Based Practice Has Three Legs

David Sackett and colleagues (1996) defined evidence-based medicine as integrating the **best available research evidence** with **clinical expertise** and **patient values**. The **APA Presidential Task Force on Evidence-Based Practice (2006)** carried this into psychology: **best available research + clinical expertise + client characteristics, culture, and preferences**.

Two errors cut off a leg: **theoretical allegiance** (loyalty to an orientation, a known bias) and **ranking by effect size alone** (research only, no client). Client preference shapes the choice among supported options but does not **define** what is supported.

ACA C.7.a (scientific basis for treatment) directs counselors to use techniques grounded in theory and/or with an empirical or scientific foundation. **C.7.b** (development and innovation) requires explaining the risks, benefits, and ethical considerations of developing or innovative techniques and minimizing harm. **C.7.c** (harmful practices) bars techniques when **substantial evidence suggests harm**, even if requested.

Chambless & Hollon (1998): What Counts as Empirically Supported

Dianne Chambless and Steven Hollon (1998) set criteria for **empirically supported therapies (ESTs)**. Evidence must come from **randomized controlled trials** (or rigorous controlled single-case experiments), with a **treatment manual**, a **clearly defined population**, reliable and valid outcome measures, and appropriate analysis. They defined three levels:

Level	Meaning
Efficacious and specific	Shown **superior to a placebo or to an alternative bona fide treatment** in at least two independent research settings
Efficacious	Shown **superior to no treatment** (e.g., wait-list), or equivalent to an already established treatment, in at least two independent research settings
Possibly efficacious	Support from only **one study or one research group**, without independent replication

A related, earlier scheme from the APA Division 12 task force (Chambless and colleagues) used "**well-established**" and "**probably efficacious**." If an item says "efficacious and specific," the owner is Chambless & Hollon (1998).

Efficacy vs. Effectiveness vs. Clinical Significance

Term	Question	Typical evidence
Efficacy	Does it work under **ideal research conditions**?	Tightly controlled RCTs with screened samples, manuals, and trained therapists

Term	Question	Typical evidence
Effectiveness	Does it work in **routine practice** with typical, often comorbid clients?	Studies in community clinics with few exclusions
Clinical significance (**Jacobson & Truax, 1991**)	Is **an individual's** change meaningful?	Reliable Change Index; movement into the functional range

Beating placebo in controlled trials with screened participants shows **efficacy**, not yet effectiveness in your setting, and not that any one client changed meaningfully.

Weighing the Evidence

A common **hierarchy of evidence**, strongest first: systematic reviews and meta-analyses (and **practice guidelines** built on them) → individual RCTs → quasi-experimental and observational studies → case series and **single case reports** → expert opinion, testimonials, and anecdote. Then apply three checks:

- **External validity (generalizability).** Rigorous evidence from people like the client weighs most. For a veteran at risk of suicide, guidelines that include at-risk veterans (such as the VA/DoD suicide-risk guideline) outweigh studies of adolescents.
- **Publication bias (the file-drawer problem).** Positive results get published more often. Publication does not prove efficacy, and article counts are not strength of evidence.
- **Best available evidence.** A thin base, such as the small adjustment-disorder literature (some support for problem-solving approaches and brief CBT), is **not zero evidence**. Appraise what exists rather than falling back on intuition or borrowing a protocol from a different diagnosis.

Build knowledge from guidelines, systematic reviews, and peer-reviewed outcome research, not from popularity or

social media trends, testimonials, workshop handouts, or aging graduate training (**C.2.f**).

Meehl: Clinical vs. Statistical Prediction

Paul Meehl (1954), in *Clinical Versus Statistical Prediction*, compared **unaided clinical judgment** with **statistical (actuarial) prediction**, which combines the same data by formula. Statistical prediction **matched or outperformed** clinical judgment in nearly every comparison, and later meta-analyses agreed. Lesson: structured data check intuition, and "thin research, so I'll trust my gut" is the weaker choice. Meehl's "**broken leg**" exception allows overriding a formula when the clinician holds decisive information it does not capture, which is why a counselor whose interview contradicts a tool's low-risk rating acts on the **full assessment** and documents the discrepancy (Part 13).

A Popular Practice That Failed: Critical Incident Stress Debriefing

Jeffrey Mitchell (1983) developed **critical incident stress debriefing (CISD)**, a structured single-session group meeting held within days of a traumatic event, originally for emergency workers, moving through set phases (introduction, facts, thoughts, reactions, symptoms, teaching, re-entry). It spread widely.

Controlled research did not support it. The **Cochrane review by Rose, Bisson, Churchill, and Wessely (2002)** found that **single-session psychological debriefing** did **not prevent PTSD**, and some trials found **worse outcomes** at follow-up. International trauma guidance, including the **World Health Organization (2013)**, now recommends **against** routine psychological debriefing. Current practice favors **psychological first aid**, monitoring, and **trauma-focused CBT** for persistent symptoms (the crisis chapter covers these). Mitchell's supporters argue

the trials tested debriefing outside its intended group format; the exam's point is unchanged.

CISD shows three errors at once: **bandwagon** (adoption is not evidence), **timing as rescue** (holding it at the "right" time does not save an unsupported intervention), and **testimonial** (a colleague's good experience is not outcome research). When evidence suggests harm, **ACA C.7.c** applies.

Part 11 — Advocacy for the Client and the Profession (1A.5)

ACA A.7.a (advocacy): when appropriate, counselors advocate at individual, group, institutional, and societal levels to address barriers that limit client access, growth, and development. **ACA A.7.b (confidentiality and advocacy):** counselors obtain **client consent** before advocating on behalf of an identifiable client.

The Advocacy Competencies

The **ACA Advocacy Competencies** were developed by **Lewis, Arnold, House, and Toporek (2002)** and endorsed by ACA. **Toporek and Daniels (2018)** led the update. The model has two dimensions: **extent of client involvement** (**collaborating with** clients vs. **acting on behalf of** clients) and **level of intervention** (individual, school/community, public arena). Crossing them gives six domains:

Level	Collaborating with clients	Acting on behalf of clients
Client/student (individual)	**Client/student empowerment**: helping the client build skills to self-advocate	**Client/student advocacy**: negotiating for a specific client, such as appealing an insurer's denial

Level	Collaborating with clients	Acting on behalf of clients
School/community	**Community collaboration**: supporting a community group that leads its own effort	**Systems advocacy**: changing a barrier within a school, agency, or community system
Public arena	**Collective action** (formerly *public information*): working with groups to change public awareness or policy	**Social/political advocacy**: working to change law or public policy, such as testifying for legislation

Advocacy Within Ethical Bounds

Situation	Keyed advocacy	Wrong answers
Insurer denies sessions; outcome scores show continued need	With consent, **appeal using outcome data** showing medical necessity (client/student advocacy)	Bill a more severe diagnosis (**upcoding is fraud**); accept the denial without challenge; post about the denial publicly (exposes the client)
A probation officer calls the client's medication for opioid use disorder "just another drug"	With **written consent**, give the officer information on medication treatment as evidence-based care	Advise the client to hide treatment; tell the client to drop medication; contact the judge without the client's consent

Advocacy for the **profession** (licensure, insurance recognition, scope of practice) usually works in the public arena, through law and policy.

Part 12 — Training in Counseling Ethics and Decision-Making Models (1A.6)

Counselors know the Code and the law (**C.1**, **I.1.a**) and, facing a dilemma, **use and document an ethical decision-making model** (**I.1.b**). The ethics chapter covers the Code's content; this part owns the models.

Kitchener's Principles

Karen Kitchener (1984) described the moral principles underlying ethical reasoning: **autonomy, nonmaleficence, beneficence, justice, and fidelity**. The **ACA Code Preamble** lists these and adds **veracity**.

Principle	Meaning
Beneficence	Working for the good of the individual and society by promoting well-being
Fidelity	Honoring commitments and **keeping promises**
Veracity	Dealing **truthfully** with people

Kitchener also separated an **intuitive** level of moral reasoning from a **critical-evaluative** level (rules, principles, theory) used when intuition is not enough.

Mandatory vs. Aspirational Ethics

Mandatory ethics is the minimum: complying with required standards. **Aspirational ethics** strives for the **highest standards** and the spirit behind the Code.

Principle Ethics vs. Virtue Ethics (Meara, Schmidt, & Day, 1996)

Naomi Meara, Lyle Schmidt, and Jeanne Day (1996) argued that principles alone are not enough.

	Principle ethics	Virtue ethics
Question	"**What shall I do?**"	"**Who shall I be?**"
Focus	Solving a specific dilemma using obligations and principles	The counselor's **character**, across all situations
Tools	Principles such as autonomy and beneficence	Core virtues: **prudence, integrity, respectfulness, benevolence**

The two work together: principles guide the decision; virtues shape the person and the small choices no rule covers. Aspirational ethics is about **how high** the standard is; virtue ethics is about **character**.

Forester-Miller & Davis (1996): The Practitioner's Seven Steps

Holly Forester-Miller and Thomas Davis set out seven steps: **(1) identify the problem, (2) apply the ACA Code, (3) determine the nature and dimensions of the dilemma, (4) generate courses of action, (5) consider consequences and choose, (6) evaluate the selected action, (7) implement it.**

Tarvydas (1998): The Integrative Decision-Making Model

Vilia Tarvydas built an **integrative** model that joins principle-based reasoning with attention to the counselor's values and the setting. It rests on **four themes** the counselor maintains throughout: a **reflective attitude**, **balance** among the issues and the parties involved, close attention to **context**, and **collaboration** with all rightful parties, including the client.

It moves through **four stages**:

Stage	Name	Key work
I	Interpreting the situation through **awareness and fact finding**	Heighten sensitivity; identify the stakeholders and their ethical claims; gather facts
II	**Formulating** an ethical decision	Review the dilemma; identify the codes, laws, principles, and policies that apply; generate courses of action; weigh consequences; **consult**; select the best ethical course

Stage	Name	Key work
III	**Selecting an action** by weighing competing **nonmoral values, personal blind spots, or prejudices**	Reflect on the counselor's own competing values and biases; consider pressures from colleagues, team, institution, and society
IV	**Planning and executing** the action	Sequence the steps; anticipate barriers and plan countermeasures; carry out, document, and evaluate

Stage III is the signature: Tarvydas builds **self-examination of the decision-maker** into the model.

When the Code and the Law Conflict

ACA I.1.c: counselors **make known their commitment to the Code and take steps to resolve the conflict**; if it cannot be resolved, they may adhere to the law. Ignoring the law is wrong; so is complying silently. **C.2.e**: consult when unsure of ethical obligations.

Part 13 — Training in Ethical and Legal Use of AI (1A.7)

The **2014 ACA Code does not mention artificial intelligence by name.** Its technology, confidentiality, and responsibility standards apply directly:

- **H.1.a (knowledge and competency):** counselors who use technology develop knowledge and skills about its **technical, ethical, and legal** considerations. Build that competence **before** using an AI tool with client data.
- **H.2.a (informed consent and disclosure):** clients are informed about the technologies used in their care, including risks and limits, and can choose. Consent cannot depend on whether the client thinks to ask.

- **B.1.c (confidentiality)** and **HIPAA:** a vendor that handles protected health information on the counselor's behalf is a **business associate** and must sign a **business associate agreement (BAA)** (45 CFR 164.504(e)). Free public chatbots generally do not. Removing a name is **not** de-identification: HIPAA's **Safe Harbor** method requires removing 18 categories of identifiers, and session details can still identify a client.
- **A.1.a (primary responsibility):** AI informs; the counselor decides. The counselor is responsible for every note and clinical decision, including AI-drafted ones.

Agency approval does not supply competence or settle consent and privacy. AI guidance from professional bodies is evolving; verify current recommendations from ACA, NBCC, and your state board.

Two Opposite Errors

Error	Meaning	Correction
Automation bias	Trusting automated output because it is automated, even over contrary evidence	Verify AI output; AI tools can be inaccurate and can carry bias
Algorithm aversion (Dietvorst et al., 2015)	Rejecting a tool wholesale after seeing it err once	Keep verifying; don't abandon a useful aid after one error

When an AI risk rating conflicts with the interview, the counselor relies on the **full clinical assessment and documents the discrepancy**.

Part 14 — Training to Work With Different Cultures and Populations (1A.8)

Cultural Humility (Hook et al., 2013)

Joshua Hook and colleagues (2013) defined **cultural humility** as an **other-oriented** interpersonal stance: openness to the client's cultural identity, curiosity about what it means to the client, and **no assumption of superiority**. They found that clients' perceptions of their counselor's cultural humility were associated with a stronger working alliance and better outcomes.

Faced with a norm the counselor knows little about, the counselor **explores what it means to this client** and **seeks training** to close the gap. Treating a cultural explanation as denial pathologizes it; referring for difference alone dodges responsibility.

The Multicultural and Social Justice Counseling Competencies (Ratts et al., 2016)

The **MSJCC** (**Ratts, Singh, Nassar-McMillan, Butler, & McCullough, 2016**) organize competence into four developmental domains: **counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions**. Within them, counselors build **attitudes and beliefs, knowledge, skills, and action**. The model attends to privilege and marginalization in both counselor and client. The multicultural material elsewhere in this book covers it in depth.

What Real Training Looks Like

Choice	Verdict
A **research-based course with skills practice and follow-up consultation** (e.g., on gender-responsive substance use care drawing on SAMHSA guidance)	**Builds competence**: knowledge, practice, and feedback transfer to sessions

Choice	Verdict
Reputable population-specific training applied to each individual (e.g., on graduate students' stressors and help-seeking)	**Builds competence** without stereotyping
A one-hour webinar with a certificate of completion	Documents **attendance, not skill**
A podcast of first-person stories	Builds empathy and awareness, not clinical skill
Online lists of "typical traits"	**Overgeneralization**: invites seeing the group instead of the client
The general multicultural course from graduate school	Treats competence as fixed; **C. 2.f** says stay current on best practices with diverse populations
Making the client the main teacher	Shifts the counselor's training burden onto the client's paid session time

What the Exam Rewards / What It Punishes

Rewards: - Naming a limit or reaction and taking a proportionate step while care continues - Skill-matched, de-identified consultation; doubts brought to the supervisor before acting - Serving clients despite values conflicts, with training and consultation - Acting on early signs of burnout, compassion fatigue, or vicarious trauma - Regulating yourself first, then responding (including asking directly about suicide) - Evidence weighed by design, replication, and fit to the client - Advocacy with consent, within ethical and legal bounds

Punishes: - Referring out to escape discomfort or because of the counselor's values - Learning a new method on the client - Disclosing the counselor's state or beliefs to the client; hiding doubts from a supervisor - Treating popularity, testimonials, or a single case as evidence - Treating a license, a purchase approval, or a certificate as competence - Upcoding, false CE attestation, or claiming

an unearned credential - Trusting AI output blindly, or pasting client data into a public tool

Practice Questions

Question 1. A client who has been making steady progress in counseling for four months arrives and sits silently for several minutes.

When the counselor asks what's on his mind, he shrugs and says, "Nothing. I don't know if this is even doing anything." The most appropriate initial response is:

A. Reassure him that progress takes time and review recent gains

B. Explore what is missing for him in the counseling relationship

C. Administer a standardized symptom measure to objectively track his progress

D. Ask about recent stressors that might explain the regression

Question 2. A counselor is working with a client from a cultural background in which family decisions are made collectively. The counselor has been encouraging the client to "set limits with your family about your time." The client has been increasingly disengaged. The most likely explanation is:

A. The client has a characterological difficulty with assertiveness

B. The counselor's goals conflict with the client's cultural values

C. The client is ambivalent about change, which is normal in the working stage

D. The client needs psychoeducation about healthy relational patterns

Question 3. Which of the following counselor responses BEST demonstrates unconditional positive regard?

A. “I understand that’s how you feel, but most people in your situation would react differently.”

B. “I’m glad you told me. That behavior concerns me, and we should address it.”

C. “I appreciate you sharing that — let’s explore whether there was a better option.”

D. “Thank you for telling me. I’d like to understand more about what that was like for you.”

Question 4. A counselor is terminating with a client who has met all treatment goals. Three weeks before the final session, the client begins arriving late, canceling sessions, and reports that old symptoms are returning. The counselor should FIRST:

A. Extend the treatment contract by four additional sessions

B. Assess for the recurrence of the original presenting problem using a structured measure

C. Consult with a supervisor about whether the planned termination is premature

D. Name the pattern and explore whether it may be related to the ending

Answer Key — Chapter 1 Practice Questions

Q1 — Answer: B

Rationale: The client is signaling a rupture — something in the alliance or the work is not working for him. The first response is to explore his experience, not to defend the treatment (A), provide objective data (C), or redirect to external events (D). The rupture must be addressed before any other clinical work can resume.

Q2 — Answer: B

Rationale: The disengagement is a goals rupture: the counselor's stated treatment goal (individual autonomy) conflicts with the client's cultural framework. The exam rewards cultural humility — the counselor needs to re-explore what the client actually wants, not double down on a culturally incongruent goal.

Q3 — Answer: D

Rationale: This response validates the client's experience without judgment and without implying a better choice existed. A implies comparison to others (evaluative). C introduces a search for a "better option" (evaluative). B names concern (conditional).

Q4 — Answer: D

Rationale: Pre-termination regression is a well-documented and expected phenomenon. The first clinical response is to name the pattern and connect it to the transition. Extending treatment (A) without exploring this dynamic reinforces the avoidance. Assessment (B) is premature — this symptom pattern in the context of ending is the most parsimonious explanation. Consultation (C) may eventually be warranted but is not the first step.

Chapter 2 — Theories & Orientations

Overview

Pattern-Matching a Stem to a Theory

Theory questions on the NCMHCE follow a predictable structure. A vignette describes a counselor doing something — asking a specific question, assigning a specific task, using a specific technique — and asks you to identify the theoretical orientation (Corey, 2017; Corey et al., 2018). The shortcut: every major theory has 1–2 signature techniques that appear on no other theory's menu. Learn those signatures and you can identify the orientation from the technique alone, without needing to analyze the whole vignette. The three traps of theory items: 1. Conflating humanistic and existential. Both emphasize meaning and authenticity, but humanistic therapy (Rogers) is nondirective and relationship-focused (Rogers, 1957); existential therapy addresses freedom, death, meaning, and isolation more directly and may be more confrontational about avoidance. 2. Mislabeling CBT sub-modalities. REBT (Ellis) uses the ABC model and targets irrational beliefs. Standard CBT (Beck) uses thought records and targets cognitive distortions. DBT targets emotion dysregulation with skills training (Linehan, 2015). These are not interchangeable. 3. Confusing the family interventions. Structural family therapy (Minuchin) focuses on boundaries and subsystems. Strategic family therapy (Haley, Madanes) uses directives and paradoxical interventions. Bowenian therapy focuses on differentiation and genograms (Bowen, 1978).

The Five Theoretical Traditions

Psychoanalytic / Psychodynamic

Core premise: Unconscious conflicts, early attachment patterns, and unresolved developmental material shape present functioning. Change comes through insight into the unconscious.

Signature Techniques	What They Do
Free association	Access unconscious material by removing censorship
Dream analysis	Examine manifest vs. latent content
Transference interpretation	Use the therapeutic relationship as a window into relational patterns
Defense analysis	Identify and gently challenge protective mechanisms

For a full list of defenses organized by maturity level, see the Defense Mechanisms Quick Reference in Part Three.

How this shows up on the exam: Questions mention the counselor "exploring childhood experiences," "interpreting a dream," "noticing the client is relating to the counselor as though they were the client's father," or "helping the client understand how the past affects the present." The exam also tests defense mechanisms (see Part Three).

Adlerian (Individual Psychology)

Core premise: People are motivated by social interest and a striving for superiority (overcoming inferiority; Adler, 1956). Birth order, lifestyle, and fictional final goals shape behavior.

Signature Techniques	What They Do
Lifestyle assessment	Understand the client's characteristic pattern of engaging the world
Early recollections	Access the client's guiding life narrative

Signature Techniques	What They Do
"The question" ("What would be different if you were well?")	Distinguish psychological from organic factors; reveal avoidance
Encouragement	Counteract discouragement, which Adler saw as the root of dysfunction

How this shows up on the exam: Look for birth order questions, early childhood recollections, and the phrase "social interest." Adlerian therapy is goal-directed and often shorter-term than classical psychoanalysis.

Humanistic / Person-Centered

Core premise: People have an innate actualizing tendency that is blocked by conditions of worth imposed by others. The therapeutic relationship is the change mechanism.

Signature Techniques	What They Do
Reflective listening	Communicate empathic understanding
Nondirective exploration	Follow the client's lead; resist the expert role
Congruent self-disclosure	Authentic presence when relevant

How this shows up on the exam: Nondirective stems, emphasis on the relationship over technique, Rogers's three conditions (see Chapter 1). Person-centered questions usually ask you to identify the response that is most empathic or least directive.

Cognitive-Behavioral Traditions

Core premise: Thoughts, feelings, and behaviors are interconnected. Changing distorted cognitions and maladaptive behaviors changes affect.

Approach	Signature Technique	Target of Change
CBT (Beck, 2021)	Thought record, Socratic questioning	Cognitive distortions

Approach	Signature Technique	Target of Change
REBT (Ellis)	ABC model, disputation	Irrational beliefs
DBT (Linehan)	Skills training (TIPP, DEAR MAN, STOP)	Emotion dysregulation
Behavioral (Skinner/ Wolpe)	Exposure, systematic desensitization, token economy	Conditioned responses

The twelve cognitive distortions most often tested are defined, with examples, in the Cognitive Distortions Quick Reference in Part Three.

Worked example: A vignette describes a counselor asking a client: "What evidence do you have that your coworkers think you're incompetent?" This is **Socratic questioning** — a core CBT technique (Beck, 2021) targeting a cognitive distortion. It is not REBT (which would target the underlying *belief*, e.g., "I *must* be competent or else I'm worthless"). It is not person-centered (which would reflect, not challenge). The answer is CBT.

Systemic / Family Systems

Core premise: The client's symptoms are best understood as expressions of a system's (family's, couple's) dynamics. Change the system; the symptoms resolve.

Approach	Signature Technique	Key Concepts
Structural (Minuchin)	Joining, enactment, boundary-setting	Subsystems, boundaries, hierarchy
Strategic (Haley, 1976)	Directives, paradoxical directives	Problem-maintaining sequences
Bowenian	Genogram, coaching differentiation	Differentiation, triangulation, emotional cutoff
Solution-Focused (de Shazer et al., 2007)	Miracle question, scaling, exception-finding	Client strengths, brief therapy

Approach	Signature Technique	Key Concepts
Narrative	Externalization, re-authoring	Dominant narratives, unique outcomes

The sections above cover five traditions. Four of the orientations below are already referenced elsewhere in the guide as though they had been taught — Gestalt's empty chair, reality therapy's WDEP, existential therapy as a distractor, and motivational interviewing as a distractor (Glasser, 1998; Perls et al., 1951; Wubbolding, 2011). This section supplies what those references assume.

Gestalt Therapy (Perls)

Core premise: Awareness is curative. Psychological difficulty arises when a person loses contact with present experience and with disowned parts of the self. The work is not insight into the past but heightened awareness in the present moment.

Signature Technique	What It Does
Empty chair / two-chair work	Externalizes an internal conflict or an unresolved relationship so the client can experience both sides directly
Here-and-now awareness	"What are you noticing in your body right now?" — shifts from talking *about* to experiencing
Exaggeration	Amplifies a gesture, phrase, or tone so its meaning becomes conscious
Staying with the feeling	Blocks the client's habitual escape from an emotion long enough for it to complete
Dream work	Every element of the dream is treated as a disowned part of the dreamer, not as a symbol to be decoded

Unfinished business is the central Gestalt concept: unexpressed feelings toward significant figures that persist

and intrude on present functioning. Empty chair work is the primary vehicle for completing it.

Contact boundary disturbances — the ways people interrupt genuine contact:

Disturbance	Description
Introjection	Swallowing others' values whole without examining them
Projection	Assigning to others what belongs to the self
Retroflection	Doing to oneself what one wants to do to another (or wants another to do to oneself)
Confluence	Merging with others; loss of the boundary between self and other
Deflection	Avoiding contact through humor, abstraction, generalization, or changing the subject

How this shows up on the exam: The empty chair is the giveaway — if a vignette has a counselor asking a client to speak *to* an absent person as though they were present, the answer is Gestalt. Also look for body-awareness prompts and any focus on what is happening *right now* rather than what happened. Gestalt is often mistaken for psychodrama; psychodrama uses other people to play roles; Gestalt uses the empty chair and the client's own voice.

Existential Therapy (Yalom, Frankl, May)

Core premise: Distress arises from the human confrontation with the conditions of existence. The work is not symptom removal but helping a person live authentically in the face of those conditions.

Yalom's four ultimate concerns (Yalom, 1980):

Concern	The Human Problem	Clinical Expression
Death	Mortality is certain and cannot be negotiated	Health anxiety, midlife crisis, avoidance after a diagnosis or a loss
Freedom	We are responsible for our choices; there is no external structure that decides for us	Paralysis in decision-making, chronic blaming, “I had no choice”
Isolation	No one can fully enter another’s experience	Fusion in relationships, inability to be alone, compulsive connection
Meaninglessness	The universe supplies no inherent meaning	Emptiness despite external success, “what is the point”

Frankl’s logotherapy is the meaning-centered branch, developed from his concentration camp experience (Frankl, 1946/2006). Its signature interventions:

- **Paradoxical intention** — the client is asked to intend the very thing they fear (an insomniac tries to stay awake), breaking anticipatory anxiety
- **Dereflection** — attention is deliberately turned away from the self and toward something outside it
- **Socratic dialogue** — questioning that helps the client discover meaning already latent in their life

How this shows up on the exam: Look for stems about mortality, freedom and responsibility, or the emptiness of a life that looks successful from the outside. The distinction from person-centered therapy is directiveness — the existential counselor will actively confront avoidance of responsibility, where the person-centered counselor follows. If the stem includes “meaning” or “purpose” as the presenting concern rather than a symptom, existential is likely.

Reality Therapy / Choice Theory (Glasser)

Core premise: All behavior is chosen and is an attempt to meet basic needs. The past is largely irrelevant; what matters is what the client is doing now and whether it is working.

The five basic needs: survival, love and belonging, power and achievement, freedom, fun. Love and belonging is considered primary — most presenting problems trace to an unsatisfying relationship.

Total behavior — Glasser's four-wheeled car metaphor. Behavior has four components: **acting, thinking, feeling, and physiology.** The client has direct control over acting and thinking (the front wheels, which steer) and only indirect control over feeling and physiology (the rear wheels, which follow). Intervention therefore targets doing and thinking.

The quality world is the client's internal picture album of the people, things, and beliefs that satisfy their needs.

The WDEP system:

Letter	Question
W — Wants	What do you want? What do you want that you are not getting?
D — Doing and Direction	What are you doing? Where is your current behavior taking you?
E — Evaluation	Is what you are doing getting you what you want?
P — Planning	What is a specific, simple, measurable plan you can commit to?

Reality therapy accepts **no excuses**, does not use **punishment**, and does not accept **blaming** of circumstances or others — but it also never gives up on the client.

How this shows up on the exam: Any vignette in which the counselor asks a client to evaluate whether their own behavior is producing the results they want is reality ther-

apy. The self-evaluation step is the signature — the counselor does not tell the client the behavior isn't working; the client is asked to judge it.

Motivational Interviewing (Miller & Rollnick)

Core premise: Ambivalence is normal, and the motivation to change is elicited from the client rather than installed by the counselor. Confrontation produces defensiveness; evocation produces change talk.

MI spirit (PACE): Partnership, Acceptance, Compassion, Evocation (Miller & Rollnick, 2013). This is the stance, not a technique — the exam tests it by punishing responses that are directive, corrective, or expert-positioned.

The righting reflex is the counselor's instinct to fix, correct, or argue for change. Yielding to it reliably produces sustain talk. Resisting it is the first discipline of MI.

OARS — the core skills:

Skill	Application
Open questions	Invite elaboration rather than yes/no
Affirmations	Recognize the client's strengths and efforts genuinely, not as praise
Reflections	Simple and complex; complex reflections carry the conversation forward
Summaries	Collect, link, and transition — especially useful for gathering change talk

The four processes, in sequence: **Engaging** (establishing the relationship) → **Focusing** (agreeing on a direction) → **Evoking** (drawing out the client's own arguments for change) → **Planning** (developing commitment and a concrete plan).

Change talk vs. sustain talk. Change talk is any client speech favoring change. Preparatory change talk is **DARN** — Desire, Ability, Reasons, Need. Mobilizing change talk is **CAT** — Commitment, Activation, Taking steps. Mobilizing change talk is the stronger predictor of actual behavior change.

Discord (formerly "resistance") is a strain in the relationship, not a client trait. The response is to roll with it — reflect, shift focus, emphasize autonomy — never to argue.

How this shows up on the exam: MI-consistent responses reflect ambivalence rather than resolving it, emphasize the client's autonomy, and avoid supplying reasons for change. If two options both sound empathic, the MI answer is the one that leaves the decision explicitly with the client.

The Transtheoretical Model / Stages of Change (Prochaska & DiClemente, 1983)

Core premise: Change is a process that unfolds through predictable stages, and an intervention is effective only if it matches the client's current stage.

Stage	Client Position	Stage-Matched Intervention
Precontemplation	No intention to change; often unaware of a problem	Raise awareness, provide information without pressure, build discrepancy
Contemplation	Aware and ambivalent; weighing costs and benefits	Decisional balance, explore ambivalence, tip the balance
Preparation	Intends to act soon; may have taken small steps	Strengthen commitment, develop a concrete plan, address barriers
Action	Actively modifying behavior	Skills, support, reinforcement, problem-solving

Stage	Client Position	Stage-Matched Intervention
Maintenance	Sustaining change; working to prevent relapse	Relapse prevention, coping with high-risk situations, consolidating identity
Relapse / Recurrence	Return to prior behavior	Reframe as normative and informative, not as failure; re-enter the cycle

How this shows up on the exam: Stage mismatch is the trap. Offering an action-stage intervention — a plan, a strategy, a referral — to a precontemplative client is the single most common wrong answer in substance-use vignettes. Identify the stage from the stem before evaluating the options.

Acceptance and Commitment Therapy (Hayes)

Core premise: Suffering is amplified by experiential avoidance and by fusion with thoughts. The goal is not to change or dispute thought content but to change the client's relationship to it, in the service of valued living.

The six hexaflex processes, which together constitute psychological flexibility:

Process	What It Involves
Acceptance	Making room for unwanted internal experience rather than struggling against it
Cognitive defusion	Seeing thoughts as thoughts rather than as literal truth
Present-moment contact	Flexible, voluntary attention to the here and now
Self-as-context	The observing self, distinct from the content of experience
Values	Chosen life directions that give behavior meaning

Process	What It Involves
Committed action	Building patterns of behavior in the service of values

How this shows up on the exam: The discriminating feature against CBT is that **ACT does not dispute or restructure thoughts (S. C. Hayes et al., 2012).** If a stem has a counselor helping a client notice a thought as a thought — "I'm having the thought that I'm a failure" — that is defusion, and the answer is ACT, not CBT. Metaphors and experiential exercises are characteristic.

Interpersonal Psychotherapy (Klerman & Weissman)

Core premise: Depression occurs in an interpersonal context. Time-limited work on the current relational problem relieves the depression, without requiring a claim that relationships caused it.

The four problem areas — one or two are selected as the focus:

Area	Focus
Grief	Complicated bereavement following a death
Interpersonal role disputes	Conflicting expectations in a significant relationship
Role transitions	Life changes requiring a shift in role — divorce, retirement, new parenthood, illness, migration
Interpersonal deficits	Long-standing difficulty forming and sustaining relationships

How this shows up on the exam: IPT appears in treatment-selection items for depression, where it is a legitimate alternative to CBT with comparable evidence (Klerman et al., 1984). Role transition is the most commonly tested of the four areas. IPT is time-limited and present-focused, which distinguishes it from psychodynamic work on similar material.

Emotionally Focused Therapy (Johnson)

Core premise: Adult romantic distress is an attachment problem. Partners caught in negative cycles are protesting a felt loss of secure connection.

The negative cycle — most commonly **pursue–withdraw**, in which one partner escalates to seek contact while the other withdraws to de-escalate, each move confirming the other's fear.

Three stages:

1. **De-escalation** — identify the cycle, and frame it, rather than either partner, as the enemy
2. **Restructuring interactions** — withdrawer re-engagement, then blamer softening, producing new bonding events
3. **Consolidation** — new solutions to old problems, and a coherent narrative of the change

Attachment injury — a specific incident of abandonment or betrayal at a moment of acute need, which blocks progress until it is directly addressed and forgiven.

How this shows up on the exam: Any couples vignette naming a repeating cycle, or in which the counselor reframes the problem as the pattern rather than either partner, points to EFT (Johnson, 2020). Softening is the signature event.

Expanded Psychodynamic Traditions

The psychoanalytic section above covers classical psychoanalysis. These branches are separately testable.

Branch	Key Figures	Core Concepts
Ego psychology	Anna Freud, Hartmann	Defense mechanisms catalogued; adaptation and ego strength

Branch	Key Figures	Core Concepts
Object relations	Klein, Winnicott (1965), Mahler et al. (1975)	Splitting; paranoid-schizoid and depressive positions; good-enough mother; holding environment; transitional object; true and false self; separation-individuation
Self psychology	Kohut	Selfobject needs — mirroring, idealizing, twinship; empathic failure; transmuting internalization
Analytical psychology	Jung	Collective unconscious; archetypes (persona, shadow, anima/animus, Self); individuation
Attachment theory	Bowlby (1969), Ainsworth et al. (1978), Main	Internal working models; Strange Situation; secure, anxious-ambivalent, avoidant, disorganized

How this shows up on the exam: Attachment classifications appear most often, particularly in child and couples vignettes. Winnicott's "good-enough" and Kohut's mirroring are the two most-tested single terms from this group (Kohut, 1971; Winnicott, 1965).

Experiential Family Therapy (Satir, Whitaker)

Core premise: Family dysfunction is a problem of blocked emotional expression and low self-esteem. Change comes through experience in the session, not explanation.

Satir's five communication stances under stress (Satir, 1988):

Stance	Pattern
Placater	Agrees with everything; sacrifices self to keep peace
Blamer	Attacks and accuses; deflects from own vulnerability
Super-reasonable (computer)	Retreats into logic and abstraction; affect absent
Irrelevant (distractor)	Changes subject, uses humor, avoids the issue entirely
Congruent	Words, affect, and body match; the therapeutic goal

Family sculpting physically arranges family members in space to externalize the emotional structure of the system (Satir, 1988). Whitaker's symbolic-experiential approach uses the therapist's own spontaneity and "therapy of the absurd" to disrupt rigid patterns.

How this shows up on the exam: The five stances are the testable content. A vignette describing a family member who responds to conflict with detached logic is describing the super-reasonable stance.

Feminist and Relational-Cultural Therapy

Core premise (feminist): Psychological distress cannot be separated from social and political context (Brown, 2018). Problems located "in" the client often originate in power arrangements.

Signature elements: **gender-role analysis**, **power analysis**, an explicitly **egalitarian therapeutic relationship** with demystification of the counselor's role, and the principle that **the personal is political**.

Core premise (relational-cultural, Jean Baker Miller and the Stone Center): People grow through and toward connection (Jordan, 2018). Isolation, not conflict, is the primary source of suffering.

Key concepts: **mutual empathy**, **growth-fostering relationships**, the **central relational paradox** (people yearn

for connection but keep parts of themselves out of relationship to protect it), **disconnection and reconnection**, and **controlling images** that constrain marginalized clients.

How this shows up on the exam: These appear within multicultural competency items. The tell is a counselor who names a social or systemic contributor rather than locating the problem solely inside the client, or who explicitly flattens the power differential.

Child and Adolescent Modalities

Modality	Core Features
Child-centered play therapy (Axline, 1947; Landreth, 2012)	Nondirective; play is the child's language and toys are the words; the relationship is the agent of change
Filial therapy (Guerney, Landreth's CPRT)	Trains parents to conduct therapeutic play sessions; targets the parent–child relationship
Sandtray	Client builds a world in sand with miniatures; symbolic, minimally verbal
Directive/prescriptive play	Structured activities targeting specific goals; used in trauma-focused cognitive behavioral therapy (TF-CBT) and behavioral work

Axline's eight principles in brief: build a warm relationship; accept the child exactly as they are; establish permissiveness so feelings can be expressed; recognize and reflect feelings; respect the child's capacity to solve their own problems; let the child lead; do not hurry the process; set only the limits necessary for safety and reality (Axline, 1947).

How this shows up on the exam: Child vignettes test whether the counselor follows the child's lead rather than

directing play toward the presenting problem, and whether limits are set on behavior without rejecting the feeling.

Trauma-Specific Protocols

These appear by name in treatment-selection items.

Protocol	Structure	Best-Tested Detail
EMDR (Shapiro)	Eight phases; bilateral stimulation while attending to the target memory	Adaptive Information Processing model; the client is not required to describe the trauma in detail
Cognitive Processing Therapy	12 sessions; identifies and modifies “stuck points”	Written trauma account is optional (CPT vs. CPT+A)
Prolonged Exposure	In vivo exposure plus imaginal exposure with processing	Avoidance is the maintaining mechanism
TF-CBT	PRACTICE components; children and caregivers together	Trauma narrative is developed gradually; caregiver involvement is integral
Seeking Safety (Najavits, 2002)	Present-focused, 25 topics	Designed for concurrent PTSD and substance use; requires no trauma narrative — the answer when a client is not stable enough for processing

Transactional Analysis (Berne, 1964)

Lower yield but occasionally tested. Three **ego states** — Parent, Adult, Child. **Transactions** are complementary (parallel, communication continues), crossed (communication breaks down), or ulterior (a hidden message beneath the social one). **Games** are repetitive ulterior transactions with a predictable payoff. **Life script** is the unconscious life plan formed in childhood; **life positions** run

from “I’m OK — You’re OK” to “I’m not OK — You’re not OK.” **Strokes** are units of recognition.

Frameworks and Tools the 2027 Specifications Name

Why it’s here: NBCC’s 2027 examination specifications list these frameworks and tools in their additional content guidance (NBCC, 2026).

Know the Owner, the Idea, and the Look-Alike

The 2027 specifications name frameworks and tools that candidates may benefit from reviewing. You do not need to practice them. You need to recognize each one from a short description, name its owner, and tell it apart from the approach it gets confused with.

Each entry gives the **owner**, the **core idea**, the **signature terms**, **when it fits**, and the **look-alike**. Items usually hand you a signature term (“exiles,” “pendulation,” “cultural opportunities”) and ask for the model, or describe a client and ask which approach fits.

Part 1 — Who Owns It

Owner	What it owns
Richard Schwartz (1995)	**Internal Family Systems (IFS)**: Self; managers, firefighters, exiles; unburdening
Owen et al. (2011); **Davis et al.** (2018)	**Multicultural Orientation (MCO)**: cultural humility, opportunities, comfort
Stanley, Brown, et al. (2009)	**CBT for suicide prevention (CBT-SP)**
Stanley & Brown (2012)	**Safety Planning Intervention**
Rudd & Bryan (Rudd et al., 2015)	**Brief CBT for suicide prevention (BCBT)**
David Jobes (2016)	**CAMS**; the Suicide Status Form
Segal, Williams, & Teasdale (2013)	**Mindfulness-Based Cognitive Therapy (MBCT)**

Owner	What it owns
Jon Kabat-Zinn (1990)	**Mindfulness-Based Stress Reduction (MBSR)**
Marsha Linehan (1993)	**DBT** mindfulness skills; wise mind
John Krumboltz	**Social learning theory of career decision making**; **planned happenstance**; **happenstance learning theory**
Peter Levine (1997)	**Somatic Experiencing (SE)**: titration, pendulation
Pat Ogden (Ogden et al., 2006)	**Sensorimotor Psychotherapy**
BCIA	Voluntary **biofeedback** and **neurofeedback** certification
Cohen, Mannarino, & Deblinger (2017)	**Trauma-Focused CBT (TF-CBT)**: PRACTICE
Patricia Resick (Resick et al., 2017)	**Cognitive Processing Therapy (CPT)**
Edna Foa (Foa et al., 2007)	**Prolonged Exposure (PE)**
NIMH (Horowitz et al., 2012)	**Ask Suicide-Screening Questions (ASQ)**
Posner et al. (2011)	**Columbia-Suicide Severity Rating Scale (C-SSRS)**
Kroenke, Spitzer, & Williams (2001)	**PHQ-9**, including item 9
Myers & Briggs, from **Jung** (1921)	**Myers-Briggs Type Indicator (MBTI)**
E. K. Strong Jr.	**Strong Interest Inventory**
John Holland (1997)	**RIASEC**; **Self-Directed Search (SDS)**
Kretzmann & McKnight (1993)	**Asset-based community development**; asset mapping
American Psychological Association (2020)	**Publication Manual, 7th ed.**; bias-free language

Part 2 — Frameworks

Internal Family Systems (Schwartz)

Owner: Richard Schwartz (1995; Schwartz & Sweezy, 2020), a family therapist who applied systems thinking to the inner world.

Core idea: The mind is naturally made of **parts**, and everyone has an undamaged core **Self** that can lead. Symptoms come from parts carrying **burdens** (extreme beliefs and feelings from painful experience) and from protectors working to keep those burdens buried.

Signature terms: - **Exiles**: young, wounded parts carrying pain, shame, or fear. - **Managers**: **proactive** protectors that prevent triggering through control, perfectionism, criticism, or caretaking. - **Firefighters**: **reactive** protectors that douse an exile's pain fast, through bingeing, substances, dissociation, or rage. - **Self**, marked by the **8 C's**: calmness, curiosity, clarity, compassion, confidence, courage, creativity, connectedness. - **Unblending** (a part steps back so Self can relate to it) and **unburdening** (after being witnessed, an exile releases its burden).

When it fits: clients who speak in parts ("part of me wants to quit"), trauma survivors with harsh inner critics, and self-destructive behavior that serves a protective function. No part is treated as bad.

Look-alike: Gestalt **two-chair** work (Perls) voices inner conflict without the Self/protector/exile map. **Bowen** family systems works with the actual family, not inner parts.

Multicultural Orientation (Owen et al.)

Owner: Jesse Owen and colleagues (Owen et al., 2011), reviewed by **Davis et al. (2018)**.

Core idea: Multicultural competence is a **way of doing** (awareness, knowledge, skills). Multicultural orientation

is a **way of being**: how the counselor engages culture moment to moment in session.

Three pillars: - **Cultural humility**: an other-oriented stance that does not assume expertise in the client's identity. - **Cultural opportunities**: in-session moments when the client's cultural identity could be explored. Does the counselor notice and take them? Missed opportunities have been linked to weaker outcomes. - **Cultural comfort**: the counselor's ease and openness when cultural topics arise.

When it fits: any client, especially when identity, faith, race, gender, or migration surfaces and the counselor must decide whether to explore it now.

Look-alike: the **Multicultural Counseling Competencies** (Sue, Arredondo, & McDavis, 1992) and the **MSJCC** (Ratts et al., 2016) are **competency** frameworks. MCO is **process**. "Noticed the moment and invited the client to say more" → MCO.

CBT-SP and Its Neighbors

Owner: Barbara Stanley, Gregory Brown, and colleagues (Stanley et al., 2009). Developed for adolescents after a suicide attempt, within the NIMH-funded **Treatment of Adolescent Suicide Attempters (TASA)** study, drawing on CBT, DBT, and Brown et al.'s (2005) cognitive therapy for adult suicide attempters.

Core idea: Suicidal behavior is the **direct target** of treatment, not a symptom expected to fade with depression.

Signature components: a **chain analysis** of the recent suicidal crisis; early **safety planning**; **skills** (cognitive restructuring, problem solving, emotion regulation, family work); and a **relapse-prevention task**, in which the client imagines a crisis in detail and rehearses using the new skills.

Neighbors: - **Safety Planning Intervention** (**Stanley & Brown, 2012**): a **brief stand-alone intervention**, not a full therapy. A written, prioritized plan: warning signs, internal coping, people and places for distraction, people to ask for help, professionals and crisis lines, and **making the environment safe**. It is **not** a no-suicide contract. - **Brief CBT for suicide prevention** (**Rudd** and **Bryan**; Rudd et al., 2015): a short structured protocol tested with active-duty soldiers, using a crisis response plan. - **CAMS** (**Jobes, 2016**): a **framework**, not a single therapy. Counselor and client complete the **Suicide Status Form** side by side and target the client's suicide **drivers**.

Mindfulness-Based Cognitive Therapy (Segal, Williams, & Teasdale)

Owner: Zindel Segal, Mark Williams, and John Teasdale (2013).

Core idea: Relapse prevention for recurrent depression. A low mood can reactivate old depressive thinking. MBCT teaches people in recovery to notice that shift early and step back from it.

Signature terms: decentering (thoughts as mental events, not facts); **doing mode vs. being mode**; an **8-week group** with home practice; the **3-minute breathing space**. MBCT adapts MBSR and adds cognitive therapy elements.

When it fits: a client **in remission** with **several prior episodes**. Benefit appears clearest with three or more. It is not designed as first-step treatment for an acute severe episode. Some national guidelines, including the UK's NICE, have listed MBCT for relapse prevention; verify current wording before citing.

Look-alikes: MBSR (**Kabat-Zinn, 1990**) is the parent program, built for **stress, pain, and illness**. **DBT mindfulness** (**Linehan, 1993**) is one skills module: "what" and "how" skills and **wise mind**, for emotion dysregula-

tion. **ACT** (**Hayes**) uses mindfulness for acceptance, defusion, and values-based action.

Krumboltz's Career Theories

Owner: John Krumboltz, who applied Bandura's social learning theory to career choice (Mitchell & Krumboltz, 1996).

Core idea: Career preferences are **learned**, and people can keep learning. Four factors shape decisions: 1. **Genetic endowment and special abilities** 2. **Environmental conditions and events** (economy, family, opportunity) 3. **Learning experiences**: **instrumental** (consequences of one's own actions) and **associative** (pairings and observation) 4. **Task-approach skills** (work habits, problem solving, goal setting)

These produce **self-observation** and **worldview generalizations**. Faulty ones block decisions; the **Career Beliefs Inventory** surfaces them.

Planned happenstance (Mitchell, Levin, & Krumboltz, 1999) and **happenstance learning theory** (Krumboltz, 2009): unplanned events are normal and useful. Counselors help clients **create and use chance** through **curiosity, persistence, flexibility, optimism, and risk taking**. Indecision becomes **open-mindedness**.

When it fits: a client who "doesn't know their passion," was derailed by a layoff, or holds limiting beliefs from narrow experience.

Look-alikes: SCCT (Lent, Brown, & Hackett, 1994) also builds on Bandura but centers **self-efficacy, outcome expectations, and goals**. **Holland** matches types to environments. **Super** maps life stages and roles.

Somatic Therapies (Levine; Ogden)

Core idea: Trauma is held in the body as well as in memory and belief. **Bottom-up** work starts with sensa-

tion, movement, and arousal. **Top-down** work (such as cognitive therapy) starts with thought and meaning.

Somatic Experiencing (**Peter Levine**, 1997): **titration** (approaching activation in small doses); **pendulation** (moving attention between activation and a resource so the system learns it can return to calm); **resourcing**; completing **thwarted defensive responses**.

Sensorimotor Psychotherapy (**Pat Ogden**; Ogden, Minton, & Pain, 2006): integrates body-based work with cognitive and emotional processing; tracks posture and movement in the moment; keeps arousal within the **window of tolerance** (Siegel, 1999).

When it fits: survivors who cannot tolerate talking about the trauma, who report body-based symptoms, or who flood or shut down in talk therapy.

Evidence: still developing. Early studies, including a randomized trial of SE for PTSD (Brom et al., 2017), are encouraging, but these are not first-line PTSD treatments in major guidelines.

Look-alikes: EMDR (Shapiro) uses bilateral stimulation in a set protocol. **Polyvagal theory** (Porges) is a **theory** many somatic clinicians draw on, not a therapy.

Biofeedback

Core idea: Sensors measure a body process and feed it back in real time so the client learns to change it.

Modality	Measures
EMG	Muscle tension
Thermal	Skin temperature
EDA / GSR	Skin conductance
HRV	Beat-to-beat heart rate variation; paced “resonance” breathing
Neurofeedback (EEG)	Brain electrical activity; evidence varies by condition

Who provides it: clinicians from many licensed disciplines. The **Biofeedback Certification International Alliance (BCIA)** offers **voluntary** certification. Certification is a training credential, not a license, and does not expand legal scope of practice. Whether biofeedback is within a counselor's scope is set by state law; check your state.

Counselor's role: obtain education, training, and supervised experience before using it (**ACA C.2.a** Boundaries of Competence; **C.2.b** New Specialty Areas of Practice), or refer. Equipment-free self-regulation skills such as paced breathing remain available.

Look-alikes: relaxation training and **mindfulness** teach self-regulation **without instrumented feedback**. Neurofeedback is a **subtype** of biofeedback.

TF-CBT vs. CPT vs. PE

Owner: Judith Cohen, Anthony Mannarino, and Esther Deblinger (2017). A short-term, components-based treatment for **children and adolescents (about ages 3–18)** with a **nonoffending caregiver** in parallel and joint sessions.

PRACTICE: Psychoeducation and **P**arenting skills · **R**elaxation · **A**ffective modulation · **C**ognitive coping · **T**rauma narrative and processing · **I**n vivo mastery · **C**onjoint child-caregiver sessions · **E**nhancing safety. Skills come first; the **trauma narrative** follows and is later shared with the caregiver.

Feature	TF-CBT	CPT (Resick)	PE (Foa)
Population	Children and teens, with caregiver	Primarily adults	Primarily adults
Mechanism	Skills, then narrative exposure	**Cognitive**: challenge **stuck points**	**Exposure**: emotional processing

Feature	TF-CBT	CPT (Resick)	PE (Foa)
Signature terms	PRACTICE; trauma narrative	**Impact statement**; safety, trust, power/control, esteem, intimacy	**Imaginal** and **in vivo** exposure

Part 3 — Tools and Assessments

Ask Suicide-Screening Questions (ASQ)

Owner: NIMH; validated with youth in pediatric emergency departments (Horowitz et al., 2012).

What it is: a brief **screen** for **medical settings**, not a risk assessment. Validated with **youth ages 10–21** and, in later NIMH-led research, with **adult medical patients** (verify current age guidance in the NIMH toolkit).

Structure: four yes/no questions (recent wish to be dead, belief that family would be better off, recent thoughts of killing oneself, any past attempt). Any yes → the **acuity question**: thoughts of killing yourself **right now**? - **Yes to acuity → acute positive**: urgent safety and full mental health evaluation; the patient cannot leave until evaluated for safety. - **Yes to any of the four, no to acuity → non-acute positive**: a **brief suicide safety assessment** to decide whether a full mental health evaluation is needed.

The screen sorts. A clinician still assesses.

Look-alikes: the **C-SSRS** (**Posner et al., 2011**) grades **ideation severity** from wish to be dead to active ideation with plan and intent, and separately asks about **behavior**. **PHQ-9 item 9** (**Kroenke, Spitzer, & Williams, 2001**) is one item on a **depression** measure; a positive answer requires follow-up, but it is not a risk assessment.

MBTI, the Strong, and the SDS

MBTI (**Isabel Briggs Myers and Katharine Cook Briggs**, from **Jung**, 1921): four **dichotomies** (E–I, S–N, T–F, J–P) combine into **16 types**. Widely used for self-understanding, career exploration, and team building. **Limits:** it forces continuous traits into either/or categories, and many people get a different type on retest (Pittenger, 2005). Its publisher's own guidance discourages using it for hiring or selection.

Strong Interest Inventory (**E. K. Strong Jr.**): measures **interests**, compared with people **satisfied in specific occupations**. Reports Holland's six General Occupational Themes, Basic Interest Scales, Occupational Scales, and Personal Style Scales. Publisher-scored; professionally interpreted.

Self-Directed Search (**John Holland**, 1997): built on **RIASEC** (Realistic, Investigative, Artistic, Social, Enterprising, Conventional). **Self-administered, self-scored, self-interpreted.** Yields a **three-letter Holland code**.

Choose instruments appropriate to the client and purpose (**ACA E.6.a** Appropriateness of Instruments) and within your training (**ACA E.2.a** Limits of Competence).

Community Needs and Strengths Assessment

Owner of the strengths side: John Kretzmann and John McKnight (1993), **asset-based community development**.

Core idea: A **needs assessment** asks what a community lacks; used alone, it defines a community by its problems. An **asset-based** approach starts with what the community **already has**. Good assessments do both. **Asset mapping** inventories the gifts of **individuals**, **associations** (congregations, clubs, mutual-aid groups), and **institutions** (schools, libraries, clinics, businesses).

Needs-assessment methods: key informants (people who know the community well); **community forums** (residents describe needs in their own words); **surveys** of residents; **social indicators** (existing data such as poverty or unemployment rates); and **service utilization** (who already uses which services).

When it fits: planning a program, starting outreach, or community-level advocacy. Ask the community first.

Look-alike: program evaluation judges an existing program. A needs and strengths assessment comes **before** the program.

The APA Publication Manual (7th Edition)

Owner: the **American Psychological Association** (2020). Counselors use it for research, journal articles, grants, and many professional reports.

Bias-free language: describe people at the **appropriate level of specificity** and only when relevant; use the labels people use for themselves. **Person-first** ("a person with schizophrenia") and **identity-first** ("an autistic person") are **both acceptable**; follow the person's or group's preference. "Older adults," not "the elderly." Singular **"they"** is accepted.

Look-alikes: the **Publication Manual** sets writing style; the **ACA Code of Ethics** sets conduct. The **DSM** comes from a different APA, the **American Psychiatric Association**.

What the Exam Rewards / What It Punishes

Rewards: - Naming the owner from one signature term. - Treating a **positive screen** as a step toward **assessment**. - MBCT for **relapse prevention** in remitted recurrent depression; TF-CBT for a **child with a caregiver**. - Building **competence before** offering biofeedback, or referring. - MBTI for **self-exploration**, with its

limits named. - Starting community work with **assets and residents' voices**.

Punishes: - Calling a safety plan a no-suicide contract. - Using PHQ-9 item 9 alone as a suicide risk assessment. - Using the MBTI to hire. - MBCT as first-step treatment for an acute severe episode. - Treating somatic therapies as the best-supported PTSD treatment. - Assuming BCIA certification changes legal scope. - Treating an IFS protector as the enemy.

Practice Questions

Question 5. A counselor asks a client struggling with procrastination: "On a scale of 1–10, where 10 means you've already finished the project and 1 is the worst it's ever been, where are you today? What would be different that would tell you it's a 7 instead of a 5?" This technique is MOST consistent with:

A. Motivational interviewing

B. Solution-focused brief therapy

C. Adlerian therapy

D. Rational emotive behavior therapy

Question 6. A family therapist observes that the parents speak for the identified patient, who remains silent. The therapist asks the child to respond directly to one parent while the therapist watches. This is BEST described as:

A. A paradoxical directive

B. An enactment

C. Externalization

D. Differentiation coaching

Question 7. A counselor says, "It sounds like the depression has been trying to convince you that you're worthless. When did you first notice it doing that?" This is MOST consistent with:

A. Cognitive behavioral therapy

B. Psychodynamic therapy

C. Narrative therapy

D. Existential therapy

Question 8. According to REBT, which of the following is the MOST direct target of intervention?

A. The activating event (A) that triggered the emotional response

B. The consequences (C), including emotional distress and behavioral reactions

C. The belief (B) the client holds about the activating event

D. The environmental reinforcers and contingencies maintaining the behavior

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Question 9. A counselor asks a client who is angry with her deceased father to imagine him in the chair opposite and speak directly to him. This technique is MOST consistent with:

A. Psychodrama

B. Existential therapy

C. Gestalt therapy

D. Narrative therapy

Question 10. A client says, “I have to keep everyone happy or something bad will happen.” She reports exhaustion and resentment but continues to accommodate everyone. In Gestalt terms, her adoption of an unexamined rule about her obligations to others BEST illustrates:

A. Introjection

B. Retroflection

C. Confluence

D. Deflection

Question 11. A counselor working with a client who cannot fall asleep instructs him to lie in bed and try as hard as possible to stay awake. This intervention is:

A. Stimulus control from CBT for insomnia

B. A behavioral experiment

C. A strategic family therapy directive

D. Paradoxical intention from logotherapy

Question 12. A counselor asks a client, “What are you doing to get what you want? Is it working?” This is the signature move of:

A. Motivational interviewing

B. Reality therapy

C. Solution-focused brief therapy

D. Cognitive behavioral therapy

Question 13. A client with alcohol use disorder says, “My wife thinks I have a problem, but I drink the same as everyone I know.” The counselor’s MI-consistent response is:

A. “Let’s look at the DSM-5-TR criteria for alcohol use disorder together so you can see how many of them apply to you.”

B. “What would your wife say about your drinking if she were here with us right now?”

C. “It sounds like you’re minimizing how much this is affecting your marriage and your health.”

D. “So from where you sit, your drinking looks pretty ordinary — and someone close to you sees it differently.”

Question 14. A client in a substance use program states he has no intention of stopping and only attends because his employer required it. The MOST stage-appropriate intervention is:

A. Provide information without pressure and explore what he values, emphasizing that the decision is his

B. Develop a concrete quit plan with a target date and identify supports for the first week

C. Teach relapse prevention and coping skills for the high-risk situations he is likely to encounter at work and at home

D. Complete a decisional balance worksheet weighing the pros and cons of continuing to use

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Question 15. A counselor helps a client notice that "I'm worthless" is a thought passing through awareness rather than a fact about herself, without examining evidence for or against it. This intervention is MOST consistent with:

A. Cognitive behavioral therapy

B. Rational emotive behavior therapy

C. Acceptance and commitment therapy

D. Psychodynamic therapy

Question 16. A client became depressed after retiring from a career that had defined him for thirty years. Using IPT, the MOST appropriate problem area is:

A. Grief

B. Role transition

C. Interpersonal role dispute

D. Interpersonal deficits

Question 17. A couple presents with a repeating pattern: she escalates and criticizes when she feels distant, he

goes quiet and leaves the room. Each reports the other's behavior as the problem. An EFT counselor's FIRST task is:

A. Help them identify and externalize the cycle as the shared enemy

B. Coach the withdrawing partner to re-engage and share his underlying fears

C. Facilitate a softening event in the pursuing partner

D. Address any attachment injury underlying the pattern of pursuit and withdrawal

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Question 18. A 4-year-old in play therapy repeatedly buries a doll in the sand and digs it up. The child-centered play therapist should:

A. Ask the child what the burial means so the play can be understood

B. Redirect to a discussion of the child's recent loss to help the child process it

C. Introduce a book about grief and death written for young children

D. Reflect the child's activity and feeling without interpreting or directing

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Question 19. A client with PTSD and active, unstable cocaine use asks for trauma treatment. She has been in and out of detox three times this year. The MOST appropriate protocol is:

A. Seeking Safety

B. Prolonged Exposure

C. Cognitive Processing Therapy

D. EMDR

Question 20. A family therapist observes that whenever conflict arises, the father responds with detailed logical analysis and no visible emotion. In Satir's framework this is:

A. Placating

B. Blaming

C. Super-reasonable

D. Irrelevant

Question 21. A counselor tells a client who blames herself for her low pay, "Before we look at what you might do differently, I want to name that women in your field earn substantially less than men for the same work." This intervention MOST reflects:

A. Cognitive restructuring

B. Narrative externalization of self-blame

C. Psychoeducation about gender pay disparities

D. Feminist therapy's power analysis

Question 22. A client describes a therapist she saw years ago who "never really seemed to see me." She now becomes distressed when her current counselor takes notes. In Kohut's self psychology, her original experience is BEST described as:

A. A failure of mirroring

B. Splitting

C. A crossed transaction

D. Disorganized attachment

Answer Key — Chapter 2 Practice Questions

Q5 — Answer: B

Rationale: Scaling anchored to the preferred future (10 = the problem already solved) and followed by "what would be different at a 7" is the SFBT scaling question (de Shazer et al., 2007). Motivational interviewing's confidence ruler scales readiness or confidence, not distance from a solved future, and asks "why not lower" (Miller & Rollnick, 2013). Adlerian therapy uses encouragement (Adler, 1956). REBT uses ABC disputation (Ellis & Dryden, 2007).

Q6 — Answer: B

Rationale: Enactment is the signature structural family therapy technique in which the therapist asks family members to interact in session so that the boundary/hierarchy problems become visible and can be restructured (Minuchin, 1974). Paradoxical directive (A) is strategic. Externalization (C) is narrative. Differentiation coaching (D) is Bowenian (Bowen, 1978).

Q7 — Answer: C

Rationale: Externalization — separating the problem from the person's identity — is the hallmark narrative therapy technique (White & Epston, 1990). CBT would examine the thought (not personify it). Psychodynamic would explore its developmental roots. Existential would explore meaning, not externalize.

Q8 — Answer: C

Rationale: The ABC model holds that it is not the activating event itself but the client's belief about it that produces emotional consequences. Changing the irrational belief (disputation) is the core REBT intervention. A and B are components of the model, not the target. D describes a behavioral, not REBT, approach.

Q9 — Answer: C

Rationale: Empty chair work is the signature Gestalt technique, used to complete unfinished business with a significant figure (Perls et al., 1951). A is the closest distractor and the deciding feature is method — psychodrama uses other group members to enact roles, while Gestalt uses the empty chair and the client's own voice. B addresses ultimate concerns such as death and meaning rather than staging a dialogue (Yalom, 1980). D would externalize the anger as an entity with its own history, not address the father directly.

Q10 — Answer: A

Rationale: Introjection is taking in others' values and rules whole, without chewing them over. The rule operates as if it were her own even though she never examined it. B would involve turning an impulse back on herself — self-criticism in place of anger at others — which is a consequence here, not the boundary disturbance itself. C is a loss of the self–other boundary through merging, closer but not what the unexamined rule describes. D is avoidance of contact through humor or abstraction, which she is not doing.

Q11 — Answer: D

Rationale: Frankl's paradoxical intention asks the client to intend the feared outcome, which breaks the anticipatory anxiety that is producing it — the mechanism is precise for insomnia, where trying to sleep is what prevents sleep (Frankl, 1946/2006). A restricts the bed to sleep and sex and would have him leave the bed, not stay in it. B tests a specific prediction to gather evidence about a belief. C uses directives and paradox within a family system to interrupt problem-maintaining sequences, and there is no system in this stem.

Q12 — Answer: B

Rationale: The self-evaluation step — asking the client to judge whether their own behavior is producing the results they want — is the defining WDEP move, and the counselor deliberately does not render the judgment (Wub-

bolding, 2011). A would reflect ambivalence and emphasize autonomy rather than direct the client toward evaluating effectiveness. C would look for exceptions and preferred futures rather than assess current behavior against wants. D would examine the thought underlying the behavior.

Q13 — Answer: D

Rationale: This is a complex reflection that holds both sides of the discrepancy without arguing for either, which is how MI develops discrepancy from inside the client's own frame. A yields to the righting reflex and supplies the counselor's evidence. B recruits an absent third party and implicitly sides with her. C labels the client, which produces discord rather than change talk.

Q14 — Answer: A

Rationale: He is in precontemplation, where the task is raising awareness and building discrepancy while explicitly preserving autonomy — pressure at this stage produces resistance. B is a preparation-stage intervention. C is a maintenance-stage intervention. D is the contemplation-stage tool and is the strongest distractor, but decisional balance presumes an ambivalence he has not yet expressed.

Q15 — Answer: C

Rationale: Cognitive defusion changes the client's relationship to the thought rather than its content, and the explicit absence of evidence-testing is what identifies it. A would run a thought record and examine evidence. B would dispute the underlying irrational belief. D would explore the developmental and relational origins of the belief rather than the client's present relationship to the thought.

Q16 — Answer: B

Rationale: Retirement requires relinquishing an old role and constructing a new one, which is precisely what the role transition focus addresses. A is reserved for bereave-

ment following a death. C requires a conflict with a specific person over expectations, and none is described. D applies to long-standing difficulty forming relationships, not to a discrete life change.

Q17 — Answer: A

Rationale: Stage one is de-escalation, and its central move is framing the cycle — not either partner — as the problem, which lowers blame enough for anything else to become possible. B and C are stage two events and are sequenced in that order, both requiring de-escalation first. D is addressed when it blocks progress, but it is not the opening move and no specific injury is described.

Q18 — Answer: D

Rationale: In child-centered play therapy the child leads, play is the communication, and the therapist's role is tracking and reflecting rather than interpreting — meaning does not need to be verbalized to be worked through. A imposes an adult verbal demand on a preschooler and interrupts the process. B and C are directive interventions that belong to prescriptive play therapy or TF-CBT, not to the child-centered approach the question specifies (Cohen et al., 2017).

Q19 — Answer: A

Rationale: Seeking Safety is present-focused, designed for concurrent PTSD and substance use, and requires no trauma narrative — which makes it the indicated option when a client is not stable enough for processing (Najavits, 2002). B, C, and D are all evidence-based for PTSD but involve trauma processing that risks destabilization during active, unstable substance use. This sequencing judgment is what the item is testing.

Q20 — Answer: C

Rationale: The super-reasonable or computer stance retreats into logic and abstraction while affect goes offline, which is exactly what the stem describes. A involves agreeing and self-sacrificing to keep peace. B involves at-

tacking and accusing. D involves changing the subject or deflecting with humor. All four are stress responses, and the therapeutic goal is congruence.

Q21 — Answer: D

Rationale: Locating a client's difficulty partly in a social and structural arrangement rather than entirely within her is the defining feminist move, and the exam tests it within multicultural competency. A would examine and test the self-blaming thought as a cognitive error. B would personify the self-blame as a separate entity with its own history. C describes the format of what the counselor did but not the theoretical orientation driving it, which is what the question asks.

Q22 — Answer: A

Rationale: Mirroring is the selfobject need to be seen, recognized, and responded to, and its failure is exactly what "never really seemed to see me" describes. B is a Kleinian object-relations concept involving all-good and all-bad representations. C is transactional analysis. D is an attachment classification derived from the Strange Situation, requiring an entirely different evidence base than a recalled therapy experience (Ainsworth et al., 1978).

Chapter 3 — Skills & Techniques

Overview

The Microskills Hierarchy as a Sequence

Counseling microskills are not a menu to pick from randomly (Ivey et al., 2018). They form a developmental sequence with a clear clinical logic: you must earn the right to move up the hierarchy by doing the lower levels well first.

Attend → Listen → Influence

Attending (nonverbal and physical presence) must come before anything else. A counselor who looks distracted, avoids eye contact, or creates physical distance has already failed before speaking. Culturally responsive attending recognizes that eye contact norms vary across cultures — sustained eye contact communicates respect in many Western contexts but can signal aggression or disrespect in others. Listening skills (paraphrase, reflection of feeling, reflection of meaning, summarization) communicate understanding and build the alliance. These are the engine of the middle phase of every session.

The exam wants you to know which listening skill to use when:

- Paraphrase: captures the cognitive content of what the client said in slightly different words. Use to demonstrate you heard the facts.
- Reflection of feeling: names the emotion underneath the content. Use when the affect is stronger than the content.
- Reflection of meaning: captures the deeper value or belief the feeling points to. Use when you want to go deeper.

- Summarization: draws together themes across a segment of the session or across sessions. Use to signal transition, to punctuate insight, or to open a session.

Influencing skills (confrontation, self-disclosure, interpretation, immediacy, information-giving) require that the alliance be solid before they will be effective. A confrontation delivered without a strong alliance lands as an attack. An interpretation without sufficient exploration is a guess. The exam frequently tempts you to intervene (influence) before the listening foundation is laid. “Validate before you challenge” is the exam’s cardinal rule.

When a client expresses distress, the first scored response reflects feeling, not content. When a client is resistant, the first scored response acknowledges the resistance rather than arguing against it. When a client disagrees with the counselor, the first scored response explores the client’s perspective rather than defending the counselor’s position.

Skills Discrimination Drills

These four skills are the most-confused group on the exam:

Skill	What It Is	What It Is Not
Paraphrase	Restate content in your words	Does not name emotion
Reflection of feeling	Name the emotion the client is experiencing	Does not summarize content
Immediacy	Address the counselor–client relationship in the here-and-now	Not about the client’s outside relationships
Confrontation	Point out an incongruity between two things the client said, or between words and behavior	Not attacking; the tone is gentle and curious

Drill:

Client: “I keep telling my wife I’m fine, but honestly, I don’t sleep, I can’t concentrate, and I feel like I’m carrying this weight I can’t put down.”

- Paraphrase: “You’re telling her things are okay even though you’re struggling with sleep and focus.”
- Reflection of feeling: “There’s something exhausting — and maybe lonely — about carrying all of this privately.”
- Immediacy: (Not applicable here unless the counselor–client relationship is the topic)
- Confrontation: “I’m noticing something — you say you’re fine to your wife, but what you’re describing to me sounds like you’re far from fine. What do you make of that difference?”

When to Use Influencing Skills

Confrontation is appropriate when: (1) the alliance is strong and established, (2) a discrepancy exists between what the client says and what they do (or between two things they say), and (3) the discrepancy is maintaining the problem. Never confront in the first session. Never confront when a client is acutely distressed. Interpretation is appropriate when: the data has been gathered (sufficient exploration), the client is in a receptive state, and the interpretation is offered tentatively (“I wonder if…”), not as a pronouncement. Self-disclosure is appropriate when: it normalizes the client’s experience, strengthens the alliance, or models a coping response — and it is brief and client-focused, not counselor-centered. The exam punishes lengthy self-disclosure that shifts the focus from client to counselor. Immediacy is appropriate when: the in-session relational dynamic is more clinically important than the content the client is discussing — for example, when the client begins to replicate a problematic relational pattern with the counselor.

The sections above cover attending, four listening skills, and five influencing skills. These are the remaining skills the exam tests.

Reframing

Reframing offers an alternative meaning for the same facts. “She calls three times a day” can be reframed from *intrusive* to *frightened of losing you* — nothing in the situation changes, but the client’s relationship to it does.

Distinguish carefully:

Skill	What Changes
Reflection	Nothing; the counselor mirrors the client’s meaning
Reframe	The meaning assigned to the same facts
Interpretation	A connection is proposed to something outside the client’s current awareness

A reframe must be plausible to the client. A reframe the client cannot accept functions as an argument.

Questioning

Open questions invite elaboration and are the default. **Closed questions** gather specific data and are appropriate for risk assessment, history, and clarifying facts — but a session built on closed questions becomes an interrogation and shifts responsibility for the session’s direction onto the counselor.

The “why” problem. “Why did you do that?” invites defensiveness, rationalization, and intellectualization, because it asks the client to justify rather than explore. Convert to “what” or “how”: *what was happening for you just before that?* This is a frequently scored distinction.

Silence

Therapeutic silence gives the client space to reach material that talking would foreclose. Counselors new to the work fill silence out of their own discomfort. The exam rewards tolerating silence after a significant disclosure or an emerging emotion; it punishes filling it with reassurance.

Cultural note: comfort with silence varies substantially across cultures, and its length should be calibrated rather than applied uniformly.

Minimal Encouragers and Clarification

Minimal encouragers — "mm-hmm," "go on," a repeated key word, a nod — signal attention and sustain the client's flow at minimal cost.

Clarification checks the counselor's understanding when a message is ambiguous: "I want to make sure I'm following — when you say it fell apart, do you mean the relationship or the plan?" It is distinct from paraphrase, which restates a message the counselor already understood.

Normalizing

Normalizing places the client's experience within the range of expected human response: "Most people who go through what you did have trouble sleeping for a while." It reduces shame and secondary distress.

The failure mode is minimizing. "Everyone feels that way sometimes" dismisses rather than normalizes. The distinction is whether the client's specific experience is still being taken seriously after the generalization.

Psychoeducation and Information-Giving

Providing accurate information about a condition, a treatment, or a process is a legitimate and frequently scored intervention — particularly around diagnosis, medication basics within scope, the course of grief, trauma responses, and treatment options.

It is effective when it is timed to the client's readiness, brief, and followed by a check on the client's reaction. It fails when it substitutes for exploration or arrives while the client is in acute distress.

Goal Setting and Contracting

Collaboratively defined goals are what make treatment measurable and are a component of Bordin's working alliance (Bordin, 1979). Well-formed goals are specific, observable, achievable, and stated in the client's language. The exam favors options in which goals are developed *with* the client over options in which the counselor specifies them.

Homework and Between-Session Tasks

Between-session assignments extend the work and are integral to CBT, DBT, and behavioral approaches (Linehan, 2015). Effective assignment involves a clear rationale, collaborative design, anticipation of obstacles, and review at the next session. **Assigning homework and then failing to review it is a scored error** — it teaches the client that the task did not matter.

Focusing

Focusing selects among the several threads a client offers. When a client presents work stress, a marital conflict, and a health worry in one paragraph, the counselor must choose — and the choice is clinical, not arbitrary.

Priority goes to safety concerns, then to affect-laden material, then to the client's stated agenda.

Advice-Giving

The exam consistently scores advice-giving below exploration. It fosters dependence, bypasses the client's own capacity, and typically encodes the counselor's values. Options beginning "You should" or "Have you tried" are rarely correct.

Information-giving is different. Providing factual information the client has requested and that falls within the counselor's scope is appropriate; telling a client what decision to make is not.

Termination and Relapse Prevention Skills

Termination is a clinical phase, not an administrative event. Its components: review progress against goals, consolidate and name gains, anticipate high-risk situations, plan for setbacks, process the ending itself including any grief or anger, and clarify how to return.

Regression shortly before a planned termination is expected and is worked with as material rather than treated as evidence that more sessions are needed.

Which Skill When — Quick Reference

Clinical Situation	Indicated Skill
Client expresses strong emotion	Reflection of feeling
Client's message is ambiguous	Clarification
Client says one thing, does another	Confrontation (with alliance established)
The counselor–client relationship is the live issue	Immediacy
Client is stuck in one meaning of an event	Reframe

Clinical Situation	Indicated Skill
Client feels their reaction is abnormal	Normalizing
Client lacks accurate information	Psychoeducation
Session has multiple competing threads	Focusing
Client has just disclosed something significant	Silence
Session or segment is ending	Summarization

Practice Questions

Question 23. A client describes her conflict with her mother in detail, then adds: “I’m sorry — I just talk too much.” The MOST clinically appropriate response is:

A. “What do you think makes you feel that your sharing isn’t welcome, or that you say too much?”

B. “Not at all — please continue telling me about your mother and what happened between the two of you last week.”

C. “I’m noticing that you apologized for taking up space — I wonder if that happens in other relationships too.”

D. “It sounds like you feel uncomfortable sharing your feelings and worry about how they’ll be received.”

Question 24. A client says: “I want to stop drinking, but then I just feel so anxious when I don’t that I pour myself a drink to calm down.” The counselor’s MOST appropriate response at this stage in treatment (second session, alliance being established) is:

A. “It sounds like alcohol has become your primary way of managing anxiety.”

B. “Have you considered that the alcohol may actually be making the anxiety worse?”

C. “There’s a real tension there — you want to stop, but not drinking brings something painful.”

D. “Let’s talk about what we could put in place to help you manage the anxiety without alcohol.”

Question 25. A client who usually speaks openly arrives and gives one-word answers, avoids eye contact, and crosses her arms. The BEST response is:

A. Continue with the planned session agenda and monitor whether she opens up on her own as the session goes on

B. Ask her directly whether something is wrong and whether she wants to talk about it

C. Ask about any stressors at work or home that may have occurred since the last session

D. Reflect the nonverbal discrepancy: “I notice you seem different today — quieter, maybe more guarded.”

Question 26. Which of the following BEST describes an appropriate use of counselor self-disclosure?

A. “I know exactly how you feel — I went through a painful divorce myself several years ago, so I’ve been where you are.”

B. “I want to be honest with you: when you describe your situation, I feel a sense of sadness for what you’ve been carrying.”

C. “This reminds me of a client I worked with once who had a very similar story and found a way through it.”

D. “I’ve struggled with anxiety too, for most of my twenties, so I really do understand why this has been so hard for you.”

Question 27. A client says, “My mother calls me every single day. She won’t leave me alone.” The counselor responds, “It sounds like she may be frightened of losing touch with you.” This response is BEST described as:

A. Interpretation

B. Reflection of meaning

C. Reframe

D. Confrontation

Question 28. A client falls silent after disclosing that she was assaulted in college. She looks at the floor. The counselor's BEST response is:

A. Allow the silence to continue

B. "Take your time. Can you tell me more about what happened?"

C. "That must have been terrifying."

D. "Many survivors find it difficult to talk about this."

Question 29. A client states that her panic attacks mean she is "going crazy." The MOST appropriate intervention is:

A. Psychoeducation about the physiology of panic and its relationship to the fight-or-flight response

B. Cognitive restructuring of the belief that she is going crazy, using a thought record to test the evidence

C. Reassurance that she is not going crazy and that the attacks will pass with time

D. Interoceptive exposure to the physical sensations she associates with panic

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Question 30. A counselor assigned a client a thought record last session. The client arrives having completed it. The counselor discusses a new topic and does not mention the record. This is:

A. Acceptable if the new topic is more clinically urgent than reviewing the completed thought record

B. Appropriate flexibility in session structure, because the agenda should follow the client's lead

C. A minor oversight without clinical consequence, as the client has already done the work

D. A clinical error, because unreviewed homework teaches the client that the task was not meaningful

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Question 31. A client asks, “Do you think I should leave my husband?” The BEST response is:

A. “I can’t answer that — that’s your decision to make.”

B. “What would it mean for you if you did?”

C. “Based on what you’ve described, I do have concerns about the relationship.”

D. “Let’s look at the pros and cons together.”

Question 32. A client describes, in a single account, a conflict with her supervisor, a recent lump she found in her breast, and her son’s failing grades. The counselor should focus FIRST on:

A. The supervisor conflict, as her stated presenting concern

B. The son’s grades, as the most concrete and solvable problem

C. The lump, and whether she has sought medical evaluation

D. Whichever topic she returns to spontaneously later in the session

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Question 33. Two sessions before a planned termination, a client who has made substantial progress reports that her symptoms have returned and asks to continue indefinitely. The counselor should:

A. Extend treatment, as symptom return indicates she is not yet ready to end counseling

B. Maintain the termination date without discussion to avoid reinforcing her dependence on the counselor

C. Refer her to another counselor for ongoing support after the planned termination date

D. Name the timing of the symptom return and explore what ending the relationship means to her

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Question 34. A client who lost his father three weeks ago says, “I keep expecting to see him. I think something’s wrong with me.” The counselor’s BEST response is:

A. “Sensing the presence of someone who died is a common part of grief, and it doesn’t mean something is wrong with you. What has that been like?”

B. “Everyone grieves differently, and there’s no right or wrong way to feel after losing a parent.”

C. “That sounds like it could be a symptom worth monitoring. Let’s keep track of how often it happens over the next few weeks, and we can look at it together.”

D. “What do you think it means that you keep expecting to see him? Tell me more about when those moments tend to happen.”

Answer Key — Chapter 3 Practice Questions

Q23 — Answer: C

Rationale: This is an immediacy response — the counselor names something happening in the room (the client's apology for speaking) and connects it to a broader pattern. It is clinically rich without being intrusive. B is reassurance (not clinically useful). A is exploratory but misses the relational parallel. D is a reflection of feeling but is weaker than C because it doesn't leverage the here-and-now moment.

Q24 — Answer: C

Rationale: The alliance is being built (second session). The correct response reflects the client's ambivalence using empathy, not information (B) or premature problem-solving (D). A is a paraphrase-adjacent response but interprets rather than reflects. C holds the ambivalence and communicates that the counselor understands both sides — which is what MI-informed and alliance-building responses do.

Q25 — Answer: D

Rationale: This is a confrontation of an incongruity between the client's usual presentation and today's nonverbal behavior. It names the observation tentatively and invites the client to explain. B is too blunt. A ignores clinical data. C redirects to external causes before the relational communication has been addressed.

Q26 — Answer: B

Rationale: This self-disclosure is brief, genuine, client-focused, and uses the counselor's felt response to deepen the alliance. A (claiming "I know exactly how you feel") overidentifies and shifts focus. C (referencing another client) breaches confidentiality norms and shifts focus. D is well-intentioned but the length and framing make it counselor-centered.

Q27 — Answer: C

Rationale: The facts are unchanged and the counselor has offered an alternative meaning for the same behavior, which is the definition of a reframe. A would propose a connection outside the client's awareness, typically to a developmental or unconscious pattern. B would articulate the value or belief beneath the client's own stated feeling, not supply a different account of the mother. D would name an inconsistency in the client's presentation.

Q28 — Answer: A

Rationale: Silence after a significant disclosure gives the client control over what comes next and communicates that the counselor can tolerate the material — filling it, however warmly, takes the moment back. B is gentle but requests more disclosure at exactly the point the client has paused. C names an emotion the counselor has assumed rather than observed. D normalizes, which is useful later but here moves from her specific experience to a general statement.

Q29 — Answer: A

Rationale: The belief is an information deficit before it is a cognitive distortion, and explaining the autonomic mechanism directly resolves the catastrophic misinterpretation that maintains panic. B treats an accurate-information problem as a thinking error and is less efficient. C reassures without supplying the understanding that makes the reassurance credible. D is an appropriate later component but is premature before the client understands what her body is doing.

Q30 — Answer: D

Rationale: Reviewing assigned work is part of the intervention, not an optional courtesy — failing to review it reliably reduces future compliance and communicates that the assignment was filler. A is the strongest distractor and would hold only if genuine urgency were described, and

none is. B misnames neglect as flexibility. C understates a well-documented effect on engagement.

Q31 — Answer: B

Rationale: The question is an invitation into exploration, and an open question about meaning keeps the decision with the client while doing clinical work with what the question expresses. A is technically correct but declines without offering anything, which can be experienced as withholding. C gives advice on a major life decision, encoding the counselor's values. D is a reasonable later step but moves to structured problem-solving before the meaning of the question has been explored.

Q32 — Answer: C

Rationale: Focusing prioritizes safety, and a potential medical concern outranks psychosocial content — verifying that she has arranged evaluation is both a clinical and an ethical necessity. A follows her stated agenda while a potential health emergency goes unaddressed. B selects on tractability rather than clinical priority. D defers a decision the counselor is responsible for making.

Q33 — Answer: D

Rationale: Pre-termination regression is an expected phenomenon, and the clinical response is to make the pattern explicit and work with the meaning of the ending — which is often the most valuable material in the treatment. A accepts the symptoms at face value and forfeits that work. B is rigid and treats a communication as a behavior to be extinguished. C hands off the termination process rather than completing it.

Q34 — Answer: A

Rationale: This normalizes accurately — sensing the presence of the deceased is a well-documented feature of ordinary grief — and then returns to his specific experience, which is what separates normalizing from minimizing. B is a generic platitude that dismisses rather than normalizes. C pathologizes a normative grief phenomenon and

would increase his alarm. D explores without first relieving the fear that something is wrong with him, which is the live concern in the stem.

Chapter 4 — Group Counseling & Group Work

Overview

The Group as Client

The single most important conceptual shift in group work is this: the group is the client, not the collection of individuals. The leader's role is not to do individual counseling with multiple people watching. It is to tend the group as a living system — its norms, its cohesion, its stage, its safety. Under pressure, the exam will test your ability to prioritize the group's needs over an individual member's immediate wishes. When one member dominates, the leader's first responsibility is to the group's ability to function. When conflict erupts between members, the leader's job is to help the group process the conflict, not to adjudicate it. When a member discloses a serious personal crisis, the leader must both tend to that member and tend to the group's response to the disclosure. Leader priority order under pressure: 1. Safety of all group members 2. Group cohesion and the ability of the group to function 3. The therapeutic progress of individual members 4. Efficient use of group time

Group Types

Groups differ by purpose and by the leader's role. The table below summarizes the major types.

Type	Purpose	Leader Role
Psychoeducational	Skill-building, information	Teacher/facilitator
Skills-training	Practice specific skills (DBT, parenting)	Coach/trainer

Type	Purpose	Leader Role
Cognitive-behavioral	Change distorted cognitions and behavior patterns	Active, structured
Support	Normalize, provide peer support	Facilitator, low-directive
Process/ interpersonal	Use group dynamics as change mechanism	Process observer, catalytic
Psychotherapy	Deep personality/ relational change	Clinician, boundary-holder

Tuckman's Stages of Group Development (Tuckman, 1965; Tuckman & Jensen, 1977)

Stage	Group Task	Leader Focus
Forming	Inclusion, safety, initial trust	Establish norms, provide structure
Storming	Conflict, challenge, jockeying for position	Normalize conflict, protect safety, don't shut it down
Norming	Cohesion, established roles, deepened trust	Step back, facilitate member-to-member interaction
Performing	Productive work, self-disclosure, insight	Catalyst, minimal direction
Adjourning	Termination, integration, grief	Process endings, transfer of learning

Exam trap: in the storming stage, the instinct is to suppress conflict. The correct response is to name and normalize conflict while keeping it safe — because navigating conflict is part of the therapeutic work. A leader who deflects all conflict keeps the group in perpetual forming.

Yalom's Therapeutic Factors

Yalom identified eleven therapeutic factors unique to group therapy (Yalom & Leszcz, 2020). The exam most commonly tests these five:

Factor	What It Means
Universality	"I'm not alone in this" — the relief of shared human experience
Cohesion	The group's felt sense of connection and belonging (the group equivalent of alliance)
Altruism	Members gain from giving help to others, not just receiving it
Instillation of hope	Seeing others improve provides evidence that change is possible
Interpersonal learning	The group becomes a social microcosm in which members can learn about their relational patterns

When the exam asks why group therapy is particularly powerful for clients with long-standing relational difficulties, the answer is interpersonal learning — the group provides live data about how the client relates to others that no individual therapy can replicate.

Group Ethics: The Confidentiality Problem

This is the most important ethical principle specific to group work: the counselor can promise confidentiality from themselves but cannot guarantee it from other group members. This must be disclosed in pre-group screening and in the group agreement. The counselor can establish a confidentiality norm and expectation, but cannot enforce it legally among members.

Exam implication: if a group member asks "Is what I share confidential?", the correct answer acknowledges the norm and its limits — it does not simply say "yes."

Leadership Skills with Mini-Scenarios

Linking connects what one member said to something another member expressed: "I notice that what Danielle is describing — feeling invisible at home — resonates with what Marcus shared last week about his work relationships. I wonder if there's something there for the group."

Blocking stops behavior that would harm the group: a member who gives unsolicited advice, asks intrusive questions, or dominates speaking time. “Let me pause you there, Derek — before you respond, I want to hear from others about their reactions to what Sarah just shared.”

Drawing out invites silent members to participate: “Jordan, you’ve been very still during this conversation. I’m curious what’s coming up for you.”

Cutting off — distinct from blocking — ends a member’s sharing when it becomes too lengthy or is keeping the group stuck. Done respectfully but firmly.

Yalom’s Remaining Therapeutic Factors

The section above teaches five of eleven. The other six:

Factor	What It Means
Imparting information	Didactic instruction and direct advice from the leader or members
Corrective recapitulation of the primary family group	The group reproduces family-of-origin dynamics, allowing old patterns to be relived and revised — members often relate to the leader as a parent and to peers as siblings
Development of socializing techniques	Members acquire basic social skills through feedback and practice
Imitative behavior	Members learn by observing and modeling the leader and one another
Catharsis	Emotional release — necessary but not sufficient; it produces change only when paired with cognitive understanding
Existential factors	Confronting responsibility, mortality, and the limits of what others can provide

Corrective recapitulation is the most-tested of these six. When a stem describes a member reacting to the leader as they did to a parent, or competing with another member as with a sibling, that is the factor being assessed.

Yalom's Here-and-Now — the Two Tiers

Yalom's central technical contribution is that here-and-now work requires **two steps, and either alone is insufficient**:

1. **Experience** — members interact affectively in the present moment
2. **Illumination of process** — the group then examines what just happened between them

Experience without illumination is an emotional event that does not generalize. Illumination without experience is an intellectual exercise. The leader's job is to keep both operating.

Corey's Stages, Alongside Tuckman's

Tuckman's model was derived from a review of research on therapy, training, and laboratory groups. Most counseling curricula and item banks use Corey's group stages, and stems may use either vocabulary (Corey et al., 2018).

Corey	Approximate Tuckman Equivalent	Central Task
Initial	Forming	Orientation, inclusion, establishing trust and norms
Transition	Storming	Anxiety, defensiveness, conflict, challenges to the leader

Corey	Approximate Tuckman Equivalent	Central Task
Working	Norming + Performing	Cohesion, productive disclosure, member-to-member work
Final	Adjourning	Consolidating learning, processing the ending, transfer

The transition stage is where most difficult leader decisions occur, and the exam's treatment of it mirrors the storming trap already noted: conflict is worked with, not suppressed.

Pre-Group Screening and Preparation

Screening is an ethical requirement, not a scheduling formality. Its purposes: assess fit with the group's purpose, assess capacity to participate without harming self or others, and prepare the member.

Generally inappropriate for heterogeneous process groups: clients in acute crisis, actively psychotic clients, clients at acute suicidal risk, clients who are actively intoxicated, and clients whose presentation would predictably monopolize or destabilize the group. These are not permanent exclusions — they are indications that a different level of care or a different group format is needed first.

Preparation covers purpose, format, norms, the confidentiality limit, what to expect in early sessions, and the right to decline participation in any exercise.

Difficult Member Roles

The Leadership Skills section above teaches the leader skills; these are the presentations they respond to.

Role	Presentation	Leader Response
Monopolizer	Fills silence, dominates airtime, often anxious underneath	Block and redirect; eventually address the underlying anxiety with the group
Silent member	Withdrawn; may be observing, frightened, or excluded	Draw out without coercion; explore what the silence means in the group
Help-rejecting complainer	Presents problems, then defeats every suggestion	Name the pattern; avoid the group's escalating rescue attempts
Self-righteous moralist	Judges other members; positions self as expert	Block harm to members; explore the need for the position
Scapegoat	Absorbs the group's projected conflict and is targeted collectively	Interrupt the process and turn the group's attention to what it is doing — this is a group dynamic, not a member problem

Scapegoating is the most testable of these because the correct response is counterintuitive: the leader addresses the group, not the scapegoated member.

Co-Leadership

Advantages: complementary perspectives, one leader can attend to process while the other attends to content, modeling of healthy adult relating, and coverage for absence.

Risks: **splitting**, in which members align with one leader against the other, and unresolved co-leader disagreement that leaks into the group. Regular co-leader processing time outside the group is a requirement, not a luxury.

Composition, Format, and Size

Dimension	Considerations
Homogeneous	Shared presenting concern; builds cohesion and universality quickly; preferred for skills, psychoeducational, and support groups
Heterogeneous	Diverse presentations; richer interpersonal learning; preferred for process and interpersonal groups
Closed	Fixed membership and start/end dates; deeper cohesion; easier stage progression
Open	Members join and leave; continuous availability; group repeatedly returns to earlier stages as members enter
Size	Roughly 6–10 for process and psychotherapy groups; too few limits interpersonal data, too many limits airtime

Ethical Standards Specific to Groups

Beyond the confidentiality limit already covered:

- **Voluntary participation and freedom to exit.** Members must not be coerced into exercises, disclosure, or continued attendance. Pressure from the group itself is a form of coercion the leader must interrupt.
- **Informed consent for groups** covers purpose, leader qualifications, format, norms, risks — including that group participation can be distressing — and the confidentiality limitation.
- **Screening** as an ethical obligation, as above.
- **Dual relationships among members.** Members who know each other outside the group require explicit discussion, and in some settings different group placement.

- **Protection from harm.** The leader must actively intervene against attacks, coercion, and scapegoating rather than treating them as material to be processed indefinitely.

Mandated and Involuntary Members

Court-ordered and agency-mandated members are common in substance use, anger management, and correctional settings. The productive stance acknowledges the coercion openly rather than pretending the member chose to attend, clarifies what is and is not reported back to the referring body, and locates whatever genuine autonomy exists within the requirement — usually how the member uses the time rather than whether they attend.

Leadership Styles (Lewin et al., 1939)

Style	Characteristics	Effect
Authoritarian	Leader directs; high structure, low member input	Efficient but dependent; limited member-to-member work
Democratic	Leader facilitates; shared decision-making	Highest member satisfaction and cohesion; the general default
Laissez-faire	Leader minimally involved	Low structure; often anxious, unproductive groups, particularly early

Termination in Groups

Two distinct events, and the exam distinguishes them:

A single member leaving an ongoing group requires advance notice, an opportunity for the departing member to say goodbye and receive feedback, and — critically —

processing with the remaining members, who may experience the departure as abandonment or as evidence that leaving is possible.

The whole group ending involves reviewing gains, addressing unfinished business between members, planning transfer of learning, and processing the loss of the group itself.

Cultural Considerations

Norms around self-disclosure, emotional expression, deference to authority, and direct confrontation vary substantially across cultures. A member from a collectivist background who is reticent in early sessions may be behaving appropriately by their own norms rather than resisting. Leaders who read cultural difference as pathology or resistance produce exactly the disengagement they then diagnose.

Practice Questions

Question 35. During the sixth session of a process group, two members begin arguing about whose disclosure is "more serious." Other members look uncomfortable. The group leader's BEST response is:

A. Remind the group that all experiences are valid and redirect the discussion back to the planned agenda

B. Ask one of the arguing members to step back and listen so the other can finish speaking

C. Name the tension in the room and invite the group to explore what is happening between them

D. Take a break and regroup after a few minutes of silence

Question 36. A new member joins an established process group and immediately begins sharing extensive personal history, causing other members to shift uncomfortably. The leader should:

A. Allow the new member to finish, as interrupting would damage the alliance

B. Provide a brief empathic reflection and redirect by checking in with the group

C. Ask the new member to schedule an individual session to share the history

D. Remind the new member of the group's disclosure norms at the start of the next session

Question 37. Which Yalom therapeutic factor is MOST specifically activated when a group member who struggled with addiction for years shares that she is celebrating two years of sobriety?

A. Universality

B. Altruism

C. Cohesion

D. Instillation of hope

Question 38. Before a new member joins a therapy group, the counselor explains that what members share in group is confidential. Which statement MOST accurately represents the counselor's ethical obligation?

A. "Everything shared in this group is confidential by law, so nothing you say here can be repeated outside of it."

B. "I will keep what you share confidential, and I strongly encourage other members to do the same — though I cannot legally bind them."

C. "Group members have agreed to confidentiality, so you can be assured your disclosures are protected and will stay within this room."

D. "Confidentiality in groups works the same way as in individual therapy, with the same legal protections and the same exceptions."

Question 39. In the eighth session of a process group, a member becomes visibly angry at the leader for "always favoring" another member, and describes feeling as he did when his younger brother was preferred at home. This BEST illustrates which of Yalom's therapeutic factors?

A. Corrective recapitulation of the primary family group

B. Interpersonal learning through the group as a social microcosm

C. Transference

D. Imitative behavior

Question 40. Over several sessions, group members increasingly direct criticism at one member, who absorbs it without defending herself. The leader's BEST response is:

A. Support the targeted member and set a limit on the criticism

B. Explore with the targeted member why she does not defend herself when the group criticizes her

C. Meet with the targeted member individually to assess her tolerance for the group's criticism

D. Turn the group's attention to what the group is doing and what it may be avoiding

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Question 41. A prospective member for an interpersonal process group is currently experiencing command auditory hallucinations and has not been stabilized on medication. The counselor should:

A. Decline group placement at this time and address the appropriate level of care

B. Admit the member, as the support and structure of the group may be stabilizing

C. Admit the member on a trial basis with close monitoring by the leader during sessions

D. Admit the member to the group but limit their participation

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Question 42. A group member says, “I don’t want to do the role play.” Other members encourage her, saying it helped them. The leader should:

A. Encourage her to try, since the group’s support suggests it is safe

B. Ask her to observe the first round and then participate in the second round

C. Explore her reluctance in front of the group until she is willing

D. Affirm her right to decline and interrupt the group’s pressure

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Question 43. Members of an open therapy group have worked productively for months. Three new members join in the same week. The leader should expect:

A. Continued work at the current depth, since the established members will carry the group

B. A brief adjustment period followed by a return to the group’s prior level of functioning

C. Resistance from established members that should be interpreted to the group as territoriality

D. A return to earlier-stage dynamics, requiring renewed attention to norms, safety, and inclusion

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Question 44. A member repeatedly presents problems and rejects every suggestion offered, saying “yes, but” to each. Members are becoming frustrated. The leader’s BEST response is:

A. Direct the group to stop offering suggestions so the member can no longer reject them

B. Ask the member to bring the concern to individual therapy instead, where it can get more attention

C. Confront the member directly about the effect of the "yes, but" behavior on the other members

D. Name the pattern and invite the group to examine what happens between this member and the group

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Question 45. A group member gives two weeks' notice that she is leaving an ongoing group. The leader's MOST important task, beyond the departing member's own process, is:

A. Determining whether to admit a replacement member

B. Assessing whether the departure reflects unresolved conflict

C. Ensuring the member has an appropriate referral

D. Processing the departure with the remaining members

Question 46. A member from a collectivist cultural background speaks rarely in the first four sessions of a process group and defers to the leader when addressed. The leader should FIRST:

A. Consider that the behavior may be culturally normative and explore the member's experience of the group without labeling it as resistance

B. Use drawing-out techniques, such as inviting the member by name, to increase participation in group discussion

C. Discuss the group norm of active participation with the member and clarify the leader's expectations for verbal contribution

D. Interpret the deference as an expression of the corrective recapitulation factor, reflecting a reenactment of early family dynamics

Answer Key — Chapter 4 Practice Questions

Q35 — Answer: C

Rationale: This is storming behavior. The therapeutic response is to use the conflict as material — to name it, hold it, and invite the group to process it together. A deflects the conflict (misses the therapeutic opportunity). B singles out a member (not the leader's adjudication role). D avoids (the conflict will persist).

Q36 — Answer: B

Rationale: The leader must balance warmth toward the new member with responsibility to the group's functioning. A brief empathic reflection honors the disclosure; redirecting to the group restores the shared process. A ignores the group's discomfort. C is not the right mechanism. D is too delayed to address the current session dynamic.

Q37 — Answer: D

Rationale: Seeing a member succeed provides evidence that recovery is possible — this is the definition of instillation of hope. Universality is about shared pain, not observed success. Altruism involves giving help. Cohesion is the relational bond, not a specific event.

Q38 — Answer: B

Rationale: This is the only response that accurately distinguishes what the counselor can guarantee (their own confidentiality) from what they cannot guarantee (member confidentiality). All other options overstate the protection.

Q39 — Answer: A

Rationale: The group is reproducing family-of-origin dynamics — the leader in the parental position, another member in the sibling position — which is exactly what this therapeutic factor names, and the explicit family reference makes it unambiguous. B is broader and describes the general mechanism by which the group functions as a

social microcosm; the family-specific reenactment is the more precise answer. C names the psychodynamic phenomenon but is not one of Yalom's therapeutic factors, which is what the group framework asks for. D involves learning by modeling others.

Q40 — Answer: D

Rationale: Scapegoating is a group process, not a property of the scapegoated member, and the intervention has to address the group — usually the conflict or anxiety it is displacing onto one person. A protects in the moment but leaves the process running. B places the burden on the member the group is targeting. C removes the issue from the room where it is occurring.

Q41 — Answer: A

Rationale: Screening exists to assess capacity to participate without harm to self or the group, and an unstabilized member with command hallucinations meets neither condition — the appropriate response addresses level of care first. B misapplies the genuine value of group support to a member who cannot currently use it. C treats a screening decision as an experiment on the group. D admits a member into a structure the leader then has to manage around, which serves neither the member nor the group.

Q42 — Answer: D

Rationale: Voluntary participation is an ethical standard, and group pressure is a form of coercion the leader is obligated to interrupt rather than to enlist. A joins the pressure. B is a softened version of the same override, presenting participation as inevitable. C is the strongest distractor — exploration can be valuable — but "until she is willing" makes the exploration instrumental to compliance rather than genuinely open.

Q43 — Answer: D

Rationale: Open groups cycle back through earlier stages when membership changes, and a substantial influx returns the group to initial-stage tasks — trust, norms, in-

clusion — regardless of how well it was functioning. A and B both underestimate an effect that is predictable and well described. C pathologizes an expected response; established members' reactions are material to be processed, not a fault to be interpreted as territorial.

Q44 — Answer: D

Rationale: The help-rejecting complainer pattern is interpersonal and is best addressed by making the transaction itself visible to everyone participating in it, which is also the here-and-now illumination the group format exists to provide. A stops the surface behavior without addressing the dynamic. C singles the member out and risks scapegoating exactly the person whose pattern invites it. B removes from the group the material the group is uniquely positioned to work with.

Q45 — Answer: D

Rationale: A member leaving is a group event, and the remaining members may experience it as abandonment, as rejection of the group, or as permission — none of which resolves on its own. A is administrative and follows. B may be worth exploring but is part of the departing member's process, which the stem sets aside. C matters only if she is leaving treatment entirely, which is not stated.

Q46 — Answer: A

Rationale: Reticence and deference to authority are normative in many cultural contexts, and reading them as resistance or pathology produces exactly the disengagement it then diagnoses — understanding precedes intervention. B applies a technique before the meaning of the behavior is known. C imposes a norm that itself carries cultural assumptions about what participation looks like. D reaches for a psychodynamic explanation when a cultural one is more parsimonious and has not been ruled out.

Chapter 5 — Intake, Assessment, and the Mental Status Exam

Overview

Intake & Assessment is the third-largest NCMHCE domain in NBCC's 2027 specifications, at 18% of items, and it includes selecting the diagnosis (NBCC, 2026). Every case opens with an intake summary, and many of the questions that follow ask what the counselor should assess, screen for, or rule out *before* acting. The diagnostic chapters in Part Two teach what each disorder looks like. This chapter teaches the process that gets you there: the order of the intake, the biopsychosocial interview, the mental status exam, cultural formulation, screening and instrument selection, level-of-care decisions, and measuring whether counseling is working.

The core rule of the domain: **assess before you intervene, and assess safety before anything else.** Most wrong answers in this domain are good interventions offered too early.

The Intake Sequence

A first session has competing demands: safety, rapport, informed consent, information-gathering, and a working diagnosis. The exam expects you to know which comes first.

Order	Task	Why it comes here
1	Informed consent and limits of confidentiality	The client must know the rules before disclosing anything (ACA, 2014, Standard A.2.a)

Order	Task	Why it comes here
2	Immediate safety screen	Suicide, homicide, abuse, medical emergency, or intoxication overrides everything else
3	Presenting problem in the client's words	Anchors the alliance and the treatment goals
4	Biopsychosocial history	Context for diagnosis and treatment planning
5	Mental status exam	Structured observation of current functioning
6	Screening instruments as indicated	Quantifies severity and flags co-occurring problems
7	Working diagnosis and level of care	Drives the treatment plan
8	Collaborative next steps	Client leaves knowing what happens next

This sequence is the Priority Ladder applied to a first session (see The Clinical Reasoning Toolkit): the safety screen is rung 1, medical and substance rule-outs gathered in the history are rung 2, and the presenting problem, history, and mental status exam make up rung 4, engagement and assessment. In practice, safety concerns can surface at any point. If a client discloses suicidal intent while describing their history, the counselor stops the history and assesses risk (see Chapter 19).

The Biopsychosocial Interview

The biopsychosocial model holds that illness and distress arise from the interaction of biological, psychological, and social factors, and that no single level explains the whole picture (Engel, 1977). The intake interview is organized around it.

Domain	What to gather
Biological	Medical conditions, medications, sleep, appetite, pain, substance use, family psychiatric and medical history, last physical exam
Psychological	Presenting symptoms and their onset, duration, and course; prior treatment and what helped; trauma history; coping strategies; strengths
Social	Living situation, relationships, supports, work or school, finances, legal issues, culture, faith, identity
Developmental	Early history, milestones, education, significant life transitions
Risk	Current and past suicidal or homicidal ideation, self-harm, violence, access to means, abuse or neglect

How this shows up on the exam: The stem gives you partial information and asks what the counselor should explore next. The rewarded answer fills the gap that would change the diagnosis or the plan. The most common gaps are medical causes (thyroid, medications, recent illness), substance use, and a history of mania or hypomania before treating depression.

The Mental Status Exam

The mental status exam (MSE) is a structured description of how the client presents *right now*. It is observation plus a few direct questions, and it becomes part of the record at intake and whenever the client's condition changes (Trzepacz & Baker, 1993). The NCMHCE uses MSE vocabulary in intake summaries and expects you to interpret it.

Category	What is observed	Terms the exam uses
Appearance	Grooming, hygiene, dress, apparent age, notable features	Well-groomed, disheveled, malodorous, appears older than stated age
Behavior and psychomotor activity	Eye contact, posture, movement, cooperation	Psychomotor agitation or retardation, tremor, tics, guarded, cooperative
Speech	Rate, volume, rhythm, amount	Pressured, slowed, soft, monotone, poverty of speech
Mood	The client's own description of emotional state, in their words	"I feel empty," "I'm fine"
Affect	The emotion the counselor observes	Full, constricted, blunted, flat, labile; congruent or incongruent with mood
Thought process	How thoughts are organized	Linear, goal-directed, circumstantial, tangential, loose associations, flight of ideas
Thought content	What the client is thinking about	Suicidal or homicidal ideation, obsessions, preoccupations, delusions
Perception	Sensory experiences without external stimulus	Auditory or visual hallucinations, illusions, depersonalization
Cognition	Orientation, attention, memory	Oriented ×3 (person, place, time), impaired recent memory, distractible
Insight	Awareness of having a problem and its nature	Good, partial, poor

Category	What is observed	Terms the exam uses
Judgment	Ability to make sound decisions	Intact, impaired (often illustrated by recent choices)

Three distinctions the exam tests repeatedly:

- **Mood vs. affect.** Mood is what the client *reports*; affect is what the counselor *observes*. A client who says "I'm fine" with tearful, constricted affect has mood–affect incongruence, which is worth exploring.
- **Thought process vs. thought content.** Tangential speech is a process problem. A fixed false belief is a content problem.
- **Orientation and acute confusion.** Disorientation with fluctuating attention and a sudden onset points to delirium, a medical emergency (see Chapter 15).

Cultural Formulation

DSM-5-TR includes the Cultural Formulation Interview (CFI), a 16-question semistructured interview that helps the counselor understand the problem from the client's cultural perspective (American Psychiatric Association [APA], 2022). It covers four areas:

1. **Cultural definition of the problem**: how the client and their family and community describe and explain it.
2. **Cultural perceptions of cause, context, and support**: what the client believes caused the problem, and which stressors and supports matter, including identity, faith, and community.
3. **Cultural factors affecting self-coping and past help-seeking**: what the client has already tried, including traditional, religious, or family remedies.
4. **Cultural factors affecting current help-seeking**: the client's preferences, and any concerns about the counselor, the setting, or the treatment.

How this shows up on the exam: When a client's presentation could be a culturally expected response (for example, grief practices, spiritual experiences, or idioms of distress), the rewarded answer asks about cultural meaning *before* diagnosing. Labeling a culturally normative experience as pathology is a classic distractor. Appendix A covers multicultural competence more broadly.

Screening, Assessment Instruments, and SBIRT

A **screen** flags possible problems quickly; a **diagnostic assessment** confirms or rules them out. A positive screen is never a diagnosis by itself. The counselor follows it with a clinical interview.

Choosing the right instrument is a common question type. Match the tool to the question being asked:

Clinical question	Typical instrument	Source
Depression severity and tracking	PHQ-9	Kroenke et al., 2001
Anxiety severity	GAD-7	Spitzer et al., 2006
Hazardous alcohol use	AUDIT or AUDIT-C	Saunders et al., 1993; Bush et al., 1998
Suicide risk	C-SSRS	Posner et al., 2011
PTSD symptoms	PCL-5	Weathers et al., 2013
Cognitive screen	MoCA	Nasreddine et al., 2005

The Assessment Tools Compendium in Part Three lists cutoffs for every instrument in this book.

Screening, Brief Intervention, and Referral to Treatment (SBIRT) is a public health approach to substance use: universal screening, a brief motivational conversation for people with risky use, and referral to specialty treatment for those who need it (Babor et al., 2007). The exam favors universal screening for substance use at in-

take rather than screening only clients who "look like" they have a problem.

At-Risk Behavior Assessment

NBCC lists assessing risk to self and others as a core assessment task (NBCC, 2026). At intake, every client is asked directly about:

- Suicidal ideation, plans, intent, preparatory behavior, and past attempts
- Homicidal or violent ideation and any identifiable target
- Nonsuicidal self-injury
- Abuse or neglect, whether the client is being harmed or a child, elder, or dependent adult in their care may be
- Access to firearms and other lethal means

Asking directly about suicide does not increase risk. Chapter 19 covers risk formulation, safety planning, and duty-to-warn decisions.

Trauma Assessment

Assess trauma exposure gently and in stages. At intake, the goal is to learn *whether* trauma occurred and how it affects current functioning, not to elicit a detailed account. Pushing for trauma details in the first session can destabilize the client. The rewarded sequence is safety and stabilization first, then structured assessment (for example, the PCL-5), and processing only later in treatment (see Chapters 10 and 22).

Determining Level of Care

Level of care is matched to risk, severity, functioning, and support, not to the client's preference alone or to what the counselor happens to offer. The continuum runs from least to most intensive:

Level	Typical indications
Outpatient	Stable, safe, functioning with some impairment
Intensive outpatient (IOP)	Needs several hours of treatment per week while living at home
Partial hospitalization (PHP)	Needs daily structured treatment but has a safe place to sleep
Residential	Needs a 24-hour structured setting; unsafe or unsupportive environment
Inpatient hospitalization	Imminent danger to self or others, inability to care for self, or medical instability

For substance use, the ASAM Criteria assess six dimensions: acute intoxication and withdrawal potential; biomedical conditions; emotional, behavioral, or cognitive conditions; readiness to change; relapse or continued-use potential; and the recovery environment (American Society of Addiction Medicine [ASAM], 2013). Chapter 11 lists the ASAM levels of care, from early intervention (Level 0.5) to medically managed intensive inpatient services (Level 4). Withdrawal risk from alcohol or benzodiazepines is a medical priority that may require medically managed detoxification before counseling can proceed (see Chapter 11).

How this shows up on the exam: The rewarded answer chooses the *least restrictive level of care that is safe*. Hospitalizing every client who mentions suicidal thoughts is wrong. So is keeping an imminently suicidal client in weekly outpatient care.

Evaluating Counseling Effectiveness

NBCC includes evaluating treatment effectiveness and client response as a continuity-of-care task (NBCC, 2026). Measurement-based care means routinely giving a brief standardized measure (such as the PHQ-9 or GAD-7), reviewing the results *with* the client, and using them to ad-

just treatment. It improves outcomes compared with relying on clinical impression alone (Lewis et al., 2019).

The exam rewards the counselor who:

- Establishes a baseline score at intake
- Re-administers the same measure at regular intervals
- Discusses the scores with the client as part of the work
- Revisits the diagnosis, the plan, or the level of care when scores do not improve

Chapter 6 covers how to revise a treatment plan when the data show the client is not improving.

What the Exam Rewards (and Punishes) — Intake and Assessment

Rewards: Informed consent and confidentiality limits before disclosure. A direct safety screen at every intake. Ruling out medical and substance causes before a psychiatric diagnosis. Asking about prior manic or hypomanic episodes before treating depression. Using MSE terms precisely. Following a positive screen with a clinical interview. Asking about cultural meaning before diagnosing. Choosing the least restrictive safe level of care. Tracking outcomes with a standardized measure.

Punishes: Starting an intervention before the assessment is complete. Treating a screening score as a diagnosis. Skipping substance use screening because the client "doesn't seem the type." Eliciting detailed trauma narratives in the first session. Confusing mood with affect. Pathologizing culturally normative experiences. Choosing a level of care by preference or convenience instead of risk.

Worked Mini-Case — Intake and Assessment

Vignette: Denise, 52, is referred by her primary care physician for "depression." At intake she reports three months of fatigue, weight gain, low mood, and trouble

concentrating. She is dressed neatly, speaks slowly, and has a constricted affect. She says her mood is “flat.” She denies suicidal ideation. She drinks two to three glasses of wine most nights “to sleep.” She has not had blood work in over a year. She says her family “doesn’t really talk about feelings — we pray and push through.”

Reasoning chain:

Step 1 — Consent and safety: Review consent and confidentiality limits, then screen directly for suicide risk (the C-SSRS screener) even though she denies ideation.

Step 2 — Medical rule-out: Fatigue, weight gain, slowed speech, and low mood can reflect hypothyroidism or another medical condition. Coordinate with her physician for labs before settling on a primary depressive diagnosis.

Step 3 — Substance screening: Nightly alcohol use for sleep calls for a structured screen (AUDIT-C) and exploration. Alcohol can cause or worsen depressive symptoms and insomnia.

Step 4 — MSE and measures: Document the MSE (neat appearance, psychomotor slowing, constricted affect congruent with a “flat” mood, linear thought process, no perceptual disturbance, oriented). Administer the PHQ-9 for a baseline.

Step 5 — Culture: Ask what “pushing through” and prayer mean to her, and how her faith community views counseling, before building a plan.

Step 6 — Level of care: With no safety risk and intact functioning, outpatient care is appropriate. Revisit this if screening or labs change the picture.

Check Your Understanding — Intake and Assessment

Assessment — MSE. A client describes his mood as “great, never better” while tearful and speaking softly. In the MSE, this is BEST documented as:

A. Euthymic mood with appropriate affect, consistent with the client's own description of feeling great.

B. Mood–affect incongruence: reported mood "great," observed affect tearful and constricted.

C. Flight of ideas, given his upbeat self-report despite visible tearfulness and soft speech.

D. Poor judgment, shown by his insistence that he feels great while visibly tearful and subdued.

Answer: B. Mood is the client's report and affect is the counselor's observation. Here they do not match. Euthymic and appropriate (A) contradicts the observation. Flight of ideas (C) describes thought process. Nothing in the vignette addresses judgment (D).

Assessment — level of care. A client with alcohol use disorder reports drinking a fifth of vodka daily and had a seizure the last time she stopped. The MOST appropriate next step is:

A. Begin weekly outpatient motivational interviewing to build her readiness for change.

B. Refer for medically managed withdrawal before or alongside counseling.

C. Recommend she taper on her own over two weeks to reduce withdrawal intensity.

D. Administer the AUDIT to confirm severity before deciding on a level of care.

Answer: B. A history of withdrawal seizures signals a risk of complicated, potentially fatal alcohol withdrawal. Medical management comes first (ASAM, 2013). A and C expose her to withdrawal without medical support. D gathers information the counselor already has.

Chapter 6 — Treatment Planning

Overview

Treatment Planning & Continuity of Care accounts for 15% of NCMHCE items in NBCC's 2027 specifications (NBCC, 2026). The diagnostic chapters tell you which treatment has the best evidence for each disorder. This chapter covers the planning half of the domain: turning an assessment into a plan the client agrees with, writing goals and objectives that can actually be measured, sequencing the work, choosing the right level of care, and revising the plan when the client is not improving. The Continuity of Care section at the end of the chapter covers the other half: referral, aftercare and discharge plans, maintaining progress, relapse prevention, planning for disruption of service, and termination criteria.

The ACA Code of Ethics frames the task: counselors and clients work jointly on counseling plans that offer reasonable promise of success, are consistent with the client's abilities, temperament, developmental level, and circumstances, and are regularly reviewed and revised (ACA, 2014, Standard A.1.c). Every clause of that standard shows up in exam questions.

Anatomy of a Treatment Plan

Most treatment plans follow the same structure (Jongsma et al., 2021):

Component	What it is	Example (client with panic disorder)
Problem statement	The presenting concern, in behavioral terms	Panic attacks 3–4 times weekly; avoids driving and grocery stores

Component	What it is	Example (client with panic disorder)
Long-term goal	The broad outcome the client wants	Manage panic without avoiding daily activities
Short-term objectives	Specific, measurable steps toward the goal	Client will drive to the grocery store alone twice weekly within 6 weeks
Interventions	What the counselor will do	Psychoeducation on the panic cycle; interoceptive and in vivo exposure (CBT)
Measures	How progress will be tracked	Panic frequency log; Panic Disorder Severity Scale (PDSS) every 2 weeks
Target dates and review	When progress will be checked	Review at session 6 and session 12

Goals describe *where* the client is going; objectives describe the *observable steps* that show they are getting there. A common exam error is a "goal" that is really an intervention ("Client will attend CBT").

Writing Measurable Objectives

Objectives should be **specific, measurable, achievable, relevant, and time-bound (SMART)**. The exam often presents several objectives and asks which one is best written.

Weak objective	Problem	Strong objective
Client will feel less depressed.	Not measurable, no timeframe	Client's PHQ-9 will drop from 18 to below 10 within 8 weeks.
Client will stop drinking.	All-or-nothing; may not match readiness	Client will record daily drinks and reduce from 5 to 2 or fewer on weekdays within 4 weeks.

Weak objective	Problem	Strong objective
Client will improve communication with spouse.	Vague	Couple will use a structured speaker–listener exercise for one disagreement per week, reported in session, for 6 weeks.
Counselor will teach coping skills.	Describes the counselor, not the client	Client will use a grounding skill during 3 of 4 reported panic episodes within 5 weeks.

Objectives belong to the client. They describe what the client will do or experience, in words the client helped choose.

Collaboration, Strengths, and Barriers

Collaboration. The client's goals, in the client's language, drive the plan. When the counselor's priorities and the client's priorities differ, the rewarded answer explores the difference instead of overriding the client — with one exception. Safety always comes first, whether or not the client lists it.

Strengths. Every plan names the client's strengths and resources: supports, skills, past successes, faith, motivation, and community. Strengths are more than encouragement — they become the tools the interventions use.

Barriers. Anticipate what could get in the way: transportation, cost, childcare, work schedule, cognitive limits, language, stigma, ambivalence, unsafe housing, or co-occurring conditions. A plan that ignores barriers sets the client up for "noncompliance" that is really a planning failure.

Sequencing: What Comes First

When a client has several problems, the Priority Ladder (see The Clinical Reasoning Toolkit) decides the order:

1. **Imminent safety**: suicide, violence, abuse or neglect, medical danger, and withdrawal risk
2. **Medical and substance rule-outs**: physical causes and substance effects that could explain the symptoms
3. **Stabilization**: acute symptoms, basic needs, crisis resources
4. **Engagement and assessment**: alliance, readiness to change, and a clear clinical picture
5. **Evidence-based treatment**: the core symptom work for the primary diagnosis
6. **Maintenance and relapse prevention**: skills consolidation, early warning signs, supports

Match interventions to the client's **stage of change** (Prochaska & DiClemente, 1983). A precontemplative client needs engagement and motivational interviewing (Miller & Rollnick, 2013), not an action plan. Offering action-stage tasks to a precontemplative client is one of the most common distractors on the exam.

For co-occurring mental health and substance use disorders, integrated treatment of both at once is preferred over treating one and making the other wait.

Referral and Level of Care in the Plan

The plan specifies the setting (see the level-of-care table in Chapter 5) and any referrals:

- **Medication evaluation** when the evidence supports combined treatment or symptoms are severe
- **Medical evaluation** when physical causes have not been ruled out
- **Higher level of care** when risk or severity exceeds what outpatient counseling can safely manage

- **Specialty services** (eating disorder, substance use, trauma, or couples and family work) when the counselor lacks the needed competence (ACA, 2014, Standard C.2.a)
- **Community supports**: peer support, case management, vocational services, support groups

A referral is coordinated, not just handed off. The counselor obtains a release, communicates with the other provider, and follows up.

Reviewing and Revising the Plan

A treatment plan is a working document. Review it at planned intervals and whenever something changes, such as a new crisis, a new diagnosis, a change in level of care, or a lack of progress. Measurement-based care makes review concrete: the same standardized measure is given at regular intervals and reviewed with the client (Lewis et al., 2019).

When the client is **not improving**, check these explanations in order before changing the treatment:

1. **Safety**: has risk increased?
2. **Alliance**: is there a rupture in bond, goals, or tasks? (see Chapter 1)
3. **Diagnosis**: was something missed, such as bipolarity, substance use, trauma, a medical cause, or a personality pattern?
4. **Dose and fidelity**: has the evidence-based treatment been delivered fully and for long enough?
5. **Barriers**: is something outside the session blocking progress?
6. **Level of care or referral**: does the client need more intensive or specialized care, or a medication evaluation?

The rewarded answer investigates *why* progress stalled before switching approaches, and makes the change *with* the client.

Planning for Termination

Termination is planned from the start. The plan's goals define when treatment is complete. Counselors terminate when the client no longer needs help, is not benefiting, or is being harmed by continued counseling, and they provide pre-termination counseling and referrals as needed (ACA, 2014, Standard A.11.c). Raise termination early enough for the client to process it, and review progress, relapse warning signs, and a plan for returning if needed.

What the Exam Rewards (and Punishes) — Treatment Planning

Rewards: Client-owned, measurable objectives with timeframes. Safety and stabilization sequenced before core symptom work. Interventions matched to stage of change. Naming strengths and barriers. Integrated treatment of co-occurring disorders. Coordinated referrals with releases and follow-up. Revising the plan based on measured progress and the client's input. Planned termination.

Punishes: Vague goals ("feel better"). Goals written as counselor tasks. Action plans for precontemplative clients. Treating one disorder while the other "waits." Switching approaches before assessing why progress stalled. Keeping an unchanged plan despite no improvement. Unilateral decisions about goals or termination.

Worked Mini-Case — Treatment Planning

Vignette: Luis, 34, has moderate major depressive disorder (baseline PHQ-9: 17) and drinks 4–5 beers most nights. He is ambivalent about the drinking ("it's the only thing that helps me sleep") but wants to "stop feeling so heavy" so he can keep his job. He has a supportive sister and played soccer until two years ago. He works rotating shifts.

Reasoning chain:

Step 1 — Safety: Screen for suicide risk and withdrawal risk. Neither is present.

Step 2 — Client-owned goal: “Feel less heavy and keep my job.” The counselor adopts his words.

Step 3 — Objectives: The PHQ-9 will drop below 10 within 10 weeks. He will schedule and complete two valued activities per week, starting with a weekly soccer game with his sister. He will log his nightly drinks for four weeks.

Step 4 — Match readiness: Luis is contemplative about alcohol, so use MI to explore the drinking–sleep–mood link rather than demand abstinence. Treat the depression and the drinking together.

Step 5 — Barriers and strengths: Rotating shifts complicate scheduling, so offer flexible session times. His sister and his soccer history are resources the plan will use.

Step 6 — Review: Re-administer the PHQ-9 every two weeks. If there is no meaningful change by week 6, check the alliance, whether the drinking is maintaining the depression, and the need for a medication evaluation.

Check Your Understanding — Treatment Planning

Planning — objectives. Which of the following is the BEST-written short-term objective?

A. Client will gain insight into how her anxiety affects her relationships with coworkers each week.

B. Counselor will provide weekly CBT for social anxiety, including cognitive restructuring and graded exposure practice.

C. Client will initiate one conversation with a coworker on at least three workdays per week within four weeks.

D. Client will stop being anxious at work and feel comfortable talking with coworkers within four weeks.

Answer: C. C is specific, observable, measurable, and time-bound, and it belongs to the client. A and D are not

measurable. B describes the counselor's intervention, not the client's outcome.

Planning — revision. After eight sessions of well-delivered CBT, a client's GAD-7 has not changed. The counselor's MOST appropriate next step is:

A. Switch immediately to a different treatment model, such as ACT or interpersonal therapy, to give the client a fresh approach.

B. Terminate treatment now, because eight sessions without any GAD-7 change show that the client is not benefiting from counseling.

C. Continue the current plan unchanged for another eight sessions, trusting that CBT gains for anxiety often emerge gradually.

D. Review the lack of progress with the client and assess for missed factors such as alliance strain, substance use, or an unrecognized diagnosis.

Answer: D. When progress stalls, the rewarded move is to find out why before changing course, and to do it collaboratively (Lewis et al., 2019). A changes treatment without understanding the problem. C ignores the data. B is premature — termination for lack of benefit comes only after the plan has been reviewed and alternatives considered (ACA, 2014, Standard A.11.c).

Continuity of Care

2027 task map: 3B.1 Refer clients and/or family members to community resources or to specialists · 3B.2 Develop an aftercare plan · 3B.3 Discuss with clients how they will maintain progress · 3B.4 Develop a discharge plan · 3B.5 Develop a relapse prevention plan · 3B.6 Develop a plan for disruption of service · 3B.7 Engage with client in review of progress toward treatment goals at planned intervals · 3B.8 Evaluate treatment effectiveness

and client response · 3B.9 Collaborate with client to determine counseling termination criteria

Care That Outlasts the Session

A treatment plan describes what happens while the client is in your office. Continuity of care describes what happens at every seam: between you and another provider, between one level of care and the next, between this week and the week you are sick, and between the last session and the rest of the client's life.

The 2027 outlines list nine tasks here. They share one test: **does the client stay connected to help, and does the help keep working, when something changes?** Wrong answers leave a gap: a phone number with no appointment, a termination with no plan, a treatment never checked, a practice with no plan for the day the counselor cannot show up.

This section builds on the treatment-planning material on goals, referral, and termination types, and on the ethics chapter's coverage of abandonment.

Part 1 — Who Owns It

Owner	What it owns
ACA A.1.c Counseling Plans	Counselor and client devise plans jointly and **regularly review and revise** them for viability and effectiveness
ACA A.2.b Types of Information Needed	Informed consent covers goals, procedures, and **continuation of services upon the counselor's incapacitation or death**
ACA A.11.c Appropriate Termination	Terminate when the client **no longer needs help, is not likely to benefit, or is being harmed**; provide **pretermination counseling** and referrals

Owner	What it owns
ACA A.11.d Appropriate Transfer of Services	Complete clinical and administrative processes and keep **open communication** with client and new provider
ACA A.12 Abandonment and Client Neglect	Do not abandon or neglect clients; arrange continuation during **vacations, illness, and after termination**
ACA A.1.d Support Network Involvement	With client consent, enlist family, friends, and community supports
ACA B.1.c Respect for Confidentiality	Disclose client information only with **appropriate consent** or sound legal or ethical justification
ACA B.6.e Client Access	Clients may have reasonable access to their records on request
ACA B.6.g Disclosure or Transfer	**Written permission** before disclosing or transferring records to third parties
ACA B.6.h Storage and Disposal After Termination	Keep records after services end for reasonable future access, per law; dispose of them securely
ACA B.6.i Reasonable Precautions	Protect confidentiality if the counselor ends practice, is incapacitated, or dies; appoint a **records custodian** when appropriate
ACA C.2.d Monitor Effectiveness	Counselors **continually monitor their effectiveness** and take steps to improve
ACA C.2.g Impairment	Refrain from providing services when physical, mental, or emotional problems are likely to harm clients
ACA C.2.h Counselor Incapacitation, Death, Retirement, or Termination of Practice	A **plan to transfer clients and records** to an identified colleague or **records custodian** (the professional will)

Owner	What it owns
ACA H.2.a Informed Consent and Disclosure	For distance counseling: **possibility of technology failure and alternate methods of service delivery**, emergency procedures
HIPAA 45 CFR 164.308(a)(7)	Security Rule **contingency plan**: data backup, disaster recovery, emergency-mode operation
988, 211, 311	Three different phone lines: **crisis**, **community resources**, **city services**
Marlatt & Gordon (1985)	**Relapse prevention** model: high-risk situations, apparently irrelevant decisions, **abstinence violation effect**, lapse vs. relapse
Witkiewitz & Marlatt (2004)	Revised **dynamic** model of relapse
Leon Festinger (1957)	**Cognitive dissonance**: discomfort from conflicting beliefs or actions
Stokes & Baer (1977)	**Generalization**: gains do not transfer automatically; they must be programmed
Lewis et al. (2019)	**Measurement-based care**: routine measures, reviewed with the client, used to adjust treatment
Michael Lambert (2010)	**Routine outcome monitoring** with the OQ-45; feedback on **not-on-track** clients reduces deterioration
Miller & Duncan (Miller et al., 2003; Duncan et al., 2003)	**PCOMS**: the Outcome Rating Scale (ORS) and Session Rating Scale (SRS)
Jacobson & Truax (1991)	**Reliable change index** and **clinically significant change**

Part 2 — Referral to Resources and Specialists (3B. 1)

Active vs. Passive Referral

The treatment-planning chapter covers *when* to refer. This part covers *how*. The exam keys the referral that makes the connection actually happen.

	Passive referral	Active referral
What the counselor does	Hands over a name, list, or number	With consent, makes contact, books or helps book the appointment, sends records with a release, confirms attendance
Who carries the burden	The client	Shared
Typical outcome	Lower follow-through	Higher follow-through

A **warm handoff** is the strongest form: the counselor introduces the client to the new provider directly, in person or on a joint call. Active referral to mutual-help groups increases attendance compared with simply suggesting them (Timko et al., 2006).

The key distinction: an active referral **closes the loop**. The counselor knows whether the client arrived. When the referral is a **transfer**, **A.11.d** adds the duty to complete the clinical and administrative steps and keep **open communication** with client and new provider.

Verifying the Resource

A referral to the wrong place is a dead end. Before referring, confirm:

- The service exists and is **accepting new clients**
- **Eligibility**, cost, waitlist, hours, and location or telehealth access

- **Language** and disability access
- **Competence** for this specific problem, including the evidence-based treatment the client needs

988, 211, 311

Line	What it is	Use it for
988	The national **Suicide & Crisis Lifeline** (call, text, or chat)	Suicidal thoughts, mental health or substance crisis, worried family members
211	**Community information and referral** for health and human services	Food, housing, utilities, transportation, childcare, local programs
311	**Non-emergency city services**, where a city offers it	Potholes, noise, trash, city information. Not a clinical resource

Put 988 in a crisis plan. Put 211 in a resource plan. Never treat 311 as a mental health line. Exact services vary by area, so check what your local 211 and 311 actually provide.

Referring Family Members

Task 3B.1 names **family members**, not only clients. A spouse, parent, or adult child may need support of their own: a caregiver group, a family education program, a mutual-help group for families of people with substance use problems, or their own counselor.

- A family member who is not your client is referred **out**, not quietly added to your caseload.
- Your client's confidentiality still applies. Share clinical information with family only with the client's **consent** (B.1.c), unless a confidentiality exception applies (the ethics chapter).

Part 3 — Discharge Plans and Aftercare Plans (3B.4, 3B.2)

The exam treats these as two different documents. Know which is which.

	Discharge plan	Aftercare plan
Question it answers	How does the client **leave this service** safely?	How does the client **keep recovery going** after formal treatment?
Time frame	The **transition**: last days of this episode and first days after	The **months after**: ongoing, often open-ended
Starts	**At admission**	Drafted before discharge; owned by the client after
Core contents	Booked next appointment, signed release, records sent, medication bridge, crisis plan, who to call	Continuing services (step-down care, boosters), mutual-help or peer support, relapse prevention plan, daily routines, supports, return plan
Main risk it prevents	**Falling through the gap** between levels of care	**Losing gains** once support thins out

The key distinction: the **discharge plan** gets the client **to** the next provider. The **aftercare plan** says what the client **does** once the intensive help ends. A strong discharge plan includes the aftercare plan; it does not replace it.

Building a Discharge Plan

1. **Begin at admission**: where is this client going next, and what will they need there?
2. **Book the follow-up** before the client leaves: a date and time, not “call when you’re ready.”
3. **Secure the release** (B.6.g) so records reach the next provider.

4. **Bridge the gaps**: medication, transportation, authorization.
5. **Give a crisis plan** with 988 and named supports. Risk rises after leaving a higher level of care (the crisis chapter).
6. **Confirm the connection** after discharge.

An **aftercare plan** is written in the client's words. Beyond services and the relapse prevention plan (Part 5), it names the supports the client wants involved (A.1.d) and the daily structure that protects gains: sleep, work or school, routines.

Part 4 — Maintaining Progress (3B.3)

Gains made in counseling do not automatically last or spread to new settings. **Stokes & Baer** (1977) made this point about behavior change: **generalization must be planned**, not hoped for. That is the logic behind task 3B.3.

The Maintenance Conversation

Step	What it sounds like
Name what helped	"Which skills made the biggest difference?"
Attribute change to the client	"You did the exposures. The skill is yours." Clients who credit their own effort are better placed to keep using it
Plan practice	When, where, and how often the client will keep using each skill
Plan for new settings	Rehearse using skills at work, with family, under stress
Name warning signs	Early signs that things are slipping, and the first response
Taper and boost	Spread sessions out; schedule **booster sessions** after ending
Keep the door open	How to return if needed

Two wrong answers recur. **False reassurance** ("your gains will last") dismisses a real question. **Indefinite sessions "just in case"** foster dependency; when the client no longer needs counseling, **A.11.c** says to terminate. Tapering and booster sessions are the middle path.

Part 5 — Relapse Prevention (3B.5)

Marlatt & Gordon's Model (1985)

Relapse prevention is a cognitive-behavioral model developed by **G. Alan Marlatt and Judith Gordon** for addictive behaviors. It is now applied to many other problems. The model treats relapse as a **process** with identifiable steps, each a point where the client can intervene.

The sequence:

1. **Lifestyle imbalance**: too many "shoulds" and too few "wants" builds a desire for relief or indulgence.
2. **Apparently irrelevant decisions (AIDs)**, later renamed **seemingly irrelevant decisions (SIDs)**: small choices that look unrelated to use but move the client toward risk. Taking the route past the old bar. Keeping one bottle "for guests." Agreeing to watch the game at a friend's house where everyone drinks.
3. **High-risk situation**: a situation that threatens the client's sense of control. Marlatt's research found the most common are **negative emotional states**, **interpersonal conflict**, and **social pressure**.
4. **Coping response or not**:
   - **Coping** → increased **self-efficacy** → lower chance of relapse.
   - **No coping** → lower self-efficacy plus **positive outcome expectancies** ("a drink will calm me down") → **initial use**.
5. **The abstinence violation effect (AVE)** → full relapse, or a recovered lapse.

Lapse vs. Relapse

Lapse	Relapse
A single slip: one drink, one binge, one cigarette	A return to the old pattern
An event	A process
A **learning opportunity**	What the plan tries to prevent

The key distinction: a lapse does not have to become a relapse. **What the client thinks about the lapse decides which way it goes.**

The Abstinence Violation Effect

The **AVE** is the reaction that turns one slip into full relapse: "I've blown it, so I might as well keep going." It has two parts:

- **Dissonance**: the use conflicts with the client's identity as someone in recovery. The fastest way to end that conflict is to give up the identity ("I guess I'm not someone who can stay sober").
- **Personal attribution**: the client blames the slip on a stable, global, internal flaw ("I have no willpower") instead of a specific, changeable situation ("I was at a party without a plan").

The stronger the all-or-nothing belief, the stronger the AVE. Relapse prevention **reframes the lapse** as a mistake to learn from: what was the high-risk situation, what decision set it up, what coping was missing?

Festinger's Cognitive Dissonance (1957)

Leon Festinger, a social psychologist, described **cognitive dissonance**: the discomfort people feel when they hold two conflicting beliefs, or act against their own beliefs. People reduce it by changing the behavior, changing the belief, adding justifying thoughts, or minimizing the conflict.

Know the boundary. Marlatt drew on dissonance to explain *why* the AVE happens. But "cognitive dissonance" is Festinger's **general social psychology** term. When an item describes the "I've blown it, why stop now" reaction **inside a relapse prevention plan**, the answer is the **abstinence violation effect**, owned by **Marlatt & Gordon**.

Writing the Relapse Prevention Plan

Plan element	Example (Owen, six months sober)
High-risk situations	Arguments with his brother; Friday payday; work happy hours
Warning signs	Sleeping badly, isolating, romanticizing past drinking
Coping responses	Leave and call his sponsor; urge surfing; text a sober friend before Friday plans
If a lapse happens	Stop, call his sponsor within the hour, review what happened at the next session. One drink is information, not failure
Supports and crisis contacts	Sponsor, sister, counselor, 988

Witkiewitz & Marlatt (2004) later revised the model into a **dynamic** one: risk factors interact, and small changes can trigger sudden shifts. Mindfulness-based relapse prevention (Bowen et al., 2010) builds on Marlatt's **urge surfing**: watching a craving rise and fall without acting on it.

Part 6 — Planning for Disruption of Service (3B.6)

Counselors get sick, take leave, move, retire, and die. Offices flood. Video platforms go down. Task 3B.6 asks for a **plan made before** the disruption, not an improvised response after.

The Standards

- **A.12**: no abandonment. Arrange **continuation of treatment** during vacations and illness. **A.2.b**: clients learn at intake how services continue if the counselor is **incapacitated or dies**.
- **C.2.h**: transfer **clients and records** to an **identified colleague or records custodian**. This is the **professional will**. **B.6.i**: protect confidentiality when practice ends.
- **B.6.h**: keep records for **reasonable future access**, for the period law requires, then dispose of them securely. Retention periods vary. Check your state.

The Professional Will

A professional will is a written plan, separate from a personal will, that tells a named colleague exactly what to do. It names a **records custodian** (and a backup) and gives them what they need to act: where records, devices, and **passwords** are; the current **client list** and contact information; a **notification script**; a **referral list**; steps to cancel appointments and close billing; who else to notify (malpractice carrier, licensing board); and how records will be kept and later destroyed (B.6.h).

Client Access Is Not a Continuity Plan

Under **B.6.e**, clients may request reasonable access to their records, and counselors provide it unless there is compelling evidence that access would cause harm. That is a client right. It is **not** a plan for disruption. Handing every client a full record does not arrange coverage or transfer care. Continuity → **C.2.h**, not B.6.e.

Types of Disruption

Disruption	The plan
Planned leave or vacation	Advance notice; a named **covering counselor**; crisis instructions; high-risk clients seen or contacted before you leave
Illness, incapacity, or death	Step back when health would impair services (C.2.g); the professional will is activated; the custodian notifies clients, protects records, and arranges transfer (C.2.h, B.6.i)
Relocation (counselor or client) or **closing practice**	Notice in time to process the ending; transfers with releases (A.11.d); records kept per B.6.h. Continuing across state lines depends on licensure: verify current rules, including Counseling Compact status in both states
Disaster (fire, flood, storm, pandemic)	Off-site **backup** of records; a way to reach clients; emergency-mode operation. HIPAA's Security Rule requires a **contingency plan** (45 CFR 164.308(a)(7))
Platform or internet outage	Agreed at intake: the **backup method** (usually phone), who calls whom, and what to do in an emergency if contact is lost (**H.2.a**)

The key distinction: the exam rewards the plan that exists **before** the crisis and that **a named person** can carry out. "Clients will find someone else" is abandonment (A.12).

Part 7 — Reviewing Progress at Planned Intervals (3B.7)

Measurement-Based Care

Measurement-based care (Lewis et al., 2019) has three parts:

1. Give a **standardized measure** routinely, often every session.
2. **Review the result with the client** at the time it is collected.
3. **Use the data** to keep or change the plan.

A measure filed without being discussed is paperwork, not measurement-based care.

Routine outcome monitoring is the measuring part. **Michael Lambert's** work with the **OQ-45** (Lambert, 2010) showed that when counselors get feedback that a client is **not on track**, outcomes improve for those at-risk clients, and fewer of them deteriorate. **Miller & Duncan's PCOMS** (Miller et al., 2003) uses two brief scales: the **Outcome Rating Scale (ORS)** for how the client is doing, and the **Session Rating Scale (SRS)** for how the client experienced the session and the alliance.

Why Planned Intervals

- **A.1.c**: counselor and client **regularly review and revise** the plan to check that it is still viable and effective.
- **C.2.d**: counselors **continually monitor their effectiveness** and take steps to improve. Routine measures are one concrete way to do it.
- Counselors' unaided judgment misses most clients who are getting worse (Hannan et al., 2005). Scheduled review catches what impressions miss.

Set review points **in the plan itself**, plus any time something major changes. Review is done **with** the client:

show the scores, ask what the client notices and what has helped, ask about the relationship, then decide together.

Part 8 — Evaluating Effectiveness and Client Response (3B.8)

Review asks "what are the numbers?" Evaluation asks "**is this treatment working for this client, and what do we do about it?**"

Reliable and Clinically Significant Change

Jacobson & Truax (1991) gave two tests:

Concept	Meaning
Reliable change	The change in score is **larger than measurement error** would produce (the **reliable change index**, RCI). A small score shift may be noise
Clinically significant change	Reliable change **plus** moving from the clinical range into the **functional (nonclinical) range**
Reliable deterioration	A reliable change in the **worse** direction

Use each measure's published reliable-change value and cutoff, not an eyeball judgment.

Four Possible Answers

Pattern	Meaning	Response
Clinically significant improvement	The client is better and in the functional range	Consolidate; move toward the agreed termination criteria (Part 9)
Reliable improvement, still in clinical range	Working, not finished	Continue; check whether goals or dose need adjusting

Pattern	Meaning	Response
No reliable change after an adequate trial	Not working yet	Review with the client; check alliance, diagnosis, dose, barriers (the treatment-planning chapter); revise the plan
Reliable deterioration	Getting worse	**Act now**: check safety first, discuss with the client, consult, consider a **higher level of care** or referral

Evaluate **client response** broadly: symptoms, functioning, goals in the client's own terms, engagement, and the client's view of the work. A score that improves while the client says she feels worse needs a conversation.

Revision and Level of Care

- **Revise the plan** when evaluation shows it is not working (A.1.c). Change with the client, not to the client.
- **Step up** when deterioration or severity exceeds what this level can safely manage.
- **Step down** when gains are stable and less intensive care will hold them, with a discharge plan (Part 3).
- **Refer or terminate** when the client is **not likely to benefit** from continued counseling with you (A.11.c), with pretermination counseling and referrals.

The key distinction: an ineffective plan is **revised or escalated**, never continued unchanged and never dropped without a referral.

Part 9 — Collaborating on Termination Criteria (3B.9)

Termination criteria answer one question: **how will we both know we are done?** Task 3B.9 says the counselor

sets them **with** the client. The rewarded timing is **early**, as part of informed consent and initial planning, then revisited at each review.

Three standards support this. **A.2.b** puts goals and the nature of services in informed consent. **A.1.c** makes the plan a joint product. **A.11.c** defines when termination is appropriate; criteria agreed in advance make that standard visible to both people. Criteria set late, or by the counselor alone, make ending feel like rejection.

Writing Termination Criteria

Good criteria are **observable** and tied to the plan's goals:

Type	Example
Measure-based	GAD-7 in the mild range or below at three sessions in a row
Functional	Back at work full time for a month
Client-defined	"I can drive on the highway without pulling over."
Skill-based	Uses the relapse prevention plan on her own when warning signs appear
Maintenance	Gains hold through two biweekly and one monthly session

Also agree on the **other ways counseling can end**, because A.11.c names them: the client is **not benefiting** (with referral), continued counseling is **harmful**, or a **fee arrangement** is not kept.

When the Criteria Are Met

Review progress against the criteria, process the ending, finalize the maintenance, aftercare, and relapse prevention plans, taper, and confirm the open door. If the client wants to continue anyway, explore why. A new goal can justify continuing. Fear of ending alone does not; work on that fear as part of termination.

What the Exam Rewards / What It Punishes

Rewards: - Active referrals that close the loop: consent, release, booked appointment, confirmation - Discharge planning that starts at admission; a booked follow-up and crisis plan at every step-down - Aftercare and maintenance plans built with the client: skills, practice, boosters, open door - Relapse prevention that names high-risk situations and SIDs and reframes a lapse as learning - A professional will with a named records custodian, made before anything goes wrong - A backup contact method agreed at intake for telehealth - Routine measures reviewed with the client; plans revised when data say so - Termination criteria set early, together, in observable terms

Punishes: - Lists, pamphlets, and “call when you’re ready” - Discharge planning on the last day - False reassurance; indefinite sessions with no clinical reason - Treating a lapse as proof of failure - “Clients will find someone else”; no coverage during leave - Measures collected and never discussed - Continuing a plan unchanged when the client is not improving or is getting worse - Termination criteria announced by the counselor near the end

Chapter 7 — Putting It Together: Worked Case Walkthrough

The case presented during the intake session:

Marcus is a 38-year-old African American male, self-referred, who presents with "stress at work and feeling disconnected from everything." He reports that for the past six weeks he has had difficulty sleeping (waking at 3–4 a.m. and unable to return to sleep), low energy, difficulty concentrating at work, and a persistent feeling of "emptiness." He denies suicidal ideation but says, "Some days I wonder what the point is." He drinks 3–4 beers most evenings, which he says "takes the edge off." He has a primary care physician but has not had a checkup in two years. He is currently taking no medications. He is married with two children and describes his marriage as "strained."

Decision Point 1: Safety

The question: What do you assess first?

Marcus said "I wonder what the point is." This phrase requires direct safety assessment. The counselor cannot assume it is not suicidal ideation. The Priority Ladder places imminent safety at rung 1.

Correct action: Conduct a direct, empathic inquiry — "When you say you wonder what the point is, I want to make sure I understand. Are you having any thoughts of harming yourself or ending your life?"

Why the distractors lose:

- Moving directly to depression screening (PHQ-9; Kroenke et al., 2001): premature until safety is established
- Exploring the marital strain: premature — safety first

- Asking about the drinking: important but lower rung than safety

Decision Point 2: Medical and Substance Rule-Outs

The question: After safety is established (he denies active suicidal ideation (SI) with no plan or intent), what is next? Two rung-2 issues are present:

1. Medical rule-out: Sleep disturbance, fatigue, and cognitive difficulties can have thyroid, metabolic, or other medical causes. He hasn't had a checkup in two years.
2. Substance consideration: 3–4 beers nightly is above the National Institute on Alcohol Abuse and Alcoholism (NIAAA) low-risk threshold. Alcohol disrupts sleep architecture (explaining the early awakening), worsens depression, and must be quantified before diagnosing a mood disorder.

Correct action: Refer for a medical evaluation. Assess the alcohol use with a standardized tool (AUDIT or CAGE; Ewing, 1984; Saunders et al., 1993).

Why this matters: If the depression is alcohol-induced, treating depression with psychotherapy alone while ignoring the substance use will fail. The diagnosis cannot be confirmed until substance contribution is ruled out.

Decision Point 3: Differential

The leading question: Is this Major Depressive Disorder, Persistent Depressive Disorder, Alcohol-Induced Depressive Disorder, or a normal stress response?

Feature	MDD	PDD	Alcohol-Induced	Normal Stress
Duration	≥2 weeks (episode)	≥2 years (chronic low-grade)	Present only during/after use	Days to weeks; resolves with stressor

Feature	MDD	PDD	Alcohol-Induced	Normal Stress
Severity	Moderate–severe	Mild–moderate, persistent	Varies	Mild–moderate
Key feature	Episodic; often had a higher baseline	Never free of depression for >2 months	Temporally linked to alcohol	Stressor-responsive
Marcus's fit	✓ (6 weeks, moderate symptoms)	Possible if longer history	Possible — alcohol use is significant	Less likely given severity

The 6-week timeline fits an MDD episode. However, without knowing whether he was depressed before the drinking escalated, the alcohol question must be resolved first (rung 2 before rung 4).

Decision Point 4: Alliance

Marcus has mentioned "feeling disconnected." He has not previously sought mental health treatment. His cultural background and self-referral both matter: self-referral suggests motivation, but "feeling disconnected" may mean the therapeutic relationship itself feels unfamiliar or unsafe. Correct action before introducing any assessment tools: validate the courage of coming in, reflect the experience of carrying all of this privately, and explicitly invite his input on what kind of help would feel useful.

Decision Point 5: Treatment Planning

Once safety is cleared, medical rule-out is initiated, substance use is assessed, and the alliance is established:

- If MDD confirmed: CBT or behavioral activation is first-line evidence-based treatment (Martell et al., 2022). If alcohol use disorder is also present, treat both (integrated treatment is superior to sequential; Substance Abuse and Mental Health Services Administration, 2020).

- If PDD confirmed: CBT + medication consultation (dysthymia responds to combined treatment).
- Involve the marriage — the marital strain is both a stressor and a maintenance factor.

Scoring Summary

Decision	Priority Rung	Correct Action
Safety	1	Direct SI inquiry before anything else
Medical / substance	2	Refer for checkup; administer AUDIT
Alliance	4	Validate before assessing
Assessment	4	PHQ-9, AUDIT, further history
Treatment	5	CBT/behavioral activation + substance treatment if indicated

The case illustrates that clinical sophistication on the NCMHCE is not about knowing more diagnoses — it is about holding the Priority Ladder correctly under the pressure of a complex presentation.

End of Part One

PART TWO: DIAGNOSTIC REFERENCE

Chapters 8–24 (the diagnostic chapters) follow a standard rhythm: Overview → Diagnosis table → Clinical picture → Differentials → Assessment tools → What the exam rewards/punishes → Worked mini-case → Practice questions. Chapters 25–28 then cover ethics and practice management, couples and family counseling, the nondiagnostic areas of clinical focus, and indirect client care.

Chapter 8 — Depressive Disorders

Overview

Depressive disorders are the most commonly tested diagnostic family on the NCMHCE. They appear in nearly every case simulation — either as a primary diagnosis, as a comorbidity, or as part of a differential. The exam's highest-yield question about this family is: how severe, how long, and what was present before? Those three dimensions separate the diagnoses from each other and from normal grief or adjustment.

When an NCMHCE case presents a working diagnosis, your job is to confirm or revise it and to demonstrate what comes after it — the correct assessment sequence, the correctly prioritized interventions, the right evidence-based treatment, and the awareness of what could go wrong. Depression questions specifically test: (1) the distinction between MDD and PDD; (2) the role of grief — when grief becomes MDD; (3) medical and substance rule-outs (hypothyroidism, B12, sleep disorders, alcohol); (4) the priority of safety assessment (suicidality is always rung 1); (5) the evidence base: CBT and behavioral activation for MDD (Martell et al., 2022); combined CBT + medication for PDD.

Diagnosis Reference Table

Diagnostic criteria, specifiers, and durations in this table follow DSM-5-TR (American Psychiatric Association [APA], 2022); ICD-10-CM codes follow the current official coding guidelines (Centers for Medicare & Medicaid Services & National Center for Health Statistics [CMS & NCHS], 2024). ICD-11 codes are the World Health Organization's ICD-11 MMS codes (WHO, 2019), shown because the 2027 NCMHCE specifications name ICD-11 alongside

DSM-5-TR; U.S. insurance claims still use ICD-10-CM. A + joins a code and its qualifier.

Diagnosis	ICD-10	ICD-11	Key Feature	Duration	First-Line Treatment
Major Depressive Disorder (MDD)	F32.x / F33.x	6A70 / 6A71	≥5 symptoms including depressed mood or anhedonia; change from baseline; significant impairment	≥2 weeks	CBT, behavi activation, S SNRI; comb for moderat severe
Persistent Depressive Disorder (PDD / Dysthymia)	F34.1	6A72	Chronically depressed mood most of the day, more days than not; lower severity than MDD	≥2 years (adults); ≥1 year (children/ adolescents)	Combined psychothera medication; interperson therapy
Premenstrual Dysphoric Disorder (PMDD)	F32.81	GA34.41	Mood symptoms (lability, irritability, depression, anxiety) in final premenstrual week; remit after menses	Luteal phase pattern, confirmed across 2+ cycles	CBT, SSRIs be cyclic), lifestyle modificatio
Disruptive Mood Dysregulation Disorder (DMDD)	F34.81	6C90.0	Severe, recurrent temper outbursts + persistently irritable/angry mood in children 6–18	≥12 months in ≥2 settings	Multimodal therapy, psychoeduc possible medication consult
Substance/ Medication-Induced Depressive Disorder	F1x. 14/.24/.94	6C4x.70	Depressive symptoms developing during/after substance use or withdrawal	During or shortly after exposure	Treat under cause; abstinence support

Diagnosis	ICD-10	ICD-11	Key Feature	Duration	First-Line Treatment
Depressive Disorder Due to Another Medical Condition	F06.31–32	6E62.0	Depression etiologically linked to a medical condition (hypothyroidism, stroke, etc.)	Variable	Treat medic condition; adjunct antidepress needed

ICD-11 notes: Persistent Depressive Disorder: chronic major depression is coded 6A70 or 6A71, not 6A72. Premenstrual Dysphoric Disorder: coded in the genitourinary chapter (GA34.41), not the mental disorders chapter. Disruptive Mood Dysregulation Disorder: no ICD-11 category; the nearest code is 6C90.0, oppositional defiant disorder with chronic irritability-anger. Substance/Medication-Induced Depressive Disorder: x = the substance (for example, 6C40.70 alcohol; 6C4E.70 medication).

The Clinical Picture

MDD in the Case Simulation: The hallmarks are depressed mood and/or anhedonia (loss of interest or pleasure), with at least five symptoms in total drawn from those two and: significant weight/appetite change, insomnia or hypersomnia, psychomotor agitation or retardation, fatigue, feelings of worthlessness or excessive guilt, difficulty concentrating, and recurrent thoughts of death or suicidal ideation. The exam will present cue words — “she used to love hiking but hasn’t left the house in weeks” signals anhedonia. “There’s no point anymore” signals passive suicidal ideation that triggers safety assessment.

PDD (Persistent Depressive Disorder): The story is longer, more chronic, and often normalized. The client may say “I’ve always been kind of a low-energy person” or “this is just how I am.” They may never have experienced a symptom-free period they can remember. The depressive state feels like their personality, not an episode. This normalization is a key clinical marker — these clients

often present later and are harder to engage in treatment.

Grief vs. MDD: Normal grief includes waves of sadness, longing, and impaired function — but the bereaved person can typically be consoled, finds moments of positive emotion, and does not experience the pervasive anhedonia and worthlessness of MDD. The DSM-5 removed the bereavement exclusion, recognizing that grief can co-occur with or precipitate MDD (APA, 2022). The exam expects you to know: persistent grief that meets MDD criteria for two weeks or more is MDD and warrants treatment.

Double Depression: A client can have both PDD and MDD simultaneously — a PDD baseline with a current MDD episode layered on top. This appears on the exam when a chronically dysthymic client suddenly deteriorates significantly.

MDD vs. Bipolar Depression: This distinction is critical — wrong diagnosis leads to wrong treatment (antidepressants alone can destabilize bipolar disorder). Look for prior hypomanic or manic episodes (elevated or expansive mood, decreased need for sleep with increased energy, grandiosity, impulsivity). A thorough history is essential. The exam often tests this by presenting a depressed client with a history — buried in the vignette — of a period of feeling unusually energized, needing little sleep, making impulsive decisions. Missing that history means missing the bipolar diagnosis.

Male Depression: May present as irritability, agitation, and anger rather than sadness. Men are more likely to self-medicate with alcohol and more likely to complete suicide. The exam rewards recognizing atypical presentations.

Differentiating Within the Family

MDD vs. PDD: The deciding feature is duration. MDD is episodic — there was a baseline the client fell from. PDD

is chronic — the client has been depressed, at a lower severity, for at least two years without a two-month remission.

MDD vs. Adjustment Disorder with Depressed Mood: Adjustment disorder occurs within three months of an identifiable stressor, involves distress out of proportion to the stressor or significant impairment, and resolves within six months of the stressor ending. If the symptoms meet MDD criteria regardless of the stressor's presence, the diagnosis is MDD.

MDD vs. DMDD: DMDD applies only in children and adolescents and centers on chronic irritability and temper outbursts as the core feature — not the episodic sadness/anhedonia that defines MDD.

MDD vs. Substance/Medication-Induced Depressive Disorder: The deciding feature is the timeline. The induced diagnosis applies when depressive symptoms develop during or soon after intoxication, withdrawal, or exposure to a medication capable of producing them (alcohol, sedatives, stimulant crash, corticosteroids, interferon, some antihypertensives and hormonal agents). Evidence for an independent MDD instead: symptoms began before the substance use, persist for a substantial period (about one month) after acute withdrawal or intoxication ends, or the client has a history of non-substance-related episodes (APA, 2022). On the exam, the right move is to map onset against use, coordinate a medication review with the prescriber, and re-evaluate mood after a period of abstinence — while still completing a suicide risk assessment, because induced depression carries real risk.

MDD vs. Depressive Disorder Due to Another Medical Condition: Here the depression is the direct physiological consequence of a medical condition, established by history, physical exam, or laboratory findings — hypothyroidism, Cushing's disease, stroke, Parkinson's disease, multiple sclerosis, and some cancers are classic examples. Specifiers: with depressive features (F06.31) or

with major depressive-like episode (F06.32). Cue words: new-onset depression with fatigue, weight change, or cognitive slowing in a client with no psychiatric history, or depression that tracks the course of the illness. The counselor does not make the medical determination; the counselor refers for medical evaluation, coordinates with the treating physician, and treats the psychological impact alongside medical care. Contrast with adjustment disorder, where the illness is the *stressor* the client is reacting to rather than the physiological cause.

Assessment Tools in Context

Cutoffs and sources for every instrument in Part Two are collected in the Assessment Tools Compendium in Part Three.

PHQ-9 (Patient Health Questionnaire-9): Nine items, each scored 0–3 (Kroenke et al., 2001). Total 0–27. Cutoffs: 0–4 minimal; 5–9 mild; 10–14 moderate; 15–19 moderately severe; 20–27 severe. Note: Item 9 asks directly about suicidal ideation — a positive response requires safety assessment regardless of total score.

Beck Depression Inventory-II (BDI-II): 21 items (Beck et al., 1996). Widely used in clinical and research settings. Distinguishes cognitive (worthlessness, guilt) from somatic (sleep, appetite) symptoms.

Hamilton Depression Rating Scale (HAM-D): Clinician-administered (not self-report; Hamilton, 1960). Used to rate severity in research and inpatient contexts.

Columbia Suicide Severity Rating Scale (C-SSRS): Not a depression measure — a suicidality measure (Posner et al., 2011). Always use when suicide risk is being assessed (see Chapter 19 for full treatment).

What the Exam Rewards (and Punishes)

Rewards: Assessing suicidality before anything else when hopelessness, worthlessness, or passive death ideation is present — rung 1 always. Ruling out medical causes and

substances before confirming the diagnosis (thyroid, B12, sleep disorders, alcohol, steroids, beta-blockers). Recognizing the grief–MDD distinction and treating grief-triggered MDD as MDD. Identifying PDD's chronicity and recommending combined treatment (therapy + medication). Recognizing male depression may present as irritability, not sadness.

Punishes: Moving to treatment without safety assessment. Diagnosing MDD without exploring prior manic/hypomanic episodes. Dismissing chronic dysthymia as "just personality." Prescribing antidepressants alone for PDD without recommending psychotherapy. Missing the somatic presentation of depression in medical comorbidity cases.

Worked Mini-Case

Vignette: Carla, 44, is referred by her primary care physician following a routine visit. She reports she has "never really been happy" — as long as she can remember. PHQ-9: 12 (moderate). She cries easily, sleeps 10–11 hours but still feels tired, and says "I've been this way my whole life — I thought everyone felt like this." She denies suicidal ideation. Her PCP has ruled out thyroid dysfunction and B12 deficiency.

Reasoning chain:

Step 1 — Safety: No suicidal ideation. Proceed.

Step 2 — Differential: Carla's presentation — "never really been happy," chronic low mood, lifelong course, hypersomnia, fatigue — is a PDD presentation. The PHQ-9 score of 12 reflects moderate severity. Key marker: she normalized her symptoms ("I thought everyone felt like this"), consistent with PDD's ego-syntonic quality.

Step 3 — Medical rule-out: Already done by PCP (thyroid, B12 clear). Document.

Step 4 — Treatment: Combined psychotherapy and medication is the evidence-based standard for PDD. Psychother-

apy alone is less effective for PDD than for MDD. Options include CBT, behavioral activation, and interpersonal therapy. Coordinate with PCP for medication evaluation (SSRI or SNRI).

Step 5 — Engagement challenge: Carla may not initially believe treatment will work — she has "always been this way." Psychoeducation about PDD's treatability is a therapeutic priority in the first few sessions.

Answer on exam questions: The exam rewards knowing that PDD requires combined treatment (not therapy alone), that medical rule-out must precede diagnosis, and that chronicity (not severity) is the defining feature.

Practice Questions

Question 47. A client with confirmed PDD presents after six months of individual CBT with minimal improvement. The counselor's MOST appropriate next step is:

A. Continue CBT for another six months to allow adequate time for the chronic nature of PDD to respond to the current treatment

B. Switch to dialectical behavior therapy, which targets the emotion dysregulation commonly associated with persistent depression

C. Add a structured behavioral activation protocol to CBT since pure cognitive work is insufficient for chronic depression

D. Recommend a psychiatric consultation for medication evaluation, as combined treatment produces better outcomes in PDD

Question 48. A client presents with three weeks of depressed mood, anhedonia, insomnia, fatigue, and difficulty concentrating following the death of her mother one month ago. The counselor's MOST accurate diagnostic impression is:

A. Adjustment disorder with depressed mood, because the depressive symptoms began within one month of an identifiable loss event

B. Normal bereavement, because three weeks of depressive symptoms after losing a parent is an expected and self-limiting grief response

C. Major depressive disorder if the symptoms meet full MDD criteria and cause significant impairment, regardless of the grief context

D. Persistent depressive disorder, because grief reactions that include sleep and concentration problems suggest a chronic course

Answer Key — Chapter 8 Practice Questions

Q47 — Answer: D

Rationale: Combined psychotherapy and medication is the standard of care for PDD, and six months of adequate CBT without meaningful improvement is the clinical signal to add a medication evaluation. A extends a treatment that has already had an adequate trial, on the mistaken premise that chronicity justifies waiting longer. B switches modality without evidence that emotion dysregulation is the problem — DBT is not first-line for PDD (Linehan, 2015). C adds a component of CBT rather than the missing half of combined treatment, and the option's rationale (that chronic depression needs behavioral activation) does not address why six months of CBT failed (Martell et al., 2022).

Q48 — Answer: C

Rationale: DSM-5 removed the bereavement exclusion: when symptoms meet full MDD criteria — here, five symptoms for three weeks with impairment — the diagnosis is MDD regardless of the precipitating loss (APA, 2022). A defaults to Adjustment Disorder because a stressor exists, but Adjustment Disorder is diagnosed only when criteria for another disorder are not met. B is the pre-DSM-5 reasoning and risks leaving a treatable depressive episode unaddressed. D is wrong on time course — PDD requires two years of symptoms, not three weeks.

Chapter 9 — Anxiety Disorders

Overview

Anxiety disorders are the most prevalent mental health conditions in the United States and among the most frequently appearing on the NCMHCE. The exam tests two primary skills with this family: (1) distinguishing the specific anxiety diagnoses from each other based on the trigger and focus of the fear, and (2) knowing the evidence-based treatment hierarchy (which always includes exposure-based work).

The central organizing principle: **anxiety disorders differ by what the person fears and what they avoid.** The feared stimulus, the physiological response, and the behavioral response (avoidance) together define the diagnosis.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Diagnosis	ICD-10	ICD-11	Core Fear / Trigger	First-Line Treatment
Generalized Anxiety Disorder (GAD)	F41.1	6B00	Pervasive, uncontrollable worry about multiple topics	CBT, relaxation training, medication (SSRI/SNRI)
Panic Disorder	F41.0	6B01	Fear of panic attacks themselves (interoceptive conditioning)	CBT with interoceptive exposure, medication

Diagnosis	ICD-10	ICD-11	Core Fear / Trigger	First-Line Treatment
Social Anxiety Disorder	F40.10	6B04	Fear of negative evaluation in social/ performance situations	CBT, exposure, medication (SSRI)
Specific Phobia	F40.2xx	6B03	Fear of specific object/ situation (heights, blood, animals, etc.)	Systematic desensitization, exposure
Agoraphobia	F40.00	6B02	Fear of situations from which escape might be difficult or embarrassing	Exposure therapy, medication
Separation Anxiety Disorder	F93.0	6B05	Fear of separation from attachment figures	CBT, family involvement, graduated exposure
Selective Mutism	F94.0	6B06	Consistent failure to speak in specific social situations despite capability	Behavioral treatment, exposure, school collaboration

The Clinical Picture

GAD presents as a worry that does not settle — the client cannot stop, cannot turn the volume down, and has been this way for months or years. The worry shifts topics: health, finances, relationships, the future. The client often describes the worry as “out of control.” Physical symptoms include muscle tension, fatigue, irritability, sleep difficulty, and difficulty concentrating. DSM-5-TR requires six months of excessive worry about multiple areas (APA, 2022). GAD clients are often high-functioning — they ap-

pear at intake because the worry has finally crossed a threshold causing relationship problems, physical health effects, or occupational impairment.

Panic Disorder is distinguished by the presence of unexpected panic attacks plus persistent concern about having more attacks or significant behavioral change to avoid them. The panic attack itself — a sudden surge of intense fear peaking within minutes — is not sufficient for the diagnosis. The disorder requires the secondary fear of the attack, or avoidance. **Key exam detail:** Panic attacks can occur in multiple disorders — they are not unique to panic disorder. A client with Social Anxiety Disorder may have situationally triggered panic attacks. The presence of a panic attack does not equal Panic Disorder.

Social Anxiety Disorder is the fear of scrutiny, humiliation, or negative evaluation in social or performance situations. The exam sometimes presents a client who seems depressed (withdrawn, isolated, not engaging with others) — the key is whether the withdrawal is driven by low mood (depression) or by fear of judgment (social anxiety).

Specific Phobia is the most focused of the anxiety disorders — one discrete feared stimulus. The blood-injection-injury subtype is notable because it produces a vasovagal response (drop in heart rate and blood pressure, sometimes leading to fainting) rather than the sympathetic arousal typical of other phobias. Treatment requires adapted exposure (applied tension technique; Öst & Sterner, 1987).

Differentiating Within the Family

GAD vs. Depression: Both involve fatigue, sleep problems, concentration difficulties, and dysphoria. The deciding feature: in GAD, worry is the central symptom; in depression, anhedonia and dysphoric mood are central. GAD clients worry about bad things that might happen in the

future; depressed clients ruminate on loss and worthlessness about the past and present.

GAD vs. Panic Disorder: GAD anxiety is chronic and diffuse (many topics, constant low-level worry). Panic Disorder anxiety is episodic and intense (discrete attacks). A client can have both.

Social Anxiety Disorder vs. Schizoid Personality Disorder: Schizoid PD involves genuine indifference to social relationships — the person does not want them and is not distressed by their absence. Social Anxiety Disorder involves a desire for social connection combined with fear of it. The client with Social Anxiety Disorder wants to attend the party but cannot; the person with Schizoid PD doesn't want to go.

Social Anxiety Disorder vs. Avoidant Personality Disorder: These exist on a continuum. AvPD is essentially more severe, pervasive social anxiety with greater functional impairment and a more ingrained self-view of being inadequate. Treatment is the same (CBT, exposure) but longer.

Assessment Tools in Context

GAD-7 (Generalized Anxiety Disorder 7-item scale): Seven items, scored 0–3 (Spitzer et al., 2006). Scores of 5, 10, and 15 are thresholds for mild, moderate, and severe anxiety. Commonly used in primary care.

Beck Anxiety Inventory (BAI): 21 items, self-report (Beck et al., 1988). Focuses on somatic and cognitive anxiety symptoms. Often used alongside the BDI-II.

Panic Disorder Severity Scale (PDSS): Seven items rating panic frequency, distress, anticipatory anxiety, avoidance, and impairment (Shear et al., 1997). Clinician-administered.

SUDS (Subjective Units of Distress Scale): A 0–100 rating of current distress/anxiety (Wolpe, 1969). Used

within exposure therapy to track anxiety throughout a session, not a standalone diagnostic measure.

What the Exam Rewards (and Punishes)

Rewards: Using exposure as the core treatment for all anxiety disorders. Recognizing that avoidance maintains anxiety — treatment must address avoidance, not just symptoms. Distinguishing GAD's chronicity from Panic Disorder's episodic intensity. Identifying the secondary fear (fear of the attack) as what defines Panic Disorder. Knowing that blood-injection-injury phobia requires adapted exposure (applied tension technique).

Punishes: Treating anxiety with relaxation alone without addressing avoidance. Diagnosing Panic Disorder based on panic attacks alone without secondary apprehension or behavioral change. Missing comorbid depression in anxiety clients. Recommending prolonged benzodiazepine use as first-line (this maintains avoidance and produces dependence). Confusing Social Anxiety Disorder's motivated-but-fearful presentation with Schizoid PD's indifferent withdrawal.

Worked Mini-Case

Vignette: Jordan, 28, presents describing episodes of heart racing, shortness of breath, dizziness, and a sense of unreality that come "out of nowhere" and last about ten minutes. He has had four such episodes in the past two months. Since the first episode, he has been monitoring his heart rate constantly, avoids exercise (because it raises his heart rate), and has stopped taking public transit because "I had one there once and I couldn't get off." He has visited the emergency department twice and been medically cleared each time.

Reasoning chain:

Step 1 — Safety/Medical: Already medically cleared twice. Rules out cardiac causes.

Step 2 — Differential: Jordan has unexpected panic attacks (confirmed), persistent concern about having more attacks (heart rate monitoring), and significant behavioral changes to avoid them (no exercise, no transit). This is **Panic Disorder**.

Step 3 — Avoidance pattern: His avoidance has expanded (transit joins exercise). Without treatment, the avoidance spreads — the hallmark of Panic Disorder's progressive course.

Step 4 — Treatment: First-line is CBT with interoceptive exposure — deliberately inducing panic-like sensations in a controlled setting to break the interoceptive fear conditioning. Psychoeducation about the physiology of panic is essential. Avoidance of feared situations must also be addressed with situational exposure.

Answer on treatment: The exam will offer relaxation training, CBT with exposure, medication referral, and supportive counseling. The correct answer is CBT with exposure. Relaxation alone is insufficient because it does not address avoidance.

Practice Questions

Question 49. A client with GAD has learned relaxation techniques over three months. She reports they help "in the moment" but the worry returns. The counselor should:

A. Teach additional relaxation strategies, such as guided imagery and diaphragmatic breathing, and increase the daily practice frequency

B. Refer for medication evaluation since three months without sustained improvement suggests the need for adjunct medication

C. Add a cognitive component targeting the function of worry, intolerance of uncertainty, and worry-as-avoidance patterns

D. Begin systematic exposure targeting the specific situations and feared outcomes she worries about most, starting with the least distressing

Question 50. Many people have specific, disproportionate fears they avoid. Which feature is the diagnostic threshold that converts such a fear into Specific Phobia?

A. The fear is persistent, causes significant distress, and produces functional impairment

B. The person actively avoids the feared stimulus or endures exposure to it with marked anxiety or distress

C. The feared stimulus is specific, identifiable, and consistently triggers an immediate anxiety response

D. The fear is clearly disproportionate to the actual danger posed by the feared object or situation

Question 51. A client reports severe fear of blood draws and has avoided medical care for two years. She describes initial panic followed by lightheadedness and has fainted twice during exposure. The counselor should:

A. Use applied tension technique, because the vasovagal fainting response requires a specific evidence-based adaptation

B. Apply standard systematic desensitization with a gradual fear hierarchy, pairing each step with progressive muscle relaxation

C. Refer to a physician first for a full cardiac workup, since fainting during blood draws suggests an underlying medical condition

D. Treat with interoceptive exposure, because the fainting and panic symptoms together suggest an underlying Panic Disorder

Question 52. A client endorses daily, pervasive worry about finances, health, relationships, and work for three years. She says “I’ve always been a worrier” and reports insomnia, muscle tension, and feeling “keyed up.” The MOST likely diagnosis is:

A. Panic Disorder, because the hyperarousal symptoms and sleep disruption suggest episodic autonomic activation

B. Major Depressive Disorder, since chronic worry and insomnia could reflect a primary mood disturbance

C. Obsessive-Compulsive Disorder, because the persistent and uncontrollable worry resembles obsessions

D. Generalized Anxiety Disorder, based on chronic multidomain worry with hyperarousal and sleep disturbance

Answer Key — Chapter 9 Practice Questions

Q49 — Answer: C

Rationale: Relaxation addresses arousal but not the cognitive and avoidance patterns that maintain GAD. CBT must target worry function and intolerance of uncertainty.

Q50 — Answer: A

Rationale: Persistence (typically six months or more) with clinically significant distress or functional impairment is the threshold criterion. Avoidance (B), specificity (C), and disproportion (D) are all present in common non-clinical fears — they describe the fear, but do not by themselves make it a disorder.

Q51 — Answer: A

Rationale: Blood-injection-injury phobia produces a vasovagal response, not sympathetic arousal. Applied tension (tensing large muscles to maintain blood pressure) is the evidence-based adaptation (Öst & Sterner, 1987).

Q52 — Answer: D

Rationale: GAD requires ≥6 months of multi-domain excessive worry with ≥3 of six associated symptoms (here, sleep disturbance, muscle tension, and feeling keyed up). The chronicity, breadth, and associated symptoms confirm GAD.

Chapter 10 — Trauma and Stressor-Related Disorders

Overview

The trauma chapter is among the highest-yield diagnostic chapters on the NCMHCE — not because PTSD is complicated to identify, but because the exam tests the nuances: subthreshold presentations, the difference between Acute Stress Disorder and PTSD, the distinction between PTSD and Adjustment Disorder, complex trauma presentations, and the critical treatment hierarchy.

The non-negotiable clinical principle: trauma clients require safety and stabilization before any processing work begins. Moving to trauma memory processing before the client has adequate distress tolerance skills, a stable therapeutic alliance, and a safe living environment is a clinical error that causes harm. The exam will present scenarios where the wrong answer is "begin trauma processing immediately" — always stabilize first.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Diagnosis	ICD-10	ICD-11	Key Feature	Duration	First-Line Treatment
PTSD	F43.10	6B40	Re-experiencing, avoidance, negative cognitions/mood, hyperarousal following Criterion A trauma	>1 month	TF-CBT, EMDR, CPT, Prolonged Exposure (PE)

Diagnosis	ICD-10	ICD-11	Key Feature	Duration	First-Line Treatment
Acute Stress Disorder (ASD)	F43.0	QE84	Same symptom domains as PTSD	3 days–1 month post-trauma	Stabilization, CBT, crisis support
Adjustment Disorder	F43.2x	6B43	Emotional/ behavioral symptoms in response to stressor; does not meet criteria for another disorder	Within 3 months of stressor; resolves within 6 months	Supportive therapy, brief CBT
Reactive Attachment Disorder (RAD)	F94.1	6B44	Disturbed social relatedness in children following inadequate caregiving	Early childhood	Caregiver-focused intervention, dyadic therapy
Disinhibited Social Engagement Disorder	F94.2	6B45	Indiscriminate attachment in children; approaches strangers with little hesitation	Early childhood	Caregiver stability, attachment-focused work

ICD-11 notes: Acute Stress Disorder: no disorder equivalent; QE84 acute stress reaction is a non-disorder code.

The Clinical Picture

PTSD presents across four symptom clusters. Understanding these helps you identify PTSD in a case simulation where the diagnosis isn't named:

1. Re-experiencing (Intrusion): Flashbacks, nightmares, intrusive memories, psychological distress when reminded of the trauma. The client "goes back" — not just remembers, but re-lives.

2. Avoidance: Avoids trauma-related thoughts, feelings, people, places, situations, conversations. The avoidance maintains the disorder by preventing natural processing.

3. Negative Alterations in Cognition and Mood: Distorted blame, persistent negative emotions, diminished interest, feeling detached from others, inability to experience positive emotions ("emotional numbing"), persistent negative beliefs ("the world is dangerous," "I am permanently damaged").

4. Hyperarousal and Reactivity: Hypervigilance, exaggerated startle, sleep disturbance, irritability, reckless or aggressive behavior, concentration problems.

Non-stereotypical presentations: A combat veteran is the clichéd presentation — but PTSD also follows childhood abuse, sexual assault, medical trauma, accidents, witnessing violence, and workplace incidents. The Criterion A trauma does not have to be experienced directly — it can be witnessed or learned about if it affects a close person.

Complex PTSD (ICD-11 recognized, not a standalone DSM-5-TR diagnosis): results from prolonged, repeated trauma (e.g., childhood abuse, domestic violence, captivity; APA, 2022; World Health Organization [WHO], 2019). Presents with affect dysregulation, dissociation, disrupted self-perception, disturbed relationships, and loss of meaning in addition to core PTSD symptoms. Treatment must include a sustained stabilization phase before any exposure-based work.

Acute Stress Disorder (ASD) is PTSD's time-limited precursor — the same symptoms, but within the first month. ASD predicts later PTSD. Early intervention (trauma-focused CBT within the acute phase) can reduce PTSD incidence.

Differentiating Within the Family

PTSD vs. Adjustment Disorder: Both follow a stressor. The key distinction: PTSD requires a Criterion A traumat-

ic stressor (actual or threatened death, serious injury, sexual violence) and specific symptom clusters (re-experiencing, avoidance, negative cognitions, hyperarousal). Adjustment Disorder occurs after a stressor of any type (job loss, divorce, moving) and doesn't require those specific symptom clusters.

PTSD vs. MDD: Depression and PTSD commonly co-occur. PTSD has re-experiencing (flashbacks, nightmares), avoidance of trauma-related cues, and hyperarousal — none of which are cardinal MDD features. When both are present, both are diagnosed.

PTSD vs. Dissociative Disorders: Dissociation can occur in PTSD as the dissociative subtype (with depersonalization and derealization). True dissociative disorders (DID, Depersonalization/Derealization Disorder) are distinct entities (see Chapter 22).

Assessment Tools in Context

PCL-5 (PTSD Checklist for DSM-5): 20-item self-report aligned with DSM-5 criteria (Weathers et al., 2013). Used for screening and monitoring treatment response. Scores ≥31–33 suggest probable PTSD. Each item maps to a DSM-5 symptom — clinically useful for tracking specific clusters.

CAPS-5 (Clinician-Administered PTSD Scale): The gold standard — a structured clinician interview assessing PTSD diagnosis and severity (Weathers et al., 2018). Used in research and for formal diagnosis. Know that it is clinician-administered.

Trauma Symptom Inventory (TSI): Broader trauma-related assessment including depression, anxiety, anger, dissociation (Briere, 2011). Used when complex trauma is suspected.

ACE Score (Adverse Childhood Experiences): Not a diagnostic tool — a cumulative risk indicator based on 10 categories of childhood adversity (Felitti et al., 1998). Higher scores correlate with more severe adult health

and mental health outcomes. Used in psychoeducation and treatment planning.

What the Exam Rewards (and Punishes)

Rewards: The three-phase treatment model: (1) safety and stabilization, (2) trauma memory processing, (3) reintegration — never skip Phase 1. Evidence-based trauma treatments: TF-CBT (children), CPT (Cognitive Processing Therapy), PE (Prolonged Exposure), EMDR (Cohen et al., 2017; Foa et al., 2019; Resick et al., 2017; Shapiro, 2018). Direct but compassionate safety assessment when suicidality or self-harm is present. Recognizing that avoidance is central — treatment must approach what the client is avoiding. Cultural sensitivity around trauma disclosure.

Punishes: Beginning trauma memory processing (exposure, EMDR) before stabilization. Forcing trauma disclosure before trust is established. Missing that PTSD and MDD, or PTSD and SUD, commonly co-occur and both require treatment. Diagnosing Adjustment Disorder when the stressor meets Criterion A and symptoms meet PTSD criteria. Treating ASD with watchful waiting only when early CBT can reduce PTSD risk.

Worked Mini-Case

Vignette: Diane, 34, is referred following a car accident eight weeks ago in which she was not injured but witnessed another driver's death. She reports nightmares about the crash nightly, avoids driving (her husband takes her to work), becomes distressed when she sees news coverage of accidents, and has been "on edge" at all times since the accident. She endorses feeling detached from her family and reports she "can't feel anything." Her husband reports she has become irritable and startles easily at sounds.

Reasoning chain:

Step 1 — Criterion A: Diane witnessed another person's death. This meets Criterion A.

Step 2 — Symptom clusters: Re-experiencing (nightmares ✓), Avoidance (avoids driving, avoids accident news ✓), Negative alterations in cognition/mood (emotional numbing, detachment ✓), Hyperarousal (on edge, irritability, exaggerated startle ✓).

Step 3 — Duration: Eight weeks (>1 month) → PTSD, not ASD.

Step 4 — Safety: No SI mentioned, but irritability and numbing warrant asking about self-harm.

Step 5 — Treatment: Begin with Phase 1 (stabilization, psychoeducation about PTSD, grounding skills, safety). Then CPT or PE to address the trauma memory and avoidance. Involve the husband as a support resource.

Key exam point: The exam might ask whether driving avoidance should be "respected" vs. addressed. The correct answer: avoidance is a maintaining factor — treatment must eventually address it.

Practice Questions

Question 53. A client with PTSD and co-occurring alcohol use disorder (she drinks to suppress nightmares) is beginning outpatient treatment. The counselor should FIRST:

A. Begin prolonged exposure to the trauma memories right away while simultaneously addressing the alcohol use in parallel

B. Focus exclusively on the PTSD since the drinking is a trauma symptom that will resolve once nightmares stop

C. Address stabilization, safety, and reduce drinking before initiating any trauma-focused processing interventions

D. Refer to a residential substance use treatment program and defer all trauma work until sustained sobriety is firmly established

Question 54. Which feature MOST clearly differentiates PTSD from Adjustment Disorder?

A. PTSD involves persistent avoidance of trauma reminders, a behavior pattern that is not present in Adjustment Disorder

B. Adjustment Disorder is expected to resolve on its own within six months without treatment, while PTSD requires intervention

C. PTSD symptoms are more severe, more persistent, and more functionally impairing than those of Adjustment Disorder

D. PTSD requires a Criterion A traumatic stressor and specific symptom clusters including intrusive re-experiencing

Question 55. A client with childhood sexual abuse history dissociates during an EMDR processing session three sessions into treatment. The MOST appropriate response is:

A. Pause the EMDR processing and extend the stabilization phase, teaching distress tolerance and grounding first

B. Continue the EMDR protocol as planned and teach grounding techniques for the client to practice between sessions

C. Refer the client to a higher level of care since dissociation during processing indicates she is too unstable

D. Switch to a non-exposure treatment such as supportive therapy for the remainder of the therapeutic relationship

Question 56. A 7-year-old removed from a neglectful home at age 3 now approaches strangers at the park, holds their hands, and has attempted to leave with unfamiliar adults twice. The MOST likely diagnosis is:

A. PTSD with behavioral dysregulation resulting from early neglect, disrupted attachment, and repeated caregiver changes

B. Social Anxiety Disorder that should be ruled out since the approach behavior may mask underlying social fears

C. Disinhibited Social Engagement Disorder, characterized by indiscriminate attachment behavior with unfamiliar adults

D. Reactive Attachment Disorder, characterized by emotional withdrawal and inhibited attachment behavior toward adult caregivers

Answer Key — Chapter 10 Practice Questions

Q53 — Answer: C

Rationale: Integrated treatment addresses both simultaneously, but stabilization and harm reduction from alcohol must precede trauma processing. The alcohol is not separate from the PTSD — it's a maintaining behavior.

Q54 — Answer: D

Rationale: The Criterion A stressor (actual/threatened death, serious injury, sexual violence) and the specific symptom clusters (re-experiencing, avoidance, negative cognitions, hyperarousal) are the defining distinctions.

Q55 — Answer: A

Rationale: Dissociation during processing is a signal that Phase 1 (stabilization) was insufficient. Distress tolerance skills must be solidified before returning to processing.

Q56 — Answer: C

Rationale: DSED is characterized by culturally inappropriate, overly familiar behavior with unfamiliar adults. RAD presents with inhibited, emotionally withdrawn behavior — the opposite pattern.

Chapter 11 — Substance-Related and Addictive Disorders

Overview

Substance use disorders occupy a unique position on the NCMHCE because they appear in two ways: as primary diagnoses, and as co-occurring conditions that complicate every other disorder. The exam tests whether you can (1) recognize the difference between use, misuse, and disorder; (2) apply the diagnostic criteria correctly; (3) identify the appropriate level of care; and (4) avoid the two classic errors — over-pathologizing normal use and under-recognizing serious use disorders in high-functioning clients.

The single most testable substance question: What is the appropriate first clinical action with a new client who reports substance use? The answer is: **assess the severity and functional impact before doing anything else.** Do not assume use = disorder, and do not assume the client knows whether they have a problem.

The second key concept: co-occurring (dual) disorders require **integrated treatment.** Treating only the mental health issue while ignoring the substance use, or treating only the substance use while ignoring the mental health issue, produces consistently worse outcomes than treating both simultaneously.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Diagnosis	ICD-10	ICD-11	Severity Specifier	Key Notes
Alcohol Use Disorder	F10.10 (mild) / F10.20 (moderate–severe)	6C40.1 / 6C40.2	2–3 Sx: mild; 4–5: moderate; ≥6: severe	Most commonly tested SUD; withdrawal can be medically dangerous (seizures, delirium tremens)
Cannabis Use Disorder	F12.10	6C41.1 / 6C41.2	Same 11-criterion scale	Withdrawal includes irritability, sleep disruption, decreased appetite; often minimized by clients
Opioid Use Disorder	F11.10	6C43.1 / 6C43.2	Same scale	Tolerance, withdrawal, compulsive use despite consequences; highest overdose mortality
Stimulant Use Disorder	F15.10 (amphetamine-type); F14.10 (cocaine)	6C46.1 / 6C46.2 (amphetamine-type); 6C45.1 / 6C45.2 (cocaine)	Same scale	Includes cocaine, amphetamine, methamphetamine; crash phase after use is clinically significant
Sedative/ Hypnotic/ Anxiolytic Use Disorder	F13.10	6C44.1 / 6C44.2	Same scale	Includes benzodiazepines; withdrawal can be medically dangerous (similar to alcohol)
Gambling Disorder	F63.0	6C50	Mild/ Moderate/ Severe	Only behavioral (non-substance) addictive disorder in DSM-5-TR; 4+ of 9 criteria

ICD-11 notes: Alcohol Use Disorder, Cannabis Use Disorder, Opioid Use Disorder, Stimulant Use Disorder, Sedative/Hypnotic/Anxiolytic Use Disorder: ICD-11 has no

severity levels; mild corresponds roughly to harmful pattern of use (.1), moderate–severe to dependence (.2).

The 11 Diagnostic Criteria (applied to each substance): (1) Taking larger amounts or longer than intended; (2) Persistent desire or unsuccessful efforts to cut down; (3) Great deal of time spent obtaining, using, or recovering; (4) Craving; (5) Failure to fulfill major role obligations; (6) Continued use despite social/interpersonal problems; (7) Important activities given up; (8) Use in hazardous situations; (9) Continued use despite knowledge of physical/psychological problems; (10) Tolerance; (11) Withdrawal.

Exam note: Tolerance and withdrawal do not automatically indicate a SUD when they occur in the context of medically prescribed use.

The Clinical Picture

The clinical picture of a substance use disorder often conceals itself. High-functioning individuals with moderate or severe alcohol use disorder may appear polished and organized. Cannabis use disorder clients may resist the diagnosis ("it's legal now"). Opioid use disorder often presents first in medical settings.

In a case simulation, look for: - **The erosion pattern:** things the person used to do that they no longer do (work performance declining, relationships deteriorating, hobbies abandoned) - **The explaining-it-away pattern:** the client attributes every problem to something other than the substance ("my boss is terrible," "my marriage was already struggling") - **The all-or-nothing disclosure:** the client's first report minimizes; the full picture emerges only with direct, nonjudgmental questions

Motivational Interviewing (MI) is the evidence-based approach for the precontemplation and contemplation stages (Miller & Rollnick, 2013). MI does not argue with the client, does not lecture, and does not try to convince. It explores ambivalence through Open questions, Reflections, Affirmations, and Summaries — the OARS skills.

FRAMES Model (Brief Intervention): Feedback (personalized) → Responsibility (emphasize client's own choice) → Advice (clear recommendation) → Menu of options → Empathy → Self-efficacy support (Miller & Sanchez, 1994).

ASAM Levels of Care

Level	Name	Setting
0.5	Early Intervention	Outpatient education/monitoring
1	Outpatient Services	Less than 9 hours/week; standard outpatient
2.1	Intensive Outpatient (IOP)	9–20 hours/week; structured day/evening
2.5	Partial Hospitalization (PHP)	20+ hours/week; day treatment
3.1–3.7	Residential/Inpatient	24-hour care; clinically managed to medically monitored
4	Medically Managed Intensive Inpatient	Hospital-based; severe withdrawal or medical complications

The exam expects you to select the appropriate level of care based on: withdrawal risk, medical stability, psychiatric comorbidity, social support, prior treatment history, and motivation.

Differentiating Within the Family

Substance Use Disorder vs. Substance-Induced Disorder: Substance-induced disorders (depression, anxiety, psychosis induced by a substance) are temporally related to the substance — they emerge during use or withdrawal and typically remit with sustained sobriety. A co-occurring independent disorder (e.g., MDD that predates substance use or persists well into sobriety) is a separate diagnosis requiring its own treatment.

Alcohol Withdrawal vs. Alcohol Use Disorder: Withdrawal is a physiological state — it does not by itself constitute a SUD. Clinically significant withdrawal is one of the 11 criteria, but the diagnosis requires at least two criteria total within the last 12 months.

Gambling Disorder: Treat like other addictive disorders — the same MI-based approach, relapse prevention, and 12-step analogues (Gamblers Anonymous). CBT is the primary evidence-based treatment.

Assessment Tools in Context

AUDIT (Alcohol Use Disorders Identification Test): 10-item self-report (Saunders et al., 1993). Scores 8+ suggest hazardous drinking. AUDIT-C (3-item abbreviated version) is widely used in primary care. Know the cutoffs.

CAGE Questionnaire: Four items — Cut down, Annoyed, Guilty, Eye-opener (Ewing, 1984). Two or more positive responses suggest significant alcohol use disorder. Brief and memorable for clinical settings.

DAST-10 (Drug Abuse Screening Test): 10-item self-report for non-alcohol substance use (Skinner, 1982). Score ≥3 suggests need for further assessment.

CRAFFT (for adolescents): Six items screening for substance use risk in young people (Car, Relax, Alone, Forget, Friends, Trouble; Knight et al., 2002). Score ≥2 is a positive screen.

What the Exam Rewards (and Punishes)

Rewards: Assessing severity (mild/moderate/severe) before any intervention. Recognizing that substance use treatment records carry heightened federal confidentiality protection under 42 CFR Part 2, which is stricter than HIPAA and requires specific written consent for most disclosures (Confidentiality of Substance Use Disorder Patient Records, 2024). Using MI for ambivalent clients — reflecting both sides of the ambivalence, not arguing for change. Recognizing that co-occurring disorders require

integrated treatment. Selecting the appropriate ASAM level of care (American Society of Addiction Medicine [ASAM], 2013). Recognizing high-risk medical withdrawal (alcohol, benzodiazepines) that requires medical monitoring.

Punishes: Confrontational techniques that break therapeutic alliance. Assuming abstinence is always the goal without assessing the client's stage of change. Treating mental health in isolation when substance use is active. Missing that alcohol and benzodiazepine withdrawal can be medically dangerous (seizures, delirium tremens). Over-relying on 12-step programming as the only evidence-based intervention.

Worked Mini-Case

Vignette: Marcus, 42, is referred by his employer after missing three Monday morning meetings. He reports drinking "a few beers" most evenings "to unwind." On further inquiry, he acknowledges that "a few" has become 6–8 beers nightly, that he has tried to cut back twice without success, and that he feels anxious and shaky on mornings he doesn't drink. His wife has threatened to leave. He has reduced his involvement in his son's baseball team because "I'm usually not feeling well on weekends." He insists he does not have a drinking problem "because I still go to work."

Reasoning chain:

Step 1 — Assessment: Identify which of the 11 SUD criteria are present. Marcus has: (1) takes larger amounts than intended (6–8 vs. "a few"); (2) unsuccessful efforts to cut down (twice); (5) role failure (missed meetings); (6) continued use despite interpersonal problems (marriage); (7) activities given up (son's baseball); (11) withdrawal (shaky, anxious on mornings without alcohol). Six criteria = **Severe Alcohol Use Disorder**.

Step 2 — Medical risk: Morning shakiness and anxiety after not drinking = withdrawal symptoms. This is medic-

ally significant. Alcohol withdrawal can progress to seizures or delirium tremens. Medical evaluation before starting outpatient treatment is indicated.

Step 3 — Stage of change: Marcus is in precontemplation ("I don't have a problem"). MI, not confrontation, is the evidence-based approach.

Step 4 — Level of care: Given severe SUD + withdrawal symptoms + social stressors: at minimum, ASAM Level 2.1 (IOP) with medical monitoring for withdrawal. Level 3 residential may be indicated.

Answer on exam questions: The exam rewards identifying the medical withdrawal risk as a first priority. A counselor who begins MI without flagging the physical symptoms has missed a safety issue.

Practice Questions

Question 57. A 35-year-old client says he drinks every day but insists "I can stop anytime — I just choose not to." He has missed work three times in the past month due to hangovers and his wife has moved out. The counselor's MOST appropriate initial response is:

A. Confront the denial directly and tell him that his belief he can stop anytime is a classic symptom of alcohol addiction

B. Accept his self-assessment and focus on the relationship problems with his wife that are his stated presenting concern

C. Use a motivational interviewing approach, reflecting his ambivalence and exploring the role alcohol plays in his life

D. Refer immediately to an intensive outpatient program since the functional impairment indicates a severe use disorder

Question 58. A client with Alcohol Use Disorder reports that she has stopped drinking on her own, without medic-

al supervision. She now reports tremors, sweating, and "seeing things that aren't there." The counselor's MOST appropriate immediate action is:

A. Refer for immediate medical evaluation, as these symptoms indicate potentially life-threatening alcohol withdrawal

B. Begin withdrawal psychoeducation, review coping strategies for cravings, and schedule a follow-up in three days

C. Provide supportive counseling and encourage her to return to drinking at a reduced level to taper safely

D. Administer a standardized withdrawal assessment scale, such as the CIWA-Ar, to determine severity before taking any other action

Question 59. A client in the contemplation stage of change regarding cannabis use says, "Part of me knows I should cut back, but it's the only thing that helps me sleep." The counselor's BEST response is:

A. "I hear that it's helping you sleep, and I'm also wondering about the part of you that's concerned — what does that part say?"

B. "Let's make a plan for you to reduce your use by 50% over the next two weeks and track how you're sleeping."

C. "Cannabis is habit-forming and disrupts sleep quality, so we need to address the sleep problem separately with evidence-based interventions."

D. "It sounds like you're not quite ready to change yet, so let's table this topic for now and check in about it again next month."

Answer Key — Chapter 11 Practice Questions

Q57 — Answer: C

Rationale: MI is the evidence-based approach for precontemplative clients. Confrontation (A) breaks alliance and is contraindicated. Referral without engagement (D) ignores stage of change.

Q58 — Answer: A

Rationale: Visual hallucinations + tremors + sweating = delirium tremens risk. This is a medical emergency requiring immediate referral, not a counseling session.

Q59 — Answer: A

Rationale: This is a double-sided reflection followed by an open question — the core MI technique for exploring ambivalence in the contemplation stage.

Chapter 12 — Personality Disorders

Overview

Personality disorders are among the most challenging topics on the NCMHCE — not because the diagnosis is hard to recognize, but because the exam tests your understanding of how personality disorders change the clinical picture and what adaptations they require in the therapeutic relationship.

The foundational principle: a personality disorder is a persistent, pervasive, inflexible pattern of experiencing and relating to the world that causes distress or impairment. The key word is **pervasive** — the pattern appears across multiple contexts (work, relationships, the counseling relationship itself), is stable over time, and is **ego-syntonic** (the client experiences the pattern as "just who I am," not as symptoms to be treated). This is why personality disorder clients often do not present seeking change in their personality — they present because the world keeps responding badly to them.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Cluster A — "Odd/Eccentric"

Diagnosis	ICD-10	ICD-11	Core Pattern
Paranoid PD	F60.0	6D10 + 6D11.0	Pervasive distrust and suspicion; interprets benign actions as malicious

Diagnosis	ICD-10	ICD-11	Core Pattern
Schizoid PD	F60.1	6D10 + 6D11.1	Detachment from social relationships; restricted affect; indifferent to others
Schizotypal PD	F21	6A22	Social isolation + odd beliefs, perceptions, speech, and behavior; not psychotic but eccentric

ICD-11 notes: Paranoid PD, Schizoid PD: ICD-11 has no specific personality disorder types; code 6D10 personality disorder plus the closest trait-domain qualifier (shown after +). Schizotypal PD: ICD-11 classifies it with schizophrenia and other primary psychotic disorders.

Cluster B — "Dramatic/Emotional/Erratic"

Diagnosis	ICD-10	ICD-11	Core Pattern
Antisocial PD (ASPD)	F60.2	6D10 + 6D11.2	Pervasive disregard for and violation of others' rights (≥18; Conduct Disorder before age 15); deceitfulness, impulsivity, recklessness
Borderline PD (BPD)	F60.3	6D10 + 6D11.5	Instability in relationships, self-image, and affect + marked impulsivity; fear of abandonment; self-harm; identity disturbance

Diagnosis	ICD-10	ICD-11	Core Pattern
Histrionic PD	F60.4	6D10 + 6D11.2	Excessive emotionality and attention-seeking; discomfort when not the center of attention; dramatic, theatrical behavior
Narcissistic PD	F60.81	6D10 + 6D11.2	Grandiosity, need for admiration, lack of empathy; sense of entitlement; exploits others

ICD-11 notes: Antisocial PD, Histrionic PD, Narcissistic PD: ICD-11 has no specific personality disorder types; code 6D10 personality disorder plus the closest trait-domain qualifier (shown after +). Borderline PD: code 6D10 personality disorder plus the borderline pattern qualifier 6D11.5.

Cluster C — "Anxious/Fearful"

Diagnosis	ICD-10	ICD-11	Core Pattern
Avoidant PD	F60.6	6D10 + 6D11.0 + 6D11.1	Social inhibition, feelings of inadequacy, hypersensitivity to negative evaluation; desires social connection but avoids it

Diagnosis	ICD-10	ICD-11	Core Pattern
Dependent PD	F60.7	6D10 + 6D11.0	Excessive need to be taken care of; submissive, clinging behavior; difficulty with decisions without others' reassurance
Obsessive-Compulsive PD (OCPD)	F60.5	6D10 + 6D11.4	Preoccupation with orderliness, perfectionism, and mental/interpersonal control; rigidity; not obsessions/compulsions

ICD-11 notes: Avoidant PD, Dependent PD, Obsessive-Compulsive PD: ICD-11 has no specific personality disorder types; code 6D10 personality disorder plus the closest trait-domain qualifier (shown after +).

The Clinical Picture

BPD in Case Simulations: The exam tests BPD in two primary ways: (1) recognizing the presentation (intense, unstable relationships; identity disturbance; self-harm; fear of abandonment; rapid mood shifts), and (2) knowing DBT as the evidence-based treatment (Linehan, 2015). BPD clients often present following a crisis — self-harm, an impulsive behavior, or a relationship rupture. The therapeutic relationship itself will show the pattern: idealization followed by devaluation.

Narcissistic PD in the Counseling Relationship: The counselor may notice excessive entitlement at intake (expecting special treatment, dismissing the counselor's expertise), difficulty with feedback, and discomfort when the session focuses on the client's contribution to their

problems. The therapeutic challenge is maintaining genuineness without triggering devaluation.

ASPD and Manipulative Behavior: ASPD clients do not present because they want to change their antisocial behavior — they present because of consequences (court-ordered, employment, or relationship). The counselor maintains clear limits, documents carefully, does not accept explanations at face value, and avoids becoming a character witness.

OCPD vs. OCD: This distinction is frequently tested. OCPD is ego-syntonic — the person's rigidity, perfectionism, and need for control feel right to them. OCD is ego-dystonic — the person recognizes the obsessions as intrusive and unwanted and the compulsions as excessive. OCPD does not require obsessions or compulsions as defined in OCD.

Differentiating Within the Family

BPD vs. Bipolar II: The critical distinguishing feature is that Bipolar II has discrete episodes (days to weeks) that are relatively autonomous from triggers; BPD mood shifts are reactive to interpersonal events (hours to days), interpersonally triggered, and accompanied by identity disturbance and self-harm. The client with BPD does not have the distinct episode structure of bipolar disorder.

BPD vs. PTSD: BPD and PTSD frequently co-occur, particularly in survivors of childhood abuse. The exam expects you to know that trauma history is common in BPD — but trauma history does not equal BPD, and BPD does not equal PTSD. Assess both independently.

Avoidant PD vs. Social Anxiety Disorder: These exist on a continuum. AvPD is essentially severe, pervasive social anxiety with a more ingrained self-view of inadequacy. Treatment is the same (CBT, exposure, medication) but more intensive and longer.

Schizoid PD vs. Autism Spectrum Disorder (ASD): Both involve limited social engagement and restricted af-

fect. The distinction: ASD involves impairment in social communication and restricted/repetitive behaviors that are neurodevelopmental in origin. Schizoid PD involves a volitional preference for solitude without the communication deficits characteristic of ASD.

Assessment Tools in Context

MMPI-3 / MMPI-2-RF: Broad-spectrum psychological assessment that includes personality scales (Ben-Porath & Tellegen, 2020). Used by psychologists; results may appear in psychological reports. The counselor reads scales for PD features without administering it.

Millon Clinical Multiaxial Inventory-IV (MCMI-IV): Designed specifically for personality disorder assessment (Millon et al., 2015). 195 true/false items. Used in forensic, medical, and clinical settings. Know it exists; counselors refer for it rather than administer it.

Structured Clinical Interview for DSM-5 Personality Disorders (SCID-5-PD): Structured clinician interview for PD diagnosis (First et al., 2016). The gold standard in research settings.

McLean Screening Instrument for BPD (MSI-BPD): 10-item self-report screen for BPD (Zanarini et al., 2003). Score of 7+ has good sensitivity for BPD. Useful at intake when BPD is suspected.

What the Exam Rewards (and Punishes)

Rewards: Recommending DBT for BPD (it has the strongest evidence base for this population). Maintaining consistent limits with ASPD clients. Distinguishing OCPD from OCD by ego-syntonicity. Recognizing that personality disorders require longer-term treatment than episodic clinical disorders. Understanding transference and countertransference as clinical tools in PD treatment.

Punishes: Diagnosing a personality disorder based on acute crisis presentation alone (must establish pervasiveness and stability over time). Recommending short-term

CBT as standalone for BPD. Responding to BPD splitting (idealization/devaluation) with rescue or rejection rather than therapeutic consistency. Missing that ASPD cannot be diagnosed before age 18 (Conduct Disorder is the childhood precursor).

Worked Mini-Case

Vignette: Lia, 26, presents following a "bad breakup" in which she says she "couldn't function." Her intake form shows three prior therapists in two years, each relationship ending abruptly ("they didn't get me" or "she was terrible"). She reports a history of cutting when "overwhelmed," describes feeling "like I don't know who I am," and has a pattern of intense relationships that follow the same arc: idealization, then sudden shift to hatred. She reports her ex was "the most amazing person I've ever met" for six months and then "pure evil." On the PHQ-9 she scores 16, but she notes "I felt fine three days ago."

Reasoning chain:

Step 1 — Recognize the pattern: Four BPD criteria are immediately apparent: (1) unstable, intense relationships with idealization/devaluation; (2) identity disturbance ("I don't know who I am"); (3) self-harm (cutting); (4) rapid mood shifts not consistent with an episodic disorder (fine three days ago, scoring 16 now).

Step 2 — Safety: History of self-harm requires direct safety assessment this session. Ask about current cutting, frequency, medical attention needed, and suicidal ideation.

Step 3 — Relationship pattern in the counseling relationship: Lia's history of three therapists in two years and abrupt endings is data. The therapeutic relationship will likely follow the same pattern unless managed carefully. DBT's structure (skills group + individual therapy + phone coaching + therapist consultation) is designed precisely for this.

Step 4 — Treatment: **DBT is the evidence-based first-line treatment for BPD.** The four skills modules (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness) directly target the features present in Lia's case.

Answer on exam questions: The exam rewards knowing DBT for BPD and recognizing that the mood instability in BPD is reactive and interpersonally driven, not episodic.

Practice Questions

Question 60. A client with Borderline Personality Disorder who is in DBT calls the counselor between sessions in distress, saying "you're the only one who's ever really understood me" and asking for an extended phone call. The counselor's MOST appropriate response is:

A. Provide the extended phone call support, as therapeutic alliance is especially critical in BPD and ruptures at this stage could be fatal

B. Schedule an emergency session for the next morning while providing empathic verbal support on the call tonight

C. Decline the call and remind the client of the session boundary, as reinforcing crisis calls creates dependency

D. Follow the established DBT phone coaching protocol, limiting the call to skill use and crisis de-escalation

Question 61. A court-ordered client with ASPD tells the counselor that he "finally gets it" after four sessions, discusses his insight extensively, and asks the counselor to write a letter to the court reflecting his progress. The counselor should:

A. Write the letter as requested since the client's expressed insight is clinically meaningful and withholding it would damage the therapeutic alliance

B. Decline, explaining to the client that the counselor cannot advocate to the court regarding his treatment progress at this early stage

C. Carefully evaluate the pattern of presentation, consider the role of consequence in motivation, and consult with a supervisor before responding to the request

D. Write the letter with appropriate qualifiers that reflect only what has been directly observed in session rather than endorsing any lasting character change

Answer Key — Chapter 12 Practice Questions

Q60 — Answer: D

Rationale: DBT structures phone coaching specifically for this scenario — brief, skill-focused, not extended support. The protocol prevents the reinforcement of crisis behavior while maintaining connection. A provides exactly the extended contact the protocol is designed to limit, and the "only one who understands me" framing is the idealization the treatment anticipates rather than a reason to depart from it. C withdraws support in a population with genuine risk and treats the call as a boundary violation rather than a coaching opportunity. B converts a between-session coaching contact into an emergency session without any assessment indicating one is needed, which reinforces crisis escalation as the route to more contact.

Q61 — Answer: C

Rationale: ASPD clients may present superficial insight and use the therapeutic relationship instrumentally. Consultation before advocacy is clinically indicated.

Chapter 13 — Bipolar and Related Disorders

Overview

Bipolar disorders are among the most serious — and most misdiagnosed — conditions the NCMHCE tests. The exam's highest concern: **missing a manic or hypomanic history when a client presents in a depressive episode.** Prescribing an antidepressant to someone with unrecognized bipolar disorder can precipitate a manic episode. The exam rewards thoroughness — always take a mood episode history, not just a current symptom snapshot.

The second critical concept: mania and hypomania differ in severity and impact, not in kind. Hypomania is a lighter, shorter, and less impairing version of mania. Understanding this distinction separates Bipolar I from Bipolar II.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Diagnosis	ICD-10	ICD-11	Defining Feature	First-Line Treatment
Bipolar I Disorder	F31.x	6A60	At least one manic episode (with or without depression)	Mood stabilizer (lithium, valproate), psychotherapy adjunct

Diagnosis	ICD-10	ICD-11	Defining Feature	First-Line Treatment
Bipolar II Disorder	F31.81	6A61	At least one hypomanic episode + at least one major depressive episode; NO full mania	Mood stabilizer, lamotrigine; psychotherapy
Cyclothymic Disorder	F34.0	6A62	≥2 years of hypomanic and depressive symptoms; never meeting full criteria for either	Mood stabilizer, psychotherapy
Substance/ Medication-Induced Bipolar and Related Disorder	F1x.x4	6C4x.70	Manic symptoms during or shortly after substance use/ withdrawal	Treat underlying cause
Bipolar and Related Disorder Due to Another Medical Condition	F06.33	6E62.1	Etiologically linked to medical condition	Treat underlying cause

ICD-11 notes: Substance/Medication-Induced Bipolar and Related Disorder: x = the substance; ICD-11 uses one substance-induced mood disorder code for depressive and manic symptoms. Bipolar and Related Disorder Due to Another Medical Condition: use 6E62.2 for mixed symptoms.

The Clinical Picture

Mania (Bipolar I): A distinct period of abnormally elevated, expansive, or irritable mood plus increased goal-directed activity or energy, lasting at least one week (or any duration if hospitalization required). During this period,

three or more of the following (four or more if the mood is only irritable): grandiosity, decreased need for sleep (feels rested after 3 hours), pressured speech, racing thoughts, distractibility, increased goal-directed activity, and reckless behavior. The key marker: **the change is distinct from baseline.** The client who is always ambitious and energetic is not manic. The client who has gone three days without sleep, maxed out four credit cards, and started four new business ventures simultaneously — that is mania.

Hypomania (Bipolar II): The same symptom criteria, but lasting at least four consecutive days, not severe enough to require hospitalization, and without psychotic features. **The crucial point: the person feels great during hypomania.** They are productive, creative, charming, and energized. They often do not identify hypomania as a problem — they come in depressed and, when asked about their "high" periods, describe them as their best functioning. This is why hypomanic episodes are missed.

Exam trap: A client presents with MDD. You review the history and she says, "Actually, there are periods — usually in the spring — when I need less sleep and I get a lot done. I feel fantastic during those times." This is a hypomanic episode. The diagnosis is **Bipolar II, not MDD.** The treatment changes significantly.

Differentiating Within the Family

Bipolar I vs. Bipolar II: The single distinguishing feature is whether a full manic episode has occurred. Bipolar I requires at least one manic episode (may or may not have depressive episodes). Bipolar II requires at least one hypomanic episode and at least one major depressive episode — and **never** a full manic episode. A Bipolar II client who later develops a full manic episode becomes Bipolar I.

Bipolar II vs. Cyclothymia: Cyclothymia involves chronic, fluctuating mood over ≥2 years, with periods of hypomanic symptoms and periods of depressive symptoms that **never** meet full criteria for either a hypomanic episode or a major depressive episode. It is milder but chronically unstable.

BPD vs. Bipolar II: The critical distinguishing feature is that Bipolar II has discrete episodes (days to weeks) that are relatively autonomous from triggers; BPD mood shifts are reactive to events (hours to days), interpersonally triggered, and accompanied by identity disturbance and self-harm.

Bipolar I vs. Substance/Medication-Induced Bipolar and Related Disorder: Manic or hypomanic symptoms that emerge during or soon after stimulant, cocaine, or corticosteroid use, or during intoxication/withdrawal, point to the induced diagnosis. Two exam-relevant rules: (1) independent bipolar disorder is supported if mood symptoms preceded the substance or persist about a month after it stops; (2) a full manic episode that emerges during antidepressant treatment and persists at a fully syndromal level beyond the drug's physiological effect is sufficient evidence for Bipolar I, not an induced disorder (APA, 2022). Correct counselor action: document the timeline, notify the prescriber promptly, and assess safety and impulsive behavior.

Bipolar I vs. Bipolar and Related Disorder Due to Another Medical Condition: Manic or hypomanic symptoms that are the direct physiological consequence of a medical condition — hyperthyroidism, Cushing's disease, multiple sclerosis, stroke, or traumatic brain injury. Red flags: first manic episode after about age 50, no family or personal mood history, or neurological signs. A first manic episode in an older adult is a medical-evaluation question before it is a bipolar question.

Assessment Tools in Context

Mood Disorder Questionnaire (MDQ): A brief screening tool (13 items) for bipolar spectrum disorders (Hirschfeld et al., 2000). A positive screen (7+ symptoms endorsed, occurring at the same time, causing moderate or serious problems) prompts further clinical assessment. The MDQ is useful at intake to screen for bipolar before starting antidepressant treatment — initiating an SSRI without screening can trigger mania in undiagnosed Bipolar II.

PHQ-9 in the Bipolar Context: The PHQ-9 measures current depressive symptom severity but does not detect bipolarity (Kroenke et al., 2001). A client who scores high on the PHQ-9 may have either unipolar MDD or bipolar depression. The PHQ-9 is necessary but not sufficient — it must be paired with a mania screen.

Young Mania Rating Scale (YMRS): A clinician-administered instrument measuring current manic symptom severity across 11 items (Young et al., 1978). Used primarily in psychiatric settings to track treatment response.

Clinical Interview for Hypomanic Episodes: The most reliable assessment tool remains a thorough clinical interview exploring the client's lifetime history of elevated mood, decreased need for sleep, increased energy, impulsive behavior, and grandiosity. This must be done before confirming any unipolar depression diagnosis.

What the Exam Rewards (and Punishes)

Rewards: Taking a full mood history before diagnosing or treating depression (rule out prior manic/hypomanic episodes). Recognizing that hypomania is often ego-syntonic and not spontaneously reported. Recommending mood stabilizer (not antidepressant monotherapy) as primary pharmacological treatment. Psychotherapy as an adjunct (CBT for bipolar, IPSRT — Interpersonal and Social Rhythm Therapy — are evidence-based; Frank, 2005).

Psychoeducation about sleep regulation as a critical component of bipolar management.

Punishes: Diagnosing and treating MDD without inquiring about prior elevated mood periods. Recommending antidepressant monotherapy for a bipolar client (can precipitate mania). Confusing BPD mood instability with hypomanic episodes. Failing to assess for psychosis during manic episodes (Bipolar I with psychotic features requires a higher level of care).

Worked Mini-Case

Vignette: Tanya, 29, self-referred. Presents with depressed mood, hypersomnia, difficulty concentrating, and feelings of worthlessness for three weeks. PHQ-9: 18 (moderately severe). She requests "the same antidepressant that helped my friend." When the counselor explores her history, she mentions a period two years ago, lasting about a week, when she "felt amazing — barely needed sleep, started three business projects, maxed out a credit card." She describes this period as "the only time I felt like myself." Her PCP prescribed an SSRI at that time; within two weeks she became "wired, couldn't sleep at all, and got into a screaming fight with my mother over nothing."

Reasoning chain:

Step 1 — Safety: No suicidal ideation reported. Proceed to step 2.

Step 2 — Differential: The history of elevated mood, decreased sleep need, impulsive spending, and increased goal-directed activity lasting at least four days meets criteria for a hypomanic episode. The SSRI-induced activation (insomnia, irritability) further suggests bipolar vulnerability. **This is Bipolar II Disorder, not MDD** — despite the current depressive presentation.

Step 3 — Treatment implication: An SSRI without a mood stabilizer is contraindicated. Treatment per the American Psychiatric Association practice guideline includes a

mood stabilizer (lithium or valproate) or atypical antipsychotic, with psychotherapy (CBT for bipolar, IPSRT) as an essential adjunct. The counselor provides psychoeducation about the bipolar diagnosis, helps Tanya understand why an antidepressant alone is not appropriate, and coordinates with the prescriber.

Answer selection logic: The exam rewards recognizing hypomania in the history. The novice trap is to focus on the current depression and miss the bipolar diagnosis entirely.

Practice Questions

Question 62. A client presents with persistent low mood, hypersomnia, and difficulty concentrating for three months, scoring 14 on the PHQ-9. Before confirming an MDD diagnosis, the counselor should:

A. Administer the BDI-II to provide a second standardized measure confirming the severity level indicated by the PHQ-9

B. Begin CBT immediately, as it is an appropriate first-line treatment for both unipolar and bipolar depressive presentations

C. Consult with a prescribing psychiatrist before beginning any form of psychotherapy

D. Screen for prior episodes of elevated mood, decreased sleep need, increased energy, or impulsive behavior

Question 63. A client with Bipolar I stable on lithium for 18 months says she “feels so normal” and wonders whether she still needs the medication. The counselor should:

A. Refer the question entirely to her prescribing psychiatrist since medication decisions are outside counseling scope

B. Inform her that Bipolar I is a lifelong condition requiring permanent uninterrupted maintenance pharmacotherapy

C. Explore her concerns while providing psychoeducation about the high relapse risk of discontinuing mood stabilizers

D. Support her autonomy by helping her work with her physician to develop a supervised medication taper plan

Answer Key — Chapter 13 Practice Questions

Q62 — Answer: D

Rationale: Bipolar disorder must be ruled out before treating depression. An SSRI initiated without this screening can trigger mania in undiagnosed Bipolar II.

Q63 — Answer: C

Rationale: This is a psychoeducation and collaborative exploration moment. The counselor does not make the medication decision, but does provide accurate information about relapse risk — which is substantial within months of lithium discontinuation. Framing it as “permanent” (B) is less collaborative; moving straight to a taper plan (D), even with her physician, skips exploring her concerns and the relapse risk first; referring the question entirely to the prescriber (A) abandons the counselor’s psychoeducational role.

Chapter 14 — Neurodevelopmental Disorders

Overview

Neurodevelopmental disorders are characterized by developmental deficits that emerge early in life. On the NCMHCE, the exam primarily tests: recognition of ADHD presentations across the lifespan, awareness of the clinical picture of Autism Spectrum Disorder (ASD) in functioning adults, appropriate use of psychological testing, and the distinction between intellectual disability and learning disorders.

Diagnosis Reference Table

Diagnostic criteria, specifiers, durations, and ICD-10-CM codes in this table follow DSM-5-TR (APA, 2022); ICD-11 codes follow WHO ICD-11 MMS (WHO, 2019).

Diagnosis	ICD-10	ICD-11	Key Feature	First-Line Treatment
ADHD — Combined Presentation	F90.2	6A05.2	≥6 inattentive + ≥6 hyperactive/impulsive symptoms in ≥2 settings, onset before age 12	Multimodal: behavioral parent training, school coordination, medication evaluation (stimulants), psychoeducation
ADHD — Predominantly Inattentive	F90.0	6A05.0	≥6 inattentive symptoms; hyperactive/impulsive symptoms below threshold	Same multimodal approach; frequently under-identified in girls and in adults

Diagnosis	ICD-10	ICD-11	Key Feature	First-Line Treatment
ADHD — Predominantly Hyperactive/ Impulsive	F90.1	6A05.1	≥6 hyperactive/ impulsive symptoms; inattentive symptoms below threshold	Same multimodal approach; most visible presentation in young children
Autism Spectrum Disorder (ASD)	F84.0	6A02	Persistent deficits in social communication AND restricted/ repetitive behaviors; present from early development	ABA (behavioral), speech-language therapy, OT, social skills training; no curative treatment
Intellectual Developmental Disorder (IDD)	F70–F73	6A00	Significant deficits in intellectual functioning (IQ ≈ 70 or below) AND adaptive behavior; onset in developmental period	Skills-based support, educational planning, family psychoeducation; severity based on adaptive functioning, not IQ alone
Specific Learning Disorder (SLD)	F81.x	6A03	Specific academic skill deficits below what intelligence and instruction would predict	Specialized evidence-based instruction; academic accommodations (504/IEP)
Developmental Coordination Disorder	F82	6A04	Marked impairment in motor skill acquisition and execution	Occupational therapy

Diagnosis	ICD-10	ICD-11	Key Feature	First-Line Treatment
Tourette's Disorder	F95.2	8A05.00	Multiple motor tics + ≥1 vocal tic, present for >1 year; begins before age 18	CBIT (Comprehensive Behavioral Intervention for Tics); medication (alpha-agonists) if severe
Social (Pragmatic) Communication Disorder	F80.82	6A01.22	Persistent difficulties in social use of language; no restricted/ repetitive behaviors	Speech-language therapy

ICD-11 notes: Intellectual Developmental Disorder: severity runs 6A00.0 (mild) to 6A00.3 (profound). Tourette's Disorder: coded in the nervous system chapter (8A05.00). Social Communication Disorder: no separate disorder; coded as a developmental language disorder subtype.

The Clinical Picture

ADHD Across the Lifespan: In elementary-age children, the hyperactive-impulsive presentation is most visible. Both teacher and parent confirmation across settings is required — symptoms that appear only in one environment are not sufficient. By middle school, hyperactivity often diminishes and inattentive symptoms become dominant.

In adults, the presenting complaint is rarely "I can't sit still." Adults report: "I can't finish anything," "I'm always late," "I lose things constantly." Many have developed compensatory strategies that partially mask symptoms until demands escalate. ADHD diagnosis in adults requires evidence of symptom presence **before age 12**, not merely that symptoms are present now.

Gender Differences: Girls and women are significantly underdiagnosed. Female ADHD more often involves the inattentive subtype, internalized distress, and social

masking. Hyperactivity in girls may manifest as excessive talking rather than gross motor activity. Many women receive their first ADHD diagnosis in adulthood.

ASD in Higher-Functioning Adults: Adults with undiagnosed ASD often present for anxiety, depression, burnout, or relationship difficulties — not for autism. ASD features emerge in the clinical relationship: unusual communication style, difficulty with reciprocity, literal interpretation of figurative language, intense focused interests, rigidity around routines, and sensory sensitivities.

Intellectual Developmental Disorder: Severity is determined by adaptive functioning, not IQ alone. Mild IDD (approximately 85% of cases): most individuals can live semi-independently with supports. The counselor's role is assessment coordination, family psychoeducation, and functional skill planning.

IDD severity is coded by level — mild (F70), moderate (F71), severe (F72), and profound (F73) — and each level is defined by adaptive functioning in the conceptual, social, and practical domains (APA, 2022). Moderate IDD (about 10% of cases) typically requires substantial support for academic and work tasks; severe IDD (about 3–4%) involves limited spoken language and support for all activities of daily living. **Profound IDD** (about 1–2%) involves very limited symbolic communication (largely nonverbal and gestural), dependence on others for all aspects of daily care, health, and safety, and frequent co-occurring sensory and motor impairments. For severe and profound levels the counselor's work shifts to the system around the client: caregiver support and respite, behavioral support planning, communication-based assessment of distress (behavior change is often the only signal of pain, illness, or abuse), and interdisciplinary coordination.

Differentiating Within the Family

ADHD vs. GAD: Both present with concentration difficulty, restlessness, and sleep disruption. In ADHD, inat-

tention is pervasive and present before age 12, not driven by worry. In GAD, cognitive difficulties are secondary to excessive worry — treat the anxiety and cognitive symptoms often improve.

Specifying the ADHD Presentation: The three presentations are specifiers of one disorder, not separate disorders. **Combined presentation (F90.2)** — criteria met for both inattention and hyperactivity-impulsivity over the past 6 months. **Predominantly inattentive presentation (F90.0)** — inattention criteria met, hyperactivity-impulsivity criteria not met. **Predominantly hyperactive/impulsive presentation (F90.1)** — hyperactivity-impulsivity criteria met, inattention criteria not met. The threshold is six symptoms per domain for children and five for clients age 17 and older (APA, 2022). Because the specifier reflects only the past 6 months, presentation can change across development — the hyperactive child often becomes the inattentive adolescent — so the exam rewards reassessing the presentation rather than assuming it is fixed.

ADHD vs. MDD: Depression causes concentration difficulty and psychomotor changes that can mimic ADHD. Key distinction: MDD has a defined episode with onset and change from previous functioning; ADHD is chronic and developmental. If inattention began with a depressive episode, MDD is the primary driver.

ASD vs. Social Anxiety Disorder: Both involve difficulty in social situations. In Social Anxiety Disorder, the person desires social connection but fears scrutiny or humiliation. In ASD, difficulty with social reciprocity and perspective-taking is present across all contexts, not only evaluative situations.

ASD vs. Social (Pragmatic) Communication Disorder: Both involve deficits in the social use of verbal and nonverbal communication — greeting and sharing information appropriately, adapting language to the listener or setting, following conversational rules such as turn-tak-

ing, and understanding inference, humor, or nonliteral language. The deciding feature is the absence of restricted, repetitive patterns of behavior, interests, or activities. If restricted/repetitive behaviors are present now or by history, the diagnosis is ASD, and SPCD cannot be diagnosed when ASD criteria are met (APA, 2022). Treatment centers on speech-language therapy and social communication skills training; the counselor's role is referral, school coordination, and addressing the secondary anxiety and peer difficulties.

OCPD vs. OCD (in this context): Students often confuse these. OCPD is an ego-syntonic personality pattern; OCD involves ego-dystonic intrusive obsessions and compulsions. See Chapter 16 for OCD.

Specific Learning Disorder vs. ADHD: SLD involves specific academic skill deficits (reading, math, written expression) below what intelligence predicts. ADHD involves broad attention and executive function deficits affecting academic performance across domains. Both can co-occur.

Assessment Tools in Context

Vanderbilt ADHD Diagnostic Rating Scale: Free parent and teacher rating scales for ADHD in children 6–12 (Wolraich et al., 2003). Captures inattentive and hyperactive-impulsive symptoms across home and school settings. Provides the cross-setting data required before ADHD can be diagnosed.

Conners Rating Scales (Conners-3): Comprehensive rating scales for parents, teachers, and self-report (adolescents). Normed by age and gender (Conners, 2008). Gold standard for multi-informant ADHD assessment.

CAARS (Conners' Adult ADHD Rating Scales): Self-report and observer-rated scales for ADHD in adults. Validated for individuals 18+ (Conners et al., 1999).

ADOS-2 (Autism Diagnostic Observation Schedule, 2nd Edition): Clinician-administered structured observa-

tional assessment (Lord et al., 2012). The gold standard for ASD diagnosis. Counselors do not administer it, but must know to refer for it.

Vineland Adaptive Behavior Scales (Vineland-3): Measures adaptive behavior across Communication, Daily Living Skills, Socialization, and Motor Skills domains (Sparrow et al., 2016). Essential for IDD diagnosis.

What the Exam Rewards (and Punishes)

Rewards: Multi-informant, multi-method, cross-setting assessment over any single measure. Recognizing that ADHD diagnosis in adults requires evidence of childhood onset. Identifying ASD presentations in adults that don't match the childhood stereotype. Neurodiversity-affirming language and strengths-based assessment.

Punishes: Diagnosing ADHD based on parent report alone without teacher/school data. Missing the age-12 onset requirement for ADHD. Attributing all academic difficulty to ADHD without assessing for comorbid SLD. Missing ASD features in women and adults who have developed compensatory/masking strategies.

Worked Mini-Case

Vignette: Jaylen, 9, is referred by his school for "inability to sit still, constant interrupting, and not finishing assignments." His teacher reports the same behaviors in class since kindergarten. His mother says he is "a handful at home but has always been this way." Jaylen is in third grade and reading at a first-grade level despite appearing bright. There is no trauma history. He was seen briefly by a pediatrician who suggested "wait and see."

Reasoning chain:

Step 1 — Safety: No safety concerns. Proceed.

Step 2 — Differential: Symptoms are consistent with ADHD (hyperactive-impulsive and inattentive symptoms, childhood onset, two settings). The reading difficulty

raises the possibility of co-occurring Specific Learning Disorder — which would require its own intervention.

Step 3 — Assessment: A comprehensive, multi-informant assessment is indicated before any diagnosis or intervention: behavioral rating scales completed by both parents and teachers, review of school records, and standardized cognitive/academic screening.

Step 4 — Treatment will depend on assessment findings. If ADHD: multimodal approach (behavioral parent training, school accommodations, possible medication referral). If comorbid SLD: specialized reading instruction (evidence-based programs like Orton-Gillingham) plus ADHD interventions.

Practice Questions

Question 64. A 28-year-old client is referred after her employer cited repeated missed deadlines and organizational failures. She reports she has "always been scattered" but managed through college with extra time accommodations. She meets criteria for ADHD — predominantly inattentive. The counselor's MOST appropriate first step in treatment planning is:

A. Refer immediately for a stimulant medication evaluation to address the attention and organization deficits her employer cited

B. Provide psychoeducation, assess for comorbidities (anxiety, depression, sleep), and develop a multimodal plan

C. Begin CBT focused on distorted cognitions about productivity and her self-perception of being "always scattered"

D. Recommend time management coaching as a standalone intervention targeting her missed deadlines and organizational failures

Question 65. Which of the following behaviors in a 5-year-old child is MOST consistent with an ASD diagnosis?

A. Difficulty sharing toys with siblings and occasional arguments over whose turn it is during play

B. Repetitive, stereotyped hand-flapping when excited, limited eye contact, and speaking in scripts from memorized TV shows

C. Excessive crying and clinginess when separated from parents at the start of the school day, easing by mid-morning

D. Impulsivity, a short attention span, frequent fidgeting, and inability to wait for turns during structured classroom activities

Answer Key — Chapter 14 Practice Questions

Q64 — Answer: B

Rationale: Psychoeducation and comprehensive assessment are the first steps in multimodal treatment planning for adult ADHD. Medication evaluation (A) is often appropriate but not necessarily the first step. CBT alone (C) is a component, not a complete plan.

Q65 — Answer: B

Rationale: The combination of repetitive motor behaviors (hand-flapping), limited eye contact, and echolalia (scripted speech) is a hallmark of ASD. Options A, C, and D are developmentally common or suggest other conditions.

Chapter 15 — Neurocognitive Disorders

Overview

Neurocognitive disorders (NCDs) are characterized by a decline in cognitive function from a previous level of performance. The exam tests two key skills: (1) recognizing delirium as a medical emergency distinct from dementia, and (2) understanding the counselor's role in supporting clients and caregivers rather than reversing the condition.

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Key Feature	Counselor Role
Major Neurocognitive Disorder (Dementia)	F02.xx	6D80–6D85	Significant decline in ≥1 cognitive domain; interferes with independence	Caregiver support, behavioral management, coordination
Mild Neurocognitive Disorder	G31.84	6D71	Modest cognitive decline; independence maintained	Psychoeducation, monitoring, lifestyle
Delirium	F05	6D70	Acute, fluctuating disturbance in attention and awareness; medical emergency	Refer immediately for medical evaluation

ICD-11 notes: Major Neurocognitive Disorder: dementia is coded by cause (for example, 6D80 dementia due to

Alzheimer disease). Mild Neurocognitive Disorder: add a code for the cause.

The Clinical Picture

Delirium vs. Dementia: The most important distinction in this chapter.

Feature	Delirium	Dementia
Onset	Acute (hours to days)	Gradual (months to years)
Course	Fluctuating	Slowly progressive
Attention	Markedly impaired	Relatively preserved early
Reversible?	Often yes (treat cause)	Rarely reversible
Common causes	Infection, medication, metabolic	Alzheimer's, vascular, Lewy body

A client who "seemed fine last week and now doesn't know where they are" has delirium until proven otherwise. Delirium is a medical emergency. Do not attempt psychotherapy. Refer immediately.

What the Exam Rewards (and Punishes)

Rewards:

- Recognizing delirium and referring for immediate medical evaluation
- Providing grief-informed support to caregivers (caregiver burden is immense)
- Functional assessment to match level of care to current ability
- Using structured, predictable environments and routines for dementia clients

Punishes:

- Treating delirium as a behavioral or psychiatric problem

- Attempting insight-oriented therapy with moderate-to-severe dementia clients
- Missing caregiver burnout in the family system

Practice Questions

Question 66. An 81-year-old client who has been in counseling for grief presents with his daughter, who reports he has been "confused and agitated since last night" and cannot remember where he is. He is usually oriented and articulate. The counselor should:

A. Contact his prescribing physician immediately and facilitate emergency evaluation

B. Administer a cognitive screening tool such as the MoCA and schedule a follow-up to track changes

C. Provide reassurance to both of them and see whether the confusion resolves by the next session

D. Increase session frequency to monitor cognitive changes and his adjustment to grief

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. An 80-year-old is brought in for confusion that began two days ago, fluctuates across the day, and includes prominent inattention and disorientation. She recently started a new medication and has a urinary tract infection. The MOST likely diagnosis is:

A. Major Neurocognitive Disorder (dementia)

B. Major Depressive Disorder

C. Mild Neurocognitive Disorder

D. Delirium

Answer: D. Acute onset, a fluctuating course, prominent inattention, and a medical/medication precipitant define delirium — a medical emergency. Dementia (A) is insidious and progressive. What would change the answer: a

gradual, months-long decline without acute change → a major or mild neurocognitive disorder.

Assessment — what to do next. The MOST appropriate immediate step is:

A. Administer the MoCA to document the extent of her cognitive deficits, and schedule a follow-up visit in one month to track change

B. Diagnose dementia and provide caregiver education about managing confusion, disorientation, and daily routines at home

C. Begin cognitive stimulation therapy to slow her decline, using structured activities such as reminiscence and orientation exercises

D. Facilitate immediate medical evaluation to identify and treat the underlying cause; use a delirium-oriented assessment (e.g., the Confusion Assessment Method)

Answer: D. Delirium is a medical emergency; the counselor refers immediately, and the CAM screens for it. Cognitive testing (A) and dementia care (B) misclassify an acute, potentially reversible state.

Treatment Decision Flow

The two skills tested: recognize delirium as a medical emergency, and understand that the counselor supports rather than reverses a major NCD.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Acute, fluctuating confusion with inattention (delirium)	Treat as a medical emergency; immediate medical referral to find the reversible cause	Managing it as dementia or beginning psychotherapy
Major NCD (dementia)	Supportive care, caregiver psychoeducation, environmental and safety strategies	Cognitive restructuring or insight therapy aimed at reversing the decline

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Caregiver burnout	Caregiver support, respite, psychoeducation	Focusing only on the identified patient

Sequencing and caveats. Delirium is distinguished by acute onset, fluctuating course, and impaired attention; it demands medical workup for a reversible cause. In major NCD the counselor's role is support and safety, not cognitive recovery.

Avoid / contraindicated. Do not attempt insight-oriented or restructuring work aimed at reversing cognitive decline.

Scope. Refer for medical evaluation; coordinate with the care team.

Differentiating Within the Family

Major NCD vs. Mild NCD (Mild Cognitive Impairment): The decisive feature is functional impact. Major NCD involves cognitive decline significant enough to interfere with independence in daily activities (managing finances, medications, driving). Mild NCD involves measurable cognitive decline that does not yet impair independence — the person compensates with strategies and extra effort.

Major NCD vs. Delirium: Both involve cognitive disturbance, but delirium is acute (onset over hours to days), fluctuating in severity across the day, and defined by a disturbance in attention and awareness. Major NCD is gradual in onset and stable within a day, even if progressive over time. Delirium in an older adult with dementia — superimposed delirium — is common and a medical emergency.

Major NCD (Alzheimer's) vs. Major NCD (Frontotemporal): Alzheimer's presents with memory impairment first, then other domains. Frontotemporal NCD presents with personality/behavioral changes (disinhibi-

tion, apathy, loss of empathy) or language problems first, with memory relatively preserved early. This distinction is clinically important because behavioral symptoms in FTD are sometimes misdiagnosed as psychiatric disorders.

Major NCD vs. Pseudodementia (Depression-Related Cognitive Impairment): Older adults with severe depression frequently present with cognitive complaints that mimic dementia. The deciding feature: depression-related cognitive impairment typically has more acute onset, the client is distressed about their cognitive difficulties (in dementia, insight is often impaired), and symptoms improve with depression treatment. GDS and neuropsychological testing help distinguish (Sheikh & Yesavage, 1986).

Assessment Tools in Context

Mini-Mental State Examination (MMSE): 30-point clinician-administered screen for cognitive impairment (Folstein et al., 1975). Tests orientation, registration, attention/calculation, recall, language, and visuospatial skills. Score ≤23 suggests significant impairment. Limitations: education and language bias; less sensitive to mild impairment.

Montreal Cognitive Assessment (MoCA): More sensitive than MMSE for mild cognitive impairment (Nasreddine et al., 2005). 30-point scale covering more domains. Score ≤25 suggests impairment. Recommended over MMSE for detecting early NCD.

Clock Drawing Test (CDT): Quick visuospatial and executive function screen (Shulman, 2000). Sensitive to frontal lobe dysfunction. Often used alongside MMSE or MoCA.

Geriatric Depression Scale (GDS): Distinguishes depression-related cognitive impairment from NCD (Sheikh & Yesavage, 1986). The 15-item short form is counselor-appropriate for older adults.

Functional Assessment: ADL (Activities of Daily Living) and IADL (Instrumental ADLs) scales measure the functional impact of cognitive decline — essential for distinguishing Major from Mild NCD. IADLs (managing finances, medications, telephone) decline before basic ADLs (dressing, eating, toileting).

Exam tip: The counselor's role in NCD is assessment, psychoeducation, caregiver support, and referral — not diagnosis of the specific etiology. Always refer to a neuropsychologist or neurologist for differential diagnosis of NCD type.

Worked Mini-Case

Vignette: Dorothy, 74, is brought in by her daughter, who reports that Dorothy has been repeating questions, missing appointments, and recently left the stove on. Dorothy says she is "just getting older" and her memory "has always been a little off." She manages her own medications but has missed doses three times this month. She denies depression and mood is pleasant. MMSE score: 22/30.

Question: The counselor's MOST appropriate next step is:

A. Reassure Dorothy that some memory decline is normal aging and schedule a follow-up in six months

B. Diagnose Major Neurocognitive Disorder and initiate a care plan with her daughter

C. Administer the MoCA for a more sensitive assessment, discuss concerns with the family, and refer for neuropsychological evaluation

D. Refer Dorothy for antidepressant evaluation since late-life depression frequently presents with cognitive symptoms

Best answer: C

Reasoning chain: MMSE of 22 indicates significant impairment, but the counselor cannot diagnose or specify

the type of NCD — that requires neuropsychological evaluation. The immediate steps are: more sensitive testing (MoCA), family engagement (with Dorothy's consent), and referral.

Why distractors lose: - A: An MMSE of 22 with functional impairment is not normal aging - B: The counselor cannot assign a specific NCD diagnosis without neuropsychological workup - D: Depression screening is valuable but the clinical picture points primarily to cognitive rather than mood etiology

Answer Key — Chapter 15 Practice Questions

Q66 — Answer: A

Rationale: Acute-onset confusion in a previously oriented elder is delirium until proven otherwise. Medical emergency. Refer immediately. B, C, and D all delay necessary medical intervention.

Chapter 16 — Obsessive-Compulsive and Related Disorders

Overview

OCD and related disorders are characterized by unwanted, repetitive thoughts and/or behaviors that the person cannot stop despite wanting to. The critical exam concept: these disorders are ego-dystonic — the client experiences the obsessions or compulsions as unwanted intrusions, not as expressions of their own values. This is what distinguishes OCD from OCPD (which is ego-syntonic).

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Core Feature	Treatment
OCD	F42.2	6B20	Obsessions (intrusive thoughts) + compulsions (rituals that reduce anxiety)	ERP (exposure and response prevention), medication (SSRI)
Body Dysmorphic Disorder	F45.22	6B21	Preoccupation with perceived defect in appearance	CBT/ERP, SSRI
Hoarding Disorder	F42.3	6B24	Difficulty discarding possessions + excessive acquisition + cluttered living space	CBT (specialized), motivational interviewing

Diagnosis	ICD-10	ICD-11	Core Feature	Treatment
Trichotillomania	F63.3	6B25.0	Recurrent hair pulling	HRT (habit reversal training)
Excoriation (Skin-Picking)	F42.4	6B25.1	Recurrent skin picking	HRT

The Clinical Picture

OCD in a case sim: A client describes intrusive thoughts ("What if I hurt someone?", "What if I left the stove on?") that they experience as horrifying and foreign to their values. They engage in rituals — checking, counting, washing, reassurance-seeking — that provide temporary relief but fuel the obsessive cycle in the long run. The checking or washing is not the problem; the problem is the attempted solution (the ritual) that prevents habituation to the obsessive thought.

The OCD cycle: Intrusive thought → anxiety spike → compulsion → temporary relief → intrusion returns stronger → repeat

ERP works by breaking the link between the obsession and the compulsion: the client confronts the feared thought without engaging the compulsion, allowing anxiety to naturally reduce through habituation (Foa et al., 2012).

Hoarding Disorder is often misunderstood. Clients with hoarding disorder form intense attachments to objects, experience severe distress at the prospect of discarding, and may not perceive the hoarding as a problem (making it partially ego-syntonic). CBT for hoarding is specialized and requires motivational enhancement, not just behavioral strategies.

Differentiating Within the Family

OCD vs. OCPD: OCD = ego-dystonic, unwanted obsessions and compulsions. OCPD = ego-syntonic, the person

values their orderliness and perfectionism. These can co-occur but they are distinct.

OCD vs. GAD: Both involve worry. In OCD, the worries are intrusive, often bizarre or ego-dystonic ("What if I'm secretly a violent person?"), and are accompanied by ritualistic responses. GAD worries are about realistic life concerns (finances, health, relationships) and are not accompanied by compulsions.

BDD vs. Normal Appearance Concern: Everyone has appearance concerns. BDD is distinguished by the preoccupation being time-consuming (DSM-5-TR sets no hourly threshold), causing significant distress, leading to repetitive behaviors (mirror-checking, excessive grooming, seeking reassurance), and being based on a perceived defect that is either absent or minimal. BDD is associated with high rates of suicidality.

What the Exam Rewards / What It Punishes

Rewards:

- ERP as the first-line treatment for OCD (not CBT without the exposure/response-prevention component)
- Recognizing that compulsions maintain the disorder by providing short-term relief
- HRT (habit reversal training; Woods & Twohig, 2008) for trichotillomania and excoriation
- Assessing suicidality in BDD (high-risk population)

Punishes:

- Using relaxation techniques to manage OCD (reduces anxiety in the moment, reinforces avoidance)
- Treating OCD with standard CBT without the ERP component
- Responding to hoarding disorder by immediately pushing the client to discard items (increases reactance)

Practice Questions

Question 67. A client with OCD describes checking the stove 30–40 times before leaving his apartment. He is 45 minutes late to work most days. He says the checking "has to be done or something terrible will happen." The MOST appropriate evidence-based treatment is:

A. Exposure and response prevention: gradually approaching the feared situation (leaving) without performing the ritual (checking)

B. CBT to identify and restructure the catastrophic belief about the stove, reducing the perceived need to check

C. Progressive muscle relaxation to reduce the anticipatory anxiety that triggers the checking before he leaves home

D. Mindfulness meditation to help him create distance from the intrusive thoughts and reduce his reactivity to the urge to check the stove

Question 68. A 19-year-old woman is referred by her dermatologist after presenting with wounds from picking at her skin for 2–3 hours daily. She describes difficulty stopping and feels distress after picking sessions. She has tried to stop independently multiple times. The counselor should:

A. Diagnose OCD and begin exposure and response prevention targeting the urge to pick

B. Diagnose Excoriation Disorder and use Habit Reversal Training as the primary treatment

C. Assess for self-harm and refer for psychiatric evaluation, as the wounds suggest intentional self-injury

D. Provide psychoeducation about the dermatological risks of picking and encourage her to use willpower to stop

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 22-year-old reports intrusive, unwanted thoughts that her hands are contaminated, causing intense anxiety that she neutralizes by washing for hours each day. She recognizes the fear is excessive but cannot stop. The most likely diagnosis is:

A. Generalized Anxiety Disorder (GAD)

B. Illness Anxiety Disorder

C. Specific Phobia

D. Obsessive-Compulsive Disorder

Answer: D. Obsessions (intrusive contamination thoughts) paired with time-consuming compulsions (washing) define OCD. GAD (A) is diffuse worry without ritualized compulsions. What would change the answer: preoccupation with having a serious illness and reassurance-seeking, without contamination rituals → Illness Anxiety Disorder.

Assessment — what to do next. The most appropriate symptom-severity measure is:

A. The Yale-Brown Obsessive Compulsive Scale (Y-BOCS)

B. The PHQ-9, to track the depressed mood that often accompanies OCD

C. The GAD-7, given her intense anxiety about contamination

D. The AUDIT, to check for alcohol use as a coping strategy

Answer: A. The Y-BOCS is the standard OCD severity measure and guides exposure and response prevention. The others target depression, anxiety, and alcohol use.

Treatment Decision Flow

OCD-spectrum items reward exposure-based methods and punish reassurance in all its disguises.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
OCD	Exposure and response prevention (ERP)	Reassurance, thought-stopping, or cognitive therapy without ERP
Hoarding disorder	Specialized CBT (motivation, sorting, decision-making skills), harm reduction	A forced cleanout
Body dysmorphic disorder	CBT with ERP-style exposure; address checking and avoidance	Referral for cosmetic procedures
Trichotillomania / excoriation	Habit reversal training	Insight-only exploration

Sequencing & caveats. ERP is first-line for OCD; habit reversal training is first-line for body-focused repetitive behaviors. Build the exposure hierarchy collaboratively. Avoid / contraindicated. Reassurance and thought-stopping feed the compulsive cycle; forced discarding fails and damages the alliance in hoarding. Scope. Where an SSRI (often at higher doses) is indicated, the counselor refers rather than prescribes.

Assessment Tools in Context

Yale-Brown Obsessive Compulsive Scale (Y-BOCS): The gold-standard clinician-administered measure of OCD severity (Goodman et al., 1989). Rates obsessions and compulsions separately on five dimensions (time occupied, distress, interference, resistance, control). Score 0–40; ≥16 indicates clinically significant OCD. The CY-BOCS is the child version. The counselor uses this to track treatment response, not to diagnose.

OCI-R (Obsessive-Compulsive Inventory–Revised): 18-item self-report screening tool for OCD symptoms across six subscales (washing, checking, ordering, obsessing, hoarding, neutralizing) (Foa et al., 2002). Score ≥21 suggests

clinically significant OCD. Faster to administer than Y-BOCS; useful in session-by-session tracking.

BDD-YBOCS / BDDQ (Body Dysmorphic Disorder): The BDDQ is a brief screen for BDD (Phillips et al., 1997). The BDD-YBOCS rates severity of preoccupation and compulsive behaviors specific to perceived physical defects. Useful when the clinician suspects BDD rather than OCD.

HRS (Hoarding Rating Scale): Brief interview assessing clutter, difficulty discarding, and distress/impairment (Tolin et al., 2010). Score ≥14 suggests clinically significant hoarding. The Clutter Image Rating (CIR) uses photographs to assess clutter levels objectively.

Exam tip: On the NCMHCE, the counselor's role is always to select the most appropriate clinical response, not the most thorough one. For OCD-spectrum disorders, "assess the nature, frequency, and content of obsessions and compulsions before initiating any intervention" is almost always a strong response when the disorder is not yet fully characterized.

Worked Mini-Case

Vignette: Amara, 29, presents reporting that she cannot leave the house without checking the locks, stove, and windows at least 12 times each. She knows the stove is off but experiences intense anxiety until she completes the checking routine. The process takes 45 minutes every morning and has caused her to be late to work repeatedly. She has tried to stop checking but finds the anxiety unbearable. She reports no depressive episodes and denies trauma history.

Question: In conducting an initial clinical interview, the counselor should FIRST:

A. Psychoeducate Amara about ERP and schedule weekly sessions

B. Refer Amara to a psychiatrist for medication evaluation before beginning counseling

C. Assess the nature, duration, triggers, and content of Amara’s obsessions and compulsions, and the degree of insight she has about them

D. Administer the Y-BOCS to establish a baseline severity score before any other intervention

Best answer: C

Reasoning chain: The NCMHCE rewards clinical sequence: assess before intervening. While Y-BOCS (D) is evidence-based, the exam sequence requires the counselor to conduct the clinical interview first — establishing the full picture of obsessions, compulsions, triggers, and insight. The clinical interview precedes a structured measure.

Why distractors lose: - A: ERP is the first-line treatment but cannot be initiated before thorough assessment - B: Medication (typically SSRIs) is appropriate adjunctive treatment but referral before assessment presumes a diagnosis and severity that hasn’t been established - D: The Y-BOCS is excellent but begins after the clinical interview establishes rapport and the presenting problem; jumping straight to measurement skips the relationship-building and clinical inquiry phase the NCMHCE consistently rewards

Answer Key — Chapter 16 Practice Questions

Q67 — Answer: A

Rationale: ERP is the gold-standard treatment for OCD (Foa et al., 2012). Cognitive restructuring (B) can be a helpful adjunct but is not sufficient alone. Relaxation (C) reduces anxiety without confronting the rituals — maintaining the disorder. Mindfulness (D) is an adjunct but not the primary treatment.

Q68 — Answer: B

Rationale: Excoriation (skin-picking) disorder is treated with HRT (habit reversal training) — awareness training, competing response training, and social support (Woods & Twohig, 2008). It is distinct from OCD (different disorder, different treatment). Self-harm (C) should be ruled out but the presentation is more consistent with excoriation. D is not evidence-based.

Chapter 17 — Schizophrenia Spectrum and Other Psychotic Disorders

Overview

Schizophrenia spectrum disorders are among the most severe on the NCMHCE, and the exam tests them differently from most other chapters — less about therapeutic interventions you would personally deliver, more about recognition, appropriate referral, and collaborative care. A key exam principle: individual psychotherapy is not the primary treatment for active psychosis. The counselor's role includes case management, psychoeducation (for client and family), and coordination with the psychiatric treatment team.

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Duration	Key Features
Schizophrenia	F20.9	6A20	≥6 months (active symptoms ≥1 month)	≥2 of: delusions, hallucinations, disorganized speech, disorganized behavior, negative symptoms
Schizophreniform Disorder	F20.81	6A20	1–6 months	Same symptoms; good premorbid functioning is a positive prognostic indicator

Diagnosis	ICD-10	ICD-11	Duration	Key Features
Brief Psychotic Disorder	F23	6A23	1 day – 1 month	Sudden onset, often following stressor; full recovery expected
Schizoaffective Disorder	F25.x	6A21	Duration variable	Meets criteria for schizophrenia + concurrent mood episode; mood episode present for majority of illness
Delusional Disorder	F22	6A24	≥1 month	Delusions (bizarre or non-bizarre; "with bizarre content" specifier); relatively intact functioning outside the delusion

ICD-11 notes: Schizophreniform Disorder: no separate category; ICD-11 schizophrenia requires only 1 month of symptoms. Brief Psychotic Disorder: nearest is acute and transient psychotic disorder, which lasts up to 3 months.

The Clinical Picture

Positive symptoms (things added to experience): hallucinations (hearing, seeing, feeling, smelling things not there), delusions (fixed false beliefs not culturally explained), disorganized speech (loosening of associations, word salad), and disorganized behavior.

Negative symptoms (things removed from experience): flat affect, alogia (poverty of speech), avolition (lack of motivation), anhedonia, social withdrawal. Negative

symptoms are harder to treat and more predictive of long-term functional impairment.

In a case simulation, recognize psychosis by: the client reports experiences no one else can verify (hearing voices, being watched by the government), the thinking pattern is tangential or incoherent, the client's behavior is bizarre or self-neglecting, and the client shows flat or inappropriate affect.

The counselor's role when a client presents with active psychosis: do not engage with the content of delusions. Neither agree ("Yes, the government may be watching you") nor argue ("That's not real"). Maintain calm, focused, reality-grounded interaction. Refer for psychiatric evaluation and medication management. The antipsychotic medications are the primary treatment.

What the Exam Rewards / What It Punishes

Rewards:

- Immediate psychiatric referral for active psychosis
- Family psychoeducation (Family-focused therapy / NAMI involvement) to reduce expressed emotion (EE), which predicts relapse
- Supported employment and cognitive remediation in the recovery/maintenance phase
- Recognizing negative symptoms as distinct from depression

Punishes:

- Attempting insight-oriented individual therapy during active psychosis
- Arguing with delusional content
- Prescribing or managing antipsychotics (outside the counselor's scope)
- Missing the medical rule-out (neurological causes, substance-induced psychosis)

Practice Questions

Question 69. A client who has been stable on antipsychotic medication tells her counselor she has stopped taking her medication because “the pills are part of the control system.” The counselor’s MOST appropriate response is:

A. Explore the client’s concerns about the medication in a nonconfrontational manner and contact the prescribing psychiatrist

B. Reassure her that the medication is safe, has been prescribed for her benefit, and will not harm her

C. Discharge the client, because she is no longer treatment-compliant and outpatient care cannot proceed safely

D. Arrange immediate hospitalization for the client due to the high likelihood of psychotic relapse after abruptly discontinuing antipsychotics

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 20-year-old has had three months of delusions, auditory hallucinations, disorganized speech, and a decline in social and occupational functioning, without a sustained mood episode. The most likely diagnosis (assuming substance and medical causes are ruled out) is:

A. Brief Psychotic Disorder

B. Schizophrenia

C. Schizophreniform Disorder

D. Schizoaffective Disorder, bipolar type

Answer: C. Active psychotic symptoms lasting one to six months define Schizophreniform Disorder. Under one month → Brief Psychotic Disorder (A); six months or more → Schizophrenia (B). What would change the answer: duration of six months or more → Schizophrenia; a mood episode present for the majority of the illness → Schizoaffective Disorder.

Assessment — what to do next. The most appropriate assessment step is:

A. Rule out substance-induced and medical causes (history and labs via referral) and establish symptom duration

B. Begin insight-oriented therapy to help the client explore the personal meaning behind the delusions and voices

C. Administer the MMSE to screen for cognitive impairment underlying the disorganized speech

D. Challenge the delusions directly to test how firmly the client holds them and gauge insight

Answer: A. Symptom duration plus substance/medical rule-out define the diagnosis within the psychotic spectrum. Therapy (B) is treatment; directly challenging delusions (D) is contraindicated.

Treatment Decision Flow

Psychosis items reward getting medication in place first and never arguing with a delusion.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Acute psychosis	Ensure safety; refer for antipsychotic medication management (the primary intervention)	Insight-oriented therapy, exposure, or directly challenging the delusion
Stabilized	CBT for psychosis (Morrison, 2017), psychoeducation, family psychoeducation, supported employment	Discontinuing medication or offering unstructured insight therapy
First-episode psychosis	Refer to coordinated specialty care (Kane et al., 2016)	Delaying treatment

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Voices persist despite adherence	CBT for psychosis (CBTp) coping strategies to change the relationship with the voices	Framing residual symptoms as treatment failure

Sequencing & caveats. Medication is the primary treatment; psychotherapy is adjunctive. Family psychoeducation reduces relapse and expressed emotion. Avoid / contraindicated. Do not argue with or directly challenge delusions; avoid exposure; do not support medication discontinuation. Scope. Refer for antipsychotic management and coordinate care.

Differentiating Within the Family

Schizophrenia vs. Schizoaffective Disorder: The key distinguishing feature is the timing of mood symptoms relative to psychotic symptoms. In Schizoaffective Disorder, a major mood episode (depressive or manic) is present for a majority of the total duration of the illness, AND there is a period of at least 2 weeks during which psychotic symptoms are present without mood symptoms. In Schizophrenia, any mood symptoms are brief relative to the duration of psychotic symptoms.

Schizophrenia vs. Brief Psychotic Disorder vs. Schizophreniform Disorder: Duration is the deciding factor. Brief Psychotic Disorder: 1 day to 1 month, then full return to premorbid functioning. Schizophreniform Disorder: 1–6 months (symptoms that don't yet meet Schizophrenia duration). Schizophrenia: 6+ months (prodromal, active, and residual symptoms combined). The NCMHCE often tests this timeline.

Schizophrenia vs. Delusional Disorder: Delusional Disorder typically involves plausible delusions (bizarre content is allowed as a specifier) (being followed, having an illness, a spouse being unfaithful) without the hallucina-

tions, disorganized speech, or negative symptoms that characterize Schizophrenia. Functioning outside the delusional domain is relatively preserved.

Schizophrenia vs. Substance-Induced Psychotic Disorder: Timing is critical. Substance-induced psychosis emerges during intoxication or withdrawal and typically resolves within days to a month of abstinence. If psychotic symptoms persist more than 1 month beyond cessation of substance use, Schizophrenia or another primary psychotic disorder becomes the appropriate consideration. Always assess for substance use with a thorough history and drug screen.

Schizophrenia vs. Major Depressive Disorder with Psychotic Features: In MDD with Psychotic Features, hallucinations and delusions occur exclusively during depressive episodes. The mood disturbance is primary; psychosis is secondary. If psychotic symptoms occur independently of mood episodes, a primary psychotic disorder is more likely.

Schizophrenia vs. Bipolar I with Psychotic Features: Same logic — psychosis in Bipolar I occurs exclusively during manic or major depressive episodes. If psychotic symptoms occur outside of mood episodes, consider Schizophrenia or Schizoaffective Disorder.

Assessment Tools in Context

Positive and Negative Syndrome Scale (PANSS): 30-item clinician-rated scale measuring positive symptoms (hallucinations, delusions, thought disorder), negative symptoms (blunted affect, alogia, avolition), and general psychopathology (Kay et al., 1987). Used to track medication response in research and clinical settings. Not administered by counselors routinely, but the NCMHCE may reference it in context.

Brief Psychiatric Rating Scale (BPRS): An 18-item clinician-rated scale covering a range of psychotic, affective, and behavioral symptoms (Overall & Gorham, 1962).

Used widely in inpatient and community mental health settings to monitor symptom change. More accessible for counselors than PANSS.

Calgary Depression Scale for Schizophrenia (CDSS): The CDSS distinguishes depression in schizophrenia from negative symptoms (both may look like low motivation and flat affect) (Addington et al., 1993). Depression treatment improves quality of life and reduces suicide risk in clients with Schizophrenia.

Columbia Suicide Severity Rating Scale (C-SSRS): Clients with Schizophrenia have 10× higher suicide risk than the general population. The C-SSRS (Posner et al., 2011) is essential for ongoing risk monitoring. Command hallucinations, depression, and substance use increase risk substantially.

Medication Adherence: Non-adherence to antipsychotic medication is the most common cause of relapse. The NCMHCE may test the counselor's role in psychoeducation around adherence, identifying barriers, and coordinating with prescribers — never adjusting or recommending changes to medication directly.

Exam tip: The counselor works within a collaborative care model with clients with Schizophrenia. The appropriate response to psychotic exacerbation is always to coordinate with the treatment team and assess for safety, not to manage psychosis independently.

Worked Mini-Case

Vignette: Victor, 25, was hospitalized six months ago following a first psychotic break in which he believed coworkers were poisoning his food and heard voices telling him he was "chosen." He was discharged on antipsychotics with outpatient follow-up. He presents to counseling reporting he stopped his medication two weeks ago because "I feel fine and the pills make me feel like a zombie." He denies current hallucinations but his affect is blunted and he is vague about his recent activities.

Question: The counselor’s MOST appropriate immediate response is:

A. Validate Victor’s autonomy in managing his own treatment and explore his feelings about the medication

B. Assess current safety and symptom status, psychoeducate about medication discontinuation risk, and contact the prescribing provider with Victor’s knowledge

C. Refer Victor back to inpatient hospitalization immediately given his medication noncompliance

D. Adjust the treatment plan to focus on coping skills so Victor can manage without medication if he chooses

Best answer: B

Reasoning chain: The highest-priority action is safety and clinical stability. Medication discontinuation within 6 months of a first psychotic episode is a significant relapse risk. The counselor assesses current symptom status and safety, provides psychoeducation (framing this as collaborative care, not coercion), and loops in the prescriber. This reflects collaborative care within scope of practice.

Why distractors lose: - A: Validating autonomy is important but cannot come before safety assessment; at this clinical juncture, autonomy validation without clinical action is insufficient - C: He is not currently in crisis — inpatient referral is premature and violates least-restrictive-environment principles - D: Counselors do not recommend clients manage Schizophrenia without medication; this option is outside scope and clinically contraindicated

Answer Key — Chapter 17 Practice Questions

Q69 — Answer: A

Rationale: The counselor explores the concern (does not argue with the delusion) and coordinates with the prescriber. This is the collaborative care model. B argues against the delusional belief. C abandons the client at a critical moment. D may eventually be appropriate but is not the immediate first step.

Chapter 18 — Feeding and Eating Disorders

Overview

Anorexia nervosa carries one of the highest mortality rates of any psychiatric disorder — a statistic the NCMHCE exam knows and tests. The exam expects you to: assess medical stability first, know the diagnostic criteria, apply the appropriate level of care, and understand the multi-team nature of eating disorder treatment.

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Core Feature	Medical Concern
Anorexia Nervosa — Restricting	F50.01x	6B80.00 / 6B80.10	Restriction, low weight, body image disturbance, fear of gaining	Cardiac arrhythmia, bone loss, lanugo, bradycardia — HIGH mortality
Anorexia Nervosa — Binge-Eating/ Purging	F50.02x	6B80.01 / 6B80.11	Restriction + binge/purge cycles, low weight	Same as above
Bulimia Nervosa	F50.2x	6B81	Binge-purge cycles, normal or above-average weight, shame cycle	Electrolyte imbalance, enamel erosion, Russell's sign

Diagnosis	ICD-10	ICD-11	Core Feature	Medical Concern
Binge-Eating Disorder	F50.81x	6B82	Recurrent binge episodes without compensatory behaviors; significant distress	Obesity-related metabolic effects
Avoidant/ Restrictive Food Intake Disorder (ARFID)	F50.82	6B83	Restriction not driven by body image; sensory, fear of aversive consequences, or low interest	Nutritional deficiency, growth failure, dependence on supplements
Pica	F98.3	6B84	Eating non-nutritive substances	Lead poisoning, GI obstruction
Rumination Disorder	F98.21	6B85	Repeated regurgitation of food	Nutritional, dental, social

ICD-11 notes: Anorexia Nervosa — Restricting, Anorexia Nervosa — Binge-Eating/Purging: split by body weight: .0x significantly low, .1x dangerously low.

Since October 2024, ICD-10-CM codes for anorexia nervosa, bulimia nervosa, and binge-eating disorder add a final digit for severity or remission (x).

The Clinical Picture

Anorexia Nervosa (AN): The client presents as underweight but may not perceive themselves as thin — body image disturbance is a core feature. They may be brought in by family (the client often does not self-refer because the disorder is ego-syntonic). They may minimize the problem (“I eat fine”) or be engaged but hide restriction severity. Physical signs: lanugo (fine hair on face/body),

bradycardia, cold intolerance, amenorrhea, dental erosion (in binge/purge type).

Bulimia Nervosa (BN): The binge-purge cycle is driven by shame. The client often eats normally in public and purges privately. They are more likely than AN clients to self-refer because the disorder is ego-dystonic — the binges and purges feel out of control and shameful. Russell's sign (calluses on knuckles from self-induced vomiting) is a physical marker.

The Refeeding Syndrome: A potentially fatal electrolyte shift that can occur when malnourished clients (particularly AN) begin eating again too quickly. It is a medical emergency. Medical monitoring is essential during refeeding.

What the Exam Rewards / What It Punishes

Rewards:

- Medical evaluation FIRST for any suspected eating disorder (cardiac monitoring, electrolytes, bone density)
- Knowing that higher levels of care (residential, partial hospital) are required for medically unstable clients
- Multidisciplinary team approach (counselor + dietitian + physician + psychiatrist)
- Family-Based Treatment (FBT/Maudsley approach; Lock & Le Grange, 2013) as first-line for adolescents with AN
- CBT-E (enhanced; Fairburn, 2008) as evidence-based treatment for BN and BED

Punishes:

- Attempting outpatient therapy with a medically unstable client with AN
- Focusing only on weight restoration without addressing the psychological maintaining factors
- Colluding with the client's body image distortion (e.g., agreeing that they "look a little heavy")

- Missing that purging in BN can cause fatal electrolyte imbalances (hypokalemia → cardiac arrhythmia)

Worked Mini-Case

Vignette: Sophia, 16, is brought in by her parents. She has lost 22 pounds over the past 8 months. Her parents became alarmed when her pulse at a school screening was 48 bpm. She says she is "eating fine" and doesn't understand why everyone is worried. She denies purging. Her BMI is 16.2.

Reasoning chain:

Step 1 — Medical emergency: Bradycardia (48 bpm) in an underweight adolescent with significant weight loss is a cardiac warning sign. Before any psychotherapy, medical evaluation is needed. Sophia may need hospitalization for medical stabilization.

Step 2 — Level of care: Based on eating-disorder level-of-care criteria (medical stability, eating disorder behaviors, weight status, psychiatric comorbidity), Sophia may need inpatient medical stabilization or residential care before outpatient counseling (APA, 2023).

Step 3 — Denial: Her insistence that she is "eating fine" reflects the ego-syntonic quality and body image distortion of AN. The counselor does not argue but maintains the medical priority.

Step 4 — Treatment once medically stable: Family-Based Treatment (FBT/Maudsley) is the first-line treatment for adolescents with AN — parents are empowered to temporarily take control of eating while the adolescent's brain is malnourished and unable to make rational food decisions.

Practice Questions

Question 70. A client with bulimia nervosa is engaging in CBT. She reports that last week she binged and purged

five times — up from two the previous week. She attributes the increase to exam stress. The counselor should:

A. Review the binge-purge logs with her and increase CBT session frequency during the exam period

B. Evaluate electrolyte levels and physical status, then address the stressor in treatment

C. Add a relaxation component to the CBT protocol to address the exam-related stress driving the increase

D. Maintain the current treatment without change, as fluctuations are expected

Question 71. Which of the following presentations warrants IMMEDIATE medical evaluation or hospitalization rather than outpatient counseling?

A. A client with bulimia nervosa who has been purging 3 times per week for 6 months, maintains normal BMI, and is motivated for treatment

B. A client with binge eating disorder who has gained 30 pounds and has comorbid depression

C. A client with ARFID who limits food textures but maintains adequate nutrition and growth

D. A client with anorexia nervosa presenting with BMI of 14.5, heart rate of 44 bpm, and reports of fainting

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 19-year-old restricts her intake, has a BMI of 16, reports intense fear of weight gain, and perceives herself as overweight despite being underweight. She does not binge or purge. The most likely diagnosis is:

A. Bulimia Nervosa

B. Anorexia Nervosa, restricting type

C. Avoidant/Restrictive Food Intake Disorder

D. Binge-Eating Disorder

Answer: B. Restriction leading to significantly low weight, intense fear of weight gain, and body-image disturbance define anorexia, restricting type. ARFID (C) lacks the weight/shape concern; bulimia (A) and BED (D) involve binge episodes. What would change the answer: normal weight with binge–purge cycles → Bulimia; food avoidance without body-image disturbance → ARFID.

Assessment — what to do next. Given the BMI of 16, the most appropriate first assessment step is:

A. Administer the EAT-26 and begin outpatient CBT-E to address her restriction and body-image concerns

B. Assess medical stability first (vitals, orthostatics, electrolytes, ECG) via medical referral before psychosocial treatment

C. Begin family-based treatment immediately, with her parents taking charge of meals and supervising weight restoration at home

D. Administer the PHQ-9 to screen for co-occurring depression, which commonly accompanies anorexia

Answer: B. At a low BMI, medical stability is assessed first, and psychosocial treatment pauses if the client is unstable. Self-report screening and therapy (A, C) follow medical clearance.

Treatment Decision Flow

Eating-disorder items are decided by medical stability first, then the stage-appropriate modality.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Anorexia with BMI < 15 (adults) or < 75% of median BMI (adolescents), HR < 50, or orthostatic hypotension	Pause psychosocial treatment; facilitate immediate medical stabilization/ hospitalization	Beginning outpatient CBT-E or FBT
Adolescent anorexia, medically stable	Family-based treatment (FBT)	Individual insight-oriented therapy

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Bulimia nervosa, medically stable	CBT-E	Nutrition education alone
Binge-eating disorder	CBT-E or IPT	Weight-loss dieting as the primary intervention

Sequencing & caveats. When the client is medically unstable, medical stabilization precedes all psychosocial treatment. FBT is first-line for medically stable adolescents with anorexia. Watch for refeeding syndrome during renutrition. Avoid / contraindicated. FBT is contraindicated where there is active parental abuse or the parents cannot supervise meals. Do not begin outpatient therapy while the client is medically unstable. Scope. Care is multidisciplinary (physician, dietitian, therapist); the counselor coordinates and refers for medical management.

Differentiating Within the Family

Anorexia Nervosa (AN) vs. Bulimia Nervosa (BN): The decisive criterion is weight. AN requires significantly low body weight for age, sex, and development. BN occurs at normal or above-normal weight. Both may involve binge-purge cycles, but the binge-purge type of AN differs from BN in that weight remains significantly low in AN. On the NCMHCE, a vignette describing binge-purge with normal or elevated weight signals BN; significantly underweight with restriction or purging signals AN.

Anorexia Nervosa vs. ARFID (Avoidant/Restrictive Food Intake Disorder): ARFID also involves restricted intake with low weight, but the motivation differs critically. AN is driven by distorted body image, intense fear of weight gain, and identity tied to thinness. ARFID is driven by sensory sensitivity, fear of choking/vomiting, or low appetite — not by body image or weight concerns. ARFID is more common in children and in individuals with autism spectrum disorder.

Bulimia Nervosa vs. Binge-Eating Disorder (BED): Both involve recurrent binge episodes with loss of control. The decisive difference is compensatory behavior. BN involves recurrent compensatory behaviors (purging, excessive exercise, fasting) following binges. BED involves no regular compensatory behaviors. Both cause significant distress.

Bulimia Nervosa vs. Purging Disorder: Purging Disorder involves purging behaviors in the absence of binge eating. This distinction is important for treatment planning, though it currently sits in the "other specified" category in DSM-5-TR (APA, 2022).

Anorexia/Bulimia vs. Body Dysmorphic Disorder: BDD involves preoccupation with perceived defects in appearance not limited to weight and shape. AN/BN body-image distortion is specifically about weight, body fat, and body shape. If a client's preoccupation involves features unrelated to weight (nose, skin, jaw), BDD is the more appropriate consideration.

Medical Monitoring Note: Anorexia nervosa carries one of the highest mortality rates of any psychiatric disorder. Medical complications — electrolyte imbalances, cardiac arrhythmias, bone density loss — require ongoing coordination with medical providers. The counselor's role includes monitoring for medical instability and making timely referrals.

Assessment Tools in Context

Eating Disorder Examination (EDE/EDE-Q): The EDE is a clinician-administered semistructured interview considered the gold standard for assessing eating disorder psychopathology (Fairburn, 2008). The EDE-Q is the self-report questionnaire version (Fairburn & Beglin, 2008). Both assess four domains: dietary restraint, eating concern, weight concern, and shape concern. Used for diagnosis, severity rating, and treatment planning.

SCOFF Questionnaire: 5-item rapid screen for eating disorders, validated for primary care and counseling intake

(Morgan et al., 1999). Scores ≥2 indicate probable AN or BN. Questions cover self-induced vomiting, loss of control over eating, recent weight loss of more than one stone (about 14 lb) in 3 months, feeling fat when others say one is thin, and whether food dominates the client's life.

Eating Attitudes Test (EAT-26): 26-item self-report screening tool for eating disorder attitudes and behaviors (Garner et al., 1982). Score ≥20 indicates need for further evaluation. Not diagnostic, but effective as a community and school-based screen.

EDI-3 (Eating Disorder Inventory–3): Comprehensive 91-item self-report measuring psychological characteristics associated with eating disorders across 12 subscales (including drive for thinness, body dissatisfaction, and interpersonal insecurity) (Garner, 2004). Used for case conceptualization rather than screening.

Exam tip: The NCMHCE tests counselors' knowledge of mandatory coordination with medical providers for eating disorders. A vignette describing a significantly underweight client should prompt the counselor to assess physical safety indicators (pulse, skin turgor, energy) and refer for medical evaluation — not to begin CBT-E without clearance.

Answer Key — Chapter 18 Practice Questions

Q70 — Answer: B

Rationale: Medical safety is always rung 2. An increase in purging frequency increases the risk of hypokalemia (from vomiting or laxative use) — electrolyte evaluation is warranted. The stressor can then be integrated into CBT work. A skips the medical check. C and D are insufficient responses to an acute escalation.

Q71 — Answer: D

Rationale: BMI 14.5 (severely low), bradycardia (44 bpm), and syncopal episodes indicate medical instability. This client requires immediate medical evaluation and likely inpatient or residential medical level of care before any outpatient therapy begins. A, B, and C can begin in outpatient settings with appropriate monitoring.

Chapter 19 — Crisis, Risk, and Safety

Overview

This chapter is non-negotiable. Every NCMHCE candidate must have a thorough, fluent understanding of risk assessment and crisis response — because safety (rung 1) applies to every single case simulation that touches on suicidality, homicidality, abuse, or danger. No clinical action precedes safety assessment when risk is present. The exam tests two specific things about suicidality: (1) your ability to conduct a thorough, direct, and compassionate risk assessment, and (2) your ability to match the appropriate level of care to the assessed risk level.

The Columbia Suicide Severity Rating Scale (C-SSRS) Framework

The C-SSRS is the most widely used structured risk assessment tool and the framework the exam implicitly uses (Posner et al., 2011). It assesses:

Ideation:

- Wish to be dead (passive)
- Active suicidal ideation — nonspecific (without plan or intent)
- Active suicidal ideation with any methods (not plan) without intent to act
- Active suicidal ideation with some intent to act, without specific plan
- Active suicidal ideation with specific plan and intent

Behavior (recent and lifetime):

- Aborted attempt
- Interrupted attempt

- Actual attempt
- Preparatory acts or behavior
- Non-suicidal self-injury (important to distinguish)

The SAL Framework for Risk Assessment

The SAL framework organizes a suicide risk assessment around three domains: **S**pecificity of plan, **A**vailability of means, and **L**ethality of method.

S — Specificity of plan: How specific is the plan (method, time, place)? Presence, frequency, intensity, duration of ideation. Active vs. passive.

A — Availability of means: Does the client have a gun at home? Pills available? Access to lethal means dramatically increases risk.

Means restriction counseling — reducing access to the most lethal means — is a primary safety intervention with high evidence.

L — Lethality of method: A plan involving firearms is higher risk than a plan involving pills. A specific plan with a time, place, and method is higher risk than vague ideation.

Additional risk factors:

- Prior attempt (strongest single predictor of future attempt)
- Current substance intoxication
- Recent significant loss
- Social isolation / lack of support
- History of impulsivity
- Male sex (higher completion rate); female sex (higher attempt rate)
- Access to means

Protective factors:

- Reasons for living (children, religious beliefs, future plans)
- Social support

- Help-seeking behavior (client is in your office)
- Engaged in treatment

Level of Care Matching

Risk Level	Features	Appropriate Action
Low	Passive ideation, no plan, protective factors present, no history of attempts	Safety plan, increase session frequency, remove means
Moderate	Active ideation, vague plan, some protective factors, prior attempt history	Safety plan, family/ support involvement, possible voluntary hospitalization; possibly intensive outpatient
High	Active ideation with specific plan, intent, means available, prior attempt, intoxicated	Emergency psychiatric evaluation; involuntary hold if imminent danger (procedures and terms vary by state)
Imminent	States intent to act immediately; has means	Call 911 / contact emergency services; do not leave client alone

The Safety Plan

A safety plan (not a “no-harm contract” — the research does not support no-harm contracts as effective) includes (Stanley & Brown, 2012):

1. Warning signs (what the client notices before crisis escalates)
2. Internal coping strategies (what the client can do alone)
3. People and social settings that provide distraction (friends, family, places — they don’t need to know the reason)
4. People the client can ask for help
5. Professionals and agencies to contact in a crisis (with numbers, including 988)

6. Making the environment safe (restricting means)

Many counselors also record the client's reasons for living.

Mandated Reporting: Child Abuse and Neglect

All mental health counselors are mandated reporters. When there is reasonable suspicion (not certainty) of child abuse or neglect, the report must be made to Child Protective Services. The counselor does not investigate; the counselor reports.

Four types of child maltreatment:

- Physical abuse
- Sexual abuse
- Emotional/psychological abuse
- Neglect (by far the most common)

Duty to Warn (Tarasoff): When a client makes a specific, credible threat against an identifiable third party, the counselor has a duty to protect that third party (*Tarasoff v. Regents of the University of California*, 1976). Actions may include: warning the intended victim, notifying law enforcement, or arranging for involuntary hospitalization. The specific obligations vary by state. The exam tests the principle — specific, identifiable threat, identifiable victim — not state-specific law.

What the Exam Rewards / What It Punishes

Rewards:

- Direct, compassionate, nonjudgmental suicidal ideation assessment ("Are you having thoughts of suicide?")
- Using the safety plan over the no-harm contract
- Means restriction counseling
- Matching level of care to risk (not automatically hospitalizing every suicidal client, but not underresponding to high-risk presentations)
- Reporting suspected abuse when there is reasonable suspicion — not waiting for certainty

Punishes:

- Avoiding direct inquiry about suicidal ideation (“I didn’t want to put the idea in their head” — this is a myth)
- Assuming chronic suicidality in BPD doesn’t need ongoing assessment
- Failing to report suspected abuse due to fear of disrupting the therapeutic alliance
- Using a no-harm contract as a substitute for a thorough risk assessment
- Discharging a high-risk client without a safety plan

Worked Mini-Case

Vignette: A client calls between sessions and says, “I’ve been thinking about taking all my sleeping pills tonight. I have them right here. I just don’t see a way out.”

Reasoning chain:

This is a high-to-imminent risk presentation:

- Active ideation ✓
- Specific plan (sleeping pills) ✓
- Access to means (has them available) ✓
- Intent (tonight) ✓
- Hopelessness (“I don’t see a way out”) ✓

Response sequence:

1. Stay on the line; do not hang up.
2. Ask her to move away from the pills while speaking with you.
3. Assess whether she has taken anything already (medical emergency if yes → 911).
4. Engage her around reasons for living; explore what led to the escalation tonight.
5. Arrange for emergency evaluation: either the client agrees to go to the ER (and a support person accompanies her), or if she refuses and the risk is

imminent, call 911 with her on the phone or with knowledge of her address.

6. Do not end the call until a safety plan is in place or emergency services have been contacted.

Practice Questions

Question 72. A client discloses active suicidal ideation with a specific plan to hang himself using a rope he has already purchased. He denies intent to act “tonight” but says “soon.” He lives alone. The counselor should:

A. Create a collaborative safety plan with him and schedule a follow-up session for tomorrow morning

B. Conduct a full risk assessment, remove the means if possible, and arrange emergency psychiatric evaluation

C. Contact his family to arrange for someone to stay with him tonight and follow up at the next session

D. Administer the PHQ-9 to quantify his depression severity and document the disclosure thoroughly in the chart

Question 73. A child client reports that her uncle “touches her in a place I don’t like” when she stays at his home on weekends. The counselor should:

A. Explore the disclosure further with the child to determine whether abuse occurred before reporting

B. Contact the client’s parents to inform them of the disclosure and let them handle the situation within the family

C. Report the disclosure to Child Protective Services immediately based on reasonable suspicion

D. Consult with a clinical supervisor to confirm whether the reporting threshold is met before making any report

Question 74. Which of the following is the MOST significant single risk factor for a future suicide attempt?

A. Diagnosis of major depressive disorder

B. History of a prior suicide attempt

C. Prolonged and persistent social isolation

D. Access to firearms

Question 75. A client with chronic BPD and a history of repeated suicidal gestures mentions, during a routine session, “I’ve been thinking about suicide again — like always.” The counselor should:

A. Conduct a current risk assessment, update the safety plan, and document

B. Acknowledge the statement and continue the session, as chronic suicidality is expected in BPD

C. Refer to the crisis line, because it is designed to handle this type of ongoing ideation

D. Contact a supervisor to discuss whether to hospitalize the client before ending the session

Answer Key — Chapter 19 Practice Questions

Q72 — Answer: B

Rationale: This is high-risk: specific plan, purchased means, lives alone, intent expressed as "soon." Emergency psychiatric evaluation is required. A safety plan alone is insufficient at this risk level. Family contact (C) may be part of the plan but not the primary action. Documentation (D) is necessary but not a safety intervention.

Q73 — Answer: C

Rationale: Mandated reporting requires reasonable suspicion, not certainty. The counselor does not investigate. A delays a mandatory report. B inappropriately transfers the legal obligation to the parents (and may alert the alleged abuser). D may be appropriate as a simultaneous step, but does not delay the report.

Q74 — Answer: B

Rationale: Prior attempt is the strongest single predictor of a future attempt. All others (A, C, D) are significant risk factors but are less predictive than prior attempt history.

Q75 — Answer: A

Rationale: Chronic suicidality in BPD does not excuse the counselor from assessing each expression. Each report requires assessment of current ideation (is this different from baseline?), means, intent, and plan. Ignoring it (B) is a clinical and ethical failure. C deflects responsibility. D may be appropriate but is not the first step — assess first.

Chapter 20 — Disruptive, Impulse-Control, and Conduct Disorders

Overview

This family groups disorders characterized by problems with self-regulation of emotions and behavior that violate the rights of others or bring the person into conflict with social norms. The exam primarily tests this in the context of youth populations and asks you to distinguish between childhood behavioral presentations.

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Key Feature	Age/ Context
Oppositional Defiant Disorder (ODD)	F91.3	6C90	Angry/irritable mood + argumentative/ defiant behavior + vindictiveness ≥6 months	Children/ adolescents; directed at authority figures
Conduct Disorder	F91.x	6C91	Serious violation of others' rights and societal norms (aggression, destruction, deceit, rule violations)	Children/ adolescents; precursor to ASPD
Intermittent Explosive Disorder	F63.81	6C73	Recurrent, impulsive aggressive outbursts disproportionate to provocation	Adolescents and adults

Diagnosis	ICD-10	ICD-11	Key Feature	Age/ Context
Antisocial PD	F60.2	6D10 + 6D11.2	Adult pattern of rights violations (requires CD before 15)	Adults ≥18
Pyromania	F63.1	6C70	Deliberate fire-setting with fascination and tension-relief pattern	Rare
Kleptomania	F63.2	6C71	Recurrent stealing not for gain; tension-relief pattern	Rare

ICD-11 notes: Conduct Disorder: renamed conduct-dissocial disorder; 6C91.0 childhood onset, 6C91.1 adolescent onset. Antisocial PD: ICD-11 has no specific personality disorder types; code 6D10 personality disorder plus the closest trait-domain qualifier (shown after +).

The Clinical Picture

ODD vs. CD: The critical distinction is severity and type of rule violation. ODD involves defiance, arguing, and irritability — annoying but not seriously harmful. CD involves aggression toward people or animals, destruction of property, deceit/theft, and serious rule violations. A child who argues with teachers and refuses chores has ODD; a child who fights, breaks into homes, and tortures animals has CD.

The developmental pathway: ODD → CD → ASPD is a known risk trajectory, especially with early-onset CD and lack of early intervention. Not all ODD children develop CD, and not all CD adolescents develop ASPD — but the association is clinically important and the exam tests it.

Parenting components: Evidence-based treatment for ODD and CD includes parent management training (PMT; Kazdin, 2005). Parents are taught to reinforce prosocial behavior consistently, set clear and enforceable limits,

use natural consequences, and reduce inadvertent reinforcement of problem behavior.

What the Exam Rewards / What It Punishes

Rewards:

- Parent management training as the core treatment for ODD/CD in children
- Functional behavioral assessment to understand the ABCs of the behavior (antecedent → behavior → consequence)
- Multisystemic therapy (MST) for severe adolescent CD (targets family, peer, school, community systems simultaneously)
- Assessing for co-occurring ADHD, trauma, or mood disorders (ODD rarely appears alone)

Punishes:

- Individual insight-oriented therapy as the sole treatment for ODD/CD (insufficient without systemic components)
- Diagnosing CD or ASPD in young children (ASPD requires ≥18 and prior CD)
- Failing to assess the family system's contribution to the behavioral pattern

Practice Questions

Question 76. A 9-year-old boy is referred for aggressive behavior. He has been suspended three times this year for fighting, was caught setting a small fire in the schoolyard, and his parents report he has stolen money from them multiple times. These behaviors have persisted for over a year. The MOST appropriate diagnosis to consider is:

A. ADHD — predominantly hyperactive/impulsive

B. Oppositional Defiant Disorder (ODD)

C. Conduct Disorder

D. Intermittent Explosive Disorder

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 15-year-old has a 14-month pattern of physical fights, bullying, shoplifting, staying out past curfew, and cruelty to a neighbor's pet, with onset at age 13. The most likely diagnosis is:

A. Oppositional Defiant Disorder

B. Attention-Deficit/Hyperactivity Disorder

C. Intermittent Explosive Disorder

D. Conduct Disorder, adolescent-onset

Answer: D. Aggression toward people and animals, theft, and serious rule violations define Conduct Disorder; onset at or after age 10 is the adolescent-onset type, which carries a better prognosis. ODD (A) is defiance and anger without rights-violating aggression or theft; IED (C) is impulsive aggression alone. What would change the answer: onset before age 10 → childhood-onset type.

Assessment — what to do next. The most appropriate assessment approach is:

A. Obtain multi-informant history (parents, school) and assess family and peer context and safety

B. Diagnose from the adolescent's self-report alone, given that he is old enough to describe his own behavior

C. Administer the MoCA to screen for cognitive deficits underlying the impulsive behavior

D. Recommend a boot-camp evaluation to determine whether a structured program could curb the behavior

Answer: A. Conduct Disorder assessment requires multi-informant data and attention to family, peer, and safety context. Self-report alone (B) is insufficient; boot camp (D) is a harmful intervention, not an assessment.

Treatment Decision Flow

Conduct and impulse-control items reward family/systemic treatment and punish anything that aggregates antisocial youth.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Adolescent conduct disorder	Evidence-based family treatment (MST or functional family therapy [FFT]); parent management training	Individual insight therapy, or "scared straight"/boot-camp programs
Oppositional defiant disorder (child)	Parent management training	Child-only individual therapy
Intermittent explosive disorder	CBT for arousal/anger regulation and skills	Cathartic "venting" approaches
Deviant peer affiliation	Build prosocial connections and activities	Group therapy that aggregates antisocial youth

Sequencing & caveats. MST and FFT have the strongest evidence for adolescent conduct disorder; parental supervision and consistency are key, modifiable targets. Avoid / contraindicated. Scared-straight and boot-camp programs can worsen outcomes; grouping antisocial youth together is iatrogenic. Scope. Standard counselor scope; coordinate school and family systems.

Differentiating Within the Family

Oppositional Defiant Disorder (ODD) vs. Conduct Disorder (CD): ODD involves persistent angry/irritable mood, argumentative/defiant behavior, and vindictiveness toward authority figures but does not include violations of others' basic rights or serious societal norms. CD requires a pattern of aggression toward people or animals, destruction of property, deceitfulness/theft, or serious rule violations. CD represents a more severe profile and can co-occur with or evolve from ODD.

Conduct Disorder vs. Antisocial Personality Disorder (ASPD): ASPD requires evidence of CD before age 15 and is diagnosed only in adults 18+. CD is the childhood/adolescent diagnosis; ASPD is the adult continuance. The NCMHCE may test this diagnostic bridge.

ODD vs. ADHD: Impulsivity and hyperactivity in ADHD can produce defiant-appearing behavior, but the defiance in ADHD is not intentionally provocative — it emerges from difficulty regulating attention and impulse. In ODD, the defiance is deliberate and targeted toward authority. Both can co-occur; when they do, both diagnoses are warranted.

ODD vs. Disruptive Mood Dysregulation Disorder (DMDD): DMDD involves chronic, severe irritability with temper outbursts that are grossly out of proportion to the situation, occurring 3+ times per week, present in two or more settings, for 12+ months, with onset before age 10. If criteria for both are met, only DMDD is diagnosed — DSM-5-TR does not permit a separate ODD diagnosis alongside DMDD. DMDD was introduced to reduce overdiagnosis of Bipolar Disorder in children.

Intermittent Explosive Disorder (IED) vs. Mania: IED involves recurrent, impulsive aggressive outbursts disproportionate to the provocation — not premeditated. The outbursts are ego-dystonic (the person feels regret). Manic aggression is embedded in a broader episode with elevated/irritable mood, decreased need for sleep, and grandiosity. IED outbursts are brief and not part of a sustained mood state.

Assessment Tools in Context

Child Behavior Checklist (CBCL): A parent-report measure of child and adolescent behavioral and emotional problems, norm-referenced by age and sex (Achenbach & Rescorla, 2001). The CBCL externalizing subscale captures aggression, rule-breaking, and conduct problems. Widely used in school, outpatient, and community set-

tings. The corresponding teacher-report version is the Teacher's Report Form (TRF); the self-report version is the Youth Self-Report (YSR).

Conners Rating Scales (3rd Edition): While primarily used for ADHD assessment, the Conners 3 includes subscales for ODD and CD — important because ADHD and ODD co-occur in 40–60% of cases. Multiple informant versions (parent, teacher, self-report) allow cross-setting comparison.

Strengths and Difficulties Questionnaire (SDQ): Brief 25-item parent/teacher/self-report for children ages 4–17 (Goodman, 1997). Covers emotional symptoms, conduct problems, hyperactivity/inattention, peer problems, and prosocial behavior. Free to use; widely used internationally.

Psychopathy Checklist: Youth Version (PCL:YV): A clinician-administered tool for adolescents adapted from Hare's PCL-R, measuring callous-unemotional (CU) traits, narcissism, and impulsivity (Forth et al., 2003). CU traits are a specifier in CD that predicts poorer treatment response and greater severity. The NCMHCE may reference this in the context of case conceptualization and evidence-based treatment selection.

Exam tip: The counselor's response to aggression and rule violations in youth should always involve family systems assessment, school coordination, and evidence-based approaches (Parent Management Training, Parent-Child Interaction Therapy [PCIT] for younger children, Multisystemic Therapy for adolescents) rather than individual insight-oriented therapy alone.

Worked Mini-Case

Vignette: Marcus Jr., 14, is brought in by his mother after his third school suspension for fighting. He argues constantly at home, refuses chores, and once damaged a neighbor's car after an argument. His mother reports he has no remorse. His pediatrician has ruled out ADHD. He

denies using substances. He was referred by the school counselor.

Question: In developing Marcus Jr.'s case conceptualization, the counselor should FIRST:

A. Diagnose Conduct Disorder and discuss long-term prognosis with the family

B. Meet with Marcus Jr. individually to build rapport and apply MI techniques

C. Conduct a comprehensive biopsychosocial assessment including school records, family history, and collateral from school personnel

D. Refer immediately for psychiatric evaluation and medication management

Best answer: C

Reasoning chain: Before diagnosis or intervention, the NCMHCE rewards comprehensive assessment — especially with disruptive behavior disorders where school, family, and peer contexts are diagnostically essential. Collateral data distinguishes ODD from CD (property destruction of a neighbor's car could meet CD criteria; three suspensions and family conflict fit both). The biopsychosocial approach is always the correct starting point.

Why distractors lose: - A: Diagnosis cannot be assigned without a full assessment; furthermore, counselors do not typically share long-term prognosis as a first-session intervention - B: Rapport matters, but launching individual MI before understanding the systemic context is premature; family context is essential in adolescent work - D: Psychiatric evaluation may be warranted but is premature as the first step; behavioral intervention is first-line for ODD/CD before considering medication

Answer Key — Chapter 20 Practice Questions

Q76 — Answer: C

Rationale: Aggression toward others, fire-setting, and theft are CD criteria — these are serious violations of others' rights, not just defiant behavior (ODD). ADHD may co-occur but doesn't account for the pattern of rights violations. IED involves impulsive aggression but not the deceit and rule-violation pattern.

Chapter 21 — Somatic Symptom and Related Disorders

Overview

Somatic symptom and related disorders involve physical symptoms that cause significant distress or impairment — but the central issue is the psychological response to those symptoms (excessive worry, disproportionate fear) rather than whether the symptoms are medically explained. The DSM-5 removed the concept of "medically unexplained symptoms" as a requirement — even a client with a diagnosed medical condition can have a somatic symptom disorder if their response to the symptoms is excessive and disabling (APA, 2022).

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Core Feature
Somatic Symptom Disorder	F45.1	6C20	≥1 somatic symptom with excessive thoughts, feelings, or behaviors related to the symptom
Illness Anxiety Disorder	F45.21	6B23	Fear of having a serious illness with minimal or no somatic symptoms
Conversion Disorder (Functional Neurological Symptom Disorder)	F44.x	6B60	Motor or sensory symptoms that cannot be explained neurologically

Diagnosis	ICD-10	ICD-11	Core Feature
Factitious Disorder	F68.10 (imposed on self); F68.A (imposed on another)	6D50; 6D51	Falsification of medical/ psychological symptoms; motivated by sick role
Psychological Factors Affecting Other Medical Conditions	F54	6E40	Psychological factors adversely affect a medical condition

ICD-11 notes: Somatic Symptom Disorder: nearest equivalent is bodily distress disorder; criteria differ from DSM-5-TR. Illness Anxiety Disorder: coded as hypochondriasis, with the obsessive-compulsive and related disorders. Conversion Disorder: coded as dissociative neurological symptom disorder, with the dissociative disorders.

What the Exam Rewards / What It Punishes

Rewards:

- Medical evaluation first (even in somatic disorders, a medical workup establishes the clinical picture)
- Avoiding dismissive language ("it's all in your head" — never appropriate)
- CBT targeting catastrophic health beliefs and hypervigilance to bodily sensations
- Maintaining the alliance with clients who have been dismissed by medical providers

Punishes:

- Diagnosing somatic symptom disorder before completing a medical evaluation
- Treating somatic disorders without addressing the psychological response patterns
- Colluding with excessive medical testing as a reassurance behavior

Practice Questions

Question 77. A client presents reporting persistent chest pain, dizziness, and shortness of breath that have been evaluated by three different cardiologists with no medical findings. She spends 4–5 hours daily researching her symptoms online and visits the ER every two weeks. She is increasingly unable to work. The MOST appropriate clinical response is:

A. Refer to a fourth specialist for additional testing, as the prior cardiac evaluations may have missed a rare or intermittent condition

B. Diagnose conversion disorder, as no medical cause has been found, and begin CBT for the symptoms

C. Acknowledge the distress caused by her symptoms, affirm the medical clearances, and begin CBT targeting health anxiety behaviors

D. Refer to a psychiatrist for anxiolytic medication to address the anxiety symptoms driving her ER visits

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 40-year-old has eight months of excessive worry about, and time consumed by, multiple physical symptoms. Repeated medical workups are negative, yet she remains highly distressed and disproportionately focused on the symptoms. The most likely diagnosis is:

A. Illness Anxiety Disorder

B. Factitious Disorder

C. Conversion Disorder (Functional Neurological Symptom Disorder)

D. Somatic Symptom Disorder

Answer: D. Distressing somatic symptoms accompanied by disproportionate thoughts, anxiety, and time devoted to them, persisting six months or more, define Somatic Symptom Disorder. Illness Anxiety (A) features preoccu-

pation with having a disease with minimal actual symptoms; Conversion (C) is a neurological deficit; Factitious (B) is intentional falsification. What would change the answer: few or no somatic symptoms but strong disease conviction → Illness Anxiety Disorder.

Assessment — what to do next. The most appropriate approach is:

A. Order additional diagnostic tests to reassure her that nothing serious has been missed, reducing her worry about the symptoms

B. Tell her the symptoms are not real and that her distress will ease once she accepts the negative workups

C. Coordinate with the medical provider to avoid redundant testing, assess symptom-related distress and functioning, and schedule regular (not symptom-contingent) contact

D. Administer the Y-BOCS to measure the severity of her repetitive preoccupation with physical symptoms

Answer: C. Care is coordinated to prevent iatrogenic over-testing while distress and functioning are assessed and visits are scheduled. More tests (A) and dismissal (B) each worsen the cycle.

Treatment Decision Flow

Somatic-symptom items reward validation plus consistent, noncontingent care — and punish both dismissal and reassurance.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Somatic symptom disorder	CBT with regular, scheduled (not symptom-contingent) visits; validate distress	Ordering more tests, or dismissing symptoms as “not real”
Illness anxiety disorder	CBT that reduces reassurance-seeking and body checking	Providing repeated reassurance

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Functional neurological (conversion) disorder	Psychoeducation, physical therapy, CBT	Confronting the client as “faking”

Sequencing & caveats. Scheduled visits decouple care from symptom escalation. Validate the reality of the client’s distress while shifting the focus from investigation to functioning. Avoid / contraindicated. Excessive testing and repeated reassurance both reinforce the symptom cycle; telling a client it is “all in your head” ruptures the alliance. Scope. Coordinate with medical providers to prevent over-testing.

Differentiating Within the Family

Somatic Symptom Disorder vs. Illness Anxiety Disorder: The key distinction: SSD has significant somatic symptoms with excessive psychological response to them. IAD has minimal or absent somatic symptoms with intense fear of having a serious illness (previously called hypochondriasis in DSM-IV).

Conversion Disorder vs. Malingering vs. Factitious Disorder:

- Conversion: Symptoms are real to the client; not consciously produced; no external incentive
- Factitious: Symptoms are consciously produced; motivated by the sick role (internal incentive)
- Malingering: Symptoms are consciously produced; motivated by external incentive (avoiding legal consequences, getting disability payments)

Somatic Symptom Disorder vs. Psychological Factors Affecting Other Medical Conditions (F54): In F54 a documented medical condition is present, and psychological or behavioral factors adversely affect its course or treatment — for example, anxiety that triggers asthma attacks, denial that leads a client with diabetes to skip in-

sulin, or chronic stress that worsens hypertension or irritable bowel symptoms (APA, 2022). The focus is the medical condition being made worse. In SSD the focus is the client's excessive thoughts, feelings, and behaviors about the symptoms themselves. Direction matters for one more distinction: in Adjustment Disorder, the medical illness is the stressor producing psychological symptoms; in F54, psychological factors are producing medical consequences. Counselor role: target the maintaining factor (adherence barriers, stress, avoidance) and collaborate with the medical team.

Assessment Tools in Context

PHQ-15 (Patient Health Questionnaire – Somatic Symptom Scale): 15-item self-report assessing somatic symptom severity over the past 4 weeks (Kroenke et al., 2002). Score ≥5 indicates mild, ≥10 moderate, ≥15 severe somatic symptom burden. Useful across primary care and counseling settings as a screen and symptom-tracking tool.

Somatic Symptom Scale-8 (SSS-8): An ultra-brief 8-item screen derived from the PHQ-15 (Gierk et al., 2014). Clinically useful for time-limited intake settings. Scores of 12–15 indicate high and 16–32 very high somatic symptom burden.

Whiteley Index (WI): Designed to screen for health anxiety (Illness Anxiety Disorder) (Pilowsky, 1967). Measures hypochondriacal concerns. The WI-7 is a brief validated version. Score >8 suggests clinically significant health anxiety warranting further assessment.

Illness Attitudes Scale (IAS): 29-item self-report measuring multiple dimensions of health anxiety including worry about illness, bodily preoccupation, disease conviction, and doctor-seeking behavior (Kellner, 1986). Used in research and specialized somatic disorder programs.

Exam tip: The NCMHCE distinguishes Somatic Symptom Disorder (distressing somatic symptoms with excessive

thoughts/feelings/behaviors regarding them) from Illness Anxiety Disorder (health anxiety with minimal or no actual somatic symptoms). The clinical strategy for both: validate, avoid unnecessary medical reassurance-seeking (which reinforces anxiety), and engage behavioral approaches that reduce hypervigilance to body signals.

Worked Mini-Case

Vignette: Rosario, 41, presents having visited four different physicians in the past year for persistent fatigue, abdominal pain, and headaches. Each physician has found no medical explanation and her last physician told her "there's nothing wrong." She is increasingly convinced she has a serious undiagnosed illness and spends several hours each day researching symptoms online. She feels dismissed by medical providers and is reluctant to engage in counseling, saying "I need a real diagnosis, not a therapist."

Question: The counselor's MOST therapeutically effective initial response is:

A. Validate Rosario's frustration, acknowledge that her symptoms are real, and collaboratively explore how her health concerns are affecting her daily life

B. Provide psychoeducation that her symptoms are psychosomatic and encourage her to stop researching her symptoms online

C. Recommend that Rosario see another specialist to rule out any remaining medical conditions

D. Explain that avoiding the doctor will reduce her anxiety and structure a behavioral contract around limiting medical visits

Best answer: A

Reasoning chain: The NCMHCE consistently rewards a stance that: (1) validates the reality of the client's experience rather than dismissing symptoms as imaginary, and (2) begins with the client's frame rather than imposing

the counselor’s. Somatic disorders involve real distress. The therapeutic alliance must be established before any psychoeducation or behavioral contracts can be effective.

Why distractors lose: - B: “Psychosomatic” is invalidating to Rosario’s experience and will rupture the already fragile alliance; psychoeducation comes after validation and rapport - C: Further specialist referral reinforces the illness-seeking behavior; the NCMHCE does not reward reinforcing somatic behavior patterns without concurrent counseling engagement - D: A behavioral contract in the first session before trust is established is premature; the framing also overlooks the counselor’s collaborative role

Answer Key — Chapter 21 Practice Questions

Q77 — Answer: C

Rationale: This presentation is consistent with Somatic Symptom Disorder or Illness Anxiety Disorder — the physical symptoms are real to her; the response (5 hours of online research, frequent ER visits) is disproportionate and maintaining the anxiety. CBT targeting reassurance-seeking and health anxiety is the evidence-based treatment. A reinforces excessive help-seeking. B misidentifies the diagnosis (conversion involves neurological symptoms). D is not first-line.

Chapter 22 — Dissociative Disorders

Overview

Dissociative disorders involve disruptions in consciousness, memory, identity, or perception. They are almost always rooted in trauma and are among the most misunderstood and stigmatized diagnoses in mental health. The exam tests recognition and appropriate treatment planning — primarily the three-phase trauma treatment model and the stabilization-before-processing principle (which is especially critical in DID).

Diagnosis Reference Table

Diagnosis	ICD-10	ICD-11	Core Feature
Dissociative Identity Disorder (DID)	F44.81	6B64	≥2 distinct personality states/ identities; amnesia between states
Dissociative Amnesia	F44.0	6B61	Inability to recall important autobiographical information (not ordinary forgetting)
Depersonalization/ Derealization Disorder	F48.1	6B66	Persistent experiences of unreality or detachment from self (depersonalization) or surroundings (derealization)

The Clinical Picture

DID in a case sim: A client reports “losing time,” being told they did or said things they don’t remember, finding

objects or writings they don't recall creating, hearing voices "from inside" (not hallucinations in the schizophrenic sense — internal voices belonging to other identity states), and extreme identity confusion. The voices are often experienced as distinctly different from the client's own thoughts.

Depersonalization/derealization: The client describes feeling like they are watching themselves from outside their body (depersonalization) or that the world looks unreal, foggy, or dreamlike (derealization). These experiences are disturbing and persistent. Common in panic disorder, PTSD, and trauma histories, but when they are the persistent, primary complaint, the disorder is DP/DR disorder.

What the Exam Rewards / What It Punishes

Rewards:

- The three-phase trauma model (stabilization → processing → integration) — applied even more carefully in DID because processing without stabilization can cause rapid destabilization between identity states
- Compassionate, non-reactive acceptance of DID presentations (not treating the diagnosis as inherently dramatic or invalid)
- Assessment of safety for all identity states in DID (some states may be suicidal even if the presenting state is not)

Punishes:

- Attempting direct trauma processing with DID clients who lack sufficient stabilization and communication between states
- Dismissing dissociative symptoms as manipulation
- Failing to recognize that reported "hearing voices" in DID clients is internal (identity-based) rather than external hallucination

Practice Questions

Question 78. A client with Dissociative Identity Disorder has been in treatment for four months. She is now requesting to begin EMDR to process childhood abuse memories. The counselor should:

A. Begin EMDR immediately, as the client has been in treatment long enough to have established a working alliance

B. Assess the current level of stabilization: communication between states, capacity to stay grounded, availability of coping skills; proceed to EMDR only if stabilization criteria are met

C. Refuse EMDR and maintain supportive treatment only

D. Increase session frequency to support the client through the EMDR process

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 30-year-old reports gaps in memory for hours at a time, finds unfamiliar items she does not remember buying, and has been told she sometimes behaves like “a different person” who uses a different name. She has an extensive childhood trauma history and no psychotic symptoms. The most likely diagnosis is:

A. Dissociative Identity Disorder

B. Schizophrenia

C. Borderline Personality Disorder

D. Dissociative Amnesia

Answer: A. Two or more distinct identity states with recurrent amnesia, in the context of severe trauma, define DID; the experiences are internal, not the external hallucinations of psychosis (B). What would change the answer: amnesia without distinct identity states → Dissociative Amnesia.

Assessment — what to do next. The most appropriate assessment step is:

A. Screen for dissociation with a validated measure (e.g., the Dissociative Experiences Scale) and prioritize current safety and stabilization

B. Use hypnosis to recover the missing memories and clarify the traumatic events that may underlie the identity states

C. Confront the client about the inconsistencies in her account to test whether the memory gaps and reported identity shifts are genuine

D. Administer the MMSE to determine whether the memory gaps reflect a cognitive impairment

Answer: A. Validated dissociation screening with a safety-and-stabilization focus is appropriate. Hypnotic memory recovery (B) risks memory contamination and destabilization.

Treatment Decision Flow

Dissociative items reward phase-oriented pacing and punish premature or suggestive memory work.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Dissociative identity disorder	Phase 1: safety and stabilization; build internal communication before any trauma processing	Moving directly to trauma processing
Recovering traumatic memories	Stabilize; let material emerge without pressure	Hypnosis or suggestive techniques to “recover” memories (contamination risk)
Hostile “protector” part	Acknowledge its protective function; build trust across the system	Trying to eliminate or forcibly integrate parts early

Sequencing & caveats. Treatment is phase-oriented: stabilization precedes trauma work; premature processing destabilizes and retraumatizes. Avoid / contraindicated. Avoid suggestive memory-recovery techniques and forced integration of parts. Scope. Standard scope; DID benefits from specialized, relationally stable treatment.

Differentiating Within the Family

Dissociative Identity Disorder (DID) vs. Other Specified Dissociative Disorder (OSDD): DID requires the presence of two or more distinct personality states/identities that recurrently take control of behavior, along with amnesia for everyday events or important personal information. OSDD is used when dissociative features are clinically significant but don't meet full criteria for DID — for example, identity disruption without distinct amnesia, or partial identity states.

DID vs. Borderline Personality Disorder (BPD): Both are associated with trauma history and involve identity instability. In BPD, identity disturbance is experienced as chronic emptiness and shifting self-image rather than distinct alternate identity states. Impulsive behaviors in BPD are driven by emotional dysregulation; in DID, impulsive behaviors may be attributed to alternate parts. Importantly, DID and BPD have high comorbidity — the diagnoses are not mutually exclusive.

Depersonalization/Derealization Disorder (DPDR) vs. Anxiety Disorders and Panic Disorder: Dissociative symptoms (feeling detached from the body or that the world is unreal) frequently occur during panic attacks and in anxiety disorders. DPDR is diagnosed only when depersonalization/derealization experiences are persistent and distressing in the absence of a panic disorder context, and when they are not better explained by another disorder or substance.

DID vs. Malingering: Clinicians sometimes question the authenticity of DID. The NCMHCE does not test skepti-

cism — it tests clinical response. The appropriate response to a client reporting identity states with amnesia is thorough assessment, trauma-informed care, and consultation, not immediate suspicion of fabrication.

Dissociative Amnesia vs. Major Neurocognitive Disorder: In dissociative amnesia, memory loss is psychologically mediated (often trauma-related) and typically involves autobiographical information (identity, personal history) while procedural memory and general knowledge remain intact. In Major NCD, memory loss is neurologically mediated, typically begins with episodic (recent) memory difficulties while procedural memory is relatively preserved early, and is progressive.

Assessment Tools in Context

Dissociative Experiences Scale (DES): 28-item self-report measuring the frequency of dissociative experiences (absorption, depersonalization, amnesia) (Bernstein & Putnam, 1986). Score ≥30 suggests clinically significant dissociation warranting further evaluation for a dissociative disorder. Score ≥30 on the DES-Taxon subscale specifically suggests pathological dissociation.

Structured Clinical Interview for DSM-5 Dissociative Disorders (SCID-D): The gold-standard clinician-administered diagnostic interview for dissociative disorders (Steinberg, 2023). Assesses five core areas: amnesia, depersonalization, derealization, identity confusion, and identity alteration. Time-intensive; used in specialized settings and forensic evaluations.

Multiscale Dissociation Inventory (MDI): 30-item self-report covering six dissociation dimensions: disengagement, depersonalization, derealization, emotional constriction, memory disturbance, and identity dissociation (Briere, 2002).

Adverse Childhood Experiences (ACE) Questionnaire: While not a dissociation measure, ACE scores are routinely used in trauma-informed practice to quantify cu-

mulative childhood adversity (Felitti et al., 1998). High ACE scores correlate with DID and complex PTSD presentations.

Exam tip: The NCMHCE tests Phase-Based Trauma Treatment in the context of dissociative disorders. Phase 1 (Safety and Stabilization) must be established before Phase 2 (Trauma Processing). Attempting trauma processing with a dissociated client who lacks stabilization skills is a clinical error the exam will punish.

Worked Mini-Case

Vignette: Lena, 32, has been in counseling for 8 months for PTSD related to prolonged childhood abuse. She recently disclosed that she has "another part" named "Alex" who she says "takes over" when she is overwhelmed, and that she sometimes loses hours with no memory of what occurred. She reports significant distress about these experiences. She is currently not in any acute crisis and has good social support.

Question: The counselor's MOST clinically appropriate next step is:

A. Immediately refer Lena to a psychiatrist for evaluation for antipsychotic medication given the presence of alternate identity states

B. Continue with EMDR and begin direct trauma processing of her childhood abuse to resolve the dissociation

C. Consult with a specialist in dissociative disorders, maintain the current safety and stabilization focus, and psychoeducate Lena about structural dissociation in a normalizing, non-pathologizing way

D. Confront Lena's "Alex" identity directly in session to assess the content of the alternate state

Best answer: C

Reasoning chain: This is a Phase 1 → Phase 2 transition question. Lena is currently in stabilization and has dis-

closed symptoms consistent with DID. The appropriate move is to (1) consult with a specialist, (2) stay in Phase 1 (safety and stabilization), and (3) provide psychoeducation that normalizes without dramatizing. Moving to trauma processing (B) before dissociative symptoms are stabilized is a clinical error. Antipsychotics (A) are not the first-line response to identity states.

Why distractors lose: - A: Alternate identity states in DID are not psychosis; antipsychotics are not indicated as a first response to this presentation - B: Direct trauma processing in an unstabilized dissociative client risks destabilization, crisis, and retraumatization - D: “Confronting” an alternate state is a clinical approach that requires specialized training and should not occur in a generalist counseling session without that expertise; it also risks reinforcing fragmentation

Answer Key — Chapter 22 Practice Questions

Q78 — Answer: B

Rationale: Phase 1 (stabilization) criteria must be met before Phase 2 (processing) begins — this is even more critical in DID where destabilization can occur rapidly and different identity states may process trauma differently. A ignores phase criteria. C withholds an evidence-based treatment unnecessarily. D adds sessions without addressing the phase readiness question.

Chapter 23 — Sleep-Wake Disorders

Overview

Sleep disorders appear on the NCMHCE primarily as co-occurring conditions that complicate other diagnoses. A counselor who misses that poor sleep is driving cognitive impairment, irritability, or mood instability will misattribute symptoms to a mood or anxiety disorder. The exam expects you to assess sleep as a routine component of the intake.

Key Sleep Disorders for the NCMHCE

Disorder	Core Feature	Counselor Role
Insomnia Disorder	Difficulty initiating/maintaining sleep ≥3 nights/week, ≥3 months	CBT-I (first-line over medication)
Hypersomnolence Disorder	Excessive sleepiness despite adequate sleep	Medical evaluation, rule out medical causes
Narcolepsy	Recurrent lapses into sleep + cataplexy, hypocretin deficiency, or short REM latency	Medical management; counselor provides psychoeducation/support
Obstructive Sleep Apnea	Repeated airway obstruction during sleep; snoring, daytime fatigue	Medical referral for sleep study, CPAP
Nightmare Disorder	Repeated, disturbing dreams causing distress; awakening with recall	IRT (Imagery Rehearsal Therapy; Krakow & Zadra, 2006), especially if trauma-related

Disorder	Core Feature	Counselor Role
REM Sleep Behavior Disorder	Acting out dreams physically (punching, kicking)	Medical evaluation; often prodromal to neurodegenerative disease

The Clinical Picture

Sleep-Wake Disorders share a common phenomenological core: disruption of the sleep-wake cycle that causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Insomnia Disorder presents with difficulty initiating sleep, difficulty maintaining sleep, or early morning awakening with inability to return to sleep — despite adequate opportunity and circumstances for sleep. It occurs at least 3 nights per week, for at least 3 months, and is not better explained by a substance or another sleep disorder. Daytime sequelae — fatigue, cognitive difficulties, mood disturbance, occupational impairment — are often what bring clients to counseling.

Hypersomnolence Disorder involves excessive sleepiness despite at least 7 hours of sleep. The person may sleep 9–11 hours and still struggle to feel alert during the day, or take naps that are not restorative. It is not explained by narcolepsy, sleep apnea, or circadian rhythm disruption.

Narcolepsy is characterized by recurrent, irresistible attacks of sleep along with cataplexy (brief loss of muscle tone triggered by strong emotion), sleep paralysis, and hypnagogic/hypnopompic hallucinations. The hallmark of Type 1 Narcolepsy is cataplexy with orexin deficiency. Type 2 Narcolepsy involves sleep attacks without cataplexy.

Breathing-Related Sleep Disorders include Obstructive Sleep Apnea Hypopnea (snoring, observed apneas, daytime sleepiness, typically in adults with elevated BMI), Central Sleep Apnea (cessation of breathing without upper airway obstruction, often associated with cardiac or

neurological conditions), and Sleep-Related Hypoventilation.

Circadian Rhythm Sleep-Wake Disorders involve misalignment between the individual's internal circadian clock and the desired sleep-wake schedule. This includes Delayed Sleep Phase Type (night owl who cannot shift earlier), Advanced Sleep Phase Type (early riser who cannot shift later), Shift Work Type, and Jet Lag Type.

Parasomnias include NREM disorders (sleepwalking, sleep terrors, confusional arousals) characterized by incomplete awakening from deep NREM sleep, and REM Sleep Behavior Disorder (acting out vivid, often violent dreams — a biomarker for neurodegenerative disorders).

Restless Legs Syndrome (RLS) presents with an uncomfortable urge to move the legs, typically worse at rest and in the evening, that is relieved by movement — often significantly disrupting sleep onset.

CBT-I (Cognitive Behavioral Therapy for Insomnia)

CBT-I is the gold-standard, first-line treatment for chronic insomnia — superior to sleep medication for long-term outcomes and without the risks of tolerance, dependence, or daytime sedation (Edinger & Carney, 2015). The NCMHCE consistently rewards recommending CBT-I over medication. The five core components work synergistically and together are more effective than any component alone.

1. Sleep Restriction Therapy: The most counterintuitive and most powerful CBT-I component. The client initially limits time in bed to their estimated actual sleep time (e.g., if averaging 5 hours of sleep, prescribed time in bed = 5 hours + a 30-minute buffer). This concentrates sleep drive, builds homeostatic sleep pressure, and consolidates fragmented sleep. As sleep efficiency improves above 85%, time in bed is gradually extended in 15- to 30-minute increments. *Critical contraindications:* Sleep re-

striction is contraindicated in Bipolar I Disorder (sleep deprivation can precipitate mania) and in seizure disorders. Screen for these before prescribing — this is tested.

2. Stimulus Control: Reconditioning the bed-wakefulness association. Instructions: (a) use the bed only for sleep and sex — no reading, TV, phone use, or worrying in bed; (b) get out of bed if unable to fall asleep within approximately 20 minutes and return only when sleepy; (c) maintain the same wake time every morning regardless of how much sleep was obtained; (d) avoid napping, or limit naps to under 20 minutes before 3 PM. Stimulus control directly targets conditioned arousal — the learned association between the bed environment and wakefulness that perpetuates insomnia.

3. Sleep Hygiene Education: Behavioral habits that support healthy sleep architecture. Consistent wake time anchors the circadian rhythm. Limiting caffeine after noon; avoiding alcohol (disrupts sleep architecture in the second half of the night); dimming lights and avoiding blue-light screens 60 minutes before bed; keeping the bedroom cool, dark, and quiet; regular physical activity (but not within 3–4 hours of bedtime). **Important:** Sleep hygiene education alone is insufficient for clinical insomnia and does not produce durable change by itself. It is a necessary but not sufficient component of CBT-I.

4. Cognitive Restructuring: Identifying and modifying dysfunctional beliefs and catastrophic cognitions about sleep. Common targets: “I must get 8 hours or I can’t function,” “One bad night means tomorrow will be ruined,” “I’m losing control of my sleep,” “Something is physically wrong with me.” These beliefs amplify pre-sleep arousal and maintain the insomnia cycle. Techniques include Socratic questioning, evidence testing, and psychoeducation about normal sleep variability and the effects of sleep debt. The Dysfunctional Beliefs and Attitudes about Sleep scale (DBAS-16) can identify specific cognitive targets for treatment.

5. Relaxation Training: Reducing physiological and cognitive hyperarousal at bedtime. Techniques include progressive muscle relaxation (PMR — systematically tensing and releasing muscle groups), diaphragmatic breathing, guided imagery, and mindfulness-based approaches. Particularly effective for clients with somatic arousal (muscle tension, racing heart, physical restlessness at bedtime). Relaxation training is one component of CBT-I, not a substitute for the behavioral components.

Clinical note on delivery: Sleep restriction + stimulus control are the active behavioral core of CBT-I. Together they target inadequate sleep drive (too much time in bed awake) and conditioned arousal. Cognitive restructuring prevents relapse. In brief clinical contacts, prioritizing these three components is evidence-supported.

Practice Questions

Question 79. A client with insomnia reports she has been taking a sleep aid for 6 months and wants to reduce the medication. She often lies awake in bed for 2–3 hours and has begun watching television in bed. The MOST appropriate evidence-based intervention is:

A. Refer to a sleep specialist for an overnight sleep study to rule out a primary sleep disorder

B. Support the gradual medication taper and add CBT-I: sleep restriction and stimulus control

C. Recommend that she use the bedroom for sleep only and introduce a consistent wake time without medication changes

D. Provide relaxation training only, because that directly addresses the hyperarousal component of her insomnia

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 45-year-old reports six months of difficulty falling and staying asleep at least four nights a week, with daytime fatigue, despite adequate op-

portunity to sleep. He reports no loud snoring or witnessed breathing pauses, and his mood is otherwise stable. The most likely diagnosis is:

A. Insomnia Disorder

B. Obstructive Sleep Apnea

C. Circadian Rhythm Sleep-Wake Disorder

D. Major Depressive Disorder

Answer: A. Difficulty initiating and maintaining sleep at least three nights a week for three months or more, with daytime impairment and adequate opportunity, defines Insomnia Disorder. The absence of snoring or apneas argues against OSA (B); stable mood argues against MDD (D). What would change the answer: loud snoring, witnessed apneas, or obesity → evaluate for OSA; a shifted but otherwise normal sleep schedule → circadian disorder.

Assessment — what to do next. The most appropriate assessment step is:

A. Have the client keep a one-to-two-week sleep diary, screen for OSA risk (e.g., STOP-BANG), and refer for polysomnography if indicated

B. Begin a hypnotic immediately to restore his sleep and relieve the daytime fatigue he describes

C. Start sleep restriction without screening, limiting time in bed to match his actual sleep time

D. Administer the PHQ-9 only, to confirm that an underlying depression is not driving the sleep problems

Answer: A. A sleep diary plus OSA screening guide the diagnosis and the need for a sleep study or referral. Note that sleep restriction (C) is screened first for bipolar and seizure contraindications.

Treatment Decision Flow

Sleep items reward CBT-I over hypnotics and include a genuine medical contraindication to screen for.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Chronic insomnia	CBT-I (first-line; superior to medication long-term)	Long-term hypnotics as the primary treatment
Sleep consolidation needed	Sleep restriction (a CBT-I component)	Advising the client to spend more time in bed
Conditioned arousal (bed associated with wakefulness)	Stimulus control (bed for sleep and sex only)	Allowing work or TV in bed

Sequencing & caveats. CBT-I is first-line and durable. Sleep restriction and stimulus control are counterintuitive but are the active ingredients; pair them with psychoeducation to sustain adherence. Avoid / contraindicated. Sleep restriction should be used with caution or avoided in bipolar I disorder (sleep loss can precipitate mania) and in seizure disorders — screen before prescribing the technique. Scope. Short-term hypnotics, if used, are accessed by referral; CBT-I remains the primary treatment.

Differentiating Within the Family

Insomnia Disorder vs. Insomnia as a Symptom: Insomnia is a transdiagnostic symptom of depression, anxiety, PTSD, and many other conditions. Insomnia Disorder is diagnosed when sleep disturbance is the primary complaint, present for 3+ months, causes significant distress/impairment, and warrants independent clinical attention beyond the comorbid condition. The NCMHCE may test whether a counselor recognizes that insomnia can be both a symptom and a co-occurring disorder requiring its own intervention.

Insomnia Disorder vs. Hypersomnolence Disorder vs. Narcolepsy: The critical question is whether the problem is getting to sleep/staying asleep (Insomnia) versus staying awake (Hypersomnolence/Narcolepsy). Narcolepsy is dis-

tinguished by cataplexy, sleep paralysis, and sleep attacks of sudden onset and irresistible force.

Insomnia vs. Circadian Rhythm Disorder: A client who presents as having "insomnia" may actually have Delayed Sleep Phase Disorder — they can sleep fine if allowed to do so at 3 AM, but cannot initiate sleep at 10 PM. Careful sleep diary review helps distinguish.

Parasomnias vs. Nocturnal Seizures: Sleepwalking and sleep terrors can be confused with nocturnal seizures. Seizure-related events tend to be stereotyped, may involve automatisms, and are associated with post-ictal confusion. Neurological referral is appropriate when seizure is possible.

NREM Parasomnias vs. REM Sleep Behavior Disorder: NREM disorders occur in the first third of the night (deep NREM sleep), the person does not recall the event, and content may involve fear or wandering without purposeful dreaming. REM Sleep Behavior Disorder occurs in the second half of the night (REM-dense periods), the person acts out vivid dreams, and recall is often present. REM Sleep Behavior Disorder is a neurological risk marker and warrants neurology referral.

Assessment Tools in Context

Pittsburgh Sleep Quality Index (PSQI): 19-item self-report assessing sleep quality and disturbances over the past month (Buysse et al., 1989). Produces a global score across 7 components (subjective sleep quality, latency, duration, efficiency, disturbances, sleep medication use, daytime dysfunction). Score >5 indicates poor sleep quality. The PSQI is the most widely used sleep assessment in counseling-adjacent settings.

Epworth Sleepiness Scale (ESS): 8-item self-report measuring daytime sleepiness across common situations (sitting and reading, watching TV, riding in a car; Johns, 1991). Score ≥10 indicates excessive daytime sleepiness; ≥16 indicates severe sleepiness. Useful for identifying hy-

persomnolence and flagging need for sleep medicine referral.

Insomnia Severity Index (ISI): 7-item self-report assessing insomnia severity over the past two weeks (Morin et al., 2011). Score ≥15 indicates moderate-to-severe clinical insomnia. Widely used to track CBT-I treatment response.

Sleep Diary: A daily log of sleep and wake times, sleep quality, naps, and daytime function. The gold standard for behavioral sleep assessment. A 2-week diary is standard before initiating CBT-I. The sleep diary informs sleep restriction protocols and sleep efficiency calculations.

Actigraphy: A wrist-worn device tracking movement as a proxy for sleep-wake states over days to weeks. Used to complement subjective reports and identify circadian pattern disruptions. Not routinely used in counseling settings but may be referenced in consultation with sleep medicine.

Exam tip: CBT-I (Cognitive Behavioral Therapy for Insomnia) is the first-line treatment for Insomnia Disorder — more effective and more durable than sleep medication. The NCMHCE rewards counselors who recommend CBT-I and avoid recommending sleep medication as a first-line approach.

What the Exam Rewards / What It Punishes

Rewards:

- Recognizing insomnia as both a transdiagnostic symptom and an independent disorder that warrants its own intervention
- Recommending CBT-I as first-line treatment over sleep hygiene advice alone or medication
- Distinguishing daytime sleepiness (suggestive of sleep apnea, narcolepsy, or hypersomnolence) from nighttime insomnia

- Referring for sleep medicine evaluation when symptoms suggest a breathing-related sleep disorder or narcolepsy
- Using a sleep diary to inform behavioral treatment planning
- Addressing the cognitive component of insomnia — catastrophic thinking about sleep, sleep-related anxiety, dysfunctional beliefs about sleep need

Punishes:

- Recommending medication as the first or primary intervention for insomnia
- Treating insomnia only as a symptom without addressing it directly when it meets criteria as a standalone disorder
- Missing the connection between REM Sleep Behavior Disorder and neurodegenerative disease risk (Parkinson's, Lewy Body Dementia)
- Ignoring daytime impairment data in sleep case vignettes
- Failing to distinguish parasomnias from nocturnal seizures in vignettes that include stereotyped nocturnal events

Worked Mini-Case

Vignette: Joelle, 38, presents with difficulty falling asleep (taking 60–90 minutes nightly) and waking at 3–4 AM and being unable to return to sleep for 3–4 hours. She has had this pattern for 7 months following a job change. She typically goes to bed at 10 PM and lies in bed watching television and scrolling her phone until she falls asleep. She naps for 1–2 hours on weekends. She reports that she worries "if I don't get 8 hours I can't function" and rates her sleep quality as 2 out of 10.

Question: The counselor's MOST appropriate initial treatment recommendation is:

A. Recommend Joelle take over-the-counter sleep aids until her sleep normalizes and revisit counseling strategies at her 6-week follow-up

B. Refer Joelle to a psychiatrist for evaluation for a short course of prescription hypnotics

C. Psychoeducate Joelle about sleep hygiene basics, including keeping a regular sleep schedule

D. Begin a CBT-I protocol including sleep restriction, stimulus control (limiting bed to sleep only), and cognitive restructuring of her beliefs about sleep

Best answer: D

Reasoning chain: This is a clear CBT-I indication. Joelle has all the hallmarks of behavioral insomnia: inappropriate sleep behaviors (TV in bed, weekend napping), cognitive distortions about sleep (“8 hours or I can’t function”), and conditioned arousal to the bed. CBT-I is the first-line, evidence-based treatment. Sleep hygiene alone (C) is insufficient for this severity.

Why distractors lose: - A: OTC sleep aids (antihistamines) cause tolerance quickly and do not address behavioral or cognitive maintaining factors - B: Medication referral as a first step is clinically inappropriate for a counseling-appropriate insomnia presentation without first attempting CBT-I - C: Sleep hygiene education alone is insufficient for 7-month chronic insomnia with cognitive components; it is a component of CBT-I but not a substitute

Answer Key — Chapter 23 Practice Questions

Q79 — Answer: B

Rationale: CBT-I (specifically sleep restriction and stimulus control, which address the TV-watching-in-bed behavior) combined with a supported medication taper is the evidence-based approach (Edinger & Carney, 2015). C is partially correct (stimulus control, wake time) but the medication taper is an important concurrent goal. D addresses only one component of CBT-I.

Chapter 24 — Sexual Dysfunctions and Gender Dysphoria

Overview

This chapter tests two distinct topic areas: (1) sexual dysfunctions — problems with desire, arousal, or orgasm that cause significant distress — and (2) gender dysphoria — distress arising from incongruence between gender identity and assigned sex. The exam's approach to gender dysphoria reflects an affirming model: the counselor's role is to support the client's self-identified gender, explore the distress, and facilitate access to care — not to question, challenge, or attempt to change the client's gender identity.

Sexual Dysfunctions Reference Table

Diagnosis	ICD-10	ICD-11	Core Feature
Female Sexual Interest/ Arousal Disorder	F52.22	HA00 / HA01.0	Reduced or absent interest in/arousal to sexual activity
Female Orgasmic Disorder	F52.31	HA02.0	Difficulty or inability to achieve orgasm
Genito-Pelvic Pain/ Penetration Disorder	F52.6	HA20	Difficulty with penetration; pelvic floor tension/pain; fear
Male Hypoactive Sexual Desire Disorder	F52.0	HA00	Persistently reduced/absent sexual desire

Diagnosis	ICD-10	ICD-11	Core Feature
Erectile Disorder	F52.21	HA01.1	Difficulty achieving/ maintaining erection
Delayed Ejaculation	F52.32	HA03.1	Marked delay or absence of ejaculation
Premature Ejaculation	F52.4	HA03.0	Ejaculation within ~1 minute of penetration, consistently

ICD-11 notes: Female Sexual Interest/Arousal Disorder: ICD-11 codes desire (HA00) and arousal (HA01.0) separately. Male Hypoactive Sexual Desire Disorder: the ICD-11 category is not sex-specific.

All sexual dysfunctions require symptoms that are persistent and recurrent, cause significant distress to the individual, and are not better explained by a nonsexual mental disorder, substance, medication, or medical condition.

Medical rule-out is essential: Many sexual dysfunctions have organic causes (hormonal, vascular, neurological, medication side effects). The counselor should always coordinate with a medical provider.

The Clinical Picture

Sexual Dysfunctions are a group of disorders characterized by significant disturbance in the ability to respond sexually or to experience sexual pleasure. DSM-5-TR distinguishes male-specific, female-specific, and gender-neutral sexual dysfunction diagnoses. Each must cause significant distress to the individual, persist for at least 6 months (except Substance/Medication-Induced Sexual Dysfunction), and not be better explained by a nonsexual mental disorder, relationship factors, substance effects, or inadequate stimulation.

Female Sexual Interest/Arousal Disorder (FSIAD): Absent or reduced sexual interest or arousal, as manifested by at least 3 of 6 criteria: reduced sexual activity, reduced erotic thoughts, reduced initiation, reduced excitement/pleasure, reduced genital sensations during sexual activity, reduced response to erotic stimuli.

Female Orgasmic Disorder: Delay, infrequency, or absence of orgasm; reduced intensity of orgasmic sensations. Present on all or almost all (approximately 75–100%) occasions of sexual activity for at least 6 months.

Genito-Pelvic Pain/Penetration Disorder (GPPPD): Consolidates vaginismus and dyspareunia from DSM-IV. Involves difficulty with vaginal penetration, marked vulvovaginal or pelvic pain during intercourse or penetration attempts, fear or anxiety about pain, and tensing of pelvic floor muscles.

Male Hypoactive Sexual Desire Disorder (MHSDD): Persistently reduced or absent sexual or erotic thoughts and desire for sexual activity.

Erectile Disorder: Difficulty obtaining or maintaining an erection, or marked decrease in erectile rigidity during sexual activity, on at least 75% of sexual encounters.

Delayed Ejaculation: Marked delay in or infrequent or absent ejaculation in approximately 75–100% of sexual encounters.

Premature (Early) Ejaculation: Pattern of ejaculation occurring within approximately 1 minute following penetration during partnered sexual activity, on approximately 75–100% of occasions.

Important specifiers for all: Lifelong vs. Acquired; Generalized vs. Situational; Mild/Moderate/Severe based on distress level.

Gender Dysphoria

DSM-5-TR Criteria: Marked incongruence between the person's experienced/expressed gender and their as-

signed gender, lasting at least 6 months, causing clinically significant distress or impairment (APA, 2022). In adolescents and adults, at least two of six specific criteria must be present (e.g., strong desire to be rid of primary/secondary sex characteristics, strong desire for the sex characteristics of another gender, strong desire to be treated as another gender, strong conviction that one has the typical feelings and reactions of another gender).

Key conceptual distinction for the exam: Distress is not inherent to having a transgender or gender-diverse identity. Distress often arises from external sources: family rejection, discrimination, lack of access to affirming care, violence risk, and barriers to transition. The counselor assesses the *source* of distress — is it the incongruence itself (dysphoria), or the social environment's response to the person's identity? This distinction guides intervention.

Affirming Stance: The NCMHCE models the affirmative approach throughout. The counselor: - Uses the client's preferred name and pronouns without qualification or exception - Validates gender identity as real, valid, and not subject to clinical "correction" - Explores the client's experience with curiosity and respect — never skepticism about the identity's authenticity - Assists the client in identifying their own goals (social transition, medical transition, or neither — all are clinically supported) - Addresses distress from rejection, discrimination, and barriers to care as primary clinical concerns - Never employs conversion or reparative practices — these are clinically harmful, ethically prohibited by the ACA and NBCC codes (American Counseling Association [ACA], 2014; NBCC, 2023), and legislatively banned in many states

WPATH Standards of Care, 8th Edition (SOC8): The World Professional Association for Transgender Health Standards of Care are the internationally recognized clinical guidelines for gender-affirming care (Coleman et al., 2022). Key points for the NCMHCE:

Role of the mental health professional: SOC8 moved substantially away from the gatekeeping model. Current guidance supports an **informed consent model** for many interventions (hormone therapy, social transition supports), with mental health professionals functioning as collaborative partners rather than approval authorities. The counselor's role is assessment, support, psychoeducation, and facilitation of access — not controlling who receives care.

When letters of support are requested: For certain surgical procedures, some providers still request documentation from a mental health professional. A letter of support includes: documentation of the client's gender identity and history of gender-related experiences; mental health history relevant to informed decision-making; confirmation that the client has persistent, well-documented gender incongruence; and that co-occurring mental health conditions, if present, are being appropriately treated and do not impair decision-making. The counselor does not "approve" the medical intervention — the medical provider does.

Transition pathways — all are valid and not sequential: Social transition (preferred name, pronouns, clothing, hair, legal documentation changes) is available at any age and requires no medical intervention or mental health approval. Hormone therapy (feminizing estrogen or masculinizing testosterone) requires informed consent and medical supervision; many providers use the informed consent model without requiring mental health letters. Gender-affirming surgeries are considered after the client has demonstrated sustained gender identity and meets specific criteria per SOC8.

Working with adolescents: SOC8 supports individualized, developmentally appropriate assessment with parental/guardian involvement when possible. Puberty-blocking hormones (GnRH agonists) and gender-affirming hormones may be considered for adolescents with persistent, well-documented gender incongruence. The counselor's

role is psychoeducation, exploration, and support — not determining whether the young person is "authentically" transgender.

Affirming stance (exam summary): The NCMHCE expects counselors to use the client's preferred pronouns and name, validate gender identity as real and valid, assess and support the client's own goals (which may include social transition, medical transition, or neither), and address distress caused by social rejection, discrimination, or dysphoria — not attempt to align the client's gender identity with their assigned sex.

Counselor role: Psychoeducation, support, advocacy, coordination with medical providers for clients seeking hormonal or surgical care (informed consent model or WPATH SOC8 framework), mental health assessment — collaborative partner, never gatekeeper.

What the Exam Rewards / What It Punishes

Rewards:

- Medical evaluation for sexual dysfunctions before initiating psychological treatment
- Affirming stance with gender-diverse clients (not questioning the validity of their identity)
- Assessment of distress source in gender dysphoria (is it the incongruence itself, or the social/family rejection around it?)

Punishes:

- Attempting conversion practices or attempting to change gender identity (clinically and ethically harmful)
- Diagnosing sexual dysfunction without ruling out medical and pharmacological causes
- Pathologizing gender diversity (non-conforming gender expression without distress is not a disorder)

Practice Questions

Question 80. A 17-year-old assigned female at birth presents with significant distress and says, “I have never felt like a girl. I feel like I’m in the wrong body, and my parents don’t believe me.” The counselor’s MOST appropriate initial response is:

A. Recommend waiting until age 18, when the client can consent independently, before discussing gender identity concerns

B. Explore the client’s experience with an affirming stance, assess distress, and provide a safe space for self-expression

C. Refer to a psychiatrist to assess for an underlying psychotic disorder before engaging with gender concerns

D. Contact the parents first to discuss the importance of their support and involvement in the client’s mental health treatment

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 55-year-old man reports new difficulty achieving erections over the past four months. He recently began a new antihypertensive medication and has poorly controlled diabetes. The most likely first consideration is:

A. Erectile disorder due to psychological factors, such as performance anxiety or relationship stress

B. A medical or medication-related contributor requiring rule-out before a primary psychiatric diagnosis

C. Male Hypoactive Sexual Desire Disorder, reflecting reduced sexual interest and motivation

D. Gender Dysphoria, as a possible source of his distress about sexual functioning

Answer: B. New onset in the context of vascular risk (diabetes) and a new antihypertensive points to a medical or medication etiology, which is ruled out before diagnosing

a primary sexual dysfunction. What would change the answer: a normal medical workup with a clear situational or psychological pattern → a psychologically based dysfunction.

Assessment — what to do next. The most appropriate step is:

A. Begin sensate-focus sex therapy immediately to reduce performance pressure and rebuild sexual confidence with his partner

B. Reassure him that it is purely stress-related and should resolve once his daily stress eases

C. Administer a personality inventory to identify traits that may contribute to his sexual difficulties

D. Refer for medical evaluation of the medication and vascular/endocrine contributors, then assess psychological factors

Answer: D. Medical and pharmacologic causes are ruled out first, then psychological factors are assessed. Beginning therapy (A) or offering reassurance (B) before the rule-out is premature.

Treatment Decision Flow

These items reward ruling out medical causes first and providing affirmative, ethical care.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Sexual dysfunction	Rule out medical and medication causes first; then sex therapy (e.g., sensate focus) and psychoeducation	Beginning psychological treatment before the medical rule-out
Suspected medication-induced dysfunction (e.g., SSRI)	Coordinate a referral to review the medication	Assuming the cause is purely psychological

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Gender dysphoria	Affirmative, informed-consent care; support exploration; refer per WPATH standards for medical steps	Conversion or “reparative” approaches; a purely gatekeeping stance

Sequencing & caveats. For sexual dysfunctions, medical and pharmacologic causes are ruled out before psychological treatment. For gender dysphoria, affirmative care is the standard. Avoid / contraindicated. Conversion or reparative therapy is unethical and harmful and is never an appropriate option. Scope. The counselor provides psychological support and refers for medical or hormonal care per accepted (WPATH) standards.

Differentiating Within the Family

Sexual Dysfunction vs. Normal Variation: Distress is the deciding criterion. Reduced sexual desire that does not distress the individual is not a disorder. A client who reports low desire but is satisfied with their sex life does not meet diagnostic criteria.

Sexual Dysfunction vs. Medical Etiology: Many sexual dysfunctions have medical contributors — diabetes (erectile and arousal disorders), hormonal imbalances (desire disorders), neurological conditions, cardiovascular disease. The DSM-5-TR requires ruling out medical causes before diagnosing a primary sexual dysfunction. A counselor’s response to sexual dysfunction always includes coordination with a physician.

Sexual Dysfunction vs. Substance/Medication-Induced: Many medications — particularly SSRIs, antipsychotics, antihypertensives, and hormonal contraceptives — cause sexual side effects. Substance use (alcohol, opioids) can cause acute and chronic sexual dysfunction. The counselor’s role is to flag this possibility and facilitate a medication review with the prescriber, not to adjust medication.

Sexual Dysfunction vs. Relationship Distress: The DSM-5-TR explicitly distinguishes sexual dysfunction from interpersonal difficulties that are not primarily sexual in nature. If sexual problems are fully explained by partner-directed distress, communication failures, or relationship conflict, a relational context diagnosis is more appropriate than an individual sexual dysfunction diagnosis.

FSIAD vs. Depression: Loss of sexual interest is a diagnostic criterion for a major depressive episode. If sexual disinterest is fully explained by depression, FSIAD is not additionally diagnosed. When sexual dysfunction persists after depression remits, independent diagnosis may be appropriate.

GPPPD vs. Somatic Symptom Disorder: GPPPD involves specific genitopelvic symptoms. Somatic Symptom Disorder is broader and not limited to pain during sexual activity. GPPPD may co-occur with chronic pelvic pain conditions that require pelvic floor physical therapy and gynecological evaluation.

Assessment Tools in Context

Female Sexual Function Index (FSFI): 19-item self-report measuring six domains of female sexual function (desire, arousal, lubrication, orgasm, satisfaction, pain) (Rosen et al., 2000). Total score ≤26.55 suggests female sexual dysfunction. Widely used in research and clinical settings.

International Index of Erectile Function (IIEF): 15-item self-report assessing male sexual function across five domains (erectile function, orgasmic function, sexual desire, intercourse satisfaction, overall satisfaction) (Rosen et al., 1997). IIEF-5 is the shorter clinical version (5 items focused on erectile function).

Male Sexual Health Questionnaire (MSHQ): Addresses multiple male sexual function domains including ejaculation, erection, and satisfaction (Rosen et al., 2004). Useful for tracking treatment response.

PROMIS Sexual Function and Satisfaction Measures: Patient-Reported Outcomes Measurement Information System measures for sexual function are available for males, females, and a gender-neutral version — important for gender-affirming practice (Flynn et al., 2013).

Exam tip: The NCMHCE tests the counselor's ability to address sexual concerns without imposing heteronormative assumptions, without pathologizing normal variation, and with awareness of the intersection between sexual dysfunction and medical conditions. Integrated sex-positive practice — validating, normalizing, coordinating with medical providers — is consistently rewarded.

Worked Mini-Case

Vignette: Darnell, 52, presents with a 14-month history of difficulty maintaining erections sufficient for satisfying intercourse. He reports morning erections "occasionally" and notes the problem occurs in all partnered sexual situations. He is mildly hypertensive and takes a beta-blocker. He and his husband are experiencing relationship strain related to the sexual difficulties. He is distressed and reports avoiding intimacy altogether.

Question: The MOST comprehensive first clinical response includes:

A. Provide psychoeducation about erectile dysfunction and recommend mindfulness-based sex therapy to reduce performance anxiety

B. Refer Darnell to a urologist, explore the potential contribution of his beta-blocker medication with his prescribing provider, and address the relationship distress in couples counseling

C. Diagnose Erectile Disorder and implement a sensate focus protocol, beginning with non-demand pleasuring exercises

D. Recommend Darnell discuss switching medications with his doctor, as SSRIs are known to cause sexual dysfunction

Best answer: B

Reasoning chain: Darnell's presentation requires multisystem thinking: (1) medical — beta-blockers are a known contributor to erectile dysfunction; morning erections suggest some vascular function is preserved, pointing toward psychogenic or medication-related factors; (2) relational — relationship strain is both a consequence and a maintaining factor; (3) counseling — ED distress and intimacy avoidance need clinical attention. B integrates all three domains without overstepping scope.

Why distractors lose: - A: Psychoeducation and mindfulness are useful but insufficient without medical evaluation — the beta-blocker must be assessed as a contributing factor first - C: Sensate focus is evidence-based, but jumping to a treatment protocol before medical causes are evaluated is clinically incomplete - D: Beta-blockers cause sexual dysfunction, not SSRIs in this case; furthermore, the counselor does not recommend medication changes — they facilitate a conversation between the client and their prescriber

Answer Key — Chapter 24 Practice Questions

Q80 — Answer: B

Rationale: An affirming, exploratory, compassionate response is the appropriate starting point. The counselor validates the client's experience and assesses the distress. A is not a clinical or ethical basis for deferring care. C pathologizes gender dysphoria (it is not a psychotic disorder). D may eventually involve the parents, but the immediate response to the client's disclosure is affirming engagement — not immediate parental contact.

Chapter 25 — Ethical and Legal Decision Points

Overview

Ethics is not a separate chapter on the NCMHCE — it is woven through every simulation. In NBCC's 2027 specifications, Legal & Ethical Compliance is tied for the largest domain, at 20% of items (NBCC, 2026). This chapter synthesizes the ethical principles, legal obligations, and decision-making frameworks that the exam tests in context. The exam never asks you to recite an ethical code; it tests ethical reasoning under realistic clinical pressure. The Five Core Ethical Principles (Beauchamp & Childress, 2019; Kitchener, 1984):

Principle	Meaning	Clinical Translation
Autonomy	The client's right to self-determination	Informed consent, voluntary participation, respecting client decisions
Beneficence	Act in the client's best interest	Choose interventions with the best evidence; prioritize client welfare
Nonmaleficence	Do no harm	Avoid harmful practices; don't exploit; manage dual relationships carefully
Justice	Fair and equal treatment	Nondiscrimination; equitable access to services
Fidelity	Honoring commitments; being trustworthy	Confidentiality, follow through on agreements, honest communication

ACA Code of Ethics (ACA, 2014) — Most-Tested Areas

Confidentiality and Its Limits

Confidentiality is the rule, and the exceptions are what the exam tests. Counselors explain the limits of confidentiality at the start of counseling and throughout it (ACA, 2014, Standard B.1.d). The main exceptions:

- **Serious and foreseeable harm**: imminent danger to self or others, including duty to warn or protect (ACA, 2014, Standard B.2.a; see Chapter 19)
- **Mandated reporting**: suspected abuse or neglect of a child, elder, or dependent adult
- **Legal requirements**: a court order or other legal mandate
- **Client waiver**: written authorization to release information
- **Consultation**: limited disclosure, with identifying information withheld whenever possible
- **Contagious, life-threatening disease**: counselors *may* disclose to an identifiable third party at high risk if the client will not inform them, following state law (ACA, 2014, Standard B.2.c)
- **Group, couple, and family counseling**: the counselor cannot guarantee that other members will keep what is shared confidential

Minors and clients who lack capacity. Counselors protect the confidentiality of minors and adults who lack capacity as far as the law allows. They also respect the legal rights of parents and guardians, and they get the appropriate third party's permission before releasing information (ACA, 2014, Standard B.5). Where a minor cannot give legal consent, the counselor seeks the minor's assent (ACA, 2014, Standard A.2.d).

Informed Consent in Depth

Informed consent is ongoing, not a one-time form (ACA, 2014, Standard A.2.a). Clients have the right to choose whether to enter or stay in counseling, and they need enough information to make that choice. The ACA Code lists what clients must be told (ACA, 2014, Standard A. 2.b):

Element	What it covers
Nature of services	Purposes, goals, techniques, procedures, limitations, potential risks, and benefits
The counselor	Qualifications, credentials, relevant experience, and approach to counseling
Continuity	What happens if the counselor becomes incapacitated or dies
Technology	The role of technology in the services
Fees and billing	Costs, billing arrangements, and consequences of nonpayment
Confidentiality	Its limits, including third-party payers and supervision
Client rights	Access to records, participation in planning, and the right to refuse services and to know the consequences of refusing

Consent must be communicated in a developmentally and culturally appropriate way, with a qualified interpreter or translator when needed (ACA, 2014, Standard A.2.c). A client who signed a form they could not understand has not given informed consent.

Dual Relationships and Boundaries

A dual relationship exists when the counselor has a professional role *and* another role with the same client, whether social, familial, financial, or romantic. The ACA Code prohibits relationships that harm the client's wel-

fare or impair the counselor's objectivity. The violations the exam tests most:

- **Sexual or romantic relationships**: always prohibited with current clients, their romantic partners, and their family members. They are also prohibited with former clients for five years after the last professional contact, and even then only if the relationship is not exploitive (ACA, 2014, Standard A.5.c).
- **Bartering**: permitted only when the client requests it, it is not exploitive or harmful, it is an accepted practice among professionals in the community, and the arrangement is put in writing (ACA, 2014, Standard A.10.e).
- **Social media contact**: counselors keep professional and personal online presences separate and do not seek out clients' personal social media (ACA, 2014, Standard H.6).
- **Self-disclosure** that serves the counselor instead of the client.

Some boundary extensions (attending a client's graduation, visiting a hospitalized client) can be appropriate. The counselor weighs the risks and benefits beforehand and documents the reasoning (ACA, 2014, Standard A.6.b).

Competence, Consultation, and Referral

Counselors practice only within the boundaries of their competence, which rests on education, training, supervised experience, credentials, and professional experience (ACA, 2014, Standard C.2.a). They continually monitor their own effectiveness (ACA, 2014, Standard C.2.d). When a client's needs fall outside the counselor's competence, the options are to consult, seek training while under supervision, or refer.

Referral must rest on competence, never on the counselor's personal values. Counselors may not refer a client solely because the counselor's beliefs, attitudes, or values conflict with the client's (ACA, 2014, Standard A.11.b).

Referrals are coordinated: the counselor provides appropriate resources and makes sure the clinical and administrative handoff is complete (ACA, 2014, Standard A.11.d).

Documentation and Records

Counselors create, safeguard, and maintain documentation of the services they provide (ACA, 2014, Standard A.1.b). Records exist to support continuity of care, to meet legal and regulatory requirements, and to show the counselor's clinical reasoning.

What a sound progress note contains: the date and type of service; the client's current presentation and any change in risk; the interventions used; the client's response; progress toward treatment plan objectives; and the plan for next steps. Common formats are SOAP (Subjective, Objective, Assessment, Plan) and DAP (Data, Assessment, Plan).

Rules the exam tests:

- Document risk assessments, consultations, and the reasoning behind high-stakes decisions such as duty to warn, hospitalization, and mandated reports.
- Never alter or backdate a record. Corrections are made as dated addenda.
- Store records securely, keep them for the period state law and regulations require, and dispose of them in a way that protects confidentiality (ACA, 2014, Standard B.6.h).
- Under HIPAA, mental health records are protected health information, and their release requires written authorization with specific exceptions (U.S. Department of Health and Human Services [HHS], 2025). Psychotherapy notes kept separately from the medical record have heightened protection.
- Substance use disorder treatment records at federally assisted programs carry additional federal confidentiality protections (Confidentiality of Substance Use Disorder Patient Records, 2024).

Distance Counseling, Telehealth, and Technology

NBCC's 2027 specifications list informed consent for telemental health as an intake task and adherence to telemental health best practices and platform compliance as legal and ethical compliance tasks (NBCC, 2026), and Section H of the ACA Code governs it. The exam rewards counselors who handle the practical and legal details *before* the first session:

- **Licensure and jurisdiction**: know the laws where the counselor practices *and* where the client is located during the session (ACA, 2014, Standard H.1.b). A counselor generally needs authorization to practice in the client's state.
- **Informed consent specific to distance counseling**: how to reach the counselor, the risks and benefits of the technology, what happens if the technology fails, expected response times, emergency procedures, time zone and cultural or language differences, possible denial of insurance coverage, and the social media policy (ACA, 2014, Standard H.2.a).
- **Emergency planning**: at intake, record the client's physical location, a local emergency contact, and local crisis resources. Confirm the client's location at the start of each session.
- **Client verification**: confirm the client's identity at the start of counseling and throughout it (ACA, 2014, Standard H.3).
- **Security and privacy**: use secure, encrypted platforms, and tell clients about the limits of electronic confidentiality (ACA, 2014, Standards H.2.c–H.2.d).
- **Suitability**: assess whether distance counseling fits the client's needs, and switch to in-person services when it does not, for example in a high-risk crisis or when a private space is unavailable.

Artificial Intelligence in Clinical Practice

AI tools are now ordinary in counseling settings: note-takers that record sessions and draft progress notes,

scheduling and billing assistants, screening and symptom-tracking apps, and consumer chatbots that clients use on their own. NBCC's 2027 specifications name AI explicitly: training in the ethical and legal use of AI, implementing technology-assisted tools such as AI note takers, and using AI within counseling practice are all listed work tasks (NBCC, 2026). The 2014 ACA Code predates AI, so expect AI items to turn on standards that already exist: Section H on technology, Section B on confidentiality, A.2 on informed consent, and C.2 on competence (ACA, 2014).

What the exam rewards:

- **Disclosure and consent.** Tell the client when an AI tool is used, what it does, and the risks; obtain consent; proceed without the tool if the client declines. An AI note-taker that records sessions is part of informed consent, not an office procedure.
- **Clinical judgment stays human.** The counselor is responsible for every clinical decision and every note, including AI-drafted ones. AI output is never the sole basis for a diagnosis, an assessment, or a risk decision. Review and correct before signing.
- **Privacy and security.** Use only tools that meet HIPAA requirements, with a business associate agreement where one is required. Never enter identifiable client information into a consumer AI tool that was not built for protected health information.
- **Bias and competence.** AI carries the bias of the data it was trained on, with greater risk of harm to marginalized clients. Understand a tool's limits before using it (ACA, 2014, Standard C.2.a), and read its output critically.
- **The relationship comes first.** AI supports counseling; it does not replace the therapeutic relationship.
- **Clients' own AI use.** Ask about it. Some clients use chatbots for emotional support between sessions. Explore the benefits and the risks — privacy, accuracy,

and above all the absence of crisis response — and build human contacts and 988 into the safety plan.

How this shows up on the exam: A stem that mentions an AI note-taker, a chatbot, or an app is a technology-and-consent item. The rewarded answer discloses the tool, obtains consent, keeps the counselor responsible for the clinical decision, and uses a HIPAA-compliant platform. The punished answer treats AI output as the assessment, enters client information into a consumer tool, or lets a chatbot stand in for crisis response.

Counselor Self-Care and Impairment

The ACA Code states that counselors engage in self-care to maintain their emotional, physical, mental, and spiritual well-being (ACA, 2014). It also requires counselors to monitor themselves for impairment and to limit, suspend, or end professional responsibilities when impaired, until it is safe to resume (ACA, 2014, Standard C.2.g).

Concept	What it is	Signs
Burnout	Exhaustion and reduced effectiveness from chronic workload and organizational stress	Cynicism, detachment, dreading sessions, reduced sense of accomplishment
Compassion fatigue	Emotional depletion from ongoing exposure to clients' suffering	Numbness, irritability, reduced empathy
Vicarious trauma	Lasting changes in the counselor's worldview from repeated exposure to trauma narratives	Changed beliefs about safety, trust, or control; intrusive images
Impairment	Functioning compromised enough to risk harm to clients	Errors, boundary lapses, substance misuse, inability to focus

How this shows up on the exam: When the counselor in a stem is exhausted, grieving, ill, or distressed, the rewarded answer is to seek supervision or consultation, ar-

range coverage or referral if functioning is compromised, and pursue their own care. Continuing to see clients while impaired is always wrong. So is abruptly abandoning clients without arranging transfer.

Accommodations and Accessibility

NBCC lists providing adequate accommodations for clients with disabilities as a legal and ethical compliance task (NBCC, 2026). Under the Americans With Disabilities Act of 1990 (1990), counseling services must be accessible to people with disabilities. In practice this means:

- Accessible physical space and materials, and accessible websites for services offered electronically (ACA, 2014, Standard H.5)
- Qualified sign language or spoken-language interpreters, not family members, especially children, for clinical communication
- Adapting pacing, format, and written materials for cognitive, sensory, or learning differences
- Asking the client what accommodations they need rather than assuming

Termination and Abandonment

Counselors may not abandon clients. Termination is planned, with adequate notice and referrals as needed. Counselors end counseling when the client no longer needs help, is not benefiting, or is being harmed by continued counseling. They may also end it when the counselor is in danger from the client or someone close to the client, or when the client does not pay agreed fees. In each case the counselor provides pre-termination counseling and recommends other providers when necessary (ACA, 2014, Standard A.11.c). Stopping treatment without any of this planning is abandonment, and it is an ethical violation.

The Ethical Decision-Making Model

When an ethical dilemma arises on the NCMHCE, apply this sequence:

1. **Identify the dilemma**: What are the competing obligations?
2. **Consult the code**: What does the ACA Code say?
3. **Apply ethical principles**: Which principle is most relevant?
4. **Consider consequences**: What are the likely outcomes of each option?
5. **Consult**: With a supervisor or colleague. This is almost always the right answer when you are uncertain.
6. **Document**: The reasoning, the decision, and the outcome.

For the formal published model, see Forester-Miller and Davis (2016), which the supplemental questions also test.

What the Exam Rewards / What It Punishes

Rewards:

- Consulting with a supervisor when uncertain about ethical decisions
- Taking a conservative approach to confidentiality (default to protect)
- Recognizing dual relationship risks and acting proactively to maintain boundaries
- Reporting abuse based on reasonable suspicion, not certainty
- Providing referrals when terminating
- Completing distance-counseling informed consent, jurisdiction checks, and an emergency plan before the first telehealth session
- Documenting risk assessments, consultations, and the reasoning behind high-stakes decisions
- Seeking supervision and arranging coverage when personally impaired

Punishes:

- Acting on a duty-to-warn breach without first attempting to address the risk within the therapeutic relationship (where clinically possible)
- Assuming HIPAA releases cover psychotherapy notes (they have separate protection)
- Providing only a "no-harm contract" without a risk assessment as an ethical response to suicidality
- Maintaining a relationship the counselor knows has become exploitive because "the client wants to continue"

Worked Mini-Case — Mandatory Reporting Threshold

Vignette: Priya is a licensed professional counselor working in a community mental health agency. She has been seeing Theo, 34, for 10 months for generalized anxiety. During a session, Theo discloses that he has been having sexual thoughts about his 9-year-old niece and has not acted on them. He insists the thoughts are ego-dystonic (he is horrified by them) and says he would never act on them. He asks Priya to keep this "strictly confidential" and says he will stop therapy if she reports it.

Question: Priya's MOST appropriate ethical and legal course of action is:

A. Maintain confidentiality, as Theo has not acted on the thoughts and they are ego-dystonic; continue to explore the thoughts in a future session

B. Report to Child Protective Services immediately, because any disclosed sexual thoughts about a specific minor automatically meet the reasonable-suspicion threshold for a mandated report

C. Consult with a supervisor and/or ethics consultant to assess whether the disclosure meets the threshold for mandated reporting under state law, while continuing to maintain the therapeutic relationship

D. Terminate with Theo immediately and refer him to a specialized provider, because sexual thoughts about a child fall outside Priya's clinical scope and training

Answer: C.

Reasoning chain: This is a nuanced mandatory reporting vignette. Most states' mandated reporting laws require a "reasonable suspicion of abuse" — past, current, or imminent. Thoughts alone, without behavioral indicators or a specific plan and intent, may not meet the legal threshold in many jurisdictions. However, the clinician cannot make this determination alone. Consultation — with a supervisor, ethics board, or legal counsel — is the appropriate step before reporting or declining to report. Priya should NOT promise confidentiality about reportable content, but she also should not report prematurely.

Why the distractors lose: - A: Maintaining confidentiality without consultation is professionally and legally precarious; the counselor has an obligation to understand their reporting duties in this situation - B: Immediately reporting may be the outcome of consultation, but reporting without consultation assumes a threshold that has not been established; impulsive reporting without clinical judgment can rupture the alliance prematurely in a case where ongoing monitoring may actually protect the child better - D: Terminating abruptly mid-process — even with a referral — raises abandonment concerns and misreads a consultation question as a scope problem; if Theo disengages from therapy, the opportunity to monitor and intervene is lost, which may increase risk to his niece

Key ethical principles at play: - Beneficence and Nonmaleficence: The counselor must protect both Theo's well-being (therapeutic relationship) and the potential victim (child safety) - Fidelity: Confidentiality cannot be promised regarding potentially reportable material - Justice: Equitable and legally informed application of mandatory reporting laws - Autonomy: Theo's autonomy does not supersede the potential protection of a minor

Practice Management and Compliance

2027 task map: 6.1 Activate (i.e., follow) an emergency preparedness plan for your counseling practice in response to a relevant incident · 6.2 Adhere to internal policies or standard operating procedures (SOPs) relevant to your clinical practice (e.g., informed consent, record retention, release of information, mandated reporting protocols) · 6.3 Adhere to relevant state and federal laws and regulations that apply to your clinical practice · 6.4 Adhere to telemental health best practice guidelines · 6.5 Adhere to the counseling codes of ethics · 6.6 Engage in activities to maintain licensure status · 6.7 Engage in billing, third-party reimbursement and other insurance panel processes · 6.8 Engage in compliance-related training (e.g., insurance, credentialing, billing, payment) · 6.9 Ensure telemental health platforms are utilized in compliance with state and federal laws · 6.10 Ensure that reimbursement requests are supported by proper clinical documentation · 6.11 Implement technology-assisted clinical tools (e.g., online therapy platform, AI note takers, virtual reality tools, calming apps, wearable devices to monitor heart rate and breathing) · 6.12 Maintain professional liability insurance · 6.13 Practice in accordance with license portability (e.g., Counseling Compact) · 6.14 Provide adequate accommodations for clients with disabilities · 6.15 Review and update an emergency preparedness plan for your counseling practice · 6.16 Securely maintain case notes, records, and files · 6.17 Solicit best practices for understanding and adhering to obligations of court proceedings · 6.18 Use an electronic health record (EHR) or electronic medical record (EMR) system · 6.19 Use artificial intelligence (AI) within counseling practice · 6.20 Market your counseling business appropriately (e.g., proper utilization of social media, proper protection of client information)

The Practice Is Part of the Care

A claim billed under the wrong code, a chart left on a desk, a video link anyone can open, or a fire alarm treated

as a nuisance can hurt clients as surely as a poor clinical choice. The 2027 outlines list twenty tasks about running a lawful, safe practice.

These items share one test: **which rule owns this situation, and does the answer follow it rather than convenience, comfort, or a myth?** Wrong answers fall into five patterns:

1. **Convenience over safeguards.** A personal phone, a free app, a shared login, or an emailed draft.
2. **Consent used as a cure-all.** A client's signature cannot waive licensure law, turn a consumer app into a compliant one, or make a forensic opinion about one's own client appropriate.
3. **One rule mistaken for another.** A subpoena treated as a court order, an authorization treated as the privacy notice, or a building plan treated as the practice plan.
4. **Payment shaping the record.** A diagnosis chosen for coverage, a code chosen for the fee, or a note written after the claim.
5. **Improvising instead of following the plan.** The practice wrote a procedure for this incident, and the counselor ignores it.

This section builds on the ethics material and adds the practice-business rules the 2027 tasks name. For the professional will, see the continuity-of-care section.

Part 1 — Who Owns It

Owner	What it owns
Practice emergency preparedness plan	Incident procedures, client notification, continuity of care
Building evacuation plan	Exits and alarms; **not** clients, records, or continuity
HIPAA contingency plan, 45 CFR 164.308(a)(7)	Data backup, disaster recovery, emergency mode operation

Owner	What it owns
HIPAA Privacy Rule	Uses and disclosures of protected health information (PHI); clients' rights
45 CFR 164.506	**Treatment, payment, and health care operations (TPO)** without authorization
45 CFR 164.508	**Authorization** for uses and disclosures outside TPO; **164.508(a)(2)** psychotherapy notes; **164.508(a)(3)** marketing
45 CFR 164.502(b), 164.514(d)	**Minimum necessary**; role-based access
45 CFR 164.502(e), 164.504(e)	**Business associate agreement (BAA)**
45 CFR 164.514(b)	**De-identification** (safe harbor or expert determination)
45 CFR 164.520	**Notice of Privacy Practices (NPP)**; **164.520(c)** how it is provided and posted
45 CFR 164.522(a)	Right to request **restrictions**; **(a)(1)(vi)** mandatory restriction when the client pays in full out of pocket
45 CFR 164.522(b)	Right to request **confidential communications**
45 CFR 164.524	**Right of access** to the designated record set
45 CFR 164.526	**Right to amend**
45 CFR 164.528	**Accounting of disclosures**
45 CFR 164.530(c)	**Safeguards** for PHI in any form, including disposal
45 CFR 164.512(e)	Disclosures for **judicial and administrative proceedings** (court orders, subpoenas)
HIPAA Security Rule	Electronic PHI: **164.310** physical safeguards, **164.312(a)(1)** access control, **164.312(a)(2)(i)** unique user ID, **164.312(b)** audit controls, **164.312(e)** transmission security; training (164.308(a)(5))

Owner	What it owns
HIPAA Breach Notification Rule, 45 CFR 164.402, 164.404, 164.410	Presumption of breach (**2013 Omnibus Rule**, replacing "risk of harm"); client notice within 60 days; business associate's duty to notify the practice
HHS Office for Civil Rights (OCR) telehealth notice	Pandemic leniency for consumer video apps; **expired August 9, 2023**
FERPA	School education records; excluded from HIPAA
42 CFR Part 2	Federally assisted substance use disorder program records
State law	Licensure, retention, recording consent, reporting, privilege, stricter privacy rules
ACA Code of Ethics (ACA, 2014)	**A.1.b** records; **A.2.a–A.2.d** consent; **A.10.b–A.10.c** referral fees, fees; **A.11.c, A.12** termination, abandonment; **B. 1.c, B.2.d, B.2.e, B.3.e, B.6.b–B.6.h, B.7.b** confidentiality, court orders, records; **C.1, C.2.f, C.2.h** standards, CE, incapacitation plan; **C.3.a–C.4.b** advertising, credentials; **C.5** nondiscrimination; **C.6.b, C.6.e** third-party reports, pro bono; **D. 1.g** employer policies; **E.5.a, E. 13.c** diagnosis, no forensic evaluation of clients; **H.1.b–H. 6.d** distance counseling and social media; **I.1.a, I.1.c** ethics and law
NBCC Code of Ethics (NBCC, 2023)	Ethics for National Certified Counselors
ACA AI Work Group recommendations (ACA, n.d.)	AI practice guidance; the 2014 Code does **not** name AI
State licensure board	Renewal, CE, audits, reporting deadlines, supervision, complaints
NBCC national certification (NCC)	A **voluntary** credential, separate from the license

Owner	What it owns
Counseling Compact	**Privilege to practice** in member states based on an unencumbered home-state license
NPI Type 1 / Type 2; NPPES	Identifiers for individuals (Type 1) and organizations (Type 2), issued through NPPES
CAQH ProView	Shared payer credentialing profile
National Practitioner Data Bank (NPDB)	Malpractice payments and adverse actions
DEA registration	Controlled substances; not a counseling identifier
Medicare MHC enrollment (Consolidated Appropriations Act, 2023)	Direct Medicare billing by mental health counselors from January 1, 2024
Medicare rules	Mandatory claim submission, opt-out, “incident to,” locum tenens, Advance Beneficiary Notice (ABN), telehealth payment flexibilities
No Surprises Act, 45 CFR 149.610	**Good faith estimate** for uninsured and self-pay clients
CPT psychotherapy codes	90832, 90834, 90837 (time), 90839/90840 (crisis), 90846/90847 (family), 90785 (interactive complexity)
Fraud laws; OIG General Compliance Program Guidance (OIG, 2023)	False claims, copay waivers; compliance program elements; HIPAA training (164.530(b))
Professional liability policy types	Occurrence, claims-made, tail, prior acts; versus general, umbrella, property, cyber
Respondeat superior	Employer liability for an employee’s acts; the employee can still be named
ADA Title III, 28 CFR 36.301(c), 36.302, 36.303	No surcharges; reasonable modifications; auxiliary aids and companion-interpreter limits (36.303(c))
Beck (2021)	CBT homework: rehearsed, linked to targets, reviewed

Owner	What it owns
Automation bias (Parasuraman & Manzey, 2010)	Over-trusting machine output; the counselor stays accountable

Part 2 — Emergency Preparedness (6.1, 6.15)

Three Plans, Three Owners

The exam separates three plans that candidates blur together.

Plan	Owner	What it covers	What it does not cover
Practice emergency preparedness plan	The practice's written policy	Each incident type, roles, staying with the client, client notification, alternate site, continuity of care	Building systems
Building evacuation plan	Landlord or building manager	Exits, alarms, assembly points for the whole building	Clients, records, continuity of care
HIPAA contingency plan	45 CFR 164.308(a)(7)	**Data backup**, **disaster recovery**, and **emergency mode operation** so electronic records survive and stay protected	People's physical safety

A building plan never replaces the practice plan, and **insurance never replaces the contingency plan**; policies pay claims but restore no records.

Activating the Plan (6.1)

The incident activates the plan. The counselor's private judgment about the incident does not. An alarm, a warning, or an announcement is enough. Waiting for smoke, a funnel cloud, or proof of an intruder is the error.

Incident	Plan procedure	Wrong-procedure trap
Fire alarm	**Evacuate**: leave with the client by the posted route to the meeting point; document the disruption afterward	Finishing the session; securing charts first; sending the client out alone
Tornado or severe-weather warning	**Shelter in place**: move with the client to the designated shelter area; check in afterward	Sending the client to drive home; continuing because "most warnings pass"
Lockdown (reported armed intruder)	**Lock down**: lock the door, lights off, stay out of sight with the client, away from doors and windows, until the all-clear	Evacuating to the parking lot; stepping into the hall to "check"
Medical emergency (new chest pain spreading to the left arm, collapse, seizure)	**Call 911**, stay with the client, have staff meet responders	Grounding exercises; "see your doctor after session"; letting the client drive
Power or EHR outage	Confirm safety, then the plan's **downtime procedures** (secured paper forms, or reschedule with a set time)	Typing PHI into a personal phone app; no return date; leaving the client alone in the dark
Office closed (overnight fire, flood)	**Notify clients** through listed contact methods; continue **securely** from the backup site or compliant telehealth; document	Canceling indefinitely; waiting for clients to call; a free consumer app "because it's an emergency"

Error patterns: **normalcy bias** (a real warning treated as nothing), **verification delay** (personal confirmation before following an announced plan), and **diagnostic overshadowing** (new physical symptoms explained away by a known diagnosis; new chest pain in a client with panic or somatic symptoms is a medical emergency until proven otherwise).

Two rules settle most items: **people before paper**, and **stay with the client** unless the plan says otherwise. Emergencies do not suspend privacy law, so care after a disaster continues **securely**.

Reviewing and Updating the Plan (6.15)

A plan is reviewed **on a schedule and whenever conditions change.**

Triggers for review: an office move, a drill or incident that exposed a gap, new staff, a new EHR or service such as telehealth, new hazards, or changed contacts.

What a complete plan contains: - An **all-hazards** risk assessment and a procedure for each incident type, with roles named - A **communication plan**, with a client contact list kept securely and reachable off site - An **alternate site** and secure telehealth option - **Encrypted offsite backups** and a records-access procedure (the contingency plan) - A **professional will** naming a colleague or records custodian (**ACA C.2.h**). Next of kin gain no right to open client files (**B.1.c**). A consent-form line saying services may end is disclosure (**A.2.b**), not a plan. - Training and drills, so staff do not improvise

A plan that closes the practice until things are rebuilt is not a plan. Stopping services without arranging care risks **abandonment (A.12)**.

Part 3 — Standard Operating Procedures and Codes of Ethics (6.2, 6.5)

Why SOPs Matter

When an item says "the practice has a written protocol," the right answer almost always **follows it and documents each step.** Accepting a job implies agreement with the employer's general policies; counselors work to change poor policies rather than ignore them (**ACA D. 1.g**).

SOP	What it requires	Common wrong answer
Informed consent	Before a new procedure, review purpose, techniques, expected distress, alternatives, and the right to refuse; document consent (**A.2.a–A.2.b**). A competent adult consents for himself.	**Retroactive consent** ("once it's working"); the **evidence-base exemption myth** ("it's evidence-based, so no consent needed"); a partner signing for a capable adult
Record retention	Keep records for the period state law and policy require, then destroy them securely. When a client asks for destruction, explain the policy and safeguards.	Shredding on request (the **record-ownership myth**); rewriting notes to suit the client; keeping "fuller" notes at home
Release of information (ROI)	A written release naming the recipient, the information, the purpose, and an expiration (**B.6.g**; HIPAA **164.508** where an authorization is required); send only what is needed (**B. 2.e**)	Acting on a verbal okay; sending the whole chart; refusing to coordinate with medical providers at all

SOP	What it requires	Common wrong answer
Mandated reporting protocol	Who reports, how fast, to which agency, how it is documented, and when to consult. State law sets the duty and threshold.	Delaying while the client "processes"; promising confidentiality the counselor cannot keep
At-risk no-show protocol	Timely outreach to a client with recent suicidal ideation through **consented** contact methods, escalating per the protocol's steps; document each step	Waiting for the client to call ("respecting autonomy"); calling the workplace; jumping straight to a welfare check
Between-session contact policy	Response limits and secure channels. Repeated reassurance-seeking messages are reviewed against the policy and brought into session as clinical material.	Detailed reassurance replies, which feed the anxiety cycle; going silent without explanation; giving out a personal cell number
Recording policy	Written, informed permission before any recording, covering purpose, who will hear it, storage, and the right to decline (**B. 6.c**); state law may add consent rules	Covert recording "just for consultation"; a verbal okay where policy requires written consent; emailing files to a consultant

Codes of Ethics (6.5)

Counselors are bound by the **ACA Code of Ethics (ACA, 2014)**. National Certified Counselors are also bound by the **NBCC Code of Ethics (NBCC, 2023)**. Most state boards adopt a code by rule, so an ethics violation can become a licensure violation.

- **C.1:** counselors read, understand, and follow the ACA Code and applicable law.

- **I.1.a:** not knowing or misunderstanding an ethical responsibility is **not a defense**.
- **I.1.c:** when ethics and law conflict, counselors make their commitment to the Code known and try to resolve the conflict. If they cannot, they may follow the law.

Part 4 — Federal and State Privacy Law (6.3)

HIPAA in One Table

HIPAA applies to **covered entities**, including providers who send standard electronic transactions such as claims. A cash-only counselor may fall outside HIPAA but is still bound by state law and ethics; exam items generally assume HIPAA applies.

Rule	What it governs	Key sections
Privacy Rule	PHI in **any form**; client rights	164.502–164.528, 164.530
Security Rule	**Electronic** PHI: administrative, physical, and technical safeguards	164.308, 164.310, 164.312
Breach Notification Rule	What counts as a breach; who must be notified, and when	164.400–164.414 (164.402, 164.404, 164.410)

HIPAA is a **floor**; a stricter state privacy law still applies.

Uses, Disclosures, and the Special Protection for Psychotherapy Notes

Concept	Rule
TPO (164.506)	Treatment, payment, and health care operations need no authorization. Ethics and many state laws still expect client consent, so practices usually get a release anyway.

Concept	Rule
Authorization (164.508)	Required for uses and disclosures **outside** TPO and outside other permitted purposes. An authorization is **not** the privacy notice, and it is not the form for choosing how the client is contacted.
Minimum necessary (164.502(b), 164.514(d))	Use, disclose, and request only what the purpose needs; staff access records only as their role requires. It does **not** apply to disclosures for treatment, to the client, or under an authorization.
Psychotherapy notes (164.508(a)(2))	The counselor's separately kept notes analyzing session content. They need a **specific authorization** for most disclosures, **even to a health plan for payment**. A plan may **not** condition payment or eligibility on the client signing it.

What psychotherapy notes are not: medications, session start and stop times, modalities and frequency, test results, and summaries of diagnosis, functional status, plan, symptoms, prognosis, and progress. Those belong to the regular record. Minimum necessary narrows a permitted disclosure; **it never substitutes for the authorization psychotherapy notes require**, and neither does a third party's confidentiality pledge.

Client Rights Under HIPAA

Right	Section	What the counselor does	Trap
Notice of Privacy Practices	164.520	Give the NPP no later than the first service; make a **good-faith effort** to obtain written acknowledgment; also post it in the office and on any practice website. If one was missed, give it now and document it.	Relying on the waiting-room posting (**164.520(c)** requires both); treating an authorization as the NPP; **withholding care** until the acknowledgment is signed
Access	164.524	Provide access to the **designated record set** in a timely way, **currently within 30 days with one 30-day extension**, or sooner if state law requires. Separately kept **psychotherapy notes are excluded.** A summary may replace the record **only if the client agrees in advance.**	Requiring a subpoena; requiring a reason; including psychotherapy notes; substituting a summary on the counselor's own initiative

Right	Section	What the counselor does	Trap
Amendment	164.526	Respond **in writing**, generally within **60 days**. Amendment of **accurate and complete** information may be denied, with an explanation and the client's right to file a **statement of disagreement**.	Deleting an accurate diagnosis; ignoring the request; referring the client to the licensing board, which handles complaints, not amendments
Accounting of disclosures	164.528	List certain disclosures to others, generally within **60 days**. The list **excludes** TPO disclosures, disclosures to the client, and disclosures the client authorized. The **first accounting in any 12 months is free.**	Treating it as an access request; refusing because logs are "internal"; charging for the first one
Restrictions	164.522(a)	Clients may ask to limit uses and disclosures; the provider usually need not agree. **Exception (164.522(a)(1)(vi)):** the provider **must** agree not to disclose to a health plan for services the client **paid for in full out of pocket.**	Billing the plan anyway (the **mandatory-submission myth**); agreeing to self-pay and then submitting claims

Right	Section	What the counselor does	Trap
Confidential communications	164.522(b)	**Providers** must accommodate reasonable requests to be contacted by other means or at other locations **without asking why. Health plans** must accommodate such a request when the client says disclosure could **endanger** them.	Requiring a reason from the client; applying the plan's endangerment standard to a provider; stopping all electronic contact

The audit log a system keeps for security (**164.312(b)**) is not the accounting of disclosures. Each is a different duty.

Business Associates and De-Identification

A **business associate** creates, receives, stores, or transmits PHI for the practice: a telehealth platform, EHR vendor, billing service, cloud backup, or AI note-taker. It needs a **business associate agreement (164.502(e), 164.504(e))** before handling PHI. A BAA sets the vendor's duties. **It does not configure the practice's settings, and it does not replace client consent** to recording or AI processing.

De-identified information is no longer PHI. HIPAA recognizes two methods (**164.514(b)**): the **safe harbor** method, which removes specified identifiers with no actual knowledge that the rest could identify the person, and **expert determination**. Removing a last name is not de-identification; details, dates, and rare facts identify people. In consultation, counselors withhold identifying information unless the client consents or disclosure cannot be avoided (**ACA B.7.b**).

Breach Notification

Step	Rule
Presumption (164.402)	An impermissible use or disclosure is **presumed a breach** unless a documented risk assessment shows a **low probability that the PHI was compromised**. The assessment weighs the nature and extent of the PHI, who received it, whether it was actually viewed or acquired, and how far the risk was mitigated.
History	The **2013 Omnibus Rule** replaced the "risk of harm" standard; proof of misuse is **not** required.
Clients (164.404)	Notice **without unreasonable delay** and **no later than 60 days** after discovery
Business associates (164.410)	The vendor must notify the practice. **The practice remains responsible** for making sure affected clients are notified.
Others	HHS must also be notified, and media notice applies to large breaches. State breach laws may add duties.
Encryption	PHI encrypted to HHS standards is not "unsecured," so losing an encrypted device generally does not trigger notice.

FERPA, HIPAA, and 42 CFR Part 2

Law	Applies to	Exam point
FERPA	**Education records** at schools receiving federal education funds, including a school-employed K–12 counselor's records	**Excluded from HIPAA**. Parents generally have access rights. Private memory-aid notes kept solely by the counselor are treated differently.

Law	Applies to	Exam point
HIPAA	Covered providers, including a community counselor who sees a student	A school referral does **not** make a private counselor's records FERPA records.
42 CFR Part 2	Records of **federally assisted substance use disorder programs**	Stricter than HIPAA; generally requires patient consent and limits use of records in proceedings against the patient. A 2024 rule aligned parts of Part 2 with HIPAA; verify current requirements.

Part 5 — Telemental Health (6.4, 6.9)

Before the First Video Session (6.4)

1. **Suitability.** Can this client use the technology, and is distance counseling appropriate for their needs (**ACA H.4.c**)? A diagnosis alone, such as a substance use disorder, does not rule telehealth out. High acute risk or no private space may rule it out for now.
2. **Jurisdiction. ACA H.1.b**: distance counselors may be subject to the laws of both their own location and the client's place of residence. Boards generally regulate telehealth **where the client is located during the session**, so the counselor needs authorization there.
3. **Distance-counseling informed consent (H.2.a).** Cover risks and benefits, what happens if the technology fails, response times, emergency procedures, privacy limits, and possible insurance differences. **The intake consent does not automatically cover video.**
4. **Identity (H.3).** Verify who is on the screen, at the start and throughout.
5. **Emergency plan.** The client's usual location, a local emergency contact, and **crisis resources local to the**

client. Your own city's 911 cannot reach a client three states away.

6. **Disconnection plan.** If the call drops, call the backup number; if a high-risk client cannot be reached, follow the safety protocol.

Intimate partner violence changes the setup. Before going to video, confirm a private space and a **device the partner cannot access**, agree on a code word and interruption plan, and use only safe channels for links. In-person sessions may be safer.

At the Start of Every Session

- **Confirm the current physical location** and privacy. Clients travel.
- A **parked**, private car can work; a client may **not** join while driving.
- Keep cameras on; camera-off sessions lose risk-relevant observation.

Licensure Follows the Client

A client's consent **cannot waive** another state's licensure law, a home-state license does not travel with the platform, and audio-only sessions are not exempt. A client away for the summer means the counselor needs a license, temporary permit, or Compact privilege there.

Platform Compliance (6.9)

Requirement	Owner	Exam point
BAA with the vendor	164.502(e), 164.504(e)	The platform is a business associate.

Requirement	Owner	Exam point
Encryption in transit	164.312(e) transmission security; **ACA H.2.d**	Live video is PHI in transit even if nothing is recorded. **Encryption alone is not compliance**, because a BAA is still required.
Access controls	164.312(a)(1)	**Unique session links or a locked waiting room** so one client cannot walk into another's session. A reusable public link fails, whatever the BAA says.
No pandemic exception	HHS OCR	Consumer-app leniency **ended August 9, 2023.**
Medicare telehealth flexibilities	Medicare payment rules	These concern **coverage and payment**, not HIPAA platform compliance. They have been extended repeatedly; verify their current status.
Recording	**ACA B.6.c**; state recording laws	The **client's** informed consent (or a legal representative's if the client lacks capacity), plus any state-required consent; secure recordings as records. A caregiver without legal authority cannot consent, and a compliant platform does not supply consent.

When the compliant platform fails, **troubleshoot it or reschedule.** A parent's consent, a client's preference, or

a signed waiver does not make a consumer app compliant. HIPAA has no looser rule for minors.

Part 6 — Licensure, Certification, and Portability (6.6, 6.13)

Keeping the License Active (6.6)

The state board owns the license. Rules vary by state, so the right answer is usually "check and follow the board's rules."

Duty	What to know
Renewal and CE	The board sets required hours, content areas such as ethics, and **approved providers**. CE that counts for NCC renewal may not count for the license. Courses labeled "for counselors" may not meet board rules. Ongoing CE is also an ethical duty (**ACA C.2.f**).
CE audits	Renewal usually involves **attesting** to completion. An audit asks for **proof**. Keep certificates for as long as board rules require, not just until the renewal is approved.
Change of information	Report a name or address change within the board's deadline, which is often shorter than the renewal cycle. Updating an NPI record in NPPES does **not** notify the board.
Self-reporting	Many boards require reporting arrests, charges, or convictions within a set period. Some require reporting **charges**, not just convictions. The duty runs to the board, not to clients.

Duty	What to know
Associates and supervision	Associates practice only under a **board-approved supervisor**. When a supervisor leaves, register a replacement under board rules before continuing. Peer consultation with a fellow associate is not supervision, and hours without approved supervision may not count.
Lapsed license	**Stop practicing** until the board reinstates the license; otherwise it is unlicensed practice, and claims may be improper. A colleague's cosignature confers nothing. Any grace period depends on the state.

Licensure versus certification. The license is state permission to practice; the **NCC** is a **voluntary** national credential. Holding one never satisfies the other.

License Portability (6.13)

Route	How it works
Traditional multistate licensure	A separate full license in each state, often by endorsement. This route remains available everywhere.
Temporary or limited permits	Some states allow short-term practice for clients temporarily located there. Rules vary by state.
Counseling Compact privilege to practice	Practice in **member states** under a privilege based on the home-state license

Counseling Compact requirements (Counseling Compact Commission, n.d.): - The **home state** is the counselor's **primary state of residence**. - The counselor holds an **active, unencumbered home-state license** and has had **no encumbrance on any license in the prior two years**. - The counselor meets the remote state's notice, fee, and **jurisprudence** requirements. -

Member states require a national examination for licensure. The Compact does **not** require the NCC. - The counselor practices under the laws of the **remote state where the client is located.**

Situation	Rule
Client moves to a **non-member** state	The privilege does not reach that state. Meet its own licensure or authorization rules. There is no general continuity-of-care exception; any temporary allowance comes from that state's law.
Counselor **moves** between member states	Apply for a license in the **new home state** under Compact rules. The former home-state license converts to a privilege once the new one is active. A privilege cannot serve as the license where the counselor lives.

Verify the current status. The Compact has been enacted in many states, and privileges are being issued state by state as member states go live. Before relying on a privilege, check that both states are active members issuing or accepting privileges.

Part 7 — Billing, Reimbursement, and Compliance (6.7, 6.8, 6.10)

Identifiers and Enrollment

Owner	What it is	Exam point
NPI Type 1	Individual provider identifier, issued once per person for life	A counselor has **one** Type 1 NPI.

Owner	What it is	Exam point
NPI Type 2	Organizational identifier	A PLLC or group that bills as an entity gets a Type 2. The counselor stays the **rendering provider** under her Type 1.
NPPES	The federal system that issues and maintains NPIs	It is not a credentialing application, and it does not update the licensing board.
CAQH ProView	The shared credentialing profile many commercial payers pull from	Providers **re-attest** periodically (currently every 120 days).
NPDB	Federal data bank of malpractice payments and adverse licensure and privileging actions	Payers and hospitals query it; providers do not build profiles there.
Medicare MHC enrollment	Under the **Consolidated Appropriations Act, 2023**, eligible **mental health counselors** may enroll and bill Medicare directly **as of January 1, 2024**	Not “incident to” billing under a physician
Medicare mandatory claim submission	Enrolled providers submit claims for covered services. A provider eligible for Medicare must **enroll or formally opt out** before privately contracting with beneficiaries.	Billing a beneficiary privately without enrolling or opting out is the error.
“Incident to”	A narrow Medicare rule for services under a supervising practitioner, with specific supervision conditions	It is not a workaround for pending commercial credentialing.

Owner	What it is	Exam point
Locum tenens (substitute) billing	A substitute fills in for an **absent** enrolled provider for a limited time	It does not cover a new clinician waiting to be credentialed.

Billing under a colleague's NPI while your own panel application is pending is a false claim: it misrepresents who rendered the service, whatever the reason.

Before the First Claim (6.7)

- **Verify eligibility and benefits** first: active coverage, mental health benefits, cost-sharing, and any **prior authorization**. Recheck after a job loss or plan change. **Billing a terminated plan is a false claim.**
- **Patient vs. subscriber.** A child on a parent's plan is the **patient**; the parent is the **subscriber**.
- **Network status.** Tell an out-of-network client the likely costs and in-network options. A **superbill** is an optional statement for the client's own reimbursement claim.
- **Good faith estimate (No Surprises Act, 45 CFR 149.610).** Give **uninsured and self-pay** clients a written estimate of expected charges when services are scheduled or on request.
- **Advance Beneficiary Notice (ABN).** Given to **Original Medicare** beneficiaries before a service Medicare may not cover. It is not for self-pay clients generally.
- **Self-pay to keep treatment from the plan.** Honor it, document the restriction, submit **no** claim (**164.522(a)(1)(vi)**).
- **Explanation of benefits (EOB).** EOBs name the provider and service and go to the **policyholder**. A client on an abusive partner's plan can ask the plan to send communications elsewhere; the plan must honor it if she says disclosure could endanger her

(**164.522(b)**). The policyholder's permission is never needed.

- **Fees.** Consider clients' finances; adjust fees for hardship when legally permissible or help find affordable care (**A.10.c**). Pausing care when coverage lapses, especially in a crisis, skips the pretermination counseling **A.11.c** requires.
- **No-shows. Never bill a payer for a missed session**, at any code. Charge a no-show fee only as the signed policy and payer contract allow. A probation office is not a payer without an agreement.

The Codes (6.10)

Code	Service	Rule
90832	Psychotherapy, 30 minutes	**16–37** minutes
90834	Psychotherapy, 45 minutes	**38–52** minutes
90837	Psychotherapy, 60 minutes	**53 or more** minutes
90839	Psychotherapy for crisis, first 60 minutes	**30–74** minutes of urgent assessment and crisis intervention
90840	Crisis add-on, each additional 30 minutes	Used with 90839 once the crisis service reaches **75 minutes** or more. A 90-minute crisis session is 90839 + 90840.
90846	Family psychotherapy **without the patient present**	Billed for the **identified patient's** treatment. The family member need not be a client.
90847	Family psychotherapy **with the patient present** (conjoint)	The identified patient is the client.

Code	Service	Rule
90785	**Interactive complexity** add-on	Requires a **documented** complicating factor in **that** session. It attaches to diagnostic evaluation and to individual and group psychotherapy. It is **not** reported with crisis codes 90839/90840.

Interactive complexity factors (any one, documented): maladaptive communication among participants, such as heated disagreement; a caregiver's emotions or behavior interfering with the plan; a sentinel event requiring a mandated report, discussed in session; or play equipment, devices, or an interpreter for a client lacking expressive or receptive language. **Age alone is not a factor**, and neither is a language difference alone.

On U.S. claims, diagnoses are reported with **ICD-10-CM** codes.

Documentation That Supports a Claim

The record supports the claim. The claim does not support itself.

Claim	What the record must show
Any psychotherapy code	**Medical necessity**: notes linking **diagnosis**, current **functional impairment**, and **plan goals** to **each session's interventions** and the **client's response**
Time-based and crisis codes	**Actual start and stop times**; for crisis codes, the urgent **risk assessment** and **interventions**. A calendar block shows a reserved slot, not a delivered service.

Claim	What the record must show
90846	**Who attended, why the patient was absent**, and **how the session served the patient's plan**. A diagnosis for the family member would make that person the patient.
90785	The specific complicating factor and how it was managed

Errors the exam punishes: a diagnosis code standing in for necessity (**diagnosis-as-justification**); attendance-only notes; verbatim quotes sent to payers, which expose more than needed and belong, if anywhere, in psychotherapy notes; **cloned** notes; and notes written after the claim or only when audited.

Fraud and Abuse Patterns (6.8)

Pattern	Why it is wrong
Upcoding	Billing 90837 for 45 minutes, or a higher code because the diagnosis seems "intensive." Complexity does not add minutes.
Undercoding	Billing 90832 for 45 minutes "to be safe." Inaccurate in either direction is inaccurate.
Diagnosis chosen for coverage	Bipolar II for more sessions, anxiety to hide an alcohol diagnosis, or a vague unspecified code. Violates **ACA C.6.b** and **E.5.a**; it is insurance fraud. Noting "doubts" or the client's preference does not cure it.
Billing for services not rendered	No-shows, sessions after coverage ended, sessions by someone else
Misidentifying the rendering provider	Using a colleague's NPI

Pattern	Why it is wrong
Routine copay waivers	Misstates the true fee and can violate payer contracts and fraud laws. A documented case-by-case hardship adjustment (**A.10.c**) differs. Pro bono (**C.6.e**) is free service, not billing an insurer while waiving the client's share.
Fee splitting and referral payments	**ACA A.10.b**: no fee splitting, and no commissions, rebates, or other payment for referring clients

Compliance Training

HIPAA requires workforce privacy training (164.530(b)) and security awareness training (164.308(a)(5)). The **OIG General Compliance Program Guidance (OIG, 2023)** lists compliance program elements: written policies, a compliance officer, training, open reporting lines, auditing and monitoring, consistent enforcement, and prompt correction. For a solo counselor, that means written billing procedures, periodic self-audits of notes against claims, and refunding overpayments.

Part 8 — Professional Liability Insurance (6.12)

Coverage	What it covers	Exam point
Occurrence policy	Incidents that **occur** during the policy period, whenever the claim is filed	No tail needed when it ends
Claims-made policy	Claims **made and reported** while the policy is active, for incidents after the retroactive date	When it ends (retirement, switching carriers), the counselor needs **tail coverage**
Tail (extended reporting endorsement)	Extends the time to report claims after a claims-made policy ends	The answer for a retiring counselor with a claims-made policy

Coverage	What it covers	Exam point
Prior acts ("nose") coverage	A new claims-made carrier covers incidents before the new policy began	The alternative to buying a tail when switching carriers
General (premises) liability	Slip-and-fall and other injuries at the office	Not professional negligence
Personal umbrella	Extra limits over personal policies	Generally **excludes professional services**
Property insurance	Damage to the office and equipment	It does not restore records or access to them.
Cyber liability	Costs of data breaches	A separate policy from professional liability

Employer coverage is not enough. An agency policy protects the agency first. It has its own limits and exclusions and may not cover off-duty work or **licensing board defense**. Under **respondeat superior** the employer can be liable for an employee's acts, but the counselor can still be named individually. A personal policy follows the counselor across jobs.

When a complaint or claim arrives, first **notify the carrier promptly**, as the policy's **notice-of-claim condition** requires; many policies include board defense and legal consultation. Do **not** call the complaining client (it can look retaliatory) or send the board a rushed, unadvised response.

Part 9 — Secure Records and EHR Use (6.16, 6.18)

Paper and Electronic Records (6.16)

ACA B.6.b requires secure records with access limited to authorized persons. HIPAA requires safeguards for PHI in **any form** (**164.530(c)**); for electronic PHI the Security Rule adds physical safeguards (**164.310**), including

device and media controls, and technical safeguards (**164.312**).

Situation	Secure practice	Error
Client brings **original documents** (school records, rating scales) that inform a diagnosis	Add **copies** to the secure record, return the originals, and limit access to those with a need	Desk tray in open view (fails **164.310** physical safeguards); shredding documents the diagnosis relied on (**A.1.b**)
Client-made **media** (phone videos of symptoms)	Upload through the practice's **secure portal** and keep in the record, limited to the care team	Texts to a personal phone; unlisted video links; a shared office drive everyone can open
Finishing notes **at home**	Work **only through the practice's encrypted remote access**, leaving no local copies	Emailing a draft to a personal account (**B. 3.e**); saving to a home computer and "deleting later"; carrying paper charts home
Retention period has **ended**	**Destroy** so the record cannot be read or reconstructed: shred, burn, or pulp paper; sanitize or physically destroy electronic media (**B.6.h**)	Office recycling bin (fails **164.530(c)**); mailing an unrequested chart to an old address

Retention periods come from state law and payer rules, not HIPAA. HIPAA requires keeping its own compliance documentation, such as policies and signed authorizations, for six years, but sets no clinical-record period. Minors' records are often kept longer. Check your state.

Using the EHR (6.18)

Safeguard	Owner	Meaning
Unique user ID	164.312(a)(2)(i)	Every user has their own login, so every action is traceable. **Never share credentials**, with a colleague or front-desk staff.
Role-based access	164.502(b), 164.514(d)	System access is not authorization. Staff open a chart only as their role in that client's care or operations requires.
Audit controls	164.312(b)	Logs of who viewed and changed what; snooping shows up and calls for sanctions and a breach risk assessment.
Access control	164.312(a)(1)	Automatic logoff, encryption, and emergency access procedures

EHR documentation rules: - **Correct by amendment.** Fix a wrong date in a signed note with a **dated, signed amendment that leaves the original visible** (**A.1.b**). Deleting and re-creating the note, or a hidden vendor edit, destroys the audit trail; leaving the error is also wrong. - **Edit copy-forward text** so the note records **only this session** (**A.1.b**; **C.6.b**). Signing cloned text, or "sign now, fix next week," attests to care that did not occur. - **No shadow records or convenience printouts.** Side files in a drawer, notes at home, and pages left in a mailbox sit outside the record's safeguards.

Part 10 — Accommodating Clients With Disabilities (6.14)

A counselor's office is a **public accommodation** under **ADA Title III** (government agencies fall under Title II, and recipients of federal funds under Section 504 of the Rehabilitation Act). **Psychiatric disabilities are covered.** Referring a client because of a disability rather than clinical need is discrimination (**ACA C.5**).

Rule	Owner	Application
No surcharges	**28 CFR 36.301(c)**	The client never pays for an accommodation: no fee for reading forms aloud, captioning, extra time, or an after-hours slot.
Reasonable modifications	28 CFR 36.302	Change policies and practices when needed, such as a **quieter waiting area** for a client whose voices worsen in a busy room or an accessible ground-floor room. Do not demand needless proof of a known need before making a simple change.
Auxiliary aids and effective communication	**28 CFR 36.303**	Provide aids such as **qualified interpreters**, captioning, and accessible formats at no cost, subject only to the narrow **undue burden** and fundamental-alteration limits. **Ask the client** what works.

Rule	Owner	Application
Companion interpreters	**28 CFR 36.303(c)**	A practice may **not require** a client to bring their own interpreter and may rely on an accompanying adult only in narrow circumstances. Family interpreting also compromises confidentiality.
Written notes	36.303	Notes may work for brief, simple exchanges. They are **not** effective communication for counseling.
Informed consent	**ACA A.2.c**	Consent must be communicated in a way the client can understand, which may mean accessible formats.
Online access	**ACA H.5.d**	Websites and online services are accessible to people with disabilities.

Accommodate rather than remove. A client with dyslexia gets audio or app-based self-monitoring logs with verbal review, not a plan with self-monitoring dropped. A client who now uses a wheelchair is seen in an accessible room, not switched to phone-only care, paused, or carried up the stairs.

Part 11 — Court Proceedings (6.17)

Beyond the ethics material on privilege and subpoenas, the practice tasks add three layers: **legal advice first, HIPAA's conditions, and the treating role.**

Step 1 — Get advice and notify the client. Consult an attorney or the liability carrier's risk-management service and **notify the client**. Do not simply send re-

cords to meet the attorney's deadline, and do not ignore the demand: privilege must be **asserted**, and silence risks sanctions.

Step 2 — Apply HIPAA 164.512(e).

Demand	What HIPAA permits
Court order (signed by a judge)	Disclose **only what the order expressly authorizes**
Subpoena or discovery request without a court order	Disclose only with the client's authorization, or with **satisfactory assurances** that the client was notified and could object, or that a **qualified protective order** was sought. Stricter **state privilege law** may require more, often a court order.

Step 3 — Apply the ethics standards. - **ACA B.2.d:** when ordered to disclose without the client's permission, seek the client's written consent, or ask that disclosure be prohibited or **limited as narrowly as possible.** - **ACA B.2.e:** release **only what is required**. Sending the whole file "to show cooperation" over-discloses. - Refusing a valid court order risks **contempt**. Any privilege claim is raised **with the court**, which decides it. A minor's own refusal does not override a court order.

The treating role. Counselors **do not evaluate current or former clients for forensic purposes (ACA E.13.c).** A treating counselor does not opine that a client is the better parent, poses no danger, or is sane; those opinions need an objective evaluator. **A release permits disclosure; it does not make the opinion appropriate.** If compelled, the counselor testifies as a **fact witness** about treatment. An unsolicited letter to the judge volunteers PHI and blurs roles.

Part 12 — Technology-Assisted Tools and AI (6.11, 6.19)

Technology-Assisted Clinical Tools (6.11)

Apps, wearables, virtual reality, and online platforms follow the same sequence: **vet the tool, get informed consent, build it into the plan, and review it.**

1. **Vet privacy.** Popularity says nothing about how an app stores or sells data, and many consumer apps fall outside HIPAA.
2. **Informed consent (H.2.a; H.4.a).** Explain **benefits and limitations**, data handling, and what the counselor will and will not do with the data.
3. **Fit (H.4.c).** Confirm the client can use the tool and that it suits their needs.
4. **Integrate.** As with CBT homework (**Beck, 2021**), **rehearse** in session, **link** to triggers or targets, and **review** use each session. A bare download link is rarely used well.
5. **Supplement, never substitute.** A calming app does not replace sessions.

Device data. Consumer wearables show **trends**, not validated diagnoses; heart-rate or sleep scores supplement assessment but do not diagnose panic disorder or mark hypomania. Client-shared data may be used in treatment and, once documented, is PHI.

Monitoring expectations. If an app shares entries, clarify **when** the counselor reviews them, that they are **not monitored live**, and where to turn in a crisis. Never hold a client's app password; it creates security risks and an unstated duty to monitor.

Consent and capacity. A tracking wearable for a client with memory loss is discussed **with the client**, seeking consent or **assent** (**ACA A.2.d**), reviewing data privacy, and tying it to the safety plan. **Covert monitoring** is paternalism; refusing all tracking is too rigid; one device

never replaces the safety plan. A parent-training reward app is likewise set up **in session** around positive targets and reviewed, not left to the child or turned into a misbehavior log.

Artificial Intelligence (6.19)

The 2014 ACA Code does not address AI by name, so "the ACA Code prohibits AI" is an invented rule. The existing standards decide AI items: informed consent (**A.2**), confidentiality (**B**), competence (**C.2**), technology (**H**, including **H.4.a** benefits and limitations), and accurate records (**A.1.b**). The **ACA AI Work Group recommendations (ACA, n.d.)** apply them to practice. Some states have passed laws limiting AI-delivered therapy, so check your state.

AI use	Before	During and after
AI note-taker (records sessions, drafts notes)	A **BAA** with the vendor (164.502(e), 164.504(e)); the client's **informed consent** to recording and AI processing. The NPP is not consent.	The counselor **reviews, corrects, and signs**, and remains accountable.
AI drafts (treatment plan summaries, accommodation suggestions)	Use only practice-approved, compliant tools	**Verify each item** against the assessment and record before it enters care or goes to a school team. Disclose AI's role; an "AI-generated" label does **not** shift liability.
Consumer chatbots	**Never** enter client details. Removing a last name is not de-identification.	—

AI use	Before	During and after
Client asks about a mental health chatbot	Help weigh its **evidence, privacy, and crisis limits**	Frame it as a **supplement** reviewed in session; chatbots are not licensed or bound by ethics codes. Keep human contacts and 988 in the safety plan.

Automation bias is trusting output because a machine produced it (Parasuraman & Manzey, 2010): signing AI drafts unread, letting app scores decide stability, or letting a chatbot set a sleep window. **The counselor answers for the clinical content.**

Part 13 — Marketing the Practice (6.20)

Owner	Rule	Application
ACA C.3.a Accurate Advertising	Advertising is accurate, not false, misleading, deceptive, or fraudulent	Specialties claimed only with real training; outcomes not promised
ACA C.3.b Testimonials	**Do not solicit** testimonials from current or former clients or anyone vulnerable to undue influence; discuss implications and get permission for any used	Do not ask satisfied clients to post reviews.
ACA C.3.c Statements by Others	Make reasonable efforts to keep others' statements about you accurate	Correct a directory listing that overstates credentials.
ACA C.4.a Accurate Representation; **C. 4.b** Credentials	Claim only qualifications actually completed, and only licenses and certifications that are **current and in good standing**	No "certified" for a certification still in progress; no lapsed credentials

Owner	Rule	Application
ACA H.6.a Virtual Professional Presence	**Separate professional and personal** profiles	A practice page, not a personal account, for marketing
ACA H.6.c Client Virtual Presence	Do not view clients' social media without consent	No searching clients online
ACA H.6.d Use of Public Social Media	Take precautions not to disclose confidential information	No case stories that could identify a client
ACA B.1.c; HIPAA	The **fact that someone is a client is protected**	**Do not reply publicly to reviews**, even with thanks; any reply confirms the relationship. At most, post a general policy statement that confirms nothing.
HIPAA 164.508(a)(3)	Using PHI for marketing generally requires **authorization**	A client's story, photo, or name in an ad needs a valid authorization, along with the ethics limits on testimonials.

What the Exam Rewards / What It Punishes

Rewards: - Following the written plan or protocol, staying with the client, and documenting each step; updating the plan after a move or drill - Matching each HIPAA request to its right - A BAA, encryption, and access controls before a vendor touches PHI, plus consent for recording and AI - Authorization where the client is located before telehealth - Following state board rules for CE, reporting, supervision, and renewal - Claims that match documented time, provider, and an accurate diagnosis - Free accommodations chosen with the client; legal advice before answering legal process

Punishes: - Waiting for proof of an emergency, or saving files before people - Personal devices, personal email, consumer apps, or shared logins for PHI - Consent, a

waiver, or a BAA treated as compliance - Billing no-shows, lapsed coverage, a colleague's NPI, or a coverage-driven diagnosis - Routine copay waivers, referral fees, or practicing on a lapsed license - Forensic opinions about one's own client - Soliciting testimonials or replying publicly to reviews

Practice Questions

Question 81. A client discloses that she has been hitting her 6-year-old son "more than I should" because "I just lose it sometimes," and mentions that he had bruises on his arm last week that she told the school came from a fall. She asks the counselor to keep this confidential. The counselor should:

A. Honor the request for confidentiality, because the disclosure was made within a protected therapeutic context

B. Assess the severity of the physical discipline and report if it meets the threshold for abuse

C. Discuss the disclosure with a clinical supervisor before deciding whether a report is required

D. Immediately report the disclosure to child protective services based on reasonable suspicion

Question 82. A counselor is offered baked goods by a client as a thank-you gift at the end of their last session. The MOST appropriate response is:

A. Decline the gift politely, because all gifts from clients create potential dual relationship concerns

B. Accept the gift with gratitude, because this is the final session and the gesture is a customary expression of thanks

C. Consider the value of the gift, the cultural context, and the clinical significance before deciding how to respond

D. Accept the gift only if the client consents in writing to its documentation in the clinical record

Question 83. A counselor has been seeing a client for individual therapy for two years. The client's spouse contacts the counselor and requests a phone appointment "to discuss my husband's treatment." The counselor should:

A. Decline the contact without the client's written authorization for this specific communication, and offer to raise it with the client so a release can be obtained if he wishes

B. Schedule the call, as family involvement is clinically valuable and the spouse can offer useful collateral information

C. Contact the client and let him decide verbally whether his spouse may speak with the counselor about his care

D. Provide general information about the client's overall progress and attendance without disclosing any specific session content

Question 84. A client in distress calls after hours and says she needs to speak to the counselor immediately. The counselor has an established after-hours protocol that includes a crisis line number and a clear policy that after-hours contact is for emergencies only. The client says, "This isn't an emergency — I just feel really alone." The counselor should:

A. Speak with the client for as long as necessary — her distress is real and the alliance depends on responsiveness during vulnerable moments

B. Affirm the client's feelings, remind her of the after-hours protocol and crisis resources, and confirm the next appointment

C. Decline to speak with her and direct her immediately to the crisis line listed in the after-hours protocol

D. End the call politely, citing the policy, and document the contact in the record

Question 85. A client on the counselor's caseload becomes a friend of the counselor's spouse. They begin attending the same social events. The counselor should:

A. Immediately terminate the therapeutic relationship and refer the client to another counselor to avoid a dual relationship

B. Discuss the situation with the client, assess the impact on the therapeutic relationship, consult with a supervisor, and document the process

C. Continue treatment without disclosure or consultation, because incidental social proximity does not constitute a dual relationship

D. Request that the client not attend social events the counselor attends so that professional boundaries remain clear

Answer Key — Chapter 25 Practice Questions

Q81 — Answer: D

Rationale: Physical discipline that rises to "more than I should" and "losing it" constitutes reasonable suspicion of physical abuse. Mandated reporting does not require certainty, and it is not delayed by the request for confidentiality (confidentiality does not protect abuse). B implies the counselor determines the abuse threshold — the role is to report the suspicion, not to investigate. C may be a concurrent step but does not delay reporting.

Q82 — Answer: C

Rationale: The ACA Code does not prohibit all gifts — it requires considering the clinical and cultural context (ACA, 2014). A small gift at termination may have significant cultural meaning. A rigid refusal could harm the alliance and cultural relationship at a significant clinical moment. B accepts without any ethical reflection. D is not an actual ACA requirement.

Q83 — Answer: A

Rationale: The counselor cannot speak with the spouse about the client's treatment without the client's explicit, written authorization. The client's name alone being known to the spouse does not constitute a waiver. C is better than B or D but is technically insufficient — authorization must be documented in writing before any contact.

Q84 — Answer: B

Rationale: The counselor acknowledges the distress (affirming), maintains the frame (after-hours policy), provides safety resources (crisis line), and bridges to the next session. A dissolves the therapeutic frame. C is abrupt and clinically inadequate. D is incomplete (no acknowledgment of the client's state).

Q85 — Answer: B

Rationale: The emergence of a social proximity dual relationship requires deliberate navigation: disclosure to the client, assessment of impact, consultation, and documentation. The outcome (continue vs. refer) depends on the clinical context and the client's wishes. Immediate termination (A) is not automatically required. Ignoring it (C) is an ethical failure. D is inappropriate and intrusive of the client's social autonomy.

Chapter 26 — Couples and Family Counseling

Overview

Couples and family counseling is relational work — the client is the relationship system, not any individual member. While the NCMHCE does not have a standalone diagnostic category for "relationship problems," systemic and relational dynamics appear throughout the exam: in how a client's presenting problem is maintained by the family system, in how the counselor conceptualizes intervention, in how treatment planning accounts for relational context, and in explicit cases involving couples conflict, family dysfunction, or parent-child relational problems. Understanding theoretical models, systemic assessment, and evidence-based couples interventions is testable material.

Theoretical Models in Practice

Bowen Family Systems Theory (Murray Bowen) The foundational systemic model. Key concepts:

Differentiation of Self: The degree to which an individual can maintain their own identity, beliefs, and emotional functioning while remaining in contact with a family system under stress. Low differentiation → emotional fusion, reactive decision-making, difficulty tolerating closeness or conflict without losing self. High differentiation → the capacity to be emotionally present without being emotionally absorbed. The NCMHCE expects you to identify low differentiation and recognize that the counselor works on differentiation, not simply "better communication."

Triangulation: When the tension between two family members is managed by routing it through a third (a

child, a symptom, an outside relationship). The identified patient in a family is often the triangle's "stabilizer." Counselor intervention breaks the triangle by reducing emotional reactivity in the dyad.

Multigenerational Transmission: Patterns of functioning, emotional reactivity, and relational dynamics transmit across generations. The genogram is Bowen's primary assessment and intervention tool — mapping three or more generations reveals patterns that cannot be seen in a nuclear family snapshot.

Emotional Cutoff: When a family member manages anxiety about closeness by physically or emotionally distancing from the family. The cutoff does not resolve the unresolved attachment — it carries it into new relationships. Reconnection (not fusion) is therapeutic.

Structural Family Therapy (Salvador Minuchin; Minuchin, 1974) Structure = the invisible rules that organize how a family interacts. Concepts:

Subsystems: Marital/parental subsystem, sibling subsystem, each with its own rules and functions.

Boundaries: Enmeshed boundaries are diffuse — too much closeness, loss of individual identity, difficulty tolerating difference. Disengaged boundaries are rigid — too little connection, members may feel unsupported or isolated. *Clear* boundaries — flexible, permeable, appropriately boundaried — are therapeutic targets.

Hierarchies: In healthy family functioning, the parental subsystem is in the executive position. Parentification (a child assuming parental roles), cross-generational coalitions (a parent aligning with a child against the other parent), and triangulated children are structural dysfunctions.

Joining and Accommodation: The therapist joins the system (accommodates to its style, pace, and language) before attempting to restructure. Premature challenge produces dropout, not change.

Enactments: The therapist asks family members to interact in session rather than describing how they interact. The live enactment reveals structure in real time and provides material for restructuring.

Emotionally Focused Therapy (EFT, Susan Johnson) The most evidence-supported couples therapy model. Based on attachment theory — adult intimate bonds function as attachment relationships. Couples distress arises when partners cannot reliably access each other as attachment figures.

The Negative Cycle: A characteristic interaction pattern driven by underlying attachment fears. The most common: pursuer-withdrawer. Partner A pursues (criticizes, demands, escalates) as an anxious attachment bid. Partner B withdraws (disengages, stonewalls) as a defensive move. Each response confirms the other's deepest fear (abandonment for A, failure/engulfment for B). The EFT therapist: 1. Identifies and names the negative cycle 2. De-escalates the cycle by accessing the primary attachment emotions underneath the cycle (fear, vulnerability, longing) 3. Facilitates enactments of new attachment moments — the softened pursuer and the engaged withdrawer reach toward each other with vulnerability

Three Stages of EFT: (1) De-escalation — cycle tracking, accessing the primary emotions underlying each partner's position; (2) Restructuring the bond — expressing attachment needs, changing interactional positions; (3) Consolidation — integrating new patterns, creating secure base narrative.

Research base: EFT has strong RCT support. Approximately 70–75% of couples move from distressed to nondistressed; 90% show significant improvement. Effects are stable at 2-year follow-up. Particularly effective for couples with depression, PTSD, chronic illness, and infidelity recovery.

Gottman Method Couples Therapy Based on 40+ years of longitudinal research on couples. Key constructs:

The Four Horsemen of the Apocalypse (predictors of relationship dissolution): - Criticism — attacking character rather than behavior ("You're so selfish" vs. "I feel alone when you don't come home for dinner") - Contempt — disgust, belittling, sarcasm, eye-rolling — the single strongest predictor of divorce - Defensiveness — counter-attacking or making excuses rather than taking responsibility - Stonewalling — emotional shutdown, withdrawal, refusal to engage

Antidotes: Gentle startup (for criticism), expressing appreciation (for contempt), taking responsibility (for defensiveness), physiological self-soothing (for stonewalling — 20 minutes minimum before returning to discussion).

The Sound Relationship House: Build Love Maps → Share Fondness and Admiration → Turn Toward → Positive Perspective → Manage Conflict → Make Life Dreams Come True → Create Shared Meaning. The foundation is trust and commitment.

Flooding: Physiological hyperarousal (heart rate >100 bpm) during conflict shuts down access to the prefrontal cortex and new learning. Partners who flood easily need to build in self-soothing time-outs (a minimum of 20 minutes) before attempting productive problem-solving.

Behavioral Couples Therapy (BCT) / Integrative Behavioral Couples Therapy (IBCT) BCT focuses on skill-building: communication training, problem-solving, behavioral exchange (increasing positive interactions). IBCT adds an acceptance component — learning to accept partner differences that are unlikely to change. IBCT has strong evidence support and may be particularly useful when partners have entrenched, temperamental differences that cannot be trained away.

The Clinical Picture

Common Presenting Concerns in Couples: - Communication breakdown — chronic conflict escalation, poor

repair after conflict - Sexual intimacy difficulties — frequency discrepancy, painful sex, low desire, impact of trauma or body image - Infidelity — disclosure crisis, ambivalence about reconciliation, affair fog/compartmentalization - Transition stress — new parenthood, blended family adjustment, empty nest, retirement - Chronic illness — caregiver dynamics, grief, changed roles and intimacy - Emotional disconnection — emotional distance, loneliness within the relationship, pursuer-withdrawer cycle

Common Presenting Concerns in Families: - Parent-child conflict — externalizing behavior, adolescent autonomy vs. parental control - Parentification and role reversal — a child functioning as emotional caretaker for a parent - Blended family adjustment — loyalty conflicts, step-parent authority, sibling rivalry - Grief and loss within the family system — how a death reorganizes family structure and roles - Impact of a family member's psychiatric illness or substance use on the system

Relational Z Codes (DSM-5-TR; APA, 2022) — Not Mental Disorders, But Clinically Relevant: - Z63.0 — Relationship Distress with Spouse or Intimate Partner - Z63.8 — High Expressed Emotion Level Within Family - Z62.820 — Parent–Biological Child Relational Problem - Z62.891 — Sibling Relational Problem These Z codes do not generate reimbursable diagnoses alone but contextualize the clinical presentation. When the relational problem exists alongside a diagnosable condition (depression, PTSD, anxiety) in one or both partners, the mental disorder is the primary diagnosis and the relational code is listed as an additional condition.

Assessment Tools in Context

Genogram: Three or more generations mapped symbolically — squares (male), circles (female), horizontal lines (couples/partnership), vertical lines (offspring) (McGoldrick et al., 2020). Notations for divorce, death,

estrangement, significant illness, migration, occupational patterns. Used in Bowen-oriented work to identify multigenerational patterns of functioning. Clinically, the process of constructing the genogram together is often as therapeutic as the product.

Dyadic Adjustment Scale (DAS-32 / DAS-4): Self-report measure of relationship quality across four subscales: consensus, satisfaction, cohesion, and affectional expression (Spanier, 1976). Brief versions (DAS-4) are feasible for ongoing monitoring. Scores below 98 on the full scale suggest clinical distress. Used pre- and post-intervention.

FACES-IV (Family Adaptability and Cohesion Evaluation Scale): Circumplex Model assessment measuring family cohesion (disengaged → connected → enmeshed) and flexibility (rigid → structured → flexible → chaotic) (Olson, 2011). Balanced families cluster in the center of both dimensions. Scores at the extremes (enmeshed + rigid or disengaged + chaotic) correlate with clinical dysfunction.

McMaster Model / Family Assessment Device (FAD): Assesses six dimensions of family functioning: problem solving, communication, roles, affective responsiveness, affective involvement, and behavior control (Epstein et al., 1983). The General Functioning subscale is widely used as a screening tool; scores above 2.0 suggest functional impairment.

Conflict Tactics Scale (CTS2): Measures conflict resolution strategies including physical assault, psychological aggression, injury, and negotiation (Straus et al., 1996). Critical for screening IPV in couples presenting for therapy. Couples counseling with active, ongoing IPV — particularly coercive control — is contraindicated. Safety planning and individual referrals take precedence.

Differentiating When Couples/Family Work Is Appropriate

Couples therapy is indicated when: - Both partners are willing to engage - There is no active coercive control or serious IPV - Both individuals have sufficient individual functioning to tolerate relational focus - The presenting concern is relational (not primarily individual psychopathology)

Couples therapy is contraindicated when: - Active domestic violence with coercive control dynamics (safety risk in conjoint sessions — victim may not be able to speak freely) - Active, untreated substance use severely impairing functioning - One partner is in crisis (acute suicidality, acute psychosis) without stabilization - One or both partners are actively pursuing legal action against the other

Family therapy is indicated when: - The identified client is a child or adolescent whose symptoms are maintained by family dynamics - A family member's psychiatric illness is significantly disrupting family functioning - Structural dysfunction (parentification, triangulation, coalition) is clinically visible - The transition (death, divorce, remarriage, developmental stage) requires systemic reorganization

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What the Exam Rewards / What It Punishes

The NCMHCE rewards the counselor who: - Conceptualizes presenting problems systemically — asks "what function does this symptom serve in the system?" even when the case frames an individual - Identifies triangulation, enmeshment, and parentification as structural problems requiring systemic intervention, not individual pathology - Chooses EFT or Gottman Method for couples — these are the evidence-based modalities the exam expects - Screens for IPV before proceeding with conjoint couples work - Uses the genogram as a clinical tool, not

just an intake exercise - Attends to cultural values around family hierarchy, role definitions, and relational expectations

The NCMHCE punishes the counselor who: - Assumes "communication skills training" is sufficient without addressing the attachment/structural substrate - Proceeds with couples therapy without screening for IPV - Takes sides — the system, not the individual, is the client - Treats a child's symptoms (acting out, school refusal, anxiety) in isolation without considering the family system - Ignores developmental stage in family work — what is "parentification" in a healthy family with an ill parent may be a necessary temporary role reorganization that deserves support, not reframing

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Worked Mini-Case

Case: Brianna (32) and Darius (35) have been married for six years. They present stating they "can't communicate." Darius describes feeling criticized constantly and says he "shuts down" during arguments; Brianna says she feels "completely alone" and that Darius doesn't care. In session, when Brianna raises a complaint, Darius's body turns away, he looks at the floor, and his verbal responses become monosyllabic. Brianna's voice escalates and she eventually says, "See? This is exactly what I mean. He never talks to me." Brianna reports that prior to marriage, communication was "never a problem." Both deny domestic violence. Brianna recently received a diagnosis of generalized anxiety disorder and is in individual therapy.

Clinical Formulation: This couple is demonstrating the classic pursuer-withdrawer cycle, the most common negative cycle in EFT. Brianna's pursuit (escalating, criticizing) is an attachment bid — her underlying emotion is fear of abandonment and longing for connection. Darius's withdrawal is a defensive move — his underlying emotion is likely inadequacy and fear of failure as a partner. Each

response reinforces the other's fear and maintains the cycle. Brianna's GAD is contextually relevant — chronic relational stress amplifies anxiety and vice versa. Individual therapy is ongoing and can continue in parallel.

Treatment Selection: Emotionally Focused Therapy (EFT) is the evidence-based first-line choice. Gottman Method is also appropriate. The counselor does not pursue communication skills training until the negative cycle is de-escalated and the underlying attachment fears are accessed — skill-teaching in an emotionally flooded couple produces compliance in session and regression at home.

Immediate Priority: Identify and name the negative cycle with the couple. Normalize both partners' contributions. Access Brianna's primary emotion ("When Darius goes quiet, what happens for you inside?"). Access Darius's primary emotion ("When Brianna escalates, what's happening for you?").

Practice Questions

Question 86. A counselor using Emotionally Focused Therapy with a distressed couple identifies that Partner A criticizes and escalates while Partner B withdraws. The counselor's FIRST therapeutic priority is to:

A. Teach Partner A communication skills for expressing needs and complaints without criticism or blame

B. Work individually with Partner B on assertiveness skills to reduce withdrawal and increase engagement during conflict

C. Name and externalize the negative cycle as the shared problem, rather than attributing it to either partner

D. Conduct individual sessions with each partner to resolve personal issues before returning to couples work

Question 87. In a family therapy session, the counselor observes that 11-year-old Mia consistently comforts her

mother when her mother cries, speaks to her father on her mother's behalf, and mediates her parents' arguments. This pattern is BEST conceptualized as:

A. Sibling relational problem resulting from Mia's position as the eldest child, requiring intervention within the sibling subsystem

B. Parentification — a structural dysfunction in which a child assumes an inappropriate parental role, requiring structural intervention

C. Healthy attachment behavior that demonstrates Mia's emotional maturity, empathy, and the secure base she has with both parents

D. Evidence of severe parent-child enmeshment that warrants Mia's removal from the family system to protect her emotional development

Question 88. A couple requests couples counseling. During the intake, the counselor asks about conflict resolution and the wife becomes visibly anxious. The husband answers all questions directed at her, minimizes her hesitation with "she's just nervous," and twice corrects what she says. The counselor's MOST appropriate immediate response is:

A. Pause the intake and find a reason to speak with the wife individually to assess for coercive control and safety before proceeding with conjoint work

B. Proceed with the couples intake as planned, because no physical violence or threats have been disclosed by either partner

C. Ask the husband to allow his wife to speak for herself, then continue the intake with both partners present in the room

D. Document the interaction carefully and provide a referral to IPV services at the end of the session

Answer Key — Chapter 26 Practice Questions

Q86 — Answer: C

Rationale: In EFT, the negative cycle is the target of first-stage intervention — not individual skill deficits or personal pathology (Johnson, 2020). Naming the cycle as a shared, external problem reduces blame and creates a collaborative therapeutic frame. Teaching skills (A) before de-escalation is premature and ineffective in a flooded couple. Assertiveness work for the withdrawer (B) addresses surface behavior without the underlying attachment fear. Individual sessions (D) split the system and delay the relational work without clinical indication.

Q87 — Answer: B

Rationale: Parentification occurs when a child assumes emotional (or instrumental) caretaker functions appropriate to an adult. Mia is performing three parentified functions: soothing a parent's distress, acting as an intermediary, and mediating adult conflict. In Structural Family Therapy, this represents a boundary and hierarchy disruption — the parental subsystem has failed to function autonomously, and Mia has been recruited into it. Structural intervention is required to restore appropriate generational roles. Secure attachment (C) does not produce adult role assumption. "Removal" (D) is not a structural intervention and is clinically inappropriate.

Q88 — Answer: A

Rationale: The husband's behavior pattern — speaking for his wife, minimizing her expressions, correcting her narrative — is a behavioral presentation consistent with coercive control dynamics. The wife's visible anxiety when asked about conflict is a safety signal. Couples counseling with active coercive control is contraindicated — the victim cannot speak freely in the partner's presence, which can escalate danger. The clinically indicated step is to

create a brief individual conversation to assess safety before making any recommendation about conjoint work. Proceeding with intake (B) ignores a safety signal. Redirecting the husband verbally (C) may escalate danger if coercive control is present. End-of-session referral (D) is too delayed and does not assess immediate safety.

Chapter 27 — Areas of Clinical Focus: Life-Span, Relational, and Situational Concerns

Overview

NBCC's 2027 specifications list the presenting concerns counselors report as most common and say candidates may benefit from reviewing them (NBCC, 2026). Many are covered in the diagnostic chapters: depression, anxiety, trauma, substance use, obsessive thoughts, eating, sleep, and dissociation. Many others are **not mental disorders at all**. Grief, divorce, caregiving, retirement, career indecision, bullying, financial strain, spiritual questions, and discrimination bring people to counseling every day. On the exam they often appear as the context of a case rather than its diagnosis.

This chapter covers those concerns. The recurring exam lesson: **do not pathologize a normal response to a hard life event, but do screen for the disorders these events can trigger.** A client going through a divorce may need support and problem-solving, not a diagnosis. The same client may also be developing major depression or drinking more, and the counselor has to check.

When a nondiagnostic concern is the focus of care, DSM-5-TR provides codes for "other conditions that may be a focus of clinical attention" — for example, relationship distress with a spouse or partner, uncomplicated bereavement, and problems related to employment or housing (American Psychiatric Association [APA], 2022). These are Z codes, not mental disorders.

A Life-Span Frame

Many presenting concerns are developmental tasks under strain. Erikson (1963) described eight psychosocial stages, each organized around a central tension. The stage tells you what the client is likely working on:

Life stage	Central task (Erikson)	Common presenting concerns
Childhood	Trust, autonomy, initiative, industry	Separation from caregivers, bullying, school problems, adoption questions
Adolescence	Identity vs. role confusion	Identity and gender identity development, peer conflict, dating
Young adulthood	Intimacy vs. isolation	Dating and relationship problems, loneliness, career entry
Middle adulthood	Generativity vs. stagnation	Parenting, caregiving for aging parents, career change, divorce, remarriage
Late adulthood	Integrity vs. despair	Retirement, health decline, loss of a partner, end-of-life issues

Transitions

Schlossberg's (1981) transition model helps explain why the same event overwhelms one client and barely affects another. How someone copes with a transition depends on four factors, often called the "4 S's" in later formulations of the model:

- **Situation**: the event itself — its timing, whether it was chosen, and what else is happening
- **Self**: personal resources, coping style, and prior experience with change
- **Support**: relationships, community, and practical help

- **Strategies**: the coping responses the person has available

How this shows up on the exam: A stem describes a client struggling after a job loss, move, divorce, retirement, or diagnosis. The rewarded answer assesses what the transition means to the client and strengthens their coping resources before labeling their distress.

Grief, Loss, and Prolonged Grief Disorder

Grief is a normal response to loss, and most people adapt without professional help. Worden (2018) describes four tasks of mourning: accepting the reality of the loss, processing the pain of grief, adjusting to a world without the deceased, and finding an enduring connection with the deceased while moving forward with life. The dual process model adds that healthy grieving moves back and forth between *loss-oriented* coping (facing the grief) and *restoration-oriented* coping (handling new roles and daily life), and that time away from grief is part of healthy adaptation, not avoidance (Stroebe & Schut, 1999).

The popular "five stages of grief" are not a required sequence. The exam will not reward pushing a client through stages in order.

Prolonged Grief Disorder (PGD) was added to DSM-5-TR in the trauma- and stressor-related disorders (APA, 2022):

- At least **12 months** since the death (at least **6 months** for children and adolescents)
- A persistent grief response marked by **intense yearning** for the deceased and/or **preoccupation** with thoughts or memories of them, nearly every day for at least the last month
- At least **3 of 8** additional symptoms: identity disruption; marked disbelief about the death; avoidance of reminders; intense emotional pain; difficulty reintegrating into life; emotional numbness; feeling that life is meaningless; intense loneliness

- Clinically significant distress or impairment, and a duration and severity that clearly exceed cultural, religious, and social norms

Grief vs. major depression. DSM-5-TR does not exclude a major depressive episode just because it follows a loss. If full MDD criteria are met, MDD is diagnosed. Grief typically comes in waves ("pangs") tied to reminders of the deceased, and self-esteem is usually preserved. Depression brings pervasive low mood, anhedonia, and feelings of worthlessness.

Exam rule: Normalize grief. Assess for suicide risk (bereavement raises it). Screen for MDD, PGD, and substance use. Respect the client's cultural and religious mourning practices.

End-of-Life and Terminal Illness

Clients facing a terminal diagnosis, and families caring for a dying person, bring existential concerns about death, meaning, isolation, and freedom (Yalom, 1980), as well as practical concerns such as advance directives, family communication, and hospice. The counselor's role is presence, meaning-making, and coordination with medical and hospice teams. It is not to hurry acceptance.

The ACA Code allows counselors, depending on applicable laws and the circumstances, to keep confidential a terminally ill client's consideration of hastening death (ACA, 2014, Standard B.2.b). This is different from suicidal ideation in a client who is not terminally ill, which calls for standard risk assessment. The rewarded answer consults, knows the relevant state law, and explores the client's reasoning without judgment.

Caregiving

Adults caring for a parent, partner, or child with a serious illness or disability carry a well-documented burden: exhaustion, isolation, financial strain, and depressive symptoms. The Zarit Burden Interview measures the care-

giver's own sense of strain (Zarit et al., 1980). The exam rewards assessing the *caregiver's* well-being, including depression and suicide risk, and connecting them with respite care, support groups, and practical services. Caregivers often minimize their own needs.

Aging and Retirement

Retirement is a transition, not simply a loss. For some clients it brings freedom; for others it means lost identity, structure, income, or social contact. Late-life concerns also include chronic illness, sensory and cognitive changes, loss of peers and partners, and ageism. The counselor screens for depression, which is common but not a normal part of aging. Suicide risk is elevated in older adults, especially older men. The counselor also screens for cognitive decline when memory complaints appear (see Chapter 15).

Career and Occupational Concerns

Work problems are mental health problems more often than clients think: job loss, underemployment, burnout, workplace conflict, and career indecision all affect mood and identity.

- **Super's life-span, life-space theory**: career development unfolds across stages (growth, exploration, establishment, maintenance, and disengagement), and work is one of several life roles, alongside partner, parent, and citizen, that compete for time and meaning (Super, 1980).
- **Holland's RIASEC typology**: people and work environments can be described by six types — Realistic, Investigative, Artistic, Social, Enterprising, and Conventional — and satisfaction is higher when person and environment match (Holland, 1997).

Exam rule: Integrate career concerns with mental health concerns rather than treating them as separate. A client

who lost a job needs both an assessment of depression and suicide risk *and* practical support with the transition.

Financial Stress

Financial strain — debt, job loss, housing insecurity, poverty — is a major driver of anxiety, depression, relationship conflict, and suicide risk. Counselors ask about finances directly and without judgment, connect clients with concrete resources (financial counseling, benefits, housing and food assistance), and adapt the treatment plan to real constraints such as cost, transportation, and time off work.

Divorce, Remarriage, and Blended Families

Divorce is a family transition, not a single event. Children adjust best when conflict between parents is low and parenting stays warm and consistent. Exposure to ongoing high conflict between parents is more harmful than the divorce itself.

Stepfamilies face predictable challenges. Insiders and outsiders form around the new couple. Children often feel loyalty binds. Parenting roles are unclear. And the family has not yet built shared culture and rituals (Papernow, 2013). A key principle: in the early years, the biological parent handles discipline while the stepparent builds a relationship with the child, rather than stepping into the disciplinarian role right away.

Exam rule: In co-parenting conflict, the rewarded answer protects children from being placed in the middle, for example as messengers between parents. The counselor stays neutral between parents, knows the legal custody arrangements before sharing a child's information, and refers for couples or family work as indicated (see Chapter 26).

Adoption

Adoption concerns can surface at any age: children's questions about identity and origins, grief and loss in all members of the adoption triad (the adoptee, birth parents, and adoptive parents), transracial and international adoption identity issues, and search and reunion. The counselor normalizes these questions as part of identity development, respects the family's cultural context, and avoids assuming that every difficulty an adoptee has is "about the adoption."

Bullying

Bullying is repeated, intentional aggression within a relationship with a power imbalance. It can be physical, verbal, relational (exclusion), or electronic (cyberbullying) (Olweus, 1993). Victims are at higher risk of depression, anxiety, school avoidance, and suicidal ideation.

Exam rule: Assess safety and suicide risk first. Involve the school through the appropriate channels, and collaborate with parents. Avoid peer mediation between the student who bullies and the target — the power imbalance makes it inappropriate.

Loneliness and Attachment

Loneliness is the distressing sense that one's relationships are fewer or less satisfying than desired. It is not the same as being alone. Attachment theory holds that early caregiving relationships shape the internal working models people bring into adult relationships (Bowlby, 1969). Chronic loneliness raises the risk of depression and poorer health. Interventions build social connection, address the thoughts and anxieties that keep people isolated, and use the therapeutic relationship itself as a corrective experience.

Religious, Spiritual, and Existential Concerns

Clients may bring religious doubt, spiritual struggle, a conflict between their faith and their identity or choices, or questions of meaning. The ASERVIC competencies call on counselors to understand spirituality and religion as part of culture and to explore clients' beliefs sensitively — neither ignoring them nor imposing the counselor's own (Cashwell & Watts, 2010).

Exam rule: Explore what the client's faith means to them, use it as a resource when the client wants that, and never impose the counselor's beliefs or refer a client out solely because their values differ from the counselor's (ACA, 2014, Standard A.11.b).

Racism, Discrimination, and Oppression

Experiences of racism and discrimination are real stressors with real effects on mental health. Carter (2007) describes race-based traumatic stress: emotional and psychological injury from racism that can produce trauma-like symptoms. Racial microaggressions — everyday slights and invalidations — also add up over time (Sue et al., 2007).

Exam rule: The counselor validates the client's experience rather than questioning whether the discrimination "really" happened. They explore its impact, strengthen coping and community connection, and consider advocacy where appropriate. They also examine their own biases (see Appendix A).

Gambling and Process Addictions

Gambling Disorder is the only behavioral addiction in DSM-5-TR's substance-related and addictive disorders chapter (APA, 2022). It requires persistent, problematic gambling with at least **4 of 9** criteria in a 12-month period, such as needing to gamble larger amounts, restlessness when cutting back, repeated failed efforts to stop,

"chasing" losses, lying to hide gambling, jeopardizing relationships or jobs, and relying on others for money. Severity is mild (4–5 criteria), moderate (6–7), or severe (8–9). A manic episode must not better explain the gambling, so rule out bipolar disorder. Suicide risk is elevated, especially when debts mount. Treatment includes CBT, motivational interviewing, financial safeguards, and Gamblers Anonymous (see Chapter 11).

Other process addictions, such as problematic pornography use or compulsive sexual behavior, are not DSM-5-TR diagnoses. ICD-11 classifies compulsive sexual behaviour disorder as an impulse control disorder (World Health Organization, 2019). The counselor assesses the behavior's impact, distress, and function without moralizing, and explores values conflicts and co-occurring conditions.

Stress Management

The transactional model of stress holds that stress depends on the person's *appraisal* of a demand and of their resources to meet it. Coping can be problem-focused (changing the situation) or emotion-focused (managing the emotional response) (Lazarus & Folkman, 1984). Effective counseling helps clients match their coping to the situation: problem-solving for what can change, and acceptance and emotion regulation for what cannot. Relaxation training, sleep and activity routines, assertiveness training, and time management are common psychoeducational tools.

Where Each Presenting Concern Is Covered

The table maps the presenting concerns NBCC's 2027 specifications list as most common, along with a few from the earlier content outline that are still worth reviewing, to the chapters that cover them (NBCC, 2026).

Presenting concern	Where to review
Hopelessness/depression; physical issues related to depression	Chapter 8
Worry, anxiety, fear, and panic; physical issues related to anxiety	Chapter 9
Physical and emotional issues related to trauma; child abuse; family abuse and violence	Chapters 10, 19, and 26
Substance use and addiction; process addictions	Chapter 11 and this chapter
Emotional dysregulation	Chapters 12 and 13
Hyper- or hypo-focus; intellectual functioning; child development	Chapter 14
Aging and geriatric concerns	Chapter 15 and this chapter
Obsessive thoughts and behaviors; ruminating and intrusive thoughts	Chapter 16
Visual and auditory hallucinations	Chapter 17
Maladaptive eating behaviors	Chapter 18
Suicidal thoughts and behaviors	Chapter 19
Behavioral problems	Chapter 20
Adjustment to physical illness or injury	Chapter 21 and this chapter
Dissociation	Chapter 22
Insomnia, sleep issues, and sleeping habits	Chapter 23
Sexual functioning concerns; gender identity development	Chapter 24
Marital and partner communication; interpersonal partner violence; parenting and co-parenting	Chapter 26
Cultural adjustment; racism, discrimination, and oppression	Appendix A and this chapter

Presenting concern	Where to review
Grief and loss; end-of-life; terminal illness; caregiving; retirement; career; financial issues; divorce; remarriage; blended families; adoption; bullying; loneliness and attachment; separation from caregivers; religious values conflict; spiritual and existential concerns; stress management	This chapter

What the Exam Rewards (and Punishes) — Areas of Clinical Focus

Rewards: Normalizing expectable reactions to life events while screening for depression, suicide risk, and substance use. Exploring what a transition or loss means to the client. Strengthening supports and coping resources. Respecting cultural and religious mourning and faith practices. Validating experiences of discrimination. Keeping children out of the middle of parental conflict. Treating gambling disorder as a true addictive disorder with elevated suicide risk.

Punishes: Diagnosing normal grief or adjustment as a disorder. Missing MDD or PGD because "it's just grief." Imposing a stage sequence on grief. Questioning whether discrimination occurred. Peer mediation in bullying. Having the stepparent take over discipline early. Moralizing about process addictions. Imposing the counselor's religious views.

Worked Mini-Case — Areas of Clinical Focus

Vignette: Harold, 67, retired from a 40-year career as a machinist eight months ago. His wife died 14 months ago. He says he "feels useless," has stopped seeing friends, and visits her grave daily, where he "talks to her for hours." He cannot bring himself to change anything in her side of the closet. He says, "Some nights I wonder

what the point of going on is." His adult daughter says he has "lost weight and stopped caring about himself."

Reasoning chain:

Step 1 — Safety: "What the point of going on is" requires a direct suicide risk assessment now. Older men who have lost a spouse are at elevated risk. Ask about access to firearms and other lethal means.

Step 2 — Differential: More than 12 months after his wife's death, daily intense yearning and preoccupation, avoidance of changing her belongings, loss of meaning, and loneliness point toward Prolonged Grief Disorder. Weight loss and self-neglect also call for screening for MDD, which can co-occur, and for a medical evaluation.

Step 3 — Transition layer: Retirement removed his structure, identity, and social contacts at the same time grief was pulling him inward. Schlossberg's 4 S's frame the assessment: situation (two losses at once), self (identity rooted in work), support (withdrawn from friends), strategies (few).

Step 4 — Plan: Safety plan and means counseling as indicated. Grief-focused treatment for PGD. A behavioral re-engagement plan that uses his identity as a craftsman (for example, volunteering). Involve his daughter with his consent. Arrange a medical follow-up.

Check Your Understanding — Areas of Clinical Focus

Clinical focus — grief. A client whose husband died four months ago cries daily, feels his absence intensely, and keeps his photos on display, but she is working, eating, and seeing friends. The MOST appropriate counselor stance is:

A. Diagnose Prolonged Grief Disorder and begin grief-focused treatment given her daily crying.

B. Normalize her grief, offer support, and monitor for depression and suicide risk.

C. Encourage her to put away his photos to speed acceptance of the loss and reduce daily crying.

D. Refer her for a medication evaluation to address the daily crying and sadness.

Answer: B. Four months after a death, with intact functioning, this is expected grief. PGD requires at least 12 months in adults (APA, 2022). Asking her to remove reminders (C) imposes the counselor's agenda. Nothing here indicates a need for medication (D).

Clinical focus — stepfamily. A newly remarried mother asks whether her husband should take over disciplining her 12-year-old son, who resents him. The counselor's BEST guidance is:

A. The stepfather should take over discipline right away to establish authority as the new head of the household.

B. The biological parent should lead discipline while the stepfather builds a relationship with the child.

C. Discipline should be suspended until the child accepts the stepfather, so that conflict at home can ease.

D. The child should choose which parent disciplines him, giving him a sense of control during the transition.

Answer: B. In early stepfamily development, the stepparent builds the relationship first and the biological parent keeps the disciplinary role (Papernow, 2013). A tends to intensify the child's resentment and loyalty conflicts. C and D abandon appropriate parental structure.

Chapter 28 — Indirect Client Care

2027 task map: 5.1 Collaborate with client's relevant support systems (e.g., advocate, family, wraparound services, legal services) with goals and objectives · 5.2 Collaborate with non-clinical stakeholders (e.g., legal counselor, community resources) · 5.3 Collaborate with other treatment providers (e.g., care coordination, interdisciplinary treatment team) · 5.4 Document the self-reported effects of client's medication · 5.5 Maintain an updated list of referral resources · 5.6 Review clinical documentation before client sessions · 5.7 Secure appropriate translation services for clients' preferred language · 5.8 Write clinical documentation during or after client sessions

The Work Around the Session

Most of what this unit covers happens when the client is not in the chair. The counselor reads the chart before the session, writes the note after it, calls the prescriber, books the interpreter, updates the referral list, and agrees on roles with a sister, a school counselor, or a domestic violence advocate. The 2027 outlines group these tasks as indirect client care.

Indirect does not mean minor. A missed line in a pre-session review can hide a risk flag. A family member asked to interpret can change what a safety plan says. A note that says "the medication is too strong for her" puts a medical judgment in the record that the counselor is not qualified to make.

Every item in this area turns on three questions, asked in order:

1. **Consent.** Has the client agreed, in writing where it matters, to this contact and this much sharing?

2. **Scope.** Is this my role, or someone else's: the prescriber's, the attorney's, the interpreter's, the advocate's?
3. **Record.** Does the record show what happened accurately, on time, and in the client's words where the client is the source?

The rewarded answer keeps the counselor responsible for the client's care, shares only what the client agreed to and the purpose needs, and connects the client to the right person instead of doing that person's job.

Part 1 — Who Owns It

Owner	What it owns
ACA A.1.b; **B.6.a**	Timely, accurate records needed for services and continuity; amendments noted per policy
ACA A.1.d Support Network Involvement	Family, friends, and community leaders as resources, **with client consent**
ACA A.2.c Developmental and Cultural Sensitivity	Culturally appropriate communication; **a qualified interpreter or translator** when language is a barrier
ACA A.2.e Mandated Clients	Telling mandated clients what is shared, with whom
ACA A.3 Clients Served by Others	Requesting a release to inform the client's other mental health professionals and collaborate
ACA A.7.a / A.7.b	Advocacy to remove barriers; client consent before advocating for an identifiable client
ACA A.10.b	No payment for referrals (no fee splitting)
ACA A.11.a	Knowing appropriate referral resources and suggesting alternatives when lacking competence
ACA A.11.d	Completing transfer steps and keeping communication open

Owner	What it owns
ACA A.12	No abandonment
ACA B.1.c Respect for Confidentiality	Disclosing only with consent or sound legal or ethical justification
ACA B.2.d / B.2.e	Limiting court-ordered disclosure; sharing only essential information
ACA B.3.b / B.3.e	Informing clients about interdisciplinary teams; protecting transmitted information
ACA B.6.g Disclosure or Transfer	Written client permission before records go to a third party
ACA B.7.a / B.7.b Case Consultation	Consultation for professional purposes with only germane data; no identifying information without consent unless unavoidable
ACA C.2.a Boundaries of Competence	The basis of the medication scope line
ACA D.1.c Interdisciplinary Teamwork	Collaborating on teams with a counseling perspective
HIPAA Privacy Rule (45 CFR Part 164)	Authorization (164.508), psychotherapy notes (164.508(a)(2)), treatment disclosures (164.506), minimum necessary (164.502(b), 164.514(d)), judicial proceedings (164.512(e)), access (164.524), amendment (164.526)
42 CFR Part 2	Added protection for federally assisted substance use disorder program records
FERPA	Education records held by schools
VAWA and FVPSA confidentiality provisions	Domestic violence programs generally share survivor information only with a written, time-limited release
ADA Title III (28 CFR 36.303; 36.301(c))	Qualified interpreters and other auxiliary aids for effective communication, at no charge to the client

Owner	What it owns
Title VI; Section 1557 of the Affordable Care Act	Language access in federally funded and health programs
National CLAS Standards (HHS Office of Minority Health, 2013)	Competent interpreters; no untrained individuals or minors as interpreters
Wraparound (Bruns et al., 2004); **systems of care** (Stroul & Friedman, 1986)	Family-driven team planning for youth across systems; the coordinated network it works within
ACT (Stein & Test, 1980); **MST** (Henggeler et al., 2009)	Community team for adults with serious mental illness; home-based treatment for youth antisocial behavior
Family-focused therapy (Miklowitz, 2008)	Family roles and early warning signs in bipolar disorder
Peer recovery support (SAMHSA)	Lived-experience support, not clinical treatment
ACA Advocacy Competencies (Toporek et al., 2009)	Client-, community-, and public-level advocacy
Collaborative care (Unützer et al., 2002)	Team-based depression care in primary care
Measurement-based care (Lewis et al., 2019); **PHQ-9** (Kroenke et al., 2001)	Repeated measures reviewed to guide treatment
SOAP (Weed, 1968); DAP; BIRP; GIRP	Progress note formats
FDA boxed warnings; **Stahl** (2020, 2021)	Drug safety warnings; mechanisms and side-effect profiles
FINISH (Berber, 1998); **AIMS** (Guy, 1976)	Antidepressant discontinuation symptoms; tardive dyskinesia screening
AGS Beers Criteria (American Geriatrics Society, 2023); **CBT-I guideline** (Qaseem et al., 2016)	Risky medications in older adults; CBT-I first-line for chronic insomnia
988; **211**; **311**	Crisis line; social service referral; city non-emergency services

Part 2 — Collaborating With Support Systems (5.1)

Consent Comes First

ACA A.1.d encourages counselors to consider enlisting family, friends, and community or spiritual leaders as positive resources, **with client consent**. **ACA B.1.c** sets the limit: disclose only with appropriate consent or a sound legal or ethical justification. A client's wish to involve family is an invitation to get a release, not a license to call.

The rewarded sequence:

1. Explore what the client wants the support person to do, and what the client wants kept private.
2. Get a **signed release** naming the person, the information, the purpose, and when it ends. A HIPAA authorization also carries the client's signature and the right to revoke (45 CFR 164.508).
3. Plan the contact or joint session around the client's goals and warning signs.
4. Document the release, the contact, and what was shared.

Risk information does not skip step 2. Disclosing a client's diagnosis or risk to family without consent requires an exception such as serious and foreseeable harm (the ethics chapter covers the exceptions). When a client in ongoing care asks for family support, the protective move is to bring family in **with** the client's agreement.

The Client Sets the Scope

Consent is not all-or-nothing. A client may involve a partner in a new bedtime routine while keeping a past trauma or a diagnosis private. The release records that scope, and the counselor stays inside it.

Error	Why it fails
Overbroad disclosure: treating any release as opening the whole record	A release covers only what it names (ACA B.1.c; ACA B.2.e)
All-or-nothing consent: no involvement unless the client discloses everything	Coercive; blocks a support the client chose
Rigid separation: keeping family out because it "blurs the frame"	Wastes a resource the client asked to use (ACA A.1.d)

A client may widen the scope later or revoke it. Revocation stops future disclosures; it cannot undo past ones.

Give Supports a Role Tied to Goals

Education alone leaves a willing helper unsure what to do. Surveillance turns a helper into a monitor. The rewarded middle is **specific roles, agreed on by the client and the support person together, tied to treatment goals.** In bipolar disorder, family-focused therapy (Miklowitz, 2008) teaches families to spot early warning signs, communicate with less criticism, and follow a relapse-prevention plan the client helped write.

Pattern	Verdict
Collaborative role clarity: client and support person agree on what to notice and how to raise it	Rewarded
Covert monitoring: the support person reports to the counselor without the client's knowledge	Erodes trust and autonomy
Psychoeducation alone: reading material, no agreed roles	Helpful but incomplete
Handing over the goals: family sets goals because of past impairment	Removes the client's voice

Advocates, Peers, and Community Helpers

Support	What they do	What they do not do
Domestic violence advocate	Safety planning, shelter, legal and protective-order advocacy	Share survivor information without the survivor's release
Certified peer recovery specialist	Shares lived experience of recovery; offers hope, mutual support, and help with recovery goals (SAMHSA)	Diagnose, write the treatment plan, or prescribe (medications for opioid use disorder come from a prescriber)
Home-visiting nurse; community health worker	Health education, home visits, links to services	Replace the client's counseling goals
Case manager	Links to housing, benefits, transportation	Provide counseling

Domestic violence advocates. Programs funded under the Violence Against Women Act (VAWA) and the Family Violence Prevention and Services Act (FVPSA) generally may share a survivor's identifying information only with an informed, written, time-limited release. With the client's release, the counselor and advocate align on **one safety plan** that follows the client's choices. Calling for updates without consent breaches confidentiality; handing all safety planning to the advocate leaves gaps.

Shared plans with any helper. Agree on shared goals, each person's role, and how you will stay in contact. Parallel plans duplicate or contradict each other. Deferring the counseling goals to the helper's priorities abandons the counseling role. Sending full progress notes exceeds what shared goals require.

Wraparound and Other Team Models

Wraparound is a planning process, not a level of care. A facilitator brings the family, natural supports, and systems such as school, child welfare, and juvenile court into

one team that writes **one individualized, strengths-based plan**. Its ten principles (Bruns et al., 2004) are family voice and choice, team-based, natural supports, collaboration, community-based, culturally competent, individualized, strengths-based, persistence, and outcome-based. Wraparound grew within the **systems of care** framework for children's mental health (Stroul & Friedman, 1986).

Model	Who it serves	Core feature
Wraparound	Youth with needs across several systems, and their families	Family-driven team; one plan; natural supports
Assertive community treatment (ACT) (Stein & Test, 1980)	Adults with serious mental illness and frequent hospital use	Multidisciplinary team delivers services in the community, around the clock
Multisystemic therapy (MST) (Henggeler et al., 2009)	Youth with serious antisocial behavior	Intensive home-based treatment of family, peer, and school systems
Partial hospitalization (PHP)	People needing structured daily treatment without an overnight stay	A level of care: a day program

Exam point: "family-driven," "natural supports," and "one plan across systems" signal wraparound. "Adults with serious mental illness" plus "a team that comes to them" signals ACT.

Part 3 — Collaborating With Non-Clinical Stakeholders (5.2)

Non-clinical stakeholders include attorneys, courts, probation officers, schools, employers, child welfare workers, and community agencies. Many have their own legal duties, so their promise of confidentiality does not protect the client. The client's informed, documented consent does.

Attorneys and Courts

A request from an attorney, **even the client's own attorney**, does not authorize release. **ACA B.6.g** requires written client permission before records are disclosed or transferred to a legitimate third party, unless an exception applies. HIPAA requires a valid authorization for disclosures that are not treatment, payment, operations, or another permitted purpose (45 CFR 164.508).

Before the client signs, talk through how release could affect the case; records sent for a custody matter may reach the other side. Share only what the purpose needs (ACA B.2.e).

Document	What to do
Client's signed authorization	Release what it names, no more
Attorney's letter or request	Not an authorization; contact the client
Subpoena	Do not ignore it and do not simply comply; contact the client, consult legal counsel, assert privilege if the client has not waived it (see the ethics chapter)
Court order	Comply, seek the client's consent or ask to limit disclosure, and release as little as possible (ACA B.2.d; 45 CFR 164.512(e))

A blanket refusal is also wrong. With a valid authorization or court order, records can go to an attorney.

Probation, Parole, and Mandated Clients

ACA A.2.e requires counselors working with mandated clients to explain what information will be shared, with whom, and the limits of confidentiality. A probation officer may have to report to the court what he hears. Before an officer joins a session or receives anything, confirm a **current signed release names the officer and states what may be shared**. If the client is in a federally assisted substance use disorder program, 42 CFR Part 2 adds its own consent rules, including rules for criminal

justice referrals; verify the current Part 2 requirements, which have been revised in recent years.

Schools

With a parent's signed consent (and the student's assent where appropriate), the counselor may coordinate with a school counselor about classroom strategies. Share **only what the strategies need**: the goal, what helps, what to watch for. Full session notes are not needed (ACA B.2.e).

HIPAA does not bar consented coordination. Records a school holds are generally **education records under FERPA**, not protected health information under HIPAA; the counselor's own records stay under HIPAA and the ACA Code.

Employers and Other Third Parties

An employer, landlord, or human resources office asking about a client gets nothing without the client's authorization, not even confirmation that the person is a client. Tell the client about the request and let the client decide.

Advocacy Within Role

ACA A.7.a supports advocacy at the individual, group, institutional, and societal levels to address barriers to client growth. **ACA A.7.b** requires client consent before advocating for an identifiable client. The ACA Advocacy Competencies (Toporek et al., 2009) run from acting with the client, to acting on the client's behalf, to community and public-arena work.

Advocacy has a boundary. With consent, the counselor documents needs and functioning, writes support letters within competence, and **links the client to legal aid** or an attorney. Giving legal advice, filing complaints, and representing the client belong to attorneys. Refusing to help at all because "advocacy is outside counseling" ignores A.7.a.

Part 4 — Collaborating With Other Treatment Providers (5.3)

Care Coordination

When counselors learn a client is seeing another mental health professional, **ACA A.3** directs them to request a release to inform that professional and to build a collaborative relationship. The same logic applies to primary care, psychiatry, substance use treatment, and dietitians.

Good coordination has four parts:

1. **One plan, not parallel plans.** Agree on shared goals and who owns which piece.
2. **Clear roles.** The prescriber manages medication; the counselor provides counseling; the dietitian manages nutrition.
3. **A contact plan.** How often, by what secure channel (ACA B.3.e), and who calls whom when something changes.
4. **Documentation.** Each contact, what was shared, and what was decided.

When a referral moves care to another provider, **ACA A.11.d** requires completing the clinical and administrative steps and keeping communication open with both the client and the new provider. Transitions between levels of care are covered in the continuity-of-care section.

Interdisciplinary Teams

ACA B.3.b requires that clients be told when an interdisciplinary team serves them: that it exists, who is on it, what is shared, and why. **ACA D.1.c** asks counselors on teams to work toward the client's welfare while contributing the counseling profession's perspective.

Integrated care places behavioral health inside primary care. In the collaborative care model (Unützer et al., 2002), a care manager, the primary care provider, and a consulting psychiatrist track symptoms with repeated

measures and adjust treatment when the client is not improving.

What May Be Shared, Under Which Rule

Rule	What it says	Exam trap
HIPAA treatment disclosures (45 CFR 164.506)	Covered providers may share PHI with each other for treatment without authorization	The ACA Code (A.3, B.1.c) and many state laws still expect client consent; the rewarded answer gets a release
HIPAA minimum necessary (164.502(b), 164.514(d))	Applies to most uses and disclosures, but not to disclosures to a provider for treatment or those the client authorizes	The ethical duty to share only what is needed (ACA B.2.e) still applies
Psychotherapy notes (164.508(a)(2))	Separate process notes need a specific authorization, even when disclosed to another treating provider	Do not send them as part of routine coordination
42 CFR Part 2	Federally assisted SUD program records need consent that meets Part 2	Verify current requirements
State law	Some states are stricter than HIPAA for mental health records	HIPAA sets a floor; stricter state law applies

Consultation: Advice In, Responsibility Stays

Consultation brings in a colleague's expertise while the **treating counselor keeps responsibility** for the client's care. **ACA B.7.a** keeps consultation to professional purposes, with only the data germane to the question and every effort to protect the client's identity. **ACA B.7.b** bars sharing identifying information unless the client consents or it is unavoidable, and limits shared information to what the consultation needs. Contrast **supervision**, which is evaluative and shares responsibility, and **refer-**

ral or transfer, which moves responsibility to the new provider once complete.

Consultation for a specific skill or for ethical questions is covered in the self-awareness section.

Part 5 — Medication: Notice, Ask, Document, Pass It On (5.4)

Counselors do not prescribe. They often see the client more than anyone else on the team, though, so they are the first to hear that a medication causes drowsiness, kills appetite, ends sexual interest, or has not been taken in a week. The task is narrow: **notice the effect, ask about it, document it as the client's report, and, with consent, communicate it to the prescriber.** The prescriber decides what changes.

The Scope Line

ACA C.2.a limits practice to the counselor's education, training, and credentials, and counselor scope-of-practice laws do not include prescribing. Prescribers include psychiatrists, primary care physicians, psychiatric-mental health nurse practitioners, physician assistants, and, in a few states, psychologists with prescriptive authority.

Inside counselor scope	Outside counselor scope
Asking what the client takes and how it is going	Recommending, starting, adjusting, or stopping a medication
Recording reported benefits, side effects, timing, and adherence	Changing when a dose is taken, even "just take it at bedtime"
Recognizing possible side effects and passing them on	Deciding a dose is too high or too low, or that a different drug is needed
General psychoeducation (some medications take weeks to work); "talk with your prescriber before stopping"	Predicting that a side effect will fade

Inside counselor scope	Outside counselor scope
Referring for a medication evaluation; exploring ambivalence with motivational interviewing	Fluid, diet, or supplement advice that alters a medication's effect
Getting urgent medical help when danger signs appear	Interpreting lab values or diagnosing toxicity

Asking About Medication

Ask at intake and whenever the client mentions a change: what the client takes (including over-the-counter drugs, supplements such as St. John's wort, alcohol, and cannabis) and who prescribed it; when it started or last changed; what it helps; what side effects occur and when; how it affects work, school, sleep, driving, eating, relationships, and sex; whether doses are missed or the client is thinking of stopping; and whether the client will let the counselor share this with the prescriber.

Sexual side effects are a common reason people stop antidepressants and are rarely volunteered. Ask directly and without judgment.

Documenting the Self-Report

Element	Example
Name as the client gives it	"my anxiety pill," or the name read off the bottle
Prescriber	Psychiatric nurse practitioner
Start or change date	Started about three weeks ago, per client
Reported benefits	"I'm not lying awake worrying as much"
Reported side effects and timing	Nausea for an hour after the morning dose
Functional impact	Skipping breakfast meetings at work
Adherence	Reports missing two doses this week
Counselor action	With client's signed release, counselor will inform prescriber

Record benefits as well as side effects; the prescriber weighs both. “Client is on medication” is too vague to use. Record the report now; the client’s report is the data, and it does not need a physician’s confirmation first.

Self-report versus attribution. Document what the client reports, label it as the client’s report, and do not assign a cause.

Entry type	Example	Verdict
Self-report	“Client reports feeling ‘wired’ at bedtime since her ADHD medication was changed.”	Correct
Counselor observation	“Client yawned repeatedly; speech slowed late in session.”	Correct, labeled as observation
Counselor attributing cause	“The new medication is causing her insomnia.”	A medical judgment
Dosing opinion	“Dose appears too high.”	Prescriber’s role
Reframing as a psychiatric symptom	“Restlessness is likely anxiety, not a medication effect.”	Speculation in the record
Omission	Leaving the report out because medication “belongs in the physician’s chart”	Reported effects belong in the counseling record too

Passing It On: Routine, Prompt, Emergency

Urgency	Examples	Counselor action
Routine	Mild nausea, sexual side effects, daytime drowsiness, weight gain, questions	Document; with consent, inform the prescriber; encourage the client to raise it

Urgency	Examples	Counselor action
Prompt	New or worse suicidal thoughts after starting an antidepressant; akathisia; signs of mania after an antidepressant; stopping a medication abruptly; new rash on lamotrigine	Assess risk; contact the prescriber the same day; document
Emergency	The patterns in the emergency table below	Help the client get immediate medical evaluation (911 or emergency department); document what was reported and done

Consent governs routine communication with the prescriber. In an emergency, getting the client to medical care comes first; the ethics chapter covers when disclosure without consent is justified.

The antidepressant boxed warning. All antidepressants carry an FDA boxed warning for increased suicidal thoughts and behaviors in **children, adolescents, and young adults under age 25**, especially early in treatment and after dose changes. New suicidal thoughts, agitation, or restlessness in a young client who recently started an antidepressant call for a risk assessment, documentation, and prompt prescriber contact. Reassurance and advice to skip doses are both wrong; skipping doses is a medication decision and can cause discontinuation symptoms.

Antidepressant-induced mania. In a client with unrecognized bipolar disorder, an antidepressant can trigger hypomania or mania. Decreased need for sleep, racing thoughts, or risky spending soon after starting one call for prompt prescriber contact and a review of the diagnosis.

Drug-Class Reference

These tables give mechanisms, uses, side effects clients report, and what the counselor watches for. Doses are left out on purpose; they are outside counselor scope. Every agent in the antidepressant table carries the under-25 boxed warning described above. Mechanisms and side-effect profiles follow Stahl (2020, 2021) and FDA labeling.

Antidepressants

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
SSRIs: fluoxetine (Prozac), sertraline (Zoloft), escitalopram (Lexapro), citalopram (Celexa), paroxetine (Paxil), fluvoxamine	Block the serotonin reuptake transporter. First-line for depression, persistent depressive disorder, GAD, social anxiety, panic, OCD, PTSD	Sexual dysfunction (desire, arousal, orgasm), nausea, diarrhea, insomnia or sedation, sweating, headache, weight change	Full effect often takes 4–6 weeks; a client who quits at week 2 "because it isn't working" needs general psychoeducation and a prescriber conversation. Fluoxetine's long half-life means fewer discontinuation symptoms
SNRIs: venlafaxine (Effexor), desvenlafaxine (Pristiq), duloxetine (Cymbalta)	Block serotonin and norepinephrine reuptake. Depression and anxiety disorders; duloxetine also for fibromyalgia and nerve and musculoskeletal pain	SSRI-like effects plus raised blood pressure (especially venlafaxine)	Discontinuation symptoms are often stronger than with SSRIs. Duloxetine is common when depression and chronic pain co-occur

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
Bupropion (Wellbutrin; Zyban)	Blocks dopamine and norepinephrine reuptake. Depression, seasonal depression, smoking cessation	Insomnia, agitation, dry mouth; few sexual side effects	Lowers the seizure threshold, a concern with eating disorders, alcohol or sedative withdrawal, or seizure history (a key warning, not boxed). Sometimes added to offset SSRI sexual effects
Mirtazapine (Remeron)	Blocks alpha-2 and some serotonin receptors. Depression with insomnia and poor appetite	Sedation, increased appetite, weight gain	Often chosen for older adults with poor sleep and appetite
Trazodone	Serotonin antagonist and reuptake inhibitor. Often used off-label for sleep	Sedation, dizziness, lightheadedness on standing	**Priapism** is rare but an emergency
Tricyclics (TCAs): amitriptyline, nortriptyline, imipramine, desipramine	Block serotonin and norepinephrine reuptake plus cholinergic, histamine, and alpha-1 receptors. Now used mostly for pain, migraine prevention, and sleep	Dry mouth, blurred vision, constipation, urinary retention, confusion, dizziness on standing, sedation, weight gain, cardiac conduction effects	**Overdose is far more lethal than with SSRIs**: a suicidal client on a TCA calls for risk assessment, means-safety counseling, and prompt prescriber contact. Anticholinergic confusion is dangerous in older adults

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
MAOIs: phenelzine (Nardil), tranylcypromine (Parnate), selegiline patch (Emsam)	Inhibit monoamine oxidase, which breaks down serotonin, norepinephrine, dopamine, and tyramine. Treatment-resistant and atypical depression	Dizziness on standing, insomnia, weight gain	Tyramine-rich foods (aged cheese, cured meats, some fermented foods, tap beer, soy sauce) can cause a **hypertensive crisis**. Serotonergic drugs (SSRIs, SNRIs, tramadol, some cough medicines) can cause **serotonin syndrome**; prescribers build in a washout period when switching, longer after fluoxetine
Dextromethorphan-bupropion (Auvelity)	NMDA receptor antagonist plus bupropion. Major depressive disorder in adults	Dizziness, headache, dry mouth	Antidepressant boxed warning; bupropion's seizure caution

Newer Agents Clients May Mention

Agent	What it is	What the counselor should know
Esketamine (Spravato) nasal spray	NMDA receptor antagonist for adults with treatment-resistant depression or with major depression and acute suicidal thoughts or behavior	Given only in a certified health care setting under a REMS program, with monitoring after each dose; never taken home. Boxed warnings: sedation, dissociation, respiratory depression, abuse and misuse, suicidal thoughts and behaviors. No driving until the next day after restful sleep, so transportation is a planning issue. Feeling "detached" after treatment is a reportable effect
Zuranolone (Zurzuvae)	Oral neuroactive steroid that enhances GABA-A activity; a short course for postpartum depression in adults	Boxed warning for impaired driving: no driving or hazardous activity for at least 12 hours after each dose. Coordinate with obstetric and psychiatric prescribers
Xanomeline-trospium (Cobenfy)	Oral antipsychotic for schizophrenia in adults that acts on muscarinic receptors instead of blocking dopamine D2 receptors; trospium limits effects outside the brain	Clients may report nausea, vomiting, indigestion, constipation, fast heartbeat, or raised blood pressure; urinary retention is a warning. It lacks the usual D2-blocker movement profile, but any new effect is still reported, not interpreted

Mood Stabilizers

Agent	Uses	Key risks	Counselor watches for
Lithium	Bipolar I and II, especially mania and maintenance; depression augmentation; linked to lower suicide risk	Boxed warning: **toxicity**. Narrow therapeutic range; the prescriber monitors blood levels. Long term: hypothyroidism, weight gain, thirst and frequent urination, acne, kidney effects	**Toxicity:** coarse tremor, vomiting, diarrhea, confusion, unsteady gait, slurred speech; in severe cases seizures and heart rhythm problems. Levels rise with dehydration, vomiting or diarrhea, heavy sweating, low-salt diets, NSAIDs (ibuprofen, naproxen), and some blood-pressure medicines. Mild, steady side effects are routine reports; a coarse tremor with GI symptoms or confusion is an emergency. Never advise on fluids or salt
Valproate (Depakote)	Bipolar disorder (including mixed features and rapid cycling), epilepsy, migraine prevention	Boxed warnings: **liver toxicity, pancreatitis, major birth-defect risk** (neural tube defects, lower IQ in exposed children)	Coordinate with the prescriber if a client who could become pregnant plans or reports a pregnancy

Agent	Uses	Key risks	Counselor watches for
Lamotrigine (Lamictal)	Bipolar maintenance, especially preventing depression; epilepsy	Boxed warning: **serious rash, including Stevens-Johnson syndrome**, more likely with rapid dose increases or with valproate	Report any new rash while starting it at once. Often preferred over valproate for people of reproductive age
Carbamazepine (Tegretol)	Bipolar disorder, epilepsy, trigeminal neuralgia	Boxed warnings: **serious skin reactions** (HLA-B*1502 screening in at-risk ancestry); **aplastic anemia and agranulocytosis**. Also liver effects and birth-defect risk	Lowers levels of many other drugs, including hormonal contraceptives. Fever, sore throat, or unusual bruising is reported promptly

Antipsychotics

Group and agents	Mechanism	Key side effects	Counselor watches for
First-generation (typical): haloperidol (Haldol), chlorpromazine, fluphenazine, perphenazine	Dopamine D2 blockade	**Extrapyramidal symptoms (EPS)**: **akathisia** (inner restlessness and urge to move, often mistaken for anxiety or worsening psychosis, and linked to distress and suicidality), **acute dystonia** (sudden spasms of neck, eyes, or jaw), **drug-induced parkinsonism** (tremor, rigidity, shuffling gait). **Tardive dyskinesia (TD)**: involuntary lip, tongue, or limb movements after long-term use; may be permanent. **Neuroleptic malignant syndrome (NMS)**	Report every movement change. TD is screened with the AIMS (Guy, 1976) and treated with VMAT2 inhibitors such as valbenazine (Ingrezza) and deutetrabenazine (Austedo). NMS is an emergency

Group and agents	Mechanism	Key side effects	Counselor watches for
Second-generation (atypical): risperidone (Risperdal), olanzapine (Zyprexa), quetiapine (Seroquel), aripiprazole (Abilify), ziprasidone (Geodon), lurasidone (Latuda), brexpiprazole (Rexulti), cariprazine (Vraylar), lumateperone (Caplyta)	D2 and serotonin 5-HT2A blockade, or D2 partial agonism (aripiprazole, brexpiprazole, cariprazine)	**Metabolic effects**: weight gain, raised blood sugar and lipids (highest with olanzapine and clozapine, notable with quetiapine); sedation; EPS and akathisia are possible	Report significant weight gain, new thirst, or frequent urination; the prescriber monitors weight, glucose, and lipids. Quetiapine is often used off-label for sleep. Agents approved for depression carry the antidepressant boxed warning

Group and agents	Mechanism	Key side effects	Counselor watches for
Clozapine (Clozaril)	Multiple receptors; weak D2 blockade	Boxed warnings: **severe neutropenia**, orthostatic hypotension and fainting, **seizures**, **myocarditis and cardiomyopathy**, mortality in older adults with dementia-related psychosis. High metabolic risk; drooling; constipation that can become dangerous	The most effective antipsychotic for treatment-resistant schizophrenia, with evidence for reducing suicidal behavior in schizophrenia. The prescriber orders regular blood (ANC) monitoring. Report fever, sore throat, chest pain, or a racing heart promptly. The FDA ended the clozapine REMS program in 2025, so registry enrollment and ANC reporting to the registry are no longer required; the monitoring itself continues

First-generation, second-generation, and clozapine labels carry a boxed warning for **increased mortality in older adults with dementia-related psychosis**; xanomeline-trospium has no boxed warning. TD and NMS are major warnings but not boxed.

Anxiolytics and Sleep Medications

Class and agents	Mechanism	Key risks	Counselor watches for
Benzodiazepines: diazepam (Valium), lorazepam (Ativan), alprazolam (Xanax), clonazepam (Klonopin), chlordiazepoxide (Librium)	Enhance GABA-A receptor activity, depressing the central nervous system	Boxed warnings: **abuse, misuse, and addiction; dependence and withdrawal reactions; profound sedation, slowed breathing, and death when combined with opioids**. Sedation, memory problems, falls in older adults	Not a long-term anxiety treatment; dependence can develop within weeks. **Withdrawal after regular use can cause seizures and can be life-threatening**, so stopping needs medical management. Ask about alcohol and opioids. Used medically for alcohol withdrawal
Buspirone	Serotonin 5-HT1A partial agonist	Dizziness, nausea, headache	For GAD; takes 2–4 weeks; useless as-needed or for panic. No dependence. Clients used to fast benzodiazepine relief may call it ineffective

Class and agents	Mechanism	Key risks	Counselor watches for
Z-drugs: zolpidem (Ambien), zaleplon (Sonata), eszopiclone (Lunesta)	Act at GABA-A receptors	Boxed warning: **complex sleep behaviors** (sleepwalking, sleep-driving, sometimes with no memory and serious injury)	Report complex sleep behavior promptly. Long-term use is a reason to suggest CBT-I, the first-line treatment for chronic insomnia (Qaseem et al., 2016), and to support a prescriber conversation

ADHD Medications

Class and agents	Mechanism	Key side effects	Counselor watches for
Stimulants: amphetamine salts (Adderall), lisdexamfetamine (Vyvanse, a prodrug with lower misuse potential), methylphenidate (Ritalin, Concerta), dexmethylphenidate (Focalin)	Increase dopamine and norepinephrine release and block reuptake	**Reduced appetite** (growth monitored in children), **trouble falling asleep**, raised heart rate and blood pressure, irritability, tics, rebound as doses wear off	Schedule II. Boxed warning: **abuse, misuse, and addiction**. Watch for taking extra, use to stay awake or study, and diversion. Cardiac conditions need medical coordination
Atomoxetine (Strattera)	Selective norepinephrine reuptake inhibitor	GI upset, decreased appetite, fatigue	Not controlled; takes weeks. Boxed warning: **suicidal thoughts in children and adolescents**

Class and agents	Mechanism	Key side effects	Counselor watches for
Viloxazine (Qelbree)	Norepinephrine reuptake inhibition, among other actions	Sleepiness, decreased appetite	Boxed warning: suicidal thoughts and behaviors
Alpha-2 agonists: guanfacine ER (Intuniv), clonidine ER (Kapvay)	Stimulate alpha-2 receptors	Sedation, low blood pressure; rebound high blood pressure if stopped abruptly	Helpful with tics or sleep problems. Clonidine, guanfacine, and the alpha-1 blocker prazosin are also used off-label for PTSD nightmares

Substance Use and Neurocognitive Medications

Medication	Use	Counselor watches for
Buprenorphine (alone or with naloxone); **methadone**	Opioid use disorder	Treatment, not "trading one drug for another." Methadone for OUD comes through opioid treatment programs. Benzodiazepines or alcohol add overdose risk
Naltrexone (oral or monthly injection)	Opioid (after withdrawal) and alcohol use disorders	Lower opioid tolerance after stopping raises overdose risk if use resumes
Disulfiram (Antabuse)	Alcohol use disorder; alcohol causes a severe reaction (flushing, nausea, pounding heart)	Boxed warning: never given to an intoxicated person or without full knowledge. Hidden alcohol (mouthwash, cooking) can trigger it

Medication	Use	Counselor watches for
Naloxone (Narcan)	Reverses opioid overdose	Belongs in the safety plan of anyone at risk; available over the counter
Cholinesterase inhibitors: donepezil (Aricept), rivastigmine (Exelon), galantamine (Razadyne); **memantine** (Namenda, an NMDA antagonist)	Alzheimer's disease; cholinesterase inhibitors also in Lewy body dementia	They give modest, temporary symptom benefit and do not stop or reverse the disease, which helps set realistic expectations with families. Nausea, diarrhea, slow heart rate; dizziness and confusion with memantine

In older adults, the AGS Beers Criteria (American Geriatrics Society, 2023) flag benzodiazepines and strongly anticholinergic drugs as medications to avoid or use with caution. Report new confusion or falls. The substance use chapter covers withdrawal management and medication decisions in addiction.

Emergencies to Recognize

The counselor does not diagnose these. The counselor recognizes the pattern well enough to get the client to medical care, then documents what was reported and done.

Emergency	Typical cause	What the client may report or show
Serotonin syndrome	MAOI with SSRIs, SNRIs, tramadol, or other serotonergic drugs; St. John's wort or MDMA combinations	Agitation, confusion, sweating, fever, fast heartbeat, muscle twitching or rigidity, diarrhea
Hypertensive crisis	MAOI with tyramine-rich foods or some decongestants	Sudden severe headache, stiff neck, pounding heart, nausea

Emergency	Typical cause	What the client may report or show
Lithium toxicity	Dehydration, GI illness, low salt, NSAIDs, some blood-pressure drugs	Coarse tremor, vomiting, diarrhea, confusion, unsteady gait, slurred speech
Neuroleptic malignant syndrome	Antipsychotics	High fever, severe rigidity, confusion, unstable pulse and blood pressure
Respiratory depression	Benzodiazepines with opioids or alcohol	Extreme drowsiness, slow or shallow breathing, unresponsiveness
Sedative or alcohol withdrawal	Abrupt stop after regular use	Tremor, sweating, agitation, hallucinations, seizures
Serious skin reaction	Lamotrigine, carbamazepine	Spreading rash, blisters, mouth sores, fever
Priapism	Trazodone, rarely others	Prolonged, painful erection

Discontinuation symptoms are not an emergency but are often mistaken for relapse. Stopping an SSRI or SNRI abruptly, especially paroxetine or venlafaxine, can cause the FINISH symptoms (Berber, 1998): flu-like symptoms, insomnia, nausea, imbalance, sensory disturbances ("brain zaps"), and hyperarousal. Document the report and encourage the client to talk with the prescriber before stopping any medication.

Part 6 — The Referral Resource List (5.5)

Why the List Is an Ethical Duty

ACA A.11.a expects counselors to know **culturally and clinically appropriate referral resources** and to suggest them when they lack the competence to help; if the client declines the referral, the counselor discontinues the relationship. An outdated list fails that duty at the mo-

ment it matters. **ACA A.12** bars abandonment, so ending services without arranging alternatives is not an option. **ACA A.10.b** prohibits fees, commissions, or rebates for referrals.

What Each Entry Holds

Field	Why it matters
Name, location, telehealth, contact	The client can reach it
Specialty, populations, and treatments offered	Fit for the need (eating disorders, trauma, adolescents, OCD)
License status, verified with the state board	The provider is qualified
Languages offered; accessibility	Language and disability access
Insurance accepted, sliding scale, cost	The client can afford it
Openings or waitlist	Avoids dead-end referrals
Level of care (outpatient, IOP, PHP, residential, inpatient)	Matches the assessed need
Date last verified	Shows when the entry was last checked

Keeping It Current

- **Verify on a schedule** (specialty, license, openings, insurance) and whenever a client reports a problem.
- **Vet before adding.** A name a colleague mentions in passing is a lead, not a listing.
- **Retire dead entries** promptly, and check what replaced them.
- **Avoid reciprocity as a criterion.** Keeping only providers who refer back is a conflict of interest.

When a listed resource turns out to be closed or full: secure a current option for the client **now**, then update the list and set a routine check. Handing over the dead listing, dropping to a lower level of care because the list failed, telling the client to search alone, or deleting

the entry and waiting for an annual review all shift the cost of a stale list onto the client.

Making the Referral

Explain why the referral is recommended and involve the client in the decision. Offer **several vetted options** that fit the need so the client chooses; naming one preferred colleague limits choice and can look like a conflict of interest, and sending the client to search online unaided skips the vetting. Get a signed release before sharing information, and **follow up** to confirm the client connected (ACA A.11.d).

Matching the Resource to the Client

Resource	Best fit
Community mental health center	Uninsured or low-income clients needing ongoing care; sliding-scale fees
Federally qualified health center (FQHC)	Integrated primary and behavioral care with sliding-fee discounts
Employee assistance program (EAP)	Employed clients; short-term, employer-sponsored; not available to someone unemployed
Out-of-network private practice	Clients who can pay or have out-of-network benefits
University training clinic	Low-cost care from supervised trainees
Mutual-help groups (AA, NA, SMART Recovery, NAMI programs)	Peer support alongside treatment
Legal aid; vocational rehabilitation; housing and benefits agencies	Legal, work, and basic-needs barriers

Crisis and National Lines

Line	What it does
988 Suicide & Crisis Lifeline	National three-digit line for suicidal and mental health crises; call, text, or chat. Veterans and service members press 1 after dialing 988 for the Veterans Crisis Line
911	Immediate danger to life; medical emergencies
Crisis Text Line	Text HOME to 741741
211	Community information and referral: housing, food, utilities, social services
311	Non-emergency city services in some cities
National Domestic Violence Hotline	1-800-799-7233
SAMHSA National Helpline	Treatment referral and information; 1-800-662-4357
Poison Control	1-800-222-1222, including medication overdoses and mix-ups

Every referral list carries 988. Do not confuse it with 211 (social services) or 311 (municipal services).

Part 7 — Reviewing the Record Before the Session (5.6)

Pre-session review turns last week's session into this week's plan. It is a clinical task, not a billing one, done before the client arrives.

What to Review

Item	Why
Last progress note and its plan	Continuity: what was assigned, what was left open
Treatment plan goals and objectives	Keeps the session tied to the client's goals

Item	Why
Homework and self-monitoring data (drinking log, thought records, activity schedule)	Shows what happened between sessions
Repeated measures (PHQ-9, GAD-7, others)	Trend over time
Risk history and current safety plan	Prior risk carries forward
Medication changes reported	Something to follow up
Collateral contacts and messages since last session	New information from family, providers, or the client
Releases on file: who, what scope, expiration	Determines who may join or be contacted
Pending tasks: referral follow-up, letters, coordination calls	Closes loops

Memory misses details and trends. Intake paperwork alone misses recent progress. Skimming notes in session spends the client's time on the counselor's preparation.

Turning Review Into a Plan

Risk carries forward. When a recent note records suicidal thinking, plan to reassess risk and review the safety plan **early** in the session. Waiting for the client to bring it up risks missing a change; jumping to a higher level of care before a current assessment goes beyond the evidence. The crisis chapter covers risk assessment.

Measurement-based care. Reviewing repeated scores lets the counselor see the trend and adjust treatment, ideally reviewing that trend with the client (Lewis et al., 2019). The PHQ-9 (Kroenke et al., 2001) is a screening and severity measure; a score does not confirm a diagnosis without further assessment.

Consent check. If someone else will attend, confirm a **current signed release** names that person and states what may be shared. An expired release is not a release.

Covering for a Colleague

Before seeing a colleague's client during a leave, review the treatment plan, recent notes, and risk history. "Starting fresh" risks missing risk factors; asking for a full retelling burdens the client with what the record already holds. The continuity-of-care section covers transfers and handoffs.

Part 8 — Language Access (5.7)

Interpreter or Translator

Role	Works with	Example
Interpreter	**Spoken or signed** language, in real time	Session with a client whose preferred language is not English; ASL interpreting for a Deaf client
Translator	**Written** text	Informed consent form, safety plan handout, intake forms
Sight translation (by an interpreter)	Reading a written document aloud in another language	Reviewing a consent form during the session

Who Is Qualified, and Who Is Not

A **qualified interpreter** is proficient in both languages, trained in interpreting, bound by confidentiality and an interpreter code of ethics, and able to interpret accurately and impartially, ideally with mental health experience. Being bilingual is not the same skill.

Do not use	Why
The client (interpreting for a family member)	Dual role; filtered or inaccurate messages; the client cannot both participate and interpret
Family members	Bias, protectiveness, secrets, and confidentiality problems; they may soften or omit

Do not use	Why
Minor children	Burdens the child, exposes the child to adult clinical content, and risks inaccuracy; only in an emergency when no qualified interpreter is available
Untrained bilingual staff (such as a receptionist)	Not trained in accuracy or role; confidentiality issues
Consumer translation apps	Inaccurate for clinical content and a privacy risk
Speaking English slowly	Does not provide meaningful access

When no in-person interpreter is available, a **qualified telephone or video remote interpreter** gives accurate access right away.

The Rules

ACA A.2.c directs counselors to communicate in culturally and developmentally appropriate ways and, when clients have difficulty understanding the language counselors use, to arrange **a qualified interpreter or translator** to ensure comprehension.

ADA Title III treats private counseling offices as places of public accommodation. They must provide auxiliary aids and services, including qualified interpreters, for **effective communication** with people who are deaf, hard of hearing, or have other communication disabilities (28 CFR 36.303). The provider may not charge the client for them (28 CFR 36.301(c)). Under 36.303(c), the provider may not require the person to bring someone to interpret; an accompanying adult may interpret only in an emergency involving an imminent threat to safety or welfare when no interpreter is available, or when the person specifically requests it, the adult agrees, and reliance is appropriate; a minor child may interpret only in that kind of emergency.

Title VI of the Civil Rights Act and **Section 1557 of the Affordable Care Act** require programs that receive

federal financial assistance, including many health programs, to provide meaningful access for people with limited English proficiency. Implementing rules have changed over time; verify current requirements.

The **National CLAS Standards** (HHS Office of Minority Health, 2013) call for free, timely language assistance, notice that it is available, competent interpreters rather than untrained individuals or minors, and materials in the languages common in the service area.

Deaf and Hard-of-Hearing Clients

For a Deaf client whose preferred language is American Sign Language, arrange a **qualified ASL interpreter**. Written notes are too slow and limited for counseling, and English may be the client's second language. Lip-reading (speechreading) captures only part of spoken English. Ask the client what works; preferences vary.

Running an Interpreted Session

Before: - Book the interpreter in advance, in the client's preferred language and dialect, and check that the interpreter does not know the client personally. - **Brief the interpreter**: the session's purpose, sensitive topics, key terms, confidentiality, and the expectation to interpret everything in the first person.

During: - Seat yourself facing the client; an ASL interpreter sits near the counselor so the client can see both. - **Speak directly to the client** in the first person ("How have you been sleeping?"), not to the interpreter ("Ask her how she has been sleeping"). - Speak in **short segments** so the interpreter can render everything accurately. - Expect everything to be interpreted. **Summaries lose meaning and emotional content.** - If the interpreter adds advice, edits, or side conversations, pause and restate the interpreter's role.

After: - Debrief the interpreter: clarify cultural or language points, and check in after distressing content, since interpreters are exposed to traumatic material too.

Written Materials and Consent

A signature on a form the client cannot read does not make consent informed. Simple English still does not reach a client whose preferred language is not English. Provide a **qualified translation** of the consent form, or have a qualified interpreter sight-translate it, and **review it together**, answering questions through the interpreter (ACA A.2.c).

Documenting Language Access

Record the client's preferred spoken and written language, the interpreter's name or ID number, the mode (in person, phone, video), and any translated documents. If a client declines a professional interpreter, document the offer and the refusal.

Part 9 — Writing Clinical Documentation (5.8)

Why the Note Exists

ACA A.1.b requires sufficient, timely documentation that supports the delivery and continuity of services and accurately reflects progress and services; **ACA B.6.a** restates the duty to keep necessary records. The note serves care first, telling the next session, a covering counselor, and coordinating providers what happened. It also supports billing, meets legal requirements, and shows the reasoning behind decisions.

What Every Progress Note Contains

- Client identifier, date, start and end time, service type, setting (telehealth included)
- Who attended, including interpreters and support persons
- The client's report, quoted where the words matter; the counselor's observations and any measure scores
- Interventions, tied to the treatment plan, and the client's response

- **Risk assessment** and any safety actions
- Progress toward goals; reported medication effects and adherence
- Collateral contacts, consultations, and releases
- The plan; the counselor's signature, credentials, and date signed

Write in plain, objective, behavioral language. Leave out judgmental labels ("manipulative"), speculation about people who are not clients ("her mother is clearly narcissistic"), and details irrelevant to care.

Note Formats

Format	Sections	Where things go
SOAP (Weed, 1968)	**S**ubjective, **O**bjective, **A**ssessment, **P**lan	Client's report in S; counselor observations and scores in O; clinical impression and progress in A; next steps in P
DAP	**D**ata, **A**ssessment, **P**lan	Client report and counselor observations combined in D
BIRP	**B**ehavior, **I**ntervention, **R**esponse, **P**lan	Client's presentation in B; what the counselor did in I; how the client responded in R
GIRP	**G**oal, **I**ntervention, **R**esponse, **P**lan	Starts from the treatment-plan goal addressed

Exam favorites: the client's quoted statement goes in S (SOAP) or D (DAP). A score of 14 on the PHQ-9 goes in O (SOAP) or D (DAP). "Client rated anxiety 7 before and 4 after the grounding exercise" is the client's response to an intervention: R in BIRP or GIRP.

Self-Report, Observation, and Attribution

The note keeps three kinds of statements distinct:

Kind	Signal words	Example
Client self-report	"Client reports," "client states," quotation marks	Client states, "I've been up every night since Sunday."
Counselor observation	"Counselor observed," "client presented with"	Speech slowed; affect flat; eye contact limited
Clinical assessment	"Assessment," "impression," "consistent with"	Symptoms consistent with continued moderate depression; progress toward objective 2 limited

Counselors may record clinical impressions within their competence, including diagnostic impressions. They do not record medical conclusions, such as what caused a side effect or whether a dose is right. Those stay as the client's report plus the action taken.

When to Write

- **During the session (collaborative documentation):** the counselor and client write part of the note together in the last minutes, reviewing what was covered and the plan. It can improve accuracy and engagement. Explain it in informed consent, and do not let typing replace attending.
- **After the session:** write as soon as practical, while memory is accurate, and within the deadline your agency, payer, or state sets. Always finish before the next session and before any handoff.

Corrections, Late Entries, and Addenda

Situation	Correct action	Wrong action
Something omitted from a signed note	A **dated, signed addendum** that references the original session date and states the added information	Rewriting the signed note (can look like falsifying the record); adding it to next week's note as new information (misdates when the client said it)

Situation	Correct action	Wrong action
An error in a note	Mark the error as corrected (in paper records, a single line through it, initialed and dated; in an EHR, the amendment function), keeping the original readable	Erasing, obliterating, or deleting
A note written late	Label it a late entry with the actual date written and the date of service	Backdating

ACA A.1.b expects amendments to be noted according to agency or institutional policy. Under HIPAA, a client may request an amendment to the record (45 CFR 164.526). The provider may deny a request when the record is accurate and complete, and the client may then file a statement of disagreement.

Psychotherapy Notes and the Clinical Record

HIPAA separates the **clinical record** (dates and times of service, diagnosis, treatment plan, progress summaries, medications, test results) from **psychotherapy notes**: the counselor's analysis of session content, kept separately from the rest of the record. Psychotherapy notes need a specific authorization for most disclosures (45 CFR 164.508(a)(2)) and are excluded from the client's right of access under HIPAA (45 CFR 164.524), though state law may differ. Medication reports, risk assessments, and anything another provider needs belong in the clinical record, not only in psychotherapy notes. Clients' access to their records under the ACA Code is covered in the ethics chapter.

Documenting Coordination

Each collateral contact gets an entry: date, who, by what means, the release that authorized it, what was shared, and what was decided.

What the Exam Rewards / What It Punishes

Rewards: - A signed release naming the person, the scope, and the purpose before any contact - Specific, client-endorsed roles for supports, tied to goals - One coordinated plan with clear roles and a contact plan - Advocacy within the counselor's role, plus links to legal services - Recording medication effects as self-report and passing them to the prescriber with consent - Immediate medical evaluation for toxicity or emergency signs - A verified referral list, several options, and active follow-up - Pre-session review of the last note, goals, data, risk, and releases - Qualified, briefed interpreters; speaking directly to the client - Timely, objective notes; dated addenda for corrections

Punishes: - Calling family, attorneys, officers, or advocates without consent - Treating a release as opening the whole record - Turning supports into covert monitors - Doing someone else's job (filing legal complaints, setting family goals for the client, adjusting medication) - Medication advice, dosing opinions, or attributing a cause in the note - Reassuring away new suicidal thoughts in a young client on an antidepressant - Handing a client a dead or unvetted referral - Walking into a session without reviewing the record - Family members, children, apps, or slow English in place of an interpreter - Rewriting signed notes or backdating entries

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End of Part Two. Continue in Part Three and Back Matter.

PART THREE: CLINICAL QUICK REFERENCE

Defense Mechanisms Quick Reference

Defense mechanisms are unconscious psychological processes that protect the ego from anxiety. The NCMHCE tests your ability to identify them in case vignettes — often as part of a psychodynamic conceptualization question.

Mature Defenses (adaptive; common in healthy adults)

Mechanism	Definition	Example on Exam
Sublimation	Channeling unacceptable impulses into socially acceptable outlets	A client with aggressive urges becomes a competitive athlete
Humor	Using comedy to manage uncomfortable feelings without denial	A client jokes about their diagnosis in a way that allows perspective-taking
Altruism	Relieving one's own distress by helping others	A client who experienced childhood poverty becomes a social worker
Suppression	Consciously setting aside a distressing thought to deal with later	A client decides not to think about the diagnosis until after the presentation

How it's tested: A vignette describes a client who "channels his anger into competitive sports" — this is sublimation. The exam may ask you to identify it or distinguish it from displacement.

Neurotic (Intermediate) Defenses (less adaptive; common in neurotic range disorders)

Mechanism	Definition	Example on Exam
Repression	Unconscious exclusion of distressing material from awareness	Client has no memory of traumatic childhood events
Reaction Formation	Expressing the opposite of an unacceptable feeling	A client who feels rage toward a parent becomes excessively devoted
Displacement	Redirecting an impulse from the true target to a safer one	Client yells at her children after a bad day with her boss
Rationalization	Creating logical justifications for unacceptable feelings or behaviors	Client who overspends: "I work hard; I deserve nice things"
Intellectualization	Using abstract thinking to distance from emotional content	A client discusses grief using clinical terminology without showing affect
Undoing	Performing an action to "undo" a transgression	Client feels guilty about a thought and then completes a ritual to cancel it
Isolation of Affect	Separating thoughts from their associated emotions	Client recounts a traumatic event in complete detail with no emotional response

How it's tested: Intellectualization frequently appears in cases where a client describes a loss using clinical or distant language. Reaction formation appears in cases where the stated feeling and the behavioral evidence are directly opposite.

Immature Defenses (maladaptive; common in personality disorders, severe pathology)

Mechanism	Definition	Example on Exam
Denial	Refusing to acknowledge a painful reality	Client with a serious medical diagnosis says “I don’t believe there’s really anything wrong”
Projection	Attributing one’s own unacceptable feelings to others	Client says “everyone here is judging me” when she is the one judging herself
Splitting	Seeing people as all-good or all-bad; inability to tolerate ambivalence	BPD client idealizes the counselor one week, devalues the next
Regression	Returning to an earlier developmental stage under stress	An adult client begins speaking in a childlike voice when discussing abuse
Somatization	Converting psychological distress into physical symptoms	Client develops unexplained stomach pain before difficult family events
Projective Identification	Causing others to actually feel what you are projecting	BPD client subtly behaves in ways that cause the counselor to feel rage — then attributes the rage to the counselor
Passive Aggression	Expressing hostility indirectly	Client “forgets” appointments consistently when angry at the counselor
Acting Out	Expressing internal conflict through action rather than words	Client gets into a bar fight after a session where trauma was discussed

How it’s tested: Splitting is almost always tested in the context of BPD. Projection and projective identification may appear together in personality disorder questions.

Acting out appears in cases where the client's behavior outside sessions reflects internal session material.

Cognitive Distortions Quick Reference

Cognitive distortions are errors in thinking identified in CBT that maintain depression, anxiety, and other conditions (Beck, 2021). The exam tests your ability to identify them in client statements.

Distortion	Definition	Client Statement Example
All-or-nothing thinking (black-and-white)	Seeing things in extreme categories	"If I'm not perfect at this, I'm a complete failure."
Catastrophizing	Predicting the worst possible outcome	"If I fail this test, my life is over."
Mind reading	Assuming knowledge of others' thoughts	"I know she thinks I'm stupid."
Fortune telling	Predicting a negative future as fact	"I'll never be able to manage on my own."
Emotional reasoning	Assuming feelings reflect reality	"I feel worthless, so I must be worthless."
Personalization	Blaming oneself for events beyond control	"My parents divorced because of me."
Overgeneralization	Drawing broad conclusions from single events	"I made one mistake — I always mess everything up."
Mental filter	Focusing exclusively on one negative detail	The client hears universally positive feedback and fixates on one mild critique
Discounting the positive	Dismissing positive experiences as not counting	"The promotion doesn't count — they just felt sorry for me."
Should statements	Rigid rules about self/others	"I should be able to handle this without help."

Distortion	Definition	Client Statement Example
Labeling	Attaching a global negative label to self or others	"I'm just a loser." "She's completely toxic."
Magnification/ minimization	Amplifying negatives; shrinking positives	"My one strength doesn't matter. My one failure defines me."

Restructuring Context

The goal of cognitive restructuring is not to replace negative thoughts with positive ones — it is to replace distorted thoughts with more accurate, evidence-based ones. The Socratic questioning sequence: 1. What is the evidence for and against this thought? 2. What would you tell a friend who had this thought? 3. Is this thought helping you or hurting you? 4. What is a more accurate way to describe this situation? Exam tip: The exam will present a client statement and ask you to identify the distortion, or present a counselor response and ask whether it correctly targets the distortion. Know the distortions by both definition and example.

Assessment Tools Compendium

All instruments organized by domain for cross-reference with the clinical chapters. Cutoffs are those published by the instrument developers; clinical interpretation always considers the individual's baseline, culture, and context. Each row names the primary source for the instrument.

Depression

Tool	Type	Items	Cutoffs / Notes	Source
PHQ-9	Self-report	9	0–4 minimal; 5–9 mild; 10–14 moderate; 15–19 moderately severe; 20–27 severe. Item 9 screens suicidal ideation.	Kroenke et al., 2001
BDI-II	Self-report	21	0–13 minimal; 14–19 mild; 20–28 moderate; 29–63 severe	Beck et al., 1996
HAM-D (HDRS)	Clinician-rated	17 or 21	0–7 normal; 8–16 mild; 17–23 moderate; ≥24 severe	Hamilton, 1960
EPDS	Self-report	10	Perinatal depression screen; ≥13 warrants follow-up; item 10 screens self-harm	Cox et al., 1987

Tool	Type	Items	Cutoffs / Notes	Source
MADRS	Clinician-rated	10	Montgomery–Åsberg; sensitive to change; widely used in medication trials	Montgomery & Åsberg, 1979
GDS-15	Self-report	15	Geriatric Depression Scale short form; ≥5 suggests depression; ≥10 almost always depression	Sheikh & Yesavage, 1986

Anxiety

Tool	Type	Items	Cutoffs / Notes	Source
GAD-7	Self-report	7	5 mild; 10 moderate; 15 severe	Spitzer et al., 2006
BAI	Self-report	21	0–21 low; 22–35 moderate; 36+ potentially concerning	Beck et al., 1988
PDSS	Clinician-rated	7	Panic disorder severity; higher score = greater severity	Shear et al., 1997
SUDS	Client-rated	1	0–100 distress rating used within exposure sessions	Wolpe, 1969

Tool	Type	Items	Cutoffs / Notes	Source
SPIN	Self-report	17	Social Phobia Inventory; ≥19 suggests social anxiety disorder	Connor et al., 2000

Trauma

Tool	Type	Items	Cutoffs / Notes	Source
PCL-5	Self-report	20	Provisional PTSD: ≥31–33; maps item-by-item to DSM-5 criteria	Weathers et al., 2013
CAPS-5	Clinician-administered	Structured interview	Gold standard for PTSD diagnosis and severity	Weathers et al., 2018
TSI-2	Self-report	136	Comprehensive trauma-related symptoms, including complex trauma	Briere, 2011
IES-R	Self-report	22	Impact of Event Scale–Revised; intrusion, avoidance, hyperarousal subscales	Weiss & Marmar, 1997
ACE Score	Self-report	10	Adverse childhood experiences; not diagnostic — cumulative risk indicator	Felitti et al., 1998

Substance Use

Tool	Type	Items	Cutoffs / Notes	Source
AUDIT	Self-report	10	8–15 brief advice; 16–19 counseling; ≥20 referral	Saunders et al., 1993
AUDIT-C	Self-report	3	≥4 (men) or ≥3 (women): positive screen	Bush et al., 1998
CAGE	Clinical interview	4	≥2 positive: clinically significant alcohol use	Ewing, 1984
DAST-10	Self-report	10	≥3 need for assessment; ≥6 substantial problems	Skinner, 1982
CRAFFT	Self-report (adolescents)	6	≥2 positive screen	Knight et al., 2002
MAST	Self-report	25	≥5 alcoholism probable	Selzer, 1971
ASI	Structured interview	Multi-domain	Addiction Severity Index; 7 domains for treatment planning	McLellan et al., 1992

Suicidality

Tool	Type	Key Features	Source
C-SSRS	Clinician-administered	Ideation (5 levels) plus behavior; gold standard for risk research	Posner et al., 2011

Tool	Type	Key Features	Source
Columbia Protocol	Clinical decision aid	Guides action based on C-SSRS responses	Posner et al., 2011
SBQ-R	Self-report	4 items; ≥7 (general) or ≥8 (psychiatric) indicates risk	Osman et al., 2001

Personality and Comprehensive Assessment

Tool	Type	Notes	Source
MMPI-2 / MMPI-3	Self-report	567 / 335 items; comprehensive personality and psychopathology; requires licensed psychologist for interpretation	Ben-Porath & Tellegen, 2020; Butcher et al., 2001
MCMI-IV	Self-report	195 items; personality disorders plus clinical syndromes	Millon et al., 2015
MSI-BPD	Self-report	10 items; BPD screen; ≥7 warrants further evaluation	Zanarini et al., 2003
PAI	Self-report	344 items; Personality Assessment Inventory	Morey, 2007

Cognitive and Neurodevelopmental

Tool	Type	Notes	Source
MMSE	Clinician-administered	Brief cognitive screen; 0–30; ≤23 suggests impairment	Folstein et al., 1975

Tool	Type	Notes	Source
MoCA	Clinician-administered	More sensitive than MMSE for mild impairment; <26 possible impairment	Nasreddine et al., 2005
WAIS-IV / WISC-V	Clinician-administered	Full intelligence assessment; requires licensed psychologist	Wechsler, 2008, 2014
Conners-3	Rating scales	Parent, teacher, self-report; ADHD across settings	Conners, 2008
ADOS-2	Clinician-administered	Gold standard ASD observational assessment	Lord et al., 2012
CARS-2	Rating scale	Childhood Autism Rating Scale; observation-based	Schopler et al., 2010

Eating Disorders

Tool	Type	Notes	Source
EDE-Q	Self-report	4 subscales; higher = more severe	Fairburn & Beglin, 2008
EAT-26	Self-report	26 items; ≥20 warrants clinical follow-up	Garner et al., 1982
SCOFF	Self-report	5 items; rapid screen; ≥2 positive: referral indicated	Morgan et al., 1999

Child and Adolescent Specific

Tool	Type	Notes	Source
CBCL	Parent/teacher rating	Child Behavior Checklist; broad symptom assessment	Achenbach & Rescorla, 2001
CDI-2	Self-report (ages 7–17)	Children's Depression Inventory, 2nd edition	Kovacs, 2011
RCADS	Self-report	Revised Children's Anxiety and Depression Scale	Chorpita et al., 2000
SCARED	Self-report; parent form	Screen for Child Anxiety Related Disorders	Birmaher et al., 1997
CPSS-5	Self-report	Child PTSD Symptom Scale, DSM-5 version	Foa et al., 2018

BACK MATTER

Alphabetical Diagnosis Index

(ICD-10-CM codes, ICD-11 codes, and diagnostic family for each of the 103 diagnoses in the reference section. U.S. claims use ICD-10-CM; the chapter tables explain ICD-11 codes that are not one-to-one.)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Acute Stress Disorder	F43.0	QE84	Trauma & Stressor-Related (Ch 10)
Adjustment Disorder	F43.2x	6B43	Trauma & Stressor-Related (Ch 10)
Agoraphobia	F40.00	6B02	Anxiety (Ch 9)
Alcohol Use Disorder — mild	F10.10	6C40.1	Substance & Addictive (Ch 11)
Alcohol Use Disorder — moderate/severe	F10.20	6C40.2	Substance & Addictive (Ch 11)
Anorexia Nervosa — Binge/Purge	F50.02x	6B80.01 / 6B80.11	Feeding & Eating (Ch 18)
Anorexia Nervosa — Restricting	F50.01x	6B80.00 / 6B80.10	Feeding & Eating (Ch 18)
Antisocial Personality Disorder	F60.2	6D10 + 6D11.2	Personality (Ch 12)
Attention-Deficit/ Hyperactivity Disorder — combined	F90.2	6A05.2	Neurodevelopmental (Ch 14)
Attention-Deficit/ Hyperactivity Disorder — predominantly inattentive	F90.0	6A05.0	Neurodevelopmental (Ch 14)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Attention-Deficit/ Hyperactivity Disorder — predominantly hyperactive/ impulsive	F90.1	6A05.1	Neurodevelopmental (Ch 14)
Autism Spectrum Disorder	F84.0	6A02	Neurodevelopmental (Ch 14)
Avoidant Personality Disorder	F60.6	6D10 + 6D11.0 + 6D11.1	Personality (Ch 12)
Avoidant/ Restrictive Food Intake Disorder	F50.82	6B83	Feeding & Eating (Ch 18)
Binge-Eating Disorder	F50.81x	6B82	Feeding & Eating (Ch 18)
Bipolar Disorder Due to Another Medical Condition	F06.33	6E62.1	Bipolar & Related (Ch 13)
Bipolar I Disorder	F31.x	6A60	Bipolar & Related (Ch 13)
Bipolar II Disorder	F31.81	6A61	Bipolar & Related (Ch 13)
Body Dysmorphic Disorder	F45.22	6B21	OCD & Related (Ch 16)
Borderline Personality Disorder	F60.3	6D10 + 6D11.5	Personality (Ch 12)
Brief Psychotic Disorder	F23	6A23	Schizophrenia Spectrum (Ch 17)
Bulimia Nervosa	F50.2x	6B81	Feeding & Eating (Ch 18)
Cannabis Use Disorder	F12.10	6C41.1 / 6C41.2	Substance & Addictive (Ch 11)
Conduct Disorder	F91.x	6C91	Disruptive/Impulse-Control (Ch 20)
Conversion Disorder	F44.x	6B60	Somatic Symptom (Ch 21)
Cyclothymic Disorder	F34.0	6A62	Bipolar & Related (Ch 13)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Delayed Ejaculation	F52.32	HA03.1	Sexual Dysfunctions (Ch 24)
Delirium	F05	6D70	Neurocognitive (Ch 15)
Delusional Disorder	F22	6A24	Schizophrenia Spectrum (Ch 17)
Dependent Personality Disorder	F60.7	6D10 + 6D11.0	Personality (Ch 12)
Depersonalization/ Derealization Disorder	F48.1	6B66	Dissociative (Ch 22)
Depressive Disorder Due to Another Medical Condition	F06.31–32	6E62.0	Depressive (Ch 8)
Developmental Coordination Disorder	F82	6A04	Neurodevelopmental (Ch 14)
Disinhibited Social Engagement Disorder	F94.2	6B45	Trauma & Stressor-Related (Ch 10)
Disruptive Mood Dysregulation Disorder	F34.81	6C90.0	Depressive (Ch 8)
Dissociative Amnesia	F44.0	6B61	Dissociative (Ch 22)
Dissociative Identity Disorder	F44.81	6B64	Dissociative (Ch 22)
Erectile Disorder	F52.21	HA01.1	Sexual Dysfunctions (Ch 24)
Excoriation (Skin-Picking) Disorder	F42.4	6B25.1	OCD & Related (Ch 16)
Factitious Disorder	F68.10; F68.A	6D50; 6D51	Somatic Symptom (Ch 21)
Female Orgasmic Disorder	F52.31	HA02.0	Sexual Dysfunctions (Ch 24)
Female Sexual Interest/Arousal Disorder	F52.22	HA00 / HA01.0	Sexual Dysfunctions (Ch 24)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Gambling Disorder	F63.0	6C50	Substance & Addictive (Ch 11)
Gender Dysphoria	F64.0	HA60	Gender Dysphoria (Ch 24)
Generalized Anxiety Disorder	F41.1	6B00	Anxiety (Ch 9)
Genito-Pelvic Pain/ Penetration Disorder	F52.6	HA20	Sexual Dysfunctions (Ch 24)
Histrionic Personality Disorder	F60.4	6D10 + 6D11.2	Personality (Ch 12)
Hoarding Disorder	F42.3	6B24	OCD & Related (Ch 16)
Hypersomnolence Disorder	F51.11	7A21	Sleep-Wake (Ch 23)
Illness Anxiety Disorder	F45.21	6B23	Somatic Symptom (Ch 21)
Insomnia Disorder	F51.01	7A00	Sleep-Wake (Ch 23)
Intellectual Disability — mild	F70	6A00.0	Neurodevelopmental (Ch 14)
Intellectual Disability — moderate	F71	6A00.1	Neurodevelopmental (Ch 14)
Intellectual Disability — severe	F72	6A00.2	Neurodevelopmental (Ch 14)
Intellectual Disability — profound	F73	6A00.3	Neurodevelopmental (Ch 14)
Intermittent Explosive Disorder	F63.81	6C73	Disruptive/Impulse-Control (Ch 20)
Kleptomania	F63.2	6C71	Disruptive/Impulse-Control (Ch 20)
Major Depressive Disorder — single episode	F32.x	6A70	Depressive (Ch 8)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Major Depressive Disorder — recurrent	F33.x	6A71	Depressive (Ch 8)
Major Neurocognitive Disorder	F02.xx	6D80–6D85	Neurocognitive (Ch 15)
Male Hypoactive Sexual Desire Disorder	F52.0	HA00	Sexual Dysfunctions (Ch 24)
Mild Neurocognitive Disorder	G31.84	6D71	Neurocognitive (Ch 15)
Narcissistic Personality Disorder	F60.81	6D10 + 6D11.2	Personality (Ch 12)
Narcolepsy	G47.419	7A20	Sleep-Wake (Ch 23)
Nightmare Disorder	F51.5	7B01.2	Sleep-Wake (Ch 23)
Obsessive-Compulsive Disorder	F42.2	6B20	OCD & Related (Ch 16)
Obsessive-Compulsive Personality Disorder	F60.5	6D10 + 6D11.4	Personality (Ch 12)
Obstructive Sleep Apnea	G47.33	7A41	Sleep-Wake (Ch 23)
Opioid Use Disorder	F11.10	6C43.1 / 6C43.2	Substance & Addictive (Ch 11)
Oppositional Defiant Disorder	F91.3	6C90	Disruptive/Impulse-Control (Ch 20)
Panic Disorder	F41.0	6B01	Anxiety (Ch 9)
Paranoid Personality Disorder	F60.0	6D10 + 6D11.0	Personality (Ch 12)
Persistent Depressive Disorder	F34.1	6A72	Depressive (Ch 8)
Pica	F98.3	6B84	Feeding & Eating (Ch 18)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Posttraumatic Stress Disorder	F43.10	6B40	Trauma & Stressor-Related (Ch 10)
Premature Ejaculation	F52.4	HA03.0	Sexual Dysfunctions (Ch 24)
Premenstrual Dysphoric Disorder	F32.81	GA34.41	Depressive (Ch 8)
Prolonged Grief Disorder	F43.81	6B42	Trauma & Stressor-Related (taught in Ch 27)
Psychological Factors Affecting Other Medical Conditions	F54	6E40	Somatic Symptom (Ch 21)
Pyromania	F63.1	6C70	Disruptive/Impulse-Control (Ch 20)
Reactive Attachment Disorder	F94.1	6B44	Trauma & Stressor-Related (Ch 10)
REM Sleep Behavior Disorder	G47.52	7B01.0	Sleep-Wake (Ch 23)
Rumination Disorder	F98.21	6B85	Feeding & Eating (Ch 18)
Schizoaffective Disorder	F25.x	6A21	Schizophrenia Spectrum (Ch 17)
Schizoid Personality Disorder	F60.1	6D10 + 6D11.1	Personality (Ch 12)
Schizophrenia	F20.9	6A20	Schizophrenia Spectrum (Ch 17)
Schizophreniform Disorder	F20.81	6A20	Schizophrenia Spectrum (Ch 17)
Schizotypal Personality Disorder	F21	6A22	Personality (Ch 12)
Sedative/ Hypnotic/ Anxiolytic Use Disorder	F13.10	6C44.1 / 6C44.2	Substance & Addictive (Ch 11)
Selective Mutism	F94.0	6B06	Anxiety (Ch 9)

Diagnosis	ICD-10-CM	ICD-11	Family (Chapter)
Separation Anxiety Disorder	F93.0	6B05	Anxiety (Ch 9)
Social Anxiety Disorder	F40.10	6B04	Anxiety (Ch 9)
Social (Pragmatic) Communication Disorder	F80.82	6A01.22	Neurodevelopmental (Ch 14)
Somatic Symptom Disorder	F45.1	6C20	Somatic Symptom (Ch 21)
Specific Learning Disorder — reading	F81.0	6A03.0	Neurodevelopmental (Ch 14)
Specific Learning Disorder — written expression	F81.81	6A03.1	Neurodevelopmental (Ch 14)
Specific Learning Disorder — mathematics	F81.2	6A03.2	Neurodevelopmental (Ch 14)
Specific Phobia	F40.2xx	6B03	Anxiety (Ch 9)
Stimulant Use Disorder	F15.10 (amphetamine-type); F14.10 (cocaine)	6C46.1 / 6C46.2 (amphetamine-type); 6C45.1 / 6C45.2 (cocaine)	Substance & Addictive (Ch 11)
Substance/ Medication-Induced Bipolar and Related Disorder	F1x.x4	6C4x.70	Bipolar & Related (Ch 13)
Substance/ Medication-Induced Depressive Disorder	F1x.14/.24/.94	6C4x.70	Depressive (Ch 8)
Tourette's Disorder	F95.2	8A05.00	Neurodevelopmental (Ch 14)
Trichotillomania	F63.3	6B25.0	OCD & Related (Ch 16)

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This guide draws on the sources below. They are organized by the section of the book they support so that readers can locate the primary source for any diagnostic criterion, assessment instrument, treatment protocol, or professional standard named in the text. Formatting follows APA 7th edition.

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No Surprises Act, good faith estimate for uninsured (or self-pay) individuals, 45 C.F.R. § 149.610.

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Supplemental Practice Questions

The following 153 questions span all 17 diagnostic families and cross-domain ethical and multicultural content covered in this guide. Use them for full-domain review after completing the diagnostic chapters, or target individual families for focused review. Answers begin in the Supplemental Answer Key.

Depressive Disorders

1. A 55-year-old man reports chronic low mood for three years. He says he has "always been this way" and cannot identify a time he felt well for more than a few weeks. PHQ-9 score is 11. The MOST likely diagnosis is:

A. Major Depressive Disorder, recurrent, in partial remission
B. Adjustment Disorder with depressed mood given the chronicity
C. Persistent Depressive Disorder
D. Dysthymic personality traits not meeting diagnostic threshold

2. A client with MDD shows a 50% reduction in PHQ-9 after 8 weeks of CBT but continues to endorse anhedonia and fatigue. The counselor should:

A. Terminate treatment since a 50% PHQ-9 reduction shows significant progress was made
B. Switch to a psychodynamic approach to explore the deeper roots of the anhedonia
C. Refer for medication as the sole remaining intervention
D. Continue CBT with added behavioral activation targeting residual anhedonia

3. A 22-year-old college student presents with depressed mood, hypersomnia, increased appetite, and leaden para-

lysis following a breakup two weeks ago. The MOST important next step is:

A. Assess duration and severity against MDD criteria, since atypical features suggest a mood disorder rather than adjustment
B. Diagnose Adjustment Disorder with depressed mood, because the symptoms began shortly after a clear and identifiable relationship stressor
C. Begin interpersonal therapy focused on the relationship loss, as grief and role transition are the evident treatment targets
D. Screen for Bipolar II as the first priority, treating the atypical symptom profile as evidence of bipolarity

4. A mother presents six weeks postpartum with tearfulness, insomnia despite infant sleeping, intrusive thoughts of accidentally harming her baby, and guilt about being a "bad mother." She is horrified by the thoughts. This presentation is MOST consistent with:

A. Postpartum psychosis requiring immediate hospitalization to protect the infant from harm
B. Normal postpartum adjustment ("baby blues") that will resolve without treatment within weeks
C. MDD with peripartum onset and intrusive ego-dystonic thoughts common in perinatal OCD overlap
D. Generalized Anxiety Disorder with onset during the perinatal period and excessive worry about infant safety

5. The DSM-5-TR specifier "with anxious distress" for MDD is clinically important because it:

A. Changes the diagnosis to a comorbid anxiety disorder that must be coded and treated alongside the depression
B. Requires the addition of a benzodiazepine to the treatment plan to manage the anxiety symptoms during the acute phase
C. Predicts poorer treatment response and higher suicide risk, warranting more aggressive treatment
D. Indicates the depression is secondary to the anxiety, so the anxiety should become the primary treatment target

6. A client in treatment for MDD reports feeling "90% better" and wants to stop therapy. She completed 12 sessions of CBT. The counselor should:

A. Support termination since treatment goals are met and she reports feeling 90% improved
B. Recommend indefinite continuation of weekly sessions to prevent any recurrence of the depressive episode
C. Discuss relapse prevention, identify early warning signs, and plan a maintenance phase before ending
D. Transition to monthly maintenance sessions without discussion — she has already completed a full CBT course

7. A client states, "I don't feel sad — I just don't feel anything at all." This presentation MOST likely reflects:

A. Emotional suppression unrelated to depression, reflecting a learned habit of avoiding painful feelings
B. Alexithymia, which is a separate condition from depression involving difficulty identifying emotions
C. Anhedonia, which can be the primary manifestation of MDD even without reported sadness
D. Dissociation requiring trauma-focused assessment before any mood disorder is considered

8. A 70-year-old reports memory difficulties, poor concentration, and psychomotor retardation. He answers "I don't know" to most cognitive screening questions. This pattern MOST suggests:

A. Major Neurocognitive Disorder requiring neurological referral — memory and concentration deficits are prominent
B. Normal aging with expected cognitive decline, given his age and the mild nature of the complaints
C. Depressive pseudodementia, since "I don't know" responses and distress about deficits are characteristic
D. Delirium requiring emergency medical evaluation for an underlying medical cause

9. When treating double depression (PDD + superimposed MDD), the treatment priority is:

A. Address the PDD first since it is the longstanding condition from which the current acute episode has emerged
B. Treat both simultaneously with the same protocol and intensity, as both are depressive disorders with shared symptoms
C. Stabilize the acute MDD episode first, then address the chronic PDD substrate with combined therapy and medication consultation
D. Focus exclusively on the MDD since PDD is untreatable and reflects a stable baseline that therapy cannot change

Anxiety Disorders

10. A client reports panic attacks exclusively in social situations where she might be judged. Between social events, she has no anxiety symptoms. The MOST accurate diagnosis is:

A. Panic Disorder with agoraphobia, because the attacks lead her to avoid social situations
B. Social Anxiety Disorder with cued panic attacks in social situations
C. Generalized Anxiety Disorder with panic features triggered by evaluative situations
D. Specific Phobia of social settings, with panic attacks as the phobic response

11. A client with GAD worries about finances, health, relationships, and work performance. He asks, “Can you just tell me everything will be okay?” The BEST therapeutic response is:

A. Provide reassurance since he is clearly distressed and needs relief before any skills work can begin
B. Redirect to cognitive restructuring of the specific worry he is currently most distressed about
C. Explore the reassurance-seeking pattern while teaching tolerance of uncertainty as a core therapeutic target
D. Validate that his worries are reasonable given current circumstances, strengthening the therapeutic alliance

12. A client avoids elevators, bridges, and crowded stores. She describes fear of being trapped with no escape if she has a panic attack. This avoidance pattern is MOST consistent with:

A. Specific Phobia, situational type, given her fear of discrete situations such as elevators and bridges
B. Agoraphobia, characterized by fear of situations where escape might be difficult during panic-like symptoms
C. Social Anxiety Disorder with avoidance of crowded public places where she may be observed
D. PTSD with avoidance symptoms, reflecting persistent efforts to avoid distressing reminders

13. Interoceptive exposure for Panic Disorder involves:

A. Deliberately inducing feared physical sensations to reduce catastrophic interpretations of bodily cues
B. Gradual exposure to feared external situations, such as crowded stores or driving on highways
C. Relaxation training to reduce physiological arousal whenever early panic sensations begin to appear
D. Cognitive restructuring of irrational beliefs about panic, such as the belief that a racing heart signals a heart attack

14. A 10-year-old refuses to attend school, clings to her mother, and reports stomachaches every morning. She is fine once at school. Medical evaluation is negative. The counselor should FIRST:

A. Recommend homeschooling to reduce distress and remove the daily trigger for her morning somatic complaints
B. Refer for gastrointestinal evaluation despite negative findings — recurrent stomachaches may reflect an undetected condition
C. Begin play therapy to explore underlying family conflicts that may be driving her reluctance to separate from her mother
D. Assess for Separation Anxiety Disorder and develop a graduated return-to-school plan with parent coaching

15. A client with Specific Phobia of blood-injection-injury type differs from other phobias because:

A. It responds better to medication than exposure, since the fainting response is physiologically driven
B. It involves a vasovagal fainting response requiring modified exposure techniques that include applied tension
C. It does not cause significant impairment, because most people rarely encounter blood, needles, or medical procedures
D. It typically develops in adulthood rather than childhood, usually following a distressing medical procedure

16. A client with Social Anxiety avoids all group therapy recommendations. The counselor should:

A. Respect the avoidance as a valid preference, since forcing group participation could damage the therapeutic alliance
B. Require group attendance as a condition of treatment, as group is the most effective format for social anxiety
C. Use individual sessions to build exposure skills, then gradually introduce group as a therapeutic exposure target
D. Modify the diagnosis since true Social Anxiety responds to group therapy and avoidance suggests another disorder

17. The key distinction between GAD and normal worry is:

A. The content of the worry, because GAD worries center on unrealistic, catastrophic themes rather than everyday matters
B. The pervasiveness, uncontrollability, and associated physical symptoms persisting for six or more months
C. Whether the worry is about realistic concerns or about unlikely, exaggerated outcomes
D. The presence of panic attacks that accompany episodes of intense or escalating worry

Trauma and Stressor-Related Disorders

18. A client meets full PTSD criteria. She also reports persistent depersonalization during flashbacks. The counselor should diagnose:

A. PTSD with dissociative subtype, which is a single diagnosis with a specifier
B. PTSD and Depersonalization/Derealization Disorder, coded as two co-occurring diagnoses
C. Complex PTSD, which replaces the standard PTSD diagnosis when dissociation is present
D. Dissociative Identity Disorder given the dissociative features during flashbacks

19. A veteran avoids Fourth of July celebrations, parking near loud noises, and watching war films. These avoidance behaviors are BEST understood as:

A. Phobic avoidance requiring exposure therapy alone, targeting each feared situation in turn
B. Agoraphobia triggered by military service, with avoidance of crowded public events
C. Behavioral restriction requiring assertiveness training to help him participate in social events
D. Trauma-related avoidance of internal and external reminders, a core PTSD symptom cluster

20. A client experienced a car accident three days ago and reports flashbacks, hypervigilance, and emotional numbness. PTSD cannot be diagnosed because:

A. Car accidents do not meet Criterion A, which requires exposure to combat or assault
B. The client is not a combat veteran, and PTSD criteria were developed for military trauma
C. Emotional numbness rules out PTSD — numbing indicates a dissociative disorder instead
D. Symptoms must persist beyond one month for PTSD; at three days, monitor for Acute Stress Disorder

21. A 14-year-old was removed from an abusive home and placed in foster care six months ago. He is hypervigilant,

has nightmares, and is defiant toward his foster parents. The MOST appropriate clinical conceptualization is:

A. Conduct Disorder given the persistent defiance toward adult authority in the foster home
B. Oppositional Defiant Disorder triggered by the placement change and the loss of familiar caregivers
C. PTSD with behavioral symptoms, where defiance reflects hyperarousal and mistrust rather than a conduct problem
D. Adjustment Disorder since the foster placement is the identifiable stressor and his symptoms followed the move to a new home

22. Prolonged Exposure (PE) therapy for PTSD works primarily by:

A. Activating the trauma memory in a safe context to allow habituation and corrective learning
B. Providing catharsis through emotional expression of the feelings associated with the traumatic event
C. Building coping skills before processing the trauma so that the client can tolerate distress safely
D. Restructuring cognitive distortions about the trauma, such as self-blame and beliefs about safety

23. Adjustment Disorder requires that symptoms develop within what timeframe of the identifiable stressor?

A. Two weeks
B. Three months
C. Six months
D. One year

24. A counselor suspects a client's PTSD symptoms are being maintained by avoidance. The client says she copes by "staying busy and not thinking about it." The counselor should:

A. Validate the strategy since it is working and allows her to function well in daily life
B. Immediately begin trauma processing to break the avoidance before it becomes further entrenched

C. Refer to a psychiatrist for medication to reduce the avoidance and the anxiety that accompanies her trauma reminders
D. Psychoeducate about avoidance as a maintaining factor while respecting readiness and pacing exposure accordingly

25. Complex PTSD (as described in ICD-11) differs from standard PTSD by adding which features?

A. Psychotic symptoms, such as trauma-related hallucinations and paranoid beliefs about others' intentions
B. Substance use disorder, reflecting chronic self-medication of hyperarousal and intrusive memories
C. Dissociative identity states that emerge following prolonged and repeated childhood trauma
D. Disturbances in self-organization: affect dysregulation, negative self-concept, and interpersonal difficulties

Substance-Related Disorders

26. A client with Alcohol Use Disorder is in the contemplation stage. The counselor should:

A. Provide information about treatment programs so he can choose a level of care that fits his needs
B. Confront the denial directly to help him recognize the seriousness of his drinking
C. Explore ambivalence using motivational interviewing, developing discrepancy between current behavior and stated values
D. Recommend AA attendance as the primary intervention — peer support strengthens commitment to sobriety in early change

27. A client prescribed opioids post-surgery has developed tolerance and withdrawal when doses are reduced. He takes the medication as prescribed and experiences no craving or impairment. This BEST represents:

A. Expected physiological dependence, which is distinct from Opioid Use Disorder
B. Opioid Use Disorder, mild, as tolerance and withdrawal

count toward the diagnostic criteria
C. Substance-induced medical condition resulting from prolonged opioid exposure after surgery
D. Iatrogenic addiction requiring immediate intervention to prevent escalation to misuse

28. The ASAM Criteria (3rd edition) for level-of-care placement assesses which dimensions?

A. Only substance type and quantity of use — the drug used and the amount consumed determine withdrawal severity and the intensity of services a client needs
B. Insurance authorization and cost considerations, including program availability, waitlist length, and the number of treatment days the client's payer will approve for coverage
C. Criminal history and legal mandates, including court orders, probation or parole requirements, and the client's record of compliance with prior treatment
D. Six dimensions including intoxication/withdrawal risk, medical conditions, emotional/behavioral conditions, readiness to change, relapse potential, and recovery environment

29. A client with Stimulant Use Disorder (cocaine) presents during a withdrawal "crash" with fatigue, hypersomnia, and depression. The counselor should:

A. Hospitalize for medical detoxification — the crash can produce serious medical complications that require continuous monitoring
B. Begin antidepressant treatment for the depressive symptoms — fatigue, hypersomnia, and low mood indicate a depressive episode
C. Interpret the symptoms as a separate Major Depressive Episode that requires its own diagnosis and a distinct treatment plan
D. Monitor, assess suicide risk during the crash, and provide supportive care; stimulant withdrawal is uncomfortable but not medically dangerous

30. A client in early recovery from Alcohol Use Disorder asks about attending social events where alcohol is served. The BEST response is:

A. Collaboratively assess readiness, develop a coping plan for high-risk situations, and identify refusal skills
B. Recommend avoiding all social situations with alcohol permanently — any exposure to drinking cues increases relapse risk
C. Encourage attendance to build confidence through exposure, because facing drinking situations early strengthens self-efficacy
D. This decision should be deferred until one year of sobriety, when the risk of relapse has substantially declined

31. Motivational Interviewing's OARS technique stands for:

A. Open questions, Affirmations, Reflections, Summaries
B. Observations, Analyses, Recommendations, Support
C. Orientation, Assessment, Reinforcement, Stabilization
D. Outcomes, Activation, Resources, Sustainability

32. Naltrexone is used in the treatment of Alcohol Use Disorder because it:

A. Produces nausea and vomiting when alcohol is consumed, creating an aversion to drinking
B. Replaces alcohol's effects on GABA receptors, easing the discomfort of early abstinence
C. Treats concurrent depression that drives alcohol use
D. Reduces craving and the rewarding effects of alcohol by blocking opioid receptors

33. A counselor discovers that a client in SUD treatment has relapsed. The MOST appropriate response is:

A. Discharge from treatment for noncompliance with the program's abstinence requirements and signed treatment contract
B. Restart treatment from the beginning with a new intake assessment and treatment plan

C. Increase session frequency without discussing the relapse, to avoid shaming the client
D. Assess the relapse nonjudgmentally, identify triggers and high-risk situations, and adjust the treatment plan

Personality Disorders

34. A client with Borderline Personality Disorder calls between sessions reporting a crisis whenever the counselor is unavailable. This pattern is BEST addressed by:

A. Establishing clear between-session contact protocols within a DBT framework, including phone coaching guidelines
B. Providing 24-hour availability to prevent self-harm — abandonment fears drive the crises and constant access may calm them
C. Setting rigid no-contact boundaries between sessions so that she learns to manage crises without relying on the counselor
D. Referring to a higher level of care, such as intensive outpatient treatment, because the crisis calls signal escalating risk

35. Narcissistic Personality Disorder is characterized by grandiosity, but the counselor should also assess for:

A. Psychotic features underlying the grandiosity, such as fixed delusions of special identity
B. Antisocial traits, since NPD always co-occurs with ASPD
C. Underlying vulnerability and fragile self-esteem that the grandiosity defends against
D. Bipolar hypomania, which explains the grandiosity and inflated self-regard

36. A client with Avoidant Personality Disorder wants friends but avoids social situations due to fear of rejection. Treatment should:

A. Focus exclusively on social skills training so that she can approach peers with greater confidence and ease
B. Recommend group therapy immediately since social

exposure is needed and a group offers a ready setting for practice
C. Build trust in the therapeutic relationship, then gradually expose to feared social situations while addressing core beliefs of inadequacy
D. Reframe avoidance as introversion to reduce distress and help her accept a quieter, more solitary lifestyle

37. The fundamental distinction between Schizotypal PD and Schizophrenia is:

A. Schizotypal PD involves eccentric beliefs and social deficits without sustained psychotic episodes; Schizophrenia involves active psychosis
B. Severity of symptoms, as Schizophrenia involves more pronounced odd beliefs and social withdrawal than Schizotypal PD
C. Age of onset, because Schizotypal PD begins in childhood whereas Schizophrenia typically emerges in early adulthood
D. Family history of psychosis, which is common in Schizophrenia but typically absent in Schizotypal PD

38. A client with Antisocial PD is court-mandated for treatment. He is charming, cooperative, and reports full commitment to change. The counselor should:

A. Accept his presentation at face value since he is engaging and a strong early alliance predicts better outcomes
B. Refuse to treat since ASPD is untreatable and court-mandated clients rarely engage genuinely in therapy
C. Maintain therapeutic skepticism, focus on behavioral accountability, and verify self-report with collateral information
D. Confront the manipulation immediately to establish honesty and set clear expectations from the very first session onward

39. Schema therapy for personality disorders focuses on:

A. Symptom reduction through behavioral techniques such as activity scheduling, exposure, and contingency management

B. Building social skills through structured exercises such as role-play, modeling, and corrective group feedback
C. Medication management of personality traits, targeting impulsivity and affective instability with mood stabilizers
D. Identifying and modifying early maladaptive schemas that drive chronic relational and emotional patterns

40. Dialectical Behavior Therapy's four skills modules are:

A. Assertiveness Training, Self-Esteem Building, Anger Management, Communication Skills
B. Cognitive Restructuring, Exposure, Relaxation, Problem-Solving
C. Attachment Repair, Mentalization, Affect Regulation, Self-Reflection
D. Mindfulness, Distress Tolerance, Emotion Regulation, Interpersonal Effectiveness

41. A client with Dependent PD has recently started making small decisions independently for the first time. The counselor should:

A. Praise the independence effusively to reinforce each decision and build her sense of self-efficacy
B. Push for larger decisions immediately to capitalize on momentum while her confidence is high
C. Interpret the independence as resistance to the therapeutic relationship and explore what she may be avoiding in session
D. Acknowledge the progress naturally without over-praising, which could reinforce dependence on the counselor's approval

Bipolar and Related Disorders

42. A client in a depressive episode has a history of one prior hypomanic episode lasting five days. The diagnosis is:

A. Major Depressive Disorder, recurrent, because the current depressive episode is the primary focus of treatment
B. Cyclothymic Disorder, given the history of both elev-

ated and depressed mood periods over time
C. Bipolar II Disorder, since one hypomanic episode plus a depressive episode meets criteria
D. Bipolar I Disorder, as any period of mood elevation indicates mania regardless of its duration or severity

43. A client with Bipolar I is currently euthymic and stable on lithium. She asks to discontinue medication because she "feels fine now." The counselor should:

A. Support her autonomy to stop medication, as she is stable and has the right to make her own treatment decisions
B. Explore her reasoning, psychoeducate about relapse risk with medication discontinuation, and coordinate with the prescriber
C. Insist she continue medication indefinitely — discontinuing lithium carries a high risk of relapse into mania
D. This is the prescriber's decision, not the counselor's domain, so the counselor should not discuss it further in session

44. Rapid cycling in Bipolar Disorder is defined as:

A. Mood changes within a single day, shifting between highs and lows
B. Four or more mood episodes within a 12-month period
C. Alternating between mania and depression within a week
D. Continuous cycling without euthymic periods

45. A client presents with pressured speech, decreased need for sleep, grandiose plans, and impulsive spending. This has been ongoing for 10 days. The MOST urgent clinical priority is:

A. Begin CBT for mania to help her identify early warning signs and challenge grandiose thinking
B. Ensure psychiatric evaluation for mood stabilization, since active mania typically requires pharmacological intervention
C. Hospitalize involuntarily, since the grandiose plans and impulsive spending indicate severely impaired judgment

D. Assess for substance-induced mania before any intervention, and hold the psychiatric referral until that question is settled

46. Interpersonal and Social Rhythm Therapy (IPSRT) for Bipolar Disorder targets:

A. Cognitive distortions about mood episodes, using thought records to challenge catastrophic predictions
B. Family communication patterns, reducing expressed emotion at home through communication and problem-solving training
C. Regularization of daily routines, sleep-wake cycles, and interpersonal relationships to prevent mood episodes
D. Medication adherence exclusively, through structured psychoeducation about mood stabilizers

47. Cyclothymic Disorder differs from Bipolar II primarily in:

A. Symptom severity — cyclothymia involves hypomanic and depressive symptoms that do not meet full episode criteria
B. Duration of illness, with cyclothymia typically running a briefer, more self-limited course than Bipolar II
C. Treatment response, since cyclothymia responds to psychotherapy alone without the need for mood stabilizers
D. Presence of psychotic features during periods of elevated mood, which distinguish Bipolar II

48. A client diagnosed with Bipolar II has been started on an SSRI by her primary care doctor without a mood stabilizer. The counselor's concern should be:

A. SSRI monotherapy without a mood stabilizer can trigger hypomania or rapid cycling in bipolar patients
B. No concern — SSRIs are appropriate for Bipolar II depression, because hypomania is milder and less risky than mania
C. The PCP should not be treating bipolar disorder, and the client should be transferred to a psychiatrist for all care

D. The client should stop the SSRI immediately without medical guidance to prevent a switch into hypomania

49. A client with Bipolar I reports that during manic episodes, she felt "more creative and productive than ever." This is therapeutically important because:

A. It suggests the mania was not severe, as significant functional impairment usually makes an episode feel distressing
B. The positive experience of mania can reduce medication adherence and treatment engagement, requiring direct exploration
C. She may not actually have Bipolar Disorder — true mania is typically experienced as chaotic and distressing
D. Creativity during mania should be channeled productively into structured projects that make use of her elevated energy

Neurodevelopmental Disorders

50. An adult presents requesting ADHD evaluation. He reports lifelong concentration difficulties but his school records show average grades and no teacher concerns. The counselor should:

A. Gather comprehensive developmental history including collateral informants, since ADHD requires evidence of symptoms before age 12 across multiple settings
B. Diagnose ADHD based on self-report — adults are the most reliable reporters of their own lifelong attention difficulties
C. Refer for neuropsychological testing immediately — objective attention measures can confirm or rule out the diagnosis
D. Rule out ADHD since school performance was adequate and no teacher ever raised concerns about attention or behavior

51. A parent asks whether their child with ADHD should take medication. The counselor should:

A. Provide psychoeducation about multimodal treatment including behavioral strategies, parent training, and medication as options, supporting informed decision-making
B. Recommend medication as the first-line treatment, explaining that stimulants have the largest effect sizes of any ADHD intervention and work quickly for most children
C. Discourage medication due to side effects such as appetite suppression, sleep disruption, and possible growth delays in children
D. Defer entirely to the prescriber — questions about medication decisions fall outside the counselor's scope of practice, training, and expertise

52. A 25-year-old software engineer was recently diagnosed with Autism Spectrum Disorder. He asks, "How did no one catch this before?" The counselor should:

A. Explain that the previous providers were negligent in missing core social communication signs that were likely present throughout childhood
B. Discuss how ASD without intellectual impairment can be masked by compensatory strategies and intellectual ability, especially without childhood behavioral concerns
C. Question the accuracy of the diagnosis, gently exploring whether a condition unrecognized for 25 years might be better explained by social anxiety or another disorder
D. Focus on treatment rather than diagnostic history, redirecting him toward current goals so the session stays productive and forward-looking

53. ADHD — Combined type requires:

A. Six or more inattentive symptoms only, persisting for at least six months, with several present before age 12 and impairment evident in two or more settings
B. The presence of a learning disability alongside ADHD symptoms, because combined type is defined by co-occurring academic skill deficits in reading, writing, or math
C. Impairment in academic settings specifically, with symptoms in each domain documented through teacher ratings rather than through home or workplace reports

D. Six or more symptoms in both the inattentive and hyperactive-impulsive domains (five or more at age 17 and older), with several present before age 12

54. Executive function deficits in ADHD are BEST addressed by:

A. Insight into why the client procrastinates, exploring the underlying fears of failure and perfectionism that keep tasks from getting started
B. Stimulant medication exclusively — executive deficits reflect dopamine dysregulation that medication directly corrects
C. External organizational supports, environmental modifications, and skills training targeting planning, time management, and task initiation
D. Cognitive restructuring of beliefs about productivity, challenging the self-critical thoughts that the client is lazy or incapable of finishing tasks

55. ASD social communication deficits differ from Social Anxiety Disorder because:

A. ASD reflects difficulty understanding social cues and reciprocity; SAD reflects intact social skills inhibited by fear of negative evaluation
B. ASD involves more severe avoidance, with the individual withdrawing from nearly all social contact rather than select situations
C. Only ASD responds to social skills training, because anxiety-driven deficits require exposure rather than skill building in groups
D. SAD involves repetitive behaviors and restricted interests, whereas ASD is defined primarily by fear of social judgment

56. A parent wants their child with ADHD to “just try harder.” The counselor should:

A. Agree that effort is important, reassuring the parent that the child could meet expectations at home and at school if he simply tried harder and cared more
B. Recommend stricter discipline, including a consistent

loss of privileges whenever the child fails to finish homework or follow instructions the first time
C. Focus on the child without addressing the parent's perspective, building the child's coping and organizational skills directly in individual sessions
D. Psychoeducate that ADHD is a neurobiological condition affecting executive function, not a willpower deficit, while validating the parent's frustration

57. Specific Learning Disorder differs from Intellectual Disability in that:

A. SLD involves circumscribed deficits in specific academic areas with otherwise normal cognitive ability; ID involves globally reduced functioning
B. Learning disorders are less severe, typically producing milder impairment that is limited to classroom performance
C. Only ID qualifies for educational accommodations, while SLD is typically addressed through general classroom differentiation rather than an IEP or 504 plan
D. SLD resolves with appropriate instruction, whereas the deficits in ID remain stable across the lifespan

Neurocognitive Disorders

58. A 78-year-old is brought to the ER with acute confusion, fluctuating attention, and visual hallucinations that developed over two days. The MOST likely diagnosis is:

A. Delirium, which is a medical emergency requiring immediate workup for underlying cause
B. Major Neurocognitive Disorder with Lewy Bodies, given the fluctuating attention and prominent visual hallucinations
C. Late-onset Schizophrenia, given the new-onset hallucinations and disorganized thinking in an older adult
D. Sundowning, which is a normal phenomenon in elderly patients and typically settles with a consistent daily routine

59. Frontotemporal NCD differs from Alzheimer's-type NCD primarily in:

A. Early personality and behavioral changes with relative sparing of memory in frontotemporal; early memory loss in Alzheimer's
B. Age of onset, which is the most reliable way to tell the two apart at presentation
C. Response to cholinesterase inhibitors, which improve memory and daily functioning in Alzheimer's but offer little benefit in frontotemporal NCD
D. Presence of motor symptoms such as parkinsonism or gait disturbance early in the course

60. A client with Mild NCD asks about evidence-based strategies to slow cognitive decline. The counselor should recommend:

A. Brain training games as the primary intervention, as daily computerized practice builds memory and processing speed
B. Cholinesterase inhibitors as prevention, started at the mild stage to delay progression to major neurocognitive disorder
C. Acceptance of inevitable decline without intervention, focusing therapy on grief, adjustment, and planning for future care
D. Regular aerobic exercise, social engagement, cognitive stimulation, sleep optimization, and management of cardiovascular risk factors

61. When a client with dementia makes a clinical decision, the counselor must assess:

A. Decision-making capacity: understanding, appreciation, reasoning, and ability to express a choice
B. Only whether the client agrees with the treatment plan — agreement indicates the client understands the recommendation
C. Whether the family agrees with the decision, since relatives know the client's values and daily functioning best
D. The client's educational background, which determines

how complex the information presented to the client can be

62. Vascular NCD is distinguished from Alzheimer's by:

A. More rapid progression, with cognitive and functional decline advancing noticeably faster from the earliest stages
B. Stepwise decline associated with cerebrovascular events rather than gradual continuous decline
C. The presence of hallucinations and fluctuating alertness early in the course of the illness
D. Younger age of onset, typically before 65, in patients with vascular risk factors

63. A caregiver for a spouse with Alzheimer's presents for her own counseling and reports exhaustion, resentment, and social isolation. The counselor's primary role is:

A. Focus on the patient with Alzheimer's, using her sessions to track his symptoms rather than address her own distress
B. Provide caregiver support, psychoeducation, respite resource referrals, and monitor for caregiver depression and burnout
C. Recommend nursing home placement so she can recover from the exhaustion and resume her own social life
D. Refer the caregiver to her own therapist, because her needs are separate from the patient's care and require a new provider

64. The "sundowning" phenomenon in dementia refers to:

A. Increased confusion, agitation, and behavioral disturbance in late afternoon and evening, likely related to circadian disruption
B. Depression that worsens in the evening, reflecting the diurnal mood variation often seen in older adults with dementia
C. A side effect of cholinesterase inhibitors taken at bedtime, which can cause restlessness and vivid dreams overnight
D. Sleep apnea causing repeated nighttime awakenings,

which then produce daytime confusion and fatigue in older adults

65. A client scores 28/30 on the MoCA. The counselor should:

A. Consider the score within context — a high-functioning individual scoring 28 may still represent meaningful decline from their baseline
B. Reassure that no cognitive impairment exists — a score of 28 falls above the standard MoCA cutoff of 26 and needs no further follow-up
C. Repeat the test in six months to establish whether the score changes over time before drawing conclusions about possible decline
D. Refer for full neuropsychological evaluation immediately — even a small deficit on a screening tool warrants comprehensive testing

OCD and Related Disorders

66. A client performs handwashing rituals for 3 hours daily. He knows the behavior is excessive but cannot stop. ERP treatment should begin with:

A. Constructing a fear hierarchy and beginning with low-to-moderate items while preventing the ritual response
B. The most anxiety-provoking contamination exposure, so that habituation occurs quickly and treatment time is shortened
C. Teaching relaxation techniques to replace the handwashing as a way of managing contamination anxiety
D. Cognitive restructuring of contamination beliefs before any exposure, so the client feels ready for the work

67. Body Dysmorphic Disorder is classified with OCD-related disorders because:

A. Both involve repetitive behaviors driven by preoccupation — OCD with intrusive thoughts, BDD with perceived appearance flaws
B. Both involve contamination fears that drive repeated washing, grooming, and cleaning rituals aimed at redu-

cing disgust and anxiety
C. Both respond to the same medication, with SSRIs at higher doses serving as the first-line pharmacological treatment for each disorder
D. Both involve hoarding behaviors, in which items connected to the obsession are saved and cannot be discarded without distress

68. A client with OCD reports that intrusive violent images are the most distressing aspect of his condition. He has never acted violently. The counselor should:

A. Assess for risk of violence given the graphic content of the images and his level of distress
B. Normalize aggressive obsessions as ego-dystonic intrusive thoughts common in OCD that do not indicate violence risk
C. Refer to a forensic specialist who is trained to evaluate whether violent intrusive images signal future dangerousness
D. Avoid discussing the content to prevent reinforcement and focus instead on general anxiety management

69. OCPD differs from OCD in that:

A. OCPD involves compulsive rituals, such as checking and counting, performed to neutralize intrusive thoughts
B. OCPD is more severe, causing greater functional impairment because the rigidity affects every area of life
C. Only OCD responds to SSRI medication, while OCPD is treated primarily with long-term psychodynamic psychotherapy and schema work
D. OCPD rigidity is ego-syntonic (the person values their standards); OCD obsessions are ego-dystonic (unwanted and distressing)

70. Trichotillomania (hair-pulling disorder) treatment typically includes:

A. Habit reversal training as the primary behavioral intervention, with competing response training
B. Insight into childhood origins of the behavior, exploring early stressors that the hair pulling may symbolize

C. SSRI medication as the sole treatment, since hair pulling is considered a variant of obsessive-compulsive disorder
D. Aversion therapy using negative stimuli, such as a rubber-band snap each time the client begins to pull

71. A client with Hoarding Disorder becomes distressed when asked to discard newspapers from 2015. The counselor should:

A. Discard them while the client is not present to avoid the distress of making each decision in session
B. Set a deadline for discarding so the client has a clear, time-limited goal that builds momentum and accountability
C. Explore the meaning and function of saving these items within the therapeutic relationship before attempting any discarding
D. Focus on preventing new acquisition only, postponing any work on discarding until the client feels less attached to the items

72. Excoriation (skin-picking) disorder is treated similarly to trichotillomania because both:

A. Are caused by anxiety, with the behavior serving mainly to relieve tension in stressful moments
B. Involve repetitive body-focused behaviors amenable to habit reversal training and stimulus control
C. Require dermatological treatment of the resulting skin or scalp damage before behavioral work can begin
D. Are symptoms of OCD rather than separate diagnoses, so the same ERP protocol applies to each

73. A client with OCD asks about the role of SSRI medication. The counselor should explain:

A. SSRIs cure OCD and should replace therapy, as medication acts faster and asks less of the client than ERP
B. SSRIs are ineffective for OCD, whose symptoms are maintained by learned avoidance rather than neurochemistry
C. Medication should only be tried after ERP has failed to

produce adequate change over a full course of treatment
D. SSRIs can reduce symptom severity and are often used alongside ERP, typically at higher doses than for depression

Psychotic Disorders

74. A 19-year-old college student becomes socially withdrawn, stops attending classes, and his roommate reports hearing him talking to himself. His parents describe a "personality change" over six months. This presentation suggests:

A. Major Depressive Disorder with psychotic features, given the withdrawal, loss of interest, and apparent hallucinations
B. Cannabis-induced psychosis, which commonly emerges in college students and accounts for the change in behavior
C. Social Anxiety Disorder with avoidance, explaining his withdrawal from classes and peers over the past semester
D. Prodromal phase of schizophrenia or early psychosis, warranting comprehensive psychiatric evaluation

75. Negative symptoms of schizophrenia include:

A. Hallucinations and delusions that persist between acute episodes
B. Disorganized speech and behavior, such as tangentiality and odd mannerisms
C. Flat affect, avolition, alogia, anhedonia, and social withdrawal
D. Paranoid ideation and suspiciousness toward others

76. A client with schizophrenia is stable on antipsychotics and engaged in therapy. He reports persistent auditory hallucinations. CBT for psychosis (CBTp) would target:

A. Eliminating the hallucinations entirely through reality testing of each voice
B. Reducing distress related to voices and modifying beliefs about their power, while improving functioning
C. Replacing antipsychotic medication with structured

cognitive and behavioral coping strategies once symptoms are stable
D. Analyzing the unconscious meaning of the voices to uncover the conflicts that produce them

77. Brief Psychotic Disorder differs from Schizophreniform Disorder in:

A. Symptom type, since Brief Psychotic Disorder involves delusions without the negative symptoms of Schizophreniform
B. Duration — Brief Psychotic Disorder lasts under one month with full return to functioning; Schizophreniform lasts one to six months
C. Treatment approach, with Brief Psychotic Disorder managed through supportive therapy and Schizophreniform requiring maintenance antipsychotics
D. Presence of hallucinations versus delusions as the predominant psychotic symptom in each disorder

78. Delusional Disorder is characterized by:

A. Severe functional deterioration across work, relationships, and self-care that progresses as the delusions become more elaborate
B. Prominent hallucinations alongside delusions, with voices that comment on the client and reinforce a single persecutory theme
C. One or more delusions with relatively preserved functioning outside the delusional system and without prominent hallucinations or disorganization
D. Disorganized thought process, with loosening of associations and tangential speech that make the delusional content difficult to follow

79. A client with schizoaffective disorder asks why she needs both a mood stabilizer and an antipsychotic. The counselor should explain:

A. One medication would be sufficient, and taking two increases side effects without adding any meaningful benefit for her particular condition
B. The disorder has both mood and psychotic components

that can persist independently, so the prescriber may target each component
C. The antipsychotic treats side effects of the mood stabilizer, such as the tremor and restlessness that commonly occur with lithium
D. She should discuss this only with her psychiatrist — questions about medication combinations fall outside the counselor's role

80. First-episode psychosis is treated more conservatively than chronic schizophrenia because:

A. First episodes are less severe, so aggressive treatment exposes patients to side effects that are not justified
B. Antipsychotics are contraindicated in first episodes because younger patients are especially vulnerable to metabolic effects and tardive dyskinesia with long-term use
C. First episodes always resolve spontaneously, so clinicians watch and wait before committing a young patient to long-term medication
D. Many first-episode patients respond to lower medication doses and may have better outcomes with early intervention including therapy and psychosocial support

81. Clozapine is reserved for treatment-resistant schizophrenia because:

A. It is less expensive than other antipsychotics, so it is kept as a lower-cost option for patients who have exhausted newer medications
B. It works faster than other antipsychotics, making it most useful for rapidly stabilizing patients whose symptoms have not responded
C. It has fewer side effects than first-generation antipsychotics, so it is held in reserve for patients who cannot tolerate haloperidol or similar agents
D. It carries serious risks including agranulocytosis requiring regular blood monitoring, but is the most effective antipsychotic for refractory cases

Eating Disorders

82. A client with Bulimia Nervosa reports purging through vomiting 5 times weekly. Medical monitoring should include:

A. Electrolyte panels, dental evaluation, and cardiac monitoring given risks of hypokalemia, dental erosion, and arrhythmia
B. Liver function tests only — repeated vomiting and bile loss primarily place strain on hepatic metabolism and function
C. Brain imaging to rule out neurological causes of the recurrent vomiting, such as a tumor or increased intracranial pressure
D. Complete blood count exclusively, to screen for the anemia, low white cell counts, and infection risk associated with malnutrition

83. Family-Based Treatment (FBT/Maudsley approach) is first-line for:

A. Adult Anorexia Nervosa, where spouses or partners take over responsibility for planning and supervising meals
B. Binge Eating Disorder in children, where parents control portions and eating schedules
C. Adolescent Anorexia Nervosa, where parents are empowered as the primary agents of refeeding
D. All eating disorders regardless of age, because family involvement improves outcomes across diagnoses

84. A client with Binge Eating Disorder reports eating large amounts of food rapidly while alone, feeling disgusted afterward, but never purging. CBT-E would target:

A. Caloric restriction to prevent binges by keeping total daily intake within a planned range
B. Identifying triggers for binge episodes, developing regular eating patterns, and addressing shape and weight overvaluation
C. Weight loss as the primary therapeutic goal, as reducing weight improves self-esteem and body image

D. Eliminating specific “trigger foods” from the diet so the client is not exposed to foods that set off a loss of control

85. ARFID (Avoidant/Restrictive Food Intake Disorder) differs from Anorexia Nervosa in that:

A. ARFID is less medically serious, because intake is limited in variety rather than in overall calories, so weight loss and instability are uncommon
B. ARFID involves no body image disturbance or fear of weight gain; restriction is driven by sensory sensitivity, fear of aversive consequences, or lack of interest
C. ARFID only occurs in children, typically as extreme picky eating that emerges in early childhood and resolves on its own by adolescence
D. ARFID does not cause nutritional deficiency, because individuals continue eating adequate amounts of preferred foods and restriction reflects sensory preference rather than thinness

86. When assessing eating disorder severity, which indicator warrants immediate medical stabilization?

A. BMI below 20, even when vital signs and laboratory values remain within normal limits
B. Bradycardia, significant electrolyte abnormalities, or BMI below 15
C. Self-reported restriction of two meals daily over the past several weeks
D. Body image disturbance alone, particularly when it is severe and persistent

87. A client with Anorexia Nervosa says she is “not that thin” while visibly emaciated. This discrepancy reflects:

A. Body image disturbance, a core diagnostic feature involving distorted perception of body weight and shape
B. Deliberate minimization to avoid hospitalization or pressure from family to gain weight
C. Poor insight suggesting a psychotic process, with the belief held at delusional intensity
D. Typical body dissatisfaction present in the general pop-

ulation, which is especially common among adolescent and young adult women

88. The counselor's role when a client with an eating disorder is also seeing a dietitian and physician includes:

A. Coordinating care across the treatment team while addressing the psychological factors maintaining the eating disorder
B. Deferring all food-related discussions to the dietitian to avoid giving the client mixed messages about eating
C. Managing the nutritional rehabilitation plan and adjusting meal targets based on weekly weight checks
D. Acting as the team leader who makes all treatment decisions and directs the dietitian and physician

Crisis, Risk, and Safety

89. A client denies suicidal ideation but has given away valued possessions, written letters to family, and settled financial affairs. The counselor should:

A. Accept the denial since the client stated no ideation and has no history of prior attempts
B. Note the behaviors in the chart and continue routine treatment while monitoring for further changes
C. Ask the client directly about each behavior but take no further action if the client provides reasonable explanations for each one
D. Recognize these as warning signs of imminent suicide risk and conduct a thorough safety assessment despite the denial

90. The most empirically supported intervention for a client presenting with suicidal ideation is:

A. No-harm contract, in which the client signs a written agreement to contact the counselor or a crisis line before acting on suicidal urges
B. Immediate hospitalization to ensure the client's safety until the suicidal ideation has fully resolved and acute risk has decreased
C. Collaborative safety planning using the Stanley-Brown

model, which is an active coping intervention superior to no-harm contracts
D. Medication referral as the first step, as treating the underlying depression is what reduces suicidal ideation over time

91. Means restriction counseling involves:

A. Telling the client to remove all dangerous items from their home, including firearms, stockpiled medications, and sharp objects
B. Collaboratively identifying lethal means, discussing temporary restriction strategies, and involving support persons in safely securing or removing access
C. Confiscating the client's property, such as firearms or stockpiled medications, and securing it at the counseling office until the crisis has passed
D. Documenting that means were discussed without specific action, so the record reflects that the topic was addressed

92. A client's partner calls the counselor and says, "I'm worried she's going to kill herself tonight." The counselor should:

A. Decline to speak with the partner, citing confidentiality, and encourage the partner to share these concerns with the client directly
B. Thank the partner for calling, take the concern seriously, attempt to reach the client, and activate the safety plan or emergency services if needed
C. Ask the partner to call 911 themselves — the partner is physically closer to the client and better positioned to judge the level of immediate danger
D. Wait until the next scheduled session to assess, when the client can address the partner's concern directly

93. A client reports that a family member is physically abusing his elderly mother who has dementia. As a mandated reporter, the counselor must:

A. Report to Adult Protective Services based on reasonable suspicion, without needing to confirm or investigate

B. Confirm the abuse before reporting by gathering specific details about the injuries and speaking with the mother
C. Encourage the client to report it himself — he has firsthand knowledge of the situation and the family relationships
D. Only report if the elderly person is the counselor's client, because the reporting duty applies to people served directly

94. Non-Suicidal Self-Injury (NSSI) differs from a suicide attempt in:

A. The method used — cutting and burning indicate NSSI while overdose and hanging indicate a suicide attempt
B. The intent — NSSI aims to manage emotional distress without intent to die, while a suicide attempt involves intent to end life
C. The severity of injury, with superficial wounds indicating NSSI and medically serious wounds indicating suicidal intent
D. Whether the client reports pain, since individuals engaging in NSSI often describe feeling little or no pain during the act of self-injury

95. A client in crisis states, "I'm going to hurt my boss. He deserves it." Before activating duty to warn, the counselor should:

A. Call the police immediately so law enforcement can intervene before the client leaves the office and acts on the statement
B. Assess specificity, imminence, and identifiability — explore whether this is venting frustration or a credible threat with a plan
C. Document and continue the session, as venting anger toward a supervisor is common among clients in crisis
D. Warn the boss as a precaution regardless of specificity — he has been clearly identified as the target

96. During a suicide risk assessment, the STRONGEST single predictor of future suicide attempt is:

A. Current depressive symptoms, especially hopelessness
B. Male gender, particularly in older adults
C. History of a previous suicide attempt
D. Family history of suicide in a first-degree relative

Disruptive, Impulse-Control, and Conduct Disorders

97. A 7-year-old is frequently angry, argues with adults, deliberately annoys siblings, and blames others for his mistakes. He has never been physically aggressive, stolen, or destroyed property. The MOST likely diagnosis is:

A. Oppositional Defiant Disorder, which involves angry/irritable mood and defiant behavior without the serious violations seen in CD
B. Conduct Disorder, childhood-onset, given the persistent pattern of defiance and deliberate annoyance of others before age 10
C. ADHD with behavioral problems — impulsivity and poor frustration tolerance often explain arguing and blaming in young children
D. Intermittent Explosive Disorder, given the frequent anger and difficulty controlling his temper with adults and siblings

98. Grouping antisocial adolescents together in group therapy is contraindicated because:

A. They will not engage with each other, because antisocial youth tend to distrust peers and resist sharing personal material in a group setting
B. Deviant peer reinforcement can strengthen antisocial behavior and worsen outcomes compared to individual or family-based treatment
C. Group therapy is never appropriate for adolescents, whose developmental need for peer approval undermines honest disclosure in groups
D. Confidentiality cannot be maintained — adolescents

are likely to share what other members disclose with friends outside the group

99. The "with limited prosocial emotions" specifier for Conduct Disorder (callous-unemotional traits) indicates:

A. The child is manipulative, using charm, flattery, and deceit to exploit peers and adults for personal gain
B. The child should be treated as an adult, including referral to adult correctional or forensic services rather than youth programs
C. Limited prosocial emotions — lack of remorse, shallow affect, and indifference to others' feelings — predicting poorer prognosis and treatment resistance
D. The diagnosis should be changed to Antisocial Personality Disorder — callous-unemotional traits define that disorder regardless of the client's current age

100. Parent Management Training (PMT) for ODD works primarily by:

A. Teaching the child emotional regulation skills to manage frustration and anger when parents set limits at home
B. Improving the parent-child attachment bond by exploring early separations and repairing the child's internal working models
C. Addressing the parents' own mental health issues, because parental stress and depression are the main drivers of the child's defiance
D. Training parents to reinforce prosocial behavior, use consistent consequences, and reduce coercive interaction cycles

101. IED aggressive outbursts must be:

A. Impulsive, disproportionate to provocation, and not premeditated or committed for tangible gain
B. Premeditated and goal-directed, typically planned in advance to intimidate others or obtain something the person wants
C. Directed at specific targets, such as the same family member or coworker, reflecting a consistent pattern of grievance

D. Accompanied by substance intoxication — disinhibition from alcohol or stimulants typically triggers the outbursts

102. Conduct Disorder childhood-onset type carries worse prognosis than adolescent-onset because:

A. Earlier onset means a longer cumulative duration of symptoms, giving the disruptive behavior pattern more years to become entrenched
B. Childhood onset is associated with higher rates of callous-unemotional traits and progression to Antisocial Personality Disorder in adulthood
C. Treatment is unavailable for younger children, because evidence-based interventions such as multisystemic therapy were developed for adolescents
D. Childhood-onset CD involves different symptom types, with more cruelty to animals and fewer rule violations such as truancy

103. Kleptomania differs from theft in Conduct Disorder because:

A. The items stolen are more valuable, because kleptomania centers on acquiring expensive objects the person could not otherwise afford
B. In kleptomania, the stealing is impulsive with rising tension beforehand and relief afterward, not motivated by the value of items or by antisocial intent
C. Kleptomania only occurs in adults, so repeated or impulsive stealing before age 18 is attributed to Conduct Disorder by default
D. Conduct Disorder theft is impulsive while kleptomania is planned, with the person carefully selecting items and timing in advance

Somatic Symptom and Related Disorders

104. A client has been to five specialists for chronic pain with no medical findings. She is frustrated that “no one can find what’s wrong.” The counselor’s FIRST priority is:

A. Explain that the pain is psychological so she can stop pursuing medical answers and redirect her energy toward

counseling instead
B. Request additional medical testing to rule out any condition the five specialists may have missed before any counseling begins
C. Validate that the pain is real and distressing, then assess the thoughts, feelings, and behaviors surrounding the symptoms
D. Diagnose Somatic Symptom Disorder immediately — repeated negative workups with persistent distress meet the full criteria

105. Conversion Disorder (Functional Neurological Symptom Disorder) presents with:

A. Chronic pain without medical explanation, accompanied by excessive thoughts and behaviors about the seriousness of the pain
B. Neurological symptoms such as paralysis, tremor, or blindness that are incompatible with recognized neurological conditions
C. Excessive worry about having a serious neurological illness, despite normal examinations and repeated medical reassurance
D. Intentional fabrication of symptoms such as feigned paralysis or seizures, typically to assume the sick role

106. Factitious Disorder differs from Somatic Symptom Disorder in that:

A. Factitious Disorder involves real physical symptoms, whereas Somatic Symptom Disorder involves symptoms that are imagined
B. Factitious Disorder is less severe — the symptoms are under voluntary control and can stop once the person is confronted
C. Only Somatic Symptom Disorder causes functional impairment, because factitious symptoms are consciously controlled
D. In Factitious Disorder, symptoms are intentionally produced for the purpose of assuming the sick role, without external incentive

107. CBT for Somatic Symptom Disorder targets:

A. Reducing excessive health-related thoughts, feelings, and behaviors while improving functioning despite persistent symptoms
B. Eliminating physical symptoms by correcting the distorted thoughts believed to generate the bodily sensations
C. Proving that symptoms are not real through repeated normal test results and logical disputation of the client's health beliefs
D. Replacing somatization with emotional expression, helping the client verbalize the feelings converted into bodily complaints

108. A client with Illness Anxiety Disorder checks his blood pressure 20 times daily. This behavior should be conceptualized as:

A. Appropriate health monitoring, given that hypertension is common and home readings help detect dangerous changes early
B. Evidence of an obsessive-compulsive disorder that should be diagnosed separately as a comorbid condition alongside illness anxiety
C. A reassurance-seeking compulsion that maintains the anxiety cycle and should be gradually reduced through behavioral intervention
D. A sign of health literacy that reflects the client's engagement in managing his cardiovascular risk

109. Malingering differs from Factitious Disorder in:

A. The type of symptoms produced, with malingering involving psychological complaints and factitious involving physical ones
B. Motivation — malingering involves external incentive (avoiding work, obtaining drugs, legal benefit); Factitious involves internal motivation (sick role)
C. Malingering only involves physical symptoms, whereas Factitious Disorder can involve both physical and psychological symptoms
D. Only malingering is intentional; in Factitious Disorder

the symptoms are produced unconsciously and outside the person's awareness

110. A physician refers a client saying the patient's symptoms are "all in their head." The counselor should:

A. Agree with the physician's characterization so the client receives a consistent message from the treatment team
B. Reframe the referral as an opportunity to address the distress and functional impact of symptoms without invalidating the client's experience
C. Decline the referral since it is inappropriately framed, and send the physician a letter explaining why that language invalidates patients' experiences
D. Explain to the client that their symptoms are psychosomatic and stem from stress rather than from any real physical cause

Dissociative Disorders

111. A client with DID switches identity states during session. The counselor should:

A. Immediately try to bring back the "host" personality, because the host is the part that consented to treatment and holds the goals
B. End the session until the host returns, then reschedule so that treatment continues with the host part that initiated therapy
C. Document the switch and continue as if nothing happened, to avoid reinforcing the switching behavior by drawing attention to it
D. Remain calm, orient to the therapeutic relationship, communicate respect for whichever part is present, and maintain safety

112. Dissociative Amnesia with dissociative fugue involves:

A. Forgetting traumatic events only, while identity and other autobiographical memory remain fully intact during the episode

B. Apparently purposeful travel or bewildered wandering with amnesia for identity or other autobiographical information
C. Memory loss that occurs only during sleep, with complex behaviors carried out in a sleepwalking state
D. Gradual memory loss over months, progressing from recent events to identity as the dissociation deepens

113. The International Society for the Study of Trauma and Dissociation (ISSTD) recommends treatment for DID in which sequence?

A. Integration first, then stabilization — unifying the alters reduces the switching that interferes with stabilization work
B. Phase 1: Safety and stabilization; Phase 2: Trauma processing; Phase 3: Integration and rehabilitation
C. Immediate trauma processing to resolve dissociation, because the amnesic barriers dissolve once the traumatic memories are processed
D. Medication-only treatment before any psychotherapy, to reduce the anxiety and flashbacks that drive dissociative switching

114. Depersonalization episodes in the context of Panic Disorder differ from Depersonalization/Derealization Disorder because:

A. In Panic Disorder, depersonalization occurs only during panic attacks and resolves between them; in DPDR, it persists independently
B. They involve different symptoms — panic-related depersonalization is a bodily sensation rather than a true change in perception
C. DPDR is more severe, producing greater distress and functional impairment than the depersonalization that accompanies panic attacks
D. Panic-related depersonalization does not involve derealization, whereas DPDR requires both experiences to be present

115. A client with DID reports that a "child part" holds trauma memories. The counselor should:

A. Directly access the child part to process trauma immediately — that part holds the memories driving current symptoms
B. Integrate the child part first so the adult can process the trauma from a more unified and emotionally stable position
C. Ignore the part system and treat only the presenting personality, to avoid reinforcing further fragmentation of the client's identity
D. Work with the system to ensure the child part feels safe and contained before any trauma processing, prioritizing stabilization

116. Grounding techniques for dissociative episodes include:

A. Analyzing why the dissociation occurs by exploring the triggering memory with the client while the episode is still unfolding
B. Sensory engagement strategies such as naming objects in the room, holding ice, or describing physical sensations to reconnect with the present moment
C. Deep relaxation and meditation, such as guided imagery or body scans, to calm the nervous system during the episode
D. Avoiding triggers that cause dissociation by identifying them and restructuring daily routines to prevent future exposure

117. A client reports feeling "unreal" but knows the feeling is not actual reality. This intact reality testing is characteristic of:

A. Depersonalization/Derealization Disorder, where the subjective experience is disturbing but reality testing remains intact
B. Psychotic depersonalization, in which the client's sense of unreality reflects an emerging break with consensual, shared reality

C. Schizophrenia prodrome, in which feelings of unreality mark an early loss of contact with reality before full psychosis emerges
D. Dissociative Identity Disorder, in which feelings of unreality occur when another identity state takes executive control

Sleep-Wake Disorders

118. A client reports taking 90 minutes to fall asleep most nights for six months. She watches TV in bed and checks her phone when she cannot sleep. The FIRST CBT-I component to implement is:

A. Stimulus control — removing screens from the bedroom and getting out of bed when unable to sleep within 20 minutes
B. Progressive muscle relaxation practiced in bed each night to reduce the physical tension and arousal that delay sleep onset
C. Sleep restriction to 5 hours per night, to build sleep drive quickly and shorten the time it takes to fall asleep
D. Cognitive restructuring of catastrophic sleep beliefs about the daytime consequences of a poor night's sleep

119. Sleep restriction therapy works by:

A. Forcing the client to sleep fewer hours permanently so the body adapts to a shorter, more efficient sleep schedule
B. Restricting caffeine, alcohol, and vigorous exercise in the hours before bed to reduce physiological arousal at night
C. Reducing total sleep need through behavioral conditioning, so the client functions well on less sleep over time
D. Temporarily limiting time in bed to actual sleep time, building sleep drive and consolidating fragmented sleep

120. A truck driver reports irresistible sleepiness and has nearly fallen asleep at the wheel. His wife notes loud

snoring and breathing pauses during sleep. The counselor should:

A. Begin CBT-I for insomnia, starting with stimulus control to improve the quality and continuity of his nighttime sleep
B. Arrange urgent referral for a sleep study to evaluate for obstructive sleep apnea, given the safety risk of driving
C. Recommend melatonin supplementation to consolidate his nighttime sleep and reduce the daytime sleepiness he experiences on the road
D. Assess for narcolepsy, given the irresistible daytime sleepiness and near-sleep episodes while driving

121. Imagery Rehearsal Therapy (IRT) for nightmare disorder involves:

A. Analyzing dream content for symbolic meaning to uncover the unresolved conflict that is driving the recurring nightmares
B. Lucid dreaming training to control dreams in real time, recognizing the nightmare as it occurs and changing its course from within
C. Writing down the nightmare, changing the storyline to a non-threatening version, and rehearsing the new version before sleep
D. Medication to suppress REM sleep, reducing time in the dream phase in which most nightmares occur

122. Paradoxical intention in insomnia treatment involves:

A. Trying harder to fall asleep by focusing attention on relaxation and on the goal of sleep onset each night
B. Instructing the client to try to stay awake, which reduces performance anxiety about sleeping
C. Sleeping at unusual times to reset the circadian rhythm, gradually shifting bedtime until it reaches the desired hour
D. Using the bedroom only for sleep-related activities so

that the bed becomes a cue for sleep rather than wakefulness

123. Chronic insomnia that persists after depression remission should be:

A. Ignored since the primary condition resolved and residual insomnia typically fades on its own
B. Treated with continued antidepressant medication — sleep typically normalizes with full maintenance dosing
C. Treated independently with CBT-I, since insomnia can become a conditioned, self-maintaining problem
D. Attributed to residual depressive symptoms and monitored closely for early signs of depressive relapse

124. A client reports sleeping 10 hours per night but waking unrefreshed, with excessive daytime sleepiness and difficulty waking. No snoring or breathing pauses. This is MOST consistent with:

A. Insomnia Disorder, reflecting poor sleep quality that leaves the client unrefreshed despite long hours in bed
B. Narcolepsy, characterized by irrepressible sleep attacks with episodes of cataplexy triggered by strong emotion
C. Circadian Rhythm Sleep-Wake Disorder, in which a delayed sleep phase produces difficulty waking and excessive daytime sleepiness
D. Hypersomnolence Disorder, characterized by excessive sleep quantity with non-restorative sleep despite adequate duration

Sexual Dysfunctions and Gender Dysphoria

125. Sensate focus exercises in sex therapy are designed to:

A. Increase sexual performance by teaching partners specific techniques to improve arousal, erection, and orgasm
B. Treat sexual pain disorders through desensitization using graduated vaginal dilators and progressive relaxation training
C. Increase libido through behavioral activation, schedul-

ing pleasurable shared activities that gradually build desire
D. Remove performance pressure by focusing on sensory experience and gradually reintroducing sexual activity without performance demands

126. Before diagnosing any sexual dysfunction, the counselor must:

A. Conduct a thorough sexual history only, covering onset, duration, and situational context of the symptoms
B. Refer to a urologist or gynecologist first — a physical examination must precede any psychological assessment
C. Rule out medical causes, medication side effects, substance use, and relationship factors that may fully account for the symptoms
D. Confirm that the dysfunction causes distress to the partner, since the relationship is the unit most directly affected by the symptoms

127. A transgender client who has completed social transition reports improved mental health but ongoing distress about chest characteristics. The counselor should:

A. Recommend waiting to see if dysphoria resolves as the client adjusts to living fully in their affirmed gender over the coming year
B. Focus on acceptance of current body, using cognitive techniques to reduce distress about chest characteristics over time
C. Conclude treatment, because the counselor's role ends once social transition is complete and mental health has improved
D. Support the client's self-determined goals, facilitate medical referral if surgical consultation is desired, and continue addressing residual dysphoria

128. SSRIs commonly cause which sexual side effect?

A. Increased libido, resulting from improved mood and energy as the depressive symptoms remit during treatment

B. Erectile dysfunction only, which is why sexual side effects are discussed primarily with male clients
C. Delayed orgasm and decreased sexual desire, which should be discussed during informed consent for medication
D. Painful intercourse, which is the sexual complaint most frequently reported by adult patients taking SSRIs

129. A couple presents because one partner has low sexual desire and the other feels rejected. The counselor should:

A. Address both partners' experiences, psychoeducate about desire discrepancy as relational rather than pathological, and assess for underlying causes
B. Treat the low-desire partner individually to identify the personal and medical factors that may be suppressing their sexual interest
C. Recommend scheduling sex to resolve the discrepancy, giving both partners a predictable and fair frequency to rely on
D. Refer to a sex therapist without further assessment — desire discrepancy falls within that specialist's scope of practice and training

130. Gender Dysphoria in children differs from that in adolescents/adults in that:

A. Children cannot experience genuine gender dysphoria, because a stable gender identity does not form until adolescence
B. Childhood presentation may or may not persist into adolescence, requiring supportive monitoring rather than immediate medical intervention
C. Children always require puberty blockers once gender dysphoria is diagnosed, to prevent distress from pubertal changes
D. The diagnostic criteria are identical across age groups, so the same assessment approach and treatment timeline apply to children, adolescents, and adults

131. The WPATH Standards of Care recommend that mental health professionals:

A. Serve as gatekeepers who approve or deny gender-affirming medical care based on their assessment of the client's readiness and psychological stability
B. Require specific therapy duration before approving any medical interventions, typically a minimum number of months of ongoing psychotherapy
C. Diagnose Gender Dysphoria before any social transition can occur, so that name and pronoun changes rest on a formal and documented clinical basis
D. Provide assessment, support, and referral within a collaborative care model that respects client autonomy in medical decision-making

Ethical and Legal Decision Points

132. A client's employer calls requesting information about the client's treatment. The counselor should:

A. Provide general information about diagnosis while withholding session content, to protect the most sensitive details
B. Confirm only that the client is in treatment, without disclosing diagnosis or any other clinical details
C. Decline without a signed release from the client, disclosing only the minimum necessary if a valid release exists
D. Share progress notes to support the client's employment and demonstrate to the employer that treatment is going well

133. A counselor realizes mid-treatment that the client's presenting issue is outside their competence. The ethical response is:

A. Seek supervision or consultation, and if the issue remains outside competence, refer to a qualified provider while ensuring continuity of care
B. Continue treatment while reading about the topic to develop the needed expertise as the work with the client

progresses
C. Terminate immediately and provide referrals so the client can begin with an appropriate provider without further delay in care
D. Competent counselors should be able to treat any presenting issue by adapting their core clinical skills to the client's needs

134. A client gives the counselor a $200 gift. The counselor should:

A. Accept to avoid damaging the therapeutic relationship, since refusing could feel like rejection to the client
B. Refuse all gifts under all circumstances, explaining that professional ethics prohibit accepting anything of value
C. Accept if the value is under a set dollar amount, treating that fixed cutoff as the sole deciding factor so gift decisions stay simple and uniform
D. Explore the meaning of the gift therapeutically, consider the client's cultural context, and make a decision that prioritizes the client's welfare

135. A counselor in a small rural town discovers that the only other therapist in the area has been showing up to sessions intoxicated. The counselor's ethical obligation includes:

A. Minding their own practice — the colleague's conduct falls under the licensing board's responsibility rather than that of a peer in the community
B. Warning the colleague's clients so they can make informed decisions about continuing treatment with that therapist
C. Addressing the concern directly with the colleague, and if unresolved, reporting to the licensing board to protect the colleague's clients
D. Only acting if a client reports harm — firsthand evidence of impaired practice is needed before any action is justified

136. Informed consent must include all of the following EXCEPT:

A. Nature and goals of treatment, including the methods and techniques the counselor anticipates using
B. The counselor's personal therapy history
C. Limits of confidentiality, such as mandated reporting and duty-to-warn situations
D. Client's right to refuse or withdraw from treatment at any time, and the possible consequences of doing so

137. A 16-year-old client discloses sexual activity with a 15-year-old peer. The counselor should:

A. Report to CPS for child sexual abuse, since both participants are legally minors and any sexual contact involving a minor is treated as reportable
B. Assess the relationship for coercion or exploitation; consensual peer sexual activity between minors of similar age may not require reporting depending on state law
C. Inform the parents immediately, because the client is a minor and parents hold the legal right to their child's health information
D. Document but take no further action — the disclosure involves a same-age peer relationship and the client describes the activity as consensual

138. When a counselor disagrees with a client's values or lifestyle choices, the counselor should:

A. Express the disagreement openly to model honesty and genuineness, which strengthens trust and authenticity in the therapeutic relationship
B. Avoid the topic entirely so that the counselor's personal reactions do not leak into the session and harm the working relationship
C. Educate the client about healthier choices, drawing on research about the health and relational outcomes associated with the client's lifestyle
D. Maintain an affirming stance, seek consultation or training if personal reactions are interfering, and refer

only on competence grounds — never solely because of a values conflict

139. The Forester-Miller and Davis ethical decision-making model recommends which step FIRST?

A. Consult with colleagues to gain perspective on the dilemma before forming any interpretation of the situation
B. Review the ACA Code of Ethics to locate the standards that apply to the situation and guide the decision
C. Generate potential courses of action and brainstorm the options available before narrowing to a single choice
D. Identify the problem and determine whether it is an ethical, legal, clinical, or professional issue

140. A counselor receives a subpoena (not a court order) for client records. The counselor should:

A. Release the records immediately, since a subpoena is a legal document that compels compliance by its deadline
B. Ignore the subpoena — it is not a court order and carries no legal obligation for the counselor
C. Release only the treatment summary, which satisfies the subpoena while protecting detailed session content, process notes, and diagnoses
D. Consult with an attorney, notify the client, and assert privilege — a subpoena alone does not compel disclosure without a court order

141. Supervision ethics require that supervisors:

A. Maintain a purely evaluative relationship so that decisions about the supervisee's licensure readiness remain fully objective
B. Focus solely on the supervisee's clinical skills, leaving personal and professional development to the supervisee's own initiative
C. Allow supervisees full autonomy in treatment decisions so they develop independent clinical judgment and professional confidence
D. Monitor supervisee competence, ensure client welfare, maintain appropriate boundaries, and address supervisee impairment

142. A counselor using teletherapy must ensure:

A. HIPAA-compliant technology, informed consent addressing teletherapy-specific risks, emergency planning for the client's location, and awareness of jurisdictional licensing requirements
B. Only that the client consents to video sessions — the client's consent to the modality is considered to cover the remaining teletherapy concerns
C. That sessions are recorded for documentation purposes, creating a complete and retrievable record of each session in case of later board or legal review
D. That teletherapy is only used when in-person sessions are impossible, due to distance, disability, illness, or other access barriers, with in-person care resuming when feasible

Additional Cross-Domain Questions

143. A client presents with symptoms that could be MDD, GAD, or PTSD. The counselor should:

A. Conduct a comprehensive assessment addressing all three differentials before committing to a diagnosis, since treatment differs significantly
B. Pick the most likely diagnosis and begin treatment, revising the diagnosis later if symptoms do not respond
C. Treat the presenting symptoms without diagnosing, as overlapping symptoms such as insomnia and irritability respond to similar interventions
D. Refer for psychological testing to determine the diagnosis — standardized measures can separate overlapping symptom patterns

144. When cultural factors influence how a client expresses distress, the counselor should:

A. Apply DSM criteria without modification, to ensure diagnostic reliability and consistency across clients
B. Avoid diagnosis to prevent cultural pathologizing of expressions of distress that are normative and expected within the client's own cultural community

C. Use the DSM-5-TR Cultural Formulation Interview to understand how culture shapes symptom expression, help-seeking, and treatment expectations
D. Refer to a counselor of the same cultural background who can interpret the client's distress from within that culture

145. A newly licensed counselor feels overwhelmed by a complex case. The MOST appropriate response is:

A. Transfer the client to a more experienced clinician who has treated similar complex cases and can offer better care
B. Research the presenting problem independently, reviewing the literature to build the knowledge the case requires
C. Inform the client of limited experience with their condition and share how overwhelmed the case has left the counselor
D. Seek supervision or consultation while continuing to provide competent care within one's current scope

146. Evidence-based practice in counseling integrates:

A. Only randomized controlled trial evidence — RCTs provide the strongest basis for causal claims
B. Whatever approach the counselor is most comfortable with — counselor confidence drives outcomes
C. Best available research evidence, clinical expertise, and client values and preferences
D. Manualized treatments without modification, delivered with fidelity to the tested protocol

147. A client in therapy for depression begins showing symptoms suggestive of hypomania. The counselor should:

A. Continue the current treatment plan — brief elevations in mood are common as depression lifts with treatment
B. Wait to see if a full manic episode develops before changing anything, to avoid overdiagnosing bipolar disorder
C. Diagnose Bipolar Disorder immediately and inform the

prescriber so the antidepressant can be discontinued and a mood stabilizer started
D. Assess for a bipolar spectrum disorder, coordinate with the prescriber about medication implications, and adjust the treatment plan

148. Therapeutic alliance research consistently shows:

A. Technique matters more than the relationship, because specific interventions account for most of the measurable variance in treatment outcomes
B. The quality of the therapeutic relationship is one of the strongest predictors of treatment outcome across all therapeutic modalities
C. Alliance is important only in humanistic therapies, where the relationship itself is considered the primary mechanism of therapeutic change
D. A strong alliance compensates for using non-evidence-based techniques — relational factors drive most of the change that occurs in therapy

149. When a client and counselor have a therapeutic rupture (the client expresses dissatisfaction with therapy), the counselor should:

A. Defend the treatment approach by explaining the rationale and evidence base behind the current interventions
B. Refer to a different counselor whose theoretical orientation and style may be a better fit for the client's needs and preferences
C. Explore the rupture directly, validate the client's experience, and use the repair process as a therapeutic intervention
D. Adjust the treatment plan without discussion, quietly shifting approach to address the client's dissatisfaction and preferences

150. Multicultural competence requires ongoing:

A. Training in specific cultural practices of all populations the counselor may serve, to ensure accurate and respectful interventions in every case they encounter

B. Matching counselor-client demographics such as race and gender whenever possible, to maximize shared understanding and trust
C. Self-awareness of one's own cultural assumptions, knowledge of diverse worldviews, and development of culturally appropriate intervention strategies
D. Avoidance of discussing cultural differences in session, to keep the focus on the client's presenting concerns

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151. A counselor begins using an AI note-taking tool that records sessions and drafts progress notes. The client has not been told. The counselor's best next step is to:

A. Continue, because progress notes are the counselor's work product and the tool is HIPAA-compliant
B. Tell the client at the next session what the tool does and its risks, obtain written consent, and offer to continue without it if the client declines
C. Stop using the tool, because AI-generated documentation is prohibited by the ACA Code
D. Add a line about AI tools to the practice's website privacy policy and continue

152. A client says she talks to an AI chatbot every night and that it "helps more than anything," including the night last week when she had thoughts of not wanting to wake up. The counselor should:

A. Tell her to stop using the chatbot because it is not a substitute for counseling
B. Explore how she uses it and what helps, name its limits — including that it cannot provide crisis response — and build human contacts and 988 into her safety plan
C. Ask her to bring the chat transcripts to the next session so the counselor can review the chatbot's advice
D. Document the chatbot use and continue with the planned session agenda

153. An AI note-taker drafts a progress note that includes a diagnosis the counselor did not make, inferred from the session transcript. The counselor should:

A. Keep the note as drafted, since the tool is HIPAA-compliant and the diagnosis is plausible
B. Delete the entire note and write a brief note by hand to avoid any AI involvement
C. Review and correct the note before signing it, removing the unmade diagnosis — the counselor is responsible for the note's content and for any diagnosis
D. Add the diagnosis to the treatment plan, since the tool identified something the counselor missed

Supplemental Answer Key and Rationales

Answers and rationales for the 153 supplemental practice questions. Each rationale states why the keyed option is correct and why the principal distractors fail.

Depressive Disorders

1. Answer: C — Persistent Depressive Disorder

Rationale: Three years of continuous low mood with no sustained well period is the defining chronicity of Persistent Depressive Disorder (≥2 years in adults, never symptom-free for more than two months). A fails because there is no premorbid baseline he fell from and no discrete recurrent episodes. B fails on time course — Adjustment Disorder requires an identifiable stressor and resolves within six months. D dismisses a diagnosable, treatable condition as personality, which is the exact clinical error Chapter 8 warns against.

2. Answer: D — Continue CBT with added behavioral activation targeting residual anhedonia

Rationale: Residual anhedonia and fatigue after a partial response are the strongest predictors of relapse, and behavioral activation directly targets anhedonia. A terminates with active symptoms present. B abandons a treatment that is working. C makes medication the sole intervention when the psychotherapy is producing measurable gains.

3. Answer: A — Assess duration and severity against MDD criteria, since atypical features suggest a mood disorder rather than adjustment

Rationale: Hypersomnia, hyperphagia, and leaden paralysis are the atypical-features profile of a major depress-

ive episode, so the priority is measuring duration and severity against MDD criteria rather than defaulting to the stressor. B assumes Adjustment Disorder because a stressor exists — but if full MDD criteria are met, MDD is the diagnosis regardless of precipitant. C begins treatment before the diagnosis is established. D is worth doing eventually, but atypical features alone do not make bipolarity the first question.

4. Answer: C — MDD with peripartum onset and intrusive ego-dystonic thoughts common in perinatal OCD overlap

Rationale: Intrusive thoughts that horrify the mother are ego-dystonic — she is distressed *by* them, which is the signature of perinatal OCD-spectrum intrusions accompanying MDD with peripartum onset. A is the dangerous misread: postpartum psychosis involves ego-syntonic delusional beliefs and loss of reality testing, neither of which is present. B dismisses clinically significant symptoms at six weeks. D omits the mood disturbance that dominates the presentation.

5. Answer: C — Predicts poorer treatment response and higher suicide risk, warranting more aggressive treatment

Rationale: The anxious distress specifier is prognostic: it predicts poorer treatment response, longer time to remission, and elevated suicide risk, which is why it changes treatment intensity. A confuses a specifier with a separate diagnosis. B prescribes, which is outside counselor scope and is not what the specifier indicates. D inverts the relationship — the specifier describes features within the depressive episode, not an etiology.

6. Answer: C — Discuss relapse prevention, identify early warning signs, and plan a maintenance phase before ending

Rationale: Ending treatment is a clinical phase: consolidate gains, identify early warning signs, and plan maintenance before termination. A ends at the point of greatest

relapse vulnerability. B pathologizes recovery by making therapy open-ended. D imposes a schedule without the collaborative relapse-prevention work that makes it useful.

7. Answer: C — Anhedonia, which can be the primary manifestation of MDD even without reported sadness

Rationale: Anhedonia is a cardinal MDD symptom and can be the dominant presentation without reported sadness — "I don't feel anything" is a depression statement, not a depression rule-out. A and B route away from the mood disorder the statement most likely signals. D reaches for trauma without any trauma history in the stem.

8. Answer: C — Depressive pseudodementia, since "I don't know" responses and distress about deficits are characteristic

Rationale: Depressive pseudodementia is distinguished by "I don't know" responses, prominent distress about the deficits, and psychomotor retardation. In a true major neurocognitive disorder the person typically confabulates or minimizes rather than reporting distress about failing. A over-reads the presentation. B normalizes clinically significant impairment. D does not fit — delirium has acute onset and fluctuating attention, not this subacute picture.

9. Answer: C — Stabilize the acute MDD episode first, then address the chronic PDD substrate with combined therapy and medication consultation

Rationale: Double depression requires stabilizing the acute major depressive episode first, then treating the underlying chronic PDD substrate with combined psychotherapy and medication consultation. A defers the more acute and more dangerous layer. B ignores that the two layers need different intensities. D is factually wrong — PDD is treatable, and combined treatment is the standard of care.

Anxiety Disorders

10. Answer: B — Social Anxiety Disorder with cued panic attacks in social situations

Rationale: Panic attacks confined to social-evaluative situations with no symptoms between them are situationally cued, which places them within Social Anxiety Disorder. A requires unexpected attacks. C requires pervasive multi-domain worry, explicitly absent here. D is incorrect — fear of social scrutiny is coded as Social Anxiety Disorder, not Specific Phobia.

11. Answer: C — Explore the reassurance-seeking pattern while teaching tolerance of uncertainty as a core therapeutic target

Rationale: Reassurance is the safety behavior that maintains GAD; giving it relieves distress for minutes and strengthens the loop. Naming the pattern and building tolerance of uncertainty targets the maintaining mechanism. A feeds the cycle. B moves to technique without addressing the in-session behavior. D validates content while reinforcing the process.

12. Answer: B — Agoraphobia, characterized by fear of situations where escape might be difficult during panic-like symptoms

Rationale: Fear of situations where escape would be difficult or help unavailable if panic-like symptoms occur, across multiple situation types, is Agoraphobia. A would require a single circumscribed stimulus, not a cluster unified by escape. C concerns judgment by others, which is not the described fear. D lacks any Criterion A trauma.

13. Answer: A — Deliberately inducing feared physical sensations to reduce catastrophic interpretations of bodily cues

Rationale: Interoceptive exposure deliberately induces the feared bodily sensations — through hyperventilation, spinning, or exertion — so the catastrophic interpretation of those cues can be disconfirmed. B describes situational

exposure. C describes relaxation, which does not test the feared prediction. D describes cognitive work, not exposure.

14. Answer: D — Assess for Separation Anxiety Disorder and develop a graduated return-to-school plan with parent coaching

Rationale: Morning somatic complaints that remit once the child is at school, with negative medical findings, point to Separation Anxiety Disorder; the intervention is graduated return to school with parent coaching. A accommodates the avoidance and entrenches it. B repeats an evaluation already completed. C defers an evidence-based behavioral plan for open-ended exploration while school absence continues.

15. Answer: B — It involves a vasovagal fainting response requiring modified exposure techniques that include applied tension

Rationale: Blood-injection-injury phobia produces a diphasic vasovagal response — blood pressure and heart rate drop, sometimes to syncope — rather than sustained sympathetic arousal, so exposure must be paired with applied tension. A is backwards; exposure remains first-line. C is false; avoidance of medical care is a serious impairment. D is incorrect; onset is typically in childhood.

16. Answer: C — Use individual sessions to build exposure skills, then gradually introduce group as a therapeutic exposure target

Rationale: Build exposure capacity in individual work, then introduce group as a graded exposure target. The group is not merely a logistical option — it is the feared stimulus, which makes it therapeutic once she is prepared. A permanently accommodates the avoidance that maintains the disorder. B coerces exposure without preparation and risks dropout. D changes the diagnosis to fit a treatment, reversing the clinical logic.

17. Answer: B — The pervasiveness, uncontrollability, and associated physical symptoms persisting for six or more months

Rationale: GAD is defined by worry that is pervasive across domains, experienced as uncontrollable, accompanied by somatic symptoms, and present for six months or more. A is wrong because content does not distinguish — GAD worry is usually about ordinary matters. C is wrong for the same reason; realistic concerns can still be pathological worry. D belongs to Panic Disorder.

Trauma and Stressor-Related Disorders

18. Answer: A — PTSD with dissociative subtype, which is a single diagnosis with a specifier

Rationale: Persistent depersonalization or derealization in a client who meets full PTSD criteria is coded as PTSD with dissociative subtype — one diagnosis with a specifier. B double-codes symptoms already accounted for. C is incorrect; Complex PTSD is an ICD-11 construct and does not replace a DSM-5-TR PTSD diagnosis (APA, 2022). D requires distinct identity states, which are not described.

19. Answer: D — Trauma-related avoidance of internal and external reminders, a core PTSD symptom cluster

Rationale: Avoiding fireworks, loud environments, and war films is avoidance of both external reminders and the internal states they provoke — a core PTSD symptom cluster, not a separate condition. A treats the avoidance as simple phobia and omits the rest of the syndrome. B misapplies agoraphobia, which concerns escape rather than trauma cues. C reframes a symptom cluster as a skills deficit.

20. Answer: D — Symptoms must persist beyond one month for PTSD; at three days, monitor for Acute Stress Disorder

Rationale: PTSD requires symptoms persisting more than one month; at three days the appropriate frame is Acute Stress Disorder, which is diagnosable from day three through week four. A is false — a motor vehicle accident can absolutely meet Criterion A. B is false; PTSD is not limited to combat. C is false; emotional numbing is part of the negative-alterations cluster.

21. Answer: C — PTSD with behavioral symptoms, where defiance reflects hyperarousal and mistrust rather than a conduct problem

Rationale: Hypervigilance and nightmares alongside defiance in a child removed from an abusive home are better explained by trauma — the defiance reflects hyperarousal and learned mistrust of adults, not a conduct problem. A and B mistake a trauma presentation for a behavior disorder and lead to interventions that replicate the coercion. D understates a Criterion A history with full trauma symptoms.

22. Answer: A — Activating the trauma memory in a safe context to allow habituation and corrective learning

Rationale: Prolonged Exposure works by repeated activation of the trauma memory in a safe context, permitting habituation and corrective learning that the feared memory is not itself dangerous (Foa et al., 2019). B describes catharsis, which is not the mechanism and is insufficient alone. C describes the stabilization phase that precedes PE. D describes Cognitive Processing Therapy's mechanism (Resick et al., 2017).

23. Answer: B — Three months

Rationale: Adjustment Disorder requires symptom onset within three months of the identifiable stressor. The six-month figure in C is the resolution window after the stressor ends, which is the most common confusion on this item. A and D do not correspond to any DSM-5-TR criterion.

24. Answer: D — Psychoeducate about avoidance as a maintaining factor while respecting readiness and pacing exposure accordingly

Rationale: Avoidance is the maintaining mechanism, so it must be addressed — but through psychoeducation and paced exposure calibrated to readiness, not by overriding her. A colludes with the maintaining factor. B moves to processing without stabilization, the central error in trauma treatment. C medicalizes a behavioral process.

25. Answer: D — Disturbances in self-organization: affect dysregulation, negative self-concept, and interpersonal difficulties

Rationale: ICD-11 Complex PTSD adds disturbances in self-organization — affect dysregulation, persistent negative self-concept, and interpersonal difficulties — to the core PTSD triad. A, B, and C name conditions that may co-occur but are not part of the CPTSD construct.

Substance-Related Disorders

26. Answer: C — Explore ambivalence using motivational interviewing, developing discrepancy between current behavior and stated values

Rationale: In contemplation the client is ambivalent, and the task is to explore that ambivalence and develop discrepancy between current behavior and stated values. A supplies an action-stage intervention prematurely. B produces discord and sustain talk. D prescribes a specific modality before the client has resolved ambivalence.

27. Answer: A — Expected physiological dependence, which is distinct from Opioid Use Disorder

Rationale: Tolerance and withdrawal occurring under appropriate medical supervision are expressly excluded from counting toward a substance use disorder. With no craving, no loss of control, and no impairment, this is physiological dependence, not Opioid Use Disorder. B ap-

plies criteria the DSM-5-TR specifically exempts. C and D pathologize an expected pharmacological effect.

28. Answer: D — Six dimensions including intoxication/withdrawal risk, medical conditions, emotional/behavioral conditions, readiness to change, relapse potential, and recovery environment

Rationale: The ASAM Criteria assess six dimensions: acute intoxication and withdrawal potential; biomedical conditions; emotional, behavioral, or cognitive conditions; readiness to change; relapse and continued-use potential; and recovery environment (ASAM, 2013). A, B, and C name factors that are either insufficient or irrelevant to clinical placement.

29. Answer: D — Monitor, assess suicide risk during the crash, and provide supportive care; stimulant withdrawal is uncomfortable but not medically dangerous

Rationale: Stimulant withdrawal is dysphoric and can carry suicide risk requiring assessment, but unlike alcohol and benzodiazepine withdrawal it is not medically dangerous; supportive care and monitoring are appropriate. A over-triages. B and C treat expected withdrawal phenomena as an independent depressive episode before the crash has resolved.

30. Answer: A — Collaboratively assess readiness, develop a coping plan for high-risk situations, and identify refusal skills

Rationale: Collaboratively assess readiness, plan for the high-risk situation, and rehearse refusal skills — this respects autonomy while building relapse-prevention capacity. B imposes permanent avoidance, which is neither realistic nor client-directed. C encourages exposure without a plan. D applies an arbitrary timeline in place of assessment.

31. Answer: A — Open questions, Affirmations, Reflections, Summaries

Rationale: OARS is Open questions, Affirmations, Reflections, and Summaries — the four core MI skills (Miller & Rollnick, 2013). The remaining options are plausible-sounding acronyms that do not correspond to the model.

32. Answer: D — Reduces craving and the rewarding effects of alcohol by blocking opioid receptors

Rationale: Naltrexone is an opioid antagonist; by blocking opioid receptors it reduces craving and blunts alcohol's rewarding effects. A describes disulfiram, which produces an aversive reaction. B loosely describes acamprosate's glutamate/GABA mechanism. C describes an antidepressant, not naltrexone.

33. Answer: D — Assess the relapse nonjudgmentally, identify triggers and high-risk situations, and adjust the treatment plan

Rationale: Relapse is clinical data. Assess it without judgment, identify the triggers and high-risk situations that preceded it, and revise the treatment plan accordingly. A punishes the client for the condition being treated. B discards accumulated progress. C increases dose without addressing cause.

Personality Disorders

34. Answer: A — Establishing clear between-session contact protocols within a DBT framework, including phone coaching guidelines

Rationale: DBT anticipates between-session crisis contact and structures it as phone coaching: brief, skills-focused, and governed by clear protocols agreed in advance. B reinforces crisis behavior and is unsustainable. C withdraws support in a population with genuine risk. D escalates level of care in response to a pattern the treatment model is designed to handle.

35. Answer: C — Underlying vulnerability and fragile self-esteem that the grandiosity defends against

Rationale: Grandiosity in NPD typically defends against fragile self-esteem and underlying vulnerability; assessing that layer is what makes the work possible and predicts the collapse that can follow narcissistic injury. A pathologizes grandiosity as psychosis. B overstates comorbidity as invariable. D misattributes a stable trait pattern to an episodic mood disorder.

36. Answer: C — Build trust in the therapeutic relationship, then gradually expose to feared social situations while addressing core beliefs of inadequacy

Rationale: AvPD involves desire for connection blocked by fear of rejection, so treatment builds safety in the therapeutic relationship first, then graduated social exposure alongside work on core beliefs of inadequacy. A addresses skills the client often already has. B exposes without preparation and typically produces dropout. D relabels impairment as temperament.

37. Answer: A — Schizotypal PD involves eccentric beliefs and social deficits without sustained psychotic episodes; Schizophrenia involves active psychosis

Rationale: Schizotypal PD involves odd beliefs, perceptual distortions, and social deficits without sustained psychotic episodes; Schizophrenia requires active-phase psychotic symptoms. B, C, and D name features that do not reliably separate the two — the presence or absence of frank psychosis does.

38. Answer: C — Maintain therapeutic skepticism, focus on behavioral accountability, and verify self-report with collateral information

Rationale: Smooth engagement and rapid declarations of change in a court-mandated ASPD client warrant therapeutic skepticism, a focus on observable behavioral accountability, and verification against collateral sources. A accepts self-report as evidence. B is an unethical refusal of care. D confronts prematurely and damages the working relationship without producing accurate information.

39. Answer: D — Identifying and modifying early maladaptive schemas that drive chronic relational and emotional patterns

Rationale: Schema therapy identifies early maladaptive schemas formed in childhood and modifies the chronic emotional and relational patterns they drive, using limited reparenting and experiential methods (Young et al., 2003). A describes standard behavioral treatment. B describes skills training. C is outside counselor scope and is not what schema therapy is.

40. Answer: D — Mindfulness, Distress Tolerance, Emotion Regulation, Interpersonal Effectiveness

Rationale: DBT's four modules are Mindfulness, Distress Tolerance, Emotion Regulation, and Interpersonal Effectiveness. A lists generic skills-group topics. B lists standard CBT components. C lists mentalization-based treatment concepts (Bateman & Fonagy, 2016).

41. Answer: D — Acknowledge the progress naturally without over-praising, which could reinforce dependence on the counselor's approval

Rationale: Acknowledge the step naturally without effusive praise — over-praising makes the counselor's approval the new source of direction and recreates the dependency inside the therapy. A does exactly that. B escalates faster than the gain can consolidate. C misreads genuine progress as resistance.

Bipolar and Related Disorders

42. Answer: C — Bipolar II Disorder, since one hypomanic episode plus a depressive episode meets criteria

Rationale: One hypomanic episode (four days or more — five qualifies) plus at least one major depressive episode, with no history of full mania, is Bipolar II. A misses the hypomania and risks antidepressant monotherapy. B requires two years of subthreshold fluctuation without a full

depressive episode. D is wrong — hypomania is not mania, and Bipolar I requires a full manic episode.

43. Answer: B — Explore her reasoning, psychoeducate about relapse risk with medication discontinuation, and coordinate with the prescriber

Rationale: Explore her reasoning, provide accurate psychoeducation about the substantial relapse risk following mood-stabilizer discontinuation, and coordinate with the prescriber. A supports a decision without informing it. C is directive and non-collaborative. D abdicates the counselor's legitimate psychoeducation and coordination role.

44. Answer: B — Four or more mood episodes within a 12-month period

Rationale: Rapid cycling is four or more mood episodes meeting full criteria within twelve months. A describes ultradian mood lability, often seen in BPD rather than rapid cycling. C and D describe patterns that do not match the specifier's definition.

45. Answer: B — Ensure psychiatric evaluation for mood stabilization, since active mania typically requires pharmacological intervention

Rationale: Ten days of elevated mood with decreased need for sleep, pressured speech, grandiosity, and impulsive spending is an active manic episode, which requires pharmacological intervention — so securing psychiatric evaluation is the priority. A offers psychotherapy that cannot stabilize acute mania. C escalates to involuntary commitment without evidence of imminent danger or refusal of care. D is a legitimate rule-out but does not outrank stabilization.

46. Answer: C — Regularization of daily routines, sleep-wake cycles, and interpersonal relationships to prevent mood episodes

Rationale: IPSRT stabilizes daily routines, sleep-wake timing, and interpersonal roles on the premise that circadian and social rhythm disruption precipitates mood episodes

(Frank, 2005). A describes CBT. B describes family-focused therapy. D reduces a full psychotherapy to one component.

47. Answer: A — Symptom severity — cyclothymia involves hypomanic and depressive symptoms that do not meet full episode criteria

Rationale: Cyclothymia involves hypomanic and depressive symptoms that never meet full episode criteria, over at least two years. Bipolar II requires at least one full hypomanic episode and one full major depressive episode. B, C, and D do not define the boundary.

48. Answer: A — SSRI monotherapy without a mood stabilizer can trigger hypomania or rapid cycling in bipolar patients

Rationale: Antidepressant monotherapy without mood-stabilizer coverage can precipitate hypomania, mixed states, or rapid cycling in bipolar spectrum illness — a well-documented risk the counselor should raise with the client and prescriber. B ignores it. C is a jurisdictional complaint rather than a clinical concern. D advises abrupt unsupervised discontinuation, which is unsafe and outside scope.

49. Answer: B — The positive experience of mania can reduce medication adherence and treatment engagement, requiring direct exploration

Rationale: When mania is remembered as the client's most creative and productive state, that positive valuation is a leading cause of medication nonadherence, and it needs direct, nonjudgmental exploration. A misreads subjective enjoyment as an index of severity. C questions an established diagnosis on the basis of a common phenomenon. D endorses pursuing a state that is dangerous to induce.

Neurodevelopmental Disorders

50. Answer: A — Gather comprehensive developmental history including collateral informants, since ADHD requires evidence of symptoms before age 12 across multiple settings

Rationale: ADHD requires several symptoms present before age 12 and impairment across two or more settings, so a comprehensive developmental history with collateral informants is the necessary step — particularly when self-report and school records diverge. B diagnoses from uncorroborated self-report. C orders testing before the history that would direct it. D rules out prematurely; adequate grades do not exclude ADHD, especially in bright or well-supported children.

51. Answer: A — Provide psychoeducation about multimodal treatment including behavioral strategies, parent training, and medication as options, supporting informed decision-making

Rationale: Provide psychoeducation about the full multimodal picture — behavioral strategies, parent training, school supports, and medication as one option — so the parent can make an informed decision. B and C substitute the counselor's position for the family's informed choice, in opposite directions. D withholds information the counselor is qualified to give.

52. Answer: B — Discuss how ASD without intellectual impairment can be masked by compensatory strategies and intellectual ability, especially without childhood behavioral concerns

Rationale: Intellectual ability and compensatory masking frequently conceal ASD through childhood, particularly in the absence of disruptive behavior, which is why diagnosis is often delayed to adulthood. A assigns blame without basis. C undermines a diagnosis that is providing him a framework. D dismisses a question that is doing real meaning-making work.

53. Answer: D — Six or more symptoms in both the inattentive and hyperactive-impulsive domains (five or more at age 17 and older), with several present before age 12

Rationale: Combined presentation requires six or more inattentive **and** six or more hyperactive-impulsive symptoms (five each for those 17 and older), with several present before age 12. A describes the predominantly inattentive presentation. B invents a comorbidity requirement. C wrongly narrows impairment to academic settings.

54. Answer: C — External organizational supports, environmental modifications, and skills training targeting planning, time management, and task initiation

Rationale: Executive dysfunction responds to external scaffolding — environmental modification, organizational systems, and explicit skills training in planning, time management, and task initiation — rather than to insight. A assumes the deficit is motivational. B omits the psychosocial half of multimodal treatment. D treats a neurocognitive deficit as a belief problem.

55. Answer: A — ASD reflects difficulty understanding social cues and reciprocity; SAD reflects intact social skills inhibited by fear of negative evaluation

Rationale: In ASD the difficulty is in reading social cues and sustaining reciprocity — a capacity difference. In Social Anxiety Disorder social skills are intact but inhibited by fear of negative evaluation. B compares severity rather than mechanism. C is false; both benefit from social skills work. D assigns an ASD feature to SAD.

56. Answer: D — Psychoeducate that ADHD is a neurobiological condition affecting executive function, not a willpower deficit, while validating the parent's frustration

Rationale: Psychoeducate that ADHD is a neurobiological condition affecting executive function rather than a willpower deficit, while validating the parent's genuine frustration — both halves are required for the parent to stay engaged. A endorses the misattribution. B escalates coercive cycles. C omits the parent, who is central to treatment.

57. Answer: A — SLD involves circumscribed deficits in specific academic areas with otherwise normal cognitive ability; ID involves globally reduced functioning

Rationale: Specific Learning Disorder involves circumscribed deficits in reading, writing, or mathematics against otherwise intact cognitive ability, while Intellectual Developmental Disorder involves global deficits in intellectual and adaptive functioning. B compares severity rather than pattern. C is false; both qualify for accommodations. D is false; SLD is persistent, though its impact is reduced with intervention.

Neurocognitive Disorders

58. Answer: A — Delirium, which is a medical emergency requiring immediate workup for underlying cause

Rationale: Acute onset over days, fluctuating attention, and visual hallucinations define delirium, which is a medical emergency requiring workup for the underlying cause — infection, medication, metabolic disturbance. B develops over months to years. C is a rare late-onset presentation that does not fit this time course. D mislabels an acute change as normal and delays urgent care.

59. Answer: A — Early personality and behavioral changes with relative sparing of memory in frontotemporal; early memory loss in Alzheimer's

Rationale: Frontotemporal NCD begins with personality change, disinhibition, and loss of social conduct while memory is relatively spared early; Alzheimer's begins

with episodic memory loss. B overlaps substantially. C and D are secondary features that do not reliably separate the two at presentation.

60. Answer: D — Regular aerobic exercise, social engagement, cognitive stimulation, sleep optimization, and management of cardiovascular risk factors

Rationale: The evidence supports a multidomain approach: aerobic exercise, social engagement, cognitive stimulation, sleep optimization, and vigorous management of cardiovascular risk factors. A overstates commercial brain training, whose gains largely fail to transfer. B misuses a symptomatic treatment as prevention. C is both inaccurate and clinically abandoning.

61. Answer: A — Decision-making capacity: understanding, appreciation, reasoning, and ability to express a choice

Rationale: Decision-making capacity has four components: understanding the information, appreciating how it applies to oneself, reasoning among options, and communicating a choice. Capacity is decision-specific rather than global. B confuses agreement with capacity. C substitutes the family's preference. D is irrelevant to capacity.

62. Answer: B — Stepwise decline associated with cerebrovascular events rather than gradual continuous decline

Rationale: Vascular NCD classically shows stepwise decline tied to discrete cerebrovascular events, against Alzheimer's gradual continuous course. A is not reliably true. C points toward Lewy body disease. D is not a dependable discriminator.

63. Answer: B — Provide caregiver support, psychoeducation, respite resource referrals, and monitor for caregiver depression and burnout

Rationale: The caregiver is the client in this encounter: provide support, psychoeducation about the disease course, respite and community referrals, and active mon-

itoring for caregiver depression and burnout, which carry real morbidity. A ignores the person in the room. C leaps to placement. D deflects care the counselor is positioned to provide.

64. Answer: A — Increased confusion, agitation, and behavioral disturbance in late afternoon and evening, likely related to circadian disruption

Rationale: Sundowning is increased confusion, agitation, and behavioral disturbance in the late afternoon and evening, associated with circadian disruption, fatigue, and reduced sensory cues. B, C, and D name other phenomena that do not match the defining time-of-day pattern.

65. Answer: A — Consider the score within context — a high-functioning individual scoring 28 may still represent meaningful decline from their baseline

Rationale: MoCA scores are interpreted against the individual's own baseline and educational attainment; 28/30 can represent meaningful decline in someone with high premorbid function (Nasreddine et al., 2005). B treats a normal-range score as proof of no impairment. C defers without gathering history. D escalates before the clinical picture warrants it.

OCD and Related Disorders

66. Answer: A — Constructing a fear hierarchy and beginning with low-to-moderate items while preventing the ritual response

Rationale: ERP builds a fear hierarchy and begins at low-to-moderate intensity while preventing the compulsive response — the response prevention is what permits new learning (Foa et al., 2012). B risks dropout by flooding. C substitutes a competing safety behavior for the ritual. D delays exposure, the active ingredient, for cognitive work that alone is less effective.

67. Answer: A — Both involve repetitive behaviors driven by preoccupation — OCD with intrusive thoughts, BDD with perceived appearance flaws

Rationale: BDD sits with the OCD-related disorders because both pair intrusive preoccupation with repetitive behaviors — obsessions and compulsions in OCD, perceived appearance defects with mirror checking, camouflaging, and reassurance seeking in BDD. B and D name features specific to particular presentations rather than the shared structure. C describes treatment response, not classification logic.

68. Answer: B — Normalize aggressive obsessions as ego-dystonic intrusive thoughts common in OCD that do not indicate violence risk

Rationale: Aggressive obsessions are among the most common and most distressing OCD themes, and they are ego-dystonic — repugnant to the person having them — which is precisely why they do not indicate violence risk. Normalizing this reduces shame and enables treatment. A and C treat a symptom as a threat. D avoids the content, which reinforces the fear that the thoughts are dangerous.

69. Answer: D — OCPD rigidity is ego-syntonic (the person values their standards); OCD obsessions are ego-dystonic (unwanted and distressing)

Rationale: OCPD rigidity is ego-syntonic: the person regards their standards as correct and does not want to change them. OCD obsessions are ego-dystonic: unwanted, intrusive, and distressing. A describes OCD, not OCPD. B compares severity rather than structure. C is false.

70. Answer: A — Habit reversal training as the primary behavioral intervention, with competing response training

Rationale: Habit reversal training — awareness training plus a competing response incompatible with pulling,

with social support — is the first-line behavioral treatment for trichotillomania. B pursues insight, which does not reduce a body-focused repetitive behavior. C overstates medication, whose evidence here is modest. D is contraindicated and unethical.

71. Answer: C — Explore the meaning and function of saving these items within the therapeutic relationship before attempting any discarding

Rationale: In Hoarding Disorder the items carry meaning — identity, memory, anticipated need — and discarding before that meaning is understood ruptures the alliance and reliably fails. A is a serious ethical violation and destroys trust. B imposes an external deadline on an internal process. D addresses only half the disorder.

72. Answer: B — Involve repetitive body-focused behaviors amenable to habit reversal training and stimulus control

Rationale: Excoriation and trichotillomania are both body-focused repetitive behaviors, and both respond to habit reversal training combined with stimulus control that alters the cues and opportunities for the behavior. A oversimplifies the function, which is often soothing or automatic rather than anxiety-driven. C addresses sequelae, not the behavior. D is incorrect; both are distinct DSM-5-TR diagnoses.

73. Answer: D — SSRIs can reduce symptom severity and are often used alongside ERP, typically at higher doses than for depression

Rationale: SSRIs reduce OCD symptom severity, are typically required at higher doses and longer trials than for depression, and work best alongside ERP rather than instead of it. A overstates medication and displaces the more effective treatment. B is false. C imposes a sequence the evidence does not support.

Psychotic Disorders

74. Answer: D — Prodromal phase of schizophrenia or early psychosis, warranting comprehensive psychiatric evaluation

Rationale: Six months of social withdrawal, functional decline, and apparent responding to internal stimuli in a young adult is a prodromal or first-episode psychosis picture, and early intervention substantially improves outcomes — so comprehensive psychiatric evaluation is the priority. A and B are legitimate differentials but cannot be assumed without evaluation. C does not account for the talking to himself or the functional collapse.

75. Answer: C — Flat affect, avolition, alogia, anhedonia, and social withdrawal

Rationale: Negative symptoms are deficits: flat affect, avolition, alogia, anhedonia, and asociality. A and B name positive and disorganized symptoms. D names a positive symptom. Negative symptoms drive long-term functional outcome and respond least to medication.

76. Answer: B — Reducing distress related to voices and modifying beliefs about their power, while improving functioning

Rationale: CBTp aims to reduce the distress and disability associated with voices and to modify beliefs about their power, omniscience, and the need to comply — improving functioning rather than eliminating the perceptual experience (Morrison, 2017). A sets an unattainable target. C is unsafe and outside scope. D applies an approach not indicated for psychosis.

77. Answer: B — Duration — Brief Psychotic Disorder lasts under one month with full return to functioning; Schizophreniform lasts one to six months

Rationale: The distinction is duration: Brief Psychotic Disorder lasts from one day to under one month with full return to premorbid functioning; Schizophreniform runs

one to six months; beyond six months the diagnosis becomes Schizophrenia. A, C, and D do not define the boundary.

78. Answer: C — One or more delusions with relatively preserved functioning outside the delusional system and without prominent hallucinations or disorganization

Rationale: Delusional Disorder involves one or more delusions — DSM-5 dropped the former non-bizarre requirement — with functioning outside the delusional system relatively preserved and without the deterioration or prominent other psychotic symptoms of schizophrenia. A and D describe schizophrenia. B is excluded — hallucinations, if present, are not prominent and relate to the delusional theme.

79. Answer: B — The disorder has both mood and psychotic components that can persist independently, so the prescriber may target each component

Rationale: Schizoaffective disorder involves both mood episodes and psychotic symptoms that persist independently — by definition delusions or hallucinations occur for two or more weeks without prominent mood symptoms — so the prescriber may target each component, whether with one agent or a combination. A contradicts the diagnosis. C is pharmacologically false. D declines a psychoeducation role squarely within scope.

80. Answer: D — Many first-episode patients respond to lower medication doses and may have better outcomes with early intervention including therapy and psychosocial support

Rationale: First-episode patients frequently respond to lower antipsychotic doses, are more sensitive to side effects, and show markedly better outcomes with coordinated specialty care combining medication, therapy, family work, and supported education or employment (Kane et al., 2016). A and C are false and dangerous. B is false; antipsychotics are indicated.

81. Answer: D — It carries serious risks including agranulocytosis requiring regular blood monitoring, but is the most effective antipsychotic for refractory cases

Rationale: Clozapine is the most effective antipsychotic for treatment-resistant schizophrenia and is the only antipsychotic with an FDA indication for reducing suicidal behavior, but it carries agranulocytosis, myocarditis, and seizure risk that call for regular absolute neutrophil count (ANC) monitoring per the prescribing information — which is why it is reserved rather than first-line. (The FDA ended the Clozapine REMS program in August 2025; ANC monitoring is still recommended by the label but is no longer a federally mandated enrollment program.) A, B, and C are all factually incorrect.

Eating Disorders

82. Answer: A — Electrolyte panels, dental evaluation, and cardiac monitoring given risks of hypokalemia, dental erosion, and arrhythmia

Rationale: Recurrent self-induced vomiting demands electrolyte panels for hypokalemia, dental evaluation for enamel erosion, and cardiac monitoring for arrhythmia — the constellation that makes purging medically dangerous. B, C, and D are either too narrow or irrelevant to the mechanism of harm.

83. Answer: C — Adolescent Anorexia Nervosa, where parents are empowered as the primary agents of refeeding

Rationale: Family-Based Treatment is first-line for adolescent Anorexia Nervosa, empowering parents as the agents of refeeding in Phase 1 before returning control to the adolescent (Lock & Le Grange, 2013). A is wrong; FBT's evidence base is adolescent. B misapplies it. D overgeneralizes a diagnosis- and age-specific treatment.

84. Answer: B — Identifying triggers for binge episodes, developing regular eating patterns, and addressing shape and weight overvaluation

Rationale: CBT-E for Binge Eating Disorder establishes regular eating patterns, identifies binge triggers, and addresses overvaluation of shape and weight — the core maintaining mechanism (Fairburn, 2008). A and D prescribe restriction, which reliably provokes further bingeing. C substitutes a weight goal for treatment of the disorder.

85. Answer: B — ARFID involves no body image disturbance or fear of weight gain; restriction is driven by sensory sensitivity, fear of aversive consequences, or lack of interest

Rationale: ARFID involves restriction driven by sensory sensitivity, fear of aversive consequences such as choking or vomiting, or low interest in eating — with no body image disturbance and no fear of weight gain, which is the defining contrast with Anorexia Nervosa. A and D are false; ARFID can cause severe malnutrition and dependence on supplementation. C is false; it occurs across the lifespan.

86. Answer: B — Bradycardia, significant electrolyte abnormalities, or BMI below 15

Rationale: Bradycardia, significant electrolyte derangement, and severe low weight are markers of medical instability requiring stabilization before outpatient psychotherapy can proceed. A sets a threshold far above the danger zone. C and D describe clinically meaningful features that do not by themselves indicate medical emergency.

87. Answer: A — Body image disturbance, a core diagnostic feature involving distorted perception of body weight and shape

Rationale: Disturbance in the way body weight or shape is experienced is a core diagnostic criterion of Anorexia

Nervosa — the perception is genuinely altered, not merely denied. B assumes strategic deception. C over-reads poor insight as psychosis. D normalizes a diagnostic feature.

88. Answer: A — Coordinating care across the treatment team while addressing the psychological factors maintaining the eating disorder

Rationale: The counselor coordinates within the multidisciplinary team while treating the psychological factors maintaining the disorder — overvaluation of shape and weight, perfectionism, affect regulation. B avoids material central to the treatment. C exceeds scope into nutritional management. D misstates the collaborative structure of eating disorder care.

Crisis, Risk, and Safety

89. Answer: D — Recognize these as warning signs of imminent suicide risk and conduct a thorough safety assessment despite the denial

Rationale: Giving away valued possessions, writing farewell letters, and putting affairs in order are classic behavioral warning signs, and behavior outweighs verbal denial — a thorough risk assessment is required regardless of what the client states. A accepts denial against contrary evidence. B documents without acting. C gathers explanations and then fails to act on the pattern.

90. Answer: C — Collaborative safety planning using the Stanley-Brown model, which is an active coping intervention superior to no-harm contracts

Rationale: The Stanley-Brown Safety Planning Intervention is an active coping plan — warning signs, internal strategies, social contacts, professionals, and means restriction — with empirical support for reducing subsequent attempts. A is not evidence-based and provides false reassurance. B is sometimes necessary but is not the default intervention. D defers the immediate clinical task.

91. Answer: B — Collaboratively identifying lethal means, discussing temporary restriction strategies, and involving support persons in safely securing or removing access

Rationale: Means restriction counseling is collaborative: identify the lethal means the client has access to, discuss temporary restriction, and involve a support person in securing or removing them. Time and access matter because suicidal crises are often short-lived. A directs rather than collaborates. C exceeds the counselor's authority. D documents without intervening.

92. Answer: B — Thank the partner for calling, take the concern seriously, attempt to reach the client, and activate the safety plan or emergency services if needed

Rationale: Receiving information does not breach confidentiality — the counselor may listen, take the concern seriously, attempt to reach the client, and activate the safety plan or emergency services. A misapplies confidentiality to incoming information. C offloads a clinical responsibility. D defers an acute risk report.

93. Answer: A — Report to Adult Protective Services based on reasonable suspicion, without needing to confirm or investigate

Rationale: Mandated reporting is triggered by reasonable suspicion; the counselor reports to Adult Protective Services and does not investigate or confirm first. B misstates the standard. C delegates a personal legal obligation. D is incorrect — the duty attaches to the suspicion, not to whether the vulnerable adult is the client.

94. Answer: B — The intent — NSSI aims to manage emotional distress without intent to die, while a suicide attempt involves intent to end life

Rationale: The distinction is intent: NSSI functions to regulate unbearable affect without intent to die, while a suicide attempt carries intent to end life. A, C, and D de-

scribe features that vary and do not reliably distinguish — which is why intent must be assessed directly rather than inferred from method or lethality.

95. Answer: B — Assess specificity, imminence, and identifiability — explore whether this is venting frustration or a credible threat with a plan

Rationale: Before any duty-to-warn action, assess specificity, imminence, and whether the target is identifiable — distinguishing venting from a credible threat with means and plan. A and D act before assessment and may breach confidentiality without justification. C fails to assess an articulated threat.

96. Answer: C — History of a previous suicide attempt

Rationale: A prior suicide attempt is the single strongest predictor of a future attempt and of death by suicide. A, B, and D are genuine risk factors with weaker predictive power — which is why attempt history must be asked about directly in every risk assessment.

Disruptive, Impulse-Control, and Conduct Disorders

97. Answer: A — Oppositional Defiant Disorder, which involves angry/irritable mood and defiant behavior without the serious violations seen in CD

Rationale: Angry and irritable mood, argumentativeness, deliberate annoyance of others, and blaming others define Oppositional Defiant Disorder. The explicit absence of aggression, theft, and property destruction rules out Conduct Disorder in B. C is not supported by the described symptoms. D requires discrete explosive outbursts grossly out of proportion to provocation.

98. Answer: B — Deviant peer reinforcement can strengthen antisocial behavior and worsen outcomes compared to individual or family-based treatment

Rationale: Aggregating antisocial youth produces deviant peer reinforcement — iatrogenic effects in which group members reinforce one another's antisocial behavior, with outcomes worse than individual or family-based treatment. A is false; they often engage readily, which is part of the problem. C overgeneralizes. D is a real concern but not the reason this composition is contraindicated.

99. Answer: C — Limited prosocial emotions — lack of remorse, shallow affect, and indifference to others' feelings — predicting poorer prognosis and treatment resistance

Rationale: The with-limited-prosocial-emotions specifier denotes lack of remorse or guilt, callousness, shallow affect, and unconcern about performance — marking a subgroup with poorer prognosis and reduced responsiveness to standard parenting interventions. A is too narrow. B and D are not clinical implications of a specifier.

100. Answer: D — Training parents to reinforce prosocial behavior, use consistent consequences, and reduce coercive interaction cycles

Rationale: Parent Management Training teaches parents to reinforce prosocial behavior, apply consistent and non-escalating consequences, and interrupt the coercive cycles in which the child's behavior is inadvertently reinforced (Kazdin, 2005). A works with the child rather than the contingencies. B describes attachment-focused work. C addresses parental mental health, which is adjunctive rather than the mechanism.

101. Answer: A — Impulsive, disproportionate to provocation, and not premeditated or committed for tangible gain

Rationale: IED outbursts must be impulsive and anger-based, grossly disproportionate to the provocation, and not premeditated or committed to achieve a tangible objective. B is the explicit exclusion. C is not required. D would indicate a substance-related diagnosis instead.

102. Answer: B — Childhood onset is associated with higher rates of callous-unemotional traits and progression to Antisocial Personality Disorder in adulthood

Rationale: Childhood-onset Conduct Disorder carries higher rates of callous-unemotional traits, greater aggression, and substantially higher progression to Antisocial Personality Disorder than adolescent-onset. A names a correlate rather than the mechanism. C is false. D is incorrect — the criteria are the same; the trajectory differs.

103. Answer: B — In kleptomania, the stealing is impulsive with rising tension beforehand and relief afterward, not motivated by the value of items or by antisocial intent

Rationale: Kleptomania is defined by failure to resist impulses to steal objects not needed for use or value, with rising tension before and relief or gratification after — not by acquisitive or antisocial motive. A is false; the items are typically of little value. C is false. D reverses the two: it is kleptomania, not Conduct Disorder theft, that is defined by impulsive stealing, and Conduct Disorder theft is more often planned or instrumental.

Somatic Symptom and Related Disorders

104. Answer: C — Validate that the pain is real and distressing, then assess the thoughts, feelings, and behaviors surrounding the symptoms

Rationale: Validate that the pain is real and distressing — it is — and then assess the thoughts, feelings, and behaviors surrounding it. Without that validation the client experiences the referral as dismissal and disengages. A and D invalidate and diagnose before assessment. B extends the cycle of testing that maintains the disorder.

105. Answer: B — Neurological symptoms such as paralysis, tremor, or blindness that are incompatible with recognized neurological conditions

Rationale: Functional Neurological Symptom Disorder presents with motor or sensory symptoms — weakness, paralysis, non-epileptic seizures, tremor, blindness — that show internal inconsistency and incompatibility with recognized neurological disease. A describes Somatic Symptom Disorder with pain. C describes Illness Anxiety Disorder. D describes factitious disorder or malingering; conversion symptoms are not intentionally produced.

106. Answer: D — In Factitious Disorder, symptoms are intentionally produced for the purpose of assuming the sick role, without external incentive

Rationale: In Factitious Disorder symptoms are deliberately produced or falsified in order to assume the sick role, with no external incentive. A is misleading — symptoms may be induced and thus physically real, but they are intentionally caused. B compares severity. C is false; both impair.

107. Answer: A — Reducing excessive health-related thoughts, feelings, and behaviors while improving functioning despite persistent symptoms

Rationale: CBT for Somatic Symptom Disorder targets the excessive thoughts, feelings, and behaviors about the symptoms — catastrophizing, body checking, reassurance seeking, activity avoidance — and restores functioning even while symptoms persist. B sets a goal treatment cannot deliver. C is adversarial and false. D describes an outdated cathartic model.

108. Answer: C — A reassurance-seeking compulsion that maintains the anxiety cycle and should be gradually reduced through behavioral intervention

Rationale: Repeated blood pressure checking is a reassurance-seeking compulsion: it relieves anxiety briefly and strengthens it over time, so it is reduced gradually through response prevention. A and D normalize a maintaining behavior. B is close but imprecise — within Illness Anxiety Disorder this is the checking behavior of that diagnosis rather than a separate OCD.

109. Answer: B — Motivation — malingering involves external incentive (avoiding work, obtaining drugs, legal benefit); Factitious involves internal motivation (sick role)

Rationale: The distinction is motivation: malingering is driven by external incentive — disability payment, avoiding work or prosecution, obtaining medication — while Factitious Disorder is driven by the internal goal of occupying the sick role. A and C are false. D is false; both involve intentional production.

110. Answer: B — Reframe the referral as an opportunity to address the distress and functional impact of symptoms without invalidating the client's experience

Rationale: Reframe the referral so the client is not invalidated: whatever the etiology, the distress and functional impact are real and treatable. A and D adopt language that ruptures the alliance before the work begins. C refuses a client who needs care because the referring language was poor.

Dissociative Disorders

111. Answer: D — Remain calm, orient to the therapeutic relationship, communicate respect for whichever part is present, and maintain safety

Rationale: Remain calm, orient to the therapeutic frame, address whichever part is present with respect, and maintain safety. Switching is not an emergency, and how the counselor responds shapes the system's trust. A and B treat the switch as a problem to be terminated. C ignores a clinically significant event.

112. Answer: B — Apparently purposeful travel or bewildered wandering with amnesia for identity or other autobiographical information

Rationale: Dissociative fugue involves apparently purposeful travel or bewildered wandering with amnesia for

identity or other autobiographical information, coded in DSM-5-TR as a specifier of Dissociative Amnesia. A describes amnesia without the fugue feature. C and D do not correspond to the diagnosis.

113. Answer: B — Phase 1: Safety and stabilization; Phase 2: Trauma processing; Phase 3: Integration and rehabilitation

Rationale: ISSTD's phasic model is Phase 1 safety and stabilization, Phase 2 trauma processing, Phase 3 integration and rehabilitation — and Phase 1 is never skipped (International Society for the Study of Trauma and Dissociation [ISSTD], 2011). A inverts the sequence. C is the classic harm-causing error. D withholds the primary treatment.

114. Answer: A — In Panic Disorder, depersonalization occurs only during panic attacks and resolves between them; in DPDR, it persists independently

Rationale: In Panic Disorder depersonalization arises only within attacks and resolves between them; in Depersonalization/Derealization Disorder the experience is persistent or recurrent and independent of panic. B is false; the phenomenology overlaps. C compares severity rather than pattern. D is false.

115. Answer: D — Work with the system to ensure the child part feels safe and contained before any trauma processing, prioritizing stabilization

Rationale: Work with the system to ensure the part holding traumatic material feels safe and contained, keeping the focus on stabilization before any processing. A forces access and reliably destabilizes. B inverts the treatment sequence — integration follows processing. C ignores the structure through which the client organizes experience.

116. Answer: B — Sensory engagement strategies such as naming objects in the room, holding ice, or describing physical sensations to reconnect with the present moment

Rationale: Grounding uses sensory and orienting strategies — naming objects in the room, holding something cold, describing physical sensations, feeling the feet on the floor — to re-establish present-moment contact. A moves to analysis while the client is not present. C, deep relaxation, can deepen dissociation. D permanently accommodates avoidance.

117. Answer: A — Depersonalization/Derealization Disorder, where the subjective experience is disturbing but reality testing remains intact

Rationale: Depersonalization/Derealization Disorder requires intact reality testing — the client knows the experience of unreality is a subjective state, which is exactly what distinguishes it from psychosis. B and C describe a loss of reality testing not present here. D requires distinct identity states.

Sleep-Wake Disorders

118. Answer: A — Stimulus control — removing screens from the bedroom and getting out of bed when unable to sleep within 20 minutes

Rationale: Stimulus control is the right first CBT-I component for this presentation: re-associate the bed with sleep by removing screens, reserving the bed for sleep and sex, and leaving the bed when sleep does not come within about twenty minutes (Edinger & Carney, 2015). B is an adjunct with weaker effects. C is a later component and the figure given is arbitrary rather than derived from a sleep diary. D addresses beliefs before the behavioral conditioning that maintains the problem.

119. Answer: D — Temporarily limiting time in bed to actual sleep time, building sleep drive and consolidating fragmented sleep

Rationale: Sleep restriction temporarily limits time in bed to actual sleep time, raising homeostatic sleep drive and consolidating fragmented sleep; time in bed is then titrated upward as efficiency improves. A misstates it as

permanent. B describes sleep hygiene. C is false — sleep need does not change.

120. Answer: B — Arrange urgent referral for a sleep study to evaluate for obstructive sleep apnea, given the safety risk of driving

Rationale: Loud snoring with witnessed apneas plus irresistible daytime sleepiness in a commercial driver is presumptive obstructive sleep apnea and a public safety risk — urgent referral for polysomnography takes precedence. A treats the wrong disorder. C is ineffective here. D is a reasonable differential but does not account for the snoring and breathing pauses.

121. Answer: C — Writing down the nightmare, changing the storyline to a non-threatening version, and rehearsing the new version before sleep

Rationale: Imagery Rehearsal Therapy has the client write the nightmare, rewrite the narrative to a non-threatening ending, and mentally rehearse the revised version daily before sleep (Krakow & Zadra, 2006). A applies dream interpretation, which is not IRT. B describes lucid dreaming therapy. D describes pharmacological management.

122. Answer: B — Instructing the client to try to stay awake, which reduces performance anxiety about sleeping

Rationale: Paradoxical intention instructs the client to try to remain awake, which removes the performance anxiety that is itself preventing sleep (Frankl, 1946/2006). A restates the problem. C describes chronotherapy. D describes stimulus control.

123. Answer: C — Treated independently with CBT-I, since insomnia can become a conditioned, self-maintaining problem

Rationale: Insomnia becomes a conditioned, self-maintaining disorder and persists after mood symptoms remit; treated independently with CBT-I it also reduces depres-

sion relapse risk. A ignores a residual symptom that predicts recurrence. B and D subordinate insomnia to the mood disorder when it now warrants its own treatment.

124. Answer: D — Hypersomnolence Disorder, characterized by excessive sleep quantity with non-restorative sleep despite adequate duration

Rationale: Hypersomnolence Disorder involves excessive sleep quantity with unrefreshing sleep, sleep inertia on waking, and daytime sleepiness despite adequate or prolonged sleep duration. A is the opposite presentation. B would involve cataplexy, sleep paralysis, or hypnagogic hallucinations with sleep attacks. C would show a misaligned but otherwise normal sleep period.

Sexual Dysfunctions and Gender Dysphoria

125. Answer: D — Remove performance pressure by focusing on sensory experience and gradually reintroducing sexual activity without performance demands

Rationale: Sensate focus removes performance demand by prohibiting intercourse initially and directing attention to sensory experience, then graduating activity as anxiety decreases (Masters & Johnson, 1970). A inverts the goal — performance focus is the problem. B and C name other techniques and targets.

126. Answer: C — Rule out medical causes, medication side effects, substance use, and relationship factors that may fully account for the symptoms

Rationale: Sexual dysfunction cannot be diagnosed until medical conditions, medication effects (SSRIs prominently), substance use, and relationship factors that would fully account for the symptoms are ruled out. A is necessary but not sufficient. B is often appropriate but is not a universal prerequisite. D is false — the distress criterion attaches to the individual.

127. Answer: D — Support the client's self-determined goals, facilitate medical referral if surgical consultation is desired, and continue addressing residual dysphoria

Rationale: Support the client's self-determined goals, facilitate referral for surgical consultation if that is what the client wants, and continue addressing residual dysphoria. A and B counsel delay and acceptance the client has not requested. C is false — social transition is not the endpoint of care.

128. Answer: C — Delayed orgasm and decreased sexual desire, which should be discussed during informed consent for medication

Rationale: SSRIs commonly cause delayed orgasm or anorgasmia and reduced desire, affecting a substantial minority of patients; this belongs in informed consent because undisclosed sexual side effects are a leading cause of covert discontinuation. A is the opposite. B is too narrow and gendered. D is not a characteristic SSRI effect.

129. Answer: A — Address both partners' experiences, psychoeducate about desire discrepancy as relational rather than pathological, and assess for underlying causes

Rationale: Desire discrepancy is a relational phenomenon rather than a defect in the lower-desire partner, so both experiences are addressed, psychoeducation reframes the difference, and underlying contributors are assessed. B locates the problem in one partner. C applies a technique before assessment. D refers without the assessment that would make the referral useful.

130. Answer: B — Childhood presentation may or may not persist into adolescence, requiring supportive monitoring rather than immediate medical intervention

Rationale: Childhood gender dysphoria may or may not persist into adolescence, which is why the standard of

care is supportive, affirming monitoring rather than immediate medical intervention, with decisions revisited as puberty approaches. A denies a documented experience. C is false. D is false — the criteria are developmentally distinct.

131. Answer: D — Provide assessment, support, and referral within a collaborative care model that respects client autonomy in medical decision-making

Rationale: WPATH describes a collaborative, informed-consent-oriented model in which mental health professionals provide assessment, support, and referral while respecting client autonomy in medical decisions (Coleman et al., 2022). A describes the gatekeeping model WPATH has moved away from. B imposes arbitrary waiting periods. C is false — social transition requires no diagnosis.

Ethical and Legal Decision Points

132. Answer: C — Decline without a signed release from the client, disclosing only the minimum necessary if a valid release exists

Rationale: Without a signed release nothing is disclosed — including whether the person is a client, since confirming attendance is itself protected information. With a valid release, disclosure is limited to the minimum necessary. A, B, and D all disclose without authorization.

133. Answer: A — Seek supervision or consultation, and if the issue remains outside competence, refer to a qualified provider while ensuring continuity of care

Rationale: Seek supervision or consultation first; if the issue remains outside competence, refer while ensuring continuity of care rather than abandoning the client. B practices beyond competence. C terminates abruptly and risks abandonment. D is false — competence is always bounded.

134. Answer: D — Explore the meaning of the gift therapeutically, consider the client's cultural context, and make a decision that prioritizes the client's welfare

Rationale: Gift-giving is evaluated case by case: the meaning of the gift, its monetary and symbolic value, cultural context, the timing in treatment, and the effect on the relationship — with client welfare governing. A and B are absolute rules in opposite directions. C substitutes an arbitrary threshold for clinical and cultural judgment.

135. Answer: C — Addressing the concern directly with the colleague, and if unresolved, reporting to the licensing board to protect the colleague's clients

Rationale: Impairment that endangers clients requires action: address it directly with the colleague where safe and feasible, and report to the licensing board if it is not resolved. A ignores a duty owed to the colleague's clients. B breaches confidentiality and exceeds the counselor's role. D waits for the harm the obligation exists to prevent.

136. Answer: B — The counselor's personal therapy history

Rationale: Informed consent covers the nature and goals of treatment, risks and benefits, limits of confidentiality, fees, the right to refuse or withdraw, and the counselor's credentials — but not the counselor's personal therapy history, which is private. A, C, and D are all required elements, which makes B the exception.

137. Answer: B — Assess the relationship for coercion or exploitation; consensual peer sexual activity between minors of similar age may not require reporting depending on state law

Rationale: Assess for coercion, exploitation, and age differential first; consensual sexual activity between similarly aged minors may not be reportable, and reporting thresholds vary by state. A reports before assessing. C

breaches adolescent confidentiality without a basis. D forecloses an assessment that safety requires.

138. Answer: D — Maintain an affirming stance, seek consultation or training if personal reactions are interfering, and refer only on competence grounds — never solely because of a values conflict

Rationale: Maintain an affirming stance, seek consultation when personal reactions interfere, and refer only on competence (skill) grounds — never solely because of a values conflict, which the ACA Code of Ethics prohibits (ACA, 2014, Standard A.11.b). A and C impose the counselor's values. B avoids material the client brought.

139. Answer: D — Identify the problem and determine whether it is an ethical, legal, clinical, or professional issue

Rationale: The Forester-Miller and Davis model begins by identifying the problem and determining whether it is ethical, legal, clinical, or professional in nature — because the category determines which framework applies (Forester-Miller & Davis, 2016). B, A, and C belong to steps two, three, and four respectively.

140. Answer: D — Consult with an attorney, notify the client, and assert privilege — a subpoena alone does not compel disclosure without a court order

Rationale: A subpoena is a request from an attorney, not a judicial order. Consult an attorney, notify the client, and assert privilege on the client's behalf; disclosure follows only a court order or the client's authorization (*Jaffee v. Redmond*, 1996). A discloses without authority. B invites sanction — the subpoena requires a response, just not disclosure. C discloses a summary that is still protected.

141. Answer: D — Monitor supervisee competence, ensure client welfare, maintain appropriate boundaries, and address supervisee impairment

Rationale: Supervisors carry dual responsibility: monitoring supervisee competence and safeguarding the welfare

of clients the supervisee serves, while maintaining appropriate boundaries and addressing impairment. A omits the developmental function. B omits client welfare and supervisee wellbeing. C abdicates the oversight that defines supervision.

142. Answer: A — HIPAA-compliant technology, informed consent addressing teletherapy-specific risks, emergency planning for the client's location, and awareness of jurisdictional licensing requirements

Rationale: Teletherapy requires HIPAA-compliant platforms, informed consent addressing the modality's specific risks and limits, an emergency plan keyed to the client's physical location, and licensure in the jurisdiction where the client is located at the time of service. B, C, and D are each incomplete or incorrect — recording is not a documentation requirement, and teletherapy is a legitimate modality rather than a last resort.

Additional Cross-Domain Questions

143. Answer: A — Conduct a comprehensive assessment addressing all three differentials before committing to a diagnosis, since treatment differs significantly

Rationale: MDD, GAD, and PTSD share insomnia, concentration difficulty, and irritability but call for different treatments, so a comprehensive differential assessment precedes commitment to a diagnosis. B risks treating the wrong condition. C avoids a diagnosis that directs treatment. D outsources clinical judgment to testing that is usually unnecessary.

144. Answer: C — Use the DSM-5-TR Cultural Formulation Interview to understand how culture shapes symptom expression, help-seeking, and treatment expectations

Rationale: The DSM-5-TR Cultural Formulation Interview systematically explores cultural identity, the client's ex-

planatory model of the problem, stressors and supports, and expectations of care. A applies criteria without cultural context and risks misdiagnosis. B withholds diagnosis and the access it provides. D assumes demographic matching substitutes for cultural competence.

145. Answer: D — Seek supervision or consultation while continuing to provide competent care within one's current scope

Rationale: Seek supervision or consultation while continuing to provide competent care — consultation is the mechanism by which competence extends, and feeling overwhelmed is not itself a competence boundary. A transfers prematurely and disrupts continuity. B substitutes self-study for oversight. C burdens the client with the counselor's anxiety.

146. Answer: C — Best available research evidence, clinical expertise, and client values and preferences

Rationale: Evidence-based practice integrates the best available research, clinical expertise, and client values and preferences — all three (American Psychological Association Presidential Task Force on Evidence-Based Practice, 2006). A elevates one leg. B abandons the research base. D applies manuals without the clinical judgment and client input the model requires.

147. Answer: D — Assess for a bipolar spectrum disorder, coordinate with the prescriber about medication implications, and adjust the treatment plan

Rationale: Emerging hypomanic symptoms during treatment for depression require assessment for a bipolar spectrum disorder, coordination with the prescriber about antidepressant risk, and adjustment of the plan. A ignores a diagnostic shift with real treatment consequences. B waits for an episode that is dangerous to allow. C diagnoses before assessment.

148. Answer: B — The quality of the therapeutic relationship is one of the strongest predictors of treatment outcome across all therapeutic modalities

Rationale: Alliance quality is among the most consistent predictors of outcome across modalities, with effects that hold after controlling for technique (Wampold & Imel, 2015). A inverts the finding. C is false — the effect appears across orientations, including CBT. D overstates it; alliance does not substitute for effective technique.

149. Answer: C — Explore the rupture directly, validate the client's experience, and use the repair process as a therapeutic intervention

Rationale: Explore the rupture directly, validate the client's experience, and treat the repair as the intervention — successfully repaired ruptures predict better outcomes than therapies with no rupture at all. A defends and deepens the breach. B refers rather than repairs. D changes the treatment while leaving the relational event unaddressed.

150. Answer: C — Self-awareness of one's own cultural assumptions, knowledge of diverse worldviews, and development of culturally appropriate intervention strategies

Rationale: Multicultural competence is a continuing developmental process built on self-awareness of one's own cultural assumptions and biases, knowledge of diverse worldviews, and culturally appropriate intervention skills. A is impossible and treats cultures as static checklists. B is neither feasible nor necessary. D avoids the conversations competence requires.

151. Answer: B — Tell the client at the next session what the tool does and its risks, obtain written consent, and offer to continue without it if the client declines

Rationale: Recording sessions and generating documentation with an AI tool falls under informed consent and Sec-

tion H of the ACA Code: clients must know when and how technology is used and may decline (ACA, 2014). HIPAA compliance (A) addresses security, not consent. The Code does not prohibit AI documentation (C). A website policy (D) is not consent.

152. Answer: B — Explore how she uses it and what helps, name its limits — including that it cannot provide crisis response — and build human contacts and 988 into her safety plan

Rationale: The disclosure contains passive suicidal ideation managed by a chatbot. The rewarded response assesses, respects what the client finds helpful, and corrects the one dangerous gap — a chatbot cannot respond to a crisis. Forbidding the tool (A) damages the alliance and ignores the risk disclosure; reviewing transcripts (C) delays the risk response; D ignores it.

153. Answer: C — Review and correct the note before signing it, removing the unmade diagnosis — the counselor is responsible for the note's content and for any diagnosis

Rationale: The counselor, not the tool, is responsible for every clinical decision and every note. AI output cannot be the basis for a diagnosis (A, D). Abandoning the tool entirely (B) is not required; reviewing and correcting its output is.

End of Supplemental Answer Key

How the 2027 NCMHCE Works — and How to Master It

What Changed in 2027

NBCC's examination specifications for exams given on or after July 1, 2027, change the NCMHCE in six ways (NBCC, 2026):

- **10 cases instead of 11.** Each case still runs in three sections — an intake summary and two counseling sessions — with 9–15 four-option questions per case.
- **Field-test items, not a field-test case.** The 30–50 unscored field-test items are scattered at random through the exam and look like every other item. You cannot tell which they are, so treat every item as scored.
- **A scaled score.** Scores range from 100 to 500, and the passing point is 360. A scaled score is not a percentage.
- **Six new domains.** Indirect Client Care is new, at 12%. Legal & Ethical Compliance rises to 20%, tied with Provision of Counseling Interventions as the largest domain.
- **A task-based outline.** The six domains hold 101 work tasks from a national work analysis, each paired with the knowledge and skills it requires.
- **ICD-11 named alongside DSM-5-TR.** The assessment knowledge list names diagnostic criteria from both, and one task asks you to select the diagnosis from the most recent DSM or ICD code.

One thing the specifications do not settle: whether a case opens with a diagnosis. Be ready for both. When a case gives a working diagnosis, confirm or revise it; when it

does not, select one from the evidence (task 2B.12). Then know what to do next.

The 10-Case Structure

The 2027 NCMHCE presents **10 case studies**. Each case is presented in three sections: an initial intake summary and two counseling sessions. Each section starts with a narrative, followed by multiple-choice questions. Each case has **9–15 questions** with four options each. The exam totals **130–150 questions**, of which **30–50 are unscored field-test items** randomly distributed throughout (NBCC, 2026).

Each case: - Opens with an intake summary, which may or may not state a working diagnosis - Moves across several domains — intake and assessment, treatment planning and continuity of care, interventions, indirect client care, and legal and ethical compliance - Advances section by section — intake, then two sessions — with new information in each

Content domain weights (NBCC, 2026):

Domain	Weight	What it asks you to do
1 Professional Development & Counselor Self-Awareness	15%	Seek consultation and supervision, judge your competence and suitability for a client, and manage countertransference, burnout, and boundaries
2 Intake & Assessment	18%	Obtain informed consent, gather self-report and collateral information, assess risk, trauma, and substance use, and select the diagnosis

Domain	Weight	What it asks you to do
3 Treatment Planning & Continuity of Care	15%	Build and revise a collaborative plan with SMART goals, then refer, plan discharge and aftercare, prevent relapse, and set termination criteria
4 Provision of Counseling Interventions	20%	Deliver empirically supported interventions, crisis intervention, and group and family work, and repair ruptures
5 Indirect Client Care	12%	Coordinate with supports and providers, document the medication effects a client reports, keep referral lists current, secure interpreters, and write notes
6 Legal & Ethical Compliance	20%	Follow ethics codes and law, telemental health rules, records and billing requirements, court obligations, and the practice's emergency plan

Section 7 of the specifications adds content guidance — common presenting concerns, theoretical frameworks, and tools and assessments — that candidates "may benefit from reviewing" (see Chapter 27 and the Assessment Tools Compendium).

The two largest domains are **Provision of Counseling Interventions** and **Legal & Ethical Compliance**, at 20% each. The exam expects you to know what to *do*, and what you are *required* to do — not just what to call it.

Testing before July 1, 2027? You will take the current NCMHCE: 11 cases built on NBCC's 2023 content outline, with 100 scored items. The clinical content in this book applies to both versions. The format and domain map in this edition are the 2027 ones. Check nbcc.org to confirm which version you will take.

How to Read a Case Vignette

When a case opens, extract five critical data points before reading any answer choices:

1. The working diagnosis — If the case states one, confirm or revise it. Read the case through it as a starting hypothesis, and watch for details that support it, change it, or add a co-occurring diagnosis.

2. The presenting context — How did this client come to counseling? Self-referred? Court-ordered? Crisis? School referral? Context shapes what the counselor's first move should be.

3. The most pressing clinical concern — Is there an acute risk factor (suicidality, danger to others, immediate crisis)? If yes, that must be addressed before anything else, regardless of the working diagnosis.

4. The stage of treatment — Is this the initial intake session, an established ongoing case, a crisis moment, or a termination discussion? The correct answer changes with stage.

5. The counselor's scope — Is the question testing what you do within your scope? Or whether you refer, consult, or coordinate? Many NCMHCE questions are testing scope as much as clinical knowledge.

Decision Patterns the Exam Rewards

These are the patterns behind the Next-Move Method. When two options both look clinically sound, one of these rules usually decides which move comes next.

The First-Things-First Rule

The NCMHCE never rewards jumping to treatment before assessment is complete, jumping to a specific technique before a therapeutic alliance is established, or skipping a safety evaluation when risk is present.

When a case presents multiple clinically valid options, ask: *What comes first in real clinical sequence?*

- Safety assessment → then diagnosis → then treatment planning → then specific interventions
- Alliance building → then psychoeducation → then evidence-based techniques
- Assessment → then goal-setting → then intervention → then evaluation

The Least Restrictive Principle

When level of care is in question, the NCMHCE rewards selecting the least restrictive level of care that addresses safety and clinical need. The correct answer is not "always hospitalize" — it is the level appropriate to the clinical picture.

The Refer Principle (for Out-of-Scope Items)

When a question touches on something outside the counselor's scope (medication, medical evaluation, neuropsychological testing, legal matters), the correct answer almost always involves a referral, consultation, or coordination — not acting independently within that out-of-scope domain.

The Collaboration Principle

The NCMHCE rewards integrated, collaborative care. Working with other providers — physicians, psychiatrists,

schools, families, social services — is almost always rewarded over working in isolation.

What to Do With Each Answer Choice

When evaluating the four answer options:

Eliminate immediately: - Any option that involves a scope violation (counselor prescribing, advising on medication doses, diagnosing a medical condition) - Any option that skips safety assessment when risk is apparent - Any option involving unethical practice (dual relationships without appropriate management, breaching confidentiality without legal basis, dismissing the client's cultural framework) - Any option that imposes a Western or dominant-culture framework when cultural factors are present

Be suspicious of: - The most aggressive option (hospitalize, terminate, report) — these are often distractors unless the specific threshold is clearly met - The most passive option (wait and see, reassure, continue without change) — these are often wrong when clinical action is needed - The option that sounds most clinically sophisticated but skips a foundational step

Favor: - Options that reflect the next logical clinical step in a defensible sequence - Options that integrate client values, culture, and context with evidence-based practice - Options that demonstrate collaborative, systemic thinking - Options that are specific enough to be actionable but not so specific as to skip earlier steps

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Managing the Sectioned Format

NCMHCE cases unfold in order — intake, then two sessions — and a later section can add information that changes the clinical picture. The 2027 specifications say you can review questions before you submit each half of the exam, and that once a half is submitted and the break

begins, you cannot return to it (NBCC, 2026). They do not say whether earlier sections within a case lock as you move forward. Confirm the navigation rules in the current candidate handbook, and until you have, work each section as if you cannot go back. Strategies:

1. Do not anchor too rigidly on a single path. If a later section presents an unexpected turn (the crisis you managed is now in follow-up, or a new piece of information changes the picture), respond to the new clinical reality — not the previous section.

2. Treat each item as its own standalone clinical judgment. Even if you suspect an earlier answer was wrong, answer each later item from the information now in front of you, not from your earlier choice. Assess each item on its own clinical merits.

3. A section about crisis management rewards a different posture than a section about ongoing care. When a case shifts from crisis to stabilization, so should your clinical posture.

Common NCMHCE Case Types and What Each Tests

Case Type	What It Primarily Tests
Suicidality/self-harm case	Risk assessment protocol; safety planning; level-of-care decision; documentation
Child/adolescent case	Developmental assessment; parental involvement; mandatory reporting; school coordination
Substance use case	SBIRT; motivational interviewing; appropriate referral; dual diagnosis
Trauma case	Trauma-informed assessment; stabilization before processing; appropriate EBP (PE, CPT, EMDR)

Case Type	What It Primarily Tests
Neurodevelopmental case	Multi-informant assessment; school coordination; diagnostic criteria; referral
Ethical dilemma case	Ethical decision-making model; confidentiality; dual relationships; mandatory reporting thresholds
Cultural difference case	Cultural formulation; explanatory models; culturally responsive practice
Multicultural case	Affirming identity; avoiding pathologizing; understanding cultural context
Medication/medical coordination case	Scope; when to refer; what to tell the prescriber; psychoeducation
Group counseling case	Group stages; leader role; confidentiality in groups; group dynamics
Termination/referral case	Ethical termination criteria; transfer of care; documentation
Indirect client care case	Documentation; care coordination with consent; recording the medication effects the client reports; interpreters; referral resources
Practice and compliance case	Telemental health rules; billing supported by documentation; record security; court obligations; emergency preparedness

Time Management

You have 225 minutes (3 hours 45 minutes) to complete the items for 10 cases — about 22 minutes per case — with the option of a 15-minute break halfway through (NBCC, 2026). Check the current candidate handbook for

the rest of the session: check-in, any tutorial, and whether the clock stops for unscheduled breaks. Strategies:

- **Do not spend more than 60–90 seconds per item.** If you are genuinely unsure, apply the elimination strategies above and commit to the best remaining option.
- **Review before you submit each half.** The specifications let you review questions before submitting each half of the exam; once a half is submitted and the break begins, you cannot return to it (NBCC, 2026). Confirm in the current candidate handbook whether sections within a case also lock, and review each section before you continue until you know.
- **Trust your preparation.** The NCMHCE tests clinical judgment, and well-prepared counselors generally find that their first instinct aligns with the correct answer more often than extended second-guessing does.
- **Read each question's final line first.** Knowing what the question is asking helps you pick out which details in the narrative are clinically relevant.

A Final Word on the 2027 Format

The 2027 exam is built from what counselors actually do at work. In practice, clients arrive with records, referrals, and existing diagnoses, and much of the job happens around the session — notes, coordination, referrals, compliance. Your job is not to identify every diagnosis from scratch. It is to confirm, revise, or select the diagnosis and know what to do with the information you have.

The counselor the NCMHCE rewards is: - Clinically grounded in evidence-based practice - Ethically principled, especially under realistic clinical pressure - Culturally humble and responsive - Collaborative, with a clear sense of scope - Systematic — assesses before treating, stabilizes before processing, and considers the whole person in context

If that describes the counselor you've become through your training, you are prepared.

Appendix A — Multicultural Counseling Competencies

Overview

The NCMHCE tests multicultural competency in every case simulation — not as a separate topic, but as a lens applied to intake, diagnosis, treatment planning, and ethics. The ACA and NBCC both require cultural competency as a foundational professional standard. This appendix provides the frameworks, models, and clinical applications the exam tests.

The Multicultural Counseling Competency Framework

The tripartite framework developed by Sue et al. (1992) and updated in the Multicultural and Social Justice Counseling Competencies (Ratts et al., 2016) organizes competency into three domains (Sue et al., 2022):

1. Awareness: The counselor's self-awareness of their own cultural background, biases, assumptions, and privilege. A counselor who has not examined their own cultural lens cannot work effectively across cultural difference. Self-of-the-therapist work — awareness of one's own racial, ethnic, gender, class, and ability-status assumptions — is foundational.

2. Knowledge: Understanding the worldviews, history, cultural practices, and social contexts of diverse client populations. This includes knowledge of the effects of racism, discrimination, colonization, immigration, and systemic oppression on mental health, help-seeking behaviors, and therapeutic alliance.

3. Skills: The ability to use culturally appropriate assessment, conceptualization, and intervention strategies. This includes adapting evidence-based practices to be culturally responsive, using culturally appropriate measures, and advocating for systemic change (American Psychological Association Presidential Task Force on Evidence-Based Practice, 2006).

The MSJCC adds: Advocacy — working to remove systemic barriers that affect clients' mental health and access to care. The culturally competent counselor is not only sensitive within the session; they advocate for equity in services, systems, and policy.

Racial/Ethnic Identity Development Models

Cross's Black Racial Identity Development Model (Nigrescence; Cross, 1991)

Stage 1 — Pre-Encounter: Individual has absorbed dominant-culture views about race; may devalue Black identity or have internalized racism. Racial identity is not salient.

Stage 2 — Encounter: A significant experience (often discriminatory or consciousness-raising) shatters the Pre-Encounter worldview. Beginning of racial identity awareness.

Stage 3 — Immersion-Emersion: Intense immersion in Black culture, identity, and community; rejection of anything perceived as "White." May be marked by anger, distrust of counselors from different racial backgrounds.

Stage 4 — Internalization: Secure, positive Black identity. Openness to working with counselors across racial backgrounds while maintaining cultural pride.

Stage 5 — Internalization-Commitment: Sustained engagement in causes that advance Black communities; active commitment to racial justice.

NCMHCE application: A Black client in Immersion-Emersion may be skeptical of or hostile toward a White counselor — this is an understandable developmental response, not a sign of pathology. The counselor who responds defensively misses this. The counselor who understands the model can engage with empathy and transparency.

Helms's White Racial Identity Development Model

Abandonment of racism: Moving through Stages 1 (Contact — unawareness of racial privilege), 2 (Disintegration — discomfort as privilege becomes visible), 3 (Reintegration — retreat to in-group racism to reduce discomfort), and into the following (Helms, 1990):

Evolution of a nonracist identity: Stage 4 (Pseudo-Independence — intellectual awareness of racism), 5 (Immersion-Emersion — deep exploration of White identity and privilege), 6 (Autonomy — internalized, nonracist identity with active antiracism commitment).

NCMHCE application: White counselors must examine their own racial identity development to avoid colluding with racism or minimizing clients' experiences of discrimination.

Cass's Homosexual Identity Formation Model

Applies to gay, lesbian, and bisexual identity development:

Stage 1 — Identity Confusion: "Am I gay?" Confusion and dissonance about sexual orientation.

Stage 2 — Identity Comparison: Accepts the possibility of being gay; experiences isolation and grief over lost heterosexual assumptions.

Stage 3 — Identity Tolerance: Seeks connection with other LGBTQ+ people; tolerance rather than acceptance of gay identity.

Stage 4 — Identity Acceptance: Positive gay identity; selective disclosure.

Stage 5 — Identity Pride: Strong LGBTQ+ identity; possible activist phase; may view heterosexual society with suspicion.

Stage 6 — Identity Synthesis: Integrated, secure LGBTQ+ identity; sexual orientation is part of but not all of identity.

NCMHCE application: A client in Identity Confusion may present with anxiety or depression, with sexual orientation as an unspoken underlying theme. The counselor creates an affirming space without assuming the client's trajectory or imposing a particular identity outcome.

The Cultural Formulation Interview (CFI)

The DSM-5-TR Cultural Formulation Interview is a 16-item semi-structured interview assessing how culture shapes the client's experience of their presenting problem, their explanatory model, and help-seeking behaviors (APA, 2022). It is organized around four domains:

1. Cultural Definition of the Problem: How does the client describe their presenting concern? What do they call it? How do they understand what is happening to them?

2. Cultural Perceptions of Cause, Context, and Support: What does the client believe is causing the problem? What role does culture, family, community, spirituality, or social context play? Who does the client turn to for help?

3. Cultural Factors Affecting Self-Coping and Past Help Seeking: What has the client tried before? What has helped? What are the cultural barriers to seeking mental health care?

4. Cultural Factors Affecting Current Help Seeking: What does the client expect from treatment? Are there concerns about how their cultural background will be received? What would make treatment feel culturally safe?

NCMHCE application: The CFI guides a culturally humble intake. The counselor does not assume a shared framework — they explore the client's explanatory model. A Haitian client who describes their depression in terms of spiritual affliction is not "lacking insight" — they are communicating within their cultural framework, and the counselor who dismisses this commits a clinical and cultural error.

Explanatory Models and Cultural Idioms of Distress

Explanatory model (Kleinman, 1980): Every individual and cultural group has a framework for understanding illness — its causes, mechanisms, course, and appropriate treatments. When the clinician's explanatory model and the client's differ significantly, treatment engagement suffers.

Cultural idioms of distress: Culture-specific ways of expressing psychological suffering that may not map onto DSM categories. Examples include: - *Nervios* (Latin American): A syndrome of anxiety, emotional distress, and somatic complaints occurring in response to stressful life events - *Hwa-byung* (Korean): A "fire illness" involving suppressed anger expressed as chest tightness, heat sensations, and palpitations - *Kufungisisa* (Zimbabwe): "Thinking too much" — excess cognition about problems leading to distress and bodily symptoms - *Ataque de nervios* (Puerto Rico/Latin America): A culturally recognized acute distress response, often following trauma or loss, involving screaming, shaking, dissociation, and loss of consciousness

NCMHCE application: When a client's presentation doesn't fit neatly into a DSM category, consider cultural idioms of distress as an alternative framework. The DSM-5-TR includes a glossary of cultural concepts of distress for exactly this purpose.

Intersectionality

Concept (Crenshaw, 1989): Multiple social identities (race, gender, class, sexual orientation, disability status, immigration status) do not operate independently — they intersect to produce qualitatively different experiences of privilege, oppression, and marginalization.

Clinical application: A Black woman's experience of workplace microaggressions cannot be fully understood by applying racial identity theory alone, or gender theory alone — the intersection creates a specific experience neither framework captures individually (Sue et al., 2007). Similarly, a queer Latinx immigrant client navigates simultaneous race, ethnicity, sexual orientation, and citizenship-status stressors that intersect.

NCMHCE application: Assess all dimensions of a client's identity and experience, and do not reduce their presenting concerns to a single axis of difference. The counselor who treats a Latino male client as representative of a single cultural monolith has missed intersectionality (Crenshaw, 1989).

Working with Specific Populations

African American Clients

- Historical and ongoing experience of racism, discrimination, and medical/psychological research exploitation (Tuskegee legacy) — justified mistrust of mental health systems
- Stigma around mental health help-seeking within some communities — contextualize, normalize
- Systemic factors (poverty, incarceration, housing instability) are clinical data, not "environmental background"
- Strengths: Extended family networks, faith community, collective identity, resilience

Latinx/Hispanic Clients

- *Familismo* (family primacy), *personalismo* (relationship-first engagement), *respeto* (respect, particularly toward authority), *confianza* (trust that must be earned before disclosure)
- Many clients may fear disclosure due to immigration status — the counselor does not have an obligation to report undocumented status
- Somatic presentation is common; the mind-body split of Western psychiatry may not reflect the client's framework

Asian American Clients

- High rates of underutilization of mental health services — stigma around emotional expression, fear of bringing shame to the family
- Tendency toward somatic presentation (depression may present as physical complaints)
- Collectivist values may affect disclosure — family systems work and psychoeducation to the family may be welcomed in some communities
- Heterogeneity: "Asian American" encompasses over 40 cultural groups with significant differences

Indigenous/Native American Clients

- Legacy of colonization, forced assimilation, boarding school trauma (historical trauma transmitted across generations)
- Relationship with time and nonlinear thinking may differ from Western clinical frameworks
- Traditional healing practices (talking circles, sweat lodges, medicine bundles, ceremony) may be therapeutic complements to clinical services — the counselor supports rather than supplants
- High rates of trauma, substance use, suicide, and poverty as consequences of structural oppression

LGBTQ+ Clients

- Minority stress model (Meyer, 2003): Discrimination, stigma, concealment, and anticipated rejection produce chronic stress that accounts for elevated mental health burden in LGBTQ+ populations
- Affirmative practice is the ethical standard — the counselor never attempts to change sexual orientation or gender identity
- Coming out is an ongoing, nonlinear process occurring across contexts (family, work, healthcare)
- Older LGBTQ+ adults may face isolation, invisibility in elder care systems, and histories of institutionalization or forced treatment for their identities

Clients with Disabilities

- Disability-first vs. person-first language — follow the client's preference and the community norms for the specific disability
- The medical model (disability as deficit to be fixed) contrasts with the social model (disability as a mismatch between the person and environment) — many clients prefer the social model
- Do not project negative affect onto disability — assess the client's own experience, which may include pride, community belonging, and adaptation

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Microaggressions

Definition (Sue et al., 2007): Brief, commonplace, and often unintentional communications that convey hostile, derogatory, or negative racial/gender/sexual-orientation slights to members of marginalized groups.

Three forms: - *Microinsults:* Communications that convey rudeness or insensitivity ("You speak English so well!") - *Microinvalidations:* Communications that negate the experiences of marginalized groups ("I don't see color") - *Microassaults:* More intentional discriminatory behaviors (use of slurs, deliberate avoidance)

NCMHCE application: The counselor does not dismiss a client's report of microaggressions as oversensitivity. The counselor who commits a microaggression and is called out should not become defensive — the culturally competent response is curiosity, acknowledgment, and correction.

Culturally Responsive Assessment

- **Standardization bias:** Most major psychological tests (WAIS, MMPI, BDI, PHQ-9) were normed primarily on White, middle-class, English-speaking populations (Ben-Porath & Tellegen, 2020; Kroenke et al., 2001; Wechsler, 2008). Results may reflect cultural differences, not pathology.
- **Language:** Clients should be assessed in their dominant language when possible. Using a family member as interpreter raises confidentiality and accuracy concerns.
- **Acculturative stress:** Immigration, acculturation conflicts, and generational value differences are clinical data that contextualize presenting symptoms.
- **The CFI** (see above) is the evidence-based tool for integrating cultural context into assessment.

NCMHCE principle: The culturally competent counselor asks about cultural factors before concluding a diagnosis explains the client's presentation. A grief response that is culturally normative in the client's community is not a disorder, even if it seems prolonged or unusual by dominant-culture standards.

Appendix B — Research Methods and Evidence-Based Practice

Overview

The NCMHCE tests research literacy primarily in two ways: (1) understanding evidence-based practice (EBP) as a framework for clinical decision-making, and (2) recognizing basic research concepts that allow a counselor to evaluate the quality of research supporting a treatment (American Psychological Association Presidential Task Force on Evidence-Based Practice, 2006). This appendix covers the most-tested concepts.

Evidence-Based Practice (EBP)

The EBP Triangle

EBP in counseling integrates three components simultaneously — a framework adapted from evidence-based medicine (Sackett et al., 1996) and formalized for psychology by the American Psychological Association Presidential Task Force on Evidence-Based Practice (2006):

1. Best available research evidence: Research that meets standards of scientific rigor — ideally randomized controlled trials (RCTs) or meta-analyses, but adapted to context when such evidence is limited (Heppner et al., 2016).

2. Clinical expertise: The counselor's training, experience, and judgment. Not all presenting problems have matching RCT evidence; expertise bridges the gap.

3. Client values, culture, and preferences: The best-researched treatment is ineffective if the client will not engage with it, if it conflicts with their values, or if it is inaccessible. EBP is not one-size-fits-all.

NCMHCE application: An exam item may test whether the counselor selects a treatment on the basis of research support *and* client fit — not research alone. A client who refuses exposure-based therapy for OCD because of religious beliefs about the content of the intrusive thoughts requires a different conversation than simply insisting ERP is the gold standard (Foa et al., 2012).

Levels of Evidence

From strongest to weakest:

Level	Design	Notes
I	Meta-analysis / systematic review of RCTs	Highest confidence; pools results across studies
II	Randomized Controlled Trial (RCT)	Random assignment to treatment vs. control; gold standard for causality
III	Quasi-experimental (non-random assignment)	Comparison without randomization; cannot rule out confounds
IV	Cohort / longitudinal studies	Follow groups over time; good for risk factor research
V	Case-control studies	Compare outcomes after the fact
VI	Case studies / expert opinion	Lowest in hierarchy; useful for rare presentations

NCMHCE point: The exam may ask you to identify the strongest or weakest research design for a given research question. RCTs are the gold standard for treatment efficacy. Meta-analyses aggregate across RCTs for the most powerful estimate of effect.

Key Research Concepts

Reliability

The consistency of a measure — does it produce the same result across time, raters, or items measuring the same construct?

- **Test-retest reliability:** Same test administered at two time points yields similar results (stable constructs should not fluctuate)
- **Inter-rater reliability:** Two clinicians using the same instrument reach the same conclusions (important for diagnostic tools)
- **Internal consistency (Cronbach's alpha):** Items within a scale measuring the same construct correlate with each other

Validity

Does the test measure what it claims to measure?

- **Content validity:** Does the test cover the full domain of the construct? (Does a depression scale include all domains of depression?)
- **Construct validity:** Does the test measure the theoretical construct? Evidence: items should correlate with other measures of the same construct (convergent validity) and not correlate with measures of different constructs (discriminant validity)
- **Criterion validity:** Does the test predict an external outcome? (Does a suicide risk scale predict suicide attempts?)
- **Face validity:** Does the test appear to measure what it intends to measure? (Lowest form of validity evidence — not sufficient alone)

NCMHCE point: Reliability is necessary but not sufficient for validity. A scale can be perfectly reliable (produces the same result every time) but measure the wrong thing. Validity is the higher standard.

Statistical vs. Clinical Significance

- **Statistical significance (p-value):** The probability of obtaining a result at least as extreme as the one observed if the null hypothesis were true. A p-value < .05 means such a result would be unlikely (under 5%) if there were no real effect — it is not the probability that the finding is due to chance. Statistical significance does not equal meaningful change.
- **Clinical significance:** Did the change matter in the client's real life? A treatment might produce a statistically significant reduction in depression scores while the client still scores in the clinical range and remains functionally impaired.
- **Effect size (Cohen's d):** The magnitude of the treatment effect, independent of sample size. Small: d = 0.2; Medium: d = 0.5; Large: d = 0.8. A large effect size in a small sample may not reach statistical significance; a tiny effect size in a huge sample may be statistically significant but clinically irrelevant.

NCMHCE application: The counselor evaluates not just whether a treatment "worked" statistically, but whether it produced meaningful improvement. This is especially relevant when explaining research findings to clients or making referral decisions.

Types of Variables

- **Independent variable (IV):** The variable manipulated or assigned (treatment condition, diagnosis category)
- **Dependent variable (DV):** The outcome measured (symptom severity, functioning, quality of life)
- **Confounding variable:** An unmeasured variable that affects the DV and may explain the apparent relationship between IV and DV — the primary reason observational studies cannot establish causation
- **Extraneous variable:** Any variable not of interest that may affect the DV

Sampling

- **Random sampling:** Every member of the population has an equal chance of selection — produces the most generalizable results
- **Convenience sampling:** Using whoever is available — common in clinical research, but limits generalizability
- **Stratified sampling:** Population is divided into subgroups (strata) and sampled proportionally — ensures representation across groups
- **Sample size:** Larger samples produce more stable estimates and greater statistical power to detect true effects

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Common Research Designs

Experimental Designs

Randomized Controlled Trial (RCT): Participants randomly assigned to treatment vs. control (waitlist, placebo, or active comparison). Random assignment controls for confounds. Gold standard for treatment efficacy. Limitations: may not reflect real-world clinical populations (strict inclusion/exclusion); ethical concerns about withholding treatment.

Within-subjects (repeated measures) design: Each participant receives all conditions (or is measured at multiple time points). Controls for individual differences. Used in single-case experimental designs.

Between-subjects design: Different participants are in different conditions. Requires random assignment to control for group differences.

Non-Experimental Designs

Correlational study: Measures the relationship between two or more variables without manipulation. **Cannot establish causation.** A positive correlation between stress and depression does not mean stress causes depression —

a third variable (e.g., biological vulnerability) might explain both.

Longitudinal / cohort study: Follows the same group over time. Better than cross-sectional for understanding developmental patterns and risk factors. Still cannot establish causation without random assignment.

Cross-sectional study: Measures variables at a single point in time. Quick and inexpensive; cannot determine temporal sequence.

Case study: In-depth examination of a single case. Rich in clinical detail; not generalizable; no control group.

Single-case experimental design (SCED): Used in behavioral research. Uses a client as their own control across baseline and intervention phases (ABAB or multiple-baseline designs). Allows causal inference at the individual level.

Ethics in Research (ACA Code / APA Standards)

Informed Consent for Research

Research participants must receive: the purpose and procedures of the study; foreseeable risks and benefits; alternative treatments available to them; the right to withdraw at any time without penalty; and confidentiality protections. Counselors who conduct research must ensure IRB (Institutional Review Board) approval.

Special Protections for Vulnerable Populations

Children and minors require parental consent AND the child's assent. Individuals with cognitive impairment require additional protections for consent. Prisoners and other institutionalized populations require special review due to vulnerability to coercion.

Deception in Research

May be used only when the research question cannot be answered otherwise, the deception does not cause harm, and participants are fully debriefed as soon as possible afterward.

Confidentiality in Research

Data should be anonymized or coded. Identifiable data requires additional protections. Aggregated results should be reported; individual data is not shared. HIPAA and FERPA may apply depending on the research context (HHS, 2025).

Evaluating Outcome Research: Questions to Ask

When reviewing a study's clinical relevance, the NCMHCE-literate counselor asks:

1. **What was the comparison?** Treatment vs. no treatment? Treatment vs. another active treatment? The second is a stronger test.
2. **Who were the participants?** Do they resemble the counselor's actual clients? Was the sample diverse?
3. **What outcome was measured?** Symptom scale score? Functioning? Quality of life? Symptom scales may show change that doesn't reflect daily life improvement.
4. **Was the effect clinically significant?** Did participants move from clinical to subclinical range? What was the effect size?
5. **How long was the follow-up?** Treatment effects that disappear within 6 months are clinically less meaningful than sustained change.
6. **Are there conflicts of interest?** Industry-funded studies show systematically more favorable results than independent research.

Evidence-Based Treatments for Key Diagnoses

Diagnosis	First-Line EBP Treatment
Major Depressive Disorder	CBT, Behavioral Activation, IPT; antidepressants (medication + therapy superior to either alone for moderate-severe)
GAD	CBT, Acceptance and Commitment Therapy (ACT), relaxation training
PTSD	Prolonged Exposure (PE), Cognitive Processing Therapy (CPT), EMDR
OCD	ERP (Exposure and Response Prevention); SSRI augmentation
Panic Disorder	CBT with interoceptive exposure
Social Anxiety Disorder	CBT, social skills training
Borderline Personality Disorder	DBT (Dialectical Behavior Therapy)
Insomnia Disorder	CBT-I (Cognitive Behavioral Therapy for Insomnia)
Alcohol Use Disorder	MI (Motivational Interviewing), CBT, 12-Step Facilitation, naltrexone (medication)
Eating Disorders (AN)	FBT (Family-Based Treatment) for adolescents; CBT-E for adults; hospitalization when medically unstable
Eating Disorders (BN, BED)	CBT, DBT, guided self-help (for BED)
ADHD (children)	Parent Management Training + school coordination; stimulant medication
Autism Spectrum Disorder	ABA, social skills training, speech-language therapy
Bipolar I Disorder	Pharmacotherapy (lithium, valproate, antipsychotics) + psychoeducation, IPSRT, CBT

Diagnosis	First-Line EBP Treatment
Schizophrenia	Antipsychotic medication + CBT for psychosis, social skills training, supported employment (IPS)

NCMHCE principle: EBP treatments should be adapted, not abandoned, for diverse populations. The evidence base is disproportionately from White, middle-class, English-speaking samples. Cultural adaptation (retaining core mechanisms while adjusting delivery, framing, and cultural content) is supported by evidence and is consistent with best practice.

Appendix C — Administrative and Professional Practice Case Simulations

Overview

Legal & Ethical Compliance (20% of items) and Professional Development & Counselor Self-Awareness (15%) run through the NCMHCE's cases in NBCC's 2027 specifications (NBCC, 2026). Some case questions do not test clinical diagnosis or treatment planning at all — they test your ability to navigate the professional, ethical, supervisory, and legal landscape of counseling practice. The following four full case simulations cover common administrative and professional practice scenarios. Each includes a vignette, three sequential questions, and full answer key rationales.

Case 1 — Mandatory Reporting Decision

Vignette: You are a licensed counselor in private practice. Your adult client, Marcella (28), discloses that her 7-year-old nephew, whom she sees regularly, has been showing her bruises on his arms and legs. She says her brother (the child's father) "gets rough" when he drinks. She tells you, "I don't want to destroy my family. Can you keep this between us?" You are in a state that requires counselors to report reasonable suspicion of child abuse — there is no threshold of certainty required.

Question 1 of 3: Your MOST appropriate response is:

A. Honor Marcella's request for confidentiality, document her disclosure in your notes for future reference, and revisit her concerns about her nephew in later sessions.

B. Contact Marcella's brother directly to assess the situation and hear his account of the bruises before deciding whether to make a formal report.

C. Refer Marcella to a family mediator who can help the family address the situation without involving child protective services.

D. Explain that you are a mandated reporter, that her disclosure creates an obligation you cannot override, and that you will be reporting to child protective services.

Question 2 of 3: Marcella becomes angry and says she will never come back to therapy if you make the report. Your BEST response to this therapeutic rupture is:

A. Acknowledge her anger and fear, validate the difficulty, affirm that the obligation stands and you will report, and explore what support you can offer her.

B. Agree to delay the report by one week so that Marcella has time to process her feelings and to prepare herself for what may happen next within her extended family.

C. Document Marcella's refusal in the record and do not make the report, so that the therapeutic relationship and her trust in you are preserved for future work.

D. Transfer Marcella to another counselor who may hold a different interpretation of the reporting law and of whether her disclosure actually triggers a report.

Question 3 of 3: After making the report, Marcella does return to session. She asks if CPS will tell her brother that you reported. Your MOST accurate response is:

A. "Yes, CPS will share the reporter's identity with your brother immediately, as soon as a caseworker reaches out to him to begin looking into the report."

B. "No — all mandatory reports are fully anonymous, and your brother will never find out that anyone in the family spoke to a counselor about his son."

C. “CPS generally keeps the reporter’s identity confidential, but as the reporter my name may be disclosed in legal proceedings, depending on state law.”

D. “I am not permitted to tell you anything about the reporting process, including what CPS does or who gets contacted once a report has been made to them.”

Answer Key — Case 1

Q1 — Answer: D. Mandatory reporting is a legal duty that supersedes client confidentiality. The threshold is reasonable suspicion — not certainty. A counselor who fails to report known or reasonably suspected child abuse is in violation of state law and professional ethics and is subject to license discipline. Honoring confidentiality (A), contacting the alleged abuser (B), or redirecting to mediation (C) all delay or circumvent the reporting obligation.

Q2 — Answer: A. A therapeutic rupture following mandatory reporting is common and workable. The counselor acknowledges the relational impact, validates the client’s emotional response, and remains clear that the legal obligation cannot be negotiated. Delaying the report (B) creates legal liability and places the child at continued risk. Failing to report due to client protest (C) is a clear ethics and legal violation. Transferring the client to avoid the obligation (D) is an avoidance strategy, not clinical judgment.

Q3 — Answer: C. State mandatory reporting laws vary, but most provide reporter confidentiality protection that limits CPS from disclosing the reporter’s identity to the alleged abuser — however, this is not absolute, and identity may be disclosed in legal proceedings. Absolute anonymity (B) is inaccurate. Refusing to answer (D) is unhelpful and clinically avoidant. Immediate identity disclosure (A) is inaccurate in most jurisdictions.

Case 2 — Supervisory Dilemma

Vignette: You are a licensed clinical supervisor. One of your supervisees, Rafael (intern), has been providing individual counseling at a community mental health agency. A colleague at the agency contacts you and reports that Rafael was observed visibly intoxicated while conducting a session with a client yesterday. When you meet with Rafael to address the concern, he denies that he was intoxicated and says the colleague "has it out for him." He does not appear impaired at the time of your meeting.

Question 1 of 3: Your MOST appropriate initial action is:

A. Take the allegation seriously, initiate a formal supervisory review, require Rafael to cease direct client contact pending the outcome, and document all actions.

B. Accept Rafael's explanation at face value — he presents as clear-headed today, and the colleague's concern about yesterday's session reached you secondhand from a coworker.

C. Contact the colleague to find out whether she will put her concern in writing before you take any action on what she reported.

D. Advise Rafael to seek a substance use evaluation voluntarily, and take no formal action on the report unless he declines to pursue one on his own.

Question 2 of 3: Rafael subsequently discloses that he has been struggling with alcohol use and has been drinking before some sessions because "it calms my nerves." He asks you to keep this between the two of you. Your MOST appropriate response is:

A. Honor Rafael's request for confidentiality, because the disclosure was made within supervision and supervisory conversations are meant to be a protected space for growth.

B. Arrange a referral to an employee assistance program and allow Rafael to continue seeing clients on his regular schedule while he seeks treatment for his alcohol use.

C. Explain that supervisory disclosures of impairment affecting client welfare are not confidential, that you must report this to the agency and licensing board, and that he must stop providing services immediately.

D. Document the disclosure in your supervision notes and inform the licensing board only if one of Rafael's clients makes a formal complaint about his conduct.

Question 3 of 3: Rafael asks what will happen to his license. Your MOST accurate and ethically appropriate response is:

A. "If you complete a treatment program, the licensing board will not take any action against you, and your license and standing in the profession will stay exactly as they are."

B. "I can't predict the board's decision; I must report and prioritize client safety. Wellness programs and following treatment recommendations often factor into its decisions."

C. "Licensing board proceedings are confidential, so your clients, your colleagues, and any future employers will never learn about any of this."

D. "You will likely lose your license over this, so you should consult an attorney and plan your defense before I report anything to the board."

Answer Key — Case 2

Q1 — Answer: A. Supervisee impairment affecting client welfare is a safety and ethics emergency. Secondhand reports require serious investigation, not dismissal. Accepting denial without further review (B) is clinically negligent. Waiting for written confirmation (C) delays necessary action. Voluntary referral without formal review (D) is insufficient when client harm may have already occurred.

Q2 — Answer: C. Supervisory confidentiality does not extend to impairment affecting client welfare. The supervisor's dual obligation — to the supervisee's development and to client safety — is explicitly resolved in favor of client safety when the two conflict. Honoring confidentiality (A) places clients at ongoing risk and violates the supervisor's ethical obligations. Allowing continued client contact during treatment (B) is contraindicated when the counselor has admitted to drinking before sessions. Reporting only if a client complains (D) is reactive and fails the preventive duty of supervision.

Q3 — Answer: B. The supervisor is not in a position to predict licensing board outcomes and should not do so. Offering false reassurance (A) is misleading. "Licensing board proceedings are confidential" (C) is partially inaccurate — boards typically publish disciplinary actions. Predicting license revocation and advising attorney consultation before reporting (D) conflates the supervisor's obligation with the supervisee's legal strategy and may function to suppress a required report.

Case 3 — Records Request and Subpoena

Vignette: You are a counselor in private practice. You receive a call from an attorney who says he represents the husband in a divorce/custody proceeding involving your current client, Sara (34). He states, "I'm sending you a subpoena for all of Sara's therapy records and I expect them within five business days." The next day, a subpoena arrives by certified mail.

Question 1 of 3: Your MOST appropriate immediate response upon receiving the subpoena is:

A. Immediately compile Sara's full record, including intake forms and all process notes, and send it to the attorney within the five-day window he specified in his call.

B. Contact Sara, consult your own attorney, and release nothing without a valid court order or Sara's written au-

thorization — a subpoena alone does not override privilege.

C. Ignore the subpoena, because you are not a party to the lawsuit and a subpoena issued by opposing counsel in a divorce/custody case creates no obligation for you to respond.

D. Contact the attorney and negotiate the scope of records to be released, and then inform Sara once you have agreed on what will be sent.

Question 2 of 3: Sara tells you she does not want her records released. She is frightened that her therapy content will be used against her in the custody proceeding. Your MOST appropriate clinical and ethical response is:

A. Release the records anyway, because the subpoena is a legal document that overrides Sara's preference about her own therapy content.

B. Offer to provide a summary letter of treatment status only, not process notes, so that you comply partially with the subpoena while limiting how much of Sara's content is shared.

C. Withdraw from Sara's case to avoid involvement in the legal proceeding and any testimony the husband's attorney may seek from you.

D. Support Sara's right to invoke privilege, document her objection, and consult your attorney about a motion to quash or a protective order if the court orders production.

Question 3 of 3: The court subsequently issues a court order (not just a subpoena) requiring you to produce Sara's records. Sara's attorney has not filed a motion to quash. Your MOST appropriate response is:

A. Comply with the order, notifying Sara in advance when possible and limiting production to what the order specifies.

B. Continue to refuse release, as a court order does not override therapeutic privilege any more than the original subpoena did.

C. Produce only session dates and diagnoses, and refuse to produce process notes under any circumstances to protect Sara's privacy.

D. Resign from Sara's case and transfer her records to another clinician so that you are no longer subject to the order.

Answer Key — Case 3

Q1 — Answer: B. A subpoena from opposing counsel initiates a legal process but does not automatically override therapeutic privilege. Privilege belongs to the client, not the counselor. The counselor's obligation is to inform the client, seek legal consultation, and object to production until either the client waives privilege in writing or a court orders production over objection. Immediate compliance (A) violates privilege. Ignoring a subpoena (C) may result in contempt. Negotiating with opposing counsel without involving the client (D) is ethically inappropriate.

Q2 — Answer: D. When a client asserts privilege, the counselor's role is to honor that assertion and assist in its legal enforcement — not to override it. Filing a motion to quash (or supporting the client's attorney in doing so) is the appropriate legal mechanism. Releasing over objection (A) violates privilege without court order. A partial summary release (B) still involves releasing privileged content without client authorization. Withdrawal (C) abandons the client at a critical juncture.

Q3 — Answer: A. A court order, unlike a subpoena from opposing counsel, is a legal mandate that overrides privilege. Once a valid court order is issued and privilege has not been successfully quashed, the counselor must comply. Continued refusal (B) constitutes contempt of court. Partial compliance limited to dates and diagnoses (C) may violate the order and is the counselor's unilateral decision

to make without legal authority. Transferring the case to evade compliance (D) does not extinguish the obligation under the court order.

Case 4 — Dual Relationship and Scope of Practice

Vignette: Jerome (45) was your individual therapy client for two years, terminating successfully 18 months ago. He calls your office and says he wants to refer his 17-year-old son, Marcus, to you for treatment of depression and school refusal. Jerome says, "I trust you more than anyone, and Marcus needs the best." You would be Marcus's primary individual therapist.

Question 1 of 3: Your MOST appropriate response to Jerome's referral request is:

A. Accept Marcus as a client — the therapeutic relationship with Jerome ended 18 months ago, which is a sufficient gap to avoid any conflict between the two treatment relationships.

B. Accept Marcus only if Jerome agrees not to discuss Marcus's treatment content with you, whether in or outside of sessions.

C. Decline the referral, as treating a parent and child in the same family system creates a problematic dual relationship, and give Jerome alternative referrals for Marcus.

D. Accept Marcus and conduct family sessions with both Jerome and Marcus together to prevent any information asymmetry between them.

Question 2 of 3: Jerome asks, "What if Marcus wants to see you? Shouldn't Marcus's wishes take priority?" Your MOST clinically and ethically appropriate response is:

A. "If Marcus specifically requests me, that changes the ethical calculus — his request resolves the concern, and I

would accept the referral and begin working with him right away."

B. "Marcus's preferences matter, but the concern is structural — my prior relationship with his father could affect my neutrality as his individual therapist, whatever he prefers."

C. "I agree — Marcus is nearly an adult, and his autonomy should be decisive in this situation over other considerations."

D. "If Marcus is in crisis, I am required to accept any referral regardless of potential dual relationship concerns involving you, him, or the rest of your family."

Question 3 of 3: Jerome expresses frustration and says, "You are the only person I trust with my son. Are you really going to turn him away?" The MOST appropriate counselor response is:

A. Reassure Jerome that you will reconsider your decision if he still feels this way in one month and Marcus is still struggling.

B. Accept the referral with the understanding that you will consult with a supervisor about managing the dual relationship as treatment unfolds.

C. Refer Marcus to a colleague in your own practice so that you can remain accessible to the family if needed during his treatment.

D. Acknowledge Jerome's trust, validate his wish for continuity of care for his son, and remain clear that a therapist with no prior tie to his parent is most protective — with a warm handoff to a qualified colleague.

Answer Key — Case 4

Q1 — Answer: C. Treating the minor child of a former adult client creates a structural dual relationship: the counselor has prior knowledge, relational history, and potential loyalty to the parent that compromises the neutrality required to serve the child's individual interests. The

18-month gap does not eliminate this concern — it reduces (but does not eliminate) the risk of using confidential information from the prior relationship. The ethical standard is whether the dual relationship could impair clinical judgment or exploit the client (ACA Code A.6.b; ACA, 2014). The clinically and ethically protective answer is to provide warm referrals and decline.

Q2 — Answer: B. Client preference is relevant but does not override a structural dual relationship concern. Marcus (at 17) cannot fully appreciate the ways in which a therapist's prior relationship with his parent might subtly influence assessment, interpretation, or disclosure comfort. The counselor explains the concern structurally, not as a personal rejection of Marcus.

Q3 — Answer: D. The counselor maintains a clear position, validates Jerome's relational trust as a sign of prior therapeutic success, and reframes the referral to another clinician as the most protective clinical decision — not as abandonment or refusal. Leaving the door open for reconsideration (A) suggests the decision is negotiable. Consulting and proceeding (B) does not resolve the structural dual relationship. Referring within the same practice (C) perpetuates the proximity concern.

Appendix D — Test-Day Logistics

Logistics change more often than content. Confirm every item below in the current NBCC candidate handbook before test day.

> **Note:** The NCMHCE is administered by NBCC through Pearson VUE. Policies, fees, and procedures change periodically. Always verify current information directly at **nbcc.org** and **pearsonvue.com/nbcc** before your exam date. This appendix reflects general procedures as of 2025–2026 and is provided as orientation, not as a substitute for official NBCC guidance.

Eligibility and Registration

Who administers the exam: NBCC (National Board for Certified Counselors) owns the NCMHCE. Pearson VUE is NBCC's testing delivery partner. You register through NBCC, not directly through Pearson VUE.

Eligibility: Candidates must meet NBCC's eligibility requirements (graduate degree in counseling, supervised experience hours, and faculty endorsement or state board referral depending on the pathway). Eligibility is verified before you are authorized to test.

Authorization to Test (ATT): Once NBCC approves your application, you receive an ATT email with a candidate ID and instructions for scheduling at Pearson VUE. The ATT has an expiration window — schedule your exam before it expires.

Scheduling: Schedule through the Pearson VUE website or phone. Testing center locations are searchable by zip code. Remote proctored (online) testing may be available — verify in the current NBCC candidate handbook.

Testing Center Procedures

Arrival: Plan to arrive at least 30 minutes before your scheduled appointment. Late arrivals may be turned away and may forfeit their exam fee. Verify arrival and fee rules in the current NBCC candidate handbook.

Identification: Two forms of ID are required. Primary ID must be government-issued with your photo and signature (driver's license, passport, state ID). Secondary ID must bear your name and signature. Names on both IDs must match the name on your NBCC registration exactly — discrepancies can result in being denied testing. Verify current ID rules in the current NBCC candidate handbook.

Check-in process: Palm vein scan or fingerprinting for identity verification. Digital photograph taken at the testing center. Signature collected electronically.

Personal items: All personal items (phone, bag, watch, jacket, wallet) must be stored in a provided locker. Nothing is permitted at your workstation except what the testing center provides.

Provided materials: Scratch paper and pencil (or a dry-erase board and marker) are provided and must be returned at the end of the exam. No personal scratch paper is permitted.

Prohibited items: Electronic devices (including smartwatches), notes, study materials, food, and drinks (water may be permitted in some centers — verify locally). Earplugs or noise-canceling earbuds are not permitted; foam earplugs provided by the testing center may be available upon request.

Breaks: The NCMHCE is a timed exam. The 2027 specifications provide an optional 15-minute break halfway through the exam (NBCC, 2026). Unscheduled breaks (restroom, etc.) have typically not stopped the exam clock — verify in the current NBCC candidate handbook. Plan accordingly.

Exam Format at the Testing Center

Delivery: Computer-based. The exam is presented on a standard monitor. No special technical skill is required — navigation is point-and-click.

Format reminder (2027): 10 case studies, each in three sections — an initial intake summary followed by two counseling sessions. Each section presents narrative and then multiple-choice items — 9–15 items per case, four options each. Total items: 130–150, of which 30–50 are unscored field-test items scattered through the exam in the same format. Total testing time: 225 minutes to complete the items, with an optional 15-minute break halfway through (NBCC, 2026). Tutorial length and total appointment time — verify in the current NBCC candidate handbook.

> **Testing before July 1, 2027?** You will take the current NCMHCE: 11 cases built on NBCC's 2023 content outline, with 100 scored items. The clinical content in this book applies to both versions. The format and domain map in this edition are the 2027 ones. Check nbcc.org to confirm which version you will take.

Reviewing answers: The specifications let you review questions before you submit each half of the exam. Once a half is submitted and the break begins, you cannot return to it (NBCC, 2026). Whether sections within a case lock as you move forward is not stated in the specifications — verify in the current NBCC candidate handbook, and review each section before you click forward.

ADA Accommodations

Who qualifies: Candidates with documented disabilities affecting their ability to take the exam under standard conditions may request testing accommodations. Common accommodations include extended time (1.5× or 2×), a

separate testing room, screen magnification, a reader, or other auxiliary aids.

How to request: Submit an accommodation request through NBCC's application process — not through Pearson VUE directly. Required documentation typically includes a current evaluation or diagnosis from a licensed professional (psychologist, physician, or licensed diagnostician), documentation of the functional limitation and its impact on testing, and the specific accommodation recommended.

Timing: Accommodation requests must be submitted and approved before your ATT is issued. Do not wait until close to your test date — processing takes time and incomplete documentation will delay approval.

Appeals: If your accommodation request is denied, NBCC has an appeals process. Contact NBCC directly for current procedures.

NBCC contact for accommodations: See nbcc.org for current contact information and the accommodation request form.

Score Reporting

Immediate results: You receive an unofficial score report after you finish testing. It shows your preliminary pass/fail status and general feedback at the domain level, and it is not the official score (NBCC, 2026). How you receive it (from the proctor at a test center, or in your Pearson dashboard for online testing) — verify in the current NBCC candidate handbook.

Official score report: Testing runs in monthly administration cycles. For state licensure, official scores are reported to your state board within 30 days of the end of the monthly test administration cycle; for NCC certification, your pass/fail status is updated in your Credentialing Gateway account (verify the timeline in the current NBCC candidate handbook).

Score scale: Scores are scaled from 100 to 500, and the passing point on every form is 360. A scaled score is not a raw score or a percentage — 400 does not mean 80% correct. Only the scored items count. NBCC sets the passing point through Angoff standard setting and keeps it fair across forms through statistical equating; the exam is criterion-referenced, so your result does not depend on how other candidates perform (NBCC, 2026).

If you do not pass: NBCC allows retesting after a mandatory waiting period (verify in the current NBCC candidate handbook). Score reports include domain-level feedback that can guide focused study before a retake.

Reporting to state boards: If your exam is taken as part of a state licensure process (rather than NCC certification), your state board typically receives score information directly from NBCC. Verify your state board's reporting process and timeline.

Practical Preparation Reminders

The following are not clinical content — they are logistics that affect performance on exam day.

Sleep the night before is more valuable than a last-minute review session. Cognitive performance under time pressure is significantly degraded by acute sleep deprivation. Stop studying by the evening before your exam.

Eat a normal meal before you go. Hunger degrades attention and working memory. The testing center will not permit food at the workstation.

Arrive early enough to complete check-in without rushing. The check-in process (ID verification, palm scan, locker assignment, palm scan again) takes longer than candidates expect.

Read the full case vignette before answering any items. When a case states a working diagnosis, your job is to confirm or revise it and use it, not ignore it. Candidates

who skim the vignette miss the clinical details that distinguish correct from plausible-but-wrong responses.

If you feel stuck on a case, mark your best answer and move on. The exam does not penalize for guessing. An unanswered item is always wrong; a guess has positive expected value.

Reasoning Skills Workbook

Multiple-choice practice shows you *whether* you chose the right answer. This workbook trains *how* you get there. Each drill isolates one reasoning tool from The Clinical Reasoning Toolkit so you can practice it on its own, find where your reasoning breaks down, and fix it before the full practice exams.

How to use it: Work each drill in writing before checking the Workbook Answer Key at the end of this section. For every item you miss, write one sentence explaining what you would do differently. Those sentences become your personal review sheet for the week before the exam.

Study Plan: Ten Weeks to Exam Day

This plan assumes about 8–10 hours of study a week. Compress it if your exam date is closer, but keep the order: learn the tools first, practice them in isolation, then apply them under exam conditions.

Week	Read	Practice
1	How to Use This Book; How This Book Maps to the NCMHCE; The Clinical Reasoning Toolkit; Chapters 1–4 (including Counselor Self-Awareness in Chapter 1)	Drills 1 and 8; the chapter practice questions

Week	Read	Practice
2	Chapters 5–7 (including Continuity of Care in Chapter 6)	**Practice Exam One, timed (225 minutes)**, as your baseline — you have not read the diagnostic chapters yet, so expect a lower score that will rise over the coming weeks; Drills 3 and 6; review every missed item with the answer key
3	Chapters 8–11	Drills 4 and 5; chapter practice questions
4	Chapters 12–15	Drill 2; chapter practice questions
5	Chapters 16–19	Drill 9; Supplemental Questions 1–50
6	Chapters 20–24	Supplemental Questions 51–100
7	Chapters 25–28 (including Practice Management and Compliance in Chapter 25 and the psychopharmacology reference in Chapter 28); Appendices A and B	Drills 7, 10, and 11; Supplemental Questions 101–153
8	Assessment Tools Compendium; Defense Mechanisms and Cognitive Distortions Quick References	**Practice Exam Two, timed**; error analysis using the log below
9	Your two weakest domains (from the log); Appendix C	Redo every missed item from Weeks 2–8; repeat the drills you scored lowest on
10	Appendix D; your personal review sheet	**Practice Exam Three, timed**; light review only in the last 48 hours

Practice Exam Log

After each timed exam, record your results. For every missed question, identify **which of the six 2027 NBCC domains** it tested and **which distractor type** caught you (Drill 2 teaches the seven types). Patterns in this log tell you what to review in Week 9.

Exam	Date	Score (of 130)	Domain missed most	Distractor type that caught me most	What I will change
Practice Exam One					
Practice Exam Two					
Practice Exam Three					

Drill 1 — Match the Question to the Tool

For each stem, name the reasoning tool that best fits the question. Some stems call for two tools; name the one that decides the answer.

1. A client disclosed in Session Two that she has been cutting her arms. What should the counselor do FIRST?
2. Which diagnosis BEST accounts for the client's symptoms?
3. Which counselor response BEST conveys empathic understanding?
4. The client says he "might cut back on drinking someday, but not now." Which intervention is MOST appropriate?
5. Given the client's current risk and supports, what level of care is MOST appropriate?

6. The client's cousin calls the office asking how the client is doing. How should the counselor respond?
7. The client asks whether she should stop taking her sertraline. How should the counselor respond?
8. A 7-year-old refuses to talk about the car accident he was in. Which approach is MOST appropriate?
9. Which factor is MOST likely maintaining the client's panic attacks?
10. During trauma processing, the client goes numb and stares blankly at the floor. What should the counselor do?
11. A client whose home burned down last night arrives distraught and shaking. What should the counselor do FIRST?
12. A client reports occasional thoughts that she would be better off dead, with no plan, and describes close ties to her children. Which response is MOST appropriate?
13. A supervisor asks the counselor to backdate a progress note for an audit. What should the counselor do?
14. A client says her family's spiritual healer believes the voices she hears are her ancestors. What should the counselor do before settling on a diagnosis?
15. A 9-year-old's tantrums occur mostly when homework is assigned and usually end with the homework postponed. What should the counselor assess?
16. A couple presents for "communication problems." The wife looks at her husband before answering every question and flinches when he raises his voice. What should the counselor do?
17. Which treatment has the strongest evidence for this client's obsessive-compulsive disorder?
18. In Session Three, a client being treated for depression says he has barely slept for four days and "feels amazing." What is the MOST important consideration?
19. When should the counselor begin trauma-memory processing with this client?

20. How should the counselor explore the ways the client's identities as an older, disabled, gay immigrant shape his distress?

Drill 2 — Name the Flaw

Each option below is a wrong answer. Name its distractor type: **right action, wrong time**; **outside scope**; **too passive**; **too aggressive**; **counselor-centered**; **mismatched to the client**; or **absolute language**.

1. A client reports command hallucinations telling him to hurt his neighbor. *"Schedule a follow-up next week to monitor the voices."*
2. At intake, a client mentions she drinks a bottle of wine every night and has had shakes in the morning. *"Begin cognitive restructuring of her negative self-talk today."*
3. *"Tell the client his antidepressant dose is probably too low and suggest he ask for a higher dose."*
4. A client has passive thoughts of death, no plan, and strong supports. *"Initiate involuntary hospitalization."*
5. *"Refer the client to another counselor because the counselor personally disagrees with her decision."*
6. A client who sees no problem with his drinking is in counseling only because his wife insisted. *"Set a quit date and give him a sobriety calendar."*
7. *"Assure the adolescent that nothing she says will ever be shared with her parents."*
8. *"Teach the 5-year-old to complete a written thought record."*
9. A client discloses that her partner has been hitting her 6-year-old. *"Encourage the client to report it herself when she feels ready."*
10. *"Tell the client that the counselor also went through a divorce and found it freeing."*
11. A client's husband died two weeks ago; she cries daily but is functioning. *"Diagnose prolonged grief disorder and begin grief-focused treatment."*

12. *“Advise the client with suspected hypothyroidism to start an over-the-counter thyroid supplement.”*
13. *“Always terminate counseling with any client who misses two sessions.”*
14. A client becomes flooded and dissociative midway through a trauma narrative. *“Encourage her to push through the memory to finish the exposure.”*
15. A client has a specific overdose plan and access to pills. *“Accept her promise not to hurt herself and end the session as scheduled.”*
16. A client with mild, occasional panic attacks is functioning well at work. *“Recommend admission to an inpatient anxiety program.”*
17. A client who is fasting for Ramadan reports afternoon fatigue. *“Tell her the fasting is unhealthy and she should stop.”*
18. *“The counselor should never discuss personal reactions to clients, even in supervision.”*
19. A client newly reports hearing voices that no one else hears. *“Wait to see whether the voices resolve on their own.”*
20. In a first session, a client minimizes his gambling losses. *“Confront his denial directly so he faces the truth today.”*

Drill 3 — What Changed?

Each item gives an intake and a later session. Write (a) what is new and (b) how it changes the counselor’s next step.

1. **Intake:** Maria, 41, has generalized anxiety disorder, and CBT has begun. **Session Two:** She says she drinks four glasses of wine every night “to turn off the worry.”
2. **Intake:** Theo, 58, has an adjustment disorder after losing his job. **Session Two:** He mentions he has given away his guitar collection and says, “They’ll be better off soon.”

3. **Intake:** Emma, 15, is referred for anxiety-related school refusal. **Session Two:** She says her mother's boyfriend "comes into my room at night and I don't like it."
4. **Intake:** Ben, 33, has major depressive disorder; his PHQ-9 is 18. By Week 4 it is 14. **Session Three (Week 8):** His PHQ-9 is still 14, unchanged for four weeks.
5. **Intake:** A couple presents for "communication problems." **Session Two:** The wife calls privately to say her husband choked her last month.
6. **Intake:** Ray, 52, reports insomnia, and CBT-I is planned. **Session Two:** His wife joins and reports that he snores loudly and "stops breathing" at night.

Drill 4 — Case Conceptualization Worksheets (The 4 Ps)

For each vignette, list the **predisposing**, **precipitating**, **perpetuating**, and **protective** factors, then name the **primary target** for treatment.

Vignette A. Janelle, 28, has had depressive episodes since adolescence, and her mother was treated for depression. Her current episode began after a breakup three months ago. Since then she has stopped going to the gym, turned down invitations from friends, and spends weekends in bed scrolling her phone, telling herself "no one wants me around." She is close to her sister and is still working.

Vignette B. Omar, 35, a combat veteran, has PTSD symptoms that worsened after a car accident last spring. He avoids driving, drinks nightly to sleep, and "keeps busy" so he does not have to think. He grew up in a violent home. He has a supportive wife and attends a veterans' group occasionally.

Vignette C. Keisha, 10, was diagnosed with generalized anxiety disorder. She has always been "a worrier." Her anxiety escalated when her family moved to a new school

district in the fall. Each night she asks her parents dozens of reassurance questions, and they answer every one to calm her. She has one close friend and loves art.

Drill 5 — Risk Formulation

For each client, rate the risk as **low**, **moderate**, or **high**, name the key factors behind your rating, and state the response it calls for.

1. Nora, 44, reports occasional thoughts that "it would be easier not to wake up," with no plan, intent, or past attempts. She is engaged in treatment and describes her two children as her reasons for living.
2. Evan, 23, says he has thought about overdosing on the pills in his bathroom cabinet but has "no intention of doing it." He has a supportive roommate. He has made no past attempts.
3. Frank, 61, recently divorced and drinking heavily, says he has a handgun at home and plans to "end things this weekend."
4. Tasha, 17, attempted suicide by overdose two weeks ago. Today she says she is "totally fine now" and mentions she has given her cat to a friend.
5. Luis, 39, has had chronic passive suicidal thoughts for years without a plan. This month he lost his job, started drinking daily, and stopped taking his medication.

Drill 6 — Mental Status Exam Write-Ups

Translate each observation into correct MSE terminology and name the MSE category.

1. When asked about his week, the client talks about his car, then his brother's car, then car prices, and never answers the question.
2. The client speaks rapidly and loudly and is very hard to interrupt.

3. The client says, “I’m doing great,” while tears run down her face.
4. The client insists that newscasters are sending him coded messages through the television.
5. The client reports hearing a man’s voice commenting on her actions when no one else is in the room.
6. The client gives long, detailed side stories but eventually answers each question.
7. Throughout the session, the client’s face shows almost no emotional expression, even when describing her mother’s death.
8. The client cannot state the date, the month, or the name of the building she is in.

Drill 7 — Rewrite the Objective

Identify what is wrong with each objective, then rewrite it as a SMART objective (specific, measurable, achievable, relevant, time-bound).

1. Client will be less anxious.
2. Counselor will provide DBT skills training.
3. Client will never drink again.
4. Client will improve his relationship with his son.
5. Client will gain insight into her trauma.
6. Client will sleep better.

Drill 8 — The Empathy Ladder

For each client statement, rate each response as **subtractive (S)**, **interchangeable (I)**, or **additive (A)**. Then write your own additive response.

1. *“I finally got the promotion, but I just feel empty.”* - a. “Congratulations! That’s a huge accomplishment.” - b. “You got the promotion, and you feel empty.” - c. “You reached something you worked hard for, and it isn’t giving you the feeling you expected — that’s confusing and maybe a little lonely.”

2. *“My kids don’t call anymore unless they need money.”* - a. “It sounds like you feel used, and maybe unseen by the people you raised.” - b. “Kids get busy at that age.” - c. “Your kids mostly call when they need money.”

3. *“I keep telling myself it wasn’t my fault, but I don’t believe it.”* - a. “It wasn’t your fault.” - b. “Part of you knows the facts, but underneath there’s still a heavy sense of blame you can’t shake.” - c. “You tell yourself it wasn’t your fault but don’t believe it.”

4. *“Everyone thinks I’m the strong one.”* - a. “Being the strong one sounds exhausting — like there’s no room for anyone to take care of you.” - b. “Everyone sees you as strong.” - c. “That’s a great quality to have.”

Drill 9 — Crisis Sequencing

Scenario: Dana, 19, calls the counseling center the morning after being sexually assaulted at a party. She is crying, says she “can’t stop shaking,” and has not told anyone.

Put these counselor actions in the correct order using the seven-stage crisis intervention model:

- 1. Help Dana identify what she needs most right now — medical care, safety, and whom to tell
- 1. Schedule a follow-up contact within a day or two and confirm how to reach support
- 1. Assess her immediate safety, injuries, and any risk of self-harm
- 1. Reflect and validate her fear, shock, and distress
- 1. Explore options together: a medical exam, advocacy services, a trusted friend, reporting choices
- 1. Establish calm, respectful contact and let her know she is believed
- 1. Agree on concrete next steps for today, such as going to the hospital with an advocate

Drill 10 — Ethics Quick-Fire

Mark each statement **true** or **false**, and name the principle or standard involved.

1. A counselor may begin a romantic relationship with a former client three years after termination.
2. Informed consent is completed once, at intake.
3. A counselor may refer a client solely because the client's values conflict with the counselor's.
4. Before providing telehealth to a client in another state, the counselor must consider the laws where the client is located.
5. A counselor must be certain that abuse occurred before making a child abuse report.
6. Psychotherapy notes kept separately from the medical record receive heightened protection under HIPAA.
7. Bartering is permitted when the client requests it, it is not exploitive or harmful, it is an accepted practice in the professional community, and the arrangement is put in writing.
8. An impaired counselor should keep seeing clients so as not to abandon them.
9. A counselor may end counseling when a client does not pay agreed fees, provided the counselor gives pre-termination counseling and referrals.
10. A minor's assent does not matter once a parent has consented.
11. To correct an error in a record, the counselor deletes the original entry and rewrites it.
12. In distance counseling, the counselor verifies the client's identity at the start of counseling and throughout it.
13. A client's family member may serve as the interpreter for clinical sessions whenever it is convenient.
14. Counselors may disclose to an identifiable third party at high risk of a contagious, life-threatening disease if

the client refuses to inform them, consistent with state law.

15. When consulting a colleague about a case, the counselor shares the client's name so the advice is accurate.

Drill 11 — Applying the ADDRESSING Framework

Vignette: Samuel, 67, immigrated from Ghana 30 years ago. He is a retired bus driver, a deacon in his church, and uses a wheelchair since a stroke two years ago. He was referred by his physician for low mood. He says, "In my culture, men do not complain. I only came because my doctor insisted." He lives with his daughter's family and worries about being "a burden."

1. Name at least five ADDRESSING factors that may shape Samuel's experience of his distress and of counseling.
2. Write two open questions that invite Samuel to describe what these identities mean to him.
3. Name one way the counselor's own identities could affect this relationship, and what the counselor would do about it.
4. Which other tool from The Clinical Reasoning Toolkit would you also apply here, and why?

Workbook Answer Key

Drill 1 — Match the Question to the Tool

1. Priority Ladder: new self-harm is rung 1 safety. Assess risk now.
2. Differential diagnosis.
3. Core counseling attributes: additive empathy.
4. Stage of change: contemplation or precontemplation calls for motivational interviewing, not an action plan.

5. Least restrictive level of care that is safe, informed by risk formulation.
6. Release check: without written authorization, the counselor neither confirms nor denies that the person is a client.
7. Scope check: medication decisions belong to the prescriber. Explore her concerns and coordinate with the prescriber.
8. Developmental fit: play-based or expressive approaches, with caregivers involved.
9. Case conceptualization (the 4 Ps): look for the perpetuating factor, such as avoidance or safety behaviors.
10. Window of tolerance: she is hypoaroused. Pause, ground, and bring her back into the window before continuing.
11. Crisis intervention model: stage 1, assess safety and immediate needs.
12. Risk formulation: this is low risk, so the response is outpatient care, safety planning, and follow-up.
13. Ethical decision-making model: backdating falsifies a record. Decline, consult, and document.
14. Cultural formulation: ask what the experience means within her cultural and spiritual framework before diagnosing.
15. Functional analysis: the tantrums are negatively reinforced by escaping homework.
16. Systems thinking plus rung 1 safety: screen privately and separately for partner violence before continuing joint sessions.
17. Evidence-based fit: exposure and response prevention (ERP).
18. What changed, then differential diagnosis: a decreased need for sleep with elevated mood suggests hypomania, which changes the diagnosis and plan. Coordinate a medical and psychiatric evaluation.

19. Phase-oriented care: only after safety and stabilization, and only when the client can stay within the window of tolerance.
20. ADDRESSING framework, with cultural humility: ask rather than assume.

Drill 2 — Name the Flaw

1. Too passive: command hallucinations to harm someone require immediate risk assessment.
2. Right action, wrong time: morning shakes signal withdrawal risk (rung 1–2) before cognitive work.
3. Outside scope: dosing is the prescriber's decision.
4. Too aggressive: overreacts to low risk.
5. Counselor-centered: referral based on values violates ACA Standard A.11.b.
6. Mismatched to the client: an action-stage task for a precontemplative client.
7. Absolute language: confidentiality has limits, especially with minors.
8. Mismatched to the client: the task does not fit a young child's development.
9. Too passive: the counselor is a mandated reporter.
10. Counselor-centered: self-disclosure that serves the counselor.
11. Right action, wrong time: prolonged grief disorder requires at least 12 months in adults.
12. Outside scope: medical advice.
13. Absolute language: termination decisions are individualized.
14. Right action, wrong time: the client is outside her window of tolerance. Stabilize first.
15. Too passive: a specific plan with access to means requires means restriction and a higher-intensity response, not a promise.
16. Too aggressive: far more restrictive than the need.
17. Mismatched to the client: it ignores cultural and religious meaning and imposes the counselor's values.

Explore instead, and coordinate medically if there are health concerns.

18. Absolute language: supervision is exactly where reactions should be discussed.
19. Too passive: new psychotic symptoms need assessment, including medical and substance causes.
20. Right action, wrong time: confrontation before engagement increases resistance. Use motivational interviewing.

Drill 3 — What Changed?

1. **New:** heavy nightly drinking used to manage anxiety. **Next:** screen with the AUDIT, assess withdrawal risk, and add integrated treatment of the alcohol use to the plan.
2. **New:** giving away possessions plus a statement implying death — warning signs. **Next:** conduct a suicide risk assessment now (rung 1), with means restriction and a safety plan as indicated.
3. **New:** a possible disclosure of sexual abuse. **Next:** gently gather only what is needed, follow mandated reporting requirements, and address her immediate safety. The school refusal plan waits.
4. **New:** a plateau with no change for four weeks. **Next:** review the plan with Ben — check safety, the alliance, the diagnosis (bipolarity, substances), whether treatment has been delivered fully, barriers, and whether a medication evaluation is needed.
5. **New:** strangulation, a strong predictor of lethal partner violence. **Next:** stop conjoint sessions, safety-plan with the wife individually, and connect her with domestic violence resources. Do not reveal her disclosure to the husband.
6. **New:** signs of possible obstructive sleep apnea. **Next:** refer for medical evaluation and a sleep study, since the insomnia plan depends on ruling out a breathing-related sleep disorder.

Drill 4 — The 4 Ps

Vignette A (Janelle): - Predisposing: an adolescent-onset depression history and a family history of depression. - Precipitating: the breakup. - Perpetuating: withdrawal and inactivity, and the belief "no one wants me around." - Protective: her sister and her job. - **Target:** withdrawal and negative self-beliefs, through behavioral activation and cognitive work.

Vignette B (Omar): - Predisposing: growing up in a violent home, and combat trauma. - Precipitating: the car accident. - Perpetuating: avoidance of driving and of memories, nightly drinking, and "keeping busy." - Protective: his wife and the veterans' group. - **Target:** avoidance and alcohol use, through integrated trauma-focused treatment after stabilization.

Vignette C (Keisha): - Predisposing: an anxious temperament. - Precipitating: the move. - Perpetuating: parental accommodation through reassurance. - Protective: her friend and her love of art. - **Target:** reassurance-seeking and accommodation, through parent coaching to reduce accommodation plus CBT with gradual exposure.

Drill 5 — Risk Formulation

1. **Low.** Passive ideation without plan, intent, or history, plus strong protective factors. Outpatient care, a collaborative safety plan, and follow-up.
2. **Moderate.** A specific method with access to means, but no intent and no history. A safety plan with means restriction (secure or remove the pills), more frequent contact, and involving his roommate with consent. Consider a higher level of care if the risk changes.
3. **High.** A specific plan, a firearm, a timeline, intent, recent loss, and alcohol use. Do not leave him alone, and arrange an emergency evaluation or hospitalization. Address the firearm.
4. **High.** A recent attempt plus a warning sign (giving away her pet); a sudden "I'm fine" does not lower risk.

Arrange an emergency evaluation and involve her parents.

5. **Moderate, and rising.** Chronic passive ideation with new dynamic risk factors: job loss, daily drinking, and stopping medication. Reassess now, update the safety plan, address the drinking, coordinate with his prescriber, and increase contact.

Drill 6 — Mental Status Exam

1. Tangential thought process (thought process).
2. Pressured speech (speech).
3. Mood–affect incongruence: reported mood "great," tearful affect (mood and affect).
4. Delusion with ideas of reference (thought content).
5. Auditory hallucination (perception).
6. Circumstantial thought process (thought process).
7. Flat or blunted affect (affect).
8. Disoriented to time and place (cognition).

Drill 7 — Rewrite the Objective (model answers)

1. *Not measurable.* → Client's GAD-7 will drop from 16 to below 10 within 10 weeks.
2. *Describes the counselor, not the client.* → Client will use a distress tolerance skill (for example, TIPP) during at least 3 of 4 urges to self-harm, recorded on a diary card, within 6 weeks.
3. *Absolute, and may not match readiness.* → Client will record daily drinks and reduce from 6 to 2 or fewer on at least 5 days per week within 4 weeks.
4. *Vague.* → Client and his son will spend 30 minutes of one-on-one time together twice weekly for 6 weeks, reported in session.
5. *Not observable.* → Client will be able to name three triggers for her trauma reactions and one grounding skill for each by session 6.
6. *Not measurable.* → Client's sleep diary will show falling asleep within 30 minutes on at least 5 nights per week within 6 weeks.

Drill 8 — The Empathy Ladder

1. a = S (shifts to congratulations and misses the feeling); b = I; c = A.
2. a = A; b = S (excuses the children and minimizes the hurt); c = I.
3. a = S (reassures instead of understanding); b = A; c = I.
4. a = A; b = I; c = S (praises instead of hearing the burden).

Drill 9 — Crisis Sequencing

Correct order: **(c), (f), (a), (d), (e), (g), (b)**, following Roberts' seven stages (Roberts, 2005):

1. Assess safety and lethality (c).
2. Make psychological contact (f).
3. Identify the major problems (a).
4. Deal with feelings (d).
5. Explore alternatives (e).
6. Make an action plan (g).
7. Follow up (b).

In practice, stages 1 and 2 happen almost at once. Calm, respectful contact makes the safety assessment possible.

Drill 10 — Ethics Quick-Fire

1. **False.** Five years, and only if the relationship is not exploitive (ACA, 2014, Standard A.5.c).
2. **False.** Informed consent is ongoing (Standard A.2.a).
3. **False.** Referral on values grounds alone is prohibited (Standard A.11.b).
4. **True** (Standard H.1.b).
5. **False.** Reasonable suspicion is the reporting threshold.
6. **True** (HIPAA).
7. **True** (Standard A.10.e).
8. **False.** Impaired counselors limit or suspend services and arrange coverage (Standard C.2.g).
9. **True** (Standard A.11.c).

10. **False.** Counselors seek the minor's assent (Standard A. 2.d).
11. **False.** Corrections are made as dated addenda. Never alter or delete the original.
12. **True** (Standard H.3).
13. **False.** Use a qualified interpreter (Standard A.2.c). Family members, especially children, compromise accuracy and confidentiality.
14. **True** (Standard B.2.c).
15. **False.** Withhold identifying information unless the client has consented.

Drill 11 — ADDRESSING (model answer)

1. **Factors:**
   - Age and generation (67, retired)
   - Disability acquired later in life (a stroke; he uses a wheelchair)
   - Religion (a church deacon)
   - Ethnic and racial identity, and national origin (Ghanaian immigrant)
   - Socioeconomic status (retired bus driver)
   - Gender (the belief that "men do not complain")
   - Family context (living with his daughter; fear of being a burden)
2. **Open questions:** "What does it mean to you, as a man in your culture, to be here talking about how you feel?" and "How does your faith help you — or weigh on you — in times like this?"
3. **Counselor's own identities:** A younger or US-born counselor may carry assumptions about aging, disability, or help-seeking, or hold privilege Samuel does not. The counselor reflects on this, stays curious, and takes it to supervision if it affects the work.
4. **Other tools:**
   - Risk formulation: older men with late-life disability and a sense of being a burden carry elevated suicide risk.

- Case conceptualization (the 4 Ps): the loss of role and independence after the stroke may be perpetuating his low mood.
- Developmental fit: the stage of integrity versus despair.

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Complete all questions in an exam before turning to the answer key at its end. The keys identify the best answer, explain why each distractor is wrong, and name the clin-

ical error each wrong option represents — the same analysis the NCMHCE rewards.

Practice Exam One

10 clinical cases · 130 questions · 225 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

Case 1 of 10

Low mood and lost interest after a demotion

Diagnosis: Major Depressive Disorder, Moderate

Intake: Daniel Okafor, a 34-year-old warehouse supervisor, presents reporting six weeks of low mood, loss of interest in coaching his daughter's soccer team, poor sleep, and difficulty concentrating after a demotion.

Session Two: He describes waking at 4 a.m. most days, eating little, and feeling worthless and slowed down. He denies any past period of elevated, expansive mood or decreased need for sleep with increased energy.

Session Three: He reports passive thoughts that his family would be better off without him but no plan, intent, or prior attempts, and he is frightened by these thoughts and willing to work on them collaboratively.

Question 1. What should the counselor confirm first to anchor the major depressive episode diagnosis at intake?

A. That he can identify the single external stressor he believes is responsible for everything he is feeling

B. That his family history includes at least one first-degree relative who was treated for a depressive disorder

C. That his current depressive symptoms are clearly more severe today than they were at the very first onset

D. That symptoms have persisted nearly every day for at least two weeks and impair his daily functioning

Question 2. The counselor has some training but limited experience delivering behavioral activation; what is the best step before using it with Daniel?

A. Use behavioral activation with Daniel anyway and rely on brief online summaries to guide each session

B. Seek consultation from a colleague experienced in behavioral activation while you build skill in the approach

C. Refer Daniel at once to another counselor rather than use any method you have not yet practiced on your own

D. Share Daniel’s full name and history with a colleague so the consultation is as specific as it can possibly be

Question 3. Daniel says he wants his family to help with his recovery; how should the counselor best collaborate with them?

A. Contact his family on your own to describe his diagnosis and passive suicidal thoughts so they can watch him

B. Tell Daniel that families should stay out of counseling so his treatment remains fully separate from home life

C. With his signed release, invite family to a session focused on his goals, warning signs, and ways to support him

D. Ask his family to schedule all of his activities and to report on his mood to you weekly without him present

Question 4. Given his passive thoughts of being better off dead, what is the most appropriate assessment step?

A. Ask him to promise in writing that he will not act on any of these thoughts before the next scheduled appointment

B. Reassure him that thoughts like these are extremely common in depression and will almost certainly fade with time

C. Defer any discussion of the thoughts until rapport is stronger so that he does not feel judged or pathologized today

D. Use a structured suicide-risk screen to clarify ideation severity, any intent, plan, and prior behavior

Question 5. What additional history is most important to gather to avoid misattributing the depressive picture?

A. A full account of every job he has held so the counselor can understand his complete occupational and earnings history

B. A review of his medications, substance use, and any thyroid or other medical conditions that mimic depression

C. A detailed genogram tracing relationship patterns and emotional cutoffs across at least three prior family generations

D. A list of the specific coping strategies that have failed him so the counselor can rule each of those approaches out

Question 6. Daniel lists his mood, sleep, lost interest in coaching, and his job as concerns; how should the counselor help him prioritize them?

A. Start with the demotion, since it triggered the episode and resolving work stress should lift the other symptoms

B. Work on every concern at once in the early phase so that no part of his presentation is neglected or left behind

C. Rank his concerns privately by clinical severity and present him with the finished order at the next appointment

D. Address safety first, then rank his other concerns together by their distress, impairment, and his own priorities

Question 7. How should the counselor track whether the treatment is actually working over time?

A. Wait until the agreed course of therapy has fully ended and then evaluate his overall response in a single review

B. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

C. Ask him at each session whether he subjectively feels that things are generally going better than they were before

D. Track only whether he is consistently attending his scheduled sessions and completing the assigned homework each week

Question 8. Daniel asks about medication. What is the most appropriate counselor action?

A. Provide general information and coordinate a referral to a prescriber for a medication evaluation with his consent

B. Suggest that he research antidepressant options online and bring his own preferred choice to a future appointment

C. Advise him that medication is generally unnecessary as long as he stays fully committed to attending his therapy

D. Tell him which specific antidepressant tends to work best for people who present with his particular symptom pattern

Question 9. Which initial behavioral activation step best fits Daniel’s presentation?

A. Have him keep a detailed daily log of every negative automatic thought before introducing any change to his routine

B. Direct him to resume the full coaching role and every other prior responsibility right away to prove he can still cope

C. Collaboratively schedule small, valued activities such as briefly attending his daughter’s practice to rebuild reward

D. Encourage him to wait to re-engage in any activities until he genuinely feels his interest and energy have returned

Question 10. How can the counselor best address Daniel’s belief that his depression makes him a failure?

A. Offer psychoeducation framing depression as a common, treatable condition while validating how heavy it currently feels

B. Redirect him away from the topic entirely and refocus the conversation on practical goal-setting for the coming week

C. Agree that the demotion was a genuine setback and focus the work on helping him accept that he underperformed at work

D. Firmly challenge the belief as irrational and present him with logical evidence from his work history that he is objectively not a failure

Question 11. Daniel says he doubts anything will help. What is the most therapeutic response?

A. Explain in detail the neurobiology of depression to convince him on scientific grounds that recovery is fully possible

B. Acknowledge the hopelessness as a symptom and collaboratively set one small, achievable goal to test it against

C. Promise him with confidence that therapy is guaranteed to work as long as he fully commits to attending every session

D. Accept his prediction that treatment is unlikely to help and shift the focus toward simply managing his daily symptoms

Question 12. Later, Daniel misses a session, and the practice has a written no-show protocol for clients with recent suicidal ideation; what should the counselor do?

A. Follow the practice's protocol, reaching out by his consented contact method and documenting each step taken

B. Wait for him to reschedule on his own, since reaching out could feel intrusive and disrespectful to an adult client

C. Request a police welfare check right away, since any missed session after suicidal thoughts signals imminent risk

D. Call his workplace to ask whether he came in today, since his supervisor would know whether he is currently safe

Question 13. Later in treatment, during an in-office session with Daniel, the building's fire alarm sounds; what should the counselor do first?

A. Follow the practice's emergency preparedness plan, leaving with Daniel by the posted route to the set meeting point

B. Keep the session going until the alarm is confirmed, since stopping now could interrupt his disclosure of risk

C. Send Daniel out of the building alone while you stay behind to lock his paper chart and the office files

D. Call Daniel's family to come pick him up right away, since the sudden disruption may sharply raise his risk

Case 2 of 10

Persistent worry across many areas of life

Diagnosis: Generalized Anxiety Disorder

Intake: Hassan, a 41-year-old civil engineer, reports about eight months of constant worry he cannot switch off. He worries about work deadlines, his children, finances, and his health, and describes feeling keyed up, tired, and tense, with poor sleep and trouble concentrating at the office.

Session Two: Hassan says the worry jumps from one topic to the next and that he cannot control it once it starts. He reports muscle tension, irritability, and lying awake replaying conversations. He denies discrete panic attacks and denies that the worry is about having a specific undiagnosed illness.

Session Three: Hassan frequently asks for reassurance, double-checks his work, and avoids opening bills until the last moment. He says caffeine helps him push through fatigue, and he drinks several cups daily. He is motivated to feel calmer but doubts that anything will quiet his mind.

Question 1. As Hassan’s intake points to generalized anxiety, how should the counselor best build current knowledge of empirically supported interventions for it?

A. Rely mainly on the approach learned in graduate school, since core counseling methods change little over time

B. Review current practice guidelines and peer-reviewed outcome research on GAD and train in the indicated approaches

C. Adopt the newest technique trending on social media, since recent popularity signals strong evidence of benefit

D. Ask Hassan to research GAD treatments on his own and then use whichever approach seems most appealing to him

Question 2. Which question best establishes the duration criterion for this disorder?

A. Whether he can identify the single life event that first triggered his worry

B. Whether the excessive worry has been present more days than not for six months or longer

C. Whether his family members have also struggled with similar anxious feelings

D. Whether his worry became noticeably worse during the past one or two weeks at work and at home

Question 3. Later, Hassan starts a medication from his physician and reports feeling drowsy at work; how should the counselor respond?

A. Advise him to lower his dose until the drowsiness passes, since the side effect is interfering with his job

B. Leave his report out of the record, since medication effects belong only in the prescribing physician’s chart

C. Record his reported effects in the progress note and, with his consent, share them with the prescribing physician

D. Tell him drowsiness is a normal side effect that will fade, so there is no need to note it or report it to anyone

Question 4. Hassan asks for a copy of his counseling records; how should the counselor respond under federal law?

A. Refuse the request, since clients may obtain their counseling records only through a court order or subpoena

B. Release the separately kept psychotherapy notes too, since federal law requires disclosing every note the counselor keeps

C. Require him to explain why he wants the records and release them only if the counselor judges his reason valid

D. Provide his records within HIPAA's required time frame, noting that separately kept psychotherapy notes are excluded

Question 5. Hassan's insurer denies further sessions although his GAD-7 scores remain elevated; what is the most appropriate advocacy step?

A. With his consent, file an appeal documenting medical necessity using his GAD-7 scores and treatment response

B. Keep seeing him but bill under a more severe diagnosis that the insurer will be more likely to approve for him

C. Tell him the insurer's decision is final and end treatment at the last authorized session without other steps

D. Post about the insurer's denial on the practice's social media page to pressure the company into reversing it

Question 6. To support Hassan's possible referral for obsessive-compulsive symptoms, how should the counselor maintain referral resources?

A. Rely on the list compiled when the practice opened, since established providers rarely change their services

B. Keep the list current by verifying providers' specialties, licensure, availability, and insurance on a schedule

C. Refer him to the first name found in an online directory, since he can judge the provider's fit after one visit

D. Keep only providers who send referrals back to the counselor, since reciprocal relationships ease care coordination

Question 7. Which target is most central to treating Hassan's worry?

A. Helping him gather more information and double-check each concern so he can finally feel certain about every worry

B. Encouraging him to avoid the situations that tend to set off his worry

C. Focusing only on relaxing his body without addressing his worry process

D. Building his tolerance of uncertainty instead of chasing certainty through checking and reassurance

Question 8. How should the counselor address Hassan's frequent reassurance-seeking and avoidance?

A. Use worry exposure while gradually reducing reassurance-seeking and the avoidance that maintains it

B. Allow him to keep avoiding stressful tasks until he feels completely ready

C. Tell him firmly to stop worrying and to simply think more positively each day

D. Provide the reassurance he requests each and every time he asks so that he feels fully supported in session

Question 9. How should measurement-based care be built into Hassan's plan?

A. Re-administer the GAD-7 at intervals to track response and step up care if he does not improve

B. Have him track only his daily coffee intake and use that single number as the main indicator of his anxiety

C. Judge progress only from his general sense of how the week felt to him

D. Wait until the final session to find out whether the treatment helped him

Question 10. In session, Hassan repeatedly asks, "But what if something goes wrong?" seeking reassurance. What is the best response?

A. Tell him his worries are irrational and that he should already know better

B. Provide a clear, confident answer each time he asks so that he leaves the session feeling calm and reassured

C. Change the subject quickly so that the worry does not take over the session

D. Name the reassurance pattern with warmth and help him practice sitting with the uncertainty

Question 11. Hassan says he doubts that anything will quiet his mind. Which response best strengthens engagement?

A. Warn him that without therapy his anxiety will only continue to get worse

B. Reassure him broadly that many people feel this way and that it will pass

C. Explore what he has already tried, affirm his strengths, and evoke his own reasons to keep going

D. Insist that the treatment will definitely work as long as he follows every instruction

Question 12. Hassan's employer calls and asks the counselor to confirm he is in treatment. How should the counselor respond?

A. Decline to confirm or deny anything and explain that disclosure requires his written consent

B. Refer the employer to Hassan directly and avoid documenting that the call occurred

C. Share a brief progress summary so the employer can support him at work

D. Confirm only that he is attending sessions, since that single detail seems harmless

Question 13. Hassan also describes possible obsessive-compulsive symptoms the counselor has limited training to treat. What is the most ethical action?

A. Continue treating all of it alone to maintain continuity of the relationship

B. Seek supervision or consultation, and refer for the components that exceed the counselor's demonstrated competence

C. Tell him those symptoms are not really a problem worth addressing in counseling

D. Transfer him immediately to another provider without any explanation, consultation, or coordinated referral planning

Case 3 of 10

Escalating evening drinking after the kids leave home

Diagnosis: Alcohol Use Disorder, Moderate

Intake: Marcus Delgado, a 50-year-old accountant, self-refers to outpatient counseling reporting that his evening drinking has crept up steadily over the past year since both of his adult children moved out, leaving long, unstructured evenings at home.

Session Two: He describes intending to have one or two beers most nights but often ending up with four or five to feel any relaxing effect, and needing noticeably more alcohol than a year ago for the same effect; he denies morning shakiness, sweating, nausea, or any legal or medical problems tied to drinking.

Session Three: He reports two attempts in the past six months to cut back to one or two drinks a night, each lasting only a few days, and a strong urge to pour a drink soon after dinner; his drinking now fills two to three hours of his evening compared with under one hour a year ago.

Question 1. What is the most important focus for the intake interview to establish Marcus's alcohol use pattern?

A. Focus mainly on his extended family's history of alcohol problems across several prior generations

B. Rely mainly on a short report from his wife about his drinking rather than interviewing Marcus about his own use

C. Systematically review the past 12 months against the DSM-5-TR substance use disorder criteria: amount, tolerance, cut-down attempts, and time spent

D. Ask mainly about any past legal consequences such as arrests or citations connected to his drinking history

Question 2. Given his reported drinking, what routine intake screening step should the counselor take?

A. Require inpatient medical detoxification before any outpatient counseling sessions can begin with him

B. Begin managing any withdrawal symptoms directly in session using relaxation and grounding techniques

C. Screen for withdrawal risk and medical complications, referring for a medical evaluation if any signs emerge

D. Skip any withdrawal screening entirely since he denies morning shakiness, sweating, or nausea at intake

Question 3. Early in treatment, Marcus asks to switch to video sessions on evenings he works late; what should the counselor do first?

A. Begin video sessions at his next appointment, since the consent he signed at intake covers any session format

B. Let him join from any app he prefers, including a personal social media messenger, so the sessions are convenient

C. Assess his suitability for telehealth, confirm his location and the counselor's licensure, and get telehealth consent

D. Decline video sessions, since telehealth is never appropriate for clients diagnosed with a substance use disorder

Question 4. Which finding set most clearly supports a moderate, rather than mild, severity specifier?

A. The observation that he now drinks beer most evenings instead of only on weekends as he once did

B. Five endorsed symptoms: drinking more than intended, time spent, two failed cut-down attempts, tolerance, and craving

C. His own subjective sense that his drinking feels somewhat heavier than it did when he was younger

D. The single fact that his drinking increased sometime after his children moved out of the family home

Question 5. Marcus describes evening drinking as normal among men in his family and circle, a cultural context the counselor knows little about; what is best?

A. Rely on Marcus as the sole source of cultural knowledge so that his counseling doubles as the counselor's training

B. Treat his description as minimization, since cultural explanations for drinking usually mask underlying denial

C. Refer him to a counselor who shares his background, since cultural differences make effective work unlikely

D. Explore what these norms mean to him and seek training on how culture shapes drinking and help-seeking

Question 6. Given Marcus's ambivalence, what is the most appropriate treatment-planning approach?

A. Insist that he commit to complete abstinence starting immediately as a condition of continuing treatment

B. Wait passively without any active planning until he independently decides on his own that change is necessary

C. Tell him to simply try harder using the same approach that already failed him on two prior occasions

D. Use a motivational, stage-matched approach that meets him at his current level of readiness to change

Question 7. Which level-of-care determination is most appropriate for Marcus at this point?

A. Outpatient counseling, since he has no withdrawal risk, medical complications, or safety concerns present

B. No formal level-of-care placement at all, since he can simply be told to monitor his own drinking informally

C. A residential program lasting several months so he is fully removed from his home drinking environment

D. Medically managed inpatient withdrawal, since any regular drinking pattern always requires hospital admission

Question 8. How should the counselor best set an initial drinking-related goal with Marcus?

A. Postpone any goal discussion indefinitely until he raises the topic of drinking again entirely on his own

B. Set a goal based only on what worked well for a different client the counselor treated in the past

C. Assign lifelong total abstinence as the only treatment goal without first discussing his own preferences

D. Collaboratively explore his goals and readiness together, rather than the counselor selecting the goal alone

Question 9. What is the best way to monitor Marcus's progress once treatment begins?

A. Ask him only once, at the very end of treatment, whether he generally feels his drinking has improved

B. Have his wife independently report on his drinking each week instead of asking Marcus directly himself

C. Track only whether he attends each scheduled session, without discussing his drinking pattern directly

D. Have him track daily drinks consumed and urges using a simple log reviewed together at each session

Question 10. Before Marcus's next session, what is the best use of the counselor's preparation time?

A. Review his last progress note, treatment plan goals, and drinking log data to plan the focus of the session

B. Skip reviewing his chart, since relying on memory keeps each session more natural and more client-centered

C. Review only his intake paperwork, since his original history matters most for every later session with him

D. Skim his notes at the start of the session while he waits, since reviewing them in real time saves prep time

Question 11. Marcus asks to pay out of pocket so his insurer will not learn of his alcohol treatment; how should the counselor respond?

A. Bill his insurer anyway, since a covered client cannot choose to pay privately for services his plan covers

B. Honor his request to self-pay in full, document the restriction, and submit no claims for those sessions

C. Bill the insurer under an anxiety code instead so that his alcohol diagnosis never appears on any claim form

D. Agree to self-pay but submit the claims later anyway, since insurers must receive a record of every session

Question 12. After months of a heavy caseload, the counselor notices dread and irritation before Marcus's sessions; what is the most appropriate response?

A. Push through the feelings without discussing them, since admitting fatigue could suggest the counselor is unfit to practice

B. Share the frustration with Marcus directly so that he understands how his drinking is affecting the counselor as well

C. Recognize possible burnout, seek supervision or consultation, and adjust self-care and caseload to protect his care

D. End treatment with Marcus right away and refer him elsewhere, since these reactions mean the work cannot continue

Question 13. Marcus asks whether he needs to see a physician about his drinking. Best counselor response?

A. Avoid answering his question directly and instead redirect the conversation back to his drinking goals

B. Explain that a brief medical evaluation is a reasonable step and offer to coordinate a referral with his consent

C. Tell him confidently that a physician visit is completely unnecessary since he denies any withdrawal symptoms

D. Suggest he research his own symptoms online and independently decide for himself whether a visit is warranted

Case 4 of 10

Intense relationships and recurrent self-harm in a graduate student

Diagnosis: Borderline Personality Disorder

Intake: Aisha Thompson, a 26-year-old graduate student, presents with a long-standing pattern of intense, unstable relationships, fear of abandonment, rapidly shifting self-image, and recurrent self-harm by cutting during periods of emotional distress.

Session Two: She describes relationships that swing between idealizing and devaluing others, chronic feelings of emptiness, and intense anger she struggles to control; her mood shifts within hours in response to perceived rejection rather than across weeks.

Session Three: She reports cutting her arms to relieve unbearable tension and has had two past low-lethality overdoses during breakups; she denies current intent but says the urges return whenever she fears someone close to her is pulling away from her.

Question 1. When Aisha asks to bring her mother, whose preferred language is not English, to a session about her safety plan, how should the counselor arrange communication?

A. Ask Aisha to interpret for her mother herself, since she knows both languages and understands the family context best

B. Use a free translation app on your phone during the session to avoid the cost and delay of arranging an interpreter

C. Proceed in English and simplify your wording, since her mother will probably understand enough to follow the session

D. Arrange a qualified professional interpreter bound by confidentiality and brief them on the session's purpose beforehand

Question 2. Aisha plans to spend the summer at her family's home in another state and asks to keep meeting by video; what should the counselor do?

A. Confirm you hold a license or other legal authorization, such as a compact privilege, in that state before meeting

B. Continue the sessions as usual, since her written consent to video sessions waives the other state's licensure rules

C. Continue the sessions as usual, since licensure rules apply only in the state where your own office is located

D. Switch her to phone calls for the summer, since audio-only sessions fall outside of state licensure requirements

Question 3. Given her recurrent self-harm and past overdoses, the most appropriate assessment step is to:

A. Focus the assessment narrowly on the specific physical wounds themselves rather than on her thoughts, intent, and triggers

B. Complete a structured suicide-risk assessment that distinguishes her self-harm urges from suicidal intent and plan

C. Assume that because her self-harm appears to be chronic and long-standing it does not require any formal ongoing assessment

D. Ask her to promise that she will not cut or harm herself again at any point before her next scheduled counseling session

Question 4. Distinguishing her self-harm from a suicide attempt most importantly requires assessing:

A. Exactly how frequently the self-harming behavior tends to occur across a typical week or month of her ongoing daily life

B. Whether the people closest to her are fully aware that she has been engaging in this self-harming behavior for some time

C. The precise depth and overall medical seriousness of the physical wounds that she has produced during her cutting episodes

D. Her intent and the function of the behavior, since cutting to relieve tension differs from acting with intent to die

Question 5. What co-occurring condition is most important to screen for given her history?

A. Co-occurring posttraumatic stress and depressive symptoms, which commonly accompany borderline personality disorder

B. An underlying neurodevelopmental disorder that could explain the impulsivity she has shown since she was a young child

C. A previously undiagnosed primary psychotic disorder that could fully account for the brief paranoid moments she sometimes has

D. A primary eating disorder that might be the true underlying driver of both her impulsivity and her emotional instability

Question 6. What is the most appropriate evidence-based treatment recommendation?

A. Open-ended supportive counseling without any particular structure, focused mostly on listening to whatever arises each week

B. Referral to a structured, evidence-based psychotherapy for borderline personality disorder such as dialectical behavior therapy

C. A primarily insight-oriented exploration of her early attachments as the single main route to relieving her current symptoms

D. Brief solution-focused work over a small number of sessions aimed at quickly resolving the presenting interpersonal conflicts she describes at intake

Question 7. For managing her recurrent suicidal and self-harm urges, the plan should center on:

A. A signed no-suicide contract in which she formally agrees in writing never to harm herself for the duration of treatment

B. A collaborative safety plan identifying warning signs, coping strategies, and supports she can use when urges escalate

C. A strict set of rules forbidding her from contacting the counselor whenever the self-harm urges happen to become stronger

D. An arrangement for her to be psychiatrically hospitalized each and every time that she experiences any urge to harm herself

Question 8. Between sessions Aisha frequently calls in crisis. The most appropriate plan element is to:

A. Make yourself personally available to her by phone at any hour of the day or night so that she never has to face a crisis alone

B. Refer her out of your practice entirely so that an on-call crisis line can take over all of the between-session contact instead

C. Tell her plainly that she may not contact you at all between sessions and that every concern must wait until the next appointment

D. Establish clear, consistent between-session contact guidelines and coach her in using skills during crises within that frame

Question 9. When Aisha alternates between idealizing and devaluing the counselor, the most therapeutic response is to:

A. Respond warmly when she idealizes you and noticeably more distantly when she devalues you so she learns about consequences

B. Point out to her directly that she is splitting and explain in detail how this same pattern is damaging all of her relationships

C. Maintain a steady, nonreactive stance and a consistent frame while validating the feelings underneath the shifts

D. Take the devaluation as accurate feedback about your work and substantially change your whole approach each time it happens

Question 10. After months of hearing Aisha describe cutting and overdoses, what should the counselor do on noticing intrusive images of her injuries and dread before sessions?

A. Dismiss the images as an ordinary part of the job that will fade on their own if you keep seeing her as scheduled

B. Assess your trauma exposure and symptoms as possible vicarious trauma and adjust caseload and self-care with support

C. Ask Aisha to stop describing her self-harm in detail so that the sessions feel less overwhelming for you to sit through

D. Transfer Aisha to another counselor immediately, since these reactions show you are no longer competent to treat her

Question 11. Aisha asks the counselor to be her friend outside of sessions because she trusts no one else. Best response?

A. Agree to occasional friendly contact outside of sessions because the therapeutic relationship is clearly so important to her

B. Decline curtly and move on quickly without acknowledging the feelings or the meaning behind her request for closeness

C. Warmly maintain the professional boundary, explain its protective purpose, and explore the longing for connection in therapy

D. Promise her that once the formal therapy has fully ended the two of you can certainly become close personal friends afterward

Question 12. The counselor notices unusually strong frustration with Aisha's frequent crises. The most appropriate step is to:

A. Begin the process of transferring her to another provider right away, since your frustration clearly means you can no longer help her

B. Disclose your frustration openly to her in session so that she can fully understand the real impact her behavior has on you

C. Address the frustration by setting noticeably firmer and more rigid limits with her in order to reduce the demands on yourself

D. Seek clinical supervision or consultation to manage the countertransference and protect the quality of her care

Question 13. Aisha discloses an urge to overdose tonight after a breakup but pleads with the counselor to keep it secret. Best response?

A. Explain the limits of confidentiality, assess imminent risk fully, and act to protect her safety as the situation requires

B. Wait until the next scheduled session to revisit the disclosure once she has had some time to fully calm down on her own first

C. Agree to keep the disclosure completely confidential so that she continues to feel safe being fully honest with you in the future

D. Reassure her that since this is most likely just her chronic pattern of urges there is no real need to take any further action tonight

Case 5 of 10

A week of soaring energy, big plans, and little sleep

Diagnosis: Bipolar I Disorder, Current or Most Recent Episode Manic

Intake: Aaron Hughes, a 30-year-old entrepreneur, is brought in by his sister, who reports that for the past week he has barely slept, talks rapidly, and has launched several grand business schemes, maxing out credit cards and approaching strangers with big plans.

Session Two: Aaron is energized and irritable when questioned, insisting he feels the best ever and needs little sleep. His speech is hard to interrupt, his thoughts race, and his sister notes a similar but milder episode last year followed by months of deep depression.

Session Three: He denies drug use, and a recent screen was negative; he has no fixed delusions but is grandiose about his abilities. His risky spending and driving worry his family, and he minimizes any concern, saying everyone else simply cannot keep up with him.

Question 1. What is most important to confirm to establish a manic episode?

A. A distinct week or more of elevated or irritable mood with increased energy plus several associated manic symptoms

B. A current full symptom count meeting the threshold for a discrete major depressive episode at this time

C. A specific stressful event that he believes is responsible for how he has been acting this week the specific stressful event that he believes is responsible for how he has been acting this week

D. A clear rise in the intensity of his elevated mood over the past day or two compared with earlier in the week

Question 2. When billing Aaron’s insurer for an extended crisis-focused session with his sister present, what best supports the reimbursement request?

A. Bill the highest-paying code available, since his acute mania clearly justifies the most intensive level of service

B. Submit the claim now and write the progress note later only if the insurer requests his records during an audit

C. List a less stigmatizing diagnosis on the claim so that the bipolar diagnosis does not follow him on his record

D. Document the session’s actual length, participants, risk assessment, and interventions to match the billed code

Question 3. In building Aaron’s treatment plan, how should the counselor weigh the counselor’s own strengths and limitations?

A. Identify what you can offer, such as relapse-prevention skills, and where his care needs psychiatric or crisis expertise

B. Assume your general counseling training covers all his needs, since mania mainly responds to a strong therapeutic alliance

C. Decline to work with him at all, since anyone with bipolar I disorder requires a specialist rather than a counselor

D. Rely on his sister’s view of whether you are helping, since family feedback is the best measure of your competence

Question 4. If Aaron agrees to use a smartphone mood-and-sleep tracking app between sessions, what is the most appropriate way to implement it?

A. Have the app automatically share all of his data with his sister so that she can intervene whenever his sleep drops

B. Review the app’s privacy and data-sharing terms with him, obtain informed consent, and use its data to support care

C. Rely on the app’s mood scores to decide when he is stable rather than continuing your own ongoing clinical assessments

D. Recommend any free app he likes without reviewing it, since mood-tracking apps carry no meaningful privacy concerns

Question 5. Given his risky behavior and irritability, what safety assessment is most important now?

A. Rely on his own reassurance that he feels great and is in full control, since he knows his own situation best of all

B. Assess danger to self and others, including suicide risk and the impulsive, high-risk behaviors he is currently engaging in

C. Focus only on whether he currently has a specific, detailed plan to end his own life and disregard the other risks for now

D. Defer any safety assessment until his mood has settled, since he is clearly far too elevated to answer the questions reliably

Question 6. What is the most appropriate immediate treatment-planning priority?

A. Start him on a mood-stabilizing medication yourself and arrange to monitor his blood levels over the coming weeks

B. Coordinate an urgent psychiatric evaluation for pharmacologic management of the acute mania and to address safety

C. Reassure the family that the episode will most likely pass on its own and simply schedule a routine follow-up in a month

D. Begin weekly insight-oriented psychotherapy to help him understand the deeper roots of his recent grand business schemes

Question 7. When Aaron talks rapidly over the counselor and grows irritable at questions, how should the counselor manage the counselor's own rising pulse and frustration?

A. Raise your voice to match his intensity so that he clearly recognizes you are firmly in control of the session

B. End the session abruptly whenever you feel frustrated, since continuing would only escalate the situation further

C. Notice your reaction, slow your breathing, and keep a calm, measured tone so the tension does not drive your response

D. Tell Aaron that his behavior is making you anxious so that he understands the effect he has on the people around him

Question 8. Aaron asks the counselor what mood stabilizer dose he should request. The most appropriate action is to:

A. Suggest a typical starting dose that tends to work for most people who are in the middle of an acute manic episode like his

B. Tell him to ask for the lowest possible dose so that the medication will not slow him down or dampen his current productivity

C. Say that medication dosing has nothing to do with counseling and decline to help him prepare for the conversation in any way

D. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them

Question 9. Aaron insists nothing is wrong and that his family is overreacting. The most therapeutic stance is to:

A. Confront him directly with all the evidence that he is manic until he is finally forced to admit that something is wrong

B. Avoid the disagreement entirely and redirect the conversation to neutral topics until he becomes more open to discussing it

C. Agree with him that his family is overreacting so that he feels understood and stays willing to keep working with you

D. Engage him respectfully, find shared goals such as protecting his business, and link safety steps to what he values

Question 10. How can the counselor best involve Aaron's family during this acute period?

A. Share the full details of his sessions with the family right away so they can take charge of managing all of his behavior

B. With his consent, provide psychoeducation about warning signs and engage the family in supporting the safety plan

C. Decline any family involvement at all and tell them this is entirely a private matter between you and the client alone

D. Encourage the family to take over his finances and his major business decisions for him without first discussing it with him

Question 11. Aaron's grandiose driving and spending raise safety concerns. When does confidentiality most clearly yield?

A. Whenever the counselor personally feels that disclosing the information would probably be in the client's own best interest

B. Never, because what a client shares in counseling must always remain completely confidential under all possible circumstances

C. When there is a serious, foreseeable risk of harm to him or others, disclosure may be required to protect safety

D. Whenever a family member simply asks for information, since concerned relatives are entitled to know how he is doing

Question 12. Once Aaron is stabilized, his sister, with his consent, asks how she can support his treatment goals; which step is most appropriate?

A. Ask his sister to report on him privately each week so that you can monitor his behavior without his knowledge

B. Let his sister set his treatment goals for him, since his judgment was too impaired during the episode to trust now

C. Agree with Aaron and his sister on specific roles, such as noting sleep shifts and spending limits, tied to his goals

D. Keep his sister out of goal-setting entirely, since involving family members undermines his autonomy as an adult

Question 13. Aaron wants the counselor to coordinate with the psychiatrist and his sister. What must come first?

A. Obtain his specific, informed consent for each party before sharing any of his protected health information

B. Begin coordinating with both parties immediately, since he has already said in session that he wants you to work with them

C. Coordinate only with his sister for now and decide about the psychiatrist later without discussing the matter with him

D. Share freely with the psychiatrist without a release because both of you are treating providers working on the same case

Case 6 of 10

Voices and growing suspicion in a college student

Diagnosis: Schizophrenia

Intake: Renee Walters, a 23-year-old college student, is referred after eight months of declining function. She reports hearing voices commenting on her actions, believes classmates are conspiring against her, and her speech at times becomes hard to follow.

Session Two: Her grades have collapsed and she has withdrawn from friends, stopped attending most classes, and shows flattened affect and little motivation. The hallucinations and suspicion have been present nearly continuously for months rather than only briefly.

Session Three: She denies sustained periods of elevated or depressed mood driving the symptoms, and a recent toxicology screen was negative. She is frightened by the voices, has limited insight that she is ill, and at times wonders whether life is worth continuing.

Question 1. What is most important to confirm to support a schizophrenia diagnosis?

A. A current full symptom count meeting the threshold for a discrete major depressive episode at this time

B. A specific stressful event on campus that she believes first triggered all of these experiences

C. A noticeable increase in the intensity of her hallucinations over the past few days compared with a week ago

D. Two or more active-phase symptoms with continuous signs of disturbance and functional decline lasting at least six months

Question 2. If the counselor realizes at intake that Renee is enrolled in a course the counselor teaches as an adjunct at her college, what is the most appropriate step?

A. Discuss the conflict openly with her and arrange a warm referral, since your teaching role compromises your suitability

B. Continue treating her and simply avoid grading her assignments, since the overlap is limited to one of her courses

C. Say nothing about the overlap so that her suspicion of classmates does not extend to you as her counselor as well

D. Ask her to drop your course so that you can keep serving as her counselor without any remaining conflict of interest

Question 3. Later, when an attorney's subpoena arrives for Renee's records in a legal matter involving her, what is the most appropriate first step?

A. Send her complete records to the requesting attorney right away, since a subpoena legally requires full compliance

B. Consult an attorney or your liability carrier, notify Renee, and release nothing without her consent or a court order

C. Ignore the subpoena entirely, since counseling records are always privileged and can never be reached by the courts

D. Prepare a summary giving your opinion of her sanity so that the court can use it to resolve the legal matter quickly

Question 4. When the treatment team suggests a structured psychosis rating scale for Renee, how should the counselor decide whether to administer it?

A. Administer it after reading the manual once, since any licensed counselor may use any published assessment instrument

B. Have a front-desk staff member give and score it, since the scale only requires reading the questions aloud to her

C. Confirm you have the training and qualification level the instrument requires before using it, or refer if you do not

D. Skip formal assessment altogether, since her symptoms are already obvious enough to make structured testing unnecessary

Question 5. Given her statement that she wonders whether life is worth continuing, the most appropriate step is to:

A. Move ahead with planning psychiatric referral first, assuming that treating the psychosis will resolve the thoughts on its own

B. Ask her to sign a brief written promise that she will stay safe and call the clinic if the thoughts get worse this week

C. Conduct a structured suicide-risk assessment, recognizing that suicide risk is elevated in early psychosis

D. Treat the comment as a fleeting and ultimately harmless part of the distress that her frightening voices are causing her

Question 6. What is the most appropriate first-line treatment recommendation?

A. Start her on an antipsychotic medication yourself and arrange to monitor her response and side effects over the coming weeks

B. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside psychosocial treatment and support

C. Recommend a course of weekly supportive counseling on its own as the complete treatment for her psychotic symptoms

D. Begin intensive insight-oriented psychotherapy to help her uncover the deeper meaning behind the voices she has been hearing

Question 7. How should the counselor respond when a colleague not involved in Renee's care asks to have her chart pulled up in the practice's shared EHR?

A. Pull up the chart and share it, since everyone in the practice already has access to the same EHR system anyway

B. Give the colleague your login so that they can view the chart directly without you needing to be involved at all

C. Print the relevant pages of her chart and leave them in the colleague's mailbox so that they can review them later

D. Decline, since EHR access requires a job-related need such as her care, and keep your own login credentials private

Question 8. What additional support is most appropriate to coordinate given her functional decline?

A. Case management and supported education or employment services to help her re-engage with her studies and daily roles

B. A strict daily schedule of demanding tasks designed to push her to return to her full prior workload as quickly as possible

C. A recommendation that she withdraw from school indefinitely and simply rest at home until all of her symptoms fully resolve

D. Detailed academic tutoring focused only on raising her grades, without addressing the symptoms driving the decline

Question 9. During sessions Renee is guarded and suspicious of the counselor. The most therapeutic stance is to:

A. Build a steady, trustworthy alliance with consistency and transparency, going at a pace she can tolerate

B. Reassure her repeatedly that her classmates would never conspire against her and that her fears are completely unfounded

C. Press her to explain her suspicions in full detail right away so that you can immediately correct the inaccurate beliefs

D. Go along with her belief that the classmates are conspiring so that she feels understood and keeps coming to her sessions

Question 10. When Renee describes the voices, the most therapeutic response is to:

A. Ask her detailed questions about exactly what the voices are instructing her to do so you can follow all of their commands

B. Acknowledge how distressing and real they feel to her without confirming or arguing about their objective accuracy

C. Tell her plainly and repeatedly that the voices are simply not real and that she needs to stop paying any attention to them

D. Agree with her that the voices are probably other real people somehow speaking to her from somewhere else nearby on campus

Question 11. A voice reportedly tells Renee to harm a specific classmate she names. The most appropriate action is to:

A. Wait to see whether she mentions the same threat again at the next session before taking any action or assessing the risk

B. Keep the disclosure entirely confidential, since anything she shares in counseling must always remain private no matter what

C. Immediately notify the classmate and the police without first assessing the seriousness or imminence of the actual risk

D. Assess the risk carefully and follow duty-to-protect obligations, which may require warning or protective steps

Question 12. Renee's parents call asking for full details of her treatment. She is 23. Best response?

A. Refuse any contact with the parents at all and tell them you cannot even acknowledge whether she is a client of yours

B. Share the full details with her parents, since family members are clearly going to be central to supporting her recovery

C. Give the parents a general update on how she is doing while leaving out the most sensitive specific clinical details of her care

D. Decline to disclose without her consent and, with her permission, explore appropriate, helpful family involvement

Question 13. The counselor has limited training in psychosis care. What is the most ethical course of action?

A. Transfer her immediately to another provider with no explanation, consultation, or coordinated plan for the transition of care

B. Continue to manage all aspects of her care alone in order to preserve the continuity of the relationship you have built with her

C. Seek consultation and coordinate with a treatment team, referring for the components beyond your demonstrated competence

D. Tell her the symptoms are not serious enough to need a psychiatric referral and keep seeing her for supportive talks only

Case 7 of 10

Lifelong distractibility and restlessness derailing work

Diagnosis: Attention-Deficit/Hyperactivity Disorder, Combined Presentation

Intake: Diego Morales, a 27-year-old graphic designer, presents with lifelong distractibility, disorganization, and restlessness that are now jeopardizing his job. He misses deadlines, loses track of tasks, interrupts others, and struggles to sit through meetings.

Session Two: He recalls similar problems since grade school — careless errors, fidgeting, and trouble finishing work — and report cards noting he was bright but unfocused. The difficulties show up both at work and at home and are not limited to one setting or one period.

Session Three: He denies discrete episodes of elevated mood or decreased need for sleep, and his concentration problems are chronic rather than episodic. He feels demoralized and calls himself lazy, though he works long hours trying to compensate for the disorganization.

Question 1. How should the counselor respond when Diego offers to redesign the counselor's practice website for free as thanks for his progress?

A. Thank him, decline the offer, and explore what the gesture means to him while keeping the relationship professional

B. Accept the offer, since free design work is a harmless gift that will strengthen the therapeutic alliance with him

C. Accept the work but pay him his usual rate so that the business arrangement remains fair to both parties going forward

D. Decline curtly and warn him that any further offers like this one will result in the end of his treatment

Question 2. Before using an AI note-taking tool to draft Diego's session notes, what must the counselor do first?

A. Begin using the tool and accept its drafted notes as written, since AI summaries of sessions are reliably accurate

B. Verify the tool is HIPAA compliant with a business associate agreement and obtain Diego’s informed consent to its use

C. Paste his session details into a free public chatbot, since removing his last name is enough to protect his identity

D. Start recording sessions for the tool without telling Diego, since note-taking is an internal matter for the practice

Question 3. What is the best step when the counselor feels impatient as Diego forgets tasks and catches the thought that he is not trying hard enough?

A. Tell Diego directly that he needs to try harder, since the counselor’s honest reaction may motivate him to change his habits

B. Ignore the reaction, since a counselor’s private thoughts have no real effect on the course of his treatment

C. Reflect on how personal beliefs about effort shape these reactions and bracket them so they do not bias his care

D. Refer Diego to another counselor right away, since this impatience means the work with him can no longer continue

Question 4. Diego brings his original grade-school report cards and ADHD rating scales completed by his mother; how should the counselor maintain these records?

A. Keep the originals in an open tray on the desk so they stay handy while his diagnostic evaluation is still under way

B. Photograph the pages on a personal phone so they can be reviewed at home while the evaluation summary is being drafted

C. Shred the report cards after reading them, since childhood school records do not belong in an adult client's file

D. Scan the relevant pages into his secure record, return the originals to him, and limit access to staff with a need

Question 5. What is most important to rule out before confirming the diagnosis?

A. A long-standing eating disorder that might be contributing to his low energy and his trouble concentrating at his job

B. An emerging neurocognitive decline that could explain the careless errors and forgetfulness he has shown since childhood

C. A previously undetected primary psychotic disorder that could account for his disorganization and trouble finishing tasks

D. Medical and substance contributors and other conditions, such as a sleep disorder or depression, that can impair attention

Question 6. A tornado warning is issued for the area during a session with Diego; what is the most appropriate response?

A. Finish the session first, since most warnings pass without harm and stopping now would disrupt his focus

B. End the session early and send him to his car, so he can drive home before the storm reaches the building

C. Direct him to the shelter area on his own while the counselor stays back to secure his paper file and laptop

D. Follow the practice's emergency plan, move with him to the designated shelter area, and check in afterward

Question 7. Which skills-based target is most appropriate for his presentation?

A. Externalizing organization through planners, reminders, task breakdown, and environmental structure to support follow-through

B. Encouraging him to keep all of his deadlines and task lists in his head so that he learns to strengthen his working memory over time

C. Teaching him deep relaxation techniques to use whenever he feels restless, without addressing his organization or planning

D. Advising him to take on even more projects at once so that he stays busy enough to avoid getting bored and distracted

Question 8. Diego asks which ADHD medication and dose he should request. The most appropriate action is to:

A. Explain that medication selection and dosing are the prescriber's role and offer, with consent, to coordinate his questions

B. Say medication is purely a medical matter and decline to help him prepare any questions at all for his upcoming appointment

C. Tell him to ask for the highest available dose so the medication is sure to keep him focused throughout his entire workday

D. Recommend the specific stimulant and dose that tends to work best for adults who present with his particular symptom pattern

Question 9. Diego calls himself lazy and feels demoralized about his difficulties. The most therapeutic response is to:

A. Agree that he does seem to lack discipline and encourage him to simply hold himself to a much higher standard going forward

B. Provide detailed statistics on how common adult ADHD is in the population to logically prove that he is not actually lazy

C. Move past the comment quickly and refocus the session on reviewing the organizational tools he has started to put in place

D. Reframe the difficulties as a treatable neurodevelopmental condition and highlight his real efforts to compensate

Question 10. How can the counselor best support Diego's follow-through between sessions?

A. Assign him a large, ambitious overhaul of his entire work system to complete on his own before the next appointment

B. Track only whether he attends his sessions and treat consistent, on-time attendance as the main sign that he is making real progress

C. Collaboratively set small, specific, achievable between-session steps and review them with a measurement-based check-in

D. Leave the between-session work entirely open-ended and trust that he will figure out the details on his own as he goes

Question 11. Diego worries that an ADHD diagnosis just gives him an excuse to fail. Best response?

A. Tell him not to worry about labels at all and move directly on to reviewing his organizational strategies for the week

B. Explain the detailed neuroscience of ADHD at length to convince him on technical grounds that it is a legitimate condition

C. Agree that a diagnosis can indeed become a convenient excuse and warn him not to rely on it to justify any future mistakes

D. Validate the worry and reframe the diagnosis as an explanation that guides effective strategies, not an excuse to give up

Question 12. Diego requests a letter for workplace accommodations. What is the most appropriate counselor action?

A. Send the letter directly to his employer yourself before discussing with him exactly what it will say or obtaining his consent

B. Refuse to provide any documentation at all and tell him that workplace accommodations are entirely outside the counseling role

C. Write whatever will most help him secure the accommodations he wants, even if it overstates how impaired he actually is

D. Provide accurate documentation within competence, with his consent, describing functional impacts relevant to accommodations

Question 13. Diego's employer calls asking whether he has a mental condition affecting his work. Best response?

A. Refuse to speak with the employer at all and hang up without explaining anything about confidentiality or the proper process

B. Decline to confirm or deny without a valid release and explain that disclosure requires his written consent

C. Confirm that he has ADHD, since the employer clearly already suspects it and is only trying to support him at work

D. Share a general summary of his condition so that the employer can decide what accommodations might be reasonable for him

Case 8 of 10

Intrusive thoughts and endless rituals

Diagnosis: Obsessive-Compulsive Disorder

Intake: Darnell, a 30-year-old teacher, reports intrusive thoughts that his hands are contaminated and that he might accidentally harm a student. To relieve the distress he washes his hands until they crack and checks his classroom repeatedly before leaving. The rituals now take more than two hours a day and make him late.

Session Two: Darnell knows the thoughts are excessive and does not want them, but he feels he must perform the rituals to prevent something terrible. The harm thoughts horrify him and are completely against his values. He has started avoiding being alone with students and feels ashamed and exhausted.

Session Three: Darnell asks repeatedly for reassurance that he is not a dangerous person. His partner has begun helping him check the apartment to save time, which has made the checking worse. He is motivated to get better and wonders whether medication or therapy would help him most.

Question 1. The counselor is trained in ERP but has never built imaginal exposure scripts for taboo harm thoughts like Darnell’s; what is the most appropriate step?

A. Consult a colleague skilled in imaginal exposure for harm obsessions, using de-identified details and keeping responsibility

B. Post Darnell’s harm thoughts on an online counselor forum, removing only his name, to gather a wide range of script ideas

C. Ask a consultant to take over writing and delivering his scripts so the counselor can avoid working with the harm content

D. Skip imaginal work and use only in-vivo handwashing exposures, since those are the skills the counselor already has

Question 2. Before starting exposure work with Darnell, how should the counselor carry out the practice's informed consent procedure?

A. Start the exposures first and obtain written consent later once he has seen that the method works for him

B. Review ERP's purpose, expected temporary distress, alternatives, and his right to decline, and document consent

C. Have his partner sign the consent form on his behalf, since the partner will be helping with practice at home

D. Skip formal consent because ERP is evidence-based and carries no meaningful risk for adult clients like him

Question 3. The counselor, still working toward full licensure, feels uneasy about Darnell's harm thoughts and unsure about the risk judgment; what is the best step?

A. Transfer Darnell to another counselor right away so that the harm content no longer has to be heard in session

B. Keep the doubts private so that the clinical supervisor does not begin to question the counselor's overall competence

C. Bring the case to the clinical supervisor to review the risk formulation and the counselor's reactions to the harm content

D. Ask Darnell to stop describing the harm thoughts in session until the counselor feels more comfortable hearing them

Question 4. What distinguishes Darnell's OCD from Generalized Anxiety Disorder?

A. He worries about more than one area of his life at times

B. He sometimes has trouble sleeping at night because his mind feels far too busy to settle

C. He feels anxious and tense during much of his typical day

D. He has specific intrusive obsessions with ritualized compulsions, not diffuse worry

Question 5. How does Darnell's OCD differ from Obsessive-Compulsive Personality Disorder?

A. He tends to be quite conscientious, careful, and thorough in nearly all of his daily life

B. He values order and high standards in the work that he does

C. He can be rather rigid about how certain tasks should be done

D. His obsessions and compulsions are unwanted and distressing, not a valued lifestyle

Question 6. Darnell asks that all reminders and messages go only to his personal cell phone, not his school district email; what does federal law require?

A. Keep using his district email, since providers may pick whichever contact method on file is most reliable

B. Agree only if he provides a written reason showing that messages to his district email would endanger him

C. Stop sending him reminders or messages entirely, since any electronic contact with clients violates HIPAA

D. Accommodate his reasonable request for confidential communications without requiring him to explain why

Question 7. How should the counselor explain the rationale for response prevention to Darnell?

A. Thinking positive thoughts whenever an obsession appears will cancel it out

B. Facing the fear without the ritual lets his anxiety fall on its own through habituation

C. Avoiding the triggers entirely will keep his anxiety at a low level

D. Performing the ritual a few more times will gradually wear out the urge

Question 8. Darnell's partner helps him check the apartment. How should the counselor address this?

A. Encourage the partner to keep helping with the checking so that his distress stays manageable

B. Tell the partner to immediately stop all support without any plan

C. Work to reduce the family accommodation and reassurance that keep the OCD going

D. Leave the partner out of treatment since OCD is an individual problem

Question 9. How should the counselor monitor Darnell's progress?

A. Ask him at each session whether he feels the rituals are getting a little better each week

B. Track his symptoms with a validated OCD measure and adjust the exposure hierarchy

C. Wait until the end of treatment to see whether the rituals changed

D. Count only how often he washes and ignore the other rituals entirely

Question 10. In session, Darnell repeatedly asks the counselor to confirm he is not dangerous. What is the best response?

A. Change the topic quickly so the reassurance request does not continue

B. Kindly limit the reassurance and support him in tolerating the uncertainty instead

C. Tell him his fear is irrational and that he should already know better

D. Firmly reassure him each and every time that he is definitely not a dangerous person at all

Question 11. Darnell is frightened that exposure work will be unbearable. What is the best response?

A. Start immediately with the most frightening exposure on the list so he sees that he can survive it

B. Give a clear rationale and build the exposure hierarchy together at a manageable pace

C. Tell him the fear is unfounded and that exposure is never difficult

D. Agree to skip exposure and rely on relaxation methods instead

Question 12. Darnell's intrusive thoughts include fears of harming a student. How should the counselor handle duty-to-protect considerations?

A. Report him to the authorities right away simply because he happened to mention having some thoughts of harm

B. Stop treatment until he can prove that he is not a danger to anyone

C. Warn the school about him just to be cautious about the students

D. Recognize the harm thoughts as ego-dystonic OCD symptoms, not intent, and avoid needless disclosure

Question 13. Darnell asks whether medication might help alongside therapy. What is the most appropriate action?

A. Explain that an SSRI is evidence-based for OCD and refer for evaluation with his consent

B. Tell him medication never helps OCD and that therapy alone is enough

C. Contact a prescriber about his case before he has consented to it

D. Recommend a specific medication along with a dose for him to begin taking on his own at home

Case 9 of 10

Acute suicidal crisis after a breakup and job loss

Diagnosis: Adjustment Disorder With Depressed Mood; Personal History of Suicidal Behavior

Intake: Jordan Ellis, a 28-year-old veteran, presents in acute distress one week after a breakup and a job loss, saying he sees no reason to keep going. He reports current thoughts of suicide, a plan involving a firearm at home, and a prior attempt two years ago.

Session Two: He describes intense hopelessness, poor sleep, and withdrawal from friends; he has been drinking more heavily this week. He says the firearm is accessible and that he has thought about using it, though part of him is frightened by how strong the thoughts are.

Session Three: He is ambivalent — he mentions his younger sister and his dog as reasons he has not acted — and he is willing to talk through options. He has not set a specific time, and he agrees to work collaboratively on staying safe over the coming days.

Question 1. What is the most appropriate first step in assessing Jordan’s presentation?

A. Conduct a structured suicide-risk assessment clarifying ideation severity, intent, plan, access to means, and prior attempts

B. Begin teaching him calming and relaxation techniques right away before gathering the details of his suicidal thoughts

C. Reassure him that the breakup and job loss are temporary setbacks and that he is certain to feel better before long

D. Refer him to a prescriber for medication first and postpone any direct discussion of the suicidal thoughts until his mood has stabilized

Question 2. How should the counselor distinguish Jordan’s presentation from nonsuicidal self-injury?

A. By focusing on whether he has visible physical injuries on his body from harming himself in the recent past

B. By asking whether the people in his life, such as family or close friends, are already aware that he has been having these kinds of thoughts lately

C. By assessing his intent, since acting with intent to die differs fundamentally from self-injury aimed at relieving tension

D. By determining how frequently he has had thoughts about harming himself across a typical week or month recently

Question 3. Given the firearm at home, what assessment is most critical to complete?

A. A precise calculation of how much alcohol he has consumed each day so the counselor can quantify his drinking pattern

B. A detailed history of every firearm he has ever owned so the counselor can document his complete background with weapons

C. A thorough exploration of the childhood experiences that he believes first shaped how he copes with painful emotions

D. A careful assessment of his access to lethal means, since available, lethal methods sharply elevate acute suicide risk

Question 4. What is most important to assess to form a balanced risk formulation?

A. Only the single most severe risk factor he reports, since the most serious item alone determines his overall level of risk

B. Both his risk factors and his protective factors, such as his ambivalence and the relationships he names as reasons to live

C. How well he is able to describe, in detail, the specific sequence of events that led up to this current crisis

D. Whether his current distress is objectively proportionate to the severity of the breakup and the job loss he experienced

Question 5. When building knowledge of empirically supported suicide interventions for Jordan, how should the counselor judge whether the research applies to him?

A. Weigh controlled trials and practice guidelines, such as the VA/DoD suicide-risk guideline, that include veterans at acute risk

B. Accept findings from studies of adolescents, since what lowers suicide risk in one age group transfers to every other age group

C. Choose the approach with the most published articles, regardless of study design or whether suicidal behavior actually fell

D. Rely on one dramatic case report heard at a workshop, since a vivid single success shows the method will work for him too

Question 6. What is the most appropriate cornerstone of the safety plan?

A. A collaborative safety plan identifying warning signs, coping strategies, supports, and crisis contacts he can use

B. A signed no-suicide contract in which he formally promises in writing not to harm himself before the next appointment

C. A firm instruction that he must call the counselor’s personal phone the very moment any suicidal thought enters his mind

D. A detailed schedule of pleasant activities for the week ahead intended to lift his mood enough that the thoughts simply fade

Question 7. How should the plan address the firearm at home?

A. Leave the firearm question alone entirely, since raising it might offend him and damage the therapeutic relationship you are building

B. Tell him that as a responsible adult it is entirely his own decision whether to keep the firearm accessible during this crisis

C. Counsel collaboratively on means restriction, such as temporarily storing the firearm away from the home with a trusted person

D. Advise him to simply unload the firearm and keep the ammunition stored in a separate drawer in the same bedroom at home

Question 8. On what basis should the counselor decide whether a higher level of care, such as hospitalization, is needed?

A. On avoiding hospitalization in all cases, since inpatient admission would only disrupt the therapeutic relationship you have built

B. On hospitalizing him automatically, since anyone who mentions suicide at all should always be admitted to an inpatient unit

C. On a clinical judgment weighing intent, plan, means, and his ability to maintain safety with the current plan and supports

D. On whether he explicitly asks to be hospitalized, since the decision should ultimately be left entirely up to the client himself

Question 9. What follow-up element most strengthens his safety after this session?

A. A plan to wait and see how he is doing at his next routine appointment in several weeks without any contact before then

B. A reminder that he can look up crisis resources online on his own if he happens to feel worse over the coming weekend

C. A suggestion that his ex-partner check in on him regularly, since reconnecting with her would clearly help him feel better

D. A warm handoff and a scheduled follow-up with caring contact rather than simply sending him out with a phone number

Question 10. If Jordan's next follow-up is held by video, which telemental health practice is most essential at the start of the session?

A. Proceed exactly as in office sessions, since the same safety procedures apply no matter where he is located

B. Confirm his current physical location and an emergency contact, and know crisis resources local to him

C. Ask him to keep his camera off so he feels less self-conscious while discussing his suicidal thoughts

D. Plan to call 911 in your own city if risk escalates, since the emergency happens during your session

Question 11. How can the counselor best instill hope without dismissing Jordan's pain?

A. Validate how unbearable things feel right now while helping him see the crisis and his options as changeable over time

B. Agree with him that his situation does look genuinely hopeless right now and focus only on getting through the next few hours

C. Promise him confidently that everything is definitely going to work out fine for him as long as he sticks with the treatment

D. Quickly change the subject to lighter topics whenever his hopelessness comes up so that the session does not feel so heavy

Question 12. Jordan worries that losing his job means he can no longer pay for sessions; what is the most appropriate response?

A. Keep billing his former employer’s health plan as usual, since the claims may still be paid for a while

B. Pause his care until new coverage is in place, since unpaid sessions cannot be offered ethically

C. Verify his current coverage and discuss options such as VA benefits or a sliding fee so care does not lapse

D. Code his sessions under a different diagnosis that his remaining coverage reimburses at a higher rate

Question 13. After assessment, Jordan can maintain safety with a plan and supports. How should the counselor document this?

A. Document the risk assessment, the formulation, the safety plan, means-restriction steps, and the clinical rationale for the disposition

B. Write a brief note simply stating that the client is fine now and that there is no further need for any ongoing concern

C. Record only that he agreed to a no-suicide contract and promised that he would not harm himself before the next appointment

D. Keep the details of the suicide assessment out of the chart entirely to protect his privacy and avoid stigmatizing him

Case 10 of 10

Recurrent binges without compensatory behavior

Diagnosis: Binge-Eating Disorder

Intake: Aaliyah Brooks, a 38-year-old nurse, presents with several months of recurrent episodes of eating unusually large amounts of food with a sense of loss of control, at least weekly. The episodes leave her feeling disgusted, guilty, and distressed, and she eats alone out of shame.

Session Two: She eats rapidly and past fullness even when not physically hungry, and she does not vomit, fast, over-exercise, or use laxatives afterward. There is no regular compensatory behavior following the binges, which distinguishes her pattern from a binge-purge cycle.

Session Three: She is at a higher body weight and reports low mood and shame about her eating, and she fears being judged or told simply to diet. She is motivated to get help and to understand why the binges happen and how to interrupt them rather than to pursue another restrictive diet.

Question 1. Aaliyah asks to meet by video because she feels too ashamed to sit in a waiting room; which platform choice is most appropriate?

A. Use a popular free consumer video app, since she already uses it and will feel most comfortable with it

B. Use your personal phone’s video calling, since a live call is not stored and therefore needs no protection

C. Use any platform she prefers, provided she signs a waiver accepting responsibility for privacy risks

D. Use a HIPAA-compliant platform with a signed business associate agreement and encrypted video sessions

Question 2. A psychotherapy session with Aaliyah runs 45 minutes; how should the claim and her record align?

A. Bill the 38–52 minute psychotherapy code (90834) and document actual start/stop times, necessity, and interventions

B. Bill the 53-minute-plus code (90837), since sessions with eating disorder clients are routinely complex and intensive

C. Bill the 16–37 minute code (90832) to stay conservative, since under-billing time protects you from any audit risk

D. Bill whichever psychotherapy code her plan reimburses best, since actual session time does not affect code choice

Question 3. The counselor suggests a calming app with guided urge-surfing exercises to help Aaliyah ride out binge urges between sessions; how should it be implemented?

A. Present the app as a replacement for her CBT sessions, since practicing urge surfing on her own will stop the binges

B. Practice an exercise with her in session, link app use to her binge triggers, and review how well it helped at each session

C. Tell her the counselor can see her app activity in real time, so she can use it to signal a crisis between sessions

D. Recommend the app without trying it together, since calming apps are self-explanatory and need no follow-up in session

Question 4. What co-occurring condition is most important to screen for given her presentation?

A. Co-occurring depression and suicide risk, which commonly accompany binge-eating disorder and shape the plan

B. A previously undetected primary psychotic disorder that might account for both her low mood and her shame about eating

C. A long-standing anxiety disorder that could be the sole and complete explanation for all of her binge episodes

D. An emerging neurocognitive decline that could explain why she keeps forgetting how much she has eaten lately

Question 5. What is the most appropriate counselor action regarding the medical aspects of her care?

A. Recommend that she begin a strict weight-loss diet right away as the primary intervention for her binge-eating disorder

B. Avoid any involvement of medical providers so that her counseling remains entirely separate from her medical care

C. Coordinate a medical evaluation and ongoing monitoring as part of a team while she engages in psychotherapy

D. Order laboratory tests and a metabolic panel yourself so that you can personally evaluate her medical status before treatment

Question 6. Aaliyah discloses that she has dyslexia and struggles to complete the written self-monitoring forms; what is the most appropriate action?

A. Refer her to another provider who specializes in learning disabilities before continuing her eating care

B. Drop self-monitoring from her plan, since the written forms are simply too demanding for her to complete

C. Offer alternatives such as audio-recorded or app-based logs with verbal review, at no added cost to her

D. Charge an additional fee for the extra session time that reviewing her logs verbally will now require

Question 7. A central target in eating-disorder-focused CBT for her would be:

A. Focusing mainly on relaxation skills so she feels calmer without addressing her eating patterns or beliefs

B. Establishing regular eating and reducing the dietary restraint and triggers that drive the binge episodes

C. Encouraging her to skip meals during the day so that she is able to save up her calories for later in the evening

D. Helping her design a stricter daily diet with tighter food rules so that she feels more in control of her eating

Question 8. Aaliyah asks whether medication could help. The most appropriate counselor action is to:

A. Recommend the specific medication and dose that tends to work best for people with binge-eating disorder like hers

B. Recommend an over-the-counter appetite suppressant so that she can reduce how much she eats during her binge episodes

C. Provide general information and coordinate a referral to a prescriber for evaluation with her consent, within scope

D. Discourage any medication and tell her she should overcome the binge eating through willpower and discipline alone

Question 9. Aaliyah is deeply ashamed and eats alone to hide her binges. The most therapeutic response is to:

A. Reassure her quickly that the binge eating is not really a big deal and that she should simply stop doing it on her own

B. Respond with a nonjudgmental, validating stance that reduces shame and supports her engaging openly in treatment

C. Tell her plainly that she simply needs more self-control and that the solution is to eat less and exercise more

D. Provide detailed statistics on how common binge eating is to logically prove to her that she should not feel ashamed

Question 10. Aaliyah fears the counselor will just put her on a diet. The most therapeutic response is to:

A. Reassure her that the very first step will be to put her on a strict calorie-controlled diet to get her eating under control

B. Clarify that the focus is on interrupting the binge cycle and the distress, not on prescribing another restrictive diet

C. Tell her that her weight is the real problem and that everything in treatment should be aimed at reducing it as fast as possible

D. Avoid answering her concern and steer the conversation toward reviewing her eating log for the week instead

Question 11. Aaliyah, distressed, mentions she sometimes feels life is not worth living because of her eating and her weight. Best step?

A. Ask her to sign a brief promise to stay safe and to reach out to you if those thoughts get any worse this week

B. Conduct a structured suicide-risk assessment and respond to the risk, recognizing eating disorders can carry elevated risk

C. Reassure her that things will improve with treatment and that there is no real need to discuss those thoughts further

D. Continue with the planned CBT agenda, assuming the comment was just a passing expression of her frustration today

Question 12. How should the counselor coordinate with Aaliyah’s primary care provider about her care?

A. Obtain her informed consent and release before communicating with the provider, then collaborate on a shared plan

B. Contact the provider right away to align the plan, since coordinating her care is clearly in her own best interest anyway

C. Share her full counseling record with the provider so that they have complete information about all of her treatment

D. Avoid any contact with the provider so that her counseling remains entirely separate from her medical care

Question 13. Aaliyah asks the counselor to keep her eating disorder out of any records her insurer might see. Best response?

A. Explain that accurate clinical documentation is required and discuss how records are protected and who may access them

B. Offer to keep a private, separate set of unofficial notes about her eating disorder outside of her formal clinical record

C. Agree to leave the diagnosis out of the record entirely so that she feels more comfortable continuing in treatment with you

D. Tell her she has no say at all in what goes into her record and move on without addressing her privacy concern further

Answer Key

Case 1: Low mood and lost interest after a demotion

Diagnosis: Major Depressive Disorder, Moderate

Q1 — Answer: D. That symptoms have persisted nearly every day for at least two weeks and impair his daily functioning

Why it is correct: Duration and impairment anchor the episode. *Common error:* Diagnosing from a single low mood report.

- **A** — Cause does not establish the diagnosis. *Trap:* Treating a trigger as the diagnostic test.
- **B** — Family history supports but does not confirm. *Trap:* Requiring family history to diagnose.
- **C** — Worsening is not required for the diagnosis. *Trap:* Demanding progression to diagnose.

Q2 — Answer: B. Seek consultation from a colleague experienced in behavioral activation while you build skill in the approach

Why it is correct: Consultation builds competence in a specific modality. *Common error:* Practicing a new modality without support.

- **A** — Informal summaries do not ensure competence. *Trap:* Learning a modality on the client.
- **C** — Referral is premature with training underway. *Trap:* Abandoning care instead of building skill.
- **D** — Consultation should protect his identifying information. *Trap:* Disclosing client identity during a routine consultation.

Q3 — Answer: C. With his signed release, invite family to a session focused on his goals, warning signs, and ways to support him

Why it is correct: Consented, goal-focused family involvement strengthens support. *Common error:* Involving family members without his consent.

- **A** — Disclosure without his consent breaches confidentiality. *Trap:* Sharing risk details without his consent.
- **B** — Excluding willing supports wastes a resource. *Trap:* Treating any family involvement as off-limits.
- **D** — Bypassing Daniel undermines his autonomy in treatment. *Trap:* Handing his treatment over to family.

Q4 — Answer: D. Use a structured suicide-risk screen to clarify ideation severity, any intent, plan, and prior behavior

Why it is correct: Structured screening clarifies risk. *Common error:* Asking vaguely or avoiding the topic.

- **A** — No-suicide contracts are not evidence-based. *Trap:* Substituting a contract for assessment.
- **B** — Premature reassurance closes assessment. *Trap:* Reassuring before assessing.
- **C** — Deferring risk assessment is unsafe. *Trap:* Postponing a safety assessment.

Q5 — Answer: B. A review of his medications, substance use, and any thyroid or other medical conditions that mimic depression

Why it is correct: Rule out medical and substance causes. *Common error:* Skipping the organic rule-out.

- **A** — Exhaustive work history is low yield. *Trap:* Collecting low-yield detail.
- **C** — Useful later, not the priority now. *Trap:* Prioritizing depth history over medical rule-outs.
- **D** — Coping review is secondary here. *Trap:* Confusing planning data with rule-outs.

Q6 — Answer: D. Address safety first, then rank his other concerns together by their distress, impairment, and his own priorities

Why it is correct: Safety first, then collaborative ranking of concerns. *Common error:* Setting treatment priorities without the client's input.

- **A** — The trigger is not automatically the priority. *Trap:* Assuming the precipitating stressor drives every symptom.
- **B** — Tackling everything at once dilutes focus. *Trap:* Avoiding the work of prioritization altogether.
- **C** — Unilateral ranking excludes the client from planning. *Trap:* Prioritizing for him rather than with him.

Q7 — Answer: B. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

Why it is correct: Measurement-based care guides decisions. *Common error:* Relying on impressions alone.

- **A** — End-only review misses early non-response. *Trap:* Skipping interim monitoring.
- **C** — Global impressions are unreliable. *Trap:* Trusting unstandardized self-report.
- **D** — Engagement is necessary but not sufficient. *Trap:* Mistaking adherence for outcome.

Q8 — Answer: A. Provide general information and coordinate a referral to a prescriber for a medication evaluation with his consent

Why it is correct: Refer and coordinate within scope. *Common error:* Giving specific medication advice.

- **B** — Outsourcing to self-research is poor care. *Trap:* Replacing referral with self-directed search.
- **C** — Discouraging meds overreaches. *Trap:* Steering a medical decision.
- **D** — Naming a drug exceeds scope. *Trap:* Overstepping professional scope.

Q9 — Answer: C. Collaboratively schedule small, valued activities such as briefly attending his daughter's practice to rebuild reward

Why it is correct: Graded valued activity rebuilds reward. *Common error:* Waiting for motivation before activity.

- **A** — Thought logs fit CBT, not BA first step. *Trap:* Confusing BA with cognitive work.
- **B** — Too much too soon invites failure. *Trap:* Ignoring graded pacing.
- **D** — Waiting on mood reverses BA logic. *Trap:* Letting mood gate behavior.

Q10 — Answer: A. Offer psychoeducation framing depression as a common, treatable condition while validating how heavy it currently feels

Why it is correct: Psychoeducation reframes self-blame. *Common error:* Either lecturing or only sympathizing.

- **B** — Avoidance leaves the belief intact. *Trap:* Avoiding emotionally central material.
- **C** — Endorsing the self-blame deepens it. *Trap:* Colluding with hopeless thinking.
- **D** — Premature disputation feels invalidating. *Trap:* Disputing before validating.

Q11 — Answer: B. Acknowledge the hopelessness as a symptom and collaboratively set one small, achievable goal to test it against

Why it is correct: Name hopelessness and test it behaviorally. *Common error:* Arguing him out of hopelessness.

- **A** — Lecturing rarely shifts hopelessness. *Trap:* Substituting facts for experience.
- **C** — Overpromising risks credibility. *Trap:* Making promises you cannot keep.
- **D** — Agreeing confirms the hopelessness. *Trap:* Adopting the client's hopeless stance.

Q12 — Answer: A. Follow the practice's protocol, reaching out by his consented contact method and documenting each step taken

Why it is correct: Follow the practice SOP and document outreach. *Common error:* Ignoring written protocols for at-risk clients.

- **B** — Passive waiting ignores the protocol and risk. *Trap:* Prioritizing politeness over the required safety procedures.
- **C** — Escalation should follow the protocol's steps. *Trap:* Equating a missed session with imminent danger.
- **D** — Calling his employer breaches his confidentiality. *Trap:* Contacting third parties without the client's consent.

Q13 — Answer: A. Follow the practice's emergency preparedness plan, leaving with Daniel by the posted route to the set meeting point

Why it is correct: Follow the established emergency plan without delay. *Common error:* Improvising or delaying during a real incident.

- **B** — Delaying evacuation endangers both you and Daniel. *Trap:* Treating the clinical moment as outranking safety.
- **C** — Paper records never outrank anyone's personal safety. *Trap:* Securing files before ensuring client safety.
- **D** — Contacting family breaches his confidentiality without need. *Trap:* Overreacting to risk with unconsented disclosure.

Case 2: Persistent worry across many areas of life

Diagnosis: Generalized Anxiety Disorder

Q1 — Answer: B. Review current practice guidelines and peer-reviewed outcome research on GAD and train in the indicated approaches

Why it is correct: Guidelines and research define empirically supported care. *Common error:* Relying on outdated or popular methods.

- **A** — The evidence base keeps evolving after graduate training. *Trap:* Treating past training as permanently current.
- **C** — Popularity is not evidence of efficacy. *Trap:* Mistaking a technique's trendiness for empirical support.
- **D** — Selecting interventions is the counselor's responsibility. *Trap:* Shifting clinical judgment onto the client.

Q2 — Answer: B. Whether the excessive worry has been present more days than not for six months or longer

Why it is correct: GAD requires 6+ months of worry more days than not. *Common error:* Diagnosing GAD from a brief recent stretch.

- **A** — A precipitant is not the duration criterion. *Trap:* Substituting a precipitant for course.
- **C** — Family history does not establish duration. *Trap:* Using family history to specify course.
- **D** — A short window does not establish chronicity. *Trap:* Confusing a recent flare with chronic course.

Q3 — Answer: C. Record his reported effects in the progress note and, with his consent, share them with the prescribing physician

Why it is correct: Document reported effects and coordinate with the prescriber. *Common error:* Ignoring or managing medication effects personally.

- **A** — Dose advice is outside counselor scope. *Trap:* Advising medication changes without the prescriber's input.
- **B** — Reported effects belong in the counseling record. *Trap:* Treating medication as only the physician's concern.

- **D** — Predicting a side effect's course exceeds counselor scope. *Trap:* Reassuring instead of documenting and coordinating.

Q4 — Answer: D. Provide his records within HIPAA's required time frame, noting that separately kept psychotherapy notes are excluded

Why it is correct: HIPAA grants access but excludes psychotherapy notes. *Common error:* Denying or overextending a client's access rights.

- **A** — Clients have a federal right of access. *Trap:* Requiring legal process for routine access.
- **B** — Psychotherapy notes are excluded from the access right. *Trap:* Confusing the record with process notes.
- **C** — Clients need not justify access requests. *Trap:* Gatekeeping access with the counselor's own criteria.

Q5 — Answer: A. With his consent, file an appeal documenting medical necessity using his GAD-7 scores and treatment response

Why it is correct: Data-supported appeals advocate within ethical bounds. *Common error:* Accepting denials or bending billing rules.

- **B** — Upcoding a diagnosis is insurance fraud. *Trap:* Misrepresenting the diagnosis to secure insurance coverage.
- **C** — Ending care without advocacy abandons him. *Trap:* Treating an insurance denial as beyond challenge.
- **D** — Public posting puts his confidentiality at risk. *Trap:* Advocating in ways that expose the client.

Q6 — Answer: B. Keep the list current by verifying providers' specialties, licensure, availability, and insurance on a schedule

Why it is correct: Verified, current resources enable appropriate referrals. *Common error:* Referring from an outdated or unvetted list.

- **A** — Provider specialties and availability change over time. *Trap:* Assuming an old list stays accurate.
- **C** — Unvetted referrals risk a poor fit. *Trap:* Shifting the vetting of providers onto the client.
- **D** — Reciprocity creates a conflict of interest. *Trap:* Basing referrals on the counselor's own business benefit.

Q7 — Answer: D. Building his tolerance of uncertainty instead of chasing certainty through checking and reassurance

Why it is correct: Intolerance of uncertainty maintains GAD. *Common error:* Trying to answer every worry instead of building tolerance.

- **A** — Chasing certainty reinforces the worry cycle. *Trap:* Solving each worry to provide certainty.
- **B** — Avoidance maintains anxiety. *Trap:* Recommending avoidance as coping.
- **C** — Relaxation alone leaves the core untreated. *Trap:* Treating arousal while ignoring worry.

Q8 — Answer: A. Use worry exposure while gradually reducing reassurance-seeking and the avoidance that maintains it

Why it is correct: Exposure plus dropping safety behaviors treats GAD. *Common error:* Leaving safety behaviors in place.

- **B** — Permitting avoidance prevents new learning. *Trap:* Waiting for readiness before facing tasks.
- **C** — Thought suppression is ineffective. *Trap:* Prescribing positive thinking as a fix.
- **D** — Giving reassurance reinforces the behavior. *Trap:* Reassuring to comfort in the moment.

Q9 — Answer: A. Re-administer the GAD-7 at intervals to track response and step up care if he does not improve

Why it is correct: Serial measures guide stepped care. *Common error:* Relying on impression instead of data.

- **B** — A single habit is not a symptom measure. *Trap:* Tracking a habit instead of symptoms.
- **C** — Global impression is less reliable. *Trap:* Substituting impression for measurement.
- **D** — End-only measurement cannot guide care. *Trap:* Deferring measurement to termination.

Q10 — Answer: D. Name the reassurance pattern with warmth and help him practice sitting with the uncertainty

Why it is correct: Address the pattern rather than feed it. *Common error:* Answering the worry to soothe it.

- **A** — Dismissiveness harms the alliance. *Trap:* Invalidating instead of guiding.
- **B** — Answering reinforces reassurance-seeking. *Trap:* Comforting by answering every worry.
- **C** — Avoiding the moment misses a chance to intervene. *Trap:* Avoiding the worry rather than using it.

Q11 — Answer: C. Explore what he has already tried, affirm his strengths, and evoke his own reasons to keep going

Why it is correct: Evoking strengths and change talk builds engagement. *Common error:* Lecturing instead of evoking motivation.

- **A** — Fear appeals tend to raise resistance. *Trap:* Using fear to drive change.
- **B** — Generic reassurance does little to engage. *Trap:* Offering generic comfort.
- **D** — Overpromising can backfire. *Trap:* Guaranteeing results to motivate.

Q12 — Answer: A. Decline to confirm or deny anything and explain that disclosure requires his written consent

Why it is correct: Disclosure to an employer needs written consent. *Common error:* Confirming treatment to an employer without consent.

- **B** — The contact should be documented. *Trap:* Failing to document a third-party call.
- **C** — Sharing progress without consent breaches confidentiality. *Trap:* Disclosing to be helpful.
- **D** — Confirming attendance is still a disclosure. *Trap:* Sharing attendance as if it were not protected.

Q13 — Answer: B. Seek supervision or consultation, and refer for the components that exceed the counselor's demonstrated competence

Why it is correct: Practice within competence; consult or refer. *Common error:* Treating beyond demonstrated competence.

- **A** — Treating beyond competence is unethical. *Trap:* Prioritizing continuity over competence.
- **C** — Minimizing the concern fails the client. *Trap:* Downplaying issues outside your skill set.
- **D** — Abrupt transfer mishandles continuity of care. *Trap:* Dropping the client without coordination.

Case 3: Escalating evening drinking after the kids leave home

Diagnosis: Alcohol Use Disorder, Moderate

Q1 — Answer: C. Systematically review the past 12 months against the DSM-5-TR substance use disorder criteria: amount, tolerance, cut-down attempts, and time spent

Why it is correct: Structured criterion review anchors the diagnosis. *Common error:* Diagnosing from a general impression instead of counting symptoms.

- **A** — Family history informs risk, not the diagnosis itself. *Trap:* Requiring family history to diagnose AUD.

- **B** — Bypassing the client's own report undermines assessment. *Trap:* Substituting collateral report for direct interview.
- **D** — Legal history is not present and not central. *Trap:* Assuming legal trouble is required for AUD.

Q2 — Answer: C. Screen for withdrawal risk and medical complications, referring for a medical evaluation if any signs emerge

Why it is correct: Routine withdrawal and medical screening is standard practice. *Common error:* Skipping the routine screen because none are expected.

- **A** — Inpatient detox is not indicated without withdrawal risk. *Trap:* Over-placing a client into a higher level of care.
- **B** — Managing withdrawal is outside counselor scope. *Trap:* Treating a medical concern as a counseling task.
- **D** — Screening should still be documented routinely. *Trap:* Omitting a routine screen based on assumption.

Q3 — Answer: C. Assess his suitability for telehealth, confirm his location and the counselor's licensure, and get telehealth consent

Why it is correct: Telehealth requires suitability, jurisdiction, and specific consent. *Common error:* Switching to video without telehealth safeguards in place.

- **A** — Telehealth needs its own informed consent. *Trap:* Assuming the intake consent already covers telehealth.
- **B** — Platforms must meet privacy and security standards. *Trap:* Choosing convenience over a HIPAA-compliant platform.
- **D** — SUD alone does not rule out telehealth. *Trap:* Applying a blanket exclusion without assessment.

Q4 — Answer: B. Five endorsed symptoms: drinking more than intended, time spent, two failed cut-down attempts, tolerance, and craving

Why it is correct: Five symptoms fall in the moderate range. *Common error:* Estimating severity without counting symptoms.

- **A** — Frequency alone does not set the specifier. *Trap:* Equating frequency change with severity level.
- **C** — Subjective comparison is not a diagnostic count. *Trap:* Substituting impression for symptom count.
- **D** — A precipitant does not establish severity. *Trap:* Using a life event to estimate severity.

Q5 — Answer: D. Explore what these norms mean to him and seek training on how culture shapes drinking and help-seeking

Why it is correct: Explore his meaning and close the counselor's own knowledge gap. *Common error:* Ignoring or outsourcing cultural knowledge gaps.

- **A** — The client should not carry the counselor's training. *Trap:* Making the client the only cultural educator.
- **B** — Dismissing cultural context harms the alliance. *Trap:* Pathologizing a client's cultural explanation as denial.
- **C** — Difference alone does not require referral. *Trap:* Referring out instead of building the counselor's own competence.

Q6 — Answer: D. Use a motivational, stage-matched approach that meets him at his current level of readiness to change

Why it is correct: Stage-matched motivational work fits ambivalence. *Common error:* Prescribing an action-stage plan to an ambivalent client.

- **A** — Mandating abstinence conflicts with his readiness level. *Trap:* Making treatment contingent on premature commitment.
- **B** — Passive waiting under-serves a moderate use disorder. *Trap:* Withholding active intervention for a diagnosable disorder.

- **C** — Repeating a failed strategy is not indicated. *Trap:* Recycling an approach that already failed.

Q7 — Answer: A. Outpatient counseling, since he has no withdrawal risk, medical complications, or safety concerns present

Why it is correct: Outpatient care matches his low-risk profile. *Common error:* Defaulting to a higher level of care without indication.

- **B** — An unplanned, informal approach under-serves a diagnosable disorder. *Trap:* Leaving a diagnosed disorder without a structured plan.
- **C** — Residential care exceeds what his profile requires. *Trap:* Choosing a more restrictive setting than needed.
- **D** — Inpatient withdrawal care is not indicated here. *Trap:* Over-placing a client without withdrawal risk.

Q8 — Answer: D. Collaboratively explore his goals and readiness together, rather than the counselor selecting the goal alone

Why it is correct: Collaborative goal-setting matches his current stage. *Common error:* Selecting the goal for the client unilaterally.

- **A** — Indefinite postponement stalls treatment planning. *Trap:* Avoiding goal-setting altogether.
- **B** — Borrowing another client's goal ignores individual readiness. *Trap:* Copying a plan instead of individualizing it.
- **C** — A unilateral abstinence mandate disregards readiness. *Trap:* Dictating the goal instead of collaborating.

Q9 — Answer: D. Have him track daily drinks consumed and urges using a simple log reviewed together at each session

Why it is correct: A structured drinking log gives objective tracking. *Common error:* Relying only on vague verbal check-ins.

- **A** — A single end-point check misses interim data. *Trap:* Skipping interim progress checks.
- **B** — Bypassing the client's own report is inappropriate. *Trap:* Replacing client self-monitoring with a third party.
- **C** — Attendance alone does not measure the target symptom. *Trap:* Mistaking attendance for symptom improvement.

Q10 — Answer: A. Review his last progress note, treatment plan goals, and drinking log data to plan the focus of the session

Why it is correct: Pre-session review ensures continuity and focus. *Common error:* Entering sessions without reviewing recent records.

- **B** — Memory alone misses details and trends. *Trap:* Trusting the counselor's recall over the written documentation.
- **C** — Intake data omits his recent progress. *Trap:* Reviewing outdated rather than current records.
- **D** — Skimming notes in session wastes his clinical time. *Trap:* Treating pre-session preparation as an optional step.

Q11 — Answer: B. Honor his request to self-pay in full, document the restriction, and submit no claims for those sessions

Why it is correct: HIPAA lets self-paying clients restrict plan disclosures. *Common error:* Billing insurers against a valid self-pay request.

- **A** — HIPAA permits this restriction when he pays in full. *Trap:* Assuming coverage mandates billing the plan.
- **C** — Changing the diagnosis code is fraud. *Trap:* Miscoding claims as a way to protect privacy.

- **D** — Submitting claims violates the agreed restriction. *Trap:* Agreeing to terms the counselor will not honor.

Q12 — Answer: C. Recognize possible burnout, seek supervision or consultation, and adjust self-care and caseload to protect his care

Why it is correct: Address burnout through supervision and self-care. *Common error:* Ignoring fatigue until it impairs care.

- **A** — Unaddressed fatigue risks impaired care for clients. *Trap:* Hiding signs of burnout to appear competent.
- **B** — Burdening the client this way misuses self-disclosure. *Trap:* Making the client manage the counselor's feelings.
- **D** — Abrupt referral is premature and disruptive. *Trap:* Terminating care instead of addressing the counselor's own fatigue.

Q13 — Answer: B. Explain that a brief medical evaluation is a reasonable step and offer to coordinate a referral with his consent

Why it is correct: Referral and coordination stay within scope. *Common error:* Assessing or ruling out medical issues personally.

- **A** — Deflecting a direct question undermines informed care. *Trap:* Avoiding a straightforward clinical question.
- **C** — A definitive medical ruling exceeds counselor scope. *Trap:* Making a medical determination as a counselor.
- **D** — Outsourcing to self-research is poor coordination of care. *Trap:* Replacing professional coordination with self-directed searching.

Case 4: Intense relationships and recurrent self-harm in a graduate student

Diagnosis: Borderline Personality Disorder

Q1 — Answer: D. Arrange a qualified professional interpreter bound by confidentiality and brief them on the session's purpose beforehand

Why it is correct: Qualified interpreters ensure accurate, confidential communication. *Common error:* Relying on family or improvised translation.

- **A** — Client interpreting distorts content and her role. *Trap:* Using the client as the interpreter.
- **B** — Consumer apps are inaccurate and risk privacy. *Trap:* Trading accuracy for convenience and cost.
- **C** — Partial understanding undermines the safety plan. *Trap:* Assuming comprehension without language access.

Q2 — Answer: A. Confirm you hold a license or other legal authorization, such as a compact privilege, in that state before meeting

Why it is correct: Licensure follows the client's physical location. *Common error:* Assuming your home-state license travels with the platform.

- **B** — Clients cannot waive state licensure law. *Trap:* Treating consent as a substitute for legal authorization.
- **C** — Jurisdiction is set by where the client sits. *Trap:* Assuming your office location governs licensure.
- **D** — Licensure applies to every modality. *Trap:* Believing audio-only care escapes licensure rules.

Q3 — Answer: B. Complete a structured suicide-risk assessment that distinguishes her self-harm urges from suicidal intent and plan

Why it is correct: Structured assessment of chronic risk. *Common error:* Treating chronic risk as unassessable.

- **A** — Wound-only focus misses risk data. *Trap:* Assessing injury without context.
- **C** — Chronic risk still needs assessment. *Trap:* Becoming desensitized to chronic risk.

- **D** — No-harm contracts are not evidence-based. *Trap:* Substituting a contract for assessment.

Q4 — Answer: D. Her intent and the function of the behavior, since cutting to relieve tension differs from acting with intent to die

Why it is correct: Intent and function separate the two. *Common error:* Assuming all self-harm is a suicide attempt.

- **A** — Frequency does not clarify intent. *Trap:* Equating frequency with risk type.
- **B** — Others' awareness is not the test. *Trap:* Using disclosure as the criterion.
- **C** — Lethality alone does not reveal intent. *Trap:* Inferring intent from wound severity.

Q5 — Answer: A. Co-occurring posttraumatic stress and depressive symptoms, which commonly accompany borderline personality disorder

Why it is correct: Screen common BPD comorbidities. *Common error:* Assessing the personality pattern in isolation.

- **B** — Neurodevelopmental framing is low yield. *Trap:* Chasing an improbable comorbidity.
- **C** — Transient paranoia is not psychosis. *Trap:* Overreading transient symptoms as psychosis.
- **D** — Eating pathology is not indicated. *Trap:* Inventing an unsupported comorbidity.

Q6 — Answer: B. Referral to a structured, evidence-based psychotherapy for borderline personality disorder such as dialectical behavior therapy

Why it is correct: Structured EBP such as DBT is indicated. *Common error:* Offering unstructured supportive care only.

- **A** — Unstructured support underperforms. *Trap:* Defaulting to unstructured care.

- **C** — Insight-only is not first-line. *Trap:* Prioritizing insight over skills.
- **D** — Brief therapy under-treats the pattern. *Trap:* Under-dosing a chronic condition.

Q7 — Answer: B. A collaborative safety plan identifying warning signs, coping strategies, and supports she can use when urges escalate

Why it is correct: Collaborative safety planning. *Common error:* Relying on a no-suicide contract.

- **A** — No-suicide contracts are not evidence-based. *Trap:* Using a contract instead of a plan.
- **C** — Cutting off crisis contact is unsafe. *Trap:* Removing crisis resources.
- **D** — Reflexive hospitalization is not the standard. *Trap:* Defaulting to hospitalization for every urge.

Q8 — Answer: D. Establish clear, consistent between-session contact guidelines and coach her in using skills during crises within that frame

Why it is correct: Consistent frame plus skills coaching. *Common error:* Either unlimited access or none.

- **A** — Unlimited access fosters dependence. *Trap:* Overextending availability.
- **B** — Offloading all contact is premature. *Trap:* Abandoning the therapeutic relationship.
- **C** — No access ignores her risk. *Trap:* Leaving a high-risk client without supports.

Q9 — Answer: C. Maintain a steady, nonreactive stance and a consistent frame while validating the feelings underneath the shifts

Why it is correct: Consistency plus validation. *Common error:* Reacting personally to the shifts.

- **A** — Reinforcing swings worsens instability. *Trap:* Letting the client's shifts drive your stance.
- **B** — Labeling can feel invalidating. *Trap:* Interpreting before validating.

- **D** — Overcorrecting destabilizes the frame. *Trap:* Abandoning the frame under pressure.

Q10 — Answer: B. Assess your trauma exposure and symptoms as possible vicarious trauma and adjust caseload and self-care with support

Why it is correct: Intrusive images and dread signal vicarious trauma. *Common error:* Ignoring cumulative exposure to client trauma.

- **A** — Unaddressed vicarious trauma tends to worsen. *Trap:* Normalizing symptoms as part of the job.
- **C** — Silencing her disclosures compromises risk assessment. *Trap:* Protecting yourself at her expense.
- **D** — Abrupt transfer risks abandonment. *Trap:* Treating a reaction as proof of incompetence.

Q11 — Answer: C. Warmly maintain the professional boundary, explain its protective purpose, and explore the longing for connection in therapy

Why it is correct: Hold the boundary, explore the need. *Common error:* Either crossing the line or rejecting coldly.

- **A** — A dual relationship harms the work. *Trap:* Trading the clinical role for friendship.
- **B** — A cold refusal can feel like rejection. *Trap:* Enforcing limits without warmth.
- **D** — Future-friendship promises are improper. *Trap:* Dangling a personal relationship.

Q12 — Answer: D. Seek clinical supervision or consultation to manage the countertransference and protect the quality of her care

Why it is correct: Use consultation for countertransference. *Common error:* Letting reactions silently shape care.

- **A** — Premature transfer abandons the client. *Trap:* Transferring instead of addressing the reaction.

- **B** — Burdening the client is inappropriate. *Trap:* Shifting your reaction onto the client.
- **C** — Reactive limits serve the clinician, not the client. *Trap:* Letting reactions drive boundary changes.

Q13 — Answer: A. Explain the limits of confidentiality, assess imminent risk fully, and act to protect her safety as the situation requires

Why it is correct: Confidentiality yields to imminent risk. *Common error:* Promising secrecy when risk is imminent.

- **B** — Delaying response to imminent risk is unsafe. *Trap:* Postponing an urgent safety response.
- **C** — Secrecy at imminent risk is unsafe. *Trap:* Prioritizing secrecy over safety.
- **D** — Dismissing imminent risk is dangerous. *Trap:* Assuming chronic means non-acute.

Case 5: A week of soaring energy, big plans, and little sleep

Diagnosis: Bipolar I Disorder, Current or Most Recent Episode Manic

Q1 — Answer: A. A distinct week or more of elevated or irritable mood with increased energy plus several associated manic symptoms

Why it is correct: Duration, mood, energy, and symptoms anchor mania. *Common error:* Diagnosing mania from a single day of high energy.

- **B** — Depressive count does not define mania. *Trap:* Applying depressive criteria to mania.
- **C** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **D** — Recent intensity is not the criterion. *Trap:* Confusing a brief peak with the episode.

Q2 — Answer: D. Document the session's actual length, participants, risk assessment, and interventions to match the billed code

Why it is correct: Claims must match documented services and time. *Common error:* Billing beyond what the record supports.

- **A** — Severity alone does not justify a code. *Trap:* Upcoding based on diagnosis.
- **B** — Notes must exist before billing. *Trap:* Documenting only when audited.
- **C** — Misstating the diagnosis is insurance fraud. *Trap:* Protecting the client through falsification.

Q3 — Answer: A. Identify what you can offer, such as relapse-prevention skills, and where his care needs psychiatric or crisis expertise

Why it is correct: Honest self-assessment defines your role and referrals. *Common error:* Overestimating or underestimating your competence.

- **B** — Acute mania needs medical management. *Trap:* Assuming the alliance covers everything.
- **C** — Counselors play a key role in bipolar care. *Trap:* Overcorrecting into needless refusal.
- **D** — Self-assessment is your own responsibility. *Trap:* Outsourcing competence judgments to family.

Q4 — Answer: B. Review the app's privacy and data-sharing terms with him, obtain informed consent, and use its data to support care

Why it is correct: Vet the tool and secure informed consent first. *Common error:* Adopting technology without privacy review.

- **A** — Automatic sharing bypasses his consent. *Trap:* Letting technology replace releases.
- **C** — Apps supplement, not replace, clinical judgment. *Trap:* Outsourcing assessment to an algorithm.
- **D** — Many apps sell or expose health data. *Trap:* Assuming wellness apps are private.

Q5 — Answer: B. Assess danger to self and others, including suicide risk and the impulsive, high-risk behaviors he is currently engaging in

Why it is correct: Assess risk across mood states. *Common error:* Overlooking risk during an elevated mood.

- **A** — Manic self-report alone is unreliable here. *Trap:* Accepting reassurance without assessment.
- **C** — Narrow suicide-only focus misses risk. *Trap:* Assessing only one channel of risk.
- **D** — Deferring risk assessment is unsafe. *Trap:* Postponing an urgent safety assessment.

Q6 — Answer: B. Coordinate an urgent psychiatric evaluation for pharmacologic management of the acute mania and to address safety

Why it is correct: Urgent psychiatric/medication evaluation. *Common error:* Offering outpatient psychotherapy as the primary acute treatment.

- **A** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.
- **C** — Watchful waiting is unsafe in acute mania. *Trap:* Delaying urgent care.
- **D** — Insight work is not the acute priority. *Trap:* Prioritizing insight during acute mania.

Q7 — Answer: C. Notice your reaction, slow your breathing, and keep a calm, measured tone so the tension does not drive your response

Why it is correct: Self-regulation keeps your responses therapeutic. *Common error:* Reacting to the client's escalation.

- **A** — Matching intensity escalates irritable mania. *Trap:* Confusing volume with control.
- **B** — Abrupt endings rupture care during risk. *Trap:* Escaping discomfort instead of regulating it.
- **D** — Burdening the client misplaces responsibility. *Trap:* Making your reaction his problem.

Q8 — Answer: D. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them

Why it is correct: Defer dosing; coordinate with consent. *Common error:* Advising on a specific medication dose.

- **A** — Naming a dose exceeds scope. *Trap:* Overstepping into medication management.
- **B** — Directing dosing is unsafe and out of scope. *Trap:* Giving specific pharmacologic direction.
- **C** — Refusing to coordinate abandons the need. *Trap:* Declining to support communication with the prescriber.

Q9 — Answer: D. Engage him respectfully, find shared goals such as protecting his business, and link safety steps to what he values

Why it is correct: Engage via shared goals despite low insight. *Common error:* Arguing with him to force insight.

- **A** — Confrontation raises resistance. *Trap:* Using confrontation to force acceptance.
- **B** — Avoiding the issue forfeits engagement. *Trap:* Sidestepping the central concern.
- **C** — Colluding ignores real risk. *Trap:* Validating the denial to keep rapport.

Q10 — Answer: B. With his consent, provide psychoeducation about warning signs and engage the family in supporting the safety plan

Why it is correct: Consented family psychoeducation and support. *Common error:* Excluding family or sharing without consent.

- **A** — Disclosing without consent breaches confidentiality. *Trap:* Sharing clinical details without consent.
- **C** — Excluding family forgoes useful support. *Trap:* Refusing collateral that could help.

- **D** — Acting without his input oversteps. *Trap:* Arranging control behind the client's back.

Q11 — Answer: C. When there is a serious, foreseeable risk of harm to him or others, disclosure may be required to protect safety

Why it is correct: Danger threshold can require disclosure. *Common error:* Treating confidentiality as absolute when risk is serious.

- **A** — A vague best-interest test is too loose. *Trap:* Using personal judgment instead of the standard.
- **B** — Absolute confidentiality is inaccurate. *Trap:* Denying any limit to confidentiality.
- **D** — Family requests do not override confidentiality. *Trap:* Disclosing on a relative's request alone.

Q12 — Answer: C. Agree with Aaron and his sister on specific roles, such as noting sleep shifts and spending limits, tied to his goals

Why it is correct: Shared, client-endorsed roles link supports to goals. *Common error:* Excluding or over-empowering the family.

- **A** — Covert reporting erodes trust and consent. *Trap:* Using family as secret monitors.
- **B** — Past impairment does not erase his voice in planning. *Trap:* Handing his goals to the family.
- **D** — Consented family support aids bipolar recovery. *Trap:* Equating family involvement with lost autonomy.

Q13 — Answer: A. Obtain his specific, informed consent for each party before sharing any of his protected health information

Why it is correct: Get a valid release for each party. *Common error:* Coordinating before obtaining consent.

- **B** — Verbal interest is not a valid release. *Trap:* Disclosing before consent is documented.
- **C** — Each disclosure is his informed choice. *Trap:* Making release decisions for the client.

- **D** — Coordination still requires consent. *Trap:* Assuming shared care waives consent.

Case 6: Voices and growing suspicion in a college student

Diagnosis: Schizophrenia

Q1 — Answer: D. Two or more active-phase symptoms with continuous signs of disturbance and functional decline lasting at least six months

Why it is correct: Symptom count, duration, and decline anchor it. *Common error:* Diagnosing schizophrenia from brief symptoms.

- **A** — Depressive count does not define it. *Trap:* Applying depressive criteria here.
- **B** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **C** — Recent intensity is not the criterion. *Trap:* Confusing a brief change with the criteria.

Q2 — Answer: A. Discuss the conflict openly with her and arrange a warm referral, since your teaching role compromises your suitability

Why it is correct: A dual role makes the pairing unsuitable. *Common error:* Minimizing a conflict of interest.

- **B** — Partial workarounds leave the dual role intact. *Trap:* Managing rather than resolving conflict.
- **C** — Concealment violates transparency and trust. *Trap:* Hiding conflicts to protect rapport.
- **D** — Burdening her education serves your interest. *Trap:* Making the client resolve your conflict.

Q3 — Answer: B. Consult an attorney or your liability carrier, notify Renee, and release nothing without her consent or a court order

Why it is correct: Seek legal guidance and protect privilege first. *Common error:* Treating a subpoena as a court order.

- **A** — A subpoena is not authorization to disclose. *Trap:* Releasing records on demand.
- **C** — Subpoenas require a timely response. *Trap:* Assuming privilege is absolute.
- **D** — Forensic opinions exceed the treating role. *Trap:* Blending treating and forensic roles.

Q4 — Answer: C. Confirm you have the training and qualification level the instrument requires before using it, or refer if you do not

Why it is correct: Use only instruments you are qualified to administer. *Common error:* Assuming licensure covers every test.

- **A** — Instruments carry specific user qualifications. *Trap:* Equating licensure with test competence.
- **B** — Untrained staff compromise validity and privacy. *Trap:* Delegating assessment beyond competence.
- **D** — Structured measures track severity and change. *Trap:* Relying on impressions alone.

Q5 — Answer: C. Conduct a structured suicide-risk assessment, recognizing that suicide risk is elevated in early psychosis

Why it is correct: Assess elevated risk in early psychosis. *Common error:* Overlooking risk amid psychotic symptoms.

- **A** — Risk must be assessed before this. *Trap:* Sequencing referral ahead of safety.
- **B** — No-suicide contracts are not evidence-based. *Trap:* Substituting a contract for assessment.
- **D** — Minimizing the ideation is unsafe. *Trap:* Dismissing the ideation.

Q6 — Answer: B. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside psychosocial treatment and support

Why it is correct: Antipsychotic eval plus psychosocial care. *Common error:* Offering counseling alone for schizophrenia.

- **A** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.
- **C** — Counseling alone is insufficient. *Trap:* Omitting medication coordination.
- **D** — Insight-only is not first-line. *Trap:* Prioritizing insight over indicated care.

Q7 — Answer: D. Decline, since EHR access requires a job-related need such as her care, and keep your own login credentials private

Why it is correct: EHR access follows role-based, minimum-necessary rules. *Common error:* Treating system access as permission.

- **A** — Technical access is not authorization. *Trap:* Assuming shared systems mean shared charts.
- **B** — Sharing credentials breaches security and audit trails. *Trap:* Lending your login for convenience.
- **C** — Printing exposes records outside safeguards. *Trap:* Moving data out of the secure system.

Q8 — Answer: A. Case management and supported education or employment services to help her re-engage with her studies and daily roles

Why it is correct: Coordinate functional supports. *Common error:* Ignoring functional and role recovery.

- **B** — Overload can overwhelm her. *Trap:* Demanding too much too soon.
- **C** — Reflexive withdrawal can worsen function. *Trap:* Defaulting to complete disengagement.
- **D** — Tutoring alone misses the cause. *Trap:* Treating the grades without the illness.

Q9 — Answer: A. Build a steady, trustworthy alliance with consistency and transparency, going at a pace she can tolerate

Why it is correct: Prioritize a trustworthy, paced alliance. *Common error:* Pushing for disclosure faster than she can tolerate.

- **B** — Flat reassurance rarely shifts delusions. *Trap:* Arguing against the belief directly.
- **C** — Pressing too fast can deepen mistrust. *Trap:* Moving faster than the alliance allows.
- **D** — Colluding with delusions is harmful. *Trap:* Validating a delusion to keep rapport.

Q10 — Answer: B. Acknowledge how distressing and real they feel to her without confirming or arguing about their objective accuracy

Why it is correct: Validate the distress, stay neutral on content. *Common error:* Either confirming or flatly disputing the voices.

- **A** — Treating commands as instructions is unsafe. *Trap:* Colluding with hallucination content.
- **C** — Bluntly disputing can rupture trust. *Trap:* Confronting the symptom dismissively.
- **D** — Endorsing the explanation reinforces the belief. *Trap:* Validating a delusional explanation.

Q11 — Answer: D. Assess the risk carefully and follow duty-to-protect obligations, which may require warning or protective steps

Why it is correct: Assess and act on duty to protect. *Common error:* Treating an identifiable threat as fully confidential.

- **A** — Delaying assessment of a threat is unsafe. *Trap:* Postponing an urgent risk assessment.
- **B** — Confidentiality yields to a serious threat. *Trap:* Treating confidentiality as absolute when there is danger.

- **C** — Act on assessed risk, not reflexively. *Trap:* Disclosing before assessing the risk.

Q12 — Answer: D. Decline to disclose without her consent and, with her permission, explore appropriate, helpful family involvement

Why it is correct: Protect confidentiality; pursue consented involvement. *Common error:* Disclosing to parents without consent.

- **A** — Total refusal forgoes consented collaboration. *Trap:* Refusing collaboration that could help.
- **B** — Disclosing without consent breaches confidentiality. *Trap:* Sharing clinical details without authorization.
- **C** — A general update is still a disclosure. *Trap:* Leaking information under the guise of an update.

Q13 — Answer: C. Seek consultation and coordinate with a treatment team, referring for the components beyond your demonstrated competence

Why it is correct: Consult, coordinate, and refer within competence. *Common error:* Managing complex psychosis alone without competence.

- **A** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.
- **B** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.
- **D** — Minimizing the illness fails the client. *Trap:* Underestimating the need for specialist care.

Case 7: Lifelong distractibility and restlessness derailing work

Diagnosis: Attention-Deficit/Hyperactivity Disorder, Combined Presentation

Q1 — Answer: A. Thank him, decline the offer, and explore what the gesture means to him while keeping the relationship professional

Why it is correct: Decline the dual relationship and explore its meaning. *Common error:* Accepting services from a client.

- **B** — Free labor creates a dual relationship. *Trap:* Treating client services as a gift.
- **C** — Paying him still creates a business relationship. *Trap:* Assuming payment removes the conflict.
- **D** — Punitive limits shame a well-meant gesture. *Trap:* Enforcing boundaries without warmth.

Q2 — Answer: B. Verify the tool is HIPAA compliant with a business associate agreement and obtain Diego's informed consent to its use

Why it is correct: Vet compliance and obtain consent before using AI. *Common error:* Adopting AI without safeguards.

- **A** — The counselor remains responsible for note accuracy. *Trap:* Trusting AI output without review.
- **C** — Details can identify him and leave the counselor's control. *Trap:* Assuming partial de-identification suffices.
- **D** — Covert recording violates informed consent. *Trap:* Treating AI use as invisible to clients.

Q3 — Answer: C. Reflect on how personal beliefs about effort shape these reactions and bracket them so they do not bias his care

Why it is correct: Self-reflection surfaces bias before it shapes care. *Common error:* Acting on unexamined reactions.

- **A** — Shaming echoes his own "lazy" self-label. *Trap:* Voicing bias as honest feedback.
- **B** — Unexamined attitudes leak into treatment. *Trap:* Assuming private thoughts are harmless.
- **D** — Reflection should precede any referral. *Trap:* Referring instead of self-examining.

Q4 — Answer: D. Scan the relevant pages into his secure record, return the originals to him, and limit access to staff with a need

Why it is correct: Collateral records that inform the diagnosis belong in the secured record. *Common error:* Handling client-supplied documents outside record safeguards.

- **A** — Unsecured papers on a desk invite unauthorized viewing. *Trap:* Trading security for convenience during an evaluation.
- **B** — Personal devices place protected information outside practice safeguards. *Trap:* Taking records home for convenience.
- **C** — The report cards document the basis for the diagnosis and are his property. *Trap:* Destroying records that support clinical decisions.

Q5 — Answer: D. Medical and substance contributors and other conditions, such as a sleep disorder or depression, that can impair attention

Why it is correct: Rule out mimics of inattention. *Common error:* Diagnosing ADHD without screening mimics.

- **A** — Eating pathology is not indicated. *Trap:* Inventing an unsupported rule-out.
- **B** — Lifelong symptoms argue against decline. *Trap:* Pursuing an improbable rule-out.
- **C** — Psychosis is not indicated. *Trap:* Pursuing an unsupported rule-out.

Q6 — Answer: D. Follow the practice’s emergency plan, move with him to the designated shelter area, and check in afterward

Why it is correct: Activate the emergency plan and protect life first. *Common error:* Delaying or improvising during a real incident.

- **A** — Assuming the warning will pass endangers them both. *Trap:* Prioritizing session continuity over safety.

- **B** — Driving during a tornado warning is more dangerous than sheltering. *Trap:* Ending contact instead of following the plan.
- **C** — Records never outrank personal safety. *Trap:* Protecting files before people.

Q7 — Answer: A. Externalizing organization through planners, reminders, task breakdown, and environmental structure to support follow-through

Why it is correct: Externalize organization and structure. *Common error:* Relying on internal willpower alone.

- **B** — Relying on memory worsens follow-through. *Trap:* Removing external supports.
- **C** — Relaxation alone misses the core. *Trap:* Treating restlessness while ignoring planning.
- **D** — Overloading worsens follow-through. *Trap:* Increasing demands instead of structuring them.

Q8 — Answer: A. Explain that medication selection and dosing are the prescriber's role and offer, with consent, to coordinate his questions

Why it is correct: Defer prescribing; coordinate with consent. *Common error:* Recommending a specific medication or dose.

- **B** — Refusing to coordinate abandons the need. *Trap:* Declining to support communication with the prescriber.
- **C** — Directing dosing is unsafe and out of scope. *Trap:* Giving specific pharmacologic direction.
- **D** — Naming a drug and dose exceeds scope. *Trap:* Overstepping into medication management.

Q9 — Answer: D. Reframe the difficulties as a treatable neurodevelopmental condition and highlight his real efforts to compensate

Why it is correct: Reframe shame; affirm effort. *Common error:* Either reinforcing or ignoring the self-blame.

- **A** — Endorsing the self-blame deepens shame. *Trap:* Colluding with the lazy self-narrative.
- **B** — Facts alone rarely shift shame. *Trap:* Answering emotion with numbers.
- **C** — Bypassing the shame misses a target. *Trap:* Avoiding emotionally salient material.

Q10 — Answer: C. Collaboratively set small, specific, achievable between-session steps and review them with a measurement-based check-in

Why it is correct: Small specific steps plus measured review. *Common error:* Assigning vague, oversized goals.

- **A** — A huge task invites failure. *Trap:* Ignoring the need to grade tasks.
- **B** — Attendance is not an outcome measure. *Trap:* Mistaking attendance for progress.
- **D** — Open-ended tasks reduce follow-through. *Trap:* Omitting structure the client needs.

Q11 — Answer: D. Validate the worry and reframe the diagnosis as an explanation that guides effective strategies, not an excuse to give up

Why it is correct: Reframe diagnosis as a guide, not an excuse. *Common error:* Either dismissing or reinforcing the belief.

- **A** — Bypassing the concern misses a target. *Trap:* Avoiding emotionally salient material.
- **B** — Lecturing rarely shifts the worry. *Trap:* Substituting facts for engagement.
- **C** — Endorsing the excuse framing harms engagement. *Trap:* Reinforcing the self-judgment.

Q12 — Answer: D. Provide accurate documentation within competence, with his consent, describing functional impacts relevant to accommodations

Why it is correct: Document accurately within competence. *Common error:* Misrepresenting or overstating the findings.

- **A** — Disclosing without consent breaches confidentiality. *Trap:* Sharing records without authorization.
- **B** — A blanket refusal overstates the limit. *Trap:* Declining appropriate documentation.
- **C** — Overstating impairment is dishonest. *Trap:* Falsifying clinical documentation.

Q13 — Answer: B. Decline to confirm or deny without a valid release and explain that disclosure requires his written consent

Why it is correct: Protect confidentiality without a release. *Common error:* Disclosing to an employer without consent.

- **A** — Abrupt refusal misses a teachable moment. *Trap:* Being needlessly obstructive.
- **C** — Confirming breaches confidentiality. *Trap:* Disclosing without authorization.
- **D** — Disclosing for accommodations still needs consent. *Trap:* Leaking details under the guise of helping.

Case 8: Intrusive thoughts and endless rituals

Diagnosis: Obsessive-Compulsive Disorder

Q1 — Answer: A. Consult a colleague skilled in imaginal exposure for harm obsessions, using de-identified details and keeping responsibility

Why it is correct: Targeted consultation builds a specific skill while the counselor retains responsibility. *Common error:* Dropping or handing off a needed technique instead of building skill.

- **B** — Harm-thought details can identify him on a public forum. *Trap:* Assuming removing a name de-identifies a case.

- **C** — Consultation supports the counselor's work; it does not transfer the case. *Trap:* Handing off care to avoid distressing content.
- **D** — Feared harm cannot be practiced in vivo, so imaginal exposure is needed. *Trap:* Fitting treatment to current skills rather than need.

Q2 — Answer: B. Review ERP's purpose, expected temporary distress, alternatives, and his right to decline, and document consent

Why it is correct: Consent must precede treatment and be documented. *Common error:* Treating evidence-based care as exempt from consent.

- **A** — Consent cannot come after the intervention. *Trap:* Obtaining consent after treatment has begun.
- **C** — A competent adult consents for himself. *Trap:* Accepting third-party consent for a capable client.
- **D** — ERP involves deliberate distress requiring consent. *Trap:* Assuming evidence-based care needs no consent.

Q3 — Answer: C. Bring the case to the clinical supervisor to review the risk formulation and the counselor's reactions to the harm content

Why it is correct: Supervision supports sound judgment and manages reactions. *Common error:* Hiding uncertainty and reactions from the clinical supervisor.

- **A** — Abrupt transfer abandons a treatable client. *Trap:* Referring out to escape counselor discomfort.
- **B** — Concealing doubts defeats the purpose of supervision. *Trap:* Protecting the counselor's image over client care.
- **D** — Silencing obsessions reinforces avoidance and shame. *Trap:* Restricting content to ease counselor discomfort.

Q4 — Answer: D. He has specific intrusive obsessions with ritualized compulsions, not diffuse worry

Why it is correct: OCD has obsessions plus rituals; GAD does not. *Common error:* Calling ritualized OCD a worry disorder.

- **A** — Multiple worries do not define OCD. *Trap:* Using worry breadth to call it OCD.
- **B** — Sleep disturbance is nonspecific. *Trap:* Using sleep to decide.
- **C** — Anxiety is common to both. *Trap:* Using anxiety to differentiate.

Q5 — Answer: D. His obsessions and compulsions are unwanted and distressing, not a valued lifestyle

Why it is correct: OCD is ego-dystonic; OCPD is ego-syntonic. *Common error:* Confusing OCD with OCPD.

- **A** — Conscientiousness is nonspecific. *Trap:* Using conscientiousness to decide.
- **B** — Valuing order points toward OCPD, not OCD. *Trap:* Using valued order to define OCD.
- **C** — Rigidity leans toward OCPD traits. *Trap:* Reading rigidity as OCD.

Q6 — Answer: D. Accommodate his reasonable request for confidential communications without requiring him to explain why

Why it is correct: HIPAA requires providers to honor reasonable confidential-communication requests. *Common error:* Demanding a reason or ignoring the request.

- **A** — Reasonable alternative-contact requests must be honored. *Trap:* Treating contact method as the provider's choice.
- **B** — Providers may not require an explanation. *Trap:* Applying the health-plan endangerment standard to providers.
- **C** — HIPAA permits secure electronic contact. *Trap:* Overcorrecting into no contact at all.

Q7 — Answer: B. Facing the fear without the ritual lets his anxiety fall on its own through habituation

Why it is correct: Habituation occurs when rituals are blocked. *Common error:* Failing to give a clear ERP rationale.

- **A** — Thought neutralizing is itself a compulsion. *Trap:* Teaching thought suppression or neutralizing.
- **C** — Avoidance maintains OCD. *Trap:* Recommending avoidance.
- **D** — Rituals strengthen the cycle, not weaken it. *Trap:* Suggesting rituals will extinguish themselves.

Q8 — Answer: C. Work to reduce the family accommodation and reassurance that keep the OCD going

Why it is correct: Reducing accommodation is part of treatment. *Common error:* Leaving accommodation in place.

- **A** — Accommodation worsens OCD. *Trap:* Recruiting family into rituals.
- **B** — Abrupt withdrawal without a plan can backfire. *Trap:* Cutting accommodation abruptly.
- **D** — Family involvement is often important. *Trap:* Ignoring the family role.

Q9 — Answer: B. Track his symptoms with a validated OCD measure and adjust the exposure hierarchy

Why it is correct: Validated tracking guides the hierarchy. *Common error:* Relying on impression instead of a measure.

- **A** — Global self-report is less reliable. *Trap:* Substituting impression for measurement.
- **C** — End-only measurement cannot guide care. *Trap:* Deferring measurement to the end.
- **D** — One ritual is not the full picture. *Trap:* Monitoring one ritual only.

Q10 — Answer: B. Kindly limit the reassurance and support him in tolerating the uncertainty instead

Why it is correct: Limiting reassurance treats the compulsion. *Common error:* Reassuring to relieve his distress.

- **A** — Avoiding the moment misses the intervention. *Trap:* Avoiding the reassurance pattern.
- **C** — Dismissiveness harms the alliance. *Trap:* Invalidating instead of guiding.
- **D** — Reassurance reinforces the OCD loop. *Trap:* Answering the reassurance request.

Q11 — Answer: B. Give a clear rationale and build the exposure hierarchy together at a manageable pace

Why it is correct: Rationale and collaborative pacing build readiness. *Common error:* Forcing exposure without preparation.

- **A** — Flooding without grading can overwhelm. *Trap:* Starting with the hardest item.
- **C** — Dismissing the fear is inaccurate. *Trap:* Overpromising that ERP is easy.
- **D** — Skipping ERP omits the active ingredient. *Trap:* Replacing ERP with relaxation.

Q12 — Answer: D. Recognize the harm thoughts as ego-dystonic OCD symptoms, not intent, and avoid needless disclosure

Why it is correct: Ego-dystonic obsessions are not intent. *Common error:* Treating OCD harm obsessions as homicidal intent.

- **A** — Reporting obsessions as intent is wrong. *Trap:* Confusing obsessions with a threat.
- **B** — Conditioning care on disproving obsessions is harmful. *Trap:* Withholding care over ego-dystonic fears.
- **C** — Unwarranted disclosure breaches confidentiality. *Trap:* Disclosing on obsession content alone.

Q13 — Answer: A. Explain that an SSRI is evidence-based for OCD and refer for evaluation with his consent

Why it is correct: SSRIs help OCD; refer to a prescriber. *Common error:* Recommending a specific drug or dose.

- **B** — Dismissing medication contradicts the evidence. *Trap:* Denying a role for medication.
- **C** — Disclosing without consent breaches confidentiality. *Trap:* Coordinating care without consent.
- **D** — Specifying a drug and dose exceeds scope. *Trap:* Making pharmacologic recommendations.

Case 9: Acute suicidal crisis after a breakup and job loss

Diagnosis: Adjustment Disorder With Depressed Mood; Personal History of Suicidal Behavior

Q1 — Answer: A. Conduct a structured suicide-risk assessment clarifying ideation severity, intent, plan, access to means, and prior attempts

Why it is correct: Structured risk assessment comes first. *Common error:* Assessing mood while skipping a structured risk screen.

- **B** — Intervening before assessing is premature. *Trap:* Jumping to coping skills before risk assessment.
- **C** — Premature reassurance forecloses assessment. *Trap:* Reassuring instead of assessing acute risk.
- **D** — Deferring risk assessment is unsafe. *Trap:* Outsourcing an acute safety assessment.

Q2 — Answer: C. By assessing his intent, since acting with intent to die differs fundamentally from self-injury aimed at relieving tension

Why it is correct: Intent to die separates the two. *Common error:* Treating all self-harm as equivalent.

- **A** — Visible injury does not establish intent. *Trap:* Inferring intent from wounds.
- **B** — Others' awareness is not the test. *Trap:* Using disclosure as the criterion.

- **D** — Frequency does not clarify intent. *Trap:* Equating frequency with risk type.

Q3 — Answer: D. A careful assessment of his access to lethal means, since available, lethal methods sharply elevate acute suicide risk

Why it is correct: Means access is a key risk driver. *Common error:* Overlooking means access in the assessment.

- **A** — Quantifying intake is not the priority. *Trap:* Confusing intake detail with means assessment.
- **B** — Exhaustive gun history is low yield. *Trap:* Collecting low-yield detail.
- **C** — Depth history is not the priority now. *Trap:* Prioritizing depth over acute safety.

Q4 — Answer: B. Both his risk factors and his protective factors, such as his ambivalence and the relationships he names as reasons to live

Why it is correct: Weigh risk and protective factors. *Common error:* Cataloging only risk factors.

- **A** — One factor does not define overall risk. *Trap:* Reducing risk to one variable.
- **C** — Narrative detail is not the risk metric. *Trap:* Equating a good history with low risk.
- **D** — Proportionality is not a risk metric. *Trap:* Judging risk by proportionality.

Q5 — Answer: A. Weigh controlled trials and practice guidelines, such as the VA/DoD suicide-risk guideline, that include veterans at acute risk

Why it is correct: Evidence carries most weight when it is rigorous and drawn from clients like him. *Common error:* Assuming any published finding applies to every client.

- **B** — Findings may not generalize across developmental groups. *Trap:* Ignoring whether the study sample resembles the client.

- **C** — Volume of publications is not strength of evidence. *Trap:* Counting articles instead of weighing design and outcomes.
- **D** — A single case report is the weakest form of evidence. *Trap:* Mistaking a vivid anecdote for empirical support.

Q6 — Answer: A. A collaborative safety plan identifying warning signs, coping strategies, supports, and crisis contacts he can use

Why it is correct: Collaborative safety planning. *Common error:* Relying on a no-suicide contract.

- **B** — No-suicide contracts are not evidence-based. *Trap:* Using a contract instead of a plan.
- **C** — A single contact rule is not a plan. *Trap:* Reducing safety planning to one instruction.
- **D** — Activity scheduling alone is insufficient. *Trap:* Substituting mood work for safety planning.

Q7 — Answer: C. Counsel collaboratively on means restriction, such as temporarily storing the firearm away from the home with a trusted person

Why it is correct: Means restriction is central. *Common error:* Leaving the lethal means unaddressed.

- **A** — Avoiding means restriction is unsafe. *Trap:* Omitting means-restriction counseling.
- **B** — Deferring abandons means counseling. *Trap:* Using autonomy to avoid the conversation.
- **D** — In-home storage still leaves access. *Trap:* Settling for an inadequate means measure.

Q8 — Answer: C. On a clinical judgment weighing intent, plan, means, and his ability to maintain safety with the current plan and supports

Why it is correct: Stratified judgment drives level of care. *Common error:* Using a single factor to decide disposition.

- **A** — Refusing admission ignores acute danger. *Trap:* Ruling out admission regardless of risk.
- **B** — Reflexive admission is not the standard. *Trap:* Admitting reflexively without assessment.
- **D** — Disposition is a clinical judgment. *Trap:* Deferring disposition to the client alone.

Q9 — Answer: D. A warm handoff and a scheduled follow-up with caring contact rather than simply sending him out with a phone number

Why it is correct: Warm handoff and follow-up. *Common error:* Ending contact without a follow-up plan.

- **A** — A long gap in an acute crisis is unsafe. *Trap:* Leaving a long gap after a crisis.
- **B** — Self-directed lookup is too passive. *Trap:* Relying on the client to self-refer in crisis.
- **C** — Outsourcing to the ex is inappropriate. *Trap:* Delegating safety to an unsuitable contact.

Q10 — Answer: B. Confirm his current physical location and an emergency contact, and know crisis resources local to him

Why it is correct: Location and local resources enable emergency response. *Common error:* Starting video sessions without verifying location.

- **A** — Remote care needs added safety steps. *Trap:* Assuming office safety procedures transfer unchanged.
- **C** — Camera off removes key risk observation. *Trap:* Sacrificing assessment data for client comfort.
- **D** — Emergency services must be local to him. *Trap:* Dispatching help to the wrong jurisdiction.

Q11 — Answer: A. Validate how unbearable things feel right now while helping him see the crisis and his options as changeable over time

Why it is correct: Validate pain, frame the crisis as time-limited. *Common error:* Either only sympathizing or only reassuring.

- **B** — Endorsing hopelessness is dangerous. *Trap:* Adopting the client's hopeless stance.
- **C** — Overpromising can ring hollow. *Trap:* Making promises you cannot keep.
- **D** — Avoiding the despair leaves it intact. *Trap:* Sidestepping the crisis to ease discomfort.

Q12 — Answer: C. Verify his current coverage and discuss options such as VA benefits or a sliding fee so care does not lapse

Why it is correct: Address payment openly without interrupting crisis care. *Common error:* Letting payment issues interrupt care during a crisis.

- **A** — Billing lapsed coverage is improper billing. *Trap:* Submitting claims without verifying current eligibility.
- **B** — Pausing care in a crisis risks abandonment. *Trap:* Letting fees override acute safety needs.
- **D** — Changing diagnoses for payment is fraud. *Trap:* Misrepresenting a diagnosis to maximize reimbursement.

Q13 — Answer: A. Document the risk assessment, the formulation, the safety plan, means-restriction steps, and the clinical rationale for the disposition

Why it is correct: Document assessment, plan, and rationale. *Common error:* Documenting only that he agreed to a no-suicide contract.

- **B** — Minimizing the record is unsafe and inaccurate. *Trap:* Understating risk in the record.
- **C** — A contract note is inadequate and misleading. *Trap:* Substituting a contract note for real documentation.
- **D** — Omitting the assessment is improper. *Trap:* Omitting essential clinical information.

Case 10: Recurrent binges without compensatory behavior

Diagnosis: Binge-Eating Disorder

Q1 — Answer: D. Use a HIPAA-compliant platform with a signed business associate agreement and encrypted video sessions

Why it is correct: Compliant platforms require encryption and a BAA. *Common error:* Choosing a platform for convenience over compliance.

- **A** — Consumer apps usually lack HIPAA safeguards. *Trap:* Prioritizing client familiarity over legal compliance.
- **B** — Live sessions still transmit protected information. *Trap:* Assuming unrecorded calls need no safeguards.
- **C** — A waiver does not satisfy HIPAA obligations. *Trap:* Shifting compliance duties onto the client.

Q2 — Answer: A. Bill the 38–52 minute psychotherapy code (90834) and document actual start/stop times, necessity, and interventions

Why it is correct: Time-based codes must match documented minutes. *Common error:* Selecting codes by complexity or payment instead of time.

- **B** — Complexity does not add billable minutes. *Trap:* Upcoding time based on the diagnosis.
- **C** — Undercoding also misrepresents the service. *Trap:* Believing inaccurate low billing is safe.
- **D** — Psychotherapy codes are defined by time. *Trap:* Choosing codes by reimbursement rate.

Q3 — Answer: B. Practice an exercise with her in session, link app use to her binge triggers, and review how well it helped at each session

Why it is correct: Tools work best when taught, tied to treatment targets, and reviewed. *Common error:* Handing over an app without integrating it into care.

- **A** — Apps supplement rather than replace evidence-based treatment. *Trap:* Treating a self-help tool as the whole intervention.
- **C** — Consumer calming apps are not monitored by the counselor. *Trap:* Creating false expectations of real-time crisis monitoring.
- **D** — Unpracticed tools are often misused or abandoned. *Trap:* Assuming a tool needs no orientation or follow-up.

Q4 — Answer: A. Co-occurring depression and suicide risk, which commonly accompany binge-eating disorder and shape the plan

Why it is correct: Screen depression and suicide risk. *Common error:* Treating the eating pattern without screening mood and risk.

- **B** — Psychosis is not indicated. *Trap:* Pursuing an unsupported rule-out.
- **C** — Anxiety alone is not the explanation. *Trap:* Over-attributing to anxiety alone.
- **D** — Cognitive decline is improbable here. *Trap:* Pursuing an improbable rule-out.

Q5 — Answer: C. Coordinate a medical evaluation and ongoing monitoring as part of a team while she engages in psychotherapy

Why it is correct: Coordinate medical evaluation and monitoring. *Common error:* Providing solo therapy without medical coordination.

- **A** — Dieting can worsen the binge cycle. *Trap:* Prescribing a diet as treatment.
- **B** — No coordination undercuts team care. *Trap:* Refusing helpful coordination.

- **D** — Ordering tests exceeds counselor scope. *Trap:* Acting outside scope of practice.

Q6 — Answer: C. Offer alternatives such as audio-recorded or app-based logs with verbal review, at no added cost to her

Why it is correct: Reasonable accommodations preserve access to effective treatment. *Common error:* Removing core treatment instead of accommodating.

- **A** — Dyslexia does not require transfer of care. *Trap:* Referring out instead of providing accommodations.
- **B** — Self-monitoring is a core CBT-ED component. *Trap:* Dropping an essential intervention for convenience.
- **D** — Surcharging for accommodations violates the ADA. *Trap:* Passing accommodation costs to the client.

Q7 — Answer: B. Establishing regular eating and reducing the dietary restraint and triggers that drive the binge episodes

Why it is correct: Regular eating to interrupt the binge cycle. *Common error:* Adding restriction that fuels binges.

- **A** — Relaxation alone misses the core. *Trap:* Treating arousal while ignoring eating.
- **C** — Meal-skipping drives binges. *Trap:* Promoting restriction that triggers binges.
- **D** — More restriction fuels binges. *Trap:* Reinforcing the restraint that drives binges.

Q8 — Answer: C. Provide general information and coordinate a referral to a prescriber for evaluation with her consent, within scope

Why it is correct: Refer and coordinate within scope. *Common error:* Giving specific medication advice.

- **A** — Naming a drug and dose exceeds scope. *Trap:* Overstepping professional limits.

- **B** — Recommending a suppressant is inappropriate and out of scope. *Trap:* Advising an unsafe, out-of-scope product.
- **D** — Willpower framing is inaccurate. *Trap:* Moralizing about a clinical disorder.

Q9 — Answer: B. Respond with a nonjudgmental, validating stance that reduces shame and supports her engaging openly in treatment

Why it is correct: Reduce shame with a nonjudgmental stance. *Common error:* Responding with judgment or diet advice that deepens shame.

- **A** — Minimizing the disorder is inaccurate. *Trap:* Downplaying a treatable condition.
- **C** — Moralizing deepens shame. *Trap:* Reducing the disorder to willpower.
- **D** — Facts alone rarely shift shame. *Trap:* Answering shame with numbers.

Q10 — Answer: B. Clarify that the focus is on interrupting the binge cycle and the distress, not on prescribing another restrictive diet

Why it is correct: Reframe the aim away from dieting. *Common error:* Confirming her fear by centering the plan on dieting.

- **A** — Centering a diet drives binges and her fear. *Trap:* Confirming the feared, counterproductive approach.
- **C** — Weight-centric framing increases shame. *Trap:* Centering the work on weight loss.
- **D** — Bypassing the concern harms engagement. *Trap:* Avoiding a key alliance issue.

Q11 — Answer: B. Conduct a structured suicide-risk assessment and respond to the risk, recognizing eating disorders can carry elevated risk

Why it is correct: Assess risk directly. *Common error:* Overlooking risk amid the eating focus.

- **A** — No-suicide contracts are not evidence-based. *Trap:* Substituting a contract for assessment.
- **C** — Minimizing the ideation is unsafe. *Trap:* Dismissing the ideation.
- **D** — Risk must be assessed before continuing. *Trap:* Proceeding past a risk disclosure.

Q12 — Answer: A. Obtain her informed consent and release before communicating with the provider, then collaborate on a shared plan

Why it is correct: Consent and release before coordinating. *Common error:* Contacting the provider before obtaining consent.

- **B** — Coordinating without consent breaches confidentiality. *Trap:* Disclosing before consent is documented.
- **C** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.
- **D** — No coordination undercuts collaborative care. *Trap:* Refusing helpful, consented coordination.

Q13 — Answer: A. Explain that accurate clinical documentation is required and discuss how records are protected and who may access them

Why it is correct: Keep honest records; explain protections. *Common error:* Falsifying or omitting clinical information.

- **B** — Shadow records are improper. *Trap:* Keeping hidden duplicate notes.
- **C** — Omitting the diagnosis falsifies the record. *Trap:* Falsifying records to reassure.
- **D** — Dismissing her concern harms rapport. *Trap:* Ignoring the client’s concern.

Practice Exam Two

10 clinical cases · 130 questions · 225 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

Case 1 of 10

Twenty years of feeling “flat” in a small hardware-store town

Diagnosis: Persistent Depressive Disorder

Intake: Tyrone Jackson, a 41-year-old man who has worked at Pruitt’s Hardware and Supply in his small rural town for eighteen years, presents reporting that he has felt “down” or “flat” for as long as he can remember, dating back to his early twenties, with no stretch of feeling fully like himself lasting more than about six weeks.

Session Two: He describes chronically low energy, a poor appetite most days, trouble falling asleep, and a nagging sense that he is not good enough, though he still opens the store most mornings and rarely misses a shift. Friends have called him the “quiet, steady one” for two decades, and he assumed this flatness was simply his personality.

Session Three: He denies any lifetime period of elevated or irritable mood, decreased need for sleep, or reckless spending, and he denies any current thoughts of suicide, a plan, or intent, saying he has “never gotten that low.” His wife recently urged him to seek help after noticing him withdraw further from their weekend routines.

Question 1. What intake detail is most essential to document to support the working diagnosis of Persistent Depressive Disorder?

A. That his low mood has been present most of the day, more days than not, for at least two years, without a symptom-free stretch longer than two months

B. That a first-degree relative, such as a parent or sibling, was also treated at some point in the past for a chronic low-mood condition resembling his own

C. That he can name the exact week his low mood first began after finishing high school

D. That his mood tends to feel somewhat heavier on workdays at the store than it does on the weekends when he is off

Question 2. During Tyrone's intake, the building loses power, the lights go out, and the practice's electronic records system goes down; what should the counselor do first?

A. Follow the practice's emergency plan for outages, confirm Tyrone's safety, and finish or reschedule using its backup procedures

B. Keep the intake going in the dark and type his answers into a personal phone app, moving them into his chart once power returns

C. Send Tyrone home right away, ending the intake without setting a new time and waiting for him to call once the power is back

D. Leave Tyrone in the darkened office while the counselor walks the building to find the cause of the outage and fix it alone

Question 3. The counselor wants to use CBASP, a therapy designed for chronic depression, with Tyrone but has only read about it; what is the most appropriate step?

A. Begin CBASP with Tyrone right away, relying on the manual, since chronic depression is already within the counselor's scope

B. Consult a colleague trained in CBASP on applying it with Tyrone while the counselor builds skill through formal training

C. Refer Tyrone to another counselor at once, since no counselor may use a modality without first holding its certification

D. Post Tyrone's case details on an online therapist forum and ask other members to explain how they would carry out CBASP with him

Question 4. Tyrone names low energy, poor sleep, feeling not good enough, and pulling away from weekend routines, and he wants to tackle them all at once; what is best?

A. Agree to work on all four concerns equally in each session so that Tyrone does not feel any of his problems are being set aside

B. Choose his sense of not being good enough as the first target, since the counselor views it as the core belief driving the rest

C. Help him rank the concerns by their impact and his readiness to change, then start with one or two targets that he chooses

D. Postpone setting any priorities until his wife can attend a session and decide which of his concerns matters most to the family

Question 5. Given his denial of any current suicidal thoughts, what is the most appropriate assessment step?

A. Wait until several sessions of rapport-building have passed before ever raising the topic of safety

B. Skip any risk questions entirely since he denies current thoughts and appears calm during the session

C. Ask him to sign a written agreement promising not to harm himself before the next scheduled appointment

D. Use a structured suicide-risk screen to confirm and document the absence of ideation, intent, plan, or prior behavior

Question 6. Tyrone agrees to involve his wife in rebuilding their weekend routines but wants his feelings of not being good enough kept private; how should the counselor proceed?

A. Share his full treatment picture with his wife, since she urged him to seek help and will support him better knowing it all

B. Ask his wife to check his mood each day and report back to the counselor so that his progress can be monitored between sessions

C. Decline to involve his wife at all, since persistent depressive disorder is best treated without any family participation

D. With his signed release, agree on what may be shared, then involve his wife in the weekend-routine goals that Tyrone selected

Question 7. How should the counselor track whether treatment is producing meaningful change over the coming months?

A. Simply ask him at each visit whether he generally feels a little better than he did the week before and note his answer in the chart

B. Wait until an entire year of sessions has passed and then evaluate his overall response in one single review

C. Track only whether he keeps showing up to sessions on time and completes the homework assigned each week

D. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

Question 8. Tyrone asks whether he should also be evaluated for medication. What is the most appropriate counselor action?

A. Advise him that medication is generally unnecessary as long as he keeps attending his counseling sessions

B. Suggest he research antidepressants online himself and bring back his preferred choice at a later appointment

C. Provide general information and, with his consent, coordinate a referral to a prescriber for a medication evaluation

D. Tell him exactly which antidepressant tends to work best for people with a chronic, low-grade symptom pattern like his

Question 9. Which initial treatment-planning goal best fits Tyrone's chronic, low-grade presentation?

A. Encourage him to wait to re-engage in any valued activities until his energy and motivation naturally return on their own

B. Set a goal for him to resume every hobby and social role he has ever held, all within the first two weeks of treatment

C. Collaboratively schedule one or two small, valued activities each week, such as a fishing trip he used to enjoy

D. Have him complete a detailed daily log of every automatic negative thought before any other treatment goal is set

Question 10. How can the counselor best respond to Tyrone's belief that his flat mood is simply his personality rather than something treatable?

A. Offer psychoeducation that chronic low mood is a treatable condition, while validating how normal it felt to him for years

B. Change the subject and move the conversation toward scheduling next week's appointment time instead

C. Agree that this is simply how he has always been and focus sessions only on coping strategies rather than change

D. Firmly tell him his belief is wrong and present a list of facts proving that his personality is not actually flat

Question 11. Tyrone says he doubts anything can really change after twenty years of feeling this way. What is the most therapeutic response?

A. Acknowledge his doubt as understandable and collaboratively set one small, achievable goal to test it against

B. Spend the session explaining the neurobiology of chronic depression in detail to convince him change is possible

C. Agree that twenty years is likely too long for real change and shift the focus to simply accepting the mood

D. Promise him confidently that his mood is guaranteed to improve within a set number of weeks of treatment

Question 12. Later, worried about small-town gossip, Tyrone asks the counselor to shred his intake notes; how should the counselor respond?

A. Explain that the practice's policy requires keeping his records for a set period, and describe how they are secured from others

B. Shred the intake notes as he asks, since the records belong to Tyrone and he can direct what happens to his information

C. Rewrite the intake notes to leave out the details he finds embarrassing, then replace the original version in his chart with these

D. Keep the official chart brief and store fuller notes of his intake at home, where no one at the small-town practice can see them

Question 13. The counselor realizes Tyrone is a regular customer-facing employee at the only hardware store in their shared small town. What is the most appropriate way to manage this?

A. Decline to continue working with him and refer him elsewhere simply because he works at a public-facing local business

B. Discuss the likelihood of unavoidable public encounters with him in advance and agree on how to handle them respectfully

C. Ask him to stop working his usual shifts at the store so that the two of them are less likely to ever encounter each other

D. Say nothing about it and simply hope that the two of them never happen to cross paths anywhere in town

Case 2 of 10

Dazed and hypervigilant three weeks after a car crash

Diagnosis: Acute Stress Disorder

Intake: Emily Navarro, a 24-year-old dental hygienist, presents for outpatient counseling ten days after being T-boned at an intersection near her apartment while stopped at a red light. She was referred by the emergency department after treatment for minor injuries and reports intrusive memories of the crash, nightmares, and a dazed, “unreal” feeling that comes and goes.

Session Two: She describes driving the long way to work every day to avoid the intersection where the crash occurred, scanning cross-traffic obsessively at every light, and startling badly at horns or screeching brakes. She has trouble concentrating on patient charts at the dental office and reports two or three nightmares about the collision each week.

Session Three: Emily denies any current suicidal ideation, self-harm, or substance use, and her emergency department workup showed no ongoing medical injury requiring further treatment. She is unsettled by how “unlike herself” she feels during the dazed episodes but is motivated, has supportive family nearby, and is willing to begin trauma-focused counseling right away.

Question 1. What should the counselor confirm first at intake to anchor a working diagnosis of Acute Stress Disorder?

A. That she can recall the exact speed of both vehicles and the precise sequence of events leading up to the collision itself

B. That her auto insurance claim regarding the collision has already been fully processed and formally resolved

C. That a family member also witnessed the crash and can independently corroborate every detail of what actually happened

D. That she directly experienced the threatened injury of the crash and that it has been under one month since it happened

Question 2. The counselor, practicing under supervision toward full licensure, has never treated acute stress disorder; what is best before finalizing Emily's treatment plan?

A. Finalize the plan independently and mention Emily's case at the next quarterly supervision meeting if any concerns come up later

B. Review her case and proposed plan with the clinical supervisor and document the supervisor's input in the supervision record

C. Ask a classmate from graduate school to approve the plan informally, since a peer review substitutes for the assigned supervisor

D. Refer Emily elsewhere at once, since counselors practicing under supervision may not treat any client with a trauma diagnosis

Question 3. Given her presentation, what safety screening is appropriate at this intake?

A. A screening focused only on whether she has ever previously been hospitalized psychiatrically for any reason at all

B. No formal risk screening is needed since she denies distress about the dazed episodes and appears to be coping well overall

C. A structured suicide risk screen covering ideation, intent, plan, and behavior, done routinely regardless of apparent risk

D. A screening deferred until the third or fourth session once the counselor has built more rapport with her over time

Question 4. In building Emily’s treatment plan, which strengths and barriers to her goals should the counselor identify from her first two sessions?

A. Strengths in the detour route that keeps her calm on the way to work; barriers in her family’s closeness and the ED referral

B. Strengths in her young age and short time since the crash; barriers in the precise memories she keeps of the collision itself

C. Strengths in her motivation and her supportive family nearby; barriers in her avoidance, hypervigilance, and poor focus

D. Strengths in her scanning of cross-traffic at every light; barriers in her willingness to talk openly about the crash itself

Question 5. Later, Emily reports that the short-course muscle relaxant the emergency department prescribed leaves her foggy each morning; how should the counselor document this?

A. Note that the medication is impairing her and recommend in the record that she cut her dose in half until the fogginess clears

B. Leave the report out of the progress note, since the effects of medication belong only in the prescribing physician’s records

C. Record that her morning fogginess is most likely a dissociative symptom of her stress disorder rather than any medication effect

D. Record the medication, the fogginess, and its timing as Emily’s own self-report, without the counselor attributing a cause

Question 6. While reviewing intake paperwork, the counselor finds that Emily was never given the practice's Notice of Privacy Practices; what does HIPAA require?

A. Provide the notice now and make a good-faith effort to obtain her written acknowledgment that she received it, documenting it

B. Have her sign an authorization for all future disclosures of her records, since that form replaces the Notice of Privacy Practices

C. Do nothing further, since posting the notice in the waiting room fully meets the federal requirement for every current client

D. Pause her counseling until she signs the acknowledgment, since services cannot continue for a client who declines to sign it

Question 7. How should the counselor monitor Emily's progress over the coming weeks?

A. Rely on her employer's occasional comments about her work performance as the main indicator of her clinical progress

B. Track only how many sessions she attends and whether she arrives on time, without reviewing her symptoms directly

C. Track her symptom cluster and functioning at each session to note whether it resolves or persists past one month

D. Ask her only once, at the very final session of the whole course of treatment, how she generally feels she has done

Question 8. Emily asks whether she should also see a prescriber about medication for her sleep and anxiety. What is the most appropriate counselor action?

A. Suggest she look into over-the-counter sleep aids on her own and report back on how well they seem to be working

B. Advise her that medication is unnecessary at this early stage as long as she keeps attending therapy sessions consistently

C. Tell her which specific sleep medication tends to work best for people going through a stressful period like hers

D. Provide general information and, with her consent, coordinate a referral to a prescriber for a full evaluation

Question 9. Which initial intervention best targets Emily's dazed, derealized episodes?

A. Begin detailed, repeated exposure to the full narrative of the crash before teaching her any coping skills first

B. Teach brief grounding techniques, such as naming sensory details in the room, to use during dazed or unreal moments

C. Explain to her at length that derealization is not a real medical concern and that she should simply try to ignore it

D. Recommend she avoid driving altogether until the dazed feelings stop occurring completely on their own over time

Question 10. Emily says she feels weak for still avoiding the intersection three weeks later. How should the counselor respond?

A. Agree that avoiding one intersection for three weeks does seem like an overreaction to what was, after all, a fairly routine traffic accident

B. Firmly challenge the belief as irrational and present a logical argument for why she should not feel this way

C. Normalize the avoidance as a common, treatable stress response rather than a personal weakness, and validate her frustration

D. Redirect the conversation away from the topic and refocus entirely on scheduling next week's session logistics

Question 11. What is the most therapeutic way to respond when Emily describes a nightmare in vivid detail during session?

A. Tell her that nightmares like this are meaningless and will stop mattering once enough time has simply passed

B. Ask her to describe the nightmare again in even more graphic detail so the counselor can fully picture every element and assess its meaning

C. Quickly change the subject to something lighter so the session does not become too heavy or emotionally difficult

D. Listen empathically, reflect her distress, and gently pace the discussion so she does not feel overwhelmed or flooded

Question 12. A colleague urges a single-session psychological debriefing for Emily; how should the counselor build knowledge to judge whether it is empirically supported?

A. Adopt the debriefing approach, since a trusted colleague's positive experience with it is enough to show that the method works

B. Review current trauma guidelines and trials, which find single-session debriefing unsupported and favor trauma-focused CBT

C. Rely on the handouts from a popular weekend trauma workshop, since a well-attended training reflects the current research base

D. Let Emily choose between debriefing and other options, since client preference alone determines which approach is supported

Question 13. Emily mentions she has retained an attorney regarding the crash and asks the counselor to send her full session notes directly to the attorney's office. What is the appropriate response?

A. Refuse to release any information at all and tell her that session notes can never be shared with an attorney

B. Send the complete session notes to the attorney’s office right away since she is the one who requested it verbally

C. Explain the release-of-information process, obtain her signed written authorization, and coordinate the disclosure appropriately

D. Summarize her progress from memory over the phone to the attorney rather than sharing any written documentation

Case 3 of 10

From prescribed pain pills to daily opioid use

Diagnosis: Opioid Use Disorder, Severe

Intake: Walt Higby, a 38-year-old construction worker, presents after a back injury led to prescribed opioids and then to buying pills and, recently, using heroin. He reports strong cravings, tolerance, failed attempts to stop, and use despite job loss and a possession arrest that has left him on probation.

Session Two: He identifies at least seven of the eleven criteria over the past year, including cravings, tolerance, withdrawal, and giving up activities he valued. He describes flu-like sickness, aching, and sweating within a day of his last use when he tries to quit.

Session Three: He is exhausted by the cycle and afraid of overdose after a friend died last month; he is unsure whether medication could help. He denies current suicidal intent but feels hopeless at times, and he has used alone, which raises his overdose risk.

Question 1. What is most important to establish to confirm a severe opioid use disorder?

A. The exact quantity of opioids he uses each day so that you can calculate precisely how physically dependent he has become

B. The number of DSM-5-TR criteria he meets over twelve months, since six or more place severity in the severe range

C. Whether members of his immediate family have also struggled with opioids or with another substance-related condition

D. Whether he first became addicted to opioids through a legitimate prescription rather than through buying them illegally

Question 2. Walt's assessment shows severe-range criteria, including cravings, withdrawal, and lost valued activities; how should the counselor integrate these results into his plan?

A. Place the assessment results in his chart and build the plan only from the goals he names, so his own priorities shape everything

B. Apply the program's standard package for severe opioid use disorder, since his severity label alone determines the plan's content

C. Link each finding to a plan element, such as cravings and withdrawal to MOUD referral and lost activities to rebuilt routines

D. Delay writing any plan until a full psychological test battery is completed, so every possible finding can be integrated at once

Question 3. Given that he has used alone and a friend recently died of overdose, what is the priority assessment and response?

A. Assess overdose risk and withdrawal safety, and provide overdose-prevention education including naloxone access

B. Reassure him that as long as he is motivated to quit, the risk of an accidental overdose is really quite low for him now

C. Focus the session on identifying the underlying emotional pain that you believe is the true root cause of all of his using

D. Advise him to taper his opioid use down on his own at home over the next week so that he can avoid the worst of the sickness

Question 4. Later, Walt's video session drops while he is describing using alone, and he does not reconnect; what should the counselor do first?

A. Wait for Walt to rejoin on his own, and if he has not returned by the scheduled end time, simply note the session as incomplete

B. Send him a brief text asking him to reschedule, since a dropped connection is a routine technical problem and not a clinical one

C. Call emergency services to his home address at once, since any disconnection during a high-risk discussion signals an overdose

D. Follow the disconnection plan set at the start of telehealth: call his backup number, then use the safety protocol if unreachable

Question 5. Later, Walt says his probation officer called buprenorphine “just another drug” that may violate his terms; what is the most appropriate advocacy step?

A. With his written consent, give the probation officer information on MOUD as evidence-based care and support his treatment

B. Advise Walt to start buprenorphine without telling his probation officer, since sharing his treatment plan could only hurt him

C. Tell Walt to drop his plan for medication and pursue counseling alone, since meeting his probation terms must come before all else

D. Contact the judge on Walt’s case directly to protest the officer’s view, without first discussing the step or getting his consent

Question 6. What is the most appropriate first-line treatment recommendation for severe opioid use disorder?

A. Suggest he first prove his commitment by staying completely abstinent on his own for thirty days before any referral is made

B. Coordinate a referral for medication for opioid use disorder, such as buprenorphine or methadone, alongside counseling

C. Recommend a course of weekly supportive counseling and self-help meetings as the complete treatment for his opioid use

D. Recommend an abstinence-only, medication-free program because relying on any medication merely substitutes one drug for another

Question 7. How should the counselor determine the appropriate level of care?

A. Automatically place him in the longest available residential program so that he is kept away from drugs for as long as possible

B. Use a multidimensional assessment of withdrawal risk, environment, and supports to match him to a safe level of care

C. Recommend only a weekly outpatient counseling slot on the assumption that talking through it regularly will be sufficient

D. Let him choose whichever program has the most convenient schedule for him regardless of his clinical needs or safety risks

Question 8. Which contextual factors should the counselor integrate into Walt’s treatment plan alongside his opioid use?

A. His opioid use alone, setting aside his probation, back injury, and work history as matters outside the scope of counseling

B. His probation terms, the back injury that started his opioid use, his recent job loss, and his friend’s overdose death

C. His likely distrust of talk therapy as a construction worker, so the plan offers him only brief, skills-based sessions

D. His friend’s recent overdose death alone, since grief is the factor most likely to drive his opioid use in the coming weeks

Question 9. Later, Walt misses a session without notice, and his insurer covers his counseling; how should the counselor handle billing for the missed visit?

A. Bill the insurer for the missed session using the usual psychotherapy code, since the appointment time was reserved for him

B. Bill the insurer for a shorter session instead, since a brief code fairly reflects the time spent waiting for him to arrive

C. Do not bill his insurer for the missed visit, and apply any no-show fee only as his signed policy and the payer contract allow

D. Bill the missed session to his probation office instead, since his counseling attendance is required as part of his probation

Question 10. Walt returns to use after ten days abstinent and is ashamed. The most therapeutic response is to:

A. Warn him sternly that another slip like this one will prove that he is simply not serious about getting clean this time

B. Reassure him that the slip really does not matter at all and encourage him to simply put it behind him and move forward

C. Frame the return to use as common in recovery, review overdose risk after abstinence, and revisit the plan without shaming him

D. Tell him that he clearly needs to be discharged to a higher level of care right away since the current outpatient plan has obviously failed him

Question 11. Walt's former employer calls asking whether he is being treated for a drug problem. The best response is to:

A. Decline to confirm or deny without a valid release, explaining the heightened confidentiality of substance-use information

B. Share only that he is attending sessions, since merely confirming attendance seems like a fairly harmless detail to give out

C. Refuse to speak with the caller at all and hang up without explaining anything about confidentiality or the proper process

D. Confirm that he is in treatment, since the employer already let him go and is now only trying to look out for his wellbeing

Question 12. Walt asks the counselor what dose of buprenorphine he should request from the prescriber. Best response?

A. Say that medication dosing is purely a medical matter and decline to help him prepare for the conversation in any way at all

B. Tell him to ask for the highest dose available so that the medication will be sure to fully block his cravings right away

C. Suggest a starting dose that usually works well for someone using the amount of opioids that he has described to you today

D. Explain that dosing is the prescriber’s decision and offer, with consent, to coordinate his questions and progress with them

Question 13. Walt wants the counselor to coordinate with his prescriber and his probation officer. What must come first?

A. Share information freely with the probation officer because the criminal-justice context overrides his usual confidentiality

B. Obtain his specific, informed written consent for each party before sharing any of his protected information

C. Coordinate only with the prescriber for now and decide about the probation officer later without discussing it further with him

D. Begin coordinating with both parties right away, since he has clearly already told you that he wants you to work with them

Case 4 of 10

Feeling detached and unreal, as if watching life from outside

Diagnosis: Depersonalization/Derealization Disorder

Intake: Owen Park, a 24-year-old graduate student, presents with months of persistent, distressing feelings of detachment — he feels like an observer of his own thoughts and actions, and the world around him often seems foggy, dreamlike, and unreal.

Session Two: He describes feeling robotic and emotionally numb and says his surroundings sometimes look flat or two-dimensional. Throughout, he knows these are subjective experiences and that things are not actually unreal; his reality testing remains intact between and during episodes.

Session Three: He is frightened he is going crazy and has been monitoring the sensations closely and avoiding stressful situations that seem to worsen them. He denies cannabis or other substance use and denies hallucinations or fixed delusional beliefs.

Question 1. What is most important to confirm to support a depersonalization/derealization disorder diagnosis?

A. That he firmly and genuinely believes that he and the world around him have actually become physically unreal

B. That he can identify the single stressful event he believes first triggered these feelings of detachment and unreality

C. Persistent or recurrent detachment or unreality with intact reality testing, causing significant distress or impairment

D. That his detachment has become more intense over the past day or two than it was earlier during the same week

Question 2. As referrals from Owen’s university grow, the counselor recognizes little experience serving graduate students as a population; which training step is best?

A. Read popular online lists of typical graduate-student traits and apply them as a guide to the counselor’s work with Owen

B. Ask Owen to teach the counselor about graduate-school culture in sessions, treating him as the main source of the training

C. Rely on the counselor’s general multicultural course from graduate school, since dissociation presents alike in every population

D. Take reputable training on graduate-student stressors and help-seeking, applying it while still assessing Owen as an individual

Question 3. Later, the vendor of the telehealth platform used for Owen’s video sessions reports a security incident that may have exposed his session data; what is required?

A. Assess the incident under the practice’s breach procedure and, unless compromise is shown unlikely, notify Owen within 60 days

B. Leave any notice to the platform vendor alone, since the security incident happened on its systems rather than within the practice

C. Notify Owen only if his information is later shown to have been misused, so he is not alarmed by an incident that caused no harm

D. Move Owen to a free consumer video app right away, since the incident shows paid platforms offer no added protection

Question 4. During exam season, with a caseload now full of distressed graduate students, the counselor starts feeling numb and going through the motions with Owen; what is best?

A. Read the numbness as a reaction to Owen's own detachment and keep working as usual, since it reflects his symptoms

B. Treat the numbness as an early sign of compassion fatigue and act now with self-care, consultation, and caseload limits

C. Push through until the semester ends and rest then, since the numbness has not yet visibly affected anyone's care

D. Tell Owen about the counselor's numbness to model that such feelings are normal and help him feel less alone with them

Question 5. When billing Owen's insurer for weekly psychotherapy sessions, which documentation practice best supports the claims?

A. Recording his diagnosis once at intake, since the diagnosis code on each claim by itself establishes medical necessity

B. Writing brief notes listing only the date and attendance, since detailed notes could expose Owen's privacy to the insurer

C. Notes linking his diagnosis, current impairment, and plan goals to each session's interventions and his response to them

D. Copying the prior week's note forward with the date updated, since Owen's symptoms change only a little from week to week

Question 6. What is the most appropriate treatment approach for his presentation?

A. An antipsychotic-focused referral on the assumption that his detachment reflects an underlying psychotic process

B. A primarily insight-oriented exploration of his early childhood as the central route to resolving his current symptoms

C. Encouraging him to keep avoiding the situations that worsen the detachment until the symptoms resolve on their own

D. Cognitive behavioral therapy with psychoeducation and grounding, addressing catastrophic appraisals of the symptoms

Question 7. How should the plan address his close monitoring of the detachment sensations?

A. Tell him to keep a detailed minute-by-minute log of exactly when the detachment is present throughout each and every day

B. Help him reduce hypervigilant self-monitoring, since constantly checking the sensations tends to intensify and maintain them

C. Advise him to avoid any activity that might bring on the sensations so that he never has to notice the detachment at all

D. Encourage him to monitor the sensations even more closely so that he can detect the earliest sign of each episode coming on

Question 8. Owen asks whether medication could help. The most appropriate counselor action is to:

A. Recommend the specific medication and dose that tends to work best for people with depersonalization symptoms like his

B. Coordinate a referral to a prescriber for evaluation, especially of comorbid anxiety or depression, while continuing therapy

C. Tell him that medication is unnecessary and that the detachment will resolve entirely through the grounding techniques alone

D. Suggest he research medications for dissociation himself and bring whichever one he prefers to a future appointment with you

Question 9. Owen says he is terrified the detachment means he is going crazy. The most therapeutic response is to:

A. Tell him quickly that there is absolutely nothing to worry about and that he should simply try to stop thinking about it

B. Agree that these symptoms are indeed a worrying sign and suggest that he may well be developing a serious mental illness

C. Validate the fear and provide psychoeducation that these symptoms, while frightening, are a recognized and treatable condition

D. Explain in technical detail the neuroscience of dissociation and depersonalization to convince him on scientific grounds that he is not going crazy

Question 10. Which technique is most useful to help Owen during an episode of detachment in session?

A. Encourage him to focus intently on exactly how detached and unreal he feels so he can fully understand the experience

B. Guide him through grounding that reconnects him with his senses and surroundings to reduce the intensity of the episode

C. Wait quietly without intervening until the episode resolves entirely by itself before continuing the conversation

D. End the session immediately and send him home so that he can rest until the detachment passes on its own at home

Question 11. Later, Owen recounts childhood abuse in detail, and the counselor, whose caseload is heavy with trauma survivors, begins feeling unsafe and hypervigilant outside work; what is best?

A. Treat the hypervigilance as an unrelated personal stressor, since it shows up outside of work rather than during sessions

B. Ask Owen to share fewer details of the abuse so that the counselor is exposed to less of his trauma history going forward

C. Stop accepting all trauma referrals at once, before examining how much the counselor's exposure is driving the change

D. Assess cumulative trauma exposure and symptoms with a tool such as the ProQOL and review the results in supervision

Question 12. As CBT with interoceptive and grounding exercises is planned, informed consent most importantly requires:

A. Having him agree up front that he will complete every exercise assigned no matter how distressing it turns out to be for him

B. Assuring him in advance that the exercises will be completely comfortable and will never bring on any detachment at all

C. Postponing any explanation of the exercises until after the first one so that his anticipatory anxiety does not build up first

D. Explaining the rationale, that some exercises may briefly heighten the sensations, and that participation remains voluntary

Question 13. Owen later admits he recently began using cannabis on weekends to cope with the sensations, well after they started, and asks the counselor to ignore it. Best response?

A. Explain that the cannabis use is clinically relevant to his symptoms and address it collaboratively rather than ignoring it

B. Agree to leave the cannabis use out of the assessment and the plan entirely so that he feels more comfortable with you

C. Tell him he must stop all cannabis use immediately or you will not be able to continue working with him in counseling at all

D. Document the cannabis use but avoid ever discussing it with him so that the topic does not create tension between you

Case 5 of 10

Recurrent depression with overlooked periods of elevated energy

Diagnosis: Bipolar II Disorder

Intake: Elena Ortiz, a 34-year-old nurse, presents seeking help for recurrent depression. She describes several depressive episodes over the years and current low mood, anhedonia, and poor sleep, and she came in mainly to ask about restarting an antidepressant.

Session Two: On careful history she recalls distinct multi-day periods of unusually elevated, energized mood with decreased need for sleep, rapid thoughts, and heightened productivity that others noticed, without psychosis or hospitalization and not severe enough to be full mania.

Session Three: She had not connected those great stretches to her mood problems and worries something is wrong with her. She describes her depressive episodes as full and impairing rather than only mild, and she denies any past episode requiring hospitalization or causing severe functional breakdown.

Question 1. What is most important to establish to support a bipolar II diagnosis?

A. That she currently meets the full symptom count required for a discrete manic episode at this point in time

B. That her current depressive symptoms are clearly more severe today than they were earlier in this same week

C. A history of at least one hypomanic episode in addition to her major depressive episodes, with no full manic episode

D. That she can identify the single stressful event she believes first triggered her recurring problems with her mood

Question 2. Elena offers to share nightly sleep data from her consumer smartwatch to help track her early warning signs; how should the counselor use it?

A. As one trend source alongside her mood reports, noting that consumer wearables are not validated diagnostic tools

B. As the deciding measure of when hypomania begins, since the watch's sleep scores are objective and free of her bias

C. Not at all, since only clinician-administered measures belong in a counseling plan for monitoring her mood episodes

D. As live overnight alerts sent to the counselor, so the counselor can respond right away whenever her sleep runs short

Question 3. Reviewing the intake, the counselor sees how nearly the counselor accepted Elena's view of plain recurrent depression without asking about elevated mood; what is best?

A. Conclude the near miss shows the counselor lacks the competence to treat mood disorders and transfer Elena's care

B. Name it as a personal limitation in assessment and build routine screening for past elevated mood into every intake

C. Attribute the near miss to Elena's incomplete reporting, since she had not linked her great stretches to her mood

D. Rely on the counselor's strong rapport to draw out such histories in the future rather than adding structured screening

Question 4. Later, Elena mentions she has a hearing loss and has trouble following speech in sessions despite her hearing aids; what should the counselor do?

A. Ask her to bring a family member to each session to repeat anything that she has trouble hearing from the counselor

B. Speak louder and more slowly in every session from now on without asking her which supports would actually help

C. Ask what works best for her and provide free aids such as captioning, a quieter room, or written session summaries

D. Offer captioned sessions for a modest added fee, since providing captioning creates an extra cost for the practice

Question 5. Given her current depressive episode, what risk assessment is most important?

A. A complete list of every antidepressant she has ever tried so the counselor can document her full medication history

B. A precise log of how many hours she sleeps each night so the counselor can quantify the full extent of her insomnia

C. A structured suicide-risk assessment, recognizing that suicide risk is elevated in bipolar depression

D. A detailed reconstruction of every job she has held so the counselor can fully map her complete occupational history

Question 6. Elena asks the counselor to support her starting an antidepressant on its own. The most appropriate action is to:

A. Reassure her that an antidepressant by itself is the clearly correct treatment for her depressive episodes and encourage her to start one right away

B. Tell her medication is entirely outside the counseling role and decline to discuss her question about it any further

C. Coordinate a psychiatric evaluation and share that antidepressant monotherapy is a concern in bipolar II, given the diagnosis

D. Tell her which specific antidepressant and dose she should ask her physician to start for her depressive symptoms

Question 7. As Elena quietly says her family might be better off without her, the counselor feels a jolt of alarm and starts speaking fast; what is best?

A. Shift to a lighter topic until the counselor's alarm settles, then come back to her comment later in the session

B. Tell Elena that the counselor feels alarmed so she understands how seriously the counselor is taking her statement

C. Call emergency services right away, since the counselor's strong alarm signals that her suicide risk must be imminent

D. Slow the counselor's breathing and speech and keep an open posture, then calmly ask direct questions about her thoughts

Question 8. What is most important to build into her ongoing plan?

A. Tracking mainly whether she attends her sessions and treating attendance as the main sign that she is doing well

B. Mood and sleep monitoring with early-warning-sign tracking so emerging hypomania or depression can be caught and managed

C. Tracking only her depressive symptoms while disregarding any signs of elevated or energized mood going forward

D. Waiting until her next routine visit in several months to reassess her mood rather than monitoring it in the meantime

Question 9. Elena is surprised and worried to learn her great periods may have been hypomania. Best response?

A. Explain the detailed neurobiology of bipolar disorder at length to convince her on technical grounds not to worry about it

B. Validate her reaction and provide psychoeducation about hypomania and bipolar II as understandable and manageable with treatment

C. Tell her that this means she has a severe, lifelong mental illness that will almost certainly only get worse over time

D. Reassure her quickly that the hypomania is nothing to be concerned about and that it does not really change anything about her care at all

Question 10. How can the counselor best help Elena recognize early signs of a hypomanic episode?

A. Collaboratively identify her personal early warning signs, such as reduced sleep need and racing thoughts, and a plan to respond

B. Advise her to treat any happy or productive day as a warning sign of hypomania so that she stays especially vigilant

C. Have her rely entirely on her family to notice any warning signs for her so that she does not have to monitor herself at all

D. Tell her not to worry about watching for any warning signs and to simply wait until a full episode is clearly underway

Question 11. Elena's primary care provider calls to coordinate her care. What must come first?

A. Share the relevant clinical information right away, since coordinating with her physician is clearly in her own best interest

B. Obtain her informed consent and release before sharing any clinical information with the primary care provider

C. Share only her diagnosis with the physician without a release, since a diagnosis alone seems like a harmless detail to provide

D. Refuse to coordinate with the physician at all so that her counseling remains entirely separate from her medical care

Question 12. Elena asks the counselor what mood-stabilizer dose she should request. The most appropriate response is to:

A. Say medication dosing has nothing to do with counseling and decline to help her prepare for the conversation in any way

B. Tell her to ask for the highest dose available so that her mood is sure to stay completely stable going forward

C. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate her questions and progress with them

D. Suggest a typical starting dose that tends to work for most people who have bipolar II disorder like hers does

Question 13. The counselor has limited training in bipolar disorder. The most ethical course of action is to:

A. Continue to manage all of her care alone to preserve the continuity of the relationship you have built with her

B. Tell her that her bipolar disorder is not serious enough to require any psychiatric involvement and continue supportive talks only

C. Transfer her at once to another provider with no explanation, consultation, or coordinated plan for the transition of care

D. Seek consultation and coordinate with a prescriber and team, or refer for the components beyond your demonstrated competence

Case 6 of 10

A fixed persecutory belief in an otherwise functioning man

Diagnosis: Delusional Disorder

Intake: Harold Means, a 52-year-old accountant, is referred by his physician. For over four months he has been convinced that a neighbor is conspiring to poison his water supply, and he has installed locks and filters and confronted the neighbor, though there is no evidence.

Session Two: Apart from this fixed belief he functions well at work and in self-care, his speech is organized, and he shows no hallucinations, disorganized behavior, or negative symptoms. The belief is non-bizarre — plausible on its face — but held with unshakable conviction.

Session Three: A recent medical and toxicology workup was unremarkable, and he denies substance use. He is angry that no one believes him and has hinted that he may "handle it himself" if the authorities will not act, which raises a concern about risk to the neighbor.

Question 1. What is most important to confirm to support a delusional disorder diagnosis?

A. That he can identify the single stressful event he believes first triggered his concerns about his neighbor

B. One or more delusions present for at least a month without other prominent active-phase psychotic symptoms

C. That his belief has become noticeably more intense over the past few days than it was the week before that

D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. The counselor plans to finish Harold's intake note at home that evening; what is the most appropriate way to keep his record secure?

A. Complete it only through the practice’s encrypted EHR access, never saving copies to a personal device or account

B. Email the draft to the counselor’s personal account so it can be finished on the home computer later that evening

C. Bring the paper intake forms home in a work bag so the note can be written from them at the kitchen table that night

D. Save the draft on the home computer’s desktop and delete it once the finished note has been uploaded to his record

Question 3. As the counselor confirms that taking on Harold’s ongoing care is appropriate, which factors best determine client-counselor suitability?

A. Whether Harold accepts that his belief about the neighbor is false, since insight is required before counseling can begin

B. The counselor’s skills or access to consultation, availability for his level of risk, and no tie to Harold or the neighbor

C. Whether the counselor and Harold share similar backgrounds and interests, since similarity is the main basis of a good fit

D. Whether Harold’s insurance pays the counselor’s full fee, since reimbursement level is the main factor in accepting clients

Question 4. Later, the neighbor seeks a protective order against Harold, and Harold asks the counselor to testify at the hearing that he poses no danger; what is best?

A. Agree to testify that he poses no danger, since he has never acted violently toward the neighbor in the past

B. Write a letter to the court describing his diagnosis so the judge can understand his behavior toward the neighbor

C. Explain that as his treating counselor the counselor should not give forensic opinions, and consult an attorney first

D. Refuse any involvement and disregard a subpoena if one arrives, to keep the counseling relationship protected

Question 5. Given his hint that he may “handle it himself,” what is the most important assessment step?

A. Focus only on whether he has a specific plan to harm himself and disregard any risk to the neighbor for the time being

B. Rely on his assurance that he would never actually do anything, since he has not harmed anyone up to this point in time

C. Defer any risk assessment until his delusion has resolved, since he is too distressed to answer the questions reliably now

D. Assess risk to the neighbor and to himself, including intent and means, and prepare to act on duty-to-protect obligations

Question 6. What is the most appropriate first-line treatment recommendation?

A. Start him on an antipsychotic medication yourself and arrange to monitor his response and side effects over the coming weeks

B. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside supportive psychological care

C. Begin intensive insight-oriented therapy to help him uncover the deeper meaning behind his belief about the neighbor

D. Recommend a course of weekly supportive counseling on its own as the complete treatment for his fixed delusional belief

Question 7. To structure Harold's violence-risk assessment, the counselor considers a structured risk instrument the counselor has never been trained to use; what is best?

A. Administer it after reading the manual, since a licensed counselor is qualified to use any violence-risk instrument

B. Use it now, since the publisher approved the counselor's purchase, which confirms the competence needed to use it

C. Skip structured assessment and rely on the counselor's clinical impression until the counselor completes training

D. Use it only after completing its required training, consulting a qualified evaluator on his risk in the meantime

Question 8. Given the risk concern, what should the plan prioritize regarding the neighbor?

A. Ongoing risk monitoring and a plan that addresses safety, including protective steps if the risk to the neighbor escalates

B. Immediate involuntary hospitalization based solely on the presence of the delusion, regardless of the assessed level of risk

C. No particular attention to the neighbor, since the client has not actually harmed anyone at any point so far

D. Encouraging him to confront the neighbor directly to resolve the conflict so that his distress about it finally settles down

Question 9. How should the counselor handle coordination with the psychiatrist and physician?

A. Avoid contacting the other providers at all so that his counseling remains entirely separate from his medical care

B. Share information with the other providers right away, since coordinating his care is clearly in his own best interest anyway

C. Share his full record with every provider so that they all have complete information about every aspect of his treatment

D. Obtain his informed consent and release before sharing information, then collaborate as part of a coordinated team

Question 10. Harold becomes angry that the counselor has not confirmed his belief. The most therapeutic response is to:

A. Tell him plainly that the belief is false and that he simply needs to accept that the neighbor is not doing anything to him

B. Agree with him that he is right about the neighbor so that he calms down and is willing to continue working with you

C. Empathize with how distressing and isolating it feels to not be believed, without endorsing or attacking the belief itself

D. End the session early to give him time to cool down rather than addressing his anger in the room with him at all

Question 11. How can the counselor best engage Harold given his limited insight into the belief?

A. Tell him that there is nothing counseling can offer him unless he is first willing to take antipsychotic medication

B. Focus the sessions mainly on gathering more and more detail about the neighbor and the alleged conspiracy each week

C. Insist that he first acknowledge that the belief is false before any other work in counseling can begin with him

D. Build trust and work on shared, concrete goals such as reducing his stress and distress that he is willing to engage with

Question 12. Harold makes a specific, credible threat to harm the named neighbor during a session. The most appropriate action is to:

A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

B. Keep the threat entirely confidential, since anything he shares in counseling must always remain completely private

C. Notify the neighbor and the police at once without first assessing the seriousness or following the proper protective process

D. Wait to see whether he repeats the threat at the next session before taking any action or assessing the risk

Question 13. The counselor has limited training in psychosis care. The most ethical course of action is to:

A. Continue managing the entire case alone to preserve the continuity of the relationship you have built with the client

B. Tell the client his symptoms are not serious enough to need psychiatric care and keep seeing him for supportive talks only

C. Seek consultation and coordinate with a treatment team, referring for the components beyond your demonstrated competence

D. Transfer the client at once to another provider with no explanation, consultation, or coordinated plan for the transition

Case 7 of 10

A bright child whose reading falls far below expectation

Diagnosis: Specific Learning Disorder, With Impairment in Reading

Intake: Sofia Alvarez, a 9-year-old, is referred by her parents and teacher for persistent reading difficulties. Despite targeted help, her word reading and comprehension remain well below what is expected for her age, and she is increasingly avoidant and discouraged about school.

Session Two: Her difficulties are specific to reading and have persisted for more than six months despite intervention. Her general reasoning and problem-solving are age-appropriate, she sustains attention in non-reading tasks, and there is no vision, hearing, or schooling deficit to explain it.

Session Three: She is articulate and curious in conversation, which contrasts sharply with her struggle to decode text. She has begun saying she is “dumb” and dreads being asked to read aloud, and her parents worry the gap is starting to affect her confidence and her mood.

Question 1. What is most important to confirm to support a specific learning disorder diagnosis?

A. That her reading has slipped a little further over the past few days than it had the week just before that

B. Reading skill well below age expectations that has persisted over six months despite targeted intervention

C. That her parents can name the single event they believe first caused her to fall behind in her reading

D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. The counselor finds that a signed note in Sofia’s EHR lists the wrong date of service; how should the counselor correct it?

A. Enter a dated, signed amendment correcting the date, leaving the original entry intact and visible in the audit trail

B. Delete the original note and re-create it with the correct date, so that her record looks clean for future readers

C. Ask the EHR vendor to change the date directly in the database, so that no visible correction appears in her record

D. Leave the error in place, since changing any note that has been signed in an EHR system is never permitted at all

Question 3. Sofia's parents ask the counselor to also tutor Sofia in reading at their home for extra pay; what is the most appropriate response?

A. Accept, since the counselor already knows Sofia well and the added tutoring could speed up her reading progress

B. Decline the tutoring role, explain how mixing roles could affect her counseling, and refer to reading specialists

C. Accept, on the condition that tutoring happens only at the office and is billed separately from the counseling sessions

D. Decline, and also stop discussing reading with Sofia in sessions so that the two roles stay fully separate from now on

Question 4. The practice adopts an approved, HIPAA-compliant AI tool that suggests accommodations from school records; how should the counselor use its suggestions for Sofia?

A. Adopt the suggestions as written, since the tool draws on far more cases than any one counselor will ever see

B. Forward the tool's suggestions directly to the school team so that her accommodations can start without delay

C. Check each suggestion against her evaluation and records, keeping final decisions and accountability with the counselor

D. Leave the AI's role out of her record, since the counselor reviewed the suggestions before they were used in her care

Question 5. The counselor, who struggled with reading as a child, notices feeling unusually protective of Sofia and angry at her teacher; what is best?

A. Share the counselor's own childhood reading struggles at length so that Sofia can see she is not dumb after all

B. Advocate forcefully against the teacher with the school, since the counselor's anger signals the school is failing her

C. Refer Sofia elsewhere at once, since any personal reaction means the counselor can no longer remain objective with her

D. Reflect on how the counselor's own school history may be shaping these reactions and keep them from steering her care

Question 6. What is the most appropriate counselor role in supporting Sofia?

A. Coordinate educational support and accommodations with the school while addressing the emotional impact in counseling

B. Tell her parents to simply have her read more at home and assume the problem will resolve on its own with practice

C. Personally deliver intensive reading remediation lessons yourself as the central component of her treatment plan

D. Start her on a medication that you will select and adjust to improve her reading over the coming weeks

Question 7. How should the counselor address the emotional impact of her struggles?

A. Build her confidence and coping, reframing the difficulty as a specific, workable challenge rather than a defect

B. Avoid discussing her feelings about school entirely so that the sessions never touch on anything that upsets her

C. Tell her she just needs to try harder and that her difficulties will disappear if she simply puts in more effort

D. Focus the sessions mainly on testing her reading repeatedly so you can see exactly how far behind she really is

Question 8. How should progress be tracked in the counseling work?

A. Rely only on whether her parents happen to mention that she seems a little happier about school lately

B. Track only her raw reading scores and base every counseling decision solely on whether those numbers improve

C. Wait until the end of the school year to review whether anything has changed at all since the first session

D. Track confidence, school avoidance, mood, and the use of accommodations over time to guide the support plan

Question 9. How should the counselor coordinate with the school regarding accommodations?

A. Contact the school directly with her clinical details right away, since the school plainly needs all of the information

B. Send the school her full clinical record so that every staff member has complete information about her counseling

C. With the parents' consent and a release, collaborate with the school team on appropriate accommodations and supports

D. Avoid contacting the school entirely so that her counseling stays completely separate from anything academic

Question 10. Sofia tearfully says she hates reading aloud and wants to give up on school. The most therapeutic response is to:

A. Validate how hard it feels, normalize the difficulty, and connect her to the supports that can make it manageable

B. Agree that school may not be for her and that giving up could be a reasonable choice given how hard it has become

C. Change the subject to something more cheerful so that she does not have to sit with the difficult feelings at all

D. Tell her that reading is simply not that important and that she should not let it bother her so much going forward

Question 11. How can the counselor best engage Sofia given her age and discouragement?

A. Talk with her mainly in abstract, adult terms about the neuropsychology of why her reading is impaired

B. Tell her she must improve her reading before any of the counseling work to help her confidence can begin

C. Use developmentally appropriate, strengths-based methods that highlight what she does well and build on it

D. Focus the sessions mainly on drilling the reading skills she struggles with most until she finally improves

Question 12. Sofia's parents ask the counselor to label her as "gifted" in a report to qualify for a program. The most appropriate action is to:

A. Take the parents' side and advocate for the label in any report, regardless of what the evaluation actually shows

B. Decline to misrepresent her profile and clarify what the assessment and your role can accurately support

C. Refuse to provide any documentation at all and decline to explain what could appropriately be reported about her

D. Write the label exactly as they request, since keeping the parents satisfied is what matters most for the work

Question 13. The counselor lacks training in psychoeducational assessment. The most ethical course of action is to:

A. Conduct and interpret the psychoeducational testing yourself to keep everything within the counseling relationship

B. Tell the parents the evaluation is unnecessary and that her difficulties will simply resolve with a bit more time

C. Refer the formal evaluation to a qualified professional and stay within your supportive counseling role

D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

Case 8 of 10

Distressing physical symptoms and constant health worry

Diagnosis: Somatic Symptom Disorder

Intake: Beatriz Romero, a 45-year-old office manager, presents with more than a year of distressing abdominal pain, fatigue, and headaches that dominate her daily life. She has seen many physicians and had repeated negative workups but remains highly distressed.

Session Two: She spends hours each day worrying about what the symptoms mean, researching diseases, and seeking reassurance, and she has cut back work and social activities. The symptoms are real to her and disabling, whether or not a medical cause has been found.

Session Three: She feels dismissed when providers say tests are normal and fears something is being missed. She is open to counseling but worried it implies her symptoms are not real. She denies that her worry is focused on one specific feared disease above the symptoms themselves.

Question 1. What is most important to confirm to support a somatic symptom disorder diagnosis?

A. Distressing somatic symptoms for over six months with excessive thoughts, feelings, and behaviors devoted to them

B. That she can identify the single stressful event she believes first triggered all of her physical symptoms

C. That her physical symptoms have grown more intense over the past week than they were the week before that

D. That every one of her physical symptoms has been medically proven, through repeated testing, to have no underlying organic basis whatsoever

Question 2. Later, during an in-office session, Beatriz suddenly reports crushing chest pain spreading to her left arm, unlike her usual symptoms; what should the counselor do first?

A. Follow the practice's medical emergency plan — call 911, stay with her, and alert the designated staff member to meet responders

B. Treat the pain as part of her familiar somatic pattern and guide her through a grounding exercise until the sensations ease

C. Ask her to compare this pain with her usual symptoms in detail before deciding whether any outside medical help is needed

D. Advise her to drive herself to her physician's office after the session so the new chest pain can be checked there today

Question 3. The counselor, skilled in CBT for depression but new to applying it to somatic symptoms like Beatriz's, plans to seek consultation; who is the best choice?

A. Her primary care physician, since her complaints are physical and the physician knows her full medical history the best

B. A counselor experienced in CBT for somatic symptom presentations, consulted in a structured way with de-identified details

C. A counselor friend who will give quick feedback over dinner without any formal arrangement, since informal talks save time

D. The practice's most senior counselor, a couples specialist, since seniority ensures sound advice on any kind of clinical case

Question 4. How should the counselor approach the medical side of her presentation?

A. Tell her plainly that since the tests are normal her symptoms are psychological and there is no real medical issue to address

B. Encourage her to keep seeking additional specialists and second opinions until a definitive physical diagnosis is finally found

C. Validate that her symptoms are real and distressing and coordinate with one consistent medical provider rather than dismissing them

D. Order a fresh round of laboratory and imaging tests yourself so that you can personally rule out any missed medical condition

Question 5. Beatriz begins sending several portal messages a day asking whether new symptoms are serious, and the practice has a written between-session contact policy; what should the counselor do?

A. Answer each message promptly with detailed reassurance so that her worry about the new symptoms does not keep building

B. Stop replying to her messages altogether without explanation so that she learns to manage her worries on her own

C. Review the practice’s contact policy with her, reply within its stated limits, and bring the messages into session work

D. Give her the counselor’s personal cell number so she can text directly and get faster answers than the portal allows

Question 6. Later, after reading her record, Beatriz asks in writing that the counselor remove the somatic symptom disorder diagnosis because she feels it calls her symptoms imaginary; what does federal law require?

A. Remove the diagnosis as she asks, since HIPAA lets clients decide which diagnoses may appear in their own health records

B. Decline to discuss the request, since clients have no legal right to ask that their counseling records be changed

C. Tell her that only her physician may change a diagnosis, so the request must be sent to her medical provider instead

D. Respond in writing within HIPAA's time limit and, if the diagnosis is accurate, explain the denial and her right to disagree

Question 7. How should care with her medical providers best be structured?

A. Recommend that she rotate among several different specialists so that she always has a fresh set of opinions to consider

B. Encourage her to go to urgent care or the emergency department each time a symptom feels especially severe or frightening to her

C. Advise her to stop seeing physicians altogether and to rely solely on counseling to manage all of her physical symptoms

D. Coordinate regular, scheduled visits with one primary provider rather than visits driven by each new symptom or crisis

Question 8. A central cognitive-behavioral target in her treatment would most appropriately be:

A. Teaching her to rest more and to limit her activity until her physical symptoms have fully and completely resolved

B. Encouraging her to research each new symptom carefully so that she can feel more informed and certain about what is happening

C. Reducing reassurance-seeking and catastrophic symptom interpretations while gradually restoring valued activity

D. Helping her develop a more thorough daily symptom-monitoring log so she can track every bodily sensation in fine detail

Question 9. Beatriz says counseling must mean the counselor thinks her symptoms are all in her head. Best response?

A. Explain that the tests are all normal, so the symptoms really are psychological and that is exactly what counseling will fix

B. Validate that her symptoms and suffering are real and explain that counseling addresses the distress and impact, not their reality

C. Avoid the question entirely and steer the session toward reviewing the activity goals she set for the coming week

D. Reassure her over and over that there is definitely nothing physically wrong with her so that she can stop worrying about it

Question 10. During a session Beatriz repeatedly asks the counselor to confirm her headaches are not a sign of something serious. Best response?

A. Provide her with a clear, confident answer every time she asks so that she can leave the session feeling calm and reassured

B. Tell her that her worries are irrational and that as an intelligent adult she really should know better than to think this way

C. Suggest she schedule another brain scan just to be sure so that she can finally put this particular worry to rest for good

D. Gently name the reassurance-seeking as part of the cycle and help her practice tolerating uncertainty rather than answering it

Question 11. Beatriz asks the counselor to tell her physician to order a specific scan she read about online. Best response?

A. Clarify that ordering tests is the physician’s decision and offer, with consent, to coordinate her concerns with the provider

B. Call her physician yourself and recommend that the specific scan she found online be ordered for her as soon as possible

C. Encourage her to insist that her physician order the scan and to keep pushing until the provider finally agrees to do it

D. Tell her this is purely a medical matter that has nothing to do with counseling and decline to discuss it any further with her

Question 12. How should the counselor coordinate with Beatriz's medical providers?

A. Obtain her informed consent and release before communicating with her providers, then collaborate on a shared plan

B. Avoid any contact with her medical providers at all so that her counseling remains entirely separate from her medical care

C. Share her full counseling records with every provider she sees so that they all have complete information about her treatment

D. Contact her physicians right away to align the plan, since coordinating her care is clearly in her own best interest anyway

Question 13. Beatriz becomes tearful and hints she sometimes feels life is not worth living with this suffering. Best step?

A. Reassure her that her symptoms will eventually improve with treatment and that there is no real need to discuss those thoughts

B. Conduct a structured suicide-risk assessment and respond to the risk, recognizing chronic somatic distress can carry suicide risk

C. Ask her to sign a brief promise that she will stay safe and reach out to you if those thoughts get any worse this week

D. Continue with the planned cognitive-behavioral agenda, assuming the comment was only a passing expression of her frustration

Case 9 of 10

Individual disclosure of intimate partner violence and a partner's request to join therapy

Diagnosis: Adjustment Disorder With Anxiety (in the context of intimate partner violence)

Intake: Maria Delgado, a 31-year-old marketing coordinator, presents for individual outpatient counseling reporting six weeks of anxiety, poor concentration, and disrupted sleep that she links to increasing tension at home with her live-in partner of fourteen months, Danny.

Session Two: In session she discloses that Danny has pushed her against a wall on two occasions in the past three months and, two weeks ago, choked her with both hands during an argument until her vision blurred; she also reports that he monitors her text messages, checks her location throughout the day, and has pressured her to stop seeing her sister and two close friends.

Session Three: Before Maria's next appointment, Danny calls the front desk and asks to be added to her "next session, so we can work on this together as a couple"; when told about the call, Maria appears frightened, says Danny does not know she disclosed the choking incident, and reports he has since questioned why she left the house for two hours the previous afternoon.

Question 1. Maria presents with six weeks of anxiety, poor concentration, and disrupted sleep and links them to tension at home with Danny but has not described any violence. What is the most appropriate intake step?

A. Concentrate the entire intake on her sleep and concentration complaints and postpone psychosocial history to a later date

B. Include direct, private questions about relationship safety as a routine part of the psychosocial intake

C. Wait until Maria raises any relationship concerns herself before asking anything about her partner or home life

D. Invite Danny to join part of the intake so the counselor can get a fuller picture of their home environment together

Question 2. After Maria discloses that Danny choked her two weeks ago, what should the counselor prioritize next in the assessment?

A. A detailed account of every disagreement the couple has had since they moved in together fourteen months ago

B. A structured danger assessment covering the frequency, severity, and escalation pattern of the violence, including access to weapons

C. A referral to a couples communication skills class so the two of them can address the conflict pattern together

D. An immediate call to Danny to hear his account of the argument before proceeding further with Maria

Question 3. Danny's choking of Maria is a well-established indicator of increased lethality risk in intimate partner violence. What follow-up is most important to include in the intake?

A. Specifically ask about any prior strangulation, threats with weapons, and whether the violence has increased in frequency or severity

B. Focus only on the most recent incident and avoid asking about any earlier acts of violence to reduce her distress

C. Ask Maria to rate on a scale of one to ten how committed she currently feels to the relationship overall

D. Refer her immediately to couples counseling so that Danny can learn healthier ways of managing his anger and she can learn to de-escalate their conflicts

Question 4. Which finding best supports Adjustment Disorder With Anxiety rather than Generalized Anxiety Disorder in Maria's case?

A. Her anxiety and hypervigilance emerged and sharply escalated in direct response to specific, ongoing violence and control

B. Her worry spans multiple areas of daily life, including work performance and finances, in addition to the relationship

C. Her anxiety has persisted for longer than six months and is accompanied by muscle tension and restlessness

D. Her symptoms are severe enough to interfere significantly with her concentration and daily functioning at work

Question 5. Beyond the presenting anxiety symptoms, what else is essential to assess given Maria's exposure to recent near-lethal violence?

A. Whether her sleep problems are better explained by an untreated primary sleep disorder such as chronic insomnia disorder unrelated to any relationship stress

B. Whether she has a family history of anxiety disorders that could help explain her current presentation

C. Her own current risk for suicide or self-harm, using a structured screen, since intimate partner violence significantly raises that risk

D. Whether her anxiety symptoms meet full criteria for panic disorder with recurrent, unexpected panic attacks

Question 6. What should be the cornerstone of the initial treatment plan for Maria at this point?

A. A structured anger-management referral for Danny that Maria can encourage him to attend on his own so the relationship can continue unchanged

B. A plan focused mainly on improving her sleep hygiene and daily relaxation practice to reduce her anxiety

C. A directive plan requiring her to end the relationship with Danny and move out within the next two weeks

D. A collaborative, individualized safety plan covering warning signs, coping steps, safe contacts, and options for leaving or staying

Question 7. Danny has called asking to be included in Maria's next session "so we can work on this together as a couple." What is the most appropriate treatment-planning response?

A. Decline the joint session and continue individual treatment, since conjoint work during active violence can raise danger and suppress disclosure

B. Tell Danny that Maria can decide for herself and, out of respect for her autonomy, let her choose whether he joins without first discussing the risks with her

C. Decline the joint session but offer to schedule Danny his own separate individual appointment with the same counselor

D. Agree to bring Danny into the next session to begin building healthier communication between the two of them

Question 8. What is the most appropriate way to help Maria access additional support while respecting her autonomy?

A. Offer information about IPV-specific resources such as a hotline and a local advocate, and let her decide what steps to take and when

B. Insist that she call a domestic violence hotline before the end of today's session as a condition of continuing treatment

C. Wait for her to ask about resources on her own rather than proactively raising the option of an IPV advocate

D. Give her a general community resource list without any information specific to intimate partner violence

Question 9. How should the safety plan specifically address the choking incident and its elevated lethality risk?

A. Focus the plan solely on long-term relationship counseling goals for the couple rather than any immediate physical-safety steps for Maria

B. Advise Maria to try to physically restrain Danny herself the next time she senses he is becoming angry

C. Recommend that Maria simply avoid arguments with Danny so the situation does not escalate to that point again

D. Include specific warning signs preceding escalation and an immediate physical-safety plan for recurrence, built collaboratively with Maria

Question 10. In session three, Maria also asks to switch to video sessions because leaving the house prompts Danny's questions; what should the counselor do before the first video session?

A. Assess whether she can meet privately on a device Danny cannot access, and agree on a code word and an interruption plan

B. Begin video sessions right away, since meeting from home removes any need for her to explain her absences to Danny

C. Send the video links and reminders to her usual phone and email so she can join each session without any extra steps

D. Proceed by video and handle any interruption by Danny as it happens, rather than planning for one with her in advance

Question 11. In the same session, Maria mentions she is insured as a domestic partner on Danny's employer plan and fears statements about her sessions will reach him; what should the counselor do?

A. Submit claims as usual, since explanation of benefits statements list only dates and never show the provider or service

B. Before billing, help her ask the plan to send statements elsewhere, which it must honor if she says she is endangered

C. Bill her sessions under a less sensitive diagnosis so that any statement reaching Danny will not raise his suspicion

D. Ask Danny's permission as the policyholder before submitting any claims for her individual counseling sessions

Question 12. When the counselor returns Danny's call, what is the appropriate ethical response regarding confidentiality?

A. Avoid returning the call at all and let Maria explain the situation to Danny on her own without further guidance

B. Tell Danny that Maria discussed him in session but withhold the specific content of what she said

C. Decline to confirm or discuss whether Maria is a client, or share any session information, without her explicit consent

D. Confirm that Maria is a client and explain that the counselor recommends individual rather than couples work

Question 13. How should the counselor document the session and support Maria's decision-making about her options going forward?

A. Note only that general relationship stress was discussed in session, without recording any of the specific safety concerns that Maria raised

B. Document the danger assessment and safety plan thoroughly, and offer protective order information while leaving the timing to Maria

C. Document that Maria must file a protective order and file the paperwork on her behalf to ensure her safety

D. Omit details of the violence from the record to protect Maria’s privacy from any future disclosure

Case 10 of 10

Progressive memory loss and lost independence in an older adult

Diagnosis: Major Neurocognitive Disorder due to Possible Alzheimer's Disease

Intake: Margaret Cole, a 78-year-old retired librarian, is brought by her daughter for two years of gradually worsening memory loss. She repeats questions, gets lost on familiar routes, has trouble managing her finances, and increasingly needs help with daily tasks.

Session Two: Her daughter reports a slow, steady decline rather than a sudden change, with clear attention most of the time and no fluctuating confusion. Margaret is aware that something is wrong and is frustrated and at times tearful about losing her independence.

Session Three: A medical workup is underway, and there is no evidence of an acute illness, fever, or new medication causing the change. The daughter worries about safety with driving, the stove, and medications, and feels exhausted and unsure how to help her mother.

Question 1. What is most important to confirm to support a major neurocognitive disorder diagnosis?

A. That she currently meets the full symptom count required for a discrete major depressive episode at this time

B. That she can identify the single stressful event she believes first triggered the memory problems she and her daughter have been noticing

C. That her memory has worsened over the past day or two more sharply than it did earlier in the same week

D. A significant decline from her prior level in one or more cognitive domains that interferes with her everyday independence

Question 2. Later, with some sessions held by video, the daughter asks that the platform record each session so she can replay them for Margaret; what must the counselor ensure?

A. Turn on the platform’s recording feature, since the daughter, as the family caregiver, can authorize it on her mother’s behalf

B. Have the daughter record the screen on her own phone so the platform keeps no copy that could be breached or subpoenaed

C. Record only with Margaret’s informed consent, or her legal representative’s if she lacks capacity, and secure files as records

D. Let the platform record by default, since a HIPAA-compliant platform makes separate consent to recording unnecessary

Question 3. How should the counselor weigh a possible depressive contribution to her cognitive problems?

A. Conclude that because she is sad her cognitive decline must be entirely due to depression and not to any neurocognitive disorder

B. Screen for depression, since depression can impair cognition and may coexist with or mimic a neurocognitive disorder

C. Determine whether her sadness is clearly more intense today than it was earlier in this same week of assessment sessions

D. Assume her sadness is simply a normal and expected reaction to aging that has no bearing on her thinking or memory at all

Question 4. What is the most appropriate counselor action regarding the diagnostic workup?

A. Use cognitive screening and collateral history and coordinate a medical and neurological evaluation rather than diagnosing alone

B. Tell the family it is simply normal aging and that there is no real need for any further medical evaluation at this point

C. Order brain imaging and laboratory tests yourself so that you can personally confirm the Alzheimer's diagnosis before treatment

D. Base the diagnosis only on how Margaret presents during the session, without collateral history or any medical evaluation

Question 5. Given the daughter's concerns, what safety assessment is most important?

A. Defer any safety assessment until the medical workup is fully complete, even though the daughter is worried about it now

B. Focus only on whether she currently has a specific plan to harm herself and disregard the broader safety concerns for now

C. Rely on Margaret's own assurance that she is managing fine at home, as she knows her own daily routine better than anyone else does

D. Assess safety risks such as driving, the stove, wandering, and medication management given her decline and impaired judgment

Question 6. What is the most appropriate treatment-planning priority for Margaret and her family?

A. Coordinate medical care and build a plan of caregiver support, safety measures, and structured non-pharmacologic approaches

B. Begin intensive insight-oriented psychotherapy to help her recover the specific memories that she has been losing over time

C. Recommend that the family wait and simply monitor her decline for several more months before arranging any support at all

D. Start her on a cognitive-enhancing medication yourself and monitor her response and dosing over the coming weeks

Question 7. Later, the counselor meets with the daughter alone about handling Margaret's repeated questions and stove safety and plans to bill family psychotherapy without the patient present; which approach best supports the reimbursement request?

A. Billing the visit as Margaret's individual psychotherapy session, since the entire discussion concerned her care at home

B. Billing the visit under the daughter's own insurance as her individual therapy, since she was the only person attending

C. Writing a brief note that a family meeting took place, since payers need only the code and date to process the claim

D. Documenting who attended, why Margaret was absent, and how the session served her treatment plan goals and safety

Question 8. How should the counselor best support the exhausted daughter?

A. Tell the daughter that caregiver stress is not really within the focus of counseling and redirect to the client's symptoms only

B. Advise the daughter to manage every aspect of her mother's care entirely on her own to keep things consistent for her mother

C. Encourage the daughter to avoid telling her mother anything about the diagnosis so that her mother never feels any distress

D. Provide caregiver psychoeducation and connect her with respite and support resources to reduce caregiver burden

Question 9. Margaret becomes tearful about losing her independence. The most therapeutic response is to:

A. Reassure her quickly that everything is completely fine and that she has absolutely nothing at all to feel upset about today

B. Provide her with detailed statistics about the typical progression of Alzheimer's disease to prepare her for what is coming

C. Validate her grief and preserve her dignity and personhood while supporting the abilities and choices she still retains

D. Speak mainly to her daughter about Margaret's care while Margaret is present, since her daughter can answer more reliably

Question 10. As her memory worsens, how should the counselor adapt the work?

A. Assign her detailed written homework to complete independently between sessions to keep her actively working on her own

B. Use simple, concrete communication, shorter sessions, and supportive engagement adapted to her current cognitive abilities

C. Continue using the same complex, insight-oriented techniques you would use with any cognitively unimpaired adult client

D. Reduce contact with her and work mainly through her daughter, since Margaret can no longer benefit from any direct engagement

Question 11. Later, the plan adds a GPS-enabled wearable to alert her daughter if Margaret becomes lost on familiar routes; how should the counselor help implement it?

A. Discuss the device with Margaret and her daughter, seek her consent or assent, review its data privacy, and tie it to the plan

B. Have the daughter slip the tracker into Margaret's purse without telling her, so that she is spared any distress about it

C. Rely on the device in place of the other safety steps in her plan, since its alerts will flag any problem as it happens

D. Leave the device choice and setup entirely to the family, since technology tools fall outside the scope of counseling

Question 12. Later, after a fall, Margaret begins using a wheelchair, and the counselor's office is up a flight of stairs with no elevator; what is the most appropriate response?

A. Have her daughter and office staff help carry Margaret and her wheelchair up the stairs to the counselor's office for each visit

B. Move her sessions to an accessible ground-floor room or other accessible space at no added cost, and ask what else she needs

C. Switch Margaret to phone-only sessions from now on, since the stairs make in-person counseling impossible for her to attend

D. Pause her counseling until she can manage the stairs again, since the building's layout is not the counselor's responsibility

Question 13. The counselor has limited training in geriatric neurocognitive care. The most ethical course of action is to:

A. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

B. Seek consultation and coordinate with a geriatric or medical team, or refer for the components beyond your competence

C. Continue managing the entire case alone to preserve the continuity of the relationship you have built with the family

D. Tell the family the situation is not serious enough to involve any medical or geriatric specialists and continue supportive talks

Answer Key

Case 1: Twenty years of feeling "flat" in a small hardware-store town

Diagnosis: Persistent Depressive Disorder

Q1 — Answer: A. That his low mood has been present most of the day, more days than not, for at least two years, without a symptom-free stretch longer than two months

Why it is correct: Chronic duration is the defining PDD criterion. *Common error:* Treating chronic low mood as simply a personality trait.

- **B** — Family history supports but does not confirm the diagnosis. *Trap:* Requiring family history to confirm a chronic mood disorder.
- **C** — A precise onset date does not establish chronicity. *Trap:* Requiring an exact onset before assessing duration.
- **D** — Day-to-day variation is not the diagnostic anchor. *Trap:* Focusing on situational triggers instead of overall course.

Q2 — Answer: A. Follow the practice's emergency plan for outages, confirm Tyrone's safety, and finish or reschedule using its backup procedures

Why it is correct: The practice's plan sets safety and backup steps. *Common error:* Improvising instead of following the established outage plan.

- **B** — A personal app puts his health information at risk. *Trap:* Using unsecured personal devices as an outage workaround.
- **C** — Ending without a plan disrupts his care. *Trap:* Dropping continuity of care during a minor emergency.
- **D** — Leaving him alone neglects his safety. *Trap:* Prioritizing the building problem over the client in session.

Q3 — Answer: B. Consult a colleague trained in CBASP on applying it with Tyrone while the counselor builds skill through formal training

Why it is correct: Consultation supports competence while skill is still developing. *Common error:* Treating reading a manual as sufficient competence.

- **A** — Reading alone does not establish competence in a modality. *Trap:* Practicing beyond training because the diagnosis is familiar.
- **C** — Immediate referral is unnecessary when consultation can bridge gaps. *Trap:* Assuming certification is required for every technique.
- **D** — Public forums risk confidentiality and lack accountable expertise. *Trap:* Mistaking crowd advice for structured consultation.

Q4 — Answer: C. Help him rank the concerns by their impact and his readiness to change, then start with one or two targets that he chooses

Why it is correct: Collaborative ranking makes a chronic problem list workable. *Common error:* Letting the counselor's theory set priorities alone.

- **A** — Spreading effort across everything dilutes early progress. *Trap:* Equating doing everything with honoring his wishes.
- **B** — Priorities should reflect his input, not only theory. *Trap:* Imposing the counselor's formulation as the plan's order.
- **D** — Deferring to his wife removes his own voice. *Trap:* Handing an adult client's priorities to a family member.

Q5 — Answer: D. Use a structured suicide-risk screen to confirm and document the absence of ideation, intent, plan, or prior behavior

Why it is correct: Structured screening confirms and documents low risk. *Common error:* Skipping a structured screen because risk seems low.

- **A** — Delaying a baseline safety check is unnecessary and unsafe practice. *Trap:* Postponing a routine safety baseline.
- **B** — Skipping the screen leaves risk undocumented. *Trap:* Assuming calm demeanor rules out the need to screen.
- **C** — No-suicide contracts are not evidence-based practice. *Trap:* Substituting a written promise for real assessment.

Q6 — Answer: D. With his signed release, agree on what may be shared, then involve his wife in the weekend-routine goals that Tyrone selected

Why it is correct: His consent defines what support-system collaboration may include. *Common error:* Assuming family involvement means sharing everything.

- **A** — Sharing beyond his consent breaches confidentiality. *Trap:* Treating a referring spouse as entitled to full disclosure.
- **B** — Turning his wife into a monitor strains the relationship. *Trap:* Converting family support into daily surveillance of him.
- **C** — Refusing involvement ignores his request and a useful resource. *Trap:* Excluding supports the client wants included.

Q7 — Answer: D. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

Why it is correct: Repeated standardized measures guide treatment decisions. *Common error:* Relying only on subjective impressions of progress.

- **A** — An informal check-in lacks a standardized benchmark. *Trap:* Trusting unstandardized self-report alone.

- **B** — A single end-point review misses early lack of progress. *Trap:* Skipping interim outcome monitoring.
- **C** — Attendance and adherence are necessary but not sufficient. *Trap:* Mistaking engagement for symptom improvement.

Q8 — Answer: C. Provide general information and, with his consent, coordinate a referral to a prescriber for a medication evaluation

Why it is correct: Referral and coordination stay within counselor scope. *Common error:* Offering specific medication advice directly.

- **A** — Discouraging a medical evaluation overreaches the counselor's role. *Trap:* Steering a medical decision away from a prescriber.
- **B** — Outsourcing to self-research replaces proper coordination of care. *Trap:* Replacing referral with unsupervised self-research.
- **D** — Naming a specific medication exceeds counselor scope. *Trap:* Overstepping into medical decision-making.

Q9 — Answer: C. Collaboratively schedule one or two small, valued activities each week, such as a fishing trip he used to enjoy

Why it is correct: Graded valued activity is an appropriate first behavioral activation step. *Common error:* Waiting for motivation to return before scheduling activity.

- **A** — Waiting for motivation reverses behavioral activation logic. *Trap:* Letting mood gate all behavioral change.
- **B** — Too much change too quickly invites early failure. *Trap:* Ignoring the need for graded pacing.
- **D** — Thought logging is useful but is not the first behavioral step here. *Trap:* Confusing cognitive work with the initial behavioral step.

Q10 — Answer: A. Offer psychoeducation that chronic low mood is a treatable condition, while validating how normal it felt to him for years

Why it is correct: Psychoeducation reframes an ego-syntonic belief without dismissing his experience. *Common error:* Either lecturing him or only agreeing that it is just who he is.

- **B** — Avoiding the belief leaves a central misconception unaddressed. *Trap:* Avoiding material central to engagement in treatment.
- **C** — Agreeing that it is unchangeable forecloses treatment. *Trap:* Colluding with the client's mistaken belief.
- **D** — Blunt correction risks feeling invalidating this early. *Trap:* Disputing before validating the client's experience.

Q11 — Answer: A. Acknowledge his doubt as understandable and collaboratively set one small, achievable goal to test it against

Why it is correct: Naming the doubt and testing it behaviorally builds realistic hope. *Common error:* Trying to argue him out of his doubt.

- **B** — A lecture rarely shifts longstanding skepticism by itself. *Trap:* Substituting information for a concrete experience of change.
- **C** — Agreeing with hopelessness abandons a treatable target. *Trap:* Matching the client's hopelessness instead of gently challenging it.
- **D** — Guaranteeing an outcome risks credibility if progress is gradual. *Trap:* Making promises that cannot reliably be kept.

Q12 — Answer: A. Explain that the practice's policy requires keeping his records for a set period, and describe how they are secured from others

Why it is correct: Retention policy governs records; security addresses his worry. *Common error:* Destroying or altering records to ease a client's worry.

- **B** — Early destruction violates retention policy and law. *Trap:* Treating client ownership of information as control over records.
- **C** — Altering a record misrepresents what occurred. *Trap:* Editing documentation to suit the client's comfort.
- **D** — Off-site shadow notes are unsecured and outside policy. *Trap:* Keeping a hidden second record to manage privacy.

Q13 — Answer: B. Discuss the likelihood of unavoidable public encounters with him in advance and agree on how to handle them respectfully

Why it is correct: Proactively addressing unavoidable overlap protects the client and the relationship. *Common error:* Ignoring foreseeable overlap until it happens unexpectedly.

- **A** — Automatic termination is unnecessary and may not be feasible in a rural area. *Trap:* Refusing rural clients over any foreseeable public overlap.
- **C** — Asking the client to change his livelihood is an inappropriate boundary solution. *Trap:* Placing the burden of boundary management on the client's job.
- **D** — Avoiding the topic leaves the client unprepared for a foreseeable situation. *Trap:* Failing to plan for a predictable boundary issue.

Case 2: Dazed and hypervigilant three weeks after a car crash

Diagnosis: Acute Stress Disorder

Q1 — Answer: D. That she directly experienced the threatened injury of the crash and that it has been under one month since it happened

Why it is correct: Exposure plus the ASD time window anchors the diagnosis. *Common error:* Diagnosing from symptom count alone without confirming exposure and timing.

- **A** — Precise recall of the crash mechanics is not diagnostic. *Trap:* Treating narrative precision as a diagnostic requirement.
- **B** — Insurance status is irrelevant to the diagnosis. *Trap:* Confusing administrative closure with clinical readiness.
- **C** — Third-party corroboration is not required to diagnose. *Trap:* Requiring outside witnesses before accepting the client's report.

Q2 — Answer: B. Review her case and proposed plan with the clinical supervisor and document the supervisor's input in the supervision record

Why it is correct: Supervision guides a new presentation for a supervisee. *Common error:* Acting alone and informing the supervisor afterward.

- **A** — Waiting months leaves the supervisor out of key decisions. *Trap:* Treating supervision as an after-the-fact formality.
- **C** — An informal peer cannot replace the responsible supervisor. *Trap:* Substituting peer approval for clinical supervision.
- **D** — Supervisees may treat trauma with supervision. *Trap:* Assuming licensure status bars all trauma cases.

Q3 — Answer: C. A structured suicide risk screen covering ideation, intent, plan, and behavior, done routinely regardless of apparent risk

Why it is correct: Routine structured screening is standard practice. *Common error:* Skipping safety screening because risk seems unlikely.

- **A** — Prior hospitalization alone does not assess current risk. *Trap:* Substituting history for a current risk assessment.

- **B** — Skipping screening based on appearance is unsafe practice. *Trap:* Assuming a calm presentation rules out risk.
- **D** — Deferring routine safety screening is inappropriate. *Trap:* Postponing a standard safety screen.

Q4 — Answer: C. Strengths in her motivation and her supportive family nearby; barriers in her avoidance, hypervigilance, and poor focus

Why it is correct: Stated motivation and support are strengths; avoidance blocks recovery. *Common error:* Mislabeling avoidance or vigilance as strengths.

- **A** — The detour is avoidance, and support is a strength. *Trap:* Counting short-term relief as a treatment strength.
- **B** — Vivid memory is a symptom, not the key barrier. *Trap:* Listing demographics instead of functional strengths.
- **D** — Hypervigilance is a symptom and openness is a strength. *Trap:* Reversing which factors help or hinder her goals.

Q5 — Answer: D. Record the medication, the fogginess, and its timing as Emily's own self-report, without the counselor attributing a cause

Why it is correct: Self-reported effects are documented accurately, without inference. *Common error:* Interpreting or omitting medication effects in notes.

- **A** — Dose advice is outside counselor scope. *Trap:* Turning a documentation task into medication management.
- **B** — Reported effects belong in the counseling record too. *Trap:* Treating medication as only the prescriber's concern.
- **C** — Attributing the cause is speculation in the record. *Trap:* Reframing a reported effect as a psychiatric symptom.

Q6 — Answer: A. Provide the notice now and make a good-faith effort to obtain her written acknowledgment that she received it, documenting it

Why it is correct: Direct providers must give the notice and seek acknowledgment. *Common error:* Confusing the privacy notice with an authorization.

- **B** — An authorization is a separate form for specific disclosures. *Trap:* Equating a blanket authorization with the privacy notice.
- **C** — Posting is required but does not replace giving the notice. *Trap:* Treating the posted notice as the only duty.
- **D** — Acknowledgment cannot be made a condition of care. *Trap:* Withholding treatment over an unsigned acknowledgment.

Q7 — Answer: C. Track her symptom cluster and functioning at each session to note whether it resolves or persists past one month

Why it is correct: Ongoing tracking clarifies whether ASD resolves or persists. *Common error:* Setting the plan once and never reassessing it.

- **A** — Third-party workplace comments are an unreliable clinical measure. *Trap:* Substituting outside opinions for direct assessment.
- **B** — Attendance alone does not measure clinical progress. *Trap:* Mistaking attendance for clinical improvement.
- **D** — A single end-point check misses the ASD-to-PTSD window. *Trap:* Skipping interim monitoring entirely.

Q8 — Answer: D. Provide general information and, with her consent, coordinate a referral to a prescriber for a full evaluation

Why it is correct: Referral and coordination stay within scope. *Common error:* Offering specific medication recommendations directly.

- **A** — Self-directed medication research is poor care coordination. *Trap:* Outsourcing a medical question instead of referring.
- **B** — Discouraging a medical evaluation overreaches. *Trap:* Steering a client away from a medical decision.
- **C** — Naming a specific medication exceeds scope. *Trap:* Overstepping professional scope of practice.

Q9 — Answer: B. Teach brief grounding techniques, such as naming sensory details in the room, to use during dazed or unreal moments

Why it is correct: Grounding directly targets dissociative episodes. *Common error:* Jumping to exposure before she can ground herself.

- **A** — Exposure before stabilization skips a needed step. *Trap:* Skipping stabilization before trauma processing.
- **C** — Dismissing the symptom is invalidating and unhelpful. *Trap:* Telling a client to ignore a distressing symptom.
- **D** — Blanket avoidance reinforces the symptom pattern. *Trap:* Prescribing more avoidance instead of coping skills.

Q10 — Answer: C. Normalize the avoidance as a common, treatable stress response rather than a personal weakness, and validate her frustration

Why it is correct: Psychoeducation reframes self-blame around avoidance. *Common error:* Either lecturing her or only sympathizing without information.

- **A** — Endorsing the self-blame deepens it. *Trap:* Colluding with a client's harsh self-judgment.
- **B** — Premature confrontation can feel invalidating. *Trap:* Disputing before validating the client's experience.

- **D** — Avoidance of the topic leaves the belief unaddressed. *Trap:* Avoiding material that feels clinically important.

Q11 — Answer: D. Listen empathically, reflect her distress, and gently pace the discussion so she does not feel overwhelmed or flooded

Why it is correct: Paced empathic listening supports processing without flooding. *Common error:* Letting the client become overwhelmed without pacing.

- **A** — Dismissing the nightmare's significance is invalidating. *Trap:* Telling a client their symptom does not matter.
- **B** — Pushing for more graphic detail risks flooding her. *Trap:* Prioritizing narrative detail over clinical pacing.
- **C** — Avoiding the material leaves it unprocessed. *Trap:* Steering away from distressing content out of discomfort.

Q12 — Answer: B. Review current trauma guidelines and trials, which find single-session debriefing unsupported and favor trauma-focused CBT

Why it is correct: Guidelines and trials define empirical support. *Common error:* Mistaking colleague endorsement for research evidence.

- **A** — Anecdotal experience is not empirical support. *Trap:* Equating clinical testimonials with outcome research.
- **C** — Workshop popularity does not equal evidence. *Trap:* Assuming any training reflects current research.
- **D** — Preference shapes choice but does not define evidence. *Trap:* Confusing client preference with empirical support.

Q13 — Answer: C. Explain the release-of-information process, obtain her signed written authorization, and coordinate the disclosure appropriately

Why it is correct: A signed release properly authorizes any disclosure. *Common error:* Sending records out without a proper signed authorization.

- **A** — A blanket refusal misstates how disclosures work. *Trap:* Refusing outright instead of following the proper process.
- **B** — Releasing records without written authorization is improper. *Trap:* Skipping the required written authorization step.
- **D** — An informal verbal summary bypasses proper documentation procedure. *Trap:* Substituting an informal verbal account for a proper release.

Case 3: From prescribed pain pills to daily opioid use

Diagnosis: Opioid Use Disorder, Severe

Q1 — Answer: B. The number of DSM-5-TR criteria he meets over twelve months, since six or more place severity in the severe range

Why it is correct: Criterion count sets severity. *Common error:* Estimating severity by drug type alone.

- **A** — Quantity alone does not set the diagnosis. *Trap:* Equating dose with severity.
- **C** — Family history is risk, not diagnosis. *Trap:* Using family history to diagnose.
- **D** — Route of onset does not set the diagnosis. *Trap:* Using the origin story to diagnose.

Q2 — Answer: C. Link each finding to a plan element, such as cravings and withdrawal to MOUD referral and lost activities to rebuilt routines

Why it is correct: Specific findings should shape specific plan objectives. *Common error:* Treating assessment as separate from planning.

- **A** — Ignoring results leaves key needs unaddressed. *Trap:* Filing assessment data without using it.

- **B** — A severity label alone misses his specific profile. *Trap:* Using a one-size package for a diagnosis.
- **D** — Waiting for more testing delays urgent care. *Trap:* Over-assessing before acting on clear findings.

Q3 — Answer: A. Assess overdose risk and withdrawal safety, and provide overdose-prevention education including naloxone access

Why it is correct: Address overdose risk and harm reduction. *Common error:* Overlooking overdose risk in the plan.

- **B** — Minimizing overdose risk is unsafe. *Trap:* Downplaying a lethal risk.
- **C** — Root-cause work is not the safety priority. *Trap:* Prioritizing insight over immediate safety.
- **D** — Unsupervised tapering can be dangerous. *Trap:* Recommending an unsupervised taper.

Q4 — Answer: D. Follow the disconnection plan set at the start of telehealth: call his backup number, then use the safety protocol if unreachable

Why it is correct: A preset disconnection plan guides the response. *Common error:* Treating a dropped session as purely technical.

- **A** — Passive waiting ignores a possible safety concern. *Trap:* Leaving reconnection to a high-risk client.
- **B** — A text alone underreacts given his overdose risk. *Trap:* Ignoring the clinical context of the dropped call.
- **C** — Jumping to emergency services skips direct contact. *Trap:* Escalating before attempting the backup plan.

Q5 — Answer: A. With his written consent, give the probation officer information on MOUD as evidence-based care and support his treatment

Why it is correct: Written-consent-based education advocates for evidence-based care. *Common error:* Abandoning or hiding needed treatment instead of advocating.

- **B** — Concealment risks his probation and his trust. *Trap:* Advising secrecy instead of addressing the barrier.
- **C** — Dropping MOUD sacrifices his first-line, evidence-based care. *Trap:* Accepting a misinformed barrier as final.
- **D** — Advocacy without consent breaches his autonomy. *Trap:* Acting on the client's behalf without his agreement.

Q6 — Answer: B. Coordinate a referral for medication for opioid use disorder, such as buprenorphine or methadone, alongside counseling

Why it is correct: MOUD is first-line; refer and coordinate. *Common error:* Offering counseling alone for severe OUD.

- **A** — Gatekeeping delays life-saving care. *Trap:* Conditioning treatment on prior abstinence.
- **C** — Psychosocial support alone is insufficient. *Trap:* Omitting medication from the plan.
- **D** — Withholding MOUD contradicts the evidence. *Trap:* Repeating the substitution myth.

Q7 — Answer: B. Use a multidimensional assessment of withdrawal risk, environment, and supports to match him to a safe level of care

Why it is correct: Use multidimensional placement. *Common error:* Defaulting to one fixed program.

- **A** — Maximal care ignores placement criteria. *Trap:* Over-placing without assessment.
- **C** — Under-placement ignores his risk. *Trap:* Under-treating a high-risk presentation.
- **D** — Preference cannot override clinical need. *Trap:* Letting convenience drive placement.

Q8 — Answer: B. His probation terms, the back injury that started his opioid use, his recent job loss, and his friend's overdose death

Why it is correct: Legal, medical, work, and loss context shape his recovery. *Common error:* Treating the substance use apart from its context.

- **A** — Excluding context leaves major relapse drivers unaddressed. *Trap:* Narrowing the plan to the diagnosis alone.
- **C** — Occupational stereotypes are not individualized context. *Trap:* Assuming preferences from a group identity.
- **D** — Grief matters but is one factor among several. *Trap:* Fixing on one factor and missing the rest.

Q9 — Answer: C. Do not bill his insurer for the missed visit, and apply any no-show fee only as his signed policy and the payer contract allow

Why it is correct: Insurers pay only for services rendered. *Common error:* Billing a payer for a session that did not occur.

- **A** — Billing for an unprovided service is fraudulent. *Trap:* Treating reserved time as a billable service.
- **B** — Billing a shorter code still misrepresents care. *Trap:* Downcoding a no-show into a billable service.
- **D** — Probation is not a payer without an agreement. *Trap:* Shifting costs to a third party without consent.

Q10 — Answer: C. Frame the return to use as common in recovery, review overdose risk after abstinence, and revisit the plan without shaming him

Why it is correct: Treat the lapse as learning and address risk. *Common error:* Treating a lapse as total failure.

- **A** — Shaming deepens the spiral. *Trap:* Amplifying shame after a lapse.
- **B** — Dismissing the lapse skips safety. *Trap:* Glossing over a dangerous lapse.

- **D** — One lapse does not mandate escalation. *Trap:* Over-escalating after one slip.

Q11 — Answer: A. Decline to confirm or deny without a valid release, explaining the heightened confidentiality of substance-use information

Why it is correct: Protect SUD records without a release. *Common error:* Disclosing SUD treatment without consent.

- **B** — Confirming attendance is still disclosure. *Trap:* Treating attendance as not protected.
- **C** — Abrupt refusal misses a teachable moment. *Trap:* Being needlessly obstructive.
- **D** — Confirming breaches confidentiality. *Trap:* Disclosing without authorization.

Q12 — Answer: D. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them

Why it is correct: Defer dosing; coordinate with consent. *Common error:* Advising on a specific medication dose.

- **A** — Refusing to coordinate abandons the need. *Trap:* Declining to support communication with the prescriber.
- **B** — Directing a dose is unsafe and out of scope. *Trap:* Giving specific pharmacologic direction.
- **C** — Naming a dose exceeds scope. *Trap:* Overstepping into medication management.

Q13 — Answer: B. Obtain his specific, informed written consent for each party before sharing any of his protected information

Why it is correct: Get a valid release for each party. *Common error:* Coordinating before obtaining consent.

- **A** — Justice involvement does not erase protections. *Trap:* Assuming probation negates confidentiality.
- **C** — Each disclosure should be his informed choice. *Trap:* Making release decisions for the client.

- **D** — Verbal interest is not a valid release. *Trap:* Disclosing before consent is documented.

Case 4: Feeling detached and unreal, as if watching life from outside

Diagnosis: Depersonalization/Derealization Disorder

Q1 — Answer: C. Persistent or recurrent detachment or unreality with intact reality testing, causing significant distress or impairment

Why it is correct: Persistent detachment with intact reality testing. *Common error:* Diagnosing from a single brief episode.

- **A** — A genuine belief in unreality suggests psychosis. *Trap:* Confusing the disorder with psychosis.
- **B** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **D** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.

Q2 — Answer: D. Take reputable training on graduate-student stressors and help-seeking, applying it while still assessing Owen as an individual

Why it is correct: Reputable population-specific training, applied without stereotyping him. *Common error:* Substituting generalizations or the client for real training.

- **A** — Unvetted trait lists breed stereotyping, not competence. *Trap:* Mistaking group generalizations for population training.
- **B** — It shifts the counselor's learning burden onto the client. *Trap:* Making the client the counselor's teacher.
- **C** — Context shapes stressors, help-seeking, and how symptoms are described. *Trap:* Treating one old course as sufficient preparation.

Q3 — Answer: A. Assess the incident under the practice's breach procedure and, unless compromise is shown unlikely, notify Owen within 60 days

Why it is correct: HIPAA requires a risk assessment and timely notice to affected clients; state laws may add duties. *Common error:* Assuming the vendor's breach is not the practice's duty.

- **B** — The vendor must notify the practice, which remains responsible for notifying Owen. *Trap:* Assuming a vendor's fault shifts the practice's duty.
- **C** — Notice turns on the probability of compromise, not proven misuse. *Trap:* Waiting for harm before notifying the client.
- **D** — Consumer apps lack required safeguards and agreements. *Trap:* Abandoning compliant platforms after one incident.

Q4 — Answer: B. Treat the numbness as an early sign of compassion fatigue and act now with self-care, consultation, and caseload limits

Why it is correct: Early warning signs call for prompt, proactive self-care steps. *Common error:* Waiting until impairment shows before addressing fatigue.

- **A** — It misattributes the counselor's depletion to the client. *Trap:* Explaining away compassion fatigue as client mirroring.
- **C** — Delay lets compassion fatigue grow and erode care. *Trap:* Postponing self-care until care visibly suffers.
- **D** — Disclosure serves the counselor and burdens the client. *Trap:* Using self-disclosure to manage the counselor's state.

Q5 — Answer: C. Notes linking his diagnosis, current impairment, and plan goals to each session's interventions and his response to them

Why it is correct: Each note must show necessity, interventions, and response. *Common error:* Treating the diagnosis code as sufficient support.

- **A** — A code alone does not document ongoing medical necessity. *Trap:* Letting the claim's code stand in for notes.
- **B** — Attendance-only notes cannot support a billed psychotherapy service. *Trap:* Trading adequate documentation for privacy concerns.
- **D** — Cloned notes misrepresent what each session actually provided. *Trap:* Copying notes forward instead of documenting each session.

Q6 — Answer: D. Cognitive behavioral therapy with psychoeducation and grounding, addressing catastrophic appraisals of the symptoms

Why it is correct: CBT, grounding, and reappraisal. *Common error:* Choosing an approach that ignores the catastrophic appraisal.

- **A** — It is not psychosis; antipsychotics are not indicated. *Trap:* Treating dissociation as psychosis.
- **B** — Insight-only is not first-line here. *Trap:* Prioritizing origins over symptom management.
- **C** — Avoidance maintains the disorder. *Trap:* Sanctioning avoidance as coping.

Q7 — Answer: B. Help him reduce hypervigilant self-monitoring, since constantly checking the sensations tends to intensify and maintain them

Why it is correct: Reduce hypervigilant self-monitoring. *Common error:* Leaving the self-monitoring in place.

- **A** — Detailed logging feeds the focus. *Trap:* Assigning a counterproductive monitoring task.
- **C** — Avoidance maintains the disorder. *Trap:* Sanctioning avoidance.
- **D** — More monitoring worsens symptoms. *Trap:* Encouraging symptom-checking.

Q8 — Answer: B. Coordinate a referral to a prescriber for evaluation, especially of comorbid anxiety or depression, while continuing therapy

Why it is correct: Refer and coordinate within scope. *Common error:* Giving specific medication advice.

- **A** — Naming a drug and dose exceeds scope. *Trap:* Overstepping professional limits.
- **C** — Discouraging meds overreaches. *Trap:* Steering a medical decision.
- **D** — Outsourcing to self-research is poor care. *Trap:* Replacing referral with self-directed search.

Q9 — Answer: C. Validate the fear and provide psychoeducation that these symptoms, while frightening, are a recognized and treatable condition

Why it is correct: Validate and psychoeducate to reduce the fear. *Common error:* Either dismissing the fear or reinforcing it.

- **A** — Dismissive reassurance can invalidate. *Trap:* Reassuring without educating.
- **B** — Confirming the catastrophe deepens the fear. *Trap:* Validating the feared interpretation.
- **D** — Lecturing rarely shifts the fear. *Trap:* Substituting dense facts for engagement.

Q10 — Answer: B. Guide him through grounding that reconnects him with his senses and surroundings to reduce the intensity of the episode

Why it is correct: Use grounding to reconnect with the present. *Common error:* Leaving him to endure the episode without grounding.

- **A** — Focusing inward intensifies the episode. *Trap:* Amplifying the symptom focus.
- **C** — Passive waiting prolongs the episode. *Trap:* Failing to intervene with grounding.
- **D** — Abruptly ending models avoidance. *Trap:* Reinforcing escape from the experience.

Q11 — Answer: D. Assess cumulative trauma exposure and symptoms with a tool such as the ProQOL and review the results in supervision

Why it is correct: Systematically assess trauma exposure and its effects with support. *Common error:* Missing vicarious trauma that appears outside sessions.

- **A** — Shifts in safety beliefs are hallmark signs of vicarious trauma. *Trap:* Dismissing symptoms because they occur outside work.
- **B** — It silences the client to protect the counselor. *Trap:* Restricting client disclosure to limit counselor exposure.
- **C** — Acting before assessing skips the needed exposure review. *Trap:* Making abrupt caseload changes without any assessment.

Q12 — Answer: D. Explaining the rationale, that some exercises may briefly heighten the sensations, and that participation remains voluntary

Why it is correct: Disclose rationale, discomfort, and choice. *Common error:* Starting exposure-based work without informed consent.

- **A** — Consent is voluntary and revocable. *Trap:* Treating consent as a binding commitment.
- **B** — Misrepresenting the method is dishonest. *Trap:* Misstating what the treatment involves.
- **C** — Withholding the method violates consent. *Trap:* Intervening before explaining.

Q13 — Answer: A. Explain that the cannabis use is clinically relevant to his symptoms and address it collaboratively rather than ignoring it

Why it is correct: Address the clinically relevant substance use. *Common error:* Agreeing to leave out clinically relevant information.

- **B** — Ignoring relevant use is poor care. *Trap:* Excluding material information at the client's request.

- **C** — A coercive ultimatum harms engagement. *Trap:* Issuing an ultimatum that ruptures the alliance.
- **D** — Avoiding the topic abandons the issue. *Trap:* Sidestepping a central clinical issue.

Case 5: Recurrent depression with overlooked periods of elevated energy

Diagnosis: Bipolar II Disorder

Q1 — Answer: C. A history of at least one hypomanic episode in addition to her major depressive episodes, with no full manic episode

Why it is correct: Hypomania plus depression defines bipolar II. *Common error:* Diagnosing unipolar depression by missing the hypomania.

- **A** — Full mania would indicate bipolar I. *Trap:* Applying manic-episode criteria to bipolar II.
- **B** — Current severity is not the criterion. *Trap:* Requiring worsening to diagnose.
- **D** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: A. As one trend source alongside her mood reports, noting that consumer wearables are not validated diagnostic tools

Why it is correct: Wearable data supplements, not replaces, clinical monitoring. *Common error:* Treating consumer device output as diagnostic.

- **B** — Consumer sleep scores are not validated to detect hypomania. *Trap:* Over-trusting a device as an objective diagnostic.
- **C** — Client-shared device data can usefully inform her monitoring plan. *Trap:* Rejecting helpful technology out of rigid caution.
- **D** — It creates a round-the-clock response the counselor cannot provide. *Trap:* Implying monitoring coverage the counselor cannot deliver.

Q3 — Answer: B. Name it as a personal limitation in assessment and build routine screening for past elevated mood into every intake

Why it is correct: Honest self-assessment turns a limitation into a practice change. *Common error:* Externalizing or overcorrecting a recognized assessment limitation.

- **A** — One near miss signals a fixable gap, not incompetence. *Trap:* Overcorrecting a limitation with an unneeded transfer.
- **C** — Clients rarely volunteer hypomania; asking is the counselor's job. *Trap:* Blaming the client for the counselor's gap.
- **D** — A real strength cannot replace systematic screening for hypomania. *Trap:* Leaning on a strength to cover a limitation.

Q4 — Answer: C. Ask what works best for her and provide free aids such as captioning, a quieter room, or written session summaries

Why it is correct: Ask the client and provide effective aids at no cost. *Common error:* Choosing accommodations without asking what works for her.

- **A** — Relying on family compromises communication and confidentiality. *Trap:* Using a companion in place of real accommodations.
- **B** — Guessing at supports ignores her own expertise on her needs. *Trap:* Assuming an accommodation without asking the client.
- **D** — The ADA bars charging clients for needed communication aids. *Trap:* Passing accommodation costs on to the client.

Q5 — Answer: C. A structured suicide-risk assessment, recognizing that suicide risk is elevated in bipolar depression

Why it is correct: Assess elevated bipolar-depression risk. *Common error:* Overlooking elevated suicide risk in bipolar depression.

- **A** — Medication history is useful but secondary now. *Trap:* Prioritizing history over the safety assessment.
- **B** — Sleep logging is secondary to risk. *Trap:* Confusing monitoring detail with risk assessment.
- **D** — Exhaustive work history is low yield. *Trap:* Collecting low-yield detail.

Q6 — Answer: C. Coordinate a psychiatric evaluation and share that antidepressant monotherapy is a concern in bipolar II, given the diagnosis

Why it is correct: Refer; flag antidepressant-monotherapy caution. *Common error:* Endorsing antidepressant monotherapy or prescribing.

- **A** — Endorsing monotherapy ignores the diagnosis. *Trap:* Endorsing a potentially destabilizing plan.
- **B** — Refusing to engage abandons the issue. *Trap:* Declining to coordinate a relevant medical issue.
- **D** — Naming a drug and dose exceeds scope. *Trap:* Overstepping into medication management.

Q7 — Answer: D. Slow the counselor’s breathing and speech and keep an open posture, then calmly ask direct questions about her thoughts

Why it is correct: Self-regulate first so a calm, direct risk inquiry follows. *Common error:* Letting the counselor’s alarm drive the response.

- **A** — Avoiding the disclosure leaves her risk unassessed. *Trap:* Managing the counselor’s anxiety by avoiding the topic.
- **B** — It shifts the counselor’s emotion onto a distressed client. *Trap:* Disclosing alarm instead of regulating it.
- **C** — Risk level comes from assessment, not the counselor’s feelings. *Trap:* Treating the counselor’s emotion as risk data.

Q8 — Answer: B. Mood and sleep monitoring with early-warning-sign tracking so emerging hypomania or depression can be caught and managed

Why it is correct: Monitor mood, sleep, and early warning signs. *Common error:* Treating the current episode without ongoing monitoring.

- **A** — Attendance is not an outcome measure. *Trap:* Mistaking adherence for outcome.
- **C** — Ignoring elevation misses hypomania. *Trap:* Monitoring only one pole.
- **D** — A long monitoring gap risks missed episodes. *Trap:* Leaving a long gap without monitoring.

Q9 — Answer: B. Validate her reaction and provide psychoeducation about hypomania and bipolar II as understandable and manageable with treatment

Why it is correct: Validate and psychoeducate about bipolar II. *Common error:* Either alarming her or dismissing the new information.

- **A** — Lecturing rarely addresses the worry. *Trap:* Substituting dense facts for engagement.
- **C** — A grim, inaccurate framing deepens fear. *Trap:* Catastrophizing the diagnosis.
- **D** — Minimizing misrepresents the diagnosis. *Trap:* Downplaying a meaningful finding.

Q10 — Answer: A. Collaboratively identify her personal early warning signs, such as reduced sleep need and racing thoughts, and a plan to respond

Why it is correct: Identify personal early warning signs and a plan. *Common error:* Leaving her without a way to detect early episodes.

- **B** — Over-pathologizing normal good days is unhelpful. *Trap:* Pathologizing ordinary good days.
- **C** — Outsourcing all monitoring is insufficient. *Trap:* Removing the client from her own monitoring.

- **D** — Waiting for a full episode misses early intervention. *Trap:* Forgoing early warning-sign monitoring.

Q11 — Answer: B. Obtain her informed consent and release before sharing any clinical information with the primary care provider

Why it is correct: Get a valid release before coordinating. *Common error:* Disclosing before obtaining consent.

- **A** — Coordinating without consent breaches confidentiality. *Trap:* Disclosing before consent is documented.
- **C** — A diagnosis is protected information. *Trap:* Disclosing a diagnosis without consent.
- **D** — No coordination undercuts integrated care. *Trap:* Refusing helpful, consented coordination.

Q12 — Answer: C. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate her questions and progress with them

Why it is correct: Defer dosing; coordinate with consent. *Common error:* Advising on a specific medication dose.

- **A** — Refusing to help abandons the need. *Trap:* Declining to support communication with the prescriber.
- **B** — Directing dosing is unsafe and out of scope. *Trap:* Giving specific pharmacologic direction.
- **D** — Naming a dose exceeds scope. *Trap:* Overstepping into medication management.

Q13 — Answer: D. Seek consultation and coordinate with a prescriber and team, or refer for the components beyond your demonstrated competence

Why it is correct: Consult, coordinate, or refer within competence. *Common error:* Managing complex bipolar care alone without competence.

- **A** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.

- **B** — Minimizing the condition fails the client. *Trap:* Underestimating the need for coordinated care.
- **C** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.

Case 6: A fixed persecutory belief in an otherwise functioning man

Diagnosis: Delusional Disorder

Q1 — Answer: B. One or more delusions present for at least a month without other prominent active-phase psychotic symptoms

Why it is correct: Delusions without other psychotic symptoms. *Common error:* Diagnosing from a belief without confirming the full pattern.

- **A** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **C** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **D** — Depressive count does not define it. *Trap:* Applying depressive criteria here.

Q2 — Answer: A. Complete it only through the practice's encrypted EHR access, never saving copies to a personal device or account

Why it is correct: Work only within secured, authorized record systems. *Common error:* Creating unsecured copies of records outside the practice.

- **B** — Personal email is an unsecured channel for client records. *Trap:* Routing PHI through personal accounts for convenience.
- **C** — Unsecured paper records off-site risk loss and exposure. *Trap:* Carrying paper records home without any safeguards.
- **D** — Local copies leave PHI on an unsecured personal device. *Trap:* Believing deletion later makes local copies safe.

Q3 — Answer: B. The counselor's skills or access to consultation, availability for his level of risk, and no tie to Harold or the neighbor

Why it is correct: Suitability rests on competence, availability, and no conflict of interest. *Common error:* Basing fit on insight, similarity, or payment.

- **A** — Limited insight is expected in delusional disorder. *Trap:* Making insight a precondition for care.
- **C** — Shared background does not establish competence or fit. *Trap:* Confusing rapport potential with suitability.
- **D** — Fee level is not a clinical suitability factor. *Trap:* Letting reimbursement drive the decision.

Q4 — Answer: C. Explain that as his treating counselor the counselor should not give forensic opinions, and consult an attorney first

Why it is correct: Avoid forensic roles with clients; get legal guidance first. *Common error:* Offering forensic opinions about one's own client.

- **A** — Vouching for safety is a forensic opinion beyond the treating role. *Trap:* Advocating in court as the treating counselor.
- **B** — An unsolicited letter discloses records and blurs roles. *Trap:* Volunteering clinical information to a court.
- **D** — Subpoenas require a legal response, not silence. *Trap:* Ignoring legal process to protect confidentiality.

Q5 — Answer: D. Assess risk to the neighbor and to himself, including intent and means, and prepare to act on duty-to-protect obligations

Why it is correct: Assess risk to others and self. *Common error:* Letting a possible threat pass without assessment.

- **A** — Narrow self-only focus misses risk to others. *Trap:* Assessing only one channel of risk.

- **B** — Past nonviolence does not remove the need to assess. *Trap:* Accepting reassurance without assessment.
- **C** — Deferring risk assessment is unsafe. *Trap:* Postponing an urgent risk assessment.

Q6 — Answer: B. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside supportive psychological care

Why it is correct: Psychiatric eval plus supportive care. *Common error:* Offering counseling alone for delusional disorder.

- **A** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.
- **C** — Insight-only is not first-line. *Trap:* Prioritizing insight over indicated care.
- **D** — Counseling alone is insufficient. *Trap:* Omitting medication coordination.

Q7 — Answer: D. Use it only after completing its required training, consulting a qualified evaluator on his risk in the meantime

Why it is correct: Competence requires instrument-specific training, not just licensure. *Common error:* Equating licensure or purchase eligibility with competence.

- **A** — A license does not confer competence with every instrument. *Trap:* Assuming a license covers every assessment instrument.
- **B** — Purchase eligibility is not proof of competence in use. *Trap:* Treating a publisher's approval as competence.
- **C** — Risk to the neighbor needs assessment now, with support. *Trap:* Dropping structured assessment while awaiting training.

Q8 — Answer: A. Ongoing risk monitoring and a plan that addresses safety, including protective steps if the risk to the neighbor escalates

Why it is correct: Monitor and plan for safety. *Common error:* Treating the delusion while ignoring the risk to others.

- **B** — Match level of care to assessed risk. *Trap:* Hospitalizing reflexively without assessment.
- **C** — Ignoring the risk is unsafe. *Trap:* Overlooking a potential risk to others.
- **D** — Encouraging confrontation increases risk. *Trap:* Advising an action that raises risk.

Q9 — Answer: D. Obtain his informed consent and release before sharing information, then collaborate as part of a coordinated team

Why it is correct: Consent and release before coordinating. *Common error:* Coordinating before obtaining consent.

- **A** — No coordination undercuts team care. *Trap:* Refusing helpful, consented coordination.
- **B** — Coordinating without consent breaches confidentiality. *Trap:* Disclosing before consent is documented.
- **C** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.

Q10 — Answer: C. Empathize with how distressing and isolating it feels to not be believed, without endorsing or attacking the belief itself

Why it is correct: Empathize with the distress, stay neutral on content. *Common error:* Either confirming or attacking the belief to manage his anger.

- **A** — Bluntly disputing entrenches the belief. *Trap:* Confronting the delusion to settle the conflict.
- **B** — Agreeing reinforces the delusion. *Trap:* Colluding to defuse the anger.
- **D** — Avoiding the moment misses an opening. *Trap:* Sidestepping the emotion rather than working it.

Q11 — Answer: D. Build trust and work on shared, concrete goals such as reducing his stress and distress that he is willing to engage with

Why it is correct: Engage via shared goals despite low insight. *Common error:* Requiring him to give up the belief before engaging.

- **A** — Conditioning care is inappropriate. *Trap:* Making counseling contingent on medication.
- **B** — Dwelling on the delusion can reinforce it. *Trap:* Centering the work on the delusional content.
- **C** — Requiring insight first stalls engagement. *Trap:* Setting insight as a precondition.

Q12 — Answer: A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

Why it is correct: Assess and act on duty to protect. *Common error:* Treating an identifiable threat as fully confidential.

- **B** — Confidentiality yields to a serious threat. *Trap:* Treating confidentiality as absolute when there is danger.
- **C** — Act on assessed risk, not reflexively. *Trap:* Disclosing before assessing the risk.
- **D** — Delaying assessment of a threat is unsafe. *Trap:* Postponing an urgent risk assessment.

Q13 — Answer: C. Seek consultation and coordinate with a treatment team, referring for the components beyond your demonstrated competence

Why it is correct: Consult, coordinate, or refer within competence. *Common error:* Managing complex psychosis alone without competence.

- **A** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.
- **B** — Minimizing the illness fails the client. *Trap:* Underestimating the need for specialist care.

- **D** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.

Case 7: A bright child whose reading falls far below expectation

Diagnosis: Specific Learning Disorder, With Impairment in Reading

Q1 — Answer: B. Reading skill well below age expectations that has persisted over six months despite targeted intervention

Why it is correct: Below-age skill persisting despite help. *Common error:* Diagnosing SLD without confirming persistence despite intervention.

- **A** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **C** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **D** — Depressive count does not define SLD. *Trap:* Applying depressive criteria here.

Q2 — Answer: A. Enter a dated, signed amendment correcting the date, leaving the original entry intact and visible in the audit trail

Why it is correct: Correct records by transparent, dated amendment. *Common error:* Erasing or hiding the original entry when correcting.

- **B** — Deleting signed entries undermines record integrity. *Trap:* Erasing the original instead of amending it.
- **C** — Hidden back-end edits defeat the record's audit trail. *Trap:* Concealing corrections to keep the chart looking clean.
- **D** — Signed notes can and should be corrected by amendment. *Trap:* Believing errors in signed notes must stand.

Q3 — Answer: B. Decline the tutoring role, explain how mixing roles could affect her counseling, and refer to reading specialists

Why it is correct: Avoid a dual role; redirect to qualified reading help. *Common error:* Taking on a second paid role with the family.

- **A** — Familiarity does not justify a potentially harmful dual role. *Trap:* Rationalizing a dual role as helpful.
- **C** — Moving the setting does not remove the dual role. *Trap:* Thinking logistics resolve a boundary problem.
- **D** — Reading struggles remain central to her counseling goals. *Trap:* Overcorrecting a boundary by avoiding clinical content.

Q4 — Answer: C. Check each suggestion against her evaluation and records, keeping final decisions and accountability with the counselor

Why it is correct: AI informs, but the counselor verifies and remains accountable. *Common error:* Deferring clinical judgment to AI-generated output.

- **A** — Volume of data does not ensure fit for this child. *Trap:* Trusting AI output over individualized judgment.
- **B** — Unverified AI output should not go straight into care. *Trap:* Sending AI suggestions out without clinical review.
- **D** — Transparency about AI use supports informed, accountable care. *Trap:* Hiding AI involvement because a human reviewed it.

Q5 — Answer: D. Reflect on how the counselor's own school history may be shaping these reactions and keep them from steering her care

Why it is correct: Self-reflection keeps personal history from steering care. *Common error:* Acting on countertransference instead of examining it.

- **A** — Lengthy self-disclosure centers the counselor's own story. *Trap:* Using disclosure to meet the counselor's needs.
- **B** — Personal anger is not evidence about the school's performance. *Trap:* Treating the counselor's countertransference as clinical data.
- **C** — Reactions call for reflection, not automatic referral. *Trap:* Overcorrecting a personal reaction with an abrupt referral.

Q6 — Answer: A. Coordinate educational support and accommodations with the school while addressing the emotional impact in counseling

Why it is correct: Coordinate school support; address the emotional impact. *Common error:* Trying to remediate the reading skill yourself instead of coordinating and supporting.

- **B** — More practice alone is not the treatment. *Trap:* Dismissing the need for structured support.
- **C** — Reading remediation is the educator's role. *Trap:* Taking on a role outside counseling scope.
- **D** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.

Q7 — Answer: A. Build her confidence and coping, reframing the difficulty as a specific, workable challenge rather than a defect

Why it is correct: Build confidence and reframe the difficulty. *Common error:* Focusing only on the skill while ignoring her eroding self-concept.

- **B** — Avoiding her feelings misses the work. *Trap:* Sidestepping the emotional impact.
- **C** — Effort-blaming is harmful and inaccurate. *Trap:* Implying the problem is insufficient effort.

- **D** — Repeated testing is not counseling. *Trap:* Turning sessions into skill testing.

Q8 — Answer: D. Track confidence, school avoidance, mood, and the use of accommodations over time to guide the support plan

Why it is correct: Track confidence, avoidance, mood, accommodations. *Common error:* Proceeding without tracking the counseling-relevant outcomes.

- **A** — Anecdote alone is insufficient. *Trap:* Substituting anecdote for measurement.
- **B** — Reading scores are the educator's metric. *Trap:* Measuring a metric outside the counseling role.
- **C** — End-only review misses steering data. *Trap:* Deferring all measurement to the end.

Q9 — Answer: C. With the parents' consent and a release, collaborate with the school team on appropriate accommodations and supports

Why it is correct: Consent and release before school coordination. *Common error:* Contacting the school without parental consent.

- **A** — Disclosing without consent breaches confidentiality. *Trap:* Disclosing to the school without consent.
- **B** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.
- **D** — No coordination undercuts the support. *Trap:* Refusing helpful, consented coordination.

Q10 — Answer: A. Validate how hard it feels, normalize the difficulty, and connect her to the supports that can make it manageable

Why it is correct: Validate, normalize, and connect to support. *Common error:* Either minimizing the difficulty or agreeing that giving up is reasonable.

- **B** — Endorsing giving up is harmful. *Trap:* Validating the wish to give up.

- **C** — Avoiding the feeling misses the work. *Trap:* Sidestepping a meaningful moment.
- **D** — Minimizing the difficulty invalidates her. *Trap:* Minimizing a genuine struggle.

Q11 — Answer: C. Use developmentally appropriate, strengths-based methods that highlight what she does well and build on it

Why it is correct: Developmentally matched, strengths-based engagement. *Common error:* Using deficit-focused, adult-oriented methods that deepen discouragement.

- **A** — Abstract methods do not fit a child. *Trap:* Pitching the work above her developmental level.
- **B** — Demanding improvement first stalls engagement. *Trap:* Setting skill improvement as a precondition.
- **D** — Skill drilling is not the counseling role. *Trap:* Taking on educational drilling in counseling.

Q12 — Answer: B. Decline to misrepresent her profile and clarify what the assessment and your role can accurately support

Why it is correct: Do not misrepresent; document accurately. *Common error:* Issuing an inaccurate label to satisfy the parents.

- **A** — Advocacy over accuracy is improper. *Trap:* Letting advocacy override accuracy.
- **C** — Flat refusal forecloses helpful options. *Trap:* Declining without clarifying the appropriate options.
- **D** — Falsifying a report is unethical. *Trap:* Misrepresenting the child's profile.

Q13 — Answer: C. Refer the formal evaluation to a qualified professional and stay within your supportive counseling role

Why it is correct: Refer evaluation; stay within role. *Common error:* Conducting a formal evaluation without the required competence.

- **A** — Testing without competence is unethical. *Trap:* Acting outside demonstrated competence.
- **B** — Minimizing the need misleads care. *Trap:* Discouraging an indicated evaluation.
- **D** — Abrupt transfer mishandles continuity. *Trap:* Dropping the family without coordination.

Case 8: Distressing physical symptoms and constant health worry

Diagnosis: Somatic Symptom Disorder

Q1 — Answer: A. Distressing somatic symptoms for over six months with excessive thoughts, feelings, and behaviors devoted to them

Why it is correct: Symptoms plus disproportionate response. *Common error:* Diagnosing from the symptoms alone without the excessive response.

- **B** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **C** — Recent change is not the criterion. *Trap:* Confusing a recent uptick with the criteria.
- **D** — Medically unexplained is not required. *Trap:* Requiring symptoms to be proven psychogenic.

Q2 — Answer: A. Follow the practice's medical emergency plan — call 911, stay with her, and alert the designated staff member to meet responders

Why it is correct: New acute symptoms trigger the practice's emergency plan. *Common error:* Dismissing new symptoms because of her diagnosis.

- **B** — Possible cardiac signs need emergency care, not grounding. *Trap:* Attributing every new symptom to the disorder.

- **C** — Detailed comparison delays a time-critical emergency response. *Trap:* Gathering history while an emergency unfolds.
- **D** — Driving with possible cardiac symptoms endangers her. *Trap:* Deferring an emergency to routine care.

Q3 — Answer: B. A counselor experienced in CBT for somatic symptom presentations, consulted in a structured way with de-identified details

Why it is correct: Consultation should match the specific skill gap. *Common error:* Choosing a consultant by convenience or rank rather than expertise.

- **A** — Physicians are not experts in counseling modalities. *Trap:* Consulting the wrong profession for a counseling skill.
- **C** — Informal social settings risk confidentiality and lack structure. *Trap:* Treating casual conversation as consultation.
- **D** — Seniority does not supply modality-specific expertise. *Trap:* Equating rank with relevant skill.

Q4 — Answer: C. Validate that her symptoms are real and distressing and coordinate with one consistent medical provider rather than dismissing them

Why it is correct: Validate symptoms; coordinate care. *Common error:* Telling her the symptoms are not real.

- **A** — Dismissive framing damages alliance. *Trap:* Invalidating the client's experience.
- **B** — Endless workups reinforce the disorder. *Trap:* Reinforcing the reassurance and testing cycle.
- **D** — Ordering tests exceeds counselor scope. *Trap:* Acting outside scope of practice.

Q5 — Answer: C. Review the practice's contact policy with her, reply within its stated limits, and bring the messages into session work

Why it is correct: Following the written policy sets safe, consistent limits. *Common error:* Bypassing policy to relieve her distress quickly.

- **A** — Reassurance replies breach limits and feed the cycle. *Trap:* Letting her anxiety override practice policy.
- **B** — Unexplained silence abandons her and ignores the policy. *Trap:* Setting limits without explaining them to the client.
- **D** — Personal texting bypasses secure, policy-governed channels. *Trap:* Creating an off-policy, unsecured route for client contact.

Q6 — Answer: D. Respond in writing within HIPAA's time limit and, if the diagnosis is accurate, explain the denial and her right to disagree

Why it is correct: HIPAA gives a right to request amendment, not removal; accurate entries may be kept. *Common error:* Altering an accurate record to ease her distress.

- **A** — Accurate entries may be kept despite objection. *Trap:* Treating the amendment right as a veto.
- **B** — HIPAA requires a timely response to amendment requests. *Trap:* Ignoring the client's right to request amendment.
- **C** — The counselor responds for the counselor's own record. *Trap:* Redirecting a request the counselor must answer.

Q7 — Answer: D. Coordinate regular, scheduled visits with one primary provider rather than visits driven by each new symptom or crisis

Why it is correct: Scheduled visits with one coordinator. *Common error:* Leaving care fragmented and symptom-driven.

- **A** — Rotating specialists fragments care. *Trap:* Encouraging doctor-shopping.

- **B** — Crisis-driven care reinforces the cycle. *Trap:* Reinforcing crisis-driven utilization.
- **C** — Cutting off medical care is inappropriate. *Trap:* Severing necessary medical involvement.

Q8 — Answer: C. Reducing reassurance-seeking and catastrophic symptom interpretations while gradually restoring valued activity

Why it is correct: Target reassurance-seeking and avoidance. *Common error:* Treating only the symptoms, not the maintaining behaviors.

- **A** — Activity restriction worsens function. *Trap:* Letting symptoms gate activity.
- **B** — Researching feeds reassurance-seeking. *Trap:* Reinforcing the maintaining behavior.
- **D** — Detailed monitoring fuels the focus. *Trap:* Encouraging hypervigilant monitoring.

Q9 — Answer: B. Validate that her symptoms and suffering are real and explain that counseling addresses the distress and impact, not their reality

Why it is correct: Affirm the symptoms are real; reframe the aim. *Common error:* Implying the symptoms are imaginary.

- **A** — This invalidates and alienates her. *Trap:* Invalidating the client's experience.
- **C** — Bypassing the concern harms engagement. *Trap:* Avoiding a key alliance issue.
- **D** — Repeated reassurance feeds the cycle. *Trap:* Providing reassurance that maintains the cycle.

Q10 — Answer: D. Gently name the reassurance-seeking as part of the cycle and help her practice tolerating uncertainty rather than answering it

Why it is correct: Name the pattern; build uncertainty tolerance. *Common error:* Answering to relieve her distress in the moment.

- **A** — Answering reinforces the cycle. *Trap:* Reinforcing the maintaining behavior.
- **B** — Shaming language harms the alliance. *Trap:* Invalidating instead of guiding.
- **C** — Another scan feeds the cycle. *Trap:* Recommending testing to relieve anxiety.

Q11 — Answer: A. Clarify that ordering tests is the physician's decision and offer, with consent, to coordinate her concerns with the provider

Why it is correct: Stay in scope; coordinate with consent. *Common error:* Directing the physician's medical workup.

- **B** — Directing the workup exceeds scope. *Trap:* Inserting yourself into medical decision-making.
- **C** — Reinforces reassurance-driven testing. *Trap:* Coaching reassurance-driven testing.
- **D** — Refusing to coordinate abandons the concern. *Trap:* Declining to help her communicate with the provider.

Q12 — Answer: A. Obtain her informed consent and release before communicating with her providers, then collaborate on a shared plan

Why it is correct: Consent and release before coordinating. *Common error:* Contacting providers before obtaining consent.

- **B** — No coordination undercuts collaborative care. *Trap:* Refusing helpful, consented coordination.
- **C** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.
- **D** — Coordinating without consent breaches confidentiality. *Trap:* Disclosing before consent is documented.

Q13 — Answer: B. Conduct a structured suicide-risk assessment and respond to the risk, recognizing chronic somatic distress can carry suicide risk

Why it is correct: Assess risk directly. *Common error:* Overlooking risk amid somatic focus.

- **A** — Minimizing the ideation is unsafe. *Trap:* Dismissing the ideation.
- **C** — No-suicide contracts are not evidence-based. *Trap:* Substituting a contract for assessment.
- **D** — Risk must be assessed before continuing. *Trap:* Proceeding past a risk disclosure.

Case 9: Individual disclosure of intimate partner violence and a partner's request to join therapy

Diagnosis: Adjustment Disorder With Anxiety (in the context of intimate partner violence)

Q1 — Answer: B. Include direct, private questions about relationship safety as a routine part of the psychosocial intake

Why it is correct: Routine private screening surfaces unreported danger. *Common error:* Waiting for the client to volunteer relationship concerns unprompted.

- **A** — Deferring psychosocial history delays context. *Trap:* Treating symptoms in isolation from psychosocial history.
- **C** — Passive waiting can delay needed screening. *Trap:* Assuming silence means there is nothing to screen for.
- **D** — Involving the partner early blocks honest disclosure. *Trap:* Assuming a fuller picture requires the partner present.

Q2 — Answer: B. A structured danger assessment covering the frequency, severity, and escalation pattern of the violence, including access to weapons

Why it is correct: Structured danger assessment anchors risk level. *Common error:* Moving to intervention before characterizing the danger.

- **A** — Exhaustive conflict history is low yield now. *Trap:* Collecting broad history instead of targeted risk data.
- **C** — Couples referral is premature and misdirected. *Trap:* Jumping to a relationship fix before assessing safety.
- **D** — Contacting the partner breaches confidentiality and safety. *Trap:* Verifying a client's disclosure with the abusive partner.

Q3 — Answer: A. Specifically ask about any prior strangulation, threats with weapons, and whether the violence has increased in frequency or severity

Why it is correct: Strangulation history strongly predicts future lethality. *Common error:* Treating the choking as an isolated event rather than a lethality marker.

- **B** — Avoiding history-taking hides escalation and worsens risk. *Trap:* Avoiding distressing questions that are clinically necessary.
- **C** — Commitment rating does not measure danger. *Trap:* Substituting a feelings check for a danger assessment.
- **D** — Couples referral bypasses the lethality assessment. *Trap:* Reframing lethal violence as a communication problem.

Q4 — Answer: A. Her anxiety and hypervigilance emerged and sharply escalated in direct response to specific, ongoing violence and control

Why it is correct: Onset tied to a specific, ongoing stressor fits an adjustment disorder. *Common error:* Diagnosing GAD from anxiety symptoms without weighing the acute precipitant.

- **B** — Diffuse worry across domains fits GAD better. *Trap:* Ignoring how broadly the worry actually spreads.

- **C** — A longer chronic course points away from an adjustment disorder. *Trap:* Overlooking the actual reported duration.
- **D** — Impairment alone does not distinguish the two. *Trap:* Using severity of impairment as the differentiator.

Q5 — Answer: C. Her own current risk for suicide or self-harm, using a structured screen, since intimate partner violence significantly raises that risk

Why it is correct: IPV exposure independently elevates suicide risk. *Common error:* Assessing only the partner's danger and skipping the client's own risk.

- **A** — Sleep disorder rule-out is secondary right now. *Trap:* Pursuing a low-yield rule-out ahead of a suicide-risk screen.
- **B** — Family history does not address acute risk. *Trap:* Prioritizing etiology over an urgent safety screen.
- **D** — Panic disorder is a lower-priority rule-out here. *Trap:* Chasing a comorbidity before the client's own safety.

Q6 — Answer: D. A collaborative, individualized safety plan covering warning signs, coping steps, safe contacts, and options for leaving or staying

Why it is correct: Collaborative safety planning fits an active-danger case. *Common error:* Substituting a directive plan for collaborative planning.

- **A** — Managing the partner's care is not the client's plan. *Trap:* Making the abusive partner's treatment the centerpiece of her plan.
- **B** — Symptom-focused planning ignores the active danger. *Trap:* Treating anxiety symptoms while leaving danger unaddressed.
- **C** — Directing her to leave overrides her autonomy. *Trap:* Telling a client she must leave rather than planning with her.

Q7 — Answer: A. Decline the joint session and continue individual treatment, since conjoint work during active violence can raise danger and suppress disclosure

Why it is correct: Conjoint therapy is contraindicated during active IPV. *Common error:* Treating a request for couples work as automatically reasonable.

- **B** — Deciding without clinical input on risk is incomplete. *Trap:* Confusing autonomy with skipping informed discussion of risk.
- **C** — Seeing the partner individually creates a conflict of interest. *Trap:* Overlooking the dual-relationship risk of also treating the partner.
- **D** — Conjoint sessions with an active abuser endanger the client. *Trap:* Mistaking a well-intentioned-sounding request as clinically safe.

Q8 — Answer: A. Offer information about IPV-specific resources such as a hotline and a local advocate, and let her decide what steps to take and when

Why it is correct: Offering resources without mandating action respects autonomy. *Common error:* Withholding IPV-specific resources out of caution.

- **B** — Conditioning care on compliance is coercive. *Trap:* Pressuring immediate action instead of offering informed choice.
- **C** — Passive waiting can leave her without key information. *Trap:* Assuming she will ask if she wants information.
- **D** — Generic resources miss IPV-specific expertise. *Trap:* Treating any resource list as sufficiently specific.

Q9 — Answer: D. Include specific warning signs preceding escalation and an immediate physical-safety plan for recurrence, built collaboratively with Maria

Why it is correct: Targeted warning-sign and safety steps address lethality. *Common error:* Leaving the plan generic rather than tailored to the choking risk.

- **A** — Long-term goals alone ignore acute lethality risk. *Trap:* Deferring urgent safety steps in favor of long-term goals.
- **B** — Instructing her to restrain him endangers her further. *Trap:* Giving advice that places the client in greater physical danger.
- **C** — This places responsibility for his violence on her. *Trap:* Framing avoidance of conflict as a safety plan.

Q10 — Answer: A. Assess whether she can meet privately on a device Danny cannot access, and agree on a code word and an interruption plan

Why it is correct: Telehealth with an abusive partner needs privacy and interruption planning; in-person may be safer. *Common error:* Assuming home sessions are automatically safe.

- **B** — Home may be where monitoring and danger are greatest. *Trap:* Equating a convenient setting with a safe one.
- **C** — Danny monitors her texts, exposing the sessions. *Trap:* Using channels her abuser can access.
- **D** — Unplanned interruptions can escalate danger to Maria. *Trap:* Improvising safety steps during a crisis.

Q11 — Answer: B. Before billing, help her ask the plan to send statements elsewhere, which it must honor if she says she is endangered

Why it is correct: HIPAA requires plans to accommodate endangerment-based requests; state laws may add protections. *Common error:* Billing routinely without considering who receives statements.

- **A** — Statements typically identify the provider and service. *Trap:* Falsely assuming claims are invisible to policyholders.

- **C** — Misrepresenting the diagnosis is insurance fraud. *Trap:* Falsifying claims to protect the client's privacy.
- **D** — Contacting Danny breaches confidentiality and endangers her. *Trap:* Treating the policyholder as needing to consent.

Q12 — Answer: C. Decline to confirm or discuss whether Maria is a client, or share any session information, without her explicit consent

Why it is correct: Confidentiality requires consent before any disclosure. *Common error:* Confirming a client relationship to a third party without consent.

- **A** — Leaving it entirely to the client misses a needed boundary. *Trap:* Assuming no response avoids the ethical issue.
- **B** — Partial disclosure still breaches confidentiality without consent. *Trap:* Believing a vague disclosure is safe because it lacks detail.
- **D** — Confirming client status without consent breaches confidentiality. *Trap:* Explaining clinical reasoning to someone who is not an authorized party.

Q13 — Answer: B. Document the danger assessment and safety plan thoroughly, and offer protective order information while leaving the timing to Maria

Why it is correct: Thorough documentation plus informed autonomy is standard. *Common error:* Either under-documenting risk or directing her legal choices.

- **A** — Vague notes obscure the actual risk present. *Trap:* Under-specifying documentation of a safety-relevant disclosure.
- **C** — Acting on her behalf removes her autonomy. *Trap:* Taking over a legal decision that belongs to the client.
- **D** — Omitting risk data undermines safe, continuous care. *Trap:* Sacrificing accurate records for perceived privacy protection.

Case 10: Progressive memory loss and lost independence in an older adult

Diagnosis: Major Neurocognitive Disorder due to Possible Alzheimer's Disease

Q1 — Answer: D. A significant decline from her prior level in one or more cognitive domains that interferes with her everyday independence

Why it is correct: Significant decline interfering with independence. *Common error:* Diagnosing major NCD from minor forgetfulness.

- **A** — Depressive count does not define NCD. *Trap:* Applying depressive criteria here.
- **B** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **C** — A sudden change suggests delirium. *Trap:* Confusing an acute change with progressive NCD.

Q2 — Answer: C. Record only with Margaret's informed consent, or her legal representative's if she lacks capacity, and secure files as records

Why it is correct: Recording needs valid consent, any state-law consent, and secure storage. *Common error:* Letting a family caregiver authorize recording.

- **A** — Caregiver status alone confers no legal authority. *Trap:* Assuming family can consent for the client.
- **B** — Personal phone recordings are unsecured protected information. *Trap:* Moving health information outside the practice's secure systems.
- **D** — Platform compliance does not replace client consent. *Trap:* Equating a compliant platform with consent.

Q3 — Answer: B. Screen for depression, since depression can impair cognition and may coexist with or mimic a neurocognitive disorder

Why it is correct: Screen depression as a contributor or mimic. *Common error:* Ignoring a treatable depressive contribution.

- **A** — Over-attributing to depression is premature. *Trap:* Forcing one explanation prematurely.
- **C** — A mood trend is not the question. *Trap:* Reframing the question as a mood trend.
- **D** — Dismissing depression misses a contributor. *Trap:* Overlooking a treatable contribution.

Q4 — Answer: A. Use cognitive screening and collateral history and coordinate a medical and neurological evaluation rather than diagnosing alone

Why it is correct: Screen and coordinate the medical workup. *Common error:* Diagnosing the medical etiology without a medical evaluation.

- **B** — Dismissing the decline is unsafe. *Trap:* Mistaking major NCD for normal aging.
- **C** — Ordering tests exceeds counselor scope. *Trap:* Acting outside scope of practice.
- **D** — One observation is insufficient. *Trap:* Diagnosing from one in-session sample.

Q5 — Answer: D. Assess safety risks such as driving, the stove, wandering, and medication management given her decline and impaired judgment

Why it is correct: Assess functional safety risks. *Common error:* Overlooking everyday safety risks.

- **A** — Deferring safety review is unsafe. *Trap:* Postponing a needed safety assessment.
- **B** — Narrow focus misses key NCD risks. *Trap:* Assessing only one channel of risk.
- **C** — Impaired insight makes self-report insufficient. *Trap:* Accepting reassurance without assessment.

Q6 — Answer: A. Coordinate medical care and build a plan of caregiver support, safety measures, and structured non-pharmacologic approaches

Why it is correct: Coordinated care plus caregiver and safety support. *Common error:* Offering individual insight therapy alone as the plan.

- **B** — Insight therapy cannot reverse NCD. *Trap:* Setting an unrealistic restorative goal.
- **C** — Passive waiting delays needed support. *Trap:* Delaying indicated support.
- **D** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.

Q7 — Answer: D. Documenting who attended, why Margaret was absent, and how the session served her treatment plan goals and safety

Why it is correct: Notes must tie the family session to the patient's plan. *Common error:* Billing a code the documentation does not support.

- **A** — An individual code misstates who was treated. *Trap:* Choosing a code that misrepresents the service.
- **B** — The daughter is not the identified client here. *Trap:* Billing a non-client's insurance for services.
- **C** — Bare notes cannot establish medical necessity. *Trap:* Assuming the claim stands without documentation.

Q8 — Answer: D. Provide caregiver psychoeducation and connect her with respite and support resources to reduce caregiver burden

Why it is correct: Caregiver education and respite support. *Common error:* Focusing only on the client and ignoring the caregiver.

- **A** — Dismissing caregiver needs is a missed target. *Trap:* Excluding the caregiver from the plan.
- **B** — Sole caregiving fuels burnout. *Trap:* Discouraging needed support and respite.
- **C** — Concealment undermines autonomy. *Trap:* Defaulting to concealment.

Q9 — Answer: C. Validate her grief and preserve her dignity and personhood while supporting the abilities and choices she still retains

Why it is correct: Validate grief; preserve dignity and choice. *Common error:* Talking past her or treating her as incapable.

- **A** — Dismissive reassurance invalidates her grief. *Trap:* Minimizing her experience.
- **B** — Heavy prognosis data is not what she needs now. *Trap:* Answering distress with statistics.
- **D** — Talking past her undermines dignity. *Trap:* Excluding the client from her own care conversation.

Q10 — Answer: B. Use simple, concrete communication, shorter sessions, and supportive engagement adapted to her current cognitive abilities

Why it is correct: Adapt communication to her abilities. *Common error:* Using complex, insight-heavy methods she cannot use.

- **A** — Independent homework exceeds her capacity. *Trap:* Assigning tasks beyond her current ability.
- **C** — Unadapted insight work does not fit. *Trap:* Failing to adapt the approach.
- **D** — Writing her off disregards personhood. *Trap:* Prematurely excluding the client.

Q11 — Answer: A. Discuss the device with Margaret and her daughter, seek her consent or assent, review its data privacy, and tie it to the plan

Why it is correct: Technology tools need consent, privacy review, and plan integration. *Common error:* Using covert monitoring to avoid upsetting her.

- **B** — Covert tracking violates her autonomy and dignity. *Trap:* Bypassing her voice because of impairment.
- **C** — One device cannot cover every safety risk. *Trap:* Over-relying on technology as the whole plan.

- **D** — Clinical tools belong within the treatment plan. *Trap:* Disowning tools that shape her care.

Q12 — Answer: B. Move her sessions to an accessible ground-floor room or other accessible space at no added cost, and ask what else she needs

Why it is correct: Practices must offer accessible alternatives without charge. *Common error:* Expecting the client to adapt to an inaccessible setting.

- **A** — Carrying her is unsafe and undignified. *Trap:* Using informal help in place of real accessibility.
- **C** — Phone-only care may not suit her cognitive needs and was not her choice. *Trap:* Unilaterally downgrading the service.
- **D** — Suspending care over access excludes her from services. *Trap:* Treating building barriers as the client's problem.

Q13 — Answer: B. Seek consultation and coordinate with a geriatric or medical team, or refer for the components beyond your competence

Why it is correct: Consult, coordinate, or refer within competence. *Common error:* Managing complex NCD alone without competence.

- **A** — Abrupt transfer mishandles continuity. *Trap:* Dropping the family without coordination.
- **C** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.
- **D** — Minimizing the case fails the family. *Trap:* Underestimating the need for specialist care.

Practice Exam Three

10 clinical cases · 130 questions · 225 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

Case 1 of 10

Tearful and doubting her competence six weeks after childbirth

Diagnosis: Major Depressive Disorder, with Peripartum Onset

Intake: Danielle Marsh, a 27-year-old woman who is 6 weeks postpartum with her first child, joins for an intake session by telehealth from her living room while her daughter naps nearby. She reports that low mood, tearfulness, and exhaustion began about ten days after delivery and have steadily worsened, leaving her convinced she is failing as a new mother.

Session Two: She describes crying multiple times most days, little interest in things she used to enjoy such as cooking or calling friends, waking well before the baby stirs and being unable to fall back asleep, a poor appetite, and trouble concentrating even on simple tasks like reading a bottle label. She denies any past period of elevated mood, decreased need for sleep, or unusually high energy.

Session Three: When asked directly, she denies any thoughts of harming her daughter or herself, any plan or intent, and any voices, visions, or beliefs that feel unreal; she says she feels warmly bonded to her daughter and is frightened by how incapable she feels, wanting help to feel like herself again.

Question 1. What should the counselor confirm first at intake to anchor the diagnosis of a major depressive episode with peripartum onset?

A. That her depressive symptoms began within four weeks of delivery and have persisted nearly daily for over two weeks with impairment

B. That her symptoms are noticeably worse this week than when she first noticed them roughly ten days after delivery

C. That she can identify a single specific event during the pregnancy or delivery that she believes triggered all of her symptoms

D. That a family member has previously been diagnosed with a mood disorder at some earlier point in their own life

Question 2. The night after Danielle's intake, a fire closes the office building from which the counselor holds her telehealth sessions; under the practice's emergency plan, what should happen next?

A. Follow the plan: notify her by her listed contact, hold her session securely from the backup site, and document the change

B. Cancel all of her sessions until the building reopens, since her care cannot continue properly without the counselor's office

C. Hold the session from the counselor's home on a free video app, since emergencies suspend the usual privacy requirements

D. Wait for Danielle to reach out about her session, since contacting clients first could make the closure seem alarming

Question 3. As part of routine peripartum risk assessment, what must the counselor screen for directly, regardless of how mild Danielle's presentation appears?

A. Whether she has already scheduled her daughter's upcoming pediatric wellness visits for the next several months

B. Any thoughts of harming her daughter or herself, and any hallucinations, delusions, or disorganized thinking

C. Whether she feels confident she can breastfeed exclusively for the recommended duration without ever supplementing

D. Whether her partner agrees that hiring outside childcare help would be an appropriate next step for the family

Question 4. After consulting a colleague experienced in perinatal IPT about adapting role-transition work for Danielle, how should the counselor use the colleague's advice?

A. Follow the advice exactly as given, since responsibility for the adapted IPT approach now rests with the consulting colleague

B. Weigh the advice against the counselor's own assessment, keep responsibility for her care, and document the consultation

C. Have the colleague cosign Danielle's treatment plan so that the colleague shares legal responsibility for the adapted approach

D. Leave the consultation out of her record, since documenting it would show that the counselor needed outside help with her care

Question 5. Danielle wants to work first on feeling like a failing mother, while the counselor sees her early waking as more central; how should they set priorities?

A. Begin with her early waking instead, since the counselor judges sleep to be the root problem and should set the order of focus

B. Work on all of her concerns with equal weight from the first session so that none of them is overlooked or set aside for later

C. Begin with her self-doubt as she wishes, share the counselor's view on sleep, and agree to revisit the order as work goes on

D. Postpone setting priorities until her symptoms ease enough that she can think clearly about what matters most to her

Question 6. Later, Danielle says the nurse from her community home-visiting program wants to support her counseling goals; what is the best way to collaborate?

A. Leave the nurse and the counselor to work separately, since coordinating with community services is outside the counseling role

B. Let the nurse take over Danielle’s activity goals, since home visits give the nurse more contact with her than weekly sessions do

C. Send the nurse Danielle’s full progress notes so the nurse can decide alone which parts of the plan to reinforce during visits

D. With her signed release, agree with Danielle and the nurse on shared goals, each one’s role, and how they will stay in contact

Question 7. How should the counselor track whether treatment is actually working over the coming weeks?

A. Wait until the full course of therapy has ended before evaluating whether her symptoms have changed at all

B. Ask her casually at each session whether she generally feels a bit better than she did the previous week

C. Track only whether she is attending sessions consistently and completing any assigned between-session tasks

D. Re-administer a validated perinatal measure such as the Edinburgh Postnatal Depression Scale at regular intervals

Question 8. Danielle asks whether she should also be evaluated for medication. What is the most appropriate counselor action?

A. Provide general information and, with her consent, coordinate a referral to a prescriber for a medication evaluation

B. Advise her that medication is unnecessary as long as she remains fully committed to attending therapy

C. Tell her which specific antidepressant is typically best for new mothers with her particular symptom pattern

D. Suggest she research antidepressant options for new mothers herself online and bring a preferred choice to a future appointment

Question 9. Which initial step best supports Danielle's re-engagement with valued activities while accounting for her new role as a mother?

A. Have her keep a detailed log of every negative thought about her parenting before introducing any change to her routine

B. Direct her to resume her full pre-baby schedule immediately to prove to herself that she can still manage everything

C. Collaboratively schedule small, manageable activities such as a short walk with the baby or a brief call to a friend

D. Encourage her to wait until she genuinely feels motivated again before attempting to resume any prior activities

Question 10. How can the counselor best address Danielle's belief that her depression makes her a bad mother?

A. Offer psychoeducation framing postpartum depression as common and treatable while validating how exhausting this feels

B. Firmly challenge the belief as irrational and present objective evidence that she is clearly a capable mother

C. Redirect the conversation away from the topic and refocus entirely on practical scheduling for the coming week

D. Agree that her exhaustion and low mood are affecting her daughter and focus on helping her accept this reality

Question 11. Danielle asks the counselor to update her obstetrician on her progress before her next checkup; under the practice's release-of-information procedure, what should happen?

A. Have her sign the practice's release form naming the obstetrician, the information to share, its purpose, and an expiration

B. Call the obstetrician today on her verbal okay, treating the practice's written release form as meant only for non-medical contacts

C. Fax her complete chart, including detailed session notes, so the obstetrician has every piece of information that might help

D. Decline the request, since the practice's policies do not allow counselors to communicate with a client's medical providers

Question 12. When reviewing informed consent, what must the counselor ensure Danielle understands?

A. The limits of confidentiality, including circumstances that would require the counselor to act to prevent serious harm

B. That the counselor will decide which specific treatment goals are appropriate for her to pursue in therapy

C. That everything she shares in telehealth sessions will remain completely confidential under all circumstances, including safety concerns

D. That she is required to complete the entire recommended course of therapy once she agrees to begin today

Question 13. Danielle asks whether it is appropriate to keep having sessions by telehealth while caring for a newborn at home. What is the appropriate response?

A. Proceed with sessions without addressing privacy or confidentiality since she already agreed to telehealth at intake

B. Tell her telehealth is never appropriate for a new mother and she must switch to in-person sessions immediately

C. Suggest she pause therapy altogether until her daughter is older and telehealth sessions are less likely to be interrupted

D. Confirm she has a private space and stable connection, review telehealth consent and confidentiality limits, and proceed

Case 2 of 10

Sad and discouraged three weeks after a layoff

Diagnosis: Adjustment Disorder, with Depressed Mood

Intake: Marcus Whitfield, a 39-year-old father of two, presents to outpatient counseling through his former employer's EAP three weeks after being laid off from his assembly-line supervisor role at an auto-parts plant. He reports feeling sad, discouraged, and "stuck," with trouble falling asleep and little motivation to do much beyond searching for work.

Session Two: He describes lying awake most nights running through bills and job applications, waking unrefreshed, and losing interest in his usual pickup basketball games, though he still gets his kids ready for school, cooks dinner, and has submitted applications to six employers this week. He denies any change in appetite and says his mood lifts somewhat when he is with his children.

Session Three: He denies any nightmares, intrusive memories, or flashbacks about the layoff, any avoidance of his old workplace or former coworkers, and any hypervigilance or exaggerated startle response. He denies current thoughts of self-harm or suicide, says he feels embarrassed but not hopeless, and states he believes "this will get better once I land something new."

Question 1. What is the most important timeline detail to establish during Marcus's intake?

A. That his sadness, sleep trouble, and low motivation began within three months of the layoff and track closely with it

B. How he would numerically rate the layoff's severity compared with other stressful events earlier in his life

C. A full chronological inventory of every position he has held across his entire manufacturing career to date

D. Whether any of his first-degree relatives, such as his parents or siblings, have ever been formally diagnosed with a depressive or anxiety disorder

Question 2. Given Marcus denies feeling hopeless, what is still the appropriate intake safety step?

A. Complete a structured suicide-risk screen as a routine part of intake regardless of how mild his presentation seems

B. Wait until a later session, once more rapport has been built, before asking him any direct safety-related questions

C. Skip any direct risk questions today since he appears to be coping well and clearly denies feeling hopeless

D. Have him complete a written pledge stating he will contact the office before acting on any distressing thoughts

Question 3. What functional information is most useful to gather to gauge the impact of Marcus's symptoms?

A. How his sleep, mood, and motivation are affecting his job search and his ability to care for his two children

B. Whether he believes his former employer treated him fairly when the decision to lay him off was made

C. A description of the exact machinery and daily tasks he performed in his prior assembly-line supervisor position

D. A detailed accounting of his monthly household budget so the counselor can calculate his exact severance runway

Question 4. Finding few trials specific to adjustment disorder, how should the counselor build knowledge of empirically supported approaches for Marcus's care?

A. Rely on the counselor's own past success with similar clients, since no research base exists to consult for this diagnosis

B. Review the limited trials and reviews on adjustment disorder, such as problem-solving and brief CBT, weighing their quality

C. Adopt the full treatment protocol for major depression, since evidence from that diagnosis transfers directly to his case

D. Choose whichever approach is most popular on counseling websites and blogs, since popularity reflects its real effectiveness

Question 5. Which treatment goal for Marcus's sleep difficulty is best written in SMART form?

A. Marcus will feel less stuck and get better sleep soon so he can focus more fully on his job search and on his two children

B. Marcus will never lie awake worrying about bills again and will sleep a full eight hours every night starting this week

C. Within four weeks, Marcus will fall asleep within 30 minutes on at least five nights a week, as tracked in his sleep log

D. Marcus will land a new supervisor job within two weeks so that his worries about money stop keeping him awake at night

Question 6. Later, Marcus reports that a sleep medication from his physician helps him sleep but leaves him groggy at morning interviews; how should the counselor document this?

A. Record only that he is "on medication," since prescription details belong solely in his physician's medical records

B. Note that the counselor advised him to skip doses on interview days, since grogginess is hurting his job search progress

C. Wait to document anything until his physician confirms the effects, since self-reports of side effects are unreliable data

D. Record the drug as he names it, its start date, his better sleep, the morning grogginess, and its effect on interviews

Question 7. How should the counselor and Marcus set initial treatment goals?

A. Have the counselor independently decide the full treatment plan and simply inform Marcus of it at the next visit

B. Collaboratively identify concrete, achievable goals tied to coping with the job loss, such as structuring his days

C. Postpone any goal-setting until his mood has fully returned to baseline on its own without any structured plan

D. Focus the entire treatment plan on exploring his childhood relationship with his own father's work history

Question 8. Marcus asks for help finding concrete job-search and financial resources. What is the best response?

A. Redirect the conversation away from finances entirely and keep every session focused only on his sleep pattern

B. Incorporate problem-solving around concrete resources such as job-placement and financial-counseling referrals

C. Tell him that discussing practical job-search resources is not appropriate work for a counseling session

D. Personally review his resume in detail and coach him directly on interview technique during each session

Question 9. How should the counselor monitor Marcus's course over the coming weeks?

A. Wait until the full course of sessions has ended and then evaluate his overall progress in one final review

B. Track only whether he attends sessions consistently, without asking directly about his mood or sleep pattern

C. Track his symptoms at each session and reassess if they persist or worsen well beyond the stressor resolving

D. Assume the diagnosis will not change and stop asking about his mood once the initial treatment plan is set

Question 10. Marcus says he feels embarrassed that a layoff has hit him this hard. What is the most therapeutic response?

A. Quickly change the subject to his job-search progress so the conversation does not dwell on his feelings

B. Tell him firmly that his embarrassment is irrational and that he has no real reason to feel that way at all

C. Validate that job loss is a significant stressor and normalize his reaction as a common, understandable response

D. Agree that most people would have handled a layoff like this with far more composure and resilience than he has

Question 11. How can the counselor best support Marcus's motivation to re-engage in activities he has pulled back from?

A. Tell him to resume his full prior schedule of activities all at once this week to prove to himself that he can still cope with his old routine

B. Advise him to set activities aside completely until his mood and motivation have fully returned to normal

C. Spend the session having him list, in detail, every reason his motivation has declined since the layoff happened

D. Collaboratively schedule a small, valued activity, like one pickup basketball game, to rebuild a sense of momentum

Question 12. Later, Marcus asks for a list of everyone outside the practice who has received his information; what does federal law require?

A. Give him a HIPAA accounting within 60 days, omitting disclosures he authorized or made for treatment, payment, or operations

B. Decline, since HIPAA gives clients access to their own records but no right to learn who has received their information

C. List only the disclosures made to the EAP, since those are the only ones federal law treats as needing to be tracked

D. Charge him a fee before preparing any list, since federal law permits a charge for every accounting that a client requests

Question 13. The EAP case manager emails asking for an update on Marcus’s progress. What is the appropriate action?

A. Ignore the email entirely and never respond to the EAP case manager about the request at any point going forward

B. Reply immediately with a full summary of Marcus’s symptoms and diagnosis since the EAP made the referral

C. Ask Marcus informally in passing whether it is fine to talk to the EAP, without documenting his answer anywhere

D. Decline to share any information unless Marcus has signed a specific, informed release authorizing that disclosure

Case 3 of 10

Cocaine use, crashes, and mounting consequences

Diagnosis: Stimulant Use Disorder (Cocaine)

Intake: Tara Brooks, a 34-year-old bartender, presents after a friend urged her to get help for her cocaine use. She reports strong cravings, using more than she intends, failed attempts to cut down, and continued use despite conflicts at work and with her partner.

Session Two: She describes needing more cocaine to get the same effect and severe crashes of fatigue and low mood for a day or two after binges. She meets at least six criteria over the past year and has missed shifts and lost money because of her use.

Session Three: She is ambivalent — relieved someone noticed yet defensive about being judged — and her crashes bring brief but intense low mood. She denies sustained elevated mood apart from intoxication and is frightened by occasional chest pain during heavy use.

Question 1. What is most important to establish to confirm a severe stimulant use disorder?

A. Whether she first started using cocaine recreationally at parties rather than to cope with stress or with difficult emotions

B. Whether anyone else in her immediate family has previously struggled with cocaine or with another substance-related condition

C. The exact quantity of cocaine she uses each week so that you can calculate precisely how dependent she has become on it

D. The number of DSM-5-TR criteria she meets over twelve months, since six or more place severity in the severe range

Question 2. Having decided to seek training on gender-responsive substance use care for clients like Tara, which training choice best builds the counselor's competence?

A. A one-hour webinar that grants a certificate of completion, since holding the certificate shows the counselor is now competent

B. A research-based course with skills practice and follow-up consultation, such as one drawing on SAMHSA's guidance for women

C. A popular podcast of women's recovery stories, since first-person accounts alone teach the clinical skills that her care requires

D. A general course on addiction pharmacology, since medication knowledge prepares a counselor equally well for any client population

Question 3. Given her chest pain during heavy use and her post-binge crashes, what assessment is most important?

A. Explore in depth the early family experiences that she believes first shaped her current relationship with substances

B. Identify the particular social situations and people that tend to prompt her heaviest cocaine use during the week

C. Determine the specific brand or source of the cocaine she uses so that you can document exactly what she has been taking

D. Assess medical risk and suicide risk, since stimulant use carries cardiac danger and crashes can bring acute low mood

Question 4. Tara's functional assessment shows most binges follow late weekend shifts and paydays; how should the counselor integrate these results into her treatment plan?

A. Record the findings in her assessment summary but keep the plan focused on general abstinence goals for every day of the week

B. Set a plan goal that she quit her bartending job at once, since the assessment shows that her work alone causes her use

C. Turn the assessed triggers into plan objectives, such as coping steps after late shifts and a set routine for handling pay

D. Delay plan changes until she completes several more months of assessment, so the trigger pattern is fully confirmed first

Question 5. Later, when Tara agrees to the intensive outpatient program her level-of-care assessment supports, the counselor learns the program on the practice's referral list closed months ago; what is best?

A. Give Tara the old program's number anyway and let her find out on her own whether another program has taken its place

B. Drop the intensive outpatient referral and keep seeing her weekly, since the list no longer offers a program at that level

C. Ask Tara to search online for programs herself, since tracking current local resources is not part of the counselor's role

D. Find a current program for her now, then update the list and set a routine check of each resource's status and services

Question 6. Tara wants to use her health insurance, but the counselor is not on her plan's provider panel; what is the most appropriate response?

A. Explain the out-of-network status, her likely costs or a good faith estimate if self-paying, and in-network referral options

B. Bill her plan under a paneled colleague’s provider number, since the colleague works in the same practice and setting

C. Tell her that out-of-network counselors cannot accept any insurance, so none of her fees could ever be reimbursed by her plan

D. Submit claims as usual and let the insurer sort out network status, since providers need not tell clients this in advance

Question 7. How should the counselor determine her appropriate level of care?

A. Use a multidimensional assessment of her medical risk, environment, and readiness to match her to the least intensive safe setting

B. Recommend only a single weekly mutual-help meeting, assuming that peer support alone will be enough to address her cocaine use

C. Let her choose whichever program has the most convenient schedule regardless of her medical risks or her clinical needs

D. Automatically refer her to the most intensive residential program available so that she is kept away from cocaine entirely

Question 8. Given her ambivalence, what is the most appropriate primary counseling approach?

A. A firm, direct confrontation about the serious damage her cocaine use is causing so that she finally faces the full reality of it

B. A structured relapse-prevention program built entirely around her committing right now to a firm quit date for complete abstinence

C. Primarily educational lectures detailing the medical harms of cocaine so that the facts alone convince her to stop using it

D. Motivational interviewing to explore her ambivalence and strengthen her own reasons for change at her current stage

Question 9. Tara says, “Maybe my using isn’t even that big a deal.” The MI-consistent response is to:

A. Agree fully that her cocaine use is not really a problem so that she feels supported and stays comfortable working with you

B. Reflect her statement and explore both sides of her ambivalence without arguing against her about how serious it is

C. Point out all of the evidence that proves her cocaine use is in fact a very serious problem that she needs to address now

D. Quickly change the subject to avoid any disagreement with her about exactly how serious her cocaine use really is right now

Question 10. Tara returns to use after a week of abstinence and is discouraged. The most therapeutic response is to:

A. Reassure her that the slip really does not matter at all and encourage her to simply forget about it and move on quickly

B. Recommend that she immediately step up to a far more restrictive level of care since the current plan has obviously failed her

C. Frame the return to use as common in recovery, explore what it taught her, and revisit the plan without shaming her

D. Warn her seriously that another slip like this one will likely undo all of her progress and prove she cannot really do this

Question 11. How should the counselor frame Tara’s post-binge crashes in their work together?

A. As high-risk periods for low mood and safety that warrant monitoring and a plan for support during the crash

B. As an unimportant and temporary discomfort that will pass on its own and therefore does not require any particular attention

C. As clear evidence that she has a separate bipolar disorder that will need to be treated entirely apart from her cocaine use

D. As proof that she simply needs to use a smaller amount of cocaine so that her crashes will not feel quite so severe afterward

Question 12. Tara's employer requests her substance-use treatment records after she missed shifts. Best response?

A. Forward the employer's request to Tara and avoid documenting in the record that the request was ever received at all

B. Send the records to the employer promptly so that the employer can decide how best to handle her recent missed work shifts

C. Decline to release records without a valid release and explain that substance-use information has heightened confidentiality protections

D. Share only her diagnosis with the employer, since a single diagnostic label seems like a relatively harmless detail to provide

Question 13. Tara asks whether there is a medication that will cure her cocaine cravings. The most appropriate response is to:

A. Recommend a specific medication and dose that she should ask her physician to prescribe to take away her cocaine cravings

B. Assure her that a reliable anti-cocaine medication exists and that starting it will be the single most important part of her treatment

C. Tell her that medication questions have nothing to do with counseling and decline to discuss anything related to her health concerns

D. Explain honestly that no medication is approved for stimulant use disorder and coordinate medical evaluation for related concerns

Case 4 of 10

Minimized suicidal thoughts and an accessible firearm after job loss and separation

Diagnosis: Adjustment Disorder With Depressed Mood

Intake: Scott Ridgeway, a 45-year-old Army veteran, presents to outpatient counseling two weeks after his wife moved out and five weeks after being laid off from his warehouse management job. He reports feeling numb and having occasional thoughts of not waking up, but denies any specific plan when asked directly.

Session Two: He describes broken sleep, drinking most nights alone, and feeling like a burden to his two teenage children. Asked about safety, he says, “I think about it sometimes, but I’d never actually do it — I’m not that guy.” Almost in passing, he mentions he keeps his old service pistol loaded in the nightstand “just in case,” and that he took it apart and cleaned it last week to make sure it still worked, out of habit.

Session Three: He continues to minimize, calling the thoughts “just background noise,” but reports growing hopelessness about reconciling with his wife and describes rehearsing what he would say in a note to his children “just to get it out of my head.” His weekend drinking has increased, and he is reluctant to discuss the firearm directly, saying he has “always been careful with guns.”

Question 1. Scott says, “I think about it sometimes, but I’d never actually do it,” then quickly changes the subject. What is the most appropriate first step?

A. Conduct a structured suicide risk assessment covering ideation, intent, plan, and access to means, without accepting his reassurance at face value

B. Refer him immediately to a psychiatrist for a medication evaluation and postpone talking further about the thoughts until that visit

C. Accept that he would never act on the thoughts since he stated this clearly, and move the conversation toward his current job search and daily routine instead

D. Begin teaching grounding and relaxation exercises right away so he has tools to manage the thoughts before anything else

Question 2. Scott mentions, almost in passing, that he keeps his sidearm loaded in the nightstand and checked recently that it still worked, then adds that he'd "never actually do it." What should the counselor do?

A. Ask his estranged wife by phone to remove the firearm from the home without first discussing the disclosure further with Scott himself

B. Explore the disclosure directly, asking follow-up questions about the firearm's accessibility and his recent thoughts about it, rather than accepting his reassurance

C. Note the comment for later in the chart and continue with the standard intake questions in the order originally planned for today

D. Take his reassurance that he would never act on the thoughts at face value and continue gathering his employment and relationship history for the intake

Question 3. What does Scott's disclosure about the loaded, recently checked firearm most significantly indicate for the risk assessment?

A. A signal to explore in depth the childhood experiences that shaped how he learned to cope with painful emotions

B. A prompt to precisely calculate how many drinks he consumes on an average night so his alcohol use can be quantified

C. Immediate access to a highly lethal method, independent of his verbal reassurance, which sharply elevates acute risk

D. A need for a detailed history of every firearm he has ever owned so the record fully documents his background with weapons

Question 4. Scott mentions his teenage children as a reason he has not acted, but also that he keeps rehearsing a note to them. What is most important for a balanced risk formulation?

A. Weigh both his risk factors, such as the accessible firearm and hopelessness, against protective factors like his children, rather than reading either alone

B. Determine whether his distress is objectively proportionate to the severity of the separation and job loss he has experienced recently, compared with others in similar circumstances

C. Focus only on the fact that he has not yet written or sent an actual note, since no completed note means the risk is currently low

D. Ask him to rate his overall mood on a simple numeric scale each session so the trend can be tracked over the coming weeks

Question 5. What co-occurring factor is most important to screen for that also raises Scott’s acute risk this week?

A. An emerging neurocognitive decline that could explain why he has trouble concentrating during the current crisis

B. A long-standing eating disorder that might be contributing to his poor sleep and his withdrawal from his children this month

C. A previously undetected primary psychotic disorder that could better explain his numbness and withdrawal from others

D. His increased weekend drinking, which can heighten impulsivity and lower the threshold for acting on suicidal thoughts

Question 6. What is the most appropriate cornerstone of Scott’s safety plan?

A. A collaborative safety plan identifying his warning signs, coping strategies, supports, and crisis contacts, built together with him

B. A detailed weekly schedule of pleasant activities intended to lift his mood enough that the suicidal thoughts eventually fade

C. A firm instruction that Scott must call the counselor’s personal phone the moment any suicidal thought enters his mind

D. A signed no-suicide contract in which Scott formally promises in writing that he will not harm himself before the next appointment

Question 7. Scott is willing to build a safety plan but says he’d rather “get into the counseling stuff first” before talking about the gun. How should the counselor proceed?

A. Spend the current session exploring the roots of his depression in depth and revisit the firearm only if it comes up again naturally

B. Address means restriction for the firearm now, alongside starting the broader safety plan, rather than delaying it to a future session

C. Tell him firmly that counseling cannot continue at all until the firearm is completely removed from the home today

D. Follow his lead and postpone the firearm conversation until rapport has developed further over the next several sessions

Question 8. What is the most appropriate way for the counselor to address the firearm itself?

A. Suggest he simply unload the firearm and keep the ammunition stored in a separate drawer within the very same bedroom

B. Respect his autonomy and leave the decision about the firearm entirely up to him without raising the topic again

C. Counsel collaboratively on means restriction, such as temporarily storing the firearm with a trusted person away from the home or using a lock

D. Contact local police directly to have the firearm confiscated from his home without first discussing the plan with Scott

Question 9. After a run of high-risk sessions, the counselor feels drained and notices a pull to accept Scott's "I'd never do it" at face value; what is best?

A. Accept his reassurance for this week and return to the firearm discussion in a later session once the counselor feels rested

B. Recognize the fatigue may be biasing judgment, finish his risk and means-safety work now, then seek consultation and support

C. Hand Scott off to a colleague before the session ends, without finishing his risk assessment, so a fresher counselor takes over

D. Ignore the tiredness and push through, since noticing personal reactions has no bearing on the quality of his risk care

Question 10. As the counselor and Scott build his treatment plan, how should his military background, job loss, and separation be integrated into it?

A. Assume that as a veteran he will reject emotional work, and build a plan limited to practical, task-focused goals only

B. Leave his military history out of the plan, since his current distress stems from civilian stressors rather than his service

C. Explore what his service, job loss, and separation mean to him, and fold his views and any fitting resources into the plan

D. Address his job loss and separation only after the safety plan is finished and all of his suicide risk has fully resolved

Question 11. Scott expresses hopelessness that his marriage and career will ever be okay again. How can the counselor best instill hope without dismissing his pain?

A. Redirect the conversation to practical job-search tasks whenever his hopelessness about the marriage and career comes up

B. Agree that his situation does look genuinely hopeless right now and focus only on getting him through the next few hours

C. Promise him confidently that his wife will likely return and that a new job will come soon if he just stays positive

D. Validate how overwhelming the losses feel right now while helping him see the current crisis and his options as changeable over time

Question 12. Later, with his risk reassessed and safety plan in place, Scott begins some video sessions, and the counselor notices the platform uses one reusable link for all clients; what is best?

A. Keep the shared link, since a vendor that signs a business associate agreement makes every platform setting compliant by default

B. Ask Scott to keep his camera off at the start of sessions in case another client joins early, since that protects his privacy

C. Post the reusable link on the practice website so clients can find it easily, since a meeting link holds no health information

D. Switch to unique session links or a locked waiting room so only Scott can join, as HIPAA access safeguards require

Question 13. Scott agrees to a collaborative safety plan and to temporarily store the firearm with his brother. How should the counselor document this session?

A. Leave the details of the suicide risk assessment out of the chart entirely to protect his privacy as a veteran

B. Record only that Scott promised he would not harm himself before the next scheduled appointment with the counselor

C. Write a brief note stating simply that the client is fine now and that no further risk monitoring is needed going forward

D. Document the risk assessment findings, the means-restriction steps agreed to, the safety plan, and the clinical rationale for the disposition

Case 5 of 10

Years of mood ups and downs that never reach full episodes

Diagnosis: Cyclothymic Disorder

Intake: Priya Nair, a 27-year-old graduate student, describes more than two years of frequent mood swings. She has stretches of elevated energy and confidence and other stretches of low mood and fatigue, neither of which has ever fully met the threshold for a manic or major depressive episode.

Session Two: Over these two years she has not been free of symptoms for more than a month or two at a time. The mood shifts are not tied to a consistent interpersonal trigger and unfold over days to weeks rather than within hours, and her sense of self and her relationships are stable between them.

Session Three: A medical and substance review was unremarkable. The swings interfere with her studies and leave her unsure which version of herself to trust, but she denies discrete episodes of clearly elevated, expansive mood with the impairment that would mark a hypomanic or manic episode.

Question 1. What is most important to confirm to support a cyclothymic disorder diagnosis?

A. Two or more years of subthreshold mood swings with no symptom-free stretch longer than about two months

B. That she currently meets the full symptom count required for a discrete major depressive episode at this time

C. That her swings have become a little more frequent over the past few days than they were the week before

D. That she can identify the single stressful event she believes first set off her pattern of mood swings

Question 2. During Priya’s early sessions, the counselor, whose caseload is heavy with trauma survivors, notices a new urge to read her ordinary low stretches as signs of hidden danger; what is the best step?

A. Check the urge against Priya’s actual reports, and assess whether trauma exposure is shifting the counselor’s sense of danger

B. Treat the urge as accurate clinical intuition and increase Priya’s session frequency so her low stretches are monitored closely

C. Attribute the urge to an ordinary busy week and plan a restful weekend, without any further review of where the reaction came from

D. Tell Priya about the counselor’s trauma-heavy caseload so she understands why her sessions may feel tense or closely watched

Question 3. A colleague suggests coding Priya’s insurance claims as bipolar II disorder because her plan authorizes more sessions for it; what is the most appropriate response?

A. Use the bipolar II code for now, since her mood swings could later progress and the extra sessions would help her

B. Bill the cyclothymic disorder diagnosis her assessment supports, with notes documenting its criteria and impairment

C. Bill an unspecified mood disorder code so that no specific diagnosis appears in any records the insurer may later review

D. Submit the claims without diagnostic notes and complete the documentation only if the insurer requests an audit

Question 4. How can the counselor most accurately judge the counselor’s own strengths and limitations for treating Priya’s cyclothymic disorder?

A. Rely mainly on the counselor’s years of general practice, since broad experience reflects skill with any mood disorder

B. Rely mainly on how comfortable and confident the counselor feels while working with Priya during their sessions

C. Compare self-appraisal with outcome data and supervisor or peer feedback from past cases of similar mood disorders

D. Ask Priya at intake to rate the counselor’s competence and treat her rating as the main gauge of the counselor’s fit

Question 5. What is most important to screen for across the course of her mood disorder?

A. A previously undetected primary psychotic disorder that might better explain her elevated, confident periods

B. Suicide risk and any escalation toward a full mood episode, monitoring across both the high and low periods

C. A long-standing eating disorder that could be the single underlying cause of her fluctuating energy levels

D. An emerging neurocognitive decline that could explain why she feels unsure which version of herself to trust

Question 6. Priya’s mood-tracking app can share her daily entries with the counselor, and she wants to turn this on; what should the counselor clarify before she does?

A. That the counselor will watch her entries every day so any warning sign is caught and acted on right away

B. That she can text her entries to the counselor’s personal phone instead, since that is quicker than the app

C. Nothing further, since her intake consent already covers any app or digital tool used during her treatment

D. When the counselor reviews shared entries, that they are not monitored live, and where to turn in a crisis

Question 7. What lifestyle target is most important to emphasize given her presentation?

A. Focusing only on her diet and leaving sleep, activity, and daily routine entirely out of the plan for now

B. Consistent sleep, activity, and daily routines, since rhythm disruption can amplify her mood swings

C. Eliminating all structure and routine so that she is free to follow whatever her mood happens to dictate each day

D. A demanding new exercise regimen pursued only during her high periods and dropped entirely during her low periods

Question 8. How should progress be tracked over the course of treatment?

A. Rely only on whether she happens to say in session that her mood feels a little steadier than before

B. Use a daily mood-and-routine chart with periodic measures to monitor cycling, function, and risk over time

C. Track only how she feels on the single day of each appointment and ignore the pattern across the weeks between visits

D. Wait until the end of treatment to review whether her mood pattern has changed at all since intake

Question 9. How should the counselor handle her questions about medication?

A. Tell her that medication would certainly ruin her creativity and that she should refuse it if it is ever offered

B. Coordinate a referral to a prescriber for evaluation while continuing the rhythm-focused psychotherapy within scope

C. Recommend a specific mood-stabilizer and dose for her to begin taking before her next set of exams

D. Ignore the medication question and simply redirect her back to the routine charting every time she raises it

Question 10. During a high period Priya says she feels great and wants to stop treatment. The most therapeutic response is to:

A. Explore the wish nonjudgmentally and use psychoeducation to connect the highs to the broader cycling pattern

B. Let the topic pass without comment and simply continue the session as though she had not raised stopping at all

C. Warn her sharply that she will inevitably crash and that stopping now would be a serious and irreversible mistake

D. Agree that she is clearly cured now and end treatment, since she reports feeling so well during this current stretch

Question 11. How can the counselor best support Priya's engagement given the chronic, fluctuating course?

A. Tell her counseling cannot help unless she first agrees to take medication exactly as a prescriber directs

B. Promise her that a few weeks of focused treatment will permanently end the mood swings for good and resolve everything she is struggling with

C. Focus the sessions mainly on cataloging each individual mood shift in exhaustive detail every single week

D. Normalize the long-term nature of the work and anchor it in goals she values across both her high and low periods

Question 12. Priya asks the counselor to leave her diagnosis out of records shared with her university for accommodations. The most appropriate action is to:

A. Refuse to provide any documentation at all and decline to discuss what could appropriately be shared with the university

B. Discuss confidentiality and the limits of any release, sharing only what she consents to and what is needed

C. Send her full clinical record, including the diagnosis, to the university right away since they will need everything anyway

D. Write whatever the university prefers to see, even if it is not accurate, so that her accommodations are approved quickly

Question 13. The counselor is unsure whether the presentation has crossed into bipolar II. The most ethical course of action is to:

A. Transfer her at once to another provider with no explanation, consultation, or coordinated plan for the transition

B. Tell her the distinction does not matter at all and that there is no need to clarify which disorder she has

C. Keep the original diagnosis fixed and never revisit it, regardless of any new information that emerges over time

D. Seek consultation and reassess against the criteria, coordinating referral if the picture exceeds your competence

Case 6 of 10

Psychosis that persists between mood episodes

Diagnosis: Schizoaffective Disorder, Bipolar Type

Intake: Hank Delaney, a 34-year-old former rideshare driver, is referred after a hospitalization. Over the past two years he has had episodes of elevated, expansive mood with little sleep and grandiose plans, as well as periods of persecutory delusions and auditory hallucinations.

Session Two: Records and his account show at least two weeks of delusions and hallucinations occurring while his mood was stable and neither high nor low. Mood episodes — both manic and depressive — have been present for the majority of the illness, and substance and medical causes were excluded.

Session Three: Currently his mood is euthymic but he still hears critical voices and remains guarded. He is taking medication intermittently, has lost housing stability, and at one low point had fleeting thoughts that life was not worth living, without current plan or intent.

Question 1. What is most important to confirm to support a schizoaffective disorder diagnosis?

A. That he currently shows the exact number of neurocognitive deficits required to diagnose a major neurocognitive disorder

B. At least two weeks of psychosis without a mood episode, with mood episodes present for most of the illness

C. That his symptoms have grown a little more intense over the past few days than they were the week before that

D. That he can identify the single stressful event he believes first set off his very first hospitalization

Question 2. Right after a tense session in which Hank accused the counselor of reporting on him, the counselor

notices lingering tension with the next client due in minutes; what is best?

A. Take a few minutes for a grounding routine, such as paced breathing, before greeting the next client calmly

B. Start the next session immediately and trust that the tension will fade once the conversation is underway

C. Vent about Hank’s accusations to a colleague in the hallway to release the tension before the next session

D. Cancel Hank’s upcoming appointments for a few weeks so the counselor can avoid another tense encounter with him

Question 3. Hank says the busy waiting room makes his critical voices louder and asks whether he could wait somewhere quieter; what is the most appropriate response?

A. Explain that accommodations apply only to physical disabilities, so he will need to cope in the waiting room

B. Treat it as a reasonable disability accommodation and arrange a quieter place to wait, at no added cost to him

C. Require a new letter from his psychiatrist proving his disability before making any change at all to where he waits

D. Book him only for the last slot of the day when the office is empty, and add an after-hours fee to cover it

Question 4. At intake, the counselor realizes Hank once drove the counselor on a single rideshare trip years ago, with no contact since; what is the most appropriate step?

A. Refer him elsewhere at once, since any prior contact outside counseling rules out a professional relationship

B. Say nothing about the trip, since raising it could feed his mistrust and harm the early working relationship

C. Judge that the brief contact poses no conflict, acknowledge it openly with him, and document the review

D. Ask the hospital team to decide whether the counselor may keep his case and follow whatever they conclude

Question 5. Given his fleeting thoughts that life was not worth living, what is the most important assessment step?

A. Defer any risk assessment until his psychotic symptoms have fully resolved and his mood has been stable for several months

B. Assume the risk has fully passed because he says the thoughts were only fleeting and he denies any plan right now

C. Focus only on his housing situation for now and plan to return to any questions about his safety at a much later date

D. Conduct a structured suicide-risk assessment of current ideation, intent, plan, and protective factors now

Question 6. What is the most appropriate first-line treatment recommendation?

A. Adjust his antipsychotic and mood-stabilizer doses yourself and monitor his blood levels over the coming weeks

B. Begin intensive insight-oriented therapy alone to help him uncover the deeper meaning of his hallucinations before anything else

C. Coordinate combined pharmacotherapy and psychosocial support within a treatment team and your counseling role

D. Recommend a course of weekly supportive talks on their own as the complete treatment for his condition going forward

Question 7. Given his intermittent medication use, what should the plan emphasize?

A. Collaborative work on engagement and adherence, addressing the barriers that interrupt his medication and care

B. Tell him that if he will not take his medication exactly as directed there is no point in continuing any counseling at all

C. Leave all decisions about his medication entirely to him without ever raising adherence as a topic in the counseling work

D. Advise him to stop the medication during good stretches and only restart it whenever his symptoms happen to come back

Question 8. How should the counselor address his unstable housing within the plan?

A. Coordinate case management and community resources for housing as part of recovery-oriented, wrap-around support

B. Personally guarantee a lease and advance him the rent yourself so that his housing problem is resolved immediately

C. Treat his housing situation as completely unrelated to his mental health and leave it out of the plan entirely

D. Tell him to sort out his own housing entirely before he returns, since counseling cannot start until his life is stable

Question 9. How should the counselor coordinate with his prescriber and the hospital team?

A. Release every detail of his record to all parties so they each hold the complete picture of everything in his treatment

B. Share his full history with everyone involved right away, since coordinating his care is plainly in his best interest anyway

C. Obtain his informed consent and a release, then collaborate with the team and share what is needed for his care

D. Avoid contacting the prescriber or the hospital at all so that his counseling stays completely separate from his other care

Question 10. During a session Hank says the voices are mocking him right now. The most therapeutic response is to:

A. Acknowledge how distressing the voices are and stay supportive, without confirming or arguing about their reality

B. Change the subject immediately so the conversation never touches anything that might upset or provoke him further

C. Agree with him that the voices are real people who are truly speaking to him so that he feels you are on his side

D. Insist firmly and repeatedly that the voices are not real until he finally agrees that there is nothing there at all

Question 11. How can the counselor best build engagement given his guardedness?

A. Press him to disclose all of his most private experiences in the very first sessions to speed the work along quickly

B. Tell him counseling can do nothing for him unless he first agrees to take his medication exactly as prescribed

C. Move at his pace, be transparent and consistent, and work toward concrete goals he chooses and values

D. Focus the sessions mainly on collecting ever more detail about the content of the voices he hears each week

Question 12. Hank discloses a specific, credible plan to harm an identifiable person. The most appropriate action is to:

A. Notify the police and the person at once without first assessing the seriousness or following the proper protective process

B. Wait to see whether he repeats the threat at the next appointment before assessing the risk or taking any action

C. Keep the threat entirely confidential, since everything shared in counseling must always stay completely private

D. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

Question 13. The counselor has limited experience with serious psychotic illness. The most ethical course of action is to:

A. Seek consultation and work within a treatment team, referring for the components beyond your competence

B. Tell him his illness is not serious enough to need a team and keep seeing him for supportive talks only

C. Transfer him at once to another provider with no explanation, consultation, or coordinated plan for the transition

D. Continue managing the entire case alone to preserve the continuity of the relationship he has built with you

Case 7 of 10

Childhood motor and vocal tics with social and school impact

Diagnosis: Tourette’s Disorder

Intake: Jamal Carter, an 11-year-old, is referred for involuntary movements and sounds. Since age seven he has had multiple motor tics — blinking, head jerks, shoulder shrugs — and at least one vocal tic, such as throat-clearing and grunting, occurring many times a day on most days.

Session Two: Both motor and vocal tics have been present for more than a year, which fits Tourette’s rather than a tic disorder limited to one type. The tics are not driven by intrusive obsessions, and they are involuntary movements rather than a functional neurological symptom pattern.

Session Three: He feels embarrassed and is being teased at school, and he has begun avoiding class participation. His parents worry about his self-esteem, and he says he sometimes feels bad about himself, without any current thoughts of self-harm, and wishes the tics would just stop.

Question 1. What is most important to confirm to support a Tourette’s disorder diagnosis?

A. That his parents can name the single event they believe first caused his tics to begin

B. Multiple motor tics and at least one vocal tic present for more than a year with childhood onset

C. That his tics have grown a little more frequent over the past few days than they were the week before

D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Jamal's parents, with his agreement, want to send phone videos of his tics so the counselor can see them as they occur at home; how should the counselor maintain these files?

A. Have them text the videos to the counselor's personal phone and delete each clip right after viewing it once

B. Ask them to post the clips to an unlisted video-sharing link so the counselor can view them at any time later

C. Save the clips to a shared office drive folder so any staff member can view them while scheduling visits

D. Have them upload clips to the practice's secure portal and keep them in his record, limited to his care team

Question 3. The counselor was trained on a clinician-rated tic severity scale but has not used it in years; what is best before relying on Jamal's scores?

A. Review the manual and scoring rules, and co-rate a session with a trained colleague to confirm reliable use of the scale

B. Use the scale at once, since training on an instrument once means the counselor's competence with it never needs refreshing

C. Have his parents complete the scale at home, since a clinician-rated measure works just as well when filled out by family

D. Skip standardized measurement entirely, since tic severity is judged more accurately by the counselor's general impressions

Question 4. Later, in a legal case the family brings over the school teasing, a judge orders release of Jamal's counseling records about his distress; what is best?

A. Refuse to release anything, since a child's counseling confidentiality outweighs any court order issued in the case

B. Consult legal counsel, inform his parents, and release only what the order specifies, seeking any protective handling

C. Send the complete file, including family session notes, since a judge’s order removes all limits on disclosure

D. Release the records only if Jamal himself agrees, since the counseling relationship is with him, not the court

Question 5. Given the teasing and his remark that he feels bad about himself, what is most important to assess?

A. A previously undetected primary psychotic disorder that might better account for his low self-esteem

B. The emotional impact on his self-esteem and mood, with developmentally appropriate screening for safety

C. An emerging neurocognitive decline that could account for why he is avoiding participating in his class

D. A long-standing eating disorder that could be the single underlying cause of his low mood and tics

Question 6. At the end of a session, Jamal shyly hands the counselor a drawing he made of the two of them playing a game; what is the most appropriate response?

A. Politely decline it, explaining that counselors may never accept any gift from a client of any age at all

B. Accept it and bring him a gift of similar value next week so that the exchange between the two of them stays even

C. Accept it warmly and explore its meaning with him, since a small handmade token from a child fits boundaries

D. Accept it and display it in the waiting room with his name so other children can enjoy his artwork too

Question 7. How should the counselor address the teasing and school impact?

A. Provide psychoeducation and coordinate school supports and anti-bullying measures to protect his participation

B. Focus only on reducing the tics and leave the teasing and school environment entirely out of the plan

C. Advise his parents to keep him out of school until the tics stop so that the teasing simply cannot happen and his embarrassment can ease

D. Tell him to just ignore the teasing and that he should not let what other children say bother him at all

Question 8. How should progress be tracked over the course of treatment?

A. Track only the raw number of tics per minute and base every decision solely on that one figure

B. Rely only on whether his parents happen to mention that he seems a little happier at home lately

C. Wait until the end of treatment to review whether anything has changed at all since the first session

D. Track tic frequency, distress, school participation, and self-esteem over time to guide the plan

Question 9. How should the counselor coordinate with the school and any medical providers?

A. Contact the school and providers directly with his clinical details right away since they will need the information

B. Send his full clinical record to the school so that every staff member has complete information about his care

C. Avoid contacting the school or any providers at all so that his counseling stays entirely separate from them

D. With the parents' consent and a release, coordinate school supports and any medical care for the tics

Question 10. Jamal says he hates his tics and wishes he were "normal like everyone else." The most therapeutic response is to:

A. Change the subject to something more fun so that he does not have to sit with the difficult feelings at all

B. Tell him he just needs to try harder to control the tics so that he can be like the other children

C. Validate his feelings, provide age-appropriate psychoeducation, and build self-acceptance and coping

D. Agree that the tics do make him quite different from everyone else so that he learns to accept that reality

Question 11. How can the counselor best engage Jamal given his age and embarrassment?

A. Use developmentally appropriate, strengths-based methods that build coping and confidence at his level

B. Tell him he must stop his tics before any of the counseling work to help his confidence can begin

C. Talk with him mainly in abstract, adult terms about the neurology of why his tics occur each week

D. Focus the sessions mainly on counting and reviewing each of his tics in exhaustive detail every week with his parents

Question 12. Jamal's parents ask the counselor to write that his tics are "fully under control" for a school activity he wants to join. The most appropriate action is to:

A. Write the statement exactly as they request, since keeping the parents satisfied is what matters most here

B. Refuse to provide any documentation at all and decline to explain what could appropriately be reported about him

C. Decline to misrepresent his status and clarify what the assessment and your role can accurately support

D. Take the parents' side and advocate for the claim in any letter, regardless of what the assessment shows

Question 13. The counselor has limited training in behavioral tic interventions. The most ethical course of action is to:

A. Seek consultation and training, and refer or coordinate for the components beyond your competence

B. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

C. Tell the parents the tics are not serious enough to need specialized help and continue with general support only

D. Continue managing the case alone to preserve the continuity of the relationship built with the family

Case 8 of 10

Months of broken sleep and exhausting days

Diagnosis: Insomnia Disorder, Persistent

Intake: Andre Bell, a 50-year-old accountant, presents with about five months of difficulty falling and staying asleep at least four nights a week despite a regular schedule and adequate time in bed. He feels exhausted, irritable, and foggy at work during the day.

Session Two: He lies awake for an hour or more, watches the clock, and worries about not sleeping; he then naps and uses caffeine to get through the day. He denies loud snoring, witnessed pauses in breathing, or gasping awake, and his partner has not noticed these signs.

Session Three: He says the sleep problem is his main concern and is not driven by pervasive daytime worry across many areas or by persistent low mood and anhedonia. He has begun spending long stretches in bed trying to force sleep, which seems to make the nights worse.

Question 1. What is most important to confirm to support a diagnosis of insomnia disorder, persistent?

A. That he has very little opportunity to sleep because of a demanding work schedule that keeps him up late most nights

B. That his sleep has become noticeably worse over the past two or three nights compared with how it was the week before

C. That he can identify the single stressful event he believes first triggered his current difficulty with falling and staying asleep at night

D. Difficulty with sleep at least three nights a week for over three months, with daytime impairment despite adequate opportunity

Question 2. When opening the progress note for Andre’s second session, the counselor sees the EHR has copied forward his entire intake note; what is best?

A. Sign the copied note as written, since the EHR feature exists to save time and his presentation has not changed much

B. Print the note, correct it by hand, and file the paper copy in a drawer kept separate from his electronic health record

C. Ask front-desk staff to finish the note under the counselor’s login so the documentation is done before the next client

D. Revise the note so it records only what occurred in this session, removing carried-over text that no longer applies

Question 3. Later, after a session, the counselor realizes the counselor kept pushing bedtime-routine suggestions while Andre tried to describe his nighttime worry; what is best?

A. Reflect on what drove the push, such as a need to show quick progress, and plan to balance structure with listening next time

B. Conclude that Andre was avoiding the work and press harder on it next session, since structure is the core of his sleep treatment

C. Apologize at length at the next session, describing in detail the counselor’s own feelings of embarrassment about the misstep

D. Move on without reviewing the session, since reflecting on past sessions cannot change anything that has already happened in them

Question 4. Later, the counselor considers suggesting an AI sleep-coaching chatbot to support Andre between sessions; what should the counselor do before recommending it?

A. Recommend it freely, since AI chatbots are held to the same licensure and ethics standards that apply to counselors and therapists

B. Review its evidence, privacy terms, and crisis limits, and frame it as a supplement to review in session, not a substitute

C. Let the chatbot set his sleep window on its own, since an AI tool calculates a sleep schedule more precisely than a counselor can

D. Rule out any AI tool, since counselors are barred by federal law from discussing or recommending AI technology to their clients

Question 5. What assessment tool best informs the evaluation and the treatment plan?

A. Ordering an overnight polysomnography sleep study yourself to confirm the diagnosis before beginning any treatment planning

B. A detailed reconstruction of his childhood sleep habits across his earliest years to find the origin of the problem

C. A single global question asking him to estimate how well he generally thinks he sleeps on an average sort of night

D. A daily sleep diary tracking bedtime, sleep onset, awakenings, rise time, and daytime functioning over a couple of weeks

Question 6. The counselor is trained in CBT-I basics but has never set or adjusted a sleep-restriction schedule; what is best before using it with Andre?

A. Start the sleep-restriction schedule now from a general online guide and adjust it later if Andre reports any problems

B. Replace sleep restriction with general sleep-hygiene tips, since those need no added skill and are just as effective

C. Consult a colleague experienced in CBT-I on setting his sleep window and safety cautions such as daytime drowsy driving

D. Refer Andre out of the practice, since lacking experience in one component means the counselor cannot treat him at all

Question 7. He spends long stretches in bed trying to force sleep. Which CBT-I component most directly addresses this?

A. Recommending that he take a long daytime nap whenever he feels tired so that he can make up for the lost nighttime sleep

B. Advising him to go to bed much earlier and to stay in bed longer so that he has more total opportunity to fall asleep

C. Encouraging him to remain in bed and keep trying to fall asleep no matter how long he ends up lying awake during the night

D. Stimulus control and sleep restriction, which strengthen the bed-sleep association and consolidate his time asleep

Question 8. Andre asks whether he should use sleeping pills. The most appropriate counselor action is to:

A. Reassure him that nightly over-the-counter sleep aids are perfectly safe to rely on indefinitely for his chronic insomnia

B. Recommend CBT-I as first-line and, if he wants to consider medication, coordinate a referral to a prescriber with his consent

C. Tell him which specific sleeping medication and dose he should ask his physician to prescribe for him at his next appointment

D. Discourage any medication entirely and tell him he must manage his insomnia through willpower and discipline alone

Question 9. How should the counselor monitor his response to CBT-I over time?

A. Track mainly whether he attends his sessions and treat consistent attendance as the main sign that he is improving

B. Ask him each week whether he subjectively feels that his sleep is generally going better than it was before

C. Review the ongoing sleep diary at intervals to track sleep efficiency and daytime functioning and adjust the plan

D. Wait until the agreed course of CBT-I is fully over and then evaluate his overall response in one single review

Question 10. Andre says he is convinced one bad night will ruin his entire next day. The most therapeutic response is to:

A. Address the catastrophic sleep beliefs with cognitive work, since such beliefs heighten arousal and maintain the insomnia

B. Explain in technical detail the neuroscience of sleep architecture to convince him on scientific grounds not to worry

C. Agree that a single poor night really will likely ruin his day and help him plan how to clear his schedule for those days

D. Reassure him repeatedly that he will definitely sleep well tonight so that he can stop worrying about it before bed

Question 11. What psychoeducation is most useful to offer Andre early in treatment?

A. Give him an extensive packet on sleep neuroscience and ask him to study it thoroughly before the next appointment

B. Explain how clock-watching, daytime napping, and effortful sleep maintain insomnia and how CBT-I targets that cycle

C. Reassure him that most people with insomnia eventually sleep fine again, so there is really nothing for him to address now

D. Tell him the most important thing is simply to relax and not to think about his sleep at all from this point forward

Question 12. Andre reveals he has been taking a friend's prescription sedative to sleep and asks the counselor to keep it off the record. Best response?

A. Agree to leave the sedative use out of the record entirely so that he feels comfortable continuing to be open with you

B. Report him immediately to the authorities for the borrowed medication before discussing the safety concern with him at all

C. Tell him this is none of your concern as a counselor since you do not prescribe, and simply move on without addressing the borrowed-medication use

D. Address the safety risk of using another person's prescription and document accurately rather than omitting it from the record

Question 13. His employer asks the counselor whether his sleep problems make him unsafe to work. Best response?

A. Share a general summary of his sleep treatment so the employer can decide for themselves whether he is fit to work

B. Decline to disclose without a valid release and clarify the limits of what a treating counselor can appropriately attest to

C. Tell the employer he is perfectly safe to work so that he does not get into any trouble at his job over his sleep problems

D. Refuse to engage with the employer at all and hang up without explaining anything about confidentiality or the process

Case 9 of 10

Persistent erectile difficulty straining a relationship

Diagnosis: Erectile Disorder

Intake: Elijah Lee, a 44-year-old accountant, seeks help for about eight months of consistent difficulty obtaining and maintaining an erection during partnered sexual activity. The difficulty causes him significant distress and has strained his relationship with his partner.

Session Two: The problem occurs on most occasions and has persisted well beyond the six-month threshold, causing clinically significant distress. It is not fully explained by another mental disorder, and he denies that the difficulty is attributable to a substance or medication he is taking.

Session Three: He describes performance anxiety and anticipatory worry that worsen the difficulty, and he denies that a primary depressive episode is the driver. A medical evaluation is underway, he is embarrassed but motivated, and he wants to restore intimacy and reduce the anxiety around sex.

Question 1. What is most important to confirm to support an erectile disorder diagnosis?

A. Difficulty obtaining or maintaining an erection on most occasions for at least six months, causing distress

B. That he currently meets the full symptom count required for a discrete major depressive episode at this time

C. That his difficulty has grown a little worse over the past few days than it was the week just before that

D. That he can pinpoint the single event he is convinced first caused his difficulties with erections

Question 2. During Elijah’s intake, building security announces a lockdown for a reported armed intruder; what should the counselor do first?

A. Leave the office with Elijah right away and head for the parking lot so both of them can get out of the building fast

B. Continue the intake quietly, since stopping the session would heighten Elijah's anxiety over an unconfirmed report

C. Step into the hallway alone to check whether the threat is real before deciding whether the lockdown plan applies

D. Follow the practice's lockdown plan: lock the door, turn off the lights, and stay with Elijah away from windows

Question 3. To support consultation, the counselor wants to audio-record Elijah's sessions, and practice policy requires written consent for any recording; what is best?

A. Explain the recording's purpose, who will hear it, how it is stored, and his right to decline, then get his written consent

B. Record the sessions without mentioning it, since recordings made only for consultation are exempt from the practice's policy

C. Ask for his verbal okay at the start of a session and begin recording, skipping the written form since he is an adult client

D. Record the sessions and send the files to the consultant by personal email, since the consultant is bound by confidentiality

Question 4. Later, Elijah's insurer asks for the counselor's separately kept psychotherapy notes to review his claims; what does HIPAA require?

A. Send the notes now without asking Elijah, since HIPAA allows disclosures for payment purposes without the client's authorization

B. Release the notes only with Elijah's specific written authorization, which the insurer may not require as a condition of payment

C. Refuse to send the insurer any information, since federal law bars sharing any part of a counseling record for payment purposes

D. Send the notes once the insurer signs a confidentiality pledge, since that pledge can take the place of the client’s authorization

Question 5. What is most important to coordinate given his presentation?

A. Recommend a specific erectile-dysfunction medication and dose for him to start yourself before his next appointment

B. Arrange a neuropsychological evaluation for an emerging neurocognitive decline that could account for his sexual difficulty

C. Arrange a psychiatric evaluation for a previously undetected primary psychotic disorder that might better account for his sexual difficulty

D. Coordinate with his medical provider so the physical evaluation and psychological care proceed together

Question 6. Later, Elijah logs on to a video session from his car in his workplace parking lot; what should the counselor do before starting?

A. Begin right away, since a car is private enough and pausing to check his setting would only add to his embarrassment

B. Let him drive home during the session as long as he uses a hands-free device and keeps his eyes on the road ahead

C. Confirm he is parked, not driving, and able to speak privately, and verify his location before discussing sensitive topics

D. End the session and require all future sessions in person, since joining a telehealth session from a car is never acceptable

Question 7. How should the counselor address his performance anxiety?

A. Use anxiety-reduction and sensate-focus-style approaches that reduce performance pressure and rebuild intimacy

B. Avoid discussing his anxiety about sex entirely so that the sessions never touch on anything uncomfortable

C. Tell him to avoid all sexual activity indefinitely so that he never has to face the difficulty or the anxiety again

D. Encourage him to focus intensely on achieving an erection every time so that he proves to himself he still can

Question 8. How should progress be tracked over the course of treatment?

A. Wait until the end of treatment to review whether his difficulty has changed at all since the first session

B. Rely only on whether he happens to mention in session that things feel a little better than they did before

C. Track only whether he achieves an erection each time and base every decision solely on that single outcome

D. Track sexual function, performance anxiety, relationship satisfaction, and distress over time to guide the plan

Question 9. How should the counselor involve his partner in the work?

A. Tell him the relationship is not relevant and keep the work focused only on his individual performance anxiety

B. Refuse any contact with the partner under all circumstances, even where he requests and consents to couple sessions

C. Bring his partner into every individual session right away without discussing it or obtaining his consent at all

D. With his consent, consider couple-based work on communication and intimacy with appropriate agreements

Question 10. Elijah is deeply embarrassed and reluctant to discuss the sexual difficulty. The most therapeutic response is to:

A. Tell him there is no reason to be embarrassed and quickly move past the topic so he does not feel uncomfortable

B. Respond with a calm, nonjudgmental, and normalizing stance that makes it safe for him to talk about it

C. Joke about the difficulty to lighten the mood so that he does not take the whole thing quite so seriously

D. Insist he describe every detail of his sexual difficulties immediately before the work can move forward at all

Question 11. How can the counselor best support Elijah's engagement and hope?

A. Focus the sessions mainly on detailing every past failed sexual encounter in exhaustive detail every week

B. Tell him that this kind of difficulty usually cannot be improved and that he should simply learn to live with it

C. Frame the difficulty as common and treatable, anchor the work in his goals, and reinforce his motivation

D. Tell him counseling cannot help him at all unless he first starts an erectile-dysfunction medication right away

Question 12. Elijah asks the counselor to keep the diagnosis out of records his insurer could access. The most appropriate action is to:

A. Send his full clinical record, including the diagnosis, to the insurer right away since they may need it anyway

B. Falsify the record to remove the diagnosis entirely so that there is no chance the insurer could ever see it

C. Clarify confidentiality and the limits of any release, sharing only what he consents to and what is needed

D. Refuse to discuss the confidentiality question at all and simply change the subject whenever he raises it

Question 13. The counselor has limited training in sexual dysfunctions. The most ethical course of action is to:

A. Continue managing the case alone to preserve the continuity of the relationship built with him

B. Transfer him at once to another provider with no explanation, consultation, or coordinated plan for the transition

C. Seek consultation and training, and coordinate or refer for the components beyond your competence

D. Tell him the difficulty is not serious enough to need specialized help and continue with general talks only

Case 10 of 10

A defiant, irritable nine-year-old at home and school

Diagnosis: Oppositional Defiant Disorder

Intake: Eli Foster, a 9-year-old boy, is brought by his mother after months of escalating conflict. He argues with adults, refuses to follow rules, deliberately annoys his sister, blames others for his mistakes, and is often touchy, angry, and resentful at home and school.

Session Two: His mother reports the pattern has lasted more than six months and occurs with several different adults. He has not been physically aggressive toward people or animals, has not destroyed property or stolen, and has no history of serious rule violations beyond defiance.

Session Three: His teacher confirms similar defiance and irritability in class. Eli can be cooperative and warm when not in conflict, and there is no indication of abuse. His mother feels exhausted and is caught in escalating power struggles and inconsistent discipline at home.

Question 1. What is most important to confirm to support an oppositional defiant disorder diagnosis?

A. That his defiant behavior has clearly become more severe over the past week than it was the week before that

B. A 6-month pattern of angry/irritable mood and defiant, vindictive behavior occurring with more than one person or setting

C. That his behavior occurs only at home with his mother and not in any other setting or with any other adults

D. That he can clearly explain the single specific event that he believes caused him to start behaving this way at home

Question 2. At intake, Eli’s mother asks to use her employer health plan, which covers Eli as a dependent; what should the counselor do before billing?

A. Submit claims under his mother’s name as the patient, since she is the policyholder who is attending the sessions

B. Waive her copays as a routine courtesy so that cost is not a barrier, without telling the health plan about the waiver

C. Begin billing right away and check his coverage only if the plan later denies one of the claims for his counseling sessions

D. Verify Eli’s eligibility, benefits, and any prior authorization, and list him as patient with his mother as subscriber

Question 3. How should the counselor weigh a possible co-occurring ADHD?

A. Whether the ADHD symptoms are clearly more severe today than they were earlier in this same school year so far

B. ADHD can co-occur and should be assessed separately, since defiance differs from inattentive and hyperactive-impulsive symptoms

C. ADHD can be ruled out entirely on the basis that he is able to be cooperative and warm when he is not in conflict

D. The defiance is almost certainly just a direct expression of ADHD, so no separate assessment of ADHD is really needed

Question 4. Later, the practice’s video platform will not load for a session with Eli and his mother, who suggests a free consumer video app; what is best?

A. Troubleshoot the practice’s compliant platform or reschedule, since pandemic enforcement leniency for consumer apps ended

B. Switch to the free app for this one session, since his mother’s consent makes any video platform acceptable under HIPAA

C. Use the app if it is encrypted, since encryption alone makes a consumer app compliant without any business agreement

D. Move all sessions to the free app, since federal telehealth rules for minors are looser than the rules that apply to adult clients

Question 5. Later, the counselor adds the interactive complexity code to Eli’s individual psychotherapy session, which his mother joined and in which the two argued heatedly; what best supports the claim?

A. A note stating only that the session was difficult, since adding the code itself shows that the work was complex

B. A note describing how their escalating conflict complicated the session and what the counselor did to manage it

C. Billing the add-on code for every session with Eli, since sessions with any child are complex by their very nature

D. A note listing only Eli’s diagnosis and the session length, with no description of what occurred during the session

Question 6. Later, the counselor suggests a reward-chart app to help Eli’s mother track and praise his cooperative behavior; how should it be implemented?

A. Give Eli the app to run on his own so he can award himself points without his mother needing to be involved at all

B. Use the app mainly to log each act of defiance so that Eli can see a running count of his misbehavior every evening

C. Set it up in session with Eli and his mother around a few target behaviors, check its privacy terms, and review data weekly

D. Recommend the app and leave his mother to set it up alone, since apps like this are simple enough to need no review at all

Question 7. A central target of the parent-focused work would most appropriately be:

A. Replacing escalating commands and inconsistent follow-through with clear expectations, consistency, and positive reinforcement

B. Encouraging his mother to give in whenever the conflict becomes intense so that the household stays calmer overall

C. Advising his mother to ignore all of Eli's behavior entirely until he simply grows out of this difficult phase on his own

D. Coaching his mother to win each confrontation decisively so that Eli learns once and for all exactly who is in charge

Question 8. How should the school be involved in the plan?

A. With consent, coordinate consistent expectations and supports across home and school given the cross-setting pattern

B. Contact the school directly to share the details of his sessions without first obtaining the family's informed consent

C. Recommend that the school apply stricter punishments so that the consequences at school are tougher than those at home

D. Keep the school entirely out of the plan so that Eli's counseling remains a completely private matter for the family

Question 9. Eli's mother says she feels like a failure as a parent. The most therapeutic response is to:

A. Validate how exhausting the conflict is and reframe parent training as a skill set, not a verdict on her worth as a parent

B. Provide her with detailed statistics on how common oppositional behavior is so she can see that she should not feel responsible

C. Agree that her parenting has indeed been the main cause of the problem and that she will need to fix her mistakes

D. Tell her quickly that she is a wonderful parent and that there is really nothing she needs to change about her approach at all

Question 10. How can the counselor best engage Eli himself in the work?

A. Confront him firmly about every instance of his misbehavior so that he clearly understands just how serious the situation is

B. Build rapport, find his strengths and goals, and involve him in age-appropriate problem-solving rather than only correcting him

C. Focus the sessions only on listing his misbehaviors so that he is fully aware of everything he has been doing wrong lately

D. Tell him that if he does not change his behavior he will face increasingly serious consequences from everyone around him

Question 11. Because Eli is a minor, how should the counselor handle confidentiality with him and his mother?

A. Promise Eli that nothing he ever says will be shared with his mother under any circumstances at all throughout treatment

B. Share everything Eli says with his mother automatically, since as a minor he has no confidentiality interests at all to consider

C. Clarify at the outset, in age-appropriate terms, what will be shared with his mother and what stays private, within legal limits

D. Avoid discussing confidentiality with either of them so that the issue does not complicate the start of the treatment

Question 12. During a session Eli hints that an adult sometimes hits him hard enough to leave marks. Best step?

A. Confront the mother directly about the marks yourself before making any report through the official channels

B. Follow mandated-reporting duties, assess safety, and make the required child-abuse report through proper channels

C. Keep the disclosure confidential because the information came up during a private counseling session with the child

D. Wait several weeks to see whether Eli mentions the same thing again before deciding whether any action is needed at all

Question 13. The counselor has limited training in child and family behavioral treatment. The most ethical course of action is to:

A. Seek consultation and training, or refer to a clinician competent in parent training and child behavioral treatment

B. Tell the mother the behavior is just a normal phase that needs no specialized treatment and continue with supportive talks only

C. Continue to manage the case entirely alone to preserve the continuity of the relationship you have built with the family

D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

Answer Key

Case 1: Tearful and doubting her competence six weeks after childbirth

Diagnosis: Major Depressive Disorder, with Peripartum Onset

Q1 — Answer: A. That her depressive symptoms began within four weeks of delivery and have persisted nearly daily for over two weeks with impairment

Why it is correct: Onset window plus duration and impairment anchor the episode. *Common error:* Diagnosing from a single tearful moment without confirming duration.

- **B** — Worsening trajectory is not required for the diagnosis. *Trap:* Demanding a worsening course before diagnosing.
- **C** — A specific trigger does not establish the diagnosis. *Trap:* Treating an identifiable trigger as the diagnostic test.
- **D** — Family history supports but does not confirm the diagnosis. *Trap:* Requiring family history before confirming a current episode.

Q2 — Answer: A. Follow the plan: notify her by her listed contact, hold her session securely from the backup site, and document the change

Why it is correct: The plan sets client contact steps and secure telehealth from a backup site. *Common error:* Improvising or suspending care instead of following the plan.

- **B** — Indefinite cancellation leaves a depressed postpartum client without care. *Trap:* Treating an office closure as the end of services.
- **C** — Emergencies do not suspend HIPAA privacy and security duties. *Trap:* Assuming a crisis justifies an unsecured platform.

- **D** — The plan places the duty to notify clients on the practice. *Trap:* Leaving clients to discover disruptions on their own.

Q3 — Answer: B. Any thoughts of harming her daughter or herself, and any hallucinations, delusions, or disorganized thinking

Why it is correct: Direct infant-harm and psychosis screening is essential. *Common error:* Skipping direct safety questions because the presentation seems mild.

- **A** — Scheduling is administrative, not a safety screen. *Trap:* Confusing administrative follow-up with risk assessment.
- **C** — Feeding preference is not a safety screen. *Trap:* Substituting lifestyle questions for a safety screen.
- **D** — This bypasses the required safety screen entirely. *Trap:* Replacing a safety screen with a childcare-logistics discussion.

Q4 — Answer: B. Weigh the advice against the counselor's own assessment, keep responsibility for her care, and document the consultation

Why it is correct: Consultation informs care, but the treating counselor decides and documents. *Common error:* Treating a consultant's advice as a transfer of responsibility.

- **A** — The consultant advises; the treating counselor stays responsible. *Trap:* Handing clinical decisions to the consulting colleague.
- **C** — A consultant is not a co-treater of record for her care. *Trap:* Confusing consultation with shared treatment responsibility.
- **D** — Consultation that shapes care belongs in the record. *Trap:* Hiding consultation to protect the counselor's image.

Q5 — Answer: C. Begin with her self-doubt as she wishes, share the counselor's view on sleep, and agree to revisit the order as work goes on

Why it is correct: With no safety concern overriding it, her priority leads and the counselor's view is shared openly. *Common error:* Letting the counselor alone set the order of problems.

- **A** — Overriding her choice weakens collaboration and buy-in. *Trap:* Ranking problems by the counselor's theory rather than her priorities.
- **B** — Equal weighting spreads effort thin and can overwhelm her. *Trap:* Avoiding prioritization in order to seem thorough.
- **D** — Priorities guide treatment now; waiting stalls her care. *Trap:* Delaying planning until the client feels better.

Q6 — Answer: D. With her signed release, agree with Danielle and the nurse on shared goals, each one's role, and how they will stay in contact

Why it is correct: Consent-based, goal-linked coordination makes wraparound support useful. *Common error:* Working in parallel with no shared goals or roles.

- **A** — Collaborating with her support services serves her goals. *Trap:* Treating coordination as beyond the counselor's role.
- **B** — Handing off goals fragments her care and her ownership of it. *Trap:* Replacing collaboration with delegation.
- **C** — Full notes exceed what shared goals require. *Trap:* Equating collaboration with full record disclosure.

Q7 — Answer: D. Re-administer a validated perinatal measure such as the Edinburgh Postnatal Depression Scale at regular intervals

Why it is correct: Standardized repeated measurement guides care. *Common error:* Relying on impressions instead of a validated measure.

- **A** — End-only review misses early non-response. *Trap:* Skipping interim progress monitoring.

- **B** — Informal check-ins lack a standardized benchmark. *Trap:* Trusting an unstandardized impression alone.
- **C** — Engagement alone does not measure symptom change. *Trap:* Mistaking adherence for clinical improvement.

Q8 — Answer: A. Provide general information and, with her consent, coordinate a referral to a prescriber for a medication evaluation

Why it is correct: Referral and coordination stay within scope. *Common error:* Giving specific medication advice directly.

- **B** — Discouraging medication overreaches. *Trap:* Steering a medical decision unilaterally.
- **C** — Naming a specific medication exceeds scope. *Trap:* Overstepping professional scope of practice.
- **D** — Outsourcing to self-research is poor coordination. *Trap:* Replacing referral with unguided self-research.

Q9 — Answer: C. Collaboratively schedule small, manageable activities such as a short walk with the baby or a brief call to a friend

Why it is correct: Graded, valued activity rebuilds reward. *Common error:* Waiting for motivation to return before scheduling activity.

- **A** — Thought logs fit cognitive work, not the first behavioral step. *Trap:* Confusing behavioral activation with cognitive restructuring.
- **B** — Too much too soon invites failure. *Trap:* Ignoring graded pacing in favor of an abrupt full schedule.
- **D** — Waiting on motivation reverses the intervention logic. *Trap:* Letting mood gate behavior instead of scheduling it.

Q10 — Answer: A. Offer psychoeducation framing postpartum depression as common and treatable while validating how exhausting this feels

Why it is correct: Psychoeducation reframes self-blame with validation. *Common error:* Either only lecturing on facts or only sympathizing without information.

- **B** — Premature disputation can feel invalidating. *Trap:* Disputing a belief before validating the feeling behind it.
- **C** — Avoidance leaves the belief unaddressed. *Trap:* Steering away from emotionally central material.
- **D** — Endorsing the self-blame deepens the distortion. *Trap:* Colluding with a client's harshest self-judgment.

Q11 — Answer: A. Have her sign the practice's release form naming the obstetrician, the information to share, its purpose, and an expiration

Why it is correct: The practice's SOP requires its written release before disclosing. *Common error:* Bypassing the practice's release procedure for convenience.

- **B** — The procedure applies to medical contacts as well. *Trap:* Inventing exceptions to the practice's written procedure.
- **C** — Only essential information should be shared (ACA B.2.e). *Trap:* Sending the full chart when a summary suffices.
- **D** — Coordination with medical providers is allowed with proper release. *Trap:* Misreading policy as banning care coordination.

Q12 — Answer: A. The limits of confidentiality, including circumstances that would require the counselor to act to prevent serious harm

Why it is correct: Confidentiality limits must be disclosed up front. *Common error:* Omitting the limits of confidentiality from consent.

- **B** — Goals are set collaboratively, not imposed. *Trap:* Removing the client from her own treatment planning.
- **C** — Absolute confidentiality is an inaccurate promise. *Trap:* Guaranteeing unconditional secrecy.

- **D** — Clients may withdraw consent at any time. *Trap:* Treating initial consent as a binding, irrevocable contract.

Q13 — Answer: D. Confirm she has a private space and stable connection, review telehealth consent and confidentiality limits, and proceed

Why it is correct: Verify privacy and consent, then continue telehealth. *Common error:* Assuming telehealth suitability without checking privacy each time.

- **A** — Skipping the ongoing privacy check is unethical. *Trap:* Failing to reverify privacy conditions on an ongoing basis.
- **B** — A blanket rule against telehealth is unsupported. *Trap:* Ruling out an appropriate modality without justification.
- **C** — Pausing indicated treatment is not necessary. *Trap:* Delaying needed treatment for a manageable logistical issue.

Case 2: Sad and discouraged three weeks after a layoff

Diagnosis: Adjustment Disorder, with Depressed Mood

Q1 — Answer: A. That his sadness, sleep trouble, and low motivation began within three months of the layoff and track closely with it

Why it is correct: Onset tied to the stressor anchors the diagnosis. *Common error:* Skipping the timeline link between stressor and symptoms.

- **B** — Comparative severity ratings are not diagnostic. *Trap:* Substituting a rating scale for timeline history.
- **C** — Exhaustive work history is low yield here. *Trap:* Collecting low-yield background instead of the timeline.
- **D** — Family history is secondary to onset timing. *Trap:* Requiring family history before establishing onset.

Q2 — Answer: A. Complete a structured suicide-risk screen as a routine part of intake regardless of how mild his presentation seems

Why it is correct: Routine structured screening applies to every intake. *Common error:* Skipping risk screening because the case looks straightforward.

- **B** — Deferring routine screening delays needed data. *Trap:* Postponing baseline safety screening.
- **C** — Omitting screening based on appearance is unsafe practice. *Trap:* Judging risk by demeanor instead of asking directly.
- **D** — A pledge is not a substitute for structured screening. *Trap:* Using a written pledge instead of screening.

Q3 — Answer: A. How his sleep, mood, and motivation are affecting his job search and his ability to care for his two children

Why it is correct: Functional impact across domains reflects impairment. *Common error:* Focusing only on mood words instead of functional impact.

- **B** — Fairness perceptions do not measure impairment. *Trap:* Exploring grievance instead of functional impact.
- **C** — Job-task detail does not inform current impairment. *Trap:* Gathering job description detail over functional impact.
- **D** — Budget arithmetic is not a clinical priority. *Trap:* Treating budgeting detail as clinical assessment.

Q4 — Answer: B. Review the limited trials and reviews on adjustment disorder, such as problem-solving and brief CBT, weighing their quality

Why it is correct: Thin evidence still warrants appraising what research exists. *Common error:* Treating a sparse research base as no evidence at all.

- **A** — Personal experience alone is not empirical support. *Trap:* Substituting anecdote when research is limited.
- **C** — Evidence for another diagnosis cannot be assumed to apply. *Trap:* Borrowing a protocol for a different, more severe disorder.
- **D** — Popularity does not establish empirical support. *Trap:* Confusing online visibility with research evidence.

Q5 — Answer: C. Within four weeks, Marcus will fall asleep within 30 minutes on at least five nights a week, as tracked in his sleep log

Why it is correct: It is specific, measurable, attainable, realistic, and time-bound. *Common error:* Writing vague goals with no measure or deadline.

- **A** — No measure or timeframe defines what success means. *Trap:* Mistaking a general wish for a SMART goal.
- **B** — “Never” and eight hours nightly are unrealistic targets. *Trap:* Setting perfectionistic goals that guarantee failure.
- **D** — Hiring decisions lie outside of Marcus’s own control. *Trap:* Making the goal depend on other people’s choices.

Q6 — Answer: D. Record the drug as he names it, its start date, his better sleep, the morning grogginess, and its effect on interviews

Why it is correct: Document reported benefits, side effects, and functional impact. *Common error:* Recording vague notes that omit reported effects.

- **A** — Vague notes lose clinically useful information. *Trap:* Assuming medication details fall outside counseling records.

- **B** — Advising dose changes exceeds a counselor's scope of practice. *Trap:* Giving medication advice instead of coordinating with prescriber.
- **C** — Self-reported effects belong in the record now. *Trap:* Discounting client reports pending medical confirmation.

Q7 — Answer: B. Collaboratively identify concrete, achievable goals tied to coping with the job loss, such as structuring his days

Why it is correct: Collaborative, concrete goals fit brief stressor-focused work. *Common error:* Setting vague, counselor-imposed goals.

- **A** — Unilateral planning removes client input. *Trap:* Excluding the client from goal-setting.
- **C** — Delaying goals removes an active treatment structure. *Trap:* Waiting for spontaneous improvement instead of planning.
- **D** — Deep historical exploration is not the priority goal. *Trap:* Prioritizing depth work over present-focused goals.

Q8 — Answer: B. Incorporate problem-solving around concrete resources such as job-placement and financial-counseling referrals

Why it is correct: Concrete problem-solving supports the treatment goal. *Common error:* Treating concrete resource needs as outside the counseling scope.

- **A** — Ignoring the stressor narrows treatment too much. *Trap:* Avoiding the client's stated practical concern.
- **C** — Refusing to engage practical needs is overly rigid. *Trap:* Excluding practical problem-solving entirely.
- **D** — Direct career coaching exceeds the counselor's role. *Trap:* Substituting counselor time for specialized vocational services.

Q9 — Answer: C. Track his symptoms at each session and reassess if they persist or worsen well beyond the stressor resolving

Why it is correct: Ongoing monitoring flags a need to reassess the diagnosis. *Common error:* Assuming the diagnosis is fixed with no need to reassess.

- **A** — End-only review misses a worsening course early. *Trap:* Skipping interim check-ins.
- **B** — Attendance alone does not capture symptom course. *Trap:* Mistaking attendance for symptom monitoring.
- **D** — Failing to monitor risks missing a worsening course. *Trap:* Treating the initial diagnosis as permanently fixed.

Q10 — Answer: C. Validate that job loss is a significant stressor and normalize his reaction as a common, understandable response

Why it is correct: Validation and normalization counter self-blame. *Common error:* Minimizing his feelings or rushing past them.

- **A** — Redirecting away avoids the emotional content. *Trap:* Avoiding emotionally significant material.
- **B** — Blunt disputation can feel invalidating. *Trap:* Disputing a feeling before validating it.
- **D** — Agreeing with the self-critical comparison deepens shame. *Trap:* Colluding with the client's harsh self-judgment.

Q11 — Answer: D. Collaboratively schedule a small, valued activity, like one pickup basketball game, to rebuild a sense of momentum

Why it is correct: Graded, valued activity rebuilds engagement. *Common error:* Waiting for motivation to return before suggesting activity.

- **A** — Too much too soon risks setting him up to fail. *Trap:* Skipping graded pacing.

- **B** — Waiting on mood reverses the usual sequence. *Trap:* Letting mood gate all behavior.
- **C** — Cataloging the causes delays actual behavioral change. *Trap:* Substituting analysis for a concrete next step.

Q12 — Answer: A. Give him a HIPAA accounting within 60 days, omitting disclosures he authorized or made for treatment, payment, or operations

Why it is correct: HIPAA grants an accounting of certain disclosures within 60 days. *Common error:* Denying the right to an accounting of disclosures.

- **B** — HIPAA does grant a right to an accounting. *Trap:* Treating record access as the only client right.
- **C** — Disclosures he authorized to the EAP are excluded. *Trap:* Misidentifying which disclosures the accounting must cover.
- **D** — The first accounting in any 12 months is free. *Trap:* Charging for an accounting HIPAA requires free.

Q13 — Answer: D. Decline to share any information unless Marcus has signed a specific, informed release authorizing that disclosure

Why it is correct: Disclosure requires a valid signed release. *Common error:* Sharing progress information without a signed release.

- **A** — Ignoring the request avoids proper handling of it. *Trap:* Avoiding the issue instead of clarifying the process.
- **B** — Disclosing without authorization breaches confidentiality. *Trap:* Assuming a referral implies open information sharing.
- **C** — Informal, undocumented consent is insufficient. *Trap:* Relying on undocumented verbal permission.

Case 3: Cocaine use, crashes, and mounting consequences

Diagnosis: Stimulant Use Disorder (Cocaine)

Q1 — Answer: D. The number of DSM-5-TR criteria she meets over twelve months, since six or more place severity in the severe range

Why it is correct: Criterion count sets severity. *Common error:* Estimating severity by amount used alone.

- **A** — Reason for onset is not the criterion. *Trap:* Using the onset story to diagnose.
- **B** — Family history is risk, not diagnosis. *Trap:* Using family history to diagnose.
- **C** — Quantity alone does not set the diagnosis. *Trap:* Equating quantity with severity.

Q2 — Answer: B. A research-based course with skills practice and follow-up consultation, such as one drawing on SAMHSA's guidance for women

Why it is correct: Competence grows from evidence-based content, skills practice, and follow-up. *Common error:* Equating a brief certificate with competence.

- **A** — A brief certificate shows attendance, not skill. *Trap:* Mistaking a credential for demonstrated competence.
- **C** — Personal stories build empathy but are not clinical training. *Trap:* Substituting anecdote for evidence-based training.
- **D** — General content misses population-specific care needs. *Trap:* Assuming generic training covers a specific population.

Q3 — Answer: D. Assess medical risk and suicide risk, since stimulant use carries cardiac danger and crashes can bring acute low mood

Why it is correct: Screen cardiac and suicide risk. *Common error:* Overlooking medical and suicide risk.

- **A** — Depth history is secondary to safety. *Trap:* Prioritizing depth over safety.
- **B** — Trigger mapping is for later planning. *Trap:* Skipping the safety assessment.

- **C** — Source detail is low yield. *Trap:* Collecting irrelevant detail.

Q4 — Answer: C. Turn the assessed triggers into plan objectives, such as coping steps after late shifts and a set routine for handling pay

Why it is correct: Assessment findings should drive specific, targeted plan objectives. *Common error:* Leaving assessment results disconnected from the plan.

- **A** — Unused findings waste the assessment's guidance for planning. *Trap:* Documenting results without changing the plan.
- **B** — Triggers inform coping plans, not mandates about her job. *Trap:* Imposing drastic directives instead of collaborative objectives.
- **D** — A clear pattern is enough to guide planning now. *Trap:* Endlessly assessing instead of acting on results.

Q5 — Answer: D. Find a current program for her now, then update the list and set a routine check of each resource's status and services

Why it is correct: Fix her referral now and keep the list current. *Common error:* Relying on a referral list that is never verified.

- **A** — Passing on a closed resource delays her needed care. *Trap:* Handing off outdated referrals without checking them.
- **B** — Her needed level of care should not hinge on a stale list. *Trap:* Downgrading care because the list is outdated.
- **C** — Maintaining referral resources is a counselor responsibility. *Trap:* Shifting the work of finding resources onto the client.

Q6 — Answer: A. Explain the out-of-network status, her likely costs or a good faith estimate if self-paying, and in-network referral options

Why it is correct: Clients need network status and cost information up front. *Common error:* Leaving clients unaware of out-of-network costs.

- **B** — Billing under another provider's number misrepresents services. *Trap:* Using a colleague's credentials to bypass panels.
- **C** — Many plans reimburse part of out-of-network care. *Trap:* Misstating how out-of-network benefits work.
- **D** — Surprise costs undermine her informed consent to treatment. *Trap:* Leaving financial disclosure to the insurer.

Q7 — Answer: A. Use a multidimensional assessment of her medical risk, environment, and readiness to match her to the least intensive safe setting

Why it is correct: Use multidimensional placement. *Common error:* Defaulting to one fixed program.

- **B** — Under-placement ignores her needs. *Trap:* Undertreating a severe presentation.
- **C** — Preference cannot override clinical need. *Trap:* Letting convenience drive placement.
- **D** — Maximal care ignores placement criteria. *Trap:* Over-placing without assessment.

Q8 — Answer: D. Motivational interviewing to explore her ambivalence and strengthen her own reasons for change at her current stage

Why it is correct: MI fits her ambivalence and stage. *Common error:* Pushing action before readiness.

- **A** — Confrontation raises resistance. *Trap:* Using confrontation to force insight.
- **B** — Action-stage tools precede her readiness. *Trap:* Applying action tools too early.
- **C** — Education alone rarely resolves ambivalence. *Trap:* Relying on information to motivate.

Q9 — Answer: B. Reflect her statement and explore both sides of her ambivalence without arguing against her about how serious it is

Why it is correct: Roll with resistance; explore ambivalence. *Common error:* Arguing her into agreement.

- **A** — Colluding with minimization is unhelpful. *Trap:* Siding with the status quo to please her.
- **C** — Arguing the change side raises resistance. *Trap:* Taking up the change side of the argument.
- **D** — Avoiding the moment wastes an opening. *Trap:* Avoiding the topic to dodge conflict.

Q10 — Answer: C. Frame the return to use as common in recovery, explore what it taught her, and revisit the plan without shaming her

Why it is correct: Treat the lapse as learning. *Common error:* Treating a lapse as total failure.

- **A** — Dismissing the lapse skips learning. *Trap:* Glossing over a teachable moment.
- **B** — One lapse does not mandate escalation. *Trap:* Over-escalating after one slip.
- **D** — Catastrophizing deepens shame. *Trap:* Amplifying shame after a lapse.

Q11 — Answer: A. As high-risk periods for low mood and safety that warrant monitoring and a plan for support during the crash

Why it is correct: Treat crashes as high-risk windows. *Common error:* Ignoring the crash as a risk window.

- **B** — Dismissing the crash misses risk. *Trap:* Overlooking a high-risk period.
- **C** — The crash is withdrawal, not bipolar. *Trap:* Misattributing the crash to bipolar disorder.
- **D** — Optimizing use is not the goal. *Trap:* Coaching safer use instead of addressing risk.

Q12 — Answer: C. Decline to release records without a valid release and explain that substance-use information has heightened confidentiality protections

Why it is correct: SUD records get heightened protection. *Common error:* Releasing SUD records without proper consent.

- **A** — The request should be documented. *Trap:* Failing to document a records request.
- **B** — Releasing without consent breaches confidentiality. *Trap:* Releasing records on request alone.
- **D** — A diagnosis is protected information. *Trap:* Sharing a diagnosis as if harmless.

Q13 — Answer: D. Explain honestly that no medication is approved for stimulant use disorder and coordinate medical evaluation for related concerns

Why it is correct: State the evidence honestly; coordinate care. *Common error:* Implying a medication cure that does not exist.

- **A** — Naming a drug and dose exceeds scope. *Trap:* Overstepping into medication management.
- **B** — Overstating a nonexistent option is dishonest. *Trap:* Promising a treatment that does not exist.
- **C** — Refusing to coordinate abandons the need. *Trap:* Declining to help with a legitimate concern.

Case 4: Minimized suicidal thoughts and an accessible firearm after job loss and separation

Diagnosis: Adjustment Disorder With Depressed Mood

Q1 — Answer: A. Conduct a structured suicide risk assessment covering ideation, intent, plan, and access to means, without accepting his reassurance at face value

Why it is correct: Structured risk assessment comes first, not the reassurance. *Common error:* Letting a client's reassurance substitute for a structured screen.

- **B** — Deferring the assessment to another provider is unsafe. *Trap:* Outsourcing an acute safety assessment.
- **C** — Taking the reassurance at face value skips assessment. *Trap:* Trusting a minimizing statement instead of assessing further.
- **D** — Intervening before assessing is premature. *Trap:* Jumping to coping skills before a risk assessment.

Q2 — Answer: B. Explore the disclosure directly, asking follow-up questions about the firearm's accessibility and his recent thoughts about it, rather than accepting his reassurance

Why it is correct: The buried disclosure must be followed up, not skipped. *Common error:* Hearing the reassurance and missing the buried disclosure of means.

- **A** — Bypassing Scott without exploring the disclosure first is premature. *Trap:* Acting on a third party before assessing with the client.
- **C** — Deferring a means disclosure is unsafe. *Trap:* Sticking rigidly to a planned question order.
- **D** — Ignoring the disclosed means after a reassurance is the core trap. *Trap:* Letting reassuring language override a disclosed lethal means.

Q3 — Answer: C. Immediate access to a highly lethal method, independent of his verbal reassurance, which sharply elevates acute risk

Why it is correct: Accessible, functional means is a major acute risk driver. *Common error:* Weighing his reassurance more heavily than the disclosed means.

- **A** — Depth history is not the priority right now. *Trap:* Prioritizing depth work over an acute safety concern.
- **B** — Quantifying drinks is not the priority the disclosure raises. *Trap:* Substituting a different data point for the means question.

- **D** — Exhaustive ownership history is low yield right now. *Trap:* Collecting low-yield background detail instead of acting on current access.

Q4 — Answer: A. Weigh both his risk factors, such as the accessible firearm and hopelessness, against protective factors like his children, rather than reading either alone

Why it is correct: Balanced formulation weighs risk and protective factors together. *Common error:* Treating the mention of his children as reassuring without weighing the note-rehearsal.

- **B** — Proportionality is not a risk metric. *Trap:* Judging risk by whether distress seems proportionate.
- **C** — Absence of a finished note does not lower acute risk. *Trap:* Treating an unfinished note as evidence that risk is low.
- **D** — A mood scale alone does not formulate acute risk. *Trap:* Substituting a mood scale for a full risk formulation.

Q5 — Answer: D. His increased weekend drinking, which can heighten impulsivity and lower the threshold for acting on suicidal thoughts

Why it is correct: Escalating alcohol use raises acute risk. *Common error:* Ignoring escalating alcohol use in a suicide risk assessment.

- **A** — Cognitive decline is an improbable explanation here. *Trap:* Over-attributing acute distress to cognitive decline.
- **B** — Eating pathology is not indicated. *Trap:* Inventing an unsupported rule-out.
- **C** — Psychosis is not indicated here. *Trap:* Pursuing an unsupported rule-out instead of the indicated one.

Q6 — Answer: A. A collaborative safety plan identifying his warning signs, coping strategies, supports, and crisis contacts, built together with him

Why it is correct: Collaborative safety planning is the evidence-based cornerstone. *Common error:* Relying on a no-suicide contract instead.

- **B** — Activity scheduling alone is insufficient. *Trap:* Substituting mood work for a full safety plan.
- **C** — A single contact rule is not a full plan. *Trap:* Reducing safety planning to one instruction.
- **D** — No-suicide contracts are not evidence-based. *Trap:* Using a contract in place of a real safety plan.

Q7 — Answer: B. Address means restriction for the firearm now, alongside starting the broader safety plan, rather than delaying it to a future session

Why it is correct: Means restriction must happen alongside deeper work, not after it. *Common error:* Letting the client's preference to defer the topic delay urgent means restriction.

- **A** — Leaving the firearm to chance is unsafe sequencing. *Trap:* Hoping a safety topic reappears instead of raising it.
- **C** — An ultimatum is coercive and can rupture engagement. *Trap:* Issuing a rigid ultimatum instead of collaborating.
- **D** — Deferring means restriction leaves lethal access in place. *Trap:* Sequencing deeper exploration before urgent means restriction.

Q8 — Answer: C. Counsel collaboratively on means restriction, such as temporarily storing the firearm with a trusted person away from the home or using a lock

Why it is correct: Collaborative means restriction is the central intervention. *Common error:* Leaving the disclosed lethal means unaddressed in the plan.

- **A** — In-home storage still leaves ready access. *Trap:* Settling for an inadequate in-home storage measure.
- **B** — Deferring entirely abandons means-restriction counseling. *Trap:* Using autonomy as a reason to avoid the conversation.

- **D** — Unilateral confiscation without collaboration is inappropriate. *Trap:* Escalating to police confiscation before working with the client.

Q9 — Answer: B. Recognize the fatigue may be biasing judgment, finish his risk and means-safety work now, then seek consultation and support

Why it is correct: Name the fatigue, keep safety work intact, then get support. *Common error:* Letting exhaustion lower the bar for risk assessment.

- **A** — Delaying means safety leaves a loaded firearm accessible. *Trap:* Letting counselor fatigue postpone urgent safety work.
- **C** — An abrupt mid-session handoff leaves his risk unaddressed. *Trap:* Escaping fatigue by abandoning an at-risk client.
- **D** — Unaddressed compassion fatigue erodes clinical judgment over time. *Trap:* Dismissing self-awareness as irrelevant to care.

Q10 — Answer: C. Explore what his service, job loss, and separation mean to him, and fold his views and any fitting resources into the plan

Why it is correct: Context integrated as he defines it makes the plan fit. *Common error:* Planning from assumptions instead of the client's own meanings.

- **A** — Assumptions about veterans are stereotypes, not assessment. *Trap:* Building a treatment plan on a group stereotype.
- **B** — Service history may shape his identity, coping, and resources. *Trap:* Dismissing context the client may see as central.
- **D** — Context informs the plan alongside safety work, not after it. *Trap:* Treating context and safety as sequential steps.

Q11 — Answer: D. Validate how overwhelming the losses feel right now while helping him see the current crisis and his options as changeable over time

Why it is correct: Validating pain while framing the crisis as changeable supports hope. *Common error:* Either only sympathizing or only reassuring him.

- **A** — Redirecting away from despair leaves it unaddressed. *Trap:* Avoiding emotionally central material with a task list.
- **B** — Endorsing hopelessness reinforces the despair driving the crisis. *Trap:* Adopting the client's hopeless stance.
- **C** — Overpromising outcomes can ring hollow and backfire. *Trap:* Making promises about outcomes you cannot control.

Q12 — Answer: D. Switch to unique session links or a locked waiting room so only Scott can join, as HIPAA access safeguards require

Why it is correct: Compliant use includes configuring the platform to block unauthorized access. *Common error:* Assuming a signed BAA alone makes platform use compliant.

- **A** — A BAA does not configure access controls. *Trap:* Treating a signed BAA as full compliance.
- **B** — Camera-off does not stop another client from hearing him. *Trap:* Shifting privacy protection onto the client.
- **C** — Public links invite unauthorized entry to sessions. *Trap:* Ignoring access control because a link seems harmless.

Q13 — Answer: D. Document the risk assessment findings, the means-restriction steps agreed to, the safety plan, and the clinical rationale for the disposition

Why it is correct: Thorough documentation captures assessment, plan, and rationale. *Common error:* Documenting only that he denied a plan.

- **A** — Omitting the assessment from the record is improper. *Trap:* Omitting essential clinical information from the chart.

- **B** — A promise-only note is inadequate and misleading. *Trap:* Substituting a promise note for real documentation.
- **C** — Minimizing the record is unsafe and inaccurate. *Trap:* Understating risk in the clinical record.

Case 5: Years of mood ups and downs that never reach full episodes

Diagnosis: Cyclothymic Disorder

Q1 — Answer: A. Two or more years of subthreshold mood swings with no symptom-free stretch longer than about two months

Why it is correct: Two years of subthreshold swings, no long remission. *Common error:* Diagnosing cyclothymia without confirming the duration and persistence.

- **B** — Full-episode count would point elsewhere. *Trap:* Applying full-episode criteria here.
- **C** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **D** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: A. Check the urge against Priya's actual reports, and assess whether trauma exposure is shifting the counselor's sense of danger

Why it is correct: Vicarious trauma can shift a counselor's sense of danger; client data is the check. *Common error:* Mistaking a trauma-shaped worldview for clinical intuition.

- **B** — Acting on the bias imposes unneeded intensity on her. *Trap:* Trusting a distorted reaction as clinical insight.
- **C** — Rest alone skips assessing the trauma exposure itself. *Trap:* Dismissing a vicarious trauma sign as ordinary fatigue.

- **D** — Disclosure burdens the client with the counselor's needs. *Trap:* Using self-disclosure in place of self-assessment.

Q3 — Answer: B. Bill the cyclothymic disorder diagnosis her assessment supports, with notes documenting its criteria and impairment

Why it is correct: Claims must match the accurate, documented diagnosis. *Common error:* Choosing a code for coverage rather than accuracy.

- **A** — Billing an unmet diagnosis misrepresents the claim. *Trap:* Justifying upcoding by a possible future course.
- **C** — A vaguer code still misstates what was assessed. *Trap:* Obscuring the diagnosis instead of documenting it.
- **D** — Documentation must support the claim before it is submitted. *Trap:* Treating notes as needed only for audits.

Q4 — Answer: C. Compare self-appraisal with outcome data and supervisor or peer feedback from past cases of similar mood disorders

Why it is correct: External data checks the biases of self-assessment. *Common error:* Trusting confidence or tenure as evidence of competence.

- **A** — Tenure alone does not prove skill with this disorder. *Trap:* Equating years of practice with specific competence.
- **B** — Comfort is a poor proxy for actual clinical skill. *Trap:* Mistaking felt confidence for demonstrated effectiveness.
- **D** — Clients cannot judge clinical competence at intake. *Trap:* Shifting the counselor's self-assessment onto the client.

Q5 — Answer: B. Suicide risk and any escalation toward a full mood episode, monitoring across both the high and low periods

Why it is correct: Screen risk and watch for escalation. *Common error:* Tracking mood without screening risk and escalation.

- **A** — Psychosis is not indicated. *Trap:* Pursuing an unsupported rule-out.
- **C** — Eating pathology is not indicated. *Trap:* Inventing an unsupported rule-out.
- **D** — Cognitive decline is improbable here. *Trap:* Pursuing an improbable rule-out.

Q6 — Answer: D. When the counselor reviews shared entries, that they are not monitored live, and where to turn in a crisis

Why it is correct: Shared app data is not live monitoring. *Common error:* Implying passive data sharing provides real-time safety.

- **A** — Promising daily watch sets an unsafe expectation. *Trap:* Overpromising monitoring a counselor cannot sustain.
- **B** — Personal texting bypasses secure, documented channels. *Trap:* Trading record security for a little convenience.
- **C** — Each new tool needs its own informed consent discussion. *Trap:* Assuming blanket intake consent covers new tools.

Q7 — Answer: B. Consistent sleep, activity, and daily routines, since rhythm disruption can amplify her mood swings

Why it is correct: Stabilize sleep and daily rhythms. *Common error:* Overlooking the role of sleep and routine in mood stability.

- **A** — Diet-only misses the key target. *Trap:* Narrowing to one factor and missing the main target.
- **C** — Removing routine destabilizes mood. *Trap:* Advising the opposite of what stabilizes mood.

- **D** — Inconsistent, pole-driven routines do not stabilize. *Trap:* Building a routine that tracks the mood swings.

Q8 — Answer: B. Use a daily mood-and-routine chart with periodic measures to monitor cycling, function, and risk over time

Why it is correct: Mood-and-routine charting plus measures. *Common error:* Proceeding without systematic mood and routine tracking.

- **A** — Anecdote alone is insufficient. *Trap:* Substituting anecdote for measurement.
- **C** — Single-day snapshots miss the cycling. *Trap:* Measuring isolated snapshots.
- **D** — End-only review misses steering data. *Trap:* Deferring all measurement to the end.

Q9 — Answer: B. Coordinate a referral to a prescriber for evaluation while continuing the rhythm-focused psychotherapy within scope

Why it is correct: Refer for prescribing, keep doing therapy. *Common error:* Advising on specific medication outside scope.

- **A** — Blanket refusal advice overreaches. *Trap:* Directing a medical decision.
- **C** — Recommending a drug and dose exceeds scope. *Trap:* Making a prescribing recommendation.
- **D** — Dismissing the question is unhelpful. *Trap:* Failing to address a legitimate question.

Q10 — Answer: A. Explore the wish nonjudgmentally and use psychoeducation to connect the highs to the broader cycling pattern

Why it is correct: Explore and psychoeducate about the pattern. *Common error:* Either simply agreeing she is done or dismissing how good she feels.

- **B** — Ignoring it misses key material. *Trap:* Sidestepping a meaningful moment.

- **C** — Fear-based pressure undermines the alliance. *Trap:* Using fear rather than collaborative exploration.
- **D** — Ending in a high ignores the cycling. *Trap:* Ending treatment based on a transient high.

Q11 — Answer: D. Normalize the long-term nature of the work and anchor it in goals she values across both her high and low periods

Why it is correct: Normalize the course and anchor in values. *Common error:* Implying a quick fix that sets her up for discouragement.

- **A** — Conditioning care is inappropriate. *Trap:* Making counseling contingent on medication.
- **B** — Promising a quick cure is misleading. *Trap:* Setting an unrealistic expectation.
- **C** — Cataloging shifts is not the work. *Trap:* Centering sessions on enumerating shifts.

Q12 — Answer: B. Discuss confidentiality and the limits of any release, sharing only what she consents to and what is needed

Why it is correct: Honor consent and minimum-necessary disclosure. *Common error:* Disclosing more than she consents to or than is necessary.

- **A** — Flat refusal forecloses a workable option. *Trap:* Declining without exploring an appropriate disclosure.
- **C** — Sending everything ignores consent. *Trap:* Over-disclosing without consent.
- **D** — Falsifying documentation is unethical. *Trap:* Misrepresenting information in records.

Q13 — Answer: D. Seek consultation and reassess against the criteria, coordinating referral if the picture exceeds your competence

Why it is correct: Consult and reassess within competence. *Common error:* Settling diagnostic uncertainty alone without consultation.

- **A** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.
- **B** — Dismissing the distinction misleads care. *Trap:* Minimizing a clinically meaningful distinction.
- **C** — Refusing to reassess is poor practice. *Trap:* Treating the diagnosis as immutable.

Case 6: Psychosis that persists between mood episodes

Diagnosis: Schizoaffective Disorder, Bipolar Type

Q1 — Answer: B. At least two weeks of psychosis without a mood episode, with mood episodes present for most of the illness

Why it is correct: Psychosis without mood plus mood majority. *Common error:* Diagnosing without confirming both timing criteria.

- **A** — Cognitive count does not define it. *Trap:* Applying neurocognitive criteria here.
- **C** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **D** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: A. Take a few minutes for a grounding routine, such as paced breathing, before greeting the next client calmly

Why it is correct: Brief regulation keeps one session from spilling into the next. *Common error:* Carrying unprocessed tension into the next session.

- **B** — Unregulated tension may leak into the next client's care. *Trap:* Assuming stress resolves without deliberate regulation.

- **C** — Hallway venting risks confidentiality and does not regulate. *Trap:* Confusing venting in a public space with self-regulation.
- **D** — Avoiding him disrupts care soon after his hospitalization. *Trap:* Managing the counselor's discomfort at his expense.

Q3 — Answer: B. Treat it as a reasonable disability accommodation and arrange a quieter place to wait, at no added cost to him

Why it is correct: Psychiatric disabilities also warrant reasonable accommodation. *Common error:* Limiting disability accommodations to physical conditions.

- **A** — The ADA covers psychiatric as well as physical disabilities. *Trap:* Excluding psychiatric disabilities from the duty to accommodate.
- **C** — Demanding proof of a known need delays a simple change. *Trap:* Adding needless documentation barriers to access.
- **D** — Accommodation costs cannot be passed on to him. *Trap:* Charging a surcharge for a needed accommodation.

Q4 — Answer: C. Judge that the brief contact poses no conflict, acknowledge it openly with him, and document the review

Why it is correct: A single past transaction rarely creates a conflict. *Common error:* Treating any prior contact as automatically disqualifying.

- **A** — Abrupt referral disrupts care without a real conflict. *Trap:* Overreacting to a trivial prior contact.
- **B** — Concealment risks trust if he later recognizes the counselor. *Trap:* Hiding relevant facts from a guarded client.
- **D** — Suitability rests on the counselor's own professional judgment. *Trap:* Handing an ethical decision to others.

Q5 — Answer: D. Conduct a structured suicide-risk assessment of current ideation, intent, plan, and protective factors now

Why it is correct: Structured suicide-risk assessment now. *Common error:* Failing to assess current suicide risk.

- **A** — Deferring assessment is unsafe. *Trap:* Postponing an urgent assessment.
- **B** — Assuming risk passed is unsafe. *Trap:* Dismissing risk on reassurance.
- **C** — Deferring safety to address housing first. *Trap:* Sequencing safety behind other needs.

Q6 — Answer: C. Coordinate combined pharmacotherapy and psychosocial support within a treatment team and your counseling role

Why it is correct: Combined meds and psychosocial care. *Common error:* Offering counseling alone for schizoaffective disorder.

- **A** — Prescribing is outside counselor scope. *Trap:* Acting outside scope of practice.
- **B** — Insight-only is not first-line. *Trap:* Prioritizing insight over indicated care.
- **D** — Talks alone are insufficient. *Trap:* Omitting medication coordination.

Q7 — Answer: A. Collaborative work on engagement and adherence, addressing the barriers that interrupt his medication and care

Why it is correct: Engagement and adherence support. *Common error:* Ignoring the adherence and engagement barriers.

- **B** — Ultimatums rupture engagement. *Trap:* Making care contingent on adherence.
- **C** — Never raising adherence neglects a key issue. *Trap:* Avoiding an important treatment target.
- **D** — Encouraging stop-start worsens course. *Trap:* Recommending an unsafe medication pattern.

Q8 — Answer: A. Coordinate case management and community resources for housing as part of recovery-oriented, wraparound support

Why it is correct: Coordinate housing and community resources. *Common error:* Treating symptoms while ignoring housing instability.

- **B** — Financial entanglement crosses boundaries. *Trap:* Crossing financial boundaries with a client.
- **C** — Housing is part of recovery. *Trap:* Ignoring a key recovery factor.
- **D** — Withholding care is inappropriate. *Trap:* Setting stability as a precondition for treatment.

Q9 — Answer: C. Obtain his informed consent and a release, then collaborate with the team and share what is needed for his care

Why it is correct: Consent and release before coordinating. *Common error:* Coordinating before obtaining consent.

- **A** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.
- **B** — Coordinating without consent breaches confidentiality. *Trap:* Disclosing before consent is documented.
- **D** — No coordination undercuts team care. *Trap:* Refusing helpful, consented coordination.

Q10 — Answer: A. Acknowledge how distressing the voices are and stay supportive, without confirming or arguing about their reality

Why it is correct: Validate distress, stay neutral on content. *Common error:* Either agreeing the voices are real or flatly arguing they are not.

- **B** — Total avoidance misses an opening. *Trap:* Sidestepping the relevant material.
- **C** — Colluding reinforces the symptom. *Trap:* Validating the hallucination to keep rapport.

- **D** — Confronting the psychosis ruptures the alliance. *Trap:* Disputing the hallucination to manage it.

Q11 — Answer: C. Move at his pace, be transparent and consistent, and work toward concrete goals he chooses and values

Why it is correct: Pace, transparency, and shared goals. *Common error:* Pushing for disclosure faster than the alliance allows.

- **A** — Pressured disclosure ruptures trust. *Trap:* Demanding disclosure too fast.
- **B** — Conditioning care is inappropriate. *Trap:* Making counseling contingent on medication.
- **D** — Dwelling on content can reinforce it. *Trap:* Centering the work on psychotic content.

Q12 — Answer: D. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

Why it is correct: Assess and act on duty to protect. *Common error:* Treating an identifiable threat as fully confidential.

- **A** — Act on assessed risk, not reflexively. *Trap:* Disclosing before assessing the risk.
- **B** — Delaying assessment is unsafe. *Trap:* Postponing an urgent risk assessment.
- **C** — Confidentiality yields to a serious threat. *Trap:* Treating confidentiality as absolute when there is danger.

Q13 — Answer: A. Seek consultation and work within a treatment team, referring for the components beyond your competence

Why it is correct: Consult, coordinate, or refer within competence. *Common error:* Managing serious psychotic illness alone without competence.

- **B** — Minimizing the illness fails the client. *Trap:* Underestimating the need for team care.

- **C** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.
- **D** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.

Case 7: Childhood motor and vocal tics with social and school impact

Diagnosis: Tourette's Disorder

Q1 — Answer: B. Multiple motor tics and at least one vocal tic present for more than a year with childhood onset

Why it is correct: Multiple motor plus a vocal tic over a year. *Common error:* Diagnosing without confirming both motor and vocal tics over the required duration.

- **A** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.
- **C** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **D** — Depressive count does not define it. *Trap:* Applying depressive criteria here.

Q2 — Answer: D. Have them upload clips to the practice's secure portal and keep them in his record, limited to his care team

Why it is correct: Client-made videos are records needing secure, limited storage. *Common error:* Handling client media outside the secure record.

- **A** — Personal phones are unsecured, and deleting bypasses the record. *Trap:* Treating clinical media as casual texts.
- **B** — Unlisted links can be forwarded and are not secured. *Trap:* Mistaking an unlisted link for real privacy.
- **C** — Access should be limited to those involved in his care. *Trap:* Giving staff broader access than needed.

Q3 — Answer: A. Review the manual and scoring rules, and co-rate a session with a trained colleague to confirm reliable use of the scale

Why it is correct: Competence must be current; refresh and verify skill before relying on scores. *Common error:* Assuming past training guarantees present competence.

- **B** — Skill with an instrument fades without use. *Trap:* Treating assessment competence as permanent.
- **C** — A clinician-rated scale is not valid as a parent report. *Trap:* Changing an instrument's administration format.
- **D** — Standardized ratings anchor his tracking over time. *Trap:* Abandoning measurement to avoid a skill gap.

Q4 — Answer: B. Consult legal counsel, inform his parents, and release only what the order specifies, seeking any protective handling

Why it is correct: Court orders require compliance limited to their scope. *Common error:* Treating a court order as all-or-nothing disclosure.

- **A** — Ignoring a valid order risks contempt and sanctions. *Trap:* Placing confidentiality above a lawful court order.
- **C** — Disclosure must stay within what the order specifies. *Trap:* Over-disclosing beyond the order's stated scope.
- **D** — A minor's assent does not govern a court order. *Trap:* Making a child's permission the legal deciding factor.

Q5 — Answer: B. The emotional impact on his self-esteem and mood, with developmentally appropriate screening for safety

Why it is correct: Assess self-esteem, mood, and safety. *Common error:* Addressing the tics while ignoring the emotional and self-esteem impact.

- **A** — Psychosis is not indicated. *Trap:* Pursuing an unsupported rule-out.
- **C** — Cognitive decline is implausible in a child. *Trap:* Pursuing an implausible rule-out.

- **D** — Eating pathology is not indicated. *Trap:* Inventing an unsupported rule-out.

Q6 — Answer: C. Accept it warmly and explore its meaning with him, since a small handmade token from a child fits boundaries

Why it is correct: Small handmade gifts from children can be appropriately accepted. *Common error:* Applying a rigid no-gift rule to a child.

- **A** — Refusal can shame a child and ignores gift context. *Trap:* Treating every gift as a boundary violation.
- **B** — Reciprocal gifts blur the professional relationship. *Trap:* Turning a token into a social exchange.
- **D** — Displaying his name reveals that he is a client. *Trap:* Breaching confidentiality while trying to affirm him.

Q7 — Answer: A. Provide psychoeducation and coordinate school supports and anti-bullying measures to protect his participation

Why it is correct: Psychoeducation and school supports. *Common error:* Treating the tics while leaving the school environment unaddressed.

- **B** — Ignoring the environment narrows the plan. *Trap:* Omitting a key part of the plan.
- **C** — Withdrawal from school is harmful. *Trap:* Endorsing harmful avoidance.
- **D** — Telling him to ignore it is insufficient. *Trap:* Minimizing the impact of the teasing.

Q8 — Answer: D. Track tic frequency, distress, school participation, and self-esteem over time to guide the plan

Why it is correct: Measurement-based tracking of tics and function. *Common error:* Proceeding without tracking the relevant outcomes.

- **A** — Tic count alone is too narrow. *Trap:* Reducing monitoring to one number.

- **B** — Anecdote alone is insufficient. *Trap:* Substituting anecdote for measurement.
- **C** — End-only review misses steering data. *Trap:* Deferring all measurement to the end.

Q9 — Answer: D. With the parents' consent and a release, coordinate school supports and any medical care for the tics

Why it is correct: Consent and release before coordinating. *Common error:* Contacting the school or providers without parental consent.

- **A** — Disclosing without consent breaches confidentiality. *Trap:* Disclosing without consent.
- **B** — Blanket sharing over-discloses. *Trap:* Releasing more than is appropriate or consented.
- **C** — No coordination undercuts support. *Trap:* Refusing helpful, consented coordination.

Q10 — Answer: C. Validate his feelings, provide age-appropriate psychoeducation, and build self-acceptance and coping

Why it is correct: Validate, psychoeducate, and build self-acceptance. *Common error:* Either minimizing his feelings or reinforcing that the tics make him abnormal.

- **A** — Avoiding the feeling misses the work. *Trap:* Sidestepping a meaningful moment.
- **B** — Blaming effort is harmful and inaccurate. *Trap:* Implying the tics reflect insufficient effort.
- **D** — Reinforcing otherness is unhelpful. *Trap:* Reinforcing a sense of being abnormal.

Q11 — Answer: A. Use developmentally appropriate, strengths-based methods that build coping and confidence at his level

Why it is correct: Developmentally matched, strengths-based engagement. *Common error:* Using abstract, adult-oriented methods that a child cannot engage with.

- **B** — Demanding tic cessation first stalls engagement. *Trap:* Setting tic cessation as a precondition.
- **C** — Abstract methods do not fit a child. *Trap:* Pitching the work above his developmental level.
- **D** — Cataloging tics is not the work. *Trap:* Centering sessions on enumerating tics.

Q12 — Answer: C. Decline to misrepresent his status and clarify what the assessment and your role can accurately support

Why it is correct: Do not misrepresent; document accurately. *Common error:* Writing an inaccurate statement to satisfy the parents.

- **A** — Falsifying a statement is unethical. *Trap:* Misrepresenting the child's status.
- **B** — Flat refusal forecloses helpful options. *Trap:* Declining without clarifying the appropriate options.
- **D** — Advocacy over accuracy is improper. *Trap:* Letting advocacy override accuracy.

Q13 — Answer: A. Seek consultation and training, and refer or coordinate for the components beyond your competence

Why it is correct: Consult, train, or refer within competence. *Common error:* Delivering a specialized intervention without the required competence.

- **B** — Abrupt transfer mishandles continuity. *Trap:* Dropping the family without coordination.
- **C** — Minimizing the need misleads care. *Trap:* Underestimating the need for specialized care.
- **D** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.

Case 8: Months of broken sleep and exhausting days

Diagnosis: Insomnia Disorder, Persistent

Q1 — Answer: D. Difficulty with sleep at least three nights a week for over three months, with daytime impairment despite adequate opportunity

Why it is correct: Frequency, duration, opportunity, and impairment. *Common error:* Diagnosing chronic insomnia from a brief sleepless stretch.

- **A** — Inadequate opportunity argues against the diagnosis. *Trap:* Diagnosing insomnia when opportunity is inadequate.
- **B** — A few bad nights is not chronic insomnia. *Trap:* Confusing a brief change with the criteria.
- **C** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: D. Revise the note so it records only what occurred in this session, removing carried-over text that no longer applies

Why it is correct: Each note must reflect that session's actual content. *Common error:* Signing cloned text that misstates the care given.

- **A** — Copied-forward text can misrepresent what actually happened. *Trap:* Trusting EHR convenience features over accuracy.
- **B** — Paper side files fragment the official record. *Trap:* Keeping shadow records outside the EHR.
- **C** — Shared logins break authorship and access controls. *Trap:* Letting others document under the counselor's credentials.

Q3 — Answer: A. Reflect on what drove the push, such as a need to show quick progress, and plan to balance structure with listening next time

Why it is correct: Reflection turns a misstep into a plan for better attunement. *Common error:* Blaming the client instead of examining the counselor's own role.

- **B** — Labeling him resistant ignores the counselor's part. *Trap:* Externalizing the problem onto the client.
- **C** — Lengthy disclosure centers the counselor's feelings. *Trap:* Confusing self-focus with self-reflection.
- **D** — Reflection shapes how future sessions go. *Trap:* Dismissing review of past work as pointless.

Q4 — Answer: B. Review its evidence, privacy terms, and crisis limits, and frame it as a supplement to review in session, not a substitute

Why it is correct: AI tools need vetting and clinical oversight before use in care. *Common error:* Assuming an AI tool is held to clinical standards.

- **A** — AI chatbots are not licensed or bound by ethics codes. *Trap:* Assuming an AI tool carries clinical accountability.
- **C** — Setting a sleep window needs clinical judgment and safety checks. *Trap:* Delegating a clinical decision to an AI tool.
- **D** — No federal law bars this, though some states limit AI therapy tools. *Trap:* Banning a tool instead of evaluating it.

Q5 — Answer: D. A daily sleep diary tracking bedtime, sleep onset, awakenings, rise time, and daytime functioning over a couple of weeks

Why it is correct: Use a sleep diary baseline. *Common error:* Planning treatment without a sleep baseline.

- **A** — Ordering a sleep study exceeds scope. *Trap:* Acting outside scope of practice.
- **B** — Early-history detail is low yield here. *Trap:* Prioritizing origins over current data.
- **C** — A global estimate is imprecise. *Trap:* Relying on a vague global rating.

Q6 — Answer: C. Consult a colleague experienced in CBT-I on setting his sleep window and safety cautions such as daytime drowsy driving

Why it is correct: Targeted consultation builds skill in a specific technique. *Common error:* Improvising an unfamiliar protocol without expert input.

- **A** — Trial and error risks harm from excess sleepiness. *Trap:* Learning a technique on the client.
- **B** — Sleep hygiene alone is less effective than CBT-I. *Trap:* Downgrading treatment to avoid a skill gap.
- **D** — A narrow skill gap calls for consultation, not referral. *Trap:* Referring out when consultation would suffice.

Q7 — Answer: D. Stimulus control and sleep restriction, which strengthen the bed-sleep association and consolidate his time asleep

Why it is correct: Stimulus control and sleep restriction. *Common error:* Leaving extended time-in-bed in place.

- **A** — Long naps undermine night sleep. *Trap:* Encouraging compensatory napping.
- **B** — More time in bed worsens insomnia. *Trap:* Increasing time in bed.
- **C** — Lying awake in bed reinforces the cycle. *Trap:* Reinforcing the maintaining behavior.

Q8 — Answer: B. Recommend CBT-I as first-line and, if he wants to consider medication, coordinate a referral to a prescriber with his consent

Why it is correct: Lead with CBT-I; refer for medication questions. *Common error:* Giving specific medication advice.

- **A** — Endorsing indefinite sleep aids is inappropriate. *Trap:* Advising long-term reliance on sleep aids.
- **C** — Naming a drug and dose exceeds scope. *Trap:* Overstepping professional limits.

- **D** — Discouraging meds overreaches. *Trap:* Steering a medical decision.

Q9 — Answer: C. Review the ongoing sleep diary at intervals to track sleep efficiency and daytime functioning and adjust the plan

Why it is correct: Use the sleep diary for measurement-based care. *Common error:* Relying on impressions alone.

- **A** — Attendance is not an outcome measure. *Trap:* Mistaking adherence for outcome.
- **B** — Global impressions are unreliable. *Trap:* Trusting unstandardized self-report.
- **D** — End-only review misses non-response. *Trap:* Skipping interim monitoring.

Q10 — Answer: A. Address the catastrophic sleep beliefs with cognitive work, since such beliefs heighten arousal and maintain the insomnia

Why it is correct: Target catastrophic sleep beliefs. *Common error:* Leaving the maintaining beliefs unaddressed.

- **B** — Lecturing rarely shifts the belief. *Trap:* Substituting facts for cognitive work.
- **C** — Endorsing the belief feeds arousal. *Trap:* Confirming the maintaining belief.
- **D** — Reassurance feeds the sleep-effort cycle. *Trap:* Providing reassurance that maintains arousal.

Q11 — Answer: B. Explain how clock-watching, daytime napping, and effortful sleep maintain insomnia and how CBT-I targets that cycle

Why it is correct: Teach the insomnia-maintaining cycle. *Common error:* Teaching techniques without the rationale.

- **A** — A heavy reading load is not the priority. *Trap:* Overloading instead of explaining.
- **C** — Dismissive reassurance forgoes treatment. *Trap:* Reassuring instead of treating.

- **D** — Not-thinking-about-it is unhelpful advice. *Trap:* Giving platitudes instead of education.

Q12 — Answer: D. Address the safety risk of using another person's prescription and document accurately rather than omitting it from the record

Why it is correct: Address the safety risk; document honestly. *Common error:* Agreeing to leave a safety-relevant fact out of the record.

- **A** — Omitting a safety fact falsifies the record. *Trap:* Falsifying records to reassure.
- **B** — Reflexive reporting is disproportionate here. *Trap:* Escalating before addressing it clinically.
- **C** — Ignoring the risk is poor care. *Trap:* Sidestepping a relevant safety issue.

Q13 — Answer: B. Decline to disclose without a valid release and clarify the limits of what a treating counselor can appropriately attest to

Why it is correct: Protect confidentiality; clarify role limits. *Common error:* Disclosing or issuing a fitness opinion without consent or competence.

- **A** — A general summary is still a disclosure. *Trap:* Leaking details under the guise of helping.
- **C** — Disclosing without consent breaches confidentiality. *Trap:* Issuing an unconsented, out-of-role opinion.
- **D** — Abrupt refusal misses a teachable moment. *Trap:* Being needlessly obstructive.

Case 9: Persistent erectile difficulty straining a relationship

Diagnosis: Erectile Disorder

Q1 — Answer: A. Difficulty obtaining or maintaining an erection on most occasions for at least six months, causing distress

Why it is correct: Persistent difficulty over six months with distress. *Common error:* Diagnosing without confirming the persistence, frequency, and distress.

- **B** — Depressive count does not define it. *Trap:* Applying depressive criteria here.
- **C** — Recent change is not the criterion. *Trap:* Confusing a brief change with the criteria.
- **D** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: D. Follow the practice's lockdown plan: lock the door, turn off the lights, and stay with Elijah away from windows

Why it is correct: An announced lockdown activates the practice's emergency plan. *Common error:* Improvising instead of following the rehearsed plan.

- **A** — Fleeing during a lockdown may move toward danger. *Trap:* Overriding the announced plan with instinct.
- **B** — Ignoring the alert leaves both people unprotected. *Trap:* Prioritizing the session over the immediate safety of both.
- **C** — Investigating alone exposes the counselor to the threat. *Trap:* Verifying the threat personally before following the plan.

Q3 — Answer: A. Explain the recording's purpose, who will hear it, how it is stored, and his right to decline, then get his written consent

Why it is correct: Policy and ACA B.6.c require informed, documented permission to record. *Common error:* Treating consultation recordings as exempt from consent.

- **B** — Covert recording violates the policy and his rights. *Trap:* Exempting professional purposes from consent.
- **C** — The policy requires written consent from adults too. *Trap:* Substituting a verbal okay for the required form.

- **D** — Personal email is not a secure channel for recordings. *Trap:* Assuming a consultant's duty secures the transfer.

Q4 — Answer: B. Release the notes only with Elijah's specific written authorization, which the insurer may not require as a condition of payment

Why it is correct: Psychotherapy notes need specific authorization, and payment cannot hinge on it. *Common error:* Applying the payment exception to psychotherapy notes.

- **A** — The payment exception excludes psychotherapy notes. *Trap:* Treating psychotherapy notes like the general record.
- **C** — Information needed for payment may be shared under HIPAA. *Trap:* Overcorrecting into refusing all payment disclosures.
- **D** — Only the client can authorize release of these notes. *Trap:* Substituting a third-party pledge for authorization.

Q5 — Answer: D. Coordinate with his medical provider so the physical evaluation and psychological care proceed together

Why it is correct: Coordinate medical and psychological care. *Common error:* Providing psychological care in isolation from the medical evaluation.

- **A** — Recommending a drug and dose exceeds scope. *Trap:* Making a prescribing recommendation.
- **B** — Cognitive decline is improbable here. *Trap:* Pursuing an improbable rule-out.
- **C** — Psychosis is not indicated. *Trap:* Pursuing an unsupported rule-out.

Q6 — Answer: C. Confirm he is parked, not driving, and able to speak privately, and verify his location before discussing sensitive topics

Why it is correct: Confirm a safe, private setting and location first. *Common error:* Starting sensitive work without checking the client's setting.

- **A** — Coworkers nearby may compromise his privacy. *Trap:* Assuming privacy instead of confirming it.
- **B** — Driving during a session is unsafe and distracting. *Trap:* Allowing therapy while the client drives.
- **D** — A parked, private car can be a workable setting. *Trap:* Replacing assessment with a blanket rule.

Q7 — Answer: A. Use anxiety-reduction and sensate-focus-style approaches that reduce performance pressure and rebuild intimacy

Why it is correct: Reduce performance pressure and rebuild intimacy. *Common error:* Increasing the focus on performance, which worsens the anxiety.

- **B** — Avoiding the anxiety misses the work. *Trap:* Sidestepping the central issue.
- **C** — Endorsing avoidance maintains the problem. *Trap:* Reinforcing avoidance.
- **D** — Increasing performance focus worsens it. *Trap:* Amplifying the performance pressure.

Q8 — Answer: D. Track sexual function, performance anxiety, relationship satisfaction, and distress over time to guide the plan

Why it is correct: Measurement-based tracking of function and relationship. *Common error:* Proceeding without tracking the relevant outcomes.

- **A** — End-only review misses steering data. *Trap:* Deferring all measurement to the end.
- **B** — Anecdote alone is insufficient. *Trap:* Substituting anecdote for measurement.
- **C** — A single performance metric is too narrow. *Trap:* Reducing monitoring to one outcome.

Q9 — Answer: D. With his consent, consider couple-based work on communication and intimacy with appropriate agreements

Why it is correct: Couple work with consent and clear agreements. *Common error:* Involving the partner without consent or role clarity.

- **A** — Excluding the relationship narrows the plan. *Trap:* Leaving the relationship out.
- **B** — Blanket refusal ignores his wishes. *Trap:* Ruling out a consented, indicated option.
- **C** — Involving the partner without consent breaches confidentiality. *Trap:* Disregarding consent and confidentiality.

Q10 — Answer: B. Respond with a calm, nonjudgmental, and normalizing stance that makes it safe for him to talk about it

Why it is correct: Calm, nonjudgmental, normalizing stance. *Common error:* Reacting awkwardly or pushing him to disclose before he feels safe.

- **A** — Rushing past it misses the work. *Trap:* Rushing past a sensitive topic.
- **C** — Joking is invalidating and unprofessional. *Trap:* Undermining a sensitive disclosure with humor.
- **D** — Demanding detail before safety is counterproductive. *Trap:* Demanding disclosure too fast.

Q11 — Answer: C. Frame the difficulty as common and treatable, anchor the work in his goals, and reinforce his motivation

Why it is correct: Normalize, anchor in goals, reinforce motivation. *Common error:* Framing the difficulty as hopeless or shameful.

- **A** — Cataloging failures is not the work. *Trap:* Centering sessions on failures.

- **B** — Endorsing hopelessness is harmful. *Trap:* Communicating hopelessness.
- **D** — Conditioning care is inappropriate. *Trap:* Making counseling contingent on medication.

Q12 — Answer: C. Clarify confidentiality and the limits of any release, sharing only what he consents to and what is needed

Why it is correct: Honor consent and minimum-necessary disclosure. *Common error:* Disclosing more than he consents to or than is necessary.

- **A** — Sending everything ignores consent. *Trap:* Over-disclosing without consent.
- **B** — Falsifying records is unethical. *Trap:* Falsifying documentation.
- **D** — Avoiding the issue is unhelpful. *Trap:* Sidestepping a needed conversation.

Q13 — Answer: C. Seek consultation and training, and coordinate or refer for the components beyond your competence

Why it is correct: Consult, train, or refer within competence. *Common error:* Providing specialized psychosexual care without the required competence.

- **A** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.
- **B** — Abrupt transfer mishandles continuity. *Trap:* Dropping the client without coordination.
- **D** — Minimizing the problem fails the client. *Trap:* Underestimating the need for specialized care.

Case 10: A defiant, irritable nine-year-old at home and school

Diagnosis: Oppositional Defiant Disorder

Q1 — Answer: B. A 6-month pattern of angry/irritable mood and defiant, vindictive behavior occurring with more than one person or setting

Why it is correct: Six-month pattern across settings. *Common error:* Diagnosing from a brief or single-setting conflict.

- **A** — Recent change is not the criterion. *Trap:* Confusing a recent uptick with the criteria.
- **C** — Single-setting-only argues against the diagnosis. *Trap:* Requiring the behavior to be limited to one setting.
- **D** — A precipitant is not the criterion. *Trap:* Using a trigger as the diagnostic test.

Q2 — Answer: D. Verify Eli's eligibility, benefits, and any prior authorization, and list him as patient with his mother as subscriber

Why it is correct: Verify coverage and identify patient and subscriber correctly. *Common error:* Billing before confirming the child's benefits.

- **A** — Eli, not his mother, is the identified patient. *Trap:* Confusing the subscriber with the patient.
- **B** — Routine undisclosed copay waivers can breach payer contracts. *Trap:* Routinely waiving cost-sharing as a courtesy to families.
- **C** — Skipping verification risks denials and surprise bills. *Trap:* Checking coverage only after a denial.

Q3 — Answer: B. ADHD can co-occur and should be assessed separately, since defiance differs from inattentive and hyperactive-impulsive symptoms

Why it is correct: Assess ADHD as a separable comorbidity. *Common error:* Assuming the defiance is simply ADHD.

- **A** — Severity trend is not the question. *Trap:* Reframing the question as a severity trend.
- **C** — Calm moments do not rule out ADHD. *Trap:* Ruling out ADHD on insufficient grounds.
- **D** — Collapsing ODD into ADHD is an error. *Trap:* Treating defiance as identical to ADHD.

Q4 — Answer: A. Troubleshoot the practice's compliant platform or reschedule, since pandemic enforcement leniency for consumer apps ended

Why it is correct: Stay on a compliant platform; OCR's pandemic enforcement discretion ended in 2023. *Common error:* Treating convenience or consent as compliance.

- **B** — Consent does not waive the practice's HIPAA obligations. *Trap:* Using parental consent to bypass compliance.
- **C** — Compliance also requires a business associate agreement. *Trap:* Equating encryption with full HIPAA compliance.
- **D** — No looser federal telehealth standard exists for minors. *Trap:* Assuming child sessions need less protection.

Q5 — Answer: B. A note describing how their escalating conflict complicated the session and what the counselor did to manage it

Why it is correct: Document the specific factor that justified the add-on code. *Common error:* Billing an add-on without describing its basis.

- **A** — A vague label does not establish the complicating factor. *Trap:* Letting the billed code stand in for documentation.
- **C** — The code requires a documented factor in each session. *Trap:* Billing add-on codes by client age.
- **D** — Diagnosis and time alone do not support the add-on. *Trap:* Omitting the session content that justifies billing.

Q6 — Answer: C. Set it up in session with Eli and his mother around a few target behaviors, check its privacy terms, and review data weekly

Why it is correct: Integrate the tool into the plan with privacy review. *Common error:* Recommending an app without setup or follow-up.

- **A** — Removing the parent defeats the parent-training purpose. *Trap:* Handing a child's behavior plan to the child.
- **B** — Tallying each act of misbehavior undercuts the positive reinforcement. *Trap:* Turning a reward tool into a misbehavior log.
- **D** — Unreviewed tools drift from the plan and risk privacy. *Trap:* Recommending a technology tool without any implementation support.

Q7 — Answer: A. Replacing escalating commands and inconsistent follow-through with clear expectations, consistency, and positive reinforcement

Why it is correct: Consistency and positive reinforcement. *Common error:* Leaving the coercive cycle in place.

- **B** — Giving in reinforces defiance. *Trap:* Rewarding escalation with capitulation.
- **C** — Blanket ignoring is not the plan. *Trap:* Assuming the pattern will self-resolve.
- **D** — Power battles escalate the cycle. *Trap:* Framing discipline as a contest to win.

Q8 — Answer: A. With consent, coordinate consistent expectations and supports across home and school given the cross-setting pattern

Why it is correct: Coordinate home-school consistency with consent. *Common error:* Addressing home while ignoring school.

- **B** — Disclosing without consent breaches confidentiality. *Trap:* Sharing information without authorization.
- **C** — Harsher punishment escalates the cycle. *Trap:* Recommending punitive escalation at school.

- **D** — Excluding school forgoes useful consistency. *Trap:* Refusing helpful, consented coordination.

Q9 — Answer: A. Validate how exhausting the conflict is and reframe parent training as a skill set, not a verdict on her worth as a parent

Why it is correct: Validate and reframe parent training. *Common error:* Either blaming or only sympathizing with the parent.

- **B** — Facts alone rarely shift the feeling. *Trap:* Answering emotion with numbers.
- **C** — Blaming the parent deepens shame. *Trap:* Assigning blame rather than building skills.
- **D** — Premature reassurance forgoes the work. *Trap:* Reassuring instead of engaging the work.

Q10 — Answer: B. Build rapport, find his strengths and goals, and involve him in age-appropriate problem-solving rather than only correcting him

Why it is correct: Strengths-based, collaborative engagement. *Common error:* Approaching the child only as a problem to correct.

- **A** — Confrontation invites more defiance. *Trap:* Leading with confrontation.
- **C** — A deficit-only focus disengages him. *Trap:* Centering only on deficits.
- **D** — Threats erode engagement. *Trap:* Using threats to motivate a child.

Q11 — Answer: C. Clarify at the outset, in age-appropriate terms, what will be shared with his mother and what stays private, within legal limits

Why it is correct: Set clear minor-confidentiality expectations up front. *Common error:* Leaving confidentiality expectations undefined.

- **A** — Absolute secrecy from the guardian is inappropriate. *Trap:* Guaranteeing total secrecy from the parent.

- **B** — Minors still have some privacy interests. *Trap:* Disregarding the minor's privacy entirely.
- **D** — Skipping the discussion undermines consent. *Trap:* Failing to set expectations.

Q12 — Answer: B. Follow mandated-reporting duties, assess safety, and make the required child-abuse report through proper channels

Why it is correct: Mandated reporting and safety first. *Common error:* Treating disclosed possible abuse as ordinary confidential material.

- **A** — Investigating yourself is not the counselor's role. *Trap:* Conducting your own investigation.
- **C** — Confidentiality yields to mandated reporting. *Trap:* Treating abuse disclosure as protected.
- **D** — Delaying a report is unsafe and improper. *Trap:* Postponing a required report.

Q13 — Answer: A. Seek consultation and training, or refer to a clinician competent in parent training and child behavioral treatment

Why it is correct: Consult, train, or refer within competence. *Common error:* Delivering specialized treatment without competence.

- **B** — Minimizing the problem fails the family. *Trap:* Underestimating the need for evidence-based care.
- **C** — Going it alone exceeds competence. *Trap:* Prioritizing continuity over competence.
- **D** — Abrupt transfer mishandles continuity. *Trap:* Dropping the family without coordination.

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